Treating Pathological Narcissism with Transference-Focused Psychotherapy



Diana Diamond, Frank E. Yeomans, Barry L. Stern, and Otto F. Kernberg



GUILFORD PRESS
e-book

TREATING PATHOLOGICAL NARCISSISM WITH TRANSFERENCE-FOCUSED PSYCHOTHERAPY

Psychoanalysis and Psychological Science

Elliot Jurist, Series Editor

Books in this series aim to bridge the work of researchers and the work of clinicians. They reflect the current empirical findings and state of the art in psychoanalysis and psychodynamic treatment. They are written to be practical and relevant to clinicians.

Attachment and Psychoanalysis:
Theory, Research, and Clinical Implications
Morris N. Eagle

Minding Emotions:
Cultivating Mentalization in Psychotherapy
Elliot Jurist

The Unconscious:
Theory, Research, and Clinical Implications
Joel Weinberger and Valentina Stoycheva

Treating Pathological Narcissism
with Transference-Focused Psychotherapy
Diana Diamond, Frank E. Yeomans,
Barry L. Stern, and Otto F. Kernberg

Treating Pathological Narcissism

with Transference-Focused Psychotherapy

Diana Diamond, Frank E. Yeomans,
Barry L. Stern, and Otto F. Kernberg

Series Editor's Note by Elliot Jurist



THE GUILFORD PRESS
New York London

Copyright © 2022 The Guilford Press
A Division of Guilford Publications, Inc.
370 Seventh Avenue, Suite 1200, New York, NY 10001
www.guilford.com

All rights reserved

No part of this book may be reproduced, translated, stored in a retrieval system, or transmitted, in any form or by any means, electronic, mechanical, photocopying, microfilming, recording, or otherwise, without written permission from the publisher.

Printed in the United States of America

This book is printed on acid-free paper.

Last digit is print number: 9 8 7 6 5 4 3 2 1

The authors have checked with sources believed to be reliable in their efforts to provide information that is complete and generally in accord with the standards of practice that are accepted at the time of publication. However, in view of the possibility of human error or changes in behavioral, mental health, or medical sciences, neither the authors, nor the editor and publisher, nor any other party who has been involved in the preparation or publication of this work warrants that the information contained herein is in every respect accurate or complete, and they are not responsible for any errors or omissions or the results obtained from the use of such information. Readers are encouraged to confirm the information contained in this book with other sources.

Library of Congress Cataloging-in-Publication Data is available from the publisher.

ISBN 978-1-4625-4668-8 (hardcover)

About the Authors

Diana Diamond, PhD, is Professor Emerita in the Doctoral Program in Clinical Psychology at the City University of New York and Senior Fellow at the Personality Disorders Institute at New York Presbyterian Hospital–Weill Cornell Medical College. She is also Adjunct Professor at the New School for Social Research and in New York University's Postdoctoral Program in Psychotherapy and Psychoanalysis. Dr. Diamond's primary interests are in psychodynamic psychotherapy for patients with personality disorders; the etiology, pathology, and treatment of narcissism; and clinical applications of attachment theory and research. Author or coauthor of numerous papers and several books, she is a recipient of the Research Award from Division 39 (Psychoanalysis) of the American Psychological Association and the Stern Distinguished Visiting Professorship Lecture award from Weill Cornell Medicine for contributions to furthering the understanding of narcissism. She serves on the editorial boards of *Psychoanalytic Psychology* and *Psychoanalytic Inquiry*, is Co-Vice President of the Margaret S. Mahler Foundation, and has a private practice in New York City.

Frank E. Yeomans, MD, PhD, is Clinical Associate Professor of Psychiatry at Weill Cornell Medical College, Director of Training at the Personality Disorders Institute at New York Presbyterian Hospital–Weill Cornell Medical College, and Adjunct Associate Clinical Professor of Psychiatry at the Columbia University Vagelos College of Physicians and Surgeons. Dr. Yeomans is an Honorary Member of the American Psychoanalytic Association, President of the International Society for

Transference-Focused Psychotherapy, and Chair of the Committee on Psychotherapy of the Group for the Advancement of Psychiatry. His primary interests are the development, investigation, teaching, and practice of psychotherapy for personality disorders. He has helped establish training programs in many countries and has published numerous articles and books.

Barry L. Stern, PhD, is Clinical Associate Professor of Psychology in Psychiatry at the Columbia University Vagelos College of Physicians and Surgeons and a faculty member at the Columbia University Center for Psychoanalytic Training and Research. Dr. Stern's work centers on the study of personality and mental health and the application of transference-focused psychotherapy with patients across the spectrum of personality functioning and with couples. He has been a lead investigator in the validation studies of the Structured Interview of Personality Organization (STIPO) and its revision (STIPO-R). His published empirical work focuses on the STIPO and the structural assessment of personality. He has a private practice in New York City.

Otto F. Kernberg, MD, is Professor of Psychiatry and Director of the Personality Disorders Institute at New York Presbyterian Hospital–Weill Cornell Medical College, Training and Supervising Analyst at the Columbia University Center for Psychoanalytic Training and Research, and Distinguished Visiting Professor of Psychiatry at the Albert Einstein College of Medicine. Dr. Kernberg is an Honorary Member of many international psychoanalytic institutes, past president of the International Psychoanalytical Association, and Distinguished Life Fellow of the American Psychiatric Association (APA). He is a recipient of the Distinguished Service Award from the APA, the first Sigourney Award for Distinguished Contributions to the Field of Psychoanalysis, the Edward A. Strecker Award from the Institute of Pennsylvania Hospital, and the Heinz Hartmann Award from the New York Psychoanalytic Society and Institute, among other honors. Dr. Kernberg has conducted research and published seminal books and articles on psychoanalytic psychotherapy, psychoanalytic object relations theory, and severe personality disorders.

Series Editor's Note

No one needs convincing that our society currently is facing the challenge of finding new ways to understand and to treat narcissistic pathology. Given its prevalence, it is imperative for mental health professionals to feel equipped to handle the range of disorders that fall under this category.

Transference-focused psychotherapy (TFP) was first developed as a treatment for borderline personality disorder by Otto Kernberg's group at the Personality Disorders Institute at New York Presbyterian Hospital–Weill Cornell Medical College. With this book, the TFP group, led by Diana Diamond and colleagues, applies TFP to the diagnosis and treatment of narcissistic pathology. Diamond and colleagues ground their approach in a contemporary version of object relations theory, which is influenced by recent work in social cognition, attachment theory, and mentalization theory. It is important to emphasize that their focus includes, but is *not* restricted to, the narrow, DSM definition of narcissistic personality disorder. Narcissistic pathology connotes the varying clinical manifestations of narcissism, from individuals who are high functioning to those functioning on a borderline level to those with malignant narcissism. Clinicians from all orientations need to be able to identify narcissistic pathology and to learn how to work effectively with this range of patients.

The most audacious aspect of Diamond and colleagues' book is to argue that it is possible to modify TFP and apply its principles, regardless of the clinicians' training and identifications, in encountering narcissistic patients. Extending TFP is especially valuable, as we have come to

appreciate the danger narcissism represents as a public health crisis. Clinicians who are reluctant about evidence-based approaches have something to gain from this book. Nonpsychoanalytic clinicians will benefit from the well-established, deep understanding of personality disorders presented here. Psychoanalytic clinicians will benefit from learning concrete steps to take in encountering patients with narcissistic disorders. Clinical researchers from all backgrounds will appreciate the balanced and nuanced effort to translate research in a pragmatic direction.

Along with mentalization-based therapy, TFP deserves recognition for pushing psychoanalysis beyond its traditional comfort zone, confronting the tension that is bound to arise when theory comes into contact with actual research findings. Indeed, it is especially heartening to encounter a psychoanalytic research agenda that depends upon a receptiveness to other fields of knowledge and a willingness to participate in and contribute to emerging dialogues across the spectrum of mental health. This book demonstrates the continuing value of psychoanalytic thinking, yet its ultimate merit is that it provides a beacon for all mental health professionals.

ELLIOT JURIST, PhD

Preface

An Object Relations Perspective on Narcissistic Personality Disorder

In 2007, I (D. D.) gave my first presentation on pathological narcissism at a conference entitled "The Pain of Narcissism and Its Psychotherapeutic Treatment." I came away from this experience determined to further develop a conceptualization and treatment approach for pathological narcissism, because the psychic pain of this increasingly prevalent group of patients is often not evident to others, including the clinicians who treat them. Narcissistic pathology is Janus-faced, in that the dominant self-presentation of narcissistic patients as supremely self-sufficient, self-absorbed, self-enhancing, and self-aggrandizing (even if only in fantasy) shields their excruciating sense of vulnerability, lacerating states of shame and self-doubt, emotional isolation, and loneliness. The conundrum of pathological narcissism and its potentially debilitating consequences was first captured in the myth of "Echo and Narcissus," the first case of pathological narcissism in recorded history. Initially recorded by Ovid in *Metamorphoses*, the myth has been a source of inspiration for psychoanalytical theorists from Freud (1914/1957) onward in comprehending the contours of narcissism and its pathological formations. Like most myths, "Echo and Narcissus" depicts universal unconscious fantasies and conflicts, which in their continued iterations and manifest forms connect the individual with fundamental human and relational strivings, anxieties, and dilemmas, distilling and revealing the essence

of character types.[1] We begin with the myth because it reveals the complex and protean nature of narcissism and narcissistic disorder—the extent to which narcissistic illusions may exalt the individual into states of self-importance and self-aggrandizement, only to plunge him or her into paralyzing states of shame, despair, and rage when shattered, and the extent to which overinvestment of love in the self and ideal others is often accompanied by states of severe self-criticism and self-torture.

Narcissus had a particularly unusual attribute—he was an extraordinarily attractive child—the product of the union of the nymph Liriope and the river god Cepheus. Concerned about whether her son would "live long / With such perfect beauty?," Liriope consulted the seer Tiresias, who replied enigmatically that her son would fare well, "unless he learns to know himself" (Hughes, 1997, p. 69). By the time he reached his 16th year Narcissus' "beauty had flowered, / But something glassy about it, a pride / Kept all his admirers at a distance" (pp. 69–70), although many male and female peers were infatuated by him. Particularly smitten was Echo, a nymph with a lovely body, but no capacity to speak other than to repeat the words of others. She followed Narcissus "like a starving wolf" (p. 71), but when she encountered him wandering in the forest and tried to embrace him, he recoiled from her caress with horror, telling her he would sooner be dead than allow her to touch him. Echo hid in the woods, brooding over the rejection until her body faded away and only her voice that endlessly repeated the words of others remained. Outraged by Narcissus' treatment of Echo, one of his many mocked and rejected suitors prayed, "Let Narcissus love and suffer / As he has made us suffer. / Let him, like us, love and know it is hopeless. / And let him like Echo, perish of anguish" (p. 73)—a prayer that was heard and granted by Nemesis, the corrector god.

One day, while quenching his thirst in a pool of water, Narcissus became enchanted by the reflection in the glassy surface. He mistook his image for that of a stranger with whom he became enchanted. Then as he leaned forward to gaze at this image and drink from the pool of water, "A strange new thirst, / a craving, unfamiliar, / Entered his body with the water, / And entered his eyes / With the reflection in the limpid mirror" (Hughes, 1997, p. 74). Mesmerized by his own image mistaken for a desired other

> He lay, like a fallen garden statue,
> Gaze fixed on his image in the water,

[1]The myth of Narcissus in Ovid's *Metamorphoses* has inspired multiple translations, the most recent of which is by the poet Ted Hughes (1997), whose translation gives the myth a more contemporary cast and which is the source of the included quotes.

Comparing it to Bacchus or Apollo,
Falling deeper and deeper in love
With what so many had loved so hopelessly.
Not recognising himself
He wanted only himself.
He had chosen from all the faces he had ever seen
Only his own. He was himself
The torturer who now began his torture. (Hughes, 1997, p. 74)

Initially, Narcissus was gratified by interacting with a beloved who perfectly mirrored his own gestures and expressions, laughing when he laughed and reaching out to him as he stretched his arms out for an embrace. But as he attempted to clasp and caress his beloved, only to see his image dissolve into a "shiver of ripples" (p. 74), he recognized that his ideal love was but an ephemeral reflection of himself, a fragile figment of his own imagination, rather than a separate being with his own corporeal reality. Narcissus laments, "I see through my own reflection. / But it is too late. / I am in love with myself. / I torture myself. What am I doing / Loving or being loved?" (p. 76). What seemed like a state of mutual recognition turned into a state of misrecognition of the self for the other. Further, the recognition that his ideal object and ideal self were one and the same precipitated a state of irreconcilable confusion about the boundaries between self and other. "Why can't I get apart from my body? / This is a new kind of lover's prayer. / To wish himself apart from the one he loves" (p. 76). In a state of rage and frustration, Narcissus ripped off his shirt and beat his breast and, finally consumed by his love, wasted away. His final words were "Farewell, you incomparable boy, / I have loved you in vain" (p. 77).

We have quoted from the myth to illustrate how it embodies multiple aspects of a contemporary object relations perspective on pathological narcissism and narcissistic disorder. Narcissus initially is infatuated with his own reflection, but when he realizes that the image has no existence separate from himself, he is plunged into despair and chooses death rather than separation from his ideal object, mistaken as another. Hence the Narcissus myth is a tale about how narcissistic pathology inevitably involves both disorders in self and interpersonal relating. In particular, the myth illustrates how excessive self-love or libidinal investment in the self may function as a distorting prism that deforms one's self-experience and capacity to live fully in reality rather than in the illusory world of the grandiose self. Initially, Narcissus embodies the arrogance, self-aggrandizement, callousness, and exploitativeness of the grandiose narcissist, but when he realizes that he can never possess or be truly loved by his ideal object that is merely a reflection of the self, the self-exaltation spirals into self-torture and even self-destruction. "What

can my courtship gain?" he laments, "What I want, I am. / But being all that I long for— / That is my destitution" (Hughes, 1997, p. 76).

In Ovid's version of the myth, Narcissus pines away and is transformed into a flower with a yellow center like an egg yolk—reminiscent of a return to an undifferentiated state—but in other versions, he plunges a dagger into his breast (Canon, Narrations, 24). Narcissus' escalating rage and disappointment, his self-flagellation (beating his breast), and wish for death when he is unable to possess his ideal image or to separate that image from himself illustrate the protean nature of pathological narcissism: how overinvestment in an ideal, illusory sense of a self merged with an ideal other remains fragile and prone to collapse, leading to "self-impoverishment nearly to the point of annihilation" (Green, 2002, p. 635; Kernberg, 1975, 2007, 2010; Ronningstam, 2011).

The myth also highlights the inability of those with pathological narcissism to tolerate separation, loss, grief, and mourning, including the loss of highly idealized views of self and others that belong to the early phases of development. In the myth, there is thus a regressive turning away from the sensual corporeal reality of others (hence the rejection of Echo's embraces) into an intoxicating union with an idealized figure—a reflection reminiscent of an early idealized bond—that remains immune to reality constraints (Tutter, 2012).

In sum, the myth prefigures several important aspects of an object relations view of pathological narcissism, including (1) the inability to sustain love relations in depth that may leave the individual tragically isolated despite desperate and compulsive seeking of admiration and approval from others; (2) the exploitative, entitled attitudes toward others who are either spurned as the repository of all that is objectionable in the self or appropriated and idealized as perfect mirrors for the self; (3) how grandiose self-states coexist with or devolve into vulnerable self-states for those with pathological narcissism; and (4) how, when unrealistic expectations of self and others are disappointed, there is often an intensification of aggression toward self and others, leading to rageful attacks on self and others or to a gradual decline toward self-destruction (Kernberg, 1997; Ronningstam, Weinberg, & Maltsberger, 2008).

It is interesting that at the time that Ovid was writing, the myth of Narcissus was a minor one, while in our narcissistic age it has achieved major salience. Given the growing number of individuals with pathological narcissism and full diagnostic narcissistic personality disorder (NPD), as well as narcissistic traits in our society—the rampant comorbidity of NPD with other disorders; the different manifestations of pathological narcissism at transitional points in the life cycle, particularly young adulthood and midlife; the heterogeneity of patients with narcissistic pathology, and the treatment complications they pose,

including high dropout rates—the development and refinement of treatment approaches for narcissistic disorder across the spectrum of personality functioning is necessary and timely.

In this book, we present a contemporary object relations conceptualization of narcissism and its disorders that integrates psychoanalytic theory with recent developments in neurocognition, social psychology, descriptive psychiatry, and developmental attachment theory—an approach that reflects the work over the past 30 years of our colleagues in the Personality Disorders Institute at New York Presbyterian Hospital–Weill Cornell Medical College. We integrated the various theoretical and technical influences that have shaped each of us, including the neo-Kleinians (Britton, 2004a, 2004b; Feldman, 2009; Joseph, 1959; Steiner, 2011) who have described the psychoanalytic approaches to narcissistic pathology and the unique combination of high-functioning and primitive mental states that characterize them. We also integrated the attachment theorists (Ainsworth, Blehar, Waters, & Wall, 1978; Bowlby, 1977, 1988; Main, Kaplan, & Cassidy, 1985; Main, 1991) who have identified the insecure and disorganized attachment representations that characterize those with pathological narcissism, as well as those who work within the conceptual framework of mentalization who have identified the deficits in imagining and understanding the intentional mental states of self and others and the epistemic hypervigilance that characterize patients with personality disorders (Bateman & Fonagy, 2016; Fonagy, Gergely, Jurist, & Target, 2002; Fonagy & Target, 1997; Fonagy & Allison, 2014). In addition, we included the contributions of the relational school (Aron, 2006; Bass, 2006; Bromberg, 1983; Harris, 2011; Greenberg, 2001) who have emphasized the unique quality of each patient–therapist dyad and the co-constructed nature of transference and countertransference. Although we incorporate the work of other theoretical traditions, our approach is most reliant on the work of the North American object relations theorists, particularly coauthor Kernberg (1975, 1976, 1984, 2007). This work integrates aspects of ego psychology and contemporary Freudian drive-based perspectives with attachment research and neurobiology that expand the understanding of fundamental motivational systems that contribute to personality functioning and development. This book thus represents our collective effort to refine and implement an intensive psychodynamic therapy with a population of patients with a challenging disorder that manifests itself in disparate ways.

In general, the development of techniques for treating those with narcissistic pathology has not kept pace with the proliferation of theories about its etiology, the social factors that might be promoting it, the descriptive criteria through which to identify it, the social-cognitive

deficits that characterize it, and the varied presentations of the disorder. More importantly, while there are a number of complex and rich theories of narcissism, both normal and pathological, it is not always clear how such theories are translated into treatment modalities. We hope this clinical guide will be used to inform further reflection and research on the treatment of individuals with narcissistic pathology, across the spectrum from narcissistic traits to fully developed NPD.

Acknowledgments

This book represents the culmination of over 25 years of clinical work and research with a broad spectrum of narcissistic patients conducted at the Personality Disorders Institute (PDI) at Weill Cornell Medical College. We want to first express our profound gratitude to the two directors of the PDI, Drs. John Clarkin and Otto Kernberg, whose leadership has continually catalyzed the creativity of its members. We also gratefully acknowledge our colleagues at Weill Cornell who have made invaluable empirical, clinical, and theoretical contributions to understanding and treating patients with pathological narcissism and NPD, including Drs. Jordan Bate, Eve Caligor, Nicole Cain, Monica Carsky, Jill Delaney, Karen Ensink, Eric Fertuck, Kenneth Levy, Catherine Haran, the late Paulina Kernberg, Lina Normandin, Mark Lenzenweger, Kevin Meehan, Robert Michels, Michael Stone, and Alan Weiner. We also thank our colleagues in the International Society for Transference-Focused Psychotherapy (ISTFP), including Drs. Anna Buchheim, Peter Buchheim, Sergio Dazzi, the late Gerhard Dammann, Chiara De Panfilis, Rosella Di Pierro, Stephan Doering, Nel Draijer, Miguel Angel Gonzalez-Torres, Mathias Lohmer, Fabio Maddedu, Emanuele Preti, Susanne Hörz-Sagstetter, Irene Sarno, Agnes Schneider-Heine, Julia Sowislo, and Luis Valenciano, many of whom have contributed significantly to advancing the study of pathological narcissism. The intense collaborative work with our colleagues in the PDI and the ISTFP have helped bring this book to fruition.

Our deepest appreciation is due also to our primary editor at The Guilford Press, Jim Nageotte, and to our series editor, Elliot Jurist.

Both gave unique and valuable feedback on the manuscript, and showed extraordinary patience and perseverance in bringing this volume into existence. We are grateful also for the editorial assistance of Drs. Jack Keefe and Andrea Fortunato, and to Vesna Bosic, MS.

We also acknowledge the support of our past and current chairs in the Department of Psychiatry, Drs. Jack Barchas and Francis Lee, for their interest in and support of our work.

We thank our students for all the ways that teaching them has enriched and expanded our ideas.

We thank our patients, whose bravery in confronting and sharing their inner lives gave us the courage to create this book.

Finally, we have had the extraordinary good fortune to have partners who have inspired us in love and in work, and have shown caring and forbearance in the face of our immersion in this project. We dedicate this book to John Alschuler, Carlos Garay, Deborah Shapira, and Kay Haran.

Contents

1. Treating Pathological Narcissism with Transference-Focused Psychotherapy: An Introduction 1

PART I. Conceptualizing Pathological Narcissism

2. Self-Functioning in Pathological Narcissism 39

3. Interpersonal Functioning in Pathological Narcissism 96

4. An Overview of TFP-N 142

PART II. Treating Pathological Narcissism

5. The Assessment and Diagnosis of Pathological Narcissism and NPD 197

6. The Treatment Contract and Frame 219

7. TFP-N Early Phases 251

8. TFP-N Later Phases 286

9. TFP-N for Patients with Combined Narcissistic and Borderline Pathology 325

PART III. Pathological Narcissism in Intimate Relationships and Society

10. Malignant Narcissism: Clinical and Social Aspects 357

11. Narcissistic Love Relations 393

12. Epilogue 409

References 417

Index 449

CHAPTER 1

Treating Pathological Narcissism with Transference-Focused Psychotherapy

An Introduction

A high-functioning narcissistic patient began treatment by saying, "I didn't learn anything about myself that I didn't already know in my previous four years of therapy, and I expect that will be the same in this treatment . . . I just want someone to hold my hand while I decide whether I should end my relationship." After several years of outpatient treatment, another formerly highly suicidal patient said, "I'm better but I can't admit it because your success is my failure." Such statements taken from intensive and ultimately productive treatments highlight the formidable clinical challenges posed by narcissistic people across the spectrum of functioning from high functioning to more severely disturbed. High-functioning narcissistic patients may present as socially engaging, charming, and successful in politics, the arts, finance, or other fields, but their grandiose self-presentation often masks the underlying vulnerability to dysregulating states of fear, rage, and despair in the face of threats to self-esteem. By contrast, more disturbed narcissistic individuals may appear on the surface as anxious, submissive, shy, and self-defeating, often withdrawing from both intimate relating and occupational challenges to preserve their fragile and often hidden sense of specialness or uniqueness. Whether this self-pathology is expressed overtly in self-aggrandizing behavior or covertly in grandiose beliefs and fantasies that the individual may be too shame ridden to reveal, the patient's difficulties allowing for a healthy dependency upon and attachment to the therapist, as well as difficulties in tolerating interpretive work that challenges rigid defenses and invites reflection on inner

turmoil and distress, often secreted and unseen, pose significant technical challenges (Behary & Davis, 2015; Stone, 1989; Kernberg, 2007, 2018; Clemence, Perry, & Plakun, 2009; Diamond, Yeomans, & Levy, 2011; Gabbard & Crisp, 2018; Ronningstam, 2018). In therapy, many individuals with pathological narcissism present as self-aggrandizing, arrogant, entitled, exploitative, and unempathic, relentlessly devaluing others, yet craving the admiration of denigrated others, including the therapist. Paradoxically, others with the disorder appear as self-effacing, introverted, and inordinately vulnerable to rejection or criticism, particularly by the therapist, whom they tend to idealize. These contradictory patient profiles may tax the skills of even the most experienced clinicians, engendering powerful countertransferential responses of feeling incompetent, bored, or disparaged in the face of the patient's devaluation, or at the other extreme, being beguiled or even captivated by the experience of being massively and unnervingly idealized (Betan, Heim, Zittel Conklin, & Westen, 2005; Gabbard, 2009; Kernberg, 1975, 2007; Kohut, 1971, 1977). Indeed, the countertransference pressures that individuals with pathological narcissism may generate in their treaters often result in stalemates, enactments, premature or precipitous terminations, or the endless prolongation of treatments with little improvement.

Perhaps because of the daunting treatment challenges they pose, theorists from almost every major psychodynamic approach have written on the nature of pathological narcissism and/or narcissistic personality disorder (NPD). These approaches include *classical and contemporary Freudian* (Akhtar & Thompson, 1982; Cooper, 1998; Ellman, 2014; Gabbard & Crisp, 2018; Ronningstam, 2000, 2005, 2012, 2016), *Kleinian and neo-Kleinian* (Britton, 1989, 2004a; Feldman, 1997, 2007, 2009; Joseph, 1959, 1989), *self-psychology* (Kohut, 1971, 1977; Lachmann, 2007; Lichtenberg, 1988), *interpersonal* (Fiscalini, 1994), North American *object relations* (Bach, 1985; Kernberg, 1975, 1984, 2010, 2018), and the *relational school* (Bromberg, 1983; Cooper, 1998; Mitchell, 1986; Shaw, 2013). Interest in narcissism is now being generated among *cognitive-behavioral* clinicians (Behary & Davis, 2015; Behary & Dieckmann, 2011; Cukrowicz, Poindexter, & Joiner, 2011), as well as those who work in the framework of *attachment-* and *mentalization-based* treatments (Unruh & Drozek, 2020; Bateman & Fonagy, 2016). These divergent approaches to understanding narcissism and narcissistic pathology have fueled among the greatest controversies in psychodynamic thinking from Freud to the present. But while they may differ in theoretical and treatment approaches, the majority of clinicians and clinical researchers concur that the current DSM NPD categorical formulation, which emphasizes grandiose behaviors and attitudes, is inadequate to understand the complex dynamics of the broad spectrum of

patients who suffer from pathological narcissism and its often devastating consequences in their work and relational lives (Paris, 2014; Pincus, Cain, & Wright, 2014; Ronningstam, 2009, 2011, 2012, 2014, 2016; Wright, 2016). However, relatively few have translated their theories or research findings into systematic, empirically informed approaches to assessment and therapy that address the specific challenges posed by individuals with pathological narcissism.

In this book, we offer a conceptualization of narcissism, both normal and pathological, based on contemporary object relations theory. We begin with a detailed description of pathological narcissism and its underlying structure, providing clinical examples, as well as research findings from social cognition, attachment, and neurobiology in support of this model. We view pathological narcissism as a form of personality disturbance that may be present in individuals across a spectrum of functioning, from the cusp of neurotic to borderline to malignant narcissism, the most severe form of the disorder where patients are at the edge of treatability.

The concepts of pathological narcissism and NPD are often used interchangeably to refer to narcissistic pathology, and they overlap to some extent. *Pathological narcissism* refers to a broad range of conditions that include the more specific NPD. Rather than conceptualizing NPD as a categorical diagnosis, we offer a developmental dimensional model of pathological narcissism which exists at different levels of severity and can encompass diverse presentations (Aslinger, Manuck, Pilkonis, Simms, & Wright, 2018). This approach is consistent with transdiagnostic and dimensional models of psychopathology (Haslam, Holland, & Kuppens, 2012; Sharp & Wall, 2021). Individuals with pathological narcissism may present with an array of varied and sometimes incongruent symptoms, and show a range of disparate characterological features from grandiose to vulnerable; from self-aggrandizing to self-effacing; from socially dominant to socially withdrawn; from histrionic or self-dramatizing to masochistic or self-defeating; and from depressive and excessively self-critical to callous, dishonest, and even antisocial. Furthermore, pathological narcissism encompasses patients who function at a different level of personality organization (e.g., neurotic, borderline, and psychotic, as described below). In our view, the terms *pathological narcissism* and *narcissistic personality disorder* go beyond the narrow DSM-5 descriptive criteria that privilege the overt attitudes and behaviors associated with grandiosity (e.g., exaggerated sense of self-importance, exploitative behaviors, lack of empathy; American Psychiatric Association, 2013). These criteria have allowed for standardized assessments for research purposes, and some of the research studies in this book are based on clinical and nonclinical groups with

DSM NPD diagnostic criteria, while others are based on those with narcissistic traits. Our object relations conceptualization of pathological narcissism encompasses DSM descriptive criteria as only one presentation of a multifaceted or pleomorphic (Gabbard & Crisp, 2018) disorder. What unites these conditions are core structural features, most notably a pathological grandiose self-structure: a specific configuration of internalized, idealized representations of self and others (described below), of which the disparate descriptive criteria that characterize the disorder are but the surface manifestations.

The focus of this book is on how the object relations model, which postulates certain core structural features of the disorder, has been translated into a therapeutic approach applicable to a broad spectrum of narcissistic patients. The development of such treatment approaches is crucial, since it is increasingly recognized that NPD represents "a disabling and major public health problem in its own right" (Pulay, Goldstein, & Grant, 2011, p. 167). NPD is associated, as are all personality disorders, with clinically significant distress and functional impairment in the areas of occupational performance and interpersonal relationships, particularly intimate or couple relationships (Grant et al., 2004). Individuals with NPD now comprise 1.3–17.0% of clinical groups (Clarkin, Levy, Lenzenweger, & Kernberg, 2007; Ronningstam, 2009) and 8.5–20.0% of patients seen in outpatient practice (Zimmerman, Rothschild, & Chelminski, 2005; Bodlund, Ekselius, & Lindström, 1993). The prevalence of NPD may be particularly high in outpatient private practice. In several independent surveys 30.0–76.0% of clinicians reported treating narcissistic patients (Westen & Arkowitz-Westen, 1998; Doidge et al., 2002; DiGiuseppe, Robin, Szeszko, & Primavera, 1995).

Narcissistic traits, if not a full NPD diagnosis, appear to be on the rise in the general nonclinical population, particularly among adolescents and young adults. According to one study, the level of narcissism among American university students (as assessed by self-report measure, the Narcissistic Personality Inventory [NPI]; Raskin & Hall, 1981) has been escalating since 1980 with an elevated level of specialness and entitlement comparable to that of celebrities (Foster, Campbell, & Twenge, 2003; Twenge, Konrath, Foster, Campbell, & Bushman, 2008b; Twenge, Miller, & Campbell, 2014). In a community study, young adults in their 20s were found to have three times the rate of NPD as were those over 65 (Stinson et al., 2008). Narcissism, as a construct with meaning beyond the consulting room, has entered our social and political discourse in unprecedented levels since 1979, when social theorist Christopher Lasch (1979) first described our society as a "culture of narcissism." More recent investigations by social psychologists have identified a "narcissism epidemic" based on a systematic rise in narcissistic traits among college students over the past 3 decades. These traits include unrealistically

inflated self-esteem, entitled expectations about careers, investment in self-development over connection to others, and indifference to the welfare of others (Foster et al., 2003; Twenge, Konrath, Foster, Campbell, & Bushman, 2008a, 2008b). Psychoanalysts, such as Aaron Stern (1979), have noted an intensification of conflict in young adult patients between the opportunities for an exaggerated sense of self-enhancement and self-adulation offered by a narcissistic culture, and the need for interdependence and mutuality that inevitably puts restraints on the pursuit of gratification of narcissistic needs. These findings invite further dialogue between social theorists and practicing clinicians.

Sociocultural trends, including the culture of competitive individualism, the breakdown of extended family structures that leave the individual unmoored without deep and enduring kin networks, and the lure of social media as a vehicle for both self-aggrandizement and transient, superficial connections with others, may promote narcissistic traits in the general population. The ubiquitous use of tweeting, texting, Instagram, Facebook, and reality TV fosters self-promotion and self-aggrandizement, along with instant, if distal, access to others, leading to superficial, often transient and shallow, encounters. Face-to-face communication that might encourage deeper and more enduring levels of intimacy is curtailed; both self and relationships are constructed and conducted on the proscenium of social media. These social trends have set the stage for a new cultural experience of self, offering unprecedented opportunities for self-aggrandizement; promotion of inflated, magnified, or distorted self-images; and ephemeral connections with others on the one hand (Twenge & Campbell, 2009), but also a multiplicity of opportunities for self-definition and self-expression on the other (Blatt, 1983; Diamond, 2006). While there is clearly a continuum between dominant personality traits and forms of psychopathology of a given era, between individual and collective narcissism, the precise relationship between the rise of narcissistic traits in the nonclinical population and the development of clinically significant pathological narcissism or NPD, which involves a specific structuring of the personality, remains in question. Indeed, a recent study has shown a decrease in the characteristics of vanity, entitlement, and leadership among college students (Wetzel et al., 2017), even while rates of narcissistic pathology appear to be on the rise among clinical groups (now from 1.3 to 17.0% of outpatients; Clarkin et al., 2007; Zimmerman et al., 2005; Ronningstam, 2018). Thus, it has become increasingly important to define the contours of pathological narcissism as a clinical diagnosis; identify its continuities and discontinuities with narcissistic traits in the general, nonclinical population (Diamond, 2006; Paris, 2014; Perry, 2014; Kealy & Ogrodniczuk, 2014); and develop approaches to assess and treat narcissistic pathology, which involves a specific maladaptive structuring of the personality.

Narcissism: Normal (Adaptive) and Pathological

Narcissism is a core psychological construct, and perhaps one of the most ubiquitous and perplexing because it may take both normal and pathological forms. From a traditional psychoanalytical perspective, narcissism involves investment in the self of both positive, loving feelings or libido (Freud, 1914/1953) and negative, hostile feelings or aggression, and thus may range from healthy self-assertion to self-abnegation (Green, 2002). From an object relations perspective, normal narcissism involves not just investment in the representation of the self but in a self that contains valued representations of others (referred to as *objects*). In psychodynamic thinking, the object refers to an internal mental representation that encompasses the actual other person but is also subject to modification by affect, impulse, wish, and fantasy. Normal or adaptive narcissism involves positive self-regard based on realistic self-appraisal. It involves pleasurable self-affirmation established and sustained through the individual's internalization of an accretion of positive, affirming experiences with others that promote stable and robust self-esteem regardless of the vicissitudes of life and relationships. With adaptive narcissism there is a sense of mastery and agency to realize the needs for self-realization and validation from others (Pincus & Roche, 2011; Kernberg, 2018; Ronningstam, 2016). The wish to shine, enhance the self through realistic goals and aspirations, be admired and respected by others, and live up to one's ideals and values are universally recognized as adaptive narcissistic strivings (Kohut, 1977; Kernberg, 1975), but from an object relations perspective, healthy adaptive narcissism has a specific meaning. Adaptive narcissism is a correlate of an integrated self-experience in which representations of self and other are clearly differentiated; positive and negative affects are well integrated and modulated; and the ideal self, which embodies the goals and aspirations one has, are commensurate with the individual's actual capacities and talents. Adaptive narcissism thus involves a sense of continuity and integration of myriad aspects of self-experience across time that sets the stage for a coherent and stable sense of identity that in turn allows for emotional engagement and investment in in-depth relationships with others.

From an object relations perspective, both an integrated self and the capacity for relatedness are built on a rich internal world of mental representations of gratifying relationships with others that shield one from the sense of emptiness, depletion, and futility, and/or the relentless striving for adulation often experienced by those with pathological narcissism. In sum, healthy narcissism is essential for adaptive self and interpersonal functioning. This is evident in the gratification and investment in realistic ambitions and achievements; in vital relationships that blend self-enhancement with love for others as unique and valued

individuals separate from the needs of the self; in an integrated system of ethical principles that encompasses both individual values and culturally sanctioned standards, while preserving a tension between the two that leaves room for independent thought and action separate from group norms; and in dedication to goals and ideals that guide work, creative, and/or intellectual pursuits. The concept of narcissism thus encompasses normative strivings for perfection, mastery, and wholeness, as well as pathological, defensive distortions of these strivings.

Pathological narcissism by contrast involves extreme, rigid needs for self enhancement and validation from others with fluctuations in self-esteem and emotional dysregulation in the form of angry outbursts, emotional withdrawal, or detachment from others when such needs are not met. Although most narcissistic individuals have tenuous self-esteem, due to a lack of integration of positive/idealized or negative/devalued aspects of experience, narcissistic pathology exists at different levels of severity. At the higher levels are individuals with non-specific self-centeredness on the cusp of neurosis who maintain a surface presentation of appropriate functioning in work and social life, and are able to sustain stable, but superficial, love relations. At the more severe levels are those who have pervasive defects in their capacity for work or profession and an inability to invest in love relations beyond transient sexual involvements. These individuals also demonstrate more aggressive reactivity to threats to their self-esteem. Whereas healthy narcissism is a component of an integrated self-experience and integrated concept of significant others, pathological narcissism involves investment in an internal world combining alternately highly idealized and devalued representations of self and others with the devalued aspects of self continually projected onto others. Consequently, others are not seen realistically as multifaceted and unique but in terms of the projected devalued or idealized aspects of self. Such a bifurcated experience of self and others creates fluctuations in self and affect regulation since the individual oscillates between a positive, exalted sense of self as superior and special, and a negative, devalued sense of self as inadequate and worthless. There are not integrated and differentiated mental representations to anchor the self in the face of the inevitable vicissitudes in life and relationships. These discontinuities in the experience of self and significant others lead to disruptions in the sense of identity and capacity to sustain relationships. Pathological narcissism thus evolves from a complex, dynamic spiral of self and relational processes—the precise strands of which may vary from individual to individual or within the individual over time. For example, many individuals with narcissistic pathology hold a view of the self as superior and others as devalued, although they may be quite charming and adept at eliciting the admiration and adulation they crave from others deemed inferior, or they may inordinately

idealize others and present the self as humble, self-effacing, insecure, shy, and subservient. But in either case, the complexity of positive and negative aspects of self and others is eliminated and the capacity for self-reflection or self-observation is limited.

Many individuals with narcissistic pathology have arranged their work and personal lives to garner the admiration and adulation they crave, but over time they have difficulty regulating their negative emotions when they are unable to obtain the veneration from others on which their psychic equilibrium depends. This may lead to increased antagonism toward others who then withdraw their positive regard thereby increasing the insularity and isolation of the narcissistic individual. Consequently, narcissistic grandiosity is inherently unstable, poised to shift into states of narcissistic vulnerability, which leaves the individual at risk for being overwhelmed by the negative affects of anger, envy, and fear. With more disturbed individuals, severe self-loathing and shame may ensue, leading to escalating and highly lethal destructive and self-destructive actions (Ansell et al., 2015; Blasco-Fontecilla et al., 2009). Indeed, for those with narcissistic pathology, the grandiosity and dismissing devaluation of others may mask a sense of "emptiness and futility," or at worst, "a hungry, enraged, empty self, full of impotent anger" (Kernberg, 1975, p. 633).

Individuals who suffer from pathological narcissism often present in contradictory ways, with their considerable strengths (verbal fluency, social competence, self-reliance) often obscuring their internal distress and suffering, which they have difficulty revealing or sometimes even experiencing consciously. Subjective feelings of distress may range from episodic feelings of worthlessness, emptiness, loneliness, and incompetence to devastating feelings of shame, isolation, crippling envy, and overwhelming self- or other-directed hostility—aspects of the vulnerable self-states to which these individuals are prone, especially in the face of threats to self-esteem, status, or control. At the more disturbed extreme, when the individual has antisocial or psychopathic features, patients with narcissistic pathology can also present with extreme levels of hostility and vengefulness, as is the case in those with the syndrome of malignant narcissism.

Thus, pathological narcissism may have varied presentations beyond the narrowly focused DSM-5 criteria, and may exist at different levels of severity depending on the underlying personality organization of the individual. At the least severe level, pathological narcissism may be evident in certain personality features, such as grandiosity, exaggerated self-importance, shame proneness, antagonism, entitlement, and/or exploitativeness (Miller, Lynam, & Campbell, 2016; Miller, Lynam, Hyatt, & Campbell, 2017). At this level, these features may be transient and catalyzed in certain situations, social contexts, or developmental

periods over the life cycle and do not necessarily involve the pathological self-structure characteristic of NPD. At the more severe end of the spectrum, pathological narcissism may manifest as an enduring personality disorder in which individuals are characterized by rigid, maladaptive, and problematic patterns of experiencing self and others that derive from a pathological grandiose self-structure. In sum, narcissistic pathology can manifest in multiple and often contradictory presentations and exist at different levels of severity, both within the individual at different points in time, and between individuals who have different levels of psychological organization. This complexity adds to the difficulty in diagnosing and treating narcissistic persons.

An Object Relations Approach to Treatment of NPD

Our treatment model is transference-focused psychotherapy (TFP), a manualized evidence-based therapy for individuals with borderline personality disorders (BPDs), including many with NPD (Clarkin et al., 2007; Doering et al., 2010). The model has an integrated object relations theoretical framework developed by the members of the Personality Disorders Institute (PDI) at Weill Cornell Medical College from their experience in treating a range of patients with narcissistic pathology at different levels of personality organization, from high-functioning to more severely disturbed individuals. In developing our treatment approach to a spectrum of narcissistic patients, including those with NPD, we have been working on two fronts. First, through in-depth clinical discussion in a weekly supervision group conducted over the past 10 years we followed a number of cases of individuals with pathological narcissism being treated with TFP. We started the supervision because we found that patients with narcissistic pathology posed unique challenges including treatment stalemates and/or prolonged nonproductive treatments, or complex intense countertransference reactions including feelings of anger, hopelessness, or idealization on the part of even the most seasoned therapists. To find out more about these patients, we turned to data from our randomized clinical trials (RCTs), which allowed the opportunity to investigate the characteristics and course of treatment of patients with co-occurring narcissistic and borderline personality pathology. TFP was developed originally for patients with BPD—however, our experience conducting TFP with a range of patients with narcissistic pathology, including those with co-occurring narcissistic and borderline pathology (NPD/BPD) in our research studies, along with research findings described later in this book, all suggest that TFP with some refinements to the technique is useful for treating individuals with pathological narcissism including specific NPD.

NPD and pathological narcissism in the broader sense have been subject to a great deal of attention from clinicians and clinical researchers but, with few exceptions (Diamond et al., 2013; Diamond & Hersh, 2020; Gabbard & Crisp, 2018; Ronningstam, 2020; Kernberg, 1975, 1984, 2018; Kohut, 1971, 1977; Unruh & Drozek, 2020), the development of systematic treatment approaches for patients with pathological narcissism across the spectrum of levels of organization have not kept pace. Consequently, the working clinician may be confronted with an almost dizzying array of theories and research on all facets of narcissistic pathology—from neurobiology to etiology to personality traits—but may have little idea how these translate into actual clinical approaches or how such approaches may be tailored for the range of narcissistic spectrum symptomatology. Significantly, the first delineation of narcissistic pathology derived from clinicians' observations of the ways that pathological narcissism influenced aspects of the psychotherapy process and outcome. Their observations included narcissistic patients' difficulty in forming a working alliance or attachment to the therapist, or even in engaging in the treatment process at all; their allergic response to interpretation; and particular maladaptive transference–countertransference patterns that evolved in the course of treatment (Ellison, Levy, Cain, Ansell, & Pincus, 2013; Hilsenroth, Holdwick, Castlebury, & Blais, 1998; Stern, 1938; Kernberg, 1975, 1984; Kohut, 1971, 1977; Bach, 1985; Bromberg, 1996; Gabbard, 1989; Ronningstam, 2012, 2018). More recently there has been a groundswell of interest in the development of psychotherapies specifically targeted for pathological narcissism and NPD (Kealy & Ogrodniczuk, 2011, 2014; Ronningstam, 2016, 2018; Drozek & Unruh, 2020; Weinberg & Ronningstam, 2020) based on treatments for personality disorders with which NPD is highly comorbid, such as BPD, but few of these treatments are based in evidence (Paris, 2014) and/or described systematically in treatment manuals (Yeomans, Clarkin, & Kernberg, 2015). Although the research findings on the efficacy and effectiveness of TFP are for BPD only, it is a therapy developed for patients with personality disorders across the spectrum of severity and diagnoses, including those with narcissistic pathology, although they may present somewhat differently from patients with BPD.

People with NPD have been distinguished from those with other personality disorders by specific factors, such as arrogance or self-importance, grandiosity, envy, and excessive need for admiration (Sharp et al., 2015)—all of which are ways to cope in the face of threats to self-esteem. Individuals with narcissistic pathology may show more stability in their sense of self, which may allow for high levels of functioning in work or creativity, but this is often in glaring contradiction to the greater degree of disturbance in interpersonal and especially intimate

relationships. Like those with BPD, they may rely on splitting-based and dissociative defenses. However, this is less evident because of the tendency to rigidly and fixedly project negative aspects of their own experience onto others and immediately appropriate all that is good in others to the self (introjection) to bolster self-esteem and ward off challenge to narcissistic equilibrium. In addition, individuals with pathological narcissism tend to deny or discount aspects of reality that do not affirm their often inflated self-concept, leading more severely disturbed individuals to grossly deficient reality testing in which whole sectors of reality that do not conform to grandiose self-concept are discounted or even denied.

These characteristics may be more or less severe depending on the level of organization and the presence of other co-occurring characterological features or personality disorders. Perhaps most central to the object relations perspective on which TFP is based is the emphasis that individuals with pathological narcissism may span the spectrum of personality organization from higher functioning to low-level borderline. This understanding sees narcissistic pathology in dimensional terms. Having said that, our object relations model of pathological narcissism is based on a particular psychological structure—the pathological grandiose self—which introduces a categorical element to our thinking. Our conceptualization of this structure is a heuristic tool that helps define a particular manifestation of pathological narcissism and how to understand it and approach it therapeutically.

Case Examples

The following case vignettes illustrate the range of individuals with narcissistic pathology across the spectrum of functioning encountered in clinical practice. All cases are composites to protect confidentiality.

Case Example: Mark—High-Functioning Narcissistic Disorder

A renowned 40-year-old architectural engineer, Mark began therapy stating that he did not expect to learn anything about himself that he did not already know but just wanted someone to hold his hand while he dealt with a crisis in his relationship. Mark was torn between a young woman with whom he had become enamored in cyberspace and his long-term on-again, off-again girlfriend, who was threatening to end their relationship if he did not make a more enduring commitment and stop his Internet liaisons, of which she had recently become aware. In this dilemma, Mark became severely anxious and sought further treatment. He expressed feelings of futility about ever being able to sustain

sexual interest in a woman for the long term and seemed driven to seek out ideal fantasy figures that embodied a sense of perfection that he himself aspired to. Mark had been referred by his former CBT therapist, who felt that he needed more intensive therapy to work through his personality difficulties, including his lingering performance anxiety despite his success at work; his inability to commit to an enduring, intimate relationship; and his intermittent binge drinking. Although he reported transient infatuations with women he met socially or on the Internet, the attraction of these idealized fantasy figures withered when he met them in person and began to make demands on him for an ongoing mutual relationship.

When he began treatment, Mark was in the grip of a conflict between his current partner and a woman he had met on the Internet (whom he described as a paragon of perfection). He believed that being with such a woman would make him the target of envy, yet he feared that he would become disillusioned should he actually embark on a real relationship with her. Although he felt little sexual attraction to his current girlfriend, he could not imagine his life without her and the sense of comfort and security she provided. Yet he refused to commit to having children with her for fear it would curtail his career or close off other relational options. He chronically devalued her as unworthy, not accomplished, or not attractive enough, even though she was from an affluent socially prominent family that had elevated his own social status. Chronically dissatisfied, he could not bring himself to end the relationship, and felt baffled by his inability to do so. Mark was at the top of his profession and a partner in his firm, but he also presented with difficulties at work that stemmed from lingering debilitating performance anxiety and self-doubt about his creative abilities. He experienced periodic bouts of depression and anxiety when his designs, which were sometimes spectacular but impractical, were criticized by his coworkers or clients. He was riding on his past accomplishments and accolades but in fact had produced little new work for the past 3 years. As a result, his client base was dwindling, where before he had turned many away, and he was unwilling to search out new clients when the economic situation led to retrenchment in his company. Mark was a devoted mentor to the junior people in his firm but would withdraw support when they became successful in their own right. On the other hand, he was extremely generous to his few close friends, and had funded an engineering scholarship at his university designed for students in the Asian countries where he had worked.

Mark's father, a successful artist, was a "mythic figure" whom the patient felt he could never live up to and who demeaned architecture as a second-rate profession, while he publicly claimed credit for his son's

success at awards ceremonies. Mark described his mother, a successful art dealer, as a stunning narcissist who skimmed the surface of life and never went beyond that. He portrayed his parents' marriage as a perfect union, an ideal relationship that he could never attain. The parents had traveled a great deal during his childhood due to the father's involvement with lecturing and exhibition tours, and he was often left with a nanny or grandmother who lived with the family.

Indeed, Mark presented initially with features of a narcissistic personality organized at a higher level. His sense of exaggerated self-importance, need for admiration, and sense of entitlement were combined with a reasonably stable sense of identity that allowed for significant achievements in work and for long-term, albeit superficial and unsatisfying, relationships. His need to be special and always in the limelight at work left him vulnerable to self-criticism and to the criticism of others. His sense of self was buttressed by real talents and accomplishments, but he remained overly reliant on the valuation of others for self-definition and self-esteem regulation. His capacity for a deeper investment in an intimate relationship was limited since others were not seen realistically as autonomous beings separate from his narcissistic needs. Consequently, Mark was plagued by feelings of loneliness, futility, and loss of meaning in his life and work. His sense of ethical values was largely intact but somewhat inconsistent. On the one hand, he was altruistic and funded an art school in an Asian country he had worked in, but there were ethical lapses around split-off areas of functioning, such as using the office computer for Internet liaisons, which was against company policy. In short, he presented with many of the contradictions observed in those with pathological narcissism at a higher-functioning level: His veneer of social competence and ease vied with sudden withdrawal into states of painful and paralyzing self-doubt, or callous behavior toward others when he felt criticized and/or his expectations for admiration and approval were not met. He lacked empathy for and investment in those he was most intimate with, yet gratification in philanthropic activities made him feel influential and worthy. His façade of invincibility and social dominance combined with episodic plunges into states of worthlessness and self-loathing when his eminence or competence at work were challenged. And a sense of supremacy and self-sufficiency masked underlying insecurities about his capacity to love and be loved.

Case Example: Rebecca—Narcissistic Disorder Functioning at a Borderline Level

Rebecca, a single Asian woman in her mid-20s, presented for therapy while in the midst of steady decline, brought on by what appeared to be

a major depressive episode. From a high-achieving family with a father who was a partner in a prestigious law firm, she had been designated as the perfect child who would fulfill the family legacy by studying law and entering the family firm. Her mother was depressed throughout Rebecca's childhood, sometimes unable to get out of bed to care for self and family. Although Rebecca had performed adequately at a prestigious college, she was in fact quite socially and sexually inhibited, at times feeling no man was good enough for her to commit to, and at other times, feeling hopelessly inadequate and unattractive. She described herself as "the pretty one" who was envied by her mother and sisters, even though she also expressed feelings of self-loathing. Her identity and sense of self were bound up with parental expectations. When Rebecca failed to achieve a score on the Law School Admission Test (LSAT) that would enable her to attend a top-tier law school, she got a job as a paralegal, but began to show symptoms of depression and anxiety, and performed poorly at work. When her best friend was admitted to law school, Rebecca cut off all contact with others, stopped going to work, and sequestered herself in her apartment, binge eating and rarely bathing. She refused to respond to phone calls, e-mails, or texts from her family, friends, and her psychiatrist. When her family became so concerned that they called 911 to check on her, she deemed this an intrusive overreaction. She had little regard for the impact of her withdrawal on her family. In the face of her decline in functioning, and her escalating suicidality, a psychiatrist whom she saw intermittently had prescribed a variety of pharmacotherapies, none of which had been particularly effective. By the time Rebecca was referred for TFP, she was on a complex cocktail of medications that made her feel "like a zombie" and was isolated in her apartment. She spent her days watching reruns of legal shows and fantasizing about being an accomplished lawyer, sporadically attempting to study for the LSAT to improve her scores, yet accomplishing very little. Although she felt controlled by the demands for contact by the significant figures in her life, she expected both parents and her boyfriend to be immediately available when she felt like contacting them, yet kept them at bay for long periods.

Rebecca epitomizes an individual with pathological narcissism in the context of *borderline personality organization* (BPO). BPO is a concept derived from the "structural" system of classification of personality disorders proposed by Kernberg (1984). This diagnostic system emphasizes dimensions (e.g., identity, defenses, ethical functioning, reality testing, object relations) in contrast to the more categorical DSM system (DSM-5; American Psychiatric Association, 2013), with a focus on both the underlying psychological structural components of personality disorders and also the levels of severity of these disorders (Caligor, Kernberg, Clarkin, & Yeomans, 2018). While these patients may or may not have the signs and symptoms of comorbid BPD, including self-destructive

behaviors, overt chronic fears of abandonment, extreme affective instability, behavioral impulsivity, and chaotic interpersonal relationships, they are organized at a broader borderline level in terms of underlying psychological structure, which is further described in Chapter 2.

Case Example: Michael—Malignant Narcissism with Borderline Personality Organization

Michael, an aspiring White male artist in his mid 20's, had a long history of self-injurious behaviors and multiple relationships based on exploitation and substance abuse, as well as bouts of depression and anxiety. He had several hospitalizations for suicidality and a number of outpatient treatments that failed to stabilize his symptoms before he was referred for TFP. During his treatment, he told his therapist, "I've been on the Internet and I don't think you talk enough about my narcissism. . . . I also found out where you live and I just might show up there." When Michael was 5 years old, his mother became disabled in a car accident that had also killed his younger brother. His mother developed a major depression and was unable to care for Michael, instead turning to him for emotional support and caregiving. His mother was also inappropriately seductive, at times exposing herself to Michael and demanding that he put her to bed. Michael's father, who was a successful executive in the entertainment business, traveled often for work, leaving Michael at home to care for his mother. His father encouraged Michael's considerable talents as an artist, performer, and athlete, and instilled in him the sense that he was unusually good-looking, gifted, and destined for unique success. However, his father drank heavily and had rage attacks, during which he sometimes was physically abusive to Michael.

Michael attended a competitive high school for gifted students and had exhibited his paintings in several galleries in shows for student work, but his intermittent self-destructiveness, affective instability, and antisocial behaviors (including missing classes, feigning illness, and lying to his mentors) made it difficult for him to develop his talents in any sustained way. He attended a prominent art and design college, but ultimately dropped out, and was hospitalized for self-injury (lacerating his arms and legs) and substance abuse, after which he took art classes without progressing toward a degree. At 23 he fell in love and married a woman from a socially prominent, wealthy family, who was captivated by his charm, while supporting him financially and excusing his erratic behaviors as the result of an "artistic temperament." Michael was chronically unfaithful to his wife, and denied her wish to start a family because he believed he was "too sick" to become a father.

In the first month of treatment, Michael began an affair with a woman he met in Alcoholics Anonymous (AA). The two developed a

murder–suicide pact to be carried out in the therapist's office, with a plan that would potentially endanger the therapist as well. Michael talked about this plan during a session with his therapist with an excitement that bordered on glee, especially about his imagined ability to trick the therapist and "pull it off," if he wished, even after having described the plan to his therapist. He imagined leaving her hurt, bewildered, and defeated. The therapist pointed out that he seemed to enjoy the idea that his talk of the plan was designed to make the therapist feel fearful and humiliated. For Michael, then, the sense of grandiosity was infused with ego-syntonic aggression fueled by paranoid feelings that the therapist might harm or injure him—beliefs that he used to justify his callous manipulation of the therapist and others, and his potentially harmful actions—all of which characterize the syndrome of malignant narcissism.

In later chapters, we return to these and other cases of individuals treated productively in TFP. We offer them here to highlight the complexities of diagnosing and treating individuals with pathological narcissism, and the myriad forms that pathological narcissism can take, as well as the range of levels of impairment that it may involve. Some of the difficulties in diagnosing and treating individuals with pathological narcissism thus stem from the fact that it can manifest itself in opposing ways (e.g., grandiose or vulnerable) that may be expressed either overtly in behavior and attitudes, or covertly in feelings, beliefs, cognitions, and fantasies (Pincus & Lukowitsky, 2010), within the same individual over time. Furthermore, the dual faces of grandiosity and vulnerability may be expressed in different admixtures for individuals at different levels of severity, as the case examples indicate. Although either grandiose or vulnerable features may dominate the initial presentation, in those with narcissistic pathology, *grandiosity and vulnerability almost always coexist in dynamic relationship to each other.* In those with a grandiose presentation, attitudes of superiority and seeming invincibility defend against inner, subjective states of vulnerability, shame, self-doubt, distress, and neediness. By contrast, a vulnerable presentation of humility, self-doubt, shame proneness, and inferiority may camouflage covert grandiose beliefs about the self as superior and special, either destined for unique greatness, or distinguished by unique suffering.

NPD and Personality Organization

Contemporary object relations theorists (Kernberg, 1984, 2007) have long stipulated that personality disorders are associated with fundamental disturbances in self and interpersonal functioning that vary in

severity within and across disorders. In this regard, the object relations approach that informs our theory and treatment of pathological narcissism and NPD is consistent with the hybrid dimensional/categorical view of personality disorders delineated in the "Alternative Model for Personality Disorders" (AMPD) Section III of DSM-5 (American Psychiatric Association, 2013). The AMPD stipulates that personality disorders, including NPD, are characterized by impairments from mild to moderate severity in the dimensions of self (self-identity, self-direction) and interpersonal functioning (intimacy and empathy), referred to as Criterion A. For those with NPD, disturbances in the sense of identity involve an overreliance on the opinions of others for self-definition and fluctuations in self-esteem regulation, which may inflate or deflate depending on the valuations of others. Difficulties in self-direction as a result or inordinate reliance on gaining approval of others, with standards either too high because of an unrealistic view of self as exceptional, or too low based on entitled expectations, can also disturb a sense of identity. Impairments in interpersonal functioning are characterized by deficits in intimacy evident in the tendency to form shallow, superficial relationships with others and/or a disinterest in those who do not provide admiration. Also evident is an impaired capacity for empathy or the ability to experience and identify with the feelings of others, although there may be a heightened intellectual understanding of the feelings of others sometimes used to advance their own interests (American Psychiatric Association, 2013). In addition, the characteristics of grandiosity, or the feelings of superiority, entitlement, and self-centeredness, and attention seeking, or the attempts to garner admiration, are required for an NPD diagnosis. Other traits, such as antagonism and negative affectivity (depression, anxiety), may also be added on as specifiers to capture vulnerable presentations of pathological narcissism.

The object relations model that informs TFP also conceptualizes the impairments of self and interpersonal functioning as core dimensions of pathological narcissism NPD that vary in severity depending on the individual's level of personality organization. But it goes beyond the AMPD model to include dynamic features, such as defensive functioning and ethical values (Clarkin, Caligor, & Sowislow, 2019). Most important is the level of integration and developmental quality of the individual's mental representations of self and significant others as crucial dimensions in understanding the intrapsychic structural features of those with pathological narcissism and NPD. In the object relations perspective, impairments in all of these dimensions are linked to a particular underlying structural constellation of ideal and devalued mental representations of self and other that characterizes the grandiose self. The descriptive criteria found in the DSM-5 NPD category, the contradictory characteristics of grandiosity or vulnerability highlighted in

the diagnostic system of the second edition of *Psychodynamic Diagnostic Manual* (PDM-2; Lingiardi & McWilliams, 2015) and the hybrid dimensional/categorical AMPD, which emphasizes impairments in self and interpersonal functioning—all may be seen as expressions of investment in different facets of the underlying grandiose self-structure that is paradigmatic of pathological narcissism. Accordingly, when we use the terms *pathological narcissism*, *narcissistic pathology*, or *narcissistic personality disorder* in this book, we are referring to a structural conception of the disorder that combines both categorical and dimensional features. We discuss these issues in more detail in later chapters.

Levels of Personality Organization

From an object relations perspective, the level of personality organization (i.e., high functioning [neurotic], borderline, or psychotic) is determined not only by the level of functioning across the dimensions of self and interpersonal domains but also the extent to which mental representations of self and other are integrated (e.g., how well positive and negative aspects are balanced and modulated) and differentiated (e.g., the extent to which concepts of self and other are separate and distinct). At the highest level, there are individuals with conflicts around narcissism. But in the context of a reasonably consolidated identity based on realistic, integrated, and differentiated mental representations that allow for the capacity to invest and take pleasure in enduring relationships to adhere to a solid system of ethical values and to maintain intact and stable reality testing—these are all characteristics of those in the normal or neurotic realm. In these individuals, we do not see the impoverishment of object relations characteristics of those with more severe narcissistic pathology—rather, there is a level of mutuality, warmth, flexibility, consistency, and complexity to the inner object world. Instead, we see transient exaggerations of the normative narcissistic strivings to receive recognition, respect, and appreciation for one's accomplishments and characteristics.

For example, a high-functioning professional woman, a partner in a major law firm with a successful career and deep, enduring friendships, occasionally regresses to childish demanding behavior when she feels not respected or attended to by her partner with whom she shares a gratifying emotional and sexual relationship. But this represents not so much distorted, polarized representations of others or an exaggerated, inflated sense of self but unresolved conflicts around an identification with an inconsistently available, but loving maternal figure that could be confronted and worked through in treatment. Similarly, a male patient, a writer who had struggled for a decade to write a definitive biography,

was supported by a wife whom he loved deeply and with whom he shared many intellectual interests. When his book soared to the top of the best-seller list, he became self-preoccupied and self-important, basking in the respect and adulation he received on book tours, neglecting the emotional needs of his wife and close friends with whom he had been previously attuned and responsive. When his wife expressed her unhappiness and dissatisfaction and requested couple therapy, he was able to acknowledge that his newfound celebrity had eclipsed other aspects of his life and was able to use the therapy productively to work on reestablishing their intimacy. The couple therapy led to the exploration of some unresolved issues about the ways his neglect of his family stemmed from an unconscious identification with his father, a talented but frustrated writer whose own career had been stalled when he took a job as an executive in publishing to support the family, thereafter projecting his aspirations onto his son while devaluing his failing marriage. In sum, individuals at a higher level of organization in some situations (escalation of prominence, wealth, etc.) may show narcissistic features, such as exaggerated self-importance, entitled expectations, desire for admiration, or antagonism. However, these are often transient and occur in the context of a relatively good overall adaptation and a reasonably stable, complex, and multifaceted sense of identity that is not distorted by an illusory rigid grandiose self, and allows for enduring—even if sometimes conflicted—relationships in depth.

Higher-functioning individuals with pathological narcissism and NPD, in contrast to those with narcissistic traits, may present similarly to those described above. They are often quite charming, accomplished, socially adept, and successful at garnering the approval, admiration, and veneration of others. But they respond to criticism or threats to self-esteem not with reflection and dialogue but with hostility, withdrawal, and/or rejection of others who challenge them, and further in the face of more serious challenges (e.g., loss of a job or relationship, or financial reversals), aspects of reality that fail to confirm their illusions about the self may be denied or rejected. In addition, higher-functioning narcissistic personalities often have seemingly gratifying relationships, but these are often superficial and lacking in mutuality and depth, and/or are organized to meet inordinate needs for admiration. They experience disturbances in the sense of self from vague feelings of futility and emptiness to more profound states of despair, and thus do not have the cohesive, integrated identity characteristic of those with higher-level organization, although they may sometimes present as such, and hence, may be seen as on the cusp of neurotic organization. However, in the face of challenges to the narcissistic equilibrium, such as the inevitable losses, developmental transitions, and/or disappointments of life, such individuals

may become quite symptomatic, experiencing anxiety, depression, or somatic symptoms that fail to abate with focal short-term treatments. In such situations, the excessive reliance on the admiration of others to regulate self-esteem, the fluctuations in self-esteem and affective instability, the compromised capacity to recognize or share their own internal distress, the lack of recognition or empathy for the distress they cause in others, the need to see the self as exceptional regardless of the views of others—all evidence of underlying narcissistic structure—emerge. So, the potential for developing narcissistic pathology may not emerge fully in such high-functioning individuals until there is a life crisis that precipitates the acute onset of more severe symptomatology and personality pathology (Ronningstam, 2011; Ronningstam, Gunderson, & Lyons, 1995; Simon, 2002). At that point, higher-functioning individuals may present with characteristics of borderline organization (e.g., identity disturbances, affective instability, impulsivity).

Clinical investigation has identified midlife as a critical period for the development of narcissistic personality pathology for individuals who function at a higher level (Kernberg, 2007; Ronningstam, 2010). Such individuals may remain asymptomatic in the early part of the life cycle (young adulthood) when their narcissism is well regulated by the admiration that their considerable talents and accomplishments evoke, but narcissistic pathology may develop or worsen in midlife, which inevitably brings confrontation with limitations, losses, and mortality. It is in midlife that characteristic attitudes of exploitativeness, entitlement, lack of empathy, and relentless use of others to regulate narcissistic needs for self-enhancement lead to erosion of love relationships, and/or disillusionment or conflicts around work (Ronningstam & Maltsberger, 1998; Kernberg, 1975, 2007, 2018). In such situations, the inability to access a rich and sustaining internal world of mental representations leave these individuals feeling a sense of emptiness or futility as they confront the gap between their external success and the impoverishment of their relational lives, even when there is not a major discrepancy between ambitions and accomplishments. Such was the case with Mark described earlier, who presented for treatment at age 40 in the midst of a crisis brought on by his conflict between a fantasy-fueled Internet relationship with an idealized unattainable woman and a long-term, devoted but devalued partner. It is noteworthy that in high-functioning individuals like Mark, grandiosity, which is likely to be more dominant earlier in the life cycle, may be relatively adaptive in that it propels the individual toward realizing his or her goals and aspirations and gives the individual the confidence to do so.

In individuals with narcissistic disorder with lower-level borderline organization, there are the core narcissistic features of grandiosity,

entitlement rage, and exploitativeness, but in addition, there is more extreme vulnerability typical of nonspecific personality pathology (Wright & Edershile, 2018). There are more extreme fluctuations between grandiose and vulnerable self-states, leading to a more fragmented sense of identity (Di Pierro, Di Sarno, Preti, Di Mattei, & Madeddu, 2017), with less integration between aspects of self that are aspired to and those that are disowned or denied. This makes it difficult to develop consistent goals and limits the capacity to invest in or sustain intimate relationships with others. These impairments in interpersonal functioning may be the outward manifestations of a deterioration or distortion in the internal world of object relations, leading to a chronic sense of emptiness and boredom that can be overcome only by the continual seeking of external excitement with an ever-changing array of people and experiences. Negative feelings, such as envy, fear, and anger, are not well integrated or tolerated, but dealt with through the systematic devaluation of others, particularly if the others have capacities or accomplishments the individual desires for his or her own. In addition, those with narcissistic pathology with BPO present with a grandiose sense of self that is grossly discrepant with their real attributes and accomplishments, along with impulsivity, poor anxiety tolerance, and split, polarized object relations. This leads to chronic, rather than episodic, failures in work and intimate relationships. Individuals are likely to react with rage, extreme withdrawal, or emotional collapse when threatened, despite some defensive functions provided by the pathological grandiose self (Kernberg, 2007). Rebecca, who collapsed when she failed to live up to her unrealistic expectations of herself, epitomizes the vulnerable narcissistic individual with a low borderline organization. Symptomatic stress may be seen in late adolescence and young adulthood with its testing of the self beyond school and family. It is often evident in the syndrome of failure to launch.

At the more disturbed end of the spectrum of pathological narcissism (lower level of borderline organization) is the inability to tolerate or manage aggressive and negative affects, such as anger or envy that may lead to the wish not only to devalue others but to destroy the good things they possess, often leading to the deterioration of relationships, both internal and external, with significant others. In addition, in individuals with more severe narcissistic pathology, there is greater deterioration of object relations and superego functioning evident in the lack of a coherent system of ethical values, with adherence to rules and standards mostly out of fear of being caught or exposed, marked antisocial features, paranoia, and ego-syntonic aggression characteristic of malignant narcissism. Michael, the third case presented earlier, had a history of psychopathic attitudes and behaviors (chronic lying), self- and other-directed destructiveness that stemmed from identification with a

corrupt parental figure who was abusive but also treated him as special. Michael showed many aspects of malignant narcissism, including vengeful destructiveness toward others evident in the murder–suicide pact he formed with another patient.

In sum, an object relations perspective proposes specific distortions in the dimensions of identity, defenses, reality testing, superego functioning (ethical values), and object relations, or the level of integration and developmental quality of mental representations of self and significant others that exist at different levels of severity. However, in our view, what distinguishes individuals with pathological narcissism across all levels of functioning from those with personality disorders in general is the specific constellation of ideal self and object representations: the pathological grandiose self.

The Pathological Grandiose Self

The pathological grandiose self is a compensatory structure in which ideal representations of self and others (i.e., what one aspires to be through identification with admired others) overshadow the real self (i.e., a realistic sense of actual abilities and potential). Typical for the entire spectrum of narcissist pathology is the absence of a stable availability of a loving parental figure that provided a significant period of safe dependency, and fostered the elaboration, differentiation, and maturation of the child's real capacities and characteristics. Also, typical across the spectrum is some source of admiration that the patient obtained from significant others, whether for physical appearance, talents, or role in the family that served as a compensating experience by which the security derived from being admired would replace the security from being loved. These developments constitute the nucleus of what crystallizes as the pathological grandiose self.

In such circumstances, a positive inward sense of an authentic and vital self, separate from idealized images of self based on the approval of others, fails to develop. In contrast to those with BPD where the split between an idealized and devalued concept of self and others leads to a lack of integration in the self, the grandiose self of those with narcissistic disorder provides a semblance of integration. In the pathological grandiose self there is a condensation of real and ideal aspects of the self with the systematic projection of the negative aspects of the self onto others. Often some special aspect of the self that makes the individual admirable as an object for the parents or others forms the basis for an idealized sense of self that merges with ideal images of others, overshadowing the realistic images of the self and others. Since the ideal self, ideal

other, and real self are one, the individual does not feel the need to live up to moral values or standards of ideal others. Also, negative or devalued aspects of the self are condensed with those of others and systematically projected onto them in order to preserve the sense of specialness, uniqueness, supremacy, and infallibility that characterizes the grandiose self (Kernberg, 1975, 2007, 2018). But systematic devaluation of others truncates the process of further internalization of others because of their devalued status. Consequently, the individual cannot receive and value nurturance from devalued others. Such a grandiose self-structure leads to difficulties with (1) identity, in that the individual believes that he or she encapsulates all that is desirable and retreats to an illusory world to preserve such an inflated view of self; (2) self-direction, in that the individual relies on the admiration and adulation of others to shape aspirations and sustain goals; (3) interpersonal functioning, in that others serve self-regulatory purposes and are seen as extensions of the individual's own needs rather than valued as separate and unique others in their own right (Kernberg, 1975, 1984, 2007, 2015; Morf & Rhodewalt, 2001; Morf, Torchetti, & Schürch, 2011); and (4) distortions in ethical values and reality testing, in that aspects of reality that do not conform to the illusions of the grandiose self are distorted or rejected, leading at times to the disregarding of rules, social norms, and the consequences of such.

In higher-functioning narcissistic persons, the ideal aspects of the grandiose self are not so extreme as to eclipse the real aspects of self, allowing for some adequate adjustment, stable if shallow relationships, and reality orientation. In high-functioning individuals, the prevalence of positive libidinal aspects over aggressive aspects in the self makes them more accessible for relationships and for therapy. Such was the case with Mark, who was highly accomplished and successful at work but was deeply conflicted between idealized fantasy-laden relationships and his unsatisfying partnership with a woman whom he systematically devalued but could not leave. By contrast, in narcissistic persons functioning on an overt borderline level and/or those with malignant narcissism, aggression-laden, highly unrealistic ideal representations of self and others fuel the need to dominate and control others, severely limiting the capacity to work or to love and leading to more severe impairments in reality orientation. Such was the case with Michael, who enacted an unconscious identification with a highly idealized destructive paternal figure in his chaotic, exploitative relationships and fantasied vengeful behavior toward the therapist.

From an object relations perspective, the pathological grandiose self is a core structural feature of those with pathological narcissism, but the core dysfunction of pathological narcissism is not necessarily grandiosity

per se. Rather, in those with pathological narcissism or NPD, states of grandiosity coexist with feelings of inferiority and fears of loss of status and control—aspects of the vulnerable self-states to which these individuals are prone when the defensive functions of the pathological grandiose self falter, or when their excessive needs for admiration and validation are unmet (Kernberg, 1975; Levy, Beeney, & Temes, 2011; Roche, Pincus, Conroy, Hyde, & Ram, 2013). When these needs are in line with the individual's actual talents and capacities, they fuel the quest for experiences that enhance the self-aspects of normal or adaptive narcissistic strivings. Pathological narcissism is born of the extreme, exaggerated nature of such needs, and the inability to translate them into mature, realistic ambitions and strivings.

While we posit such a structural underpinning to narcissistic pathology, we are aware that each individual's structure may fluctuate according to internal and external stressors. We are also aware that clinicians may find in their practice individuals who may have characteristics of narcissism but may not have a pathological grandiose self as firmly fixed in place as do others with full NPD. In such cases, one may encounter patients with narcissistic features or traits without a pathological grandiose self-organization per se. In any case, it should be noted that structures including the pathological grandiose self are enduring and stable patterns of mental functions that organize the individual's behavior, perceptions, and subjective experience (Yeomans et al., 2015).

Grandiose (Thick-Skinned) and Vulnerable (Thin-Skinned) Presentations of NPD

Orthogonal to the view of NPD as a dimensional disorder that exists along a spectrum of pathology, is thus the distinction between the two different presentations of narcissistic pathology. A grandiose or thick-skinned presentation involves an exaggerated sense of self-importance, strivings for social dominance and exploitation of others to attain it, and a sense of entitlement rage when it is threatened or challenged. These are all the overt manifestations of more covert cognitions, such as fantasies of unlimited success. A vulnerable or thin-skinned presentation is characterized by hypersensitivity to rejection, a sense of unworthiness, and feelings of inadequacy in the context of covert grandiose features evident primarily in fantasies, beliefs, and entitled expectations, the behavioral expression of which is curtailed by inhibition, self-effacement, and shame proneness (Kernberg, 1984; Cain, Pincus, & Ansell, 2008). While there is some evidence that the pathological grandiose self figures in both types of presentations, it may take somewhat different configurations.

The grandiose or thick-skinned narcissistic person may be characterized by a robust grandiose self-representation in which a thick defensive barrier keeps devalued aspects of the self more securely at bay. This was the case with Mark, described earlier, whose idealized Internet figure represented, in part, a projection of his own superiority, while his partner represented the devalued, dependent, disavowed aspects of self. By contrast, in the thin-skinned vulnerable individual, prone to states of extreme vulnerability, hypersensitivity, self-doubt, and even paranoia, the defensive barrier between idealized and devalued aspects of the self is thinner and more permeable. This was the case with Rebecca, whose preoccupation with grandiose fantasies of having a successful legal career was the primary driving force behind her retreat into a world of autistic fantasy when her unrealistic ambitions could not be realized. The vulnerable narcissistic organization was masked by her depressive symptomatology and collapse.

In sum, grandiosity and vulnerability may coexist, oscillate, or defend against one another in different ways in individuals with different levels of personality organization, and different characterological features. That pathological narcissism always coexists with impaired regulatory capacities that can lead to vulnerable states of helplessness, shame, fear, and social withdrawal is now backed up by clinical research on ecological momentary shifts in self-states on a daily or even hourly basis in those with narcissistic pathology. Several studies have shown mental state fluctuations from grandiosity to vulnerability in those with pathological narcissism and NPD in response to interpersonal interactions on a daily basis (Giacomin & Jordan, 2016; Roche et al., 2013). However, at least one study shows that the extent to which the individual is dispositionally vulnerable or grandiose affects the extent to which there were momentary fluctuations between the two states, with those who were rated higher on vulnerability showing the most variability between grandiosity and vulnerability (Edershile & Wright, 2019). Thus, further clinical and empirical research is necessary to understand whether these two presentations represent distinct categorical phenotypes or subtypes of the disorder, or fluctuating intra-individual mental states that oscillate over time in response to different conditions and life circumstances. As clinicians we do not observe a simple dichotomy between grandiosity or vulnerability. Rather, these dimensions, although fundamental to narcissistic pathology, may take on different configurations in individuals dependent on level of organization and characterological features. Those high in vulnerable narcissism have been found to suffer from more identity diffusion and may be more likely to be in the borderline realm in several studies (Di Pierro, Costantini, Benzi, Madeddu, & Preti, 2019).

The Prevalence of NPD and Its Relationship with Other Disorders

Recent studies have shown rising rates of pathological narcissism and NPD both in the nonclinical and clinical groups. In a recent community study, the Wave 2 National Epidemiologic Survey on Alcohol and Related Conditions (NESARC), a representative sample of 35,000 Americans were asked in face-to-face interviews whether they had experienced symptoms of NPD over the course of their lifetime. Individuals had to endorse the requisite number of DSM-IV criteria for NPD, including at least one that caused significant social or occupational impairment or distress (Stinson et al., 2008). In the total sample, the prevalence of NPD was 6.2%. Interestingly, men were found to have higher rates of NPD than women (7.7% in contrast to 4.8%), and rates of NPD were particularly high among young adults between the ages of 20 and 29 (9.4%). They were also higher among those who were separated, widowed, divorced, or never married, perhaps indicating the difficulty the latter group of individuals have in forming and sustaining relationships. Also, higher rates were seen among Black males and Hispanic females, indicating perhaps the defensive function of NPD to deal with the injuries of minority status, a finding that necessitates further investigation. The overall prevalence rate of 6.2% in this study is markedly greater than that noted in the only other comparable study previously conducted in Oslo, Norway, which yielded an NPD rate of 0.8% (Torgersen, Kringlen, & Cramer, 2001), indicating perhaps that narcissistic pathology may be more prevalent in North American culture. However, a reanalysis of the data conducted by trained clinical researchers rather than research assistants who conducted the first study, and with more rigorous cutoff criteria that included the level of subjective distress and functional impairment, as well as DSM-IV diagnostic criteria, yielded lower prevalence rates of 1.0% (Trull, Jahng, Tomko, Wood, & Sher, 2010). A comparable prevalence rate of 1.0% was reported in a systematic review of several other community studies of NPD (Dhawan, Kunik, Oldham, & Coverdale, 2010). Even at these lower rates, as Paris (2014) points out, NPD is a highly prevalent disorder. Furthermore, it is likely that these prevalence rates are underestimated—first, because they are largely based on self-reports and persons with pathological narcissism have been found to be unreliable in their self-assessments. In fact, informants (peers or family) are over twice as likely to identify narcissistic pathology in individuals as are the individuals themselves on self-reports (Cooper, Balsis, & Oltmanns, 2012; Klonsky & Oltmanns, 2002). Second, this focus on the grandiose features of NPD may also account for the discrepancy in the rates of NPD in men and women, since self-report

assessments privilege the overt grandiose features of NPD, and women may be more prone to be characterized by the more covert, vulnerable features of the disorder (e.g., self-defeating attitudes and behavior, shame proneness, preoccupation with grandiose fantasy). A recent meta-analysis affirms that men tend to be more narcissistic than women across different cohorts and age groups and are more likely to be diagnosed with NPD, with measures of grandiose narcissism showing more prominent gender differences then do measures of vulnerable narcissism (Grijalva et al., 2015). Thus, the rates of NPD in both clinical and community groups may be increasing along with the rise in narcissistic traits in the general population, although the impact of gender, age, and cohort necessitates further investigation.[1] In clinical groups, the prevalence of NPD has been found to range from 2% (Zimmerman et al., 2005) to 6% (Grilo, McGlashan, & Quinlan, 1998) to 17% (Clarkin et al., 2007; Ronningstam, 2011), with greater levels of narcissistic pathology reported by psychoanalytic clinicians who tend to see patients for lengthier and in-depth treatments (Gabbard, 1997). These higher prevalence rates in clinical samples indicate that those with NPD experience enough distress to seek treatment, although the narcissistic pathology initially may be masked by other co-occurring disorders.

Comorbidity and NPD

Many individuals with pathological narcissism and NPD often present with other co-occurring disorders, particularly unipolar and bipolar depression and anxiety disorders, although substance abuse, posttraumatic stress disorder, and eating disorders are also prevalent (Fossati et al., 2000; Simonsen & Simonsen, 2012; Zimmerman et al., 2005; Stinson et al., 2008). Depression in individuals with pathological narcissism often has distinct characteristics in that it is characterized by emptiness, nihilism, anhedonia, and a sense of futility and meaninglessness rather than by pervasive sadness, guilt, or preoccupation with a sense of unworthiness or loss. Our clinical observation is that depressive symptoms in patients with pathological narcissism tend to be precipitated by the collapse of entitled expectations and by recognition of the gap between the ideal and real self. Furthermore, mood and anxiety symptoms are often infused with anger, shame, and envy stemming from

[1]In a recent meta-analysis of gender differences in narcissism, men were more likely to be diagnosed with NPD, and scored higher on exploitativeness/entitlement (the most socially problematic aspect of narcissism; Ackerman, Hands, Donnellan, Hopwood, & Witt, 2017) and leadership/authority facets (Grijalva et al., 2015), than did women.

failure to meet perfectionistic standards. In sum, co-occurring mood disorders in the context of pathological narcissism may involve a particular configuration of external symptoms and internal experience of self and others.

In addition, there is a high degree of co-occurrence of NPD with other personality disorders, including histrionic, antisocial, borderline, schizotypal, and paranoid, in both community samples (i.e., 62.9% rate of comorbidity of NPD with other disorders; Stinson et al., 2008) and clinical samples (i.e., >50.0% rate of comorbidity; Clarkin et al., 2007; Gunderson, Ronningstam, & Smith, 1996; Pfohl, Coryell, Zimmerman, & Stangl, 1986; Zimmerman et al., 2005). The high rates of comorbidity between NPD and other personality disorders have led to debates about whether NPD is a distinct personality disorder or whether narcissistic pathology is a dimension that may characterize several other disorders. Interestingly, in a recent study, Sharp and colleagues (2015) evaluated the general (g) factors that might account for both common (underlying) variables or factors shared across DSM-5 personality disorders (e.g., identity, impulsivity), as well as specific, unique factors that may represent more specific forms of personality disorder pathology. Clear specific factors with strong average loadings emerged for only three personality disorders: narcissistic, antisocial, and schizotypal. Interestingly, the borderline disorder items loaded most strongly on the so-called general (g) factor of personality pathology (e.g., identity disturbance, affective instability, emptiness, self-harming impulsivity, interpersonal instability, intense anger, frantic efforts to avoid abandonment, and suicidality). Those with NPD were relatively weak on this g factor, but quite strong on the factors specific to NPD, such as grandiosity, envy, and arrogance. These findings suggest that while there is an overlap between NPD and other personality disorders reflected in the high rates of comorbidity, there are also specific characteristics of the disorder that set it apart as unique and distinct. This study supports the conceptualization of personality disorders both as distinct categories and as having varying dimensions and levels of personality pathology that are consistent with our object relations model.

In addition, the high rates of comorbidity make it likely that patients with other disorders will have significant narcissistic pathology that may affect their presentation and that will complicate the treatment process and outcome (Kernberg, 1984; Diamond & Yeomans, 2008). Interestingly, although assessing narcissistic pathology is complicated by the high rates of comorbidity, Gabbard (1997) has proposed that in more in-depth, intensive psychodynamic treatments, NPD may emerge as a primary diagnosis.

NPD More Prevalent When Diagnosed by Clinicians

Interestingly, clinicians in outpatient private practice report higher rates of narcissistic pathology and NPD than do those in outpatient hospital-based clinics or community samples. For example, in one study, 510 psychoanalysts across three countries reported that 20.0% of their patients suffered from NPD as assessed by DSM-III-R criteria (Doidge et al., 2002), making it one of the most prevalent disorders in the United States and Ontario, Canada, and the second-ranked disorder in Australia. Regarding the difficulty in treating NPD patients, Doidge and colleagues also found that the majority of these patients had sought previous short-term treatments, which were unsuccessful. In another study in which NPD was assessed through interviews with patients that focused on their interpersonal functioning, as well as observations of their behavior with the interviewer, 76.0% of expert clinicians reported treating patients who met criteria for NPD (Westen, 1997). In an attempt to extend these findings and show that DSM NPD criteria do not capture the range of patients with nacissistic personality pathology seen by clinicians in practice, Westen and Arkowitz-Westen (1998) asked a sample of randomly selected experienced clinicians (psychiatrists and psychologists) with a range of theoretical orientations (44.8% psychodynamic, 16.1% cognitive behavioral, 34.3% eclectic; 36.4% psychiatrists, 63.6% psychologists) to report on the percentage of their patients who were diagnosable with DSM-IV NPD. They found that overall 8.5% of the 714 patients involved were reported to have a diagnosis of NPD. It is noteworthy that the psychodynamic clinicians reported the highest number of NPD patients (11.2%), followed by eclectic (5.7%), and then cognitive-behavioral clinicians (3.9%). The authors concluded that DSM-IV criteria are inadequate to assess the range of personality problems of those with personality disorders, including NPD. For example, a random sample of clinicians (psychologists and psychiatrists) defined pathological narcissism as a complex multifaceted construct that exists on a continuum of severity with core features of interpersonal vulnerability, inner distress, competitiveness, and difficulty regulating affect, particularly anger, not represented in the DSM (Russ, Shedler, Bradley, & Westen, 2008).

It is likely that psychodynamic clinicians tend to report somewhat higher levels of patients with pathological narcissism and NPD in their practices because they have a greater capacity to recognize the underlying personality dynamics, the impairments in self and interpersonal functioning, and the vulnerability to states of shame and self-loathing that may be obscured by the grandiose self-presentation of those with NPD. They also have more extensive training in theories of narcissism,

both normal and pathological, and hence, a greater capacity to recognize pathological narcissism and NPD, which may be masked by syptoms of depression, anxiety, and/or substance abuse and thus escape detection by colleagues without psychodynamic training who often refer patients for more intensive psychodynamic therapy after numerous short-term treatments have failed to alleviate their distress and/or behavioral problems. Indeed, a study conducted among CBT therapists found 14.0% of 742 patients were diagnosed with NPD, a figure that approaches the number diagnosed by the psychodynamic clinicians in the previous study (Westen & Arkowitz-Westen, 1998), indicating that in this study, there was a more rigorous assessment of the patients and/or that the CBT therapists making their own diagnosis in the Westen and Arkowitz-Westen study may have been underreporting.

In sum, recent studies have indicated that those with NPD now comprise 1.0–6.2% of those in community samples (Stinson et al., 2008; Trull et al., 2010), with higher rates (from 2.0 to 17.0%) in clinical samples (Westen & Shedler, 1999; Grilo et al., 1998; Clarkin et al., 2007; Zimmerman et al., 2005), and 8.5–20.0% in outpatient private practice (Westen & Arkowtiz-Westen, 1998; Doidge et al., 2002; DiGiuseppe et al., 1995).

One factor that may account for the discrepancies between the rate of NPD diagnoses gleaned from community studies and those reported by clinicians is the nature of assessment of narcissistic pathology. First, most existing assessment instruments used in large-scale research are tied to DSM criteria that privilege the overt grandiose dimensions of the disorder, such as arrogance, entitlement, and admiration seeking, and underplay the vulnerable aspects of narcissistic pathology, including hypersensitivity and emotional reactivity. The latter have been associated with clinical levels of distress and seeking therapy (Ellison et al., 2013), so that clinicians are likely to see those with narcissistic pathology when they are in a vulnerable state of mind. In addition, clinicians may conceptualize pathological narcissism dynamically as a disorder in which pathological grandiose features vie with regulatory impairments that catalyze vulnerable states of distress, shame, and self-doubt. Nonetheless, the high rates of comorbidity suggest that further investigation is warranted on how pathological narcissism may present differently in the context of other co-occurring disorders.

Functional Impairments Associated with NPD

Although they may be asymptomatic for periods of time and appear to have a good superficial social and occupational adjustment, individuals with narcissistic disorder have significant impairments in the spheres of

love, work, and social life. In fact, individuals with narcissistic pathology are more likely to be divorced or unemployed, which may make them vulnerable to the emergence of more overt symptoms (Ronningstam, 2011; Stinson et al., 2008). In other words, when the individual's characteristic narcissistic defenses (of denial, projection, devaluation, idealization, and retreat into autistic [grandiose] fantasy, see chapter 2) are challenged by life's vicissitudes, they often become highly symptomatic, presenting for therapy with co-occurring mood disorders (depression, bipolar disorders), anxiety disorders, other personality disorders, substance abuse, and suicidality (Stinson et al., 2008; Ronningstam & Maltsberger, 1998; Ronningstam, 2009, 2010, 2013). In fact, narcissistic individuals have been shown to have excessive psychophysiological reactivity (e.g., elevated cortisol, cardiovascular reactivity) in response to everyday negative emotions (Edelstein, Yim, & Quas, 2010; Cheng, Tracy, & Miller, 2013; Kelsey, Ornduff, Reiff, & Arthur, 2002) and in response to imagined experiences of rejection (Sommer, Kirkland, Newman, Estrella, & Andreassi, 2009). Even under conditions of low threat, they show higher levels of stress hormones than individuals who are rated low on narcissism (Reinhard, Konrath, Lopez, & Cameron, 2012), even though those who self-report high on narcissism also report feeling less stress and/or more competence in stressful situations. Based on these and other findings we might surmise that those with pathological narcissism are vulnerable to stress reactions and somatic disorders. Thus NPD indivduals may have painful or frightening internal experiences of which they may or may not be fully aware or able to share, and this compromised internal processing may in fact cause pain and suffering in others. Interestingly, it has been noted that individuals with NPD are distinguished from those with other personality disorders by their proclivity to generate distress in significant others (Miller, Campbell, & Pilkonis, 2007). Thus, narcissistic disorder takes a toll on self *and* others, and it is often the partners, parents, children, or coworkers who precipitate the crisis that brings an individual to treatment.

Given these characteristics, it is not surprising that individuals with narcissistic disorder often generate intense countertransference pressures that may interfere with therapists' observational and analytic functions. One survey of countertransference reactions of psychologists and psychiatrists working with narcissistic patients found that "clinicians reported feeling anger, resentment and dread in working with patients with NPD; feeling devalued and criticized by the patient, and finding themselves distracted, avoidant and wishing to terminate the treatment" (Betan et al., 2005, p. 894). However, we have also experienced and encountered in our colleagues the opposite extreme, in that clinicians report being overinvolved and even fascinated or captivated by individuals with

pathological narcissism, leading to subtle to overt enactments or difficulties with maintaining boundaries (Luchner, 2013).

TFP for Pathological Narcissism: Translation of Object Relations Theory into Practice

The contemporary object relations perspective presented above has been translated into a treatment approach. TFP is a manualized psychoanalytically oriented, evidence-based therapy designed to treat personality disorders, including NPD (Yeomans et al., 2015). It is most typically a twice-weekly psychotherapy.

TFP for pathological narcissism and NPD (TFP-N) is uniquely suited to treat a spectrum of patients with pathological narcissism with different levels of personality organization from higher to lower functioning, with diverse presentations and levels of severity. Since TFP emphasizes identifying the totality of the individual's internal dyadic experience (e.g., grandiose self, devalued other; vulnerable self, idealized other), it is effective in addressing the different phenotypic presentations, forms of expression, and/or fluctuating mental states that may characterize those with narcissistic pathology across the spectrum of personality functioning.

The major goal of TFP-N with patients with pathological narcissism is to promote modulation and integration of the highly contradictory, distorted idealized and devalued representations that comprise the grandiose self and that underlie so many of the maladaptive patterns in self and interpersonal functioning that we observe as clinicians. Change in these structures over the course of treatment would include the narcissistic individual's increased capacity to recognize, modulate, and integrate the rigidly held idealized and devalued representations of self and others that comprise the grandiose self, and to understand how they have distorted aspects of self and interpersonal experience. These distortions include limitations in the capacity to love; an inability to connect with others beyond the instrumental functions they serve or the adulation they provide; the sense of insularity, emotional isolation, and inauthenticity; and the tortuous strivings for perfection—all of which are often the source of the narcissistic person's suffering. As the compensatory functions of the grandiose self are explored and it begins to dissolve, there is a shift to the capacity to invest in and sustain object relations in depth and also a shift from primitive defenses based on splitting and projection to a more mature defensive system that allows for a successful balance between gratifying one's needs and desires while adapting to the complexity of reality.

The specific tactics and techniques of TFP, modified for narcissistic disorder, are presented in Chapters 4–9. Two features distinguish TFP from other psychoanalytic therapies. The first is prioritizing a focus on the interaction in the session covering the content of the patient's discourse. The second is the dual focus on internal and external reality, and the integration of aspects of external reality into the ongoing transference work.

Each patient is unique, and each therapist is unique, unable to separate his or her subjectivity, personal clinical influences, tolerances, and temperament from the implementation of TFP-N. Each patient brings his or her specific challenges to the treatment frame, his or her particular ways of acting out and of challenging the therapist in the treatment process, and his or her particular defensive signature (Kernberg, Diamond, Yeomans, Clarkin, & Levy, 2008; Yeomans & Diamond, 2010). Furthermore, each patient reacts differently to the therapist's personal style as he or she implements TFP-N. Through this interface of patient and therapist, no two TFP treatments can feel or look exactly the same—however, each clinician can follow what we feel to be the best practices that we distilled from our years of work with narcissistic patients.

Research Evidence for TFP

TFP has been shown to be an effective treatment for BPD in one uncontrolled study (Clarkin et al., 2001) and two RCTs (Clarkin et al., 2007; Doering et al., 2010). Like several other structured treatments for personality disorders, TFP has been found to foster changes in symptoms (depression, anxiety), psychosocial functioning, impulsive aggression (Clarkin et al., 2007), suicidal and self-injurious behaviors, and service utilization (emergency room visits, hospitalizations; Clarkin et al., 2001; Doering et al., 2010). However, in our studies, only TFP has been shown to be effective in improving the security of attachment representations and the capacity for mentalization, both of which are thought to be indicators of structural change in levels of personality organization (Fonagy, Gergely, Jurist, & Target, 2002; George, Kaplan, & Main, 1996; Levy et al., 2006; Buchheim et al., 2017). After 1 year of TFP, patients have shown shifts from insecure to secure (Levy et al., 2006) and from disorganized to organized attachment representations (Buchheim et al., 2017).

In addition, improvements in reflective functioning (RF; Fonagy, Steele, Steele, & Target, 1998), or the capacity for mentalization in the context of attachment relationships, were also found in two RCTs (Fischer-Kern et al., 2015; Levy et al., 2006). Changes in mentalization

and in the internal working models of attachment relationships are particularly important for patients with pathological narcissism or NPD. Those with pathological narcissism have been found in many empirical and clinical investigations to have difficulty with forming and sustaining attachments and with reflecting on and differentiating between their own and others' mental states—both sequelae of their limited capacity for empathy. TFP is now considered an evidence-based therapy by Division 12 (Society of Clinical Psychology) of the American Psychological Association (*www.div12.org/treatment/transference-focused-therapy-for-borderline-personality-disorder*). It is notable that the research studies described above have included a number of patients with NPD/BPD, from 10% (Doering et al., 2010) to 17% (Clarkin et al., 2007) to 70% (Clarkin et al., 2001).

Recently, we expanded our understanding of NPD through in-depth investigation of the clinical process and outcome of patients with NPD/BPD, and how they differ from those with BPD alone. We also conducted empirical research that shows differential patterns of attachment and mentalization, and how they change over the course of TFP in patients with NPD/BPD (Diamond, Clarkin, et al., 2014; Diamond, Levy, et al., 2014; Hörz-Sagstetter et al., 2018). Inspired by these clinical and research investigations, we adapted the tactics and techniques of TFP with specific modifications for patients with pathological narcissism and NPD. The goal is to refine TFP techniques to address the particular clinical challenges posed by this group of patients, including their high rates of dropout, dismissing devaluation and/or idealization in relationship to the therapist who is enlisted as a mirror or sounding board, and their intolerance of dependency and fear of exposure. All of the foregoing may curtail the engagement with the therapist and contour the nature of transference, as well as the therapist's countertransference, which as noted earlier may range from inordinate pleasure in the patient's idealization and/or to anger and dread in the face of the patient's dismissing devaluation of the therapist, leading to retaliation, detachment, and/or overinvolvement (Betan et al., 2005; Russ et al., 2008). Interestingly, a recent study (Levy, Kivity, Diamond, Kernberg, & Clarkin, 2018) showed that a higher level of narcissism assessed dimensionally predicted both lower rates of dropout and longer time to dropout in patients in TFP, but not in dialectical behavior therapy (DBT) or supportive psychodynamic therapy in the New York–Cornell RCT. Thus, in TFP only, patients with a high level of narcissism (but not those with low levels of narcissism) were significantly less likely to drop out (13%, as compared to 33% for the latter group, which is comparable to ordinary rates of dropout for those with BPD). In addition, in

the same study we found lower rates of dropout in patients with DSM-IV/-5 NPD/BPD diagnoses relative to those with a BPD diagnosis alone (Diamond & Hörz-Sagstetter, 2012). These findings have encouraged clinical researchers at the PDI to extrapolate and refine the empirically supported treatment of TFP for the near-neighbor disorder, NPD.

This book provides a clinical guide that elaborates specific tactics and techniques for patients with narcissistic disorder at different levels of severity and with different presentations. The effectiveness of TFP for patients with BPD, including a number of patients with comorbid NPD, suggests that TFP is an effective treatment for NPD. However, to date there have been no clinical trials for TFP with patients with NPD only, as there have been no clinical trials for NPD individuals in other treatment modalities, such as DBT, mentalization-based therapy (MBT), schema-focused therapy, or short-term dynamic therapy. A first step toward establishing an evidence base for therapy with individuals with pathological narcissism and NPD is thus to further refine our techniques to treat them, and to demonstrate through case material the effectiveness of those techniques. To distinguish our approach from TFP for BPD, we refer to the approach outlined in this book as TFP-N.

A Look Ahead

In Part I of the book (Chapters 2–4), we outline the object relations model of conceptualizing pathological narcissism and NPD along with research from attachment, cognitive neuropsychology, developmental psychopathology, and social psychology to support it. In Part II (Chapters 5–9), we present how our contemporary object relations model is translated into an approach to assessment and therapy for narcissistic individuals at different levels of organization from high- to low-functioning NPD. Chapter 10 focuses on the theory and treatment of malignant narcissism, the most severe variant of pathological narcissism, and how leaders with this form of narcissistic pathology may extend it into the social and political realms by promising the restoration of supremacy and power at the group level, usually at the expense of out-groups, thereby catalyzing collective narcissism in the citizenry. Finally, since the functional impairments in those with narcissistic pathology are most evident in the sphere of intimate and family relationships, in Chapter 11 we focus on the characteristic difficulties in love relations when one or both members of the couple suffer from pathological narcissism. Chapter 12 provides an overview and summary of the object relations approach to understanding and treating pathological narcissism and NPD.

Summary and Conclusion

The development and refinement of treatment approaches for a spectrum of individuals with pathological narcissism from low to high functioning is necessary and timely. This is particularly true given the number of individuals both with narcissistic traits that characterize the general population and the prevalence of NPD as a clinically significant personality disorder; the rampant comorbidity of narcissistic pathology with other disorders that may complicate the treatment process; the heterogeneity of patients with narcissistic disorder; and the treatment complications they pose, including high dropout rates, treatment stalemates, and/or maladaptive transference–countertransference patterns. Several studies have established continuity between the characteristics of individuals with narcissistic traits and the more severe impairments of those with NPD; these include impairments in self-esteem and relational difficulties (Miller et al., 2011; Di Pierro et al., 2019; Paris, 2014), and even neurocognitive and neurobiological structures and functions (Fan et al., 2011; Chester, Lynam, Powell, & DeWall, 2016; Schulze et al., 2013). Thus, in the following chapters that delineate our model of theory and treatment, we include clinical and research studies on those with a full NPD diagnosis, as well as those with narcissistic traits. However, we do not believe that narcissistic pathology is the result of pathological amplification (Wright, 2016) of narcissistic traits but rather is distinguished by core structural features involving a particular constellation of idealized self- and object representations. Nonetheless, research investigations that look at narcissistic traits, as well as NPD, are useful to define the full spectrum of narcissistic pathology in its subclinical and clinical aspects. Our goal was to synthesize contemporary object relations perspectives with empirical research from other disciplines in order to develop an understanding of pathological narcissism and NPD that honors its multifaceted nature and presentations, and the challenges of treating those who suffer from it. We hope this clinical guide is useful for clinicians and clinical researchers from a variety of theoretical orientations in understanding the dynamic processes that contribute to the impairments in self and interpersonal functioning, and the symptomatology of narcissistic patients, and will spur them to further reflection and research on the treatment of pathological narcissism.

PART I

CONCEPTUALIZING PATHOLOGICAL NARCISSISM

CHAPTER 2

Self-Functioning in Pathological Narcissism

In this and the next chapter, we provide a detailed description of the contemporary object relations model of narcissistic pathology that we introduced in Chapter 1. The discussion incorporates research and theory from the psychoanalytical, developmental, social-cognitive, neurobiological, and sociocultural domains. First, we detail our object relations developmental model of normal and pathological narcissism. Second, we discuss further how an object relations model that emphasizes core structural features of narcissistic pathology, including distortions in self- (identity, self-esteem and affect regulation, maladaptive representations of self and others, primitive defenses) and interpersonal (empathy, intimacy, ethical standards) functioning, dovetails and diverges from the hybrid categorical/dimensional conceptualization of NPD delineated in the AMPD in Section III of DSM-5 (American Psychiatric Association, 2013). The AMPD is a dimensional diagnostic approach that emphasizes deficits in self- (e.g., identity, affect regulation) and interpersonal (e.g., intimacy, empathy) functioning at different levels of severity that are at the heart of narcissistic disorders across the spectrum. This formulation of NPD was derived after considerable debate between researchers who felt there was an inadequate empirical basis for the disorder and clinicians who found the disorder indispensable for understanding and diagnosing their patients. The latter group recognized that pathological narcissism encompasses a spectrum of disorders across varied levels of personality organization and functioning, and at different levels of severity that are not adequately accounted for by categorical criteria (Ronningstam, 2009). Third, we demonstrate how these deficits

in self- and interpersonal functioning may vary for individuals across the spectrum of personality functioning from higher functioning on the cusp of neurotic organization to lower borderline organization, as well as those with different expressions of pathological narcissism: grandiose, vulnerable, or malignant. We present case examples of individuals with pathological narcissism across this spectrum. In Chapter 3, we focus on the interpersonal functioning of those with narcissistic pathology; here, we concentrate on intrapersonal (self-) functioning.

Internalized Object Relations

An object relations model focuses on the central role of the internalization of the experience of early caregiving and other relationships. These come to be represented in the mind as cognitive-affective schemas of self in relation to others. Developmental researchers have now established that infants have inborn proclivities to relate to others through reciprocal imitation (imitating facial expressions and actions of others) and mutual emotional regulation (communicating visually, vocally, and kinesthetically with caregivers) in patterns whose synchrony or asynchrony affect adaptation and development. By the age of 1 year these co-regulated patterns are internalized in mental representations of self in relation to attachment figures (Ammaniti & Gallese, 2014; Ainsworth, Blehar, Waters, & Wall, 1978; Beebe & Lachmann, 2013). It is noteworthy, however, that recent infant research that looks at microanalysis of mother–infant interactions at four months shows that although mothers may show more varied and flexible behaviors, how infants regulate their behaviors in response to mothers varied considerably. This suggests that a complex, dynamic model of mutual regulation that takes into account infant intrapersonal (temperamental) as well as mother–infant interpersonal factors affects the nature of internalized mental representations (Beebe et al., 2010, 2012, 2016; Tronick, 2007).

These representations have their roots in actual interpersonal transactions with primary caregivers, in combination with the internal press of impulse, defense, fantasy, drive, and affect, both conscious and unconscious, as they evolve and are consolidated over different developmental stages. From an object relations perspective impulses or drives are inherently object seeking—that is, they are experienced and expressed in the context of early affectively charged relationships with others. Intra-individual factors (temperament, bodily experiences, affects, fantasy, impulses, and drives) enrich and expand the texture and arc of the representational world at different developmental stages—creating, from the beginning, differentiation and tension between self and other,

the psychological and the social, individual and society—that cannot be totally eradicated.

The term *representation* thus entails a stable mental organization, separate and distinct from external reality or momentary subjective experience that, once internalized, organizes interpersonal experience, as well as motivation and behavior. Internalized object relations are psychological structures composed of representations of self in interaction with significant others, along with their affective valence. These representations range from primitive and phantasmagorical to the relatively realistic, and are infused with affects of love and pleasure, anger and hatred, sadness and fear, as well as with wishes and fantasies. It should be noted that the internal world comprises myriad self–other affect dyads that organize consistent relational patterns into enduring mental structures, which in turn shape the individual's interpersonal experience. Object relations theorists, including Kernberg (1975; Kernberg & Caligor, 2005) and Fairbairn (1952) among others, have defined such internalized representations as crucial aspects of the tripartite structure of the id, ego, and superego. In Fairbairn's view, negative experiences with rejecting and overstimulating or tantalizing others are internalized as object relations constellations of a rejected self-persecutory other and an overstimulated self-exciting/tantalizing other, which then form an aspect of the unconcious. As the distinct and disparate object relations dyads of early psychological life become integrated into more complex structures, they form a representational core to the drives (id), the self (ego), and the system of ethical values (superego). Over the course of development the child struggles with contradictory and even polarized representations and associated feelings that are eventually integrated into coherent images of self and others with multiple complex aspects that form the basis of stable self-esteem, tolerance of a range of affects, and gratification of wishes and drives consonant with reality-based strictures and opportunities.

The relatively harmonious integration of multifaceted images of self and significant others promotes adequate reality testing in that the self is seen realistically while still encompassing guiding ideals. This also involves the capacity to manage impulses, affects, drives, and conflicts through higher-level repression-based defensive operations, as opposed to more primitive splitting-based mechanisms. From an object relations perspective conflicts between impulse and defense are embedded in patterns of opposing self–object affect dyads that comprise the representational world so that "the struggle between impulse and defense becomes the struggle between contradictory conflictual internalized object relations" (Kernberg, 2018, p. 52). For example, a self-concept as self-sufficient and grandiose in relation to devalued, inferior others, infused

with positive feelings of mastery and self-importance, may coexist and defend against a conflicting sense of self as inadequate in relation to others experienced as superior, powerful, or persecutory, imbued with negative feelings of shame and fear that are split off or denied.

It is precisely the lack of this integration that characterizes individuals with personality disorders, including narcissistic disorder. In those with narcissistic pathology, negative or painful affects and the unacceptable aspects of self along with the representations of feared or longed-for others with whom they are linked, are excluded from conscious awareness through splitting-based defenses that isolate positive from negative, and idealized from devalued aspects of experience. In those with personality disorders, including narcissistic disorder, what is internalized are both repeated interactions with others but also traumatic experiences that cannot be symbolically represented through language and other representational modes but instead remain split off or dissociated. Hence, the level and quality of the self–other affect dyads that comprise the internal world—and the extent to which they are integrated into a cohesive identity versus leading to a discontinuous or fragmented sense of self—is seen as a major etiological factor in and feature of the development of personality disorders. The work of TFP-N can be understood as helping an individual move from the internal division between extreme, unrealistic inflated and/or deflated views of self that lead to self-regulatory exploitation of others, entitled expectations, and compromised empathic capacity to a higher level (normal or neurotic organization) characterized by integrated, differentiated, and complex concepts of self and object representations. The latter are at the foundation of a cohesive, stable, if multifaceted, sense of identity; consistent ethical values; and intact sense of reality—all of which deepen the capacity for reciprocal love, genuine empathy, and pleasure in creativity and work.

Although object relations theory encompasses a range of psychoanalytic theories and approaches, the unifying factor is the central role of the internalization of aspects of early caregiving and other relationships for personality organization. The object relations model places maladaptive, distorted mental representations of self and others at the heart of understanding personality disorders.

The idea that cognitive-affective representational structures are the basis of personality development, as well as personality pathology, including narcissistic pathology, converges with the formulations of theorists and researchers from other conceptual frameworks, including:

1. Parent–infant researchers who have observed and documented the processes by which early caregiver–child visual, vocal, kinesthetic, and affective exchanges form rudimentary action schemas (e.g.,

representations of interactions that have been generalized [RIGs]; Stern, 1985) that set the template for later adaptive or maladaptive relational patterns (Beebe & Lachmann, 2013; Ainsworth et al., 1978; Stern, 1985; Tronick, 2008).

2. Attachment theory and research, a variant of object relations theory developed by Bowlby (1977, 1980) and his followers who have demonstrated how early parent–child attachment patterns are anchored in the representational world as secure or insecure internal working models (IWMs) of attachment (Main & Goldwyn, 1998). These IWMs affect subsequent cognitive, emotional, and relational development from infancy through early adulthood (Main, Kaplan, & Cassidy, 1985; Waters, Merrick, Treboux, Crowell, & Albersheim, 2000; Waters, Hamilton, & Weinfield, 2000; Lyons-Ruth & Spielman, 2004) and have been found in numerous international studies to be insecure and/or disorganized in those with personality disorders, including NPD, and are transmitted across generations. The contributions of attachment theory and research on our model is discussed further in Chapter 9.

3. Cognitive-behavioral approaches, which stipulate that cognitive schemas based on early interactions with others serve as organizers of current and future interpersonal transactions (Beck, Freeman, & Davis, 2004; Morf & Rhodewalt, 2001; Young & Flanagan, 1998; Zeigler-Hill, Green, Arnau, Sisemore, & Myers, 2011). Specific to narcissistic pathology are the investigations of Zeigler-Hill and colleagues, who found that a number of early maladaptive schemas were associated with aspects of narcissistic pathology—including the entitlement schema involving the belief that one should be able to do or have whatever one wants regardless of the consequences for self and others—and the self-aggrandizer and bully and attack schemas (Lobbestael, Van Vreeswijk, & Arntz, 2008).

Representations of self and others in all of these traditions are thought to evolve out of the interaction between early relational experiences and the intra-individual factors, although each of these conceptual frameworks may give somewhat different weight to the balance between these two dimensions (Diamond & Blatt, 1994; Eagle, 2003; Parish & Eagle, 2003; Steele & Steele, 2008; Blatt, Auerbach, & Behrends, 2008). Relatively enduring representations of self and others are thought to underlie the development of increasingly mature modes of interpersonal relatedness and the gradual consolidation and emergence of a differentiated, cohesive identity in all of these conceptual frameworks (Blatt, 1974, 1990; Kernberg, 1975, 1976, 1994; Kohut, 1971; Loewald, 1960,

1979; Mahler, Pine, & Bergman, 1975; Sandler & Rosenblatt, 1962; Stern, 1985).

The object relations perspective is distinguished from these other models in two major ways. First, how early parent–child interactions are experienced is a result, in part, of the internal press of the child's impulses, affects, and fantasies as he or she goes through different developmental stages, as well as of individual temperament, including constitutional capacities and limitations (Kernberg, 1975, 1984, 1997; Ammaniti & Gallese, 2014). Second, the dynamic nature of these representations of self and other means that different representations come to the fore or recede dynamically depending on the individual's developmental stage, activated mental state, defenses, or interpersonal experience. Hence, internal object relations are never veridical representations of actual interactions, but rather a dynamic synthesis of actual and fantasied, conscious and unconscious experiences, deriving from multiple behavioral systems, such as attachment, sexuality, affiliation/play, and aggression (Bowlby, 1982a, 1982b; Kernberg, 2018; Lichtenberg, Lachmann, & Fosshage, 2011; Panksepp & Biven, 2012).

From the contemporary object relations perspective that informs TFP-N, we recognize that narcissistic pathology at any level of organization, from borderline to high functioning, results in a dynamic interaction of the following dimensions:

1. Familial factors, such as exploitative, overinvolved parental attitudes in which the child is treated as an extension of the self (Kernberg, 1974; Otway & Vignoles, 2006), and certain attributes of the child (e.g., unusual physical, intellectual, or creative characteristics) are inordinately valorized, while the real needs are neglected. In more extreme cases, the child may suffer from abuse, neglect, or loss (Cater, Zeigler-Hill, & Vonk, 2011; Cohen et al., 2014; Ensink, Berthelot, Bégin, Maheux, & Normandin, 2017; Maxwell & Huprich, 2014).

2. Temperamental factors, such as early sensitivity to approach and rewards that may make the child more prone to seek praise and attention (Elliot & Thrash, 2002; Thomaes, Brummelman, Bushman, Reijntjes, & Orobio de Castro, 2013). Alternatively, the child may avoid closeness with others and cope with adversity through extreme self-reliance (Bowlby, 1988), or have difficulty with affect regulation and poor frustration tolerance (Schore, 1994). A number of twin studies have shown a 25–79% heritability rate of NPD, with higher rates (70%) found in the clinical studies and lower rates (25%) from the nonclinical groups (Jang, Livesley, Vernon, & Jackson, 1996; Kendler et al., 2008; Torgersen et al., 2000, 2008, 2012). Although these

studies suggest that genetic or constitutional factors may contribute to the development of narcissistic disorder, it is not clear how heritability estimates encompass both main genetic effects and gene/environment interaction effects. Thus, the meaning and relevance of these findings for understanding the development of NPD and pathological narcissism in adolescence or adulthood necessitate further research investigation.

3. The development of insecure and/or disorganized internal working models of attachment, particularly the dismissing/avoidant and/or preoccupied/anxious states of mind with respect to early attachment experiences and relationships, or disorganized oscillation between the two (Levy et al., 2006; Diamond, Levy, et al., 2014). Related to insecure attachment are deficits in mentalization and epistemic trust (the capacity to understand behavior in terms of intentional mental states in order to recognize who it is safe to trust and learn from in the family and larger social group [Bateman & Fonagy, 2016; Fonagy et al., 2002; Fonagy, Luyten, Bateman, Gergely, Strathearn, et al., 2010; Fonagy & Allison, 2014]).

4. Neurocognitive deficits, such as deficits in empathic processes—that is, impaired emotional empathy (the capacity to experience the feelings of others while still differentiating their emotional state from one's own) in the context of intact cognitive empathy (the capacity to understand and reflect on the perspectives of others without necessarily sharing their emotional state (Baskin-Summers, Krusemark, & Ronningstam, 2014; Ritter et al., 2011); impairments in emotion recognition and affect regulation, particularly the expression and management of negative emotions, including sadness, envy, and anger (Cascio, Konrath, & Falk, 2015; Marissen, Deen, & Franken, 2012). These may be linked to neurobiological deficits, such as abnormalities in brain structure and function (e.g., grey and white matter deficits and variants in the right anterior insula), although further research is necessary to understand the role of such neurobiological factors in the development of NPD (Schulze et al., 2013; Chester et al., 2016).

5. Sociocultural influences, such as the diminution of parental authority in the face of the powerful image veil of social media that allows for the construction of a chimerical, protean sense of self; and social mobility and the fragmentation of family structures that undermines community cohesion and with it a continuous and coherent sense of identity (Lasch, 1979; Twenge, Campbell, Hoffman, & Lance, 2010; Twenge & Campbell, 2009).

6. Intrapsychic structural features, notably the development of a specific configuration of mental representations of self and others—the

pathological grandiose self. In the pathological grandiose self, aspects of the real self (the actual qualities and characteristics of the self) are combined with idealized aspects of self (the ego ideal or who one aspires to become) and idealized aspects of others (admired characteristics of others). All that is positive or ideal tends to be appropriated to the self, while negative aspects of the self are split off and condensed with negative aspects of others on whom they are systematically and rigidly projected. The pathological grandiose self is a defensive structure that provides some stability of functioning, but at the cost of genuine interpersonal relating, in that it empties the mind of complex, realistic mental representations of self and others, and enables a devotion to a narrative of the self that is at a distance from how the person is objectively perceived by others (Kernberg, 1974, 2007, 2010; Rosenfeld, 1964).

Figure 2.1 depicts the interaction of these six dimensions. Each of these factors contributes to our contemporary object relations model of pathological narcissism and NPD, and each is further discussed in the subsequent sections of this chapter. We first discuss our object relations model of narcissistic pathology and then present illustrative case material to show how it diverges or converges with the formulations of the disorder found in DSM-5.

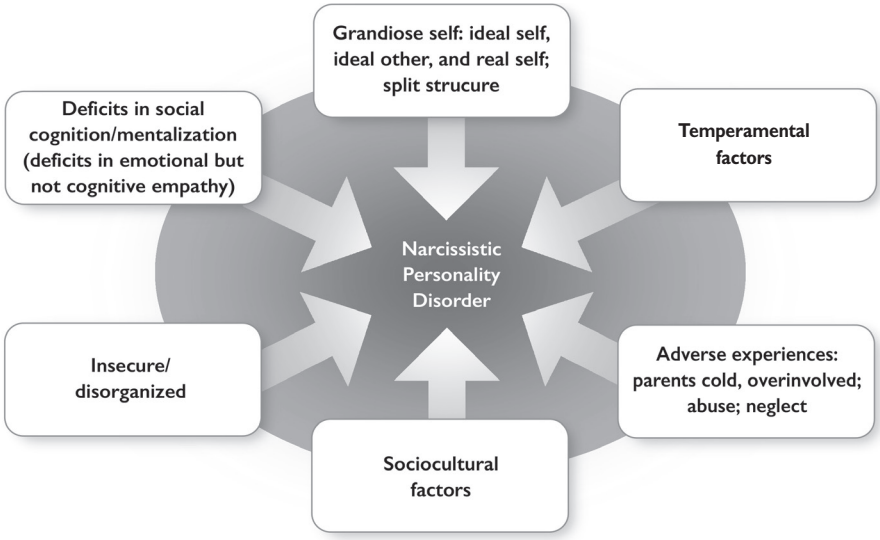


FIGURE 2.1. Factors contributing to narcissistic personality disorder.

The Development of Normal and Pathological Narcissism

How do representational structures that underlie normal and pathological narcissism develop? From an object relations perspective, mental representations of self and others are initially encoded in the developing mind under the sway of two disparate but constitutionally determined affective axes: a rewarding, gratifying, and pleasurable one reflected initially in positive self–object affect dyads, or representations of self and others; and a frustrating, aversive, and painful one reflected in negative self–object affect dyads. Through multiple transactions with primary caregivers, inborn affect dispositions are activated, eventually crystallizing in the mind as a representation of the self in interaction with a representation of another infused with a dominant affect.

The array of such self–object affect dyads forms the basic psychological structures of the mind. The developmental literature suggests that the potential for developing representations of a subjective experience with others begins at birth, as evidenced by the newborn's preference for schematic drawings of human faces versus facial features randomly presented (Mondloch et al., 1999). By 2 months, infants show the capacity for object representation (Carey, 2009; Erreich, 2015). The origins of the self and object representations are inherently social in that they grow out of an inborn proclivity toward reciprocal interactions and mutual affective exchanges from birth on. In numerous studies that are beyond the scope of this work, developmental researchers have now shown that infants from birth on are adept in responding to their caregivers, in imitating their caregiver's motor actions and facial expressions (Legerstee, 2005), and in engaging them in social interactions that they internalize in increasingly complex ways. Indeed, rudimentary capacities for internalization of such interactions are evident in the infant's capacity to imitate the motor actions and gestures of others (Legerstee, 2005), recognize and reproduce a caregiver's facial expressions, and show primitive empathy or affective resonance with the states of others (e.g., infants cry in response to the cry of other infants, although this may vary according to infant temperament; Bernhardt & Singer, 2012)—a point that may have relevance for the deficits in empathy in those with narcissistic disorder. Infants have also been observed to prefer attachment figures over others, and to engage in contingent, synchronous interactions with them that are observable as early as 4 months in highly idiosyncratic dyadic patterns of mutual regulation and response (Beebe et al., 2016; Ammaniti & Gallese, 2014; Cohn & Tronick, 1988; Tronick, 1998, 2008). Interestingly, infants have been observed to also show evidence of self-conscious emotions, such as embarrassment, pride, and coyness—which we might link to the development of self-awareness

and narcissism—suggesting that "self-conscious emotions . . . exist in simple form in the first year and are crucial for shaping the infant's emerging conceptualization of self and other" (Reddy, 2005).

The foregoing aspects of infant self and interpersonal functioning are internalized in increasingly complex sets of integrated and differentiated mental representations. Such representations have their origins in the infant's experience of actual parent–child transactions and embody not so much images of the parent and child but of their relational transactions. In addition, the representational world continues to evolve as multiple behavioral systems, such as attachment, sexuality, and aggression (Bowlby, 1982a; Lichtenberg et al., 2011; Panksepp & Biven, 2012), become more or less salient over the course of development. Gradually, the internal world of mental representations coalesces into a separate sphere of intrapsychic reality that we refer to as object relations (Ammaniti & Gallese, 2014; Kernberg, 2015). Object relations form an enduring intrapsychic organization or template that both shapes interpersonal experience and is shaped by it in a hierarchical spiral of increasing complexity throughout the life cycle. As the child matures, he or she is able to make use of dialogue with internal objects, at both the conscious and unconscious levels, for self-esteem and affect regulation.

From an object relations perspective, the self is a composite of the constellations of dyadic and triadic mental representations. In normative situations, the self–object affect dyads become increasingly more integrated (i.e., combine positive and negative qualities) and differentiated (i.e., images of self become increasingly separate and distinct from those of others). The increased integration changes how the child experiences others, in that in the face of frustration by a significant other, the child may rely on a wellspring of benign representations, allowing him or her to tolerate and modulate negative feelings rather than enacting them. In similar fashion, differentiation involves the capacity to experience the self as separate and autonomous and to tolerate aloneness and exclusion from intimate relationships between significant others. In terms of the latter, there is growing awareness from birth on of triangulation in which the child becomes cognizant not only of dyadic links between self and caregiver but of the exclusive link between the caregiver and significant others in his or her family. The ability to hold in mind such multiple representations of self in relation to others forms the basis for autobiographical memory or "the establishment of an abstract historical causal self concept" (Fonagy et al., 2002, p. 247) that forms the basis for a unified and coherent self. This is referred to as a *third position* or *triangular space*, that enables the child to take multiple perspectives on the self in relation to others, seeing the self from multiple perspectives in interaction with others and "entertaining another point of view whilst

retaining our own" (Britton, 1989, p. 87). Thus, while triangular object relations may catalyze conflicts around infantile sexual strivings and associated hostile, rivalrous, and loving fantasies and wishes (Kernberg, 2018), they also shore up reality testing in that they involve a more realistic sense of the complex relationship between self and significant others, and the differentiation between wished-for and real aspects of such relationships.

Ideal aspects of the relationship between self and others become internalized as the ego ideal, an internalized structure that embodies ideal, valued aspects of caregivers, allowing for their internalization and toning down of parental prohibitions. The ego ideal encapsulates the young child's early experience of perfection, both of the self and of cherished objects, and of oneness with the idealized attachment objects. In the ego ideal, we see the seat of the idealizing mechanisms so evident in narcissism and its disorders. The ego ideal involves "a point of contact between infantile omnipotence and the object relationship, between the pleasure principle and the reality principle" (Chasseguet-Smirgel, 1985, p. 28). Thus, an integrated self involves positive, realistic, and stable, as well as ideal, concepts of the self, but also the capacity to tolerate episodic times when one does not live up to the ideal without dysregulation in self-esteem, or harsh self-criticism. Put differently, in the normative situation, the ego ideal becomes an aspect of the superego, constituting a benign and even protective, stabilizing internal function for the individual, especially in the face of disappointment, deprivation, or danger (Schafer, 1960). While an integrated self protects against the threat to the sense of self in the face of failure, an integrated concept of others ensures that shortcomings, disappointments, and conflicts can be tolerated, and negative, aggressive feelings managed without ruptures.

Over the course of development such dyadic and triadic mental representations become integrated into a cohesive self-concept and concept of significant others as whole, separate objects. Such an integrated internal world of self and object relations defines the quality of the individual's identity. A strong identity is predicated on mental representations of self and others that are well differentiated, mature, and complex, and infused with a range of nuanced affects. Disturbances in the sense of identity, on the other hand, stem from an internal world that is poorly differentiated (e.g., aspects of self and others are confused or not totally distinct, although a sense of physical boundaries between self and other may remain relatively intact), unintegrated (e.g., positive and negative, and idealized or devalued aspects remain polarized), and inflexible (e.g., certain extreme self–object affect dyads remain rigidly dominant while others are split off or dissociated and infused with primitive and extreme unmodulated affect states).

Recent research has affirmed that positive and negative dimensions of experience are activated and processed by different regions and structures of the brain, and hence initially remain segregated from each other. Integration and modulation of these polarized affects and representational states occur at the psychological level over the course of development, both in terms of the cognitive framing of affects and in terms of the modulation of affect states. Such integration occurs only with the maturation of limbic structures and functions in conjunction with cortical structures and functions, particularly the prefrontal and preorbital cortex and the anterior cingular junction (Ripoll, Snyder, Steele, & Siever, 2013; Kernberg, 2014; Decety, 2012). It should be noted that while the hippocampus represents the structure involved in recording and maintaining affective memory, it is hypothesized that only at the higher cortical level, at the prefrontal and preorbital cortex–anterior cingular junction, that primitive positive and negative affect systems and the representations linked to them can be modulated and integrated into coherent, mature cognitive-affective structures (Kernberg, 2014; LeDoux, 1996, 2002).

Healthy or adaptive narcissism is predicated on such integrated and modulated mental representations of others that allow for the regulation of affect and self-esteem regardless of the vicissitudes of life and relationships. Healthy narcissism requires the capacity to experience pleasure in self-expression, self-affirmation, and creativity; to dedicate oneself to a set of consistent and realistic ideals and goals, and the capacity to invest deeply in enduring relationships with others while retaining autonomy. These aspects of adaptive narcissism, like object love, are to some extent a function of the internal world of object relations. A benign and integrated internal world provides depth and continuity to the experience of self, forming the basis for a consolidated identity that allows one to invest consistently and deeply in gratifying relationships that withstand the inevitable developmental and environmental pressures that ensue over the life course, conferring internal strength in the face of the inevitable losses, disappointments, and failures in life. As Kernberg (1975) puts it, the development of healthy narcissism and healthy object relations "tend to go hand in hand" (p. 323). It should be noted that such an integrated identity is by no means one-dimensional or constricted—rather, it involves the capacity to integrate the multiple roles and aspects of the self into a unitary self that is felt to be continuous and consistent over time and to reflect on how different self states operate in relation to each other and to reality.

Pathological narcissism diverges from both normal adult narcissism and fixation at stages of infantile narcissism, in that it involves the investment of libido (loving feelings) and aggression (hateful feelings), not only

in the self but in a pathological self-structure (Kernberg, 1984). From an object relations perspective, there is not necessarily a direct continuity between pathological and normal narcissism, or between narcissistic disorders at the individual level and narcissistic traits in the general populace, although many individuals in contemporary society have been found to have narcissistic traits, such as obsession with self-aggrandizement, inflated self-esteem, lack of concern and empathy for others, and entitled expectations (Twenge et al., 2008a, 2014) that converge with aspects of pathological narcissism. But in our view, those with pathological narcissism are characterized by a distinct intrapsychic structure, the pathological grandiose self, which is absent in normal narcissism even when the individual is high on narcissistic traits. In the pathological grandiose self, the idealized mental representations of others combine with ideal representations of the self to form the core of the self, while negative aspects of the self are projected and combined with negative aspects of others. In both cases, aspects of others are condensed with images of the self in ways that distort the internal world of object relations and interpersonal experience. The degree of pathology varies depending on the extent to which positive, libidinal feelings predominate over aggressive ones in the idealized self–other configuration, as is the case with higher-functioning narcissistic personalities. Negative, aggressive affects may predominate, as is the case with lower-functioning narcissistic personalities and those with malignant narcissism. In either case, the narcissistic structure, or pathological grandiose self, sustains the stability of the self, but at the cost of interfering with the development of a more realistic, mature view of self and others separate from one's projections, and therefore the possibility of genuine closeness with others.

In individuals with personality disorders, including narcissistic disorder, one-dimensional and caricatured representations of self and others remain highly polarized, retaining a split between the positive and negative dimensions of experience. An example of such polarized dyads in individuals with pathological narcissism would be a grandiose, superior self in interaction with an inferior, depreciated other infused with affects of contempt, or a vulnerable, fragile self in interaction with an idealized other with linking affects of fear and/or anger, although one self-experience is usually dominant. Such polarized representations remain unintegrated, resulting in the absence of the more complex, modulated mental representations that provide a sense of coherence and richness to the individual's experience of self and others. The self-experience of those with personality pathology, including pathological narcissism and NPD, vacillates between contradictory identifications that are set apart from one another, resulting in shifts between the self as grandiose and complete, and the self as vulnerable, divided, and depleted—mobilized

particularly in situations that threaten the narcissistic equilibrium. This leads to discontinuities in the sense of identity.

Structural Features of Pathological Narcissism

In the object relations framework that initially guided the conceptualization of both borderline and narcissistic disorders, both disorders were hypothesized to share core structural features based on maladaptive, distorted mental representations of self and others (Kernberg, 1975, 2007, 2018). These structures underlie the disturbances of self, identity, interpersonal functioning, and reality testing—with disturbances across all of these dimensions buttressed by "primitive" defensive strategies for the management of intolerable self-states, affects, and dynamic conflicts. This structural model presupposes that individuals with personality disorders are characterized by distinct ways of experiencing self and significant others that lead to consistent relational patterns, and further that such relational patterns reflect the underlying structures that catalyze the maladaptive, descriptive, observable features of the disorder (Kernberg & Caligor, 2005; Caligor et al., 2018). From a structural point of view, individuals with personality disorders, including those with narcissistic personality pathology, have impairments in the following domains of self- and interpersonal functioning:

- *Identity*. The capacity to maintain a coherent, integrated sense of self without undue reliance on others for self-definition or self-esteem regulation versus identity diffusion or the lack of integration in the sense of self, which, in those with narcissistic pathology, leads to contradictory self-appraisals, ranging from grandiose/inflated to vulnerable/deflated.
- *Defenses*. The reliance on primitive defenses based on splitting and dissociative mechanisms that involve the need to defensively sequester contradictory representations of self and others versus more mature defenses that take into account the complex nature of reality even while more troublesome aspects may be banished from awareness through repressive mechanisms.
- *Quality of object relations*. The degree to which the internal world of mental representations of self and others is well differentiated (i.e., seen as separate) and integrated (i.e., a nuanced and modulated blend of positive and negative aspects) versus split, extreme, and rigidly polarized into idealized and devalued sectors. While the former fosters the development of enduring, in-depth relationships with

others, the latter leads to shallow, superficial relationships or interpersonal patterns of exploitativeness, self-promotion, and entitlement.

- *Empathy.* The capacity to share and understand the emotions of others while still discriminating between one's own feelings and those of others versus callous disregard of others' feelings, or their use to manipulate others in one's own interests.
- *Ethical values.* The capacity to develop and sustain a set of ethical principles and values that blends adherence to universal and culturally sanctioned moral standards with individualized values versus adhering to standards that are overly rigid, perfectionistic, and constrictive, or overly lax and permissive (e.g., bending rules in the interest of self-aggrandizement).
- *Reality testing.* The capacity to recognize the consensual criteria of reality and accept it, and to distinguish between what belongs to the realms of self and other, internal and external, versus rejection of alternative realities that do not correspond to the individual's grandiose view of self.

Pathological narcissism encompasses a spectrum from higher functioning to lower borderline organization, depending on the level of impairment along the above dimensions of personality functioning. These levels of organization are depicted in Figure 2.2 and are described below.

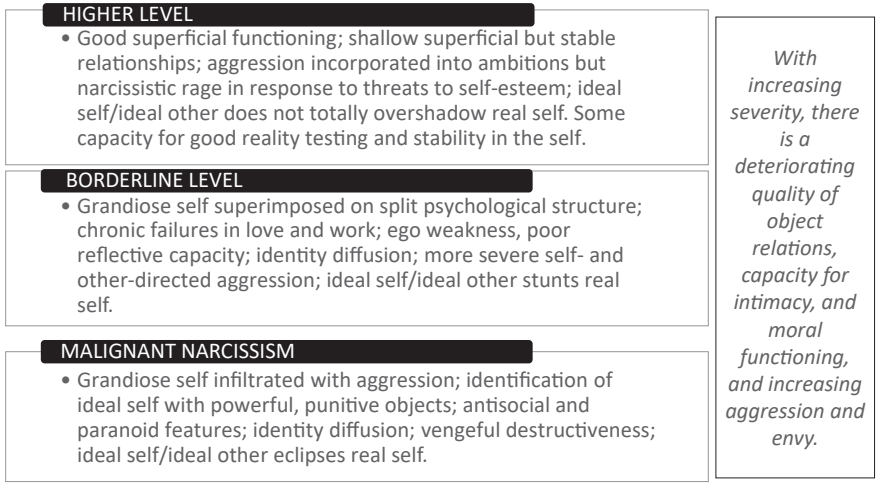


FIGURE 2.2. The spectrum of narcissistic pathology.

Pathological Narcissism at Different Levels of Organization

High-Functioning Narcissistic Personalities

High-functioning narcissistic individuals may have a relatively stable grandiose self-organization often maintained by a thick defensive system that keeps at bay any devalued aspects of self that are projected onto others seen as inferior or subservient. As long as such defensive operations are successful, those narcissistic personalities who function at a higher level are able to maintain consistent and even high self-esteem and stable, if somewhat superficial, and/or exploitative interpersonal relationships, and show a high level of accomplishment and success in work and/or creative pursuits. This is because positive affects predominate over aggressive, negative ones in the grandiose self, and the combination of ideal others and ideal self is not so pathological that it eclipses some reality-based aspects of the self (the real self). Aspects of the real self are well developed in line with the perspective of consensual reality so that reality testing about self and others remains relatively intact most of the time. In addition, in those with higher-functioning narcissism, there is adherence to ideals and values beyond self-aggrandizement, although there are occasional lapses in ethical functioning. Individuals with narcissistic pathology functioning at a higher level may engage in altruistic acts and show remarkable generosity toward others and to cultural institutions, particularly when they receive public recognition that enhances their sense of agency and social position by doing so (Gabbard & Crisp-Han, 2016; Di Pierro et al., 2017). The foregoing features bespeak some intact superego functioning, which also shores up reality testing and protects the individual from the flat-out rejection of aspects of reality that challenge the narcissistic illusions noted in the more disturbed narcissistic personalities. It should be noted that the very qualities that fuel narcissistic pathology, such as extreme self-focus to the exclusion of relationships, self-serving ambition, and self-promotion, may in fact contribute to the narcissistic individual's success in the occupational and social spheres.

Higher-functioning narcissistic personalities may also have some intact capacity for relatedness, including long-term love relationships and friendships that appear to be mutual and sustaining. Couple relationships that are perceived to be ideal may be superficial and lacking in mutual emotional sharing and enduring sexual passion, or they may be based on narcissistically sustaining fantasies of perfect mirroring or twinship (Kohut, 1971, 1977; e.g., that the other intuitively knows at all times what the individual is thinking and feeling), or on exploitation of the other, who is expected to provide endless admiration and

self-abnegating devotion. Higher-level narcissistic individuals are often attractive, accomplished, and socially adept, inspiring respect and veneration, and creating an aura of specialness and uniqueness that may encompass others who seek to enhance their own status accordingly. If such relationships remain intact, the higher-functioning narcissistic person may negotiate life successfully, and indeed may never seek treatment. However, if such social or love relations are threatened or break down, as they are prone to do when others tire of meeting the expectations for adulation and admiration, if there are sudden work-related failures or disappointments, or even a slow decline of prominence or achievement that may come with aging, higher-functioning narcissistic individuals may experience enough emotional distress to seek treatment. In such cases, what has been a stable, thick, defensive wall between the grandiose and vulnerable aspects of the self may collapse. And since self-esteem and affect regulation are intersected, threats to the self may plunge the individual into dysregulated states of fear, rage, or despair. In such situations, the higher-functioning narcissistic person may experience debilitating symptoms of depression, anxiety, panic attacks, and somatic preoccupations, and are more prone to seek symptom-relieving treatment when in that state. Thus, a careful assessment of the differential diagnosis is warranted between major depressive disorder proper versus the states of emptiness, depletion, and collapse associated with the experience of acute withdrawal of narcissistic supplies.

Case Example: Mark—High-Functioning Narcissistic Disorder

Mark, presented in Chapter 1, epitomizes a high-functioning narcissistic personality in whom a relatively stable grandiose self ensures characteristics of social potency, extroversion, and assertiveness, coupled with vulnerable features, including sudden plunges into states of self-disgust, self-doubt, and behavioral or affective dysregulation when faced with limitations, ego threats, or failures (Gore & Widiger, 2016; Miller & Maples, 2011; Hyatt et al., 2017). As is the case with many higher-functioning narcissistic individuals, the gap between ideal and real self-representations is not so drastic that real ambitions and aspirations cannot be met. He was quite talented and successful, a leader in his field, and his sense of uniqueness and specialness had some grounding in real accomplishments and status.

- *Identity.* Exaggerated sense of self-importance but also harsh critical elements of internal world; not in sync with realistic self-appraisal. Sense of self-importance and specialness may have some

grounding in real accomplishments and status. Gives a semblance of identity integration and stability. Lack of integration in self-concept, with oscillations between states of exaggerated self-importance and feelings of self-denigration, which are often hidden. Excessive reliance on admiration and accolades of others for self-definition and self-esteem regulation. Suffers from subtle feelings of inauthenticity and futility.

- *Self-direction*. Although realistic, goals are often unreasonably high (need to be special and always in limelight of field).
- *Empathy*. Impaired ability to recognize or identify with the feelings and needs of others; excessively attuned to reactions of others but only if perceived as relevant to the self. Perceptive about the motivations, thoughts, and feelings of others, particularly when their reactions are relevant to his own needs and imperatives.
- *Intimacy*. Relationships are superficial and structured to meet his self-regulatory and self-esteem needs. Some capacity for mutuality and commitment to friends, but difficulties with establishing and maintining intimacy on a deeper level. Others either idealized as individuals who reflect the self's perfection or are devalued. Mutuality constrained by little genuine interest in others' experiences and predominance of need for self-promotion or personal gain. Incapacity to deal with dependency needs in a mature, mutual manner. Leads to detachment, feelings of aloneness, and isolation.
- *Ethical functioning*. Adheres to ethical standards and values most of the time, but has a tendency to bend or disregard rules to promote or aggrandize the self.

Pathological Narcissism in Low Borderline Personality Organization (BPO)

Individuals with pathological narcissism in the context of BPO may show some of the typical deficits in the self and interpersonal dimensions described above for high-functioning narcissistic persons. However, they present with more severe identity diffusion that shows up in the inability to develop and pursue long-term goals or directions, with more drastic fluctuations between arrogant superiority and contempt for others, and feelings of inferiority, humiliation, and self-disgust, that lead in more extreme cases to compromised reality testing. In addition, narcissistic personalities organized at a borderline level show a lack of anxiety tolerance, impulse control, and affect regulation—all evidence of a pathological grandiose self who is not as well constituted (Kernberg,

2007, 2010). With more disturbed narcissistic personalities, the ideal self/ideal other constellation that forms the core of the grandiose self casts a longer shadow over the real self, leading to highly distorted views of others that interfere with the capacity to work toward realistic goals, sustain enduring intimate relationships, or maintain commitments to ethical standards or ideals beyond self-aggrandizement or slavish adherence to an ideal other, with the two often not well differentiated. In fact, what distinguishes pathological narcissism in the context of low borderline organization is the propensity to be sustained by a hidden conviction of unique greatness unrecognized by others but still to be realized, or of fantasies of perfect love. There is often a pervasive denigration of others combined with intense envy of what others have, so that devaluation protects against envy. These individuals also have an unusual hunger for appropriating things (e.g., attributes, connections, material possessions) of others. They believe that having everything unto themselves protects against envy, but this alternates with extreme entitlement, rage, and envious attacks if they do not get what they think they deserve. Superego functioning is compromised, and there is a tendency to project prohibitive aspects of the superego onto the external world and to adhere to norms not out of clear and consistent ethical standards but out of fear of punishment.

Deficits in the capacity to develop and adhere to consistent goals is evident in chronic unemployment, underemployment, or failure in work. More severe distortions in the internal world of object relations manifests itself either in immersion in exploitative, controlling, and/or chaotic relationships, or in chronic failure to establish or maintain enduring relationships of any sort. This may be evident in rapid shifts in transference developments between periods of idealization in the relationship with the therapist and wholesale devaluation often expressed in wishes to leave treatment. There is a limited ability to understand and reflect on the thoughts, feelings, and motivations underlying the behaviors of self and others, and little awareness of the impact of behavior on others. This limits empathy for others and the depth of engagement with them. In some cases, the person withdraws to an illusory world where painful feelings about failure to achieve grandiose, unrealistic goals may be avoided, often leading to detachment from and devaluation of existing connections and opportunities, including the connection to the therapist. This is particularly the case where the grandiose self has served a defensive function to protect against the sequaele of traumatic experiences (e.g., sexual or emotional abuse and neglect in early childhood or adolescence).

In those with lower-level borderline organization, the projection of the devalued, intolerable aspects of self is less stable, leaving the

individual less capable of managing the negative feelings of envy, anger, fear, and humiliation. In contrast to higher-functioning narcissistic individuals, those with structural weakness in the grandiose self may have a thinner defensive barrier between idealized and devalued aspects of self, leading to more severe, chronic fluctuations in self-esteem, more affect instability, and more chaotic interpersonal functioning. Projective mechanisms do not work as seamlessly as they do in those with higher-functioning narcissistic disorder, and as a result, individuals may oscillate between fearful submission to idealized others and triumph over those they devalue. Feelings of inferiority, shame, and sensitivity to rejection sometimes obscure the extent to which grandiosity is a major organizing aspect of the self, and accounts for the persistent sense of having been misunderstood or deprived. Such grandiosity may also take the form of a "syndrome of arrogance" (Bion, 1957) involving withering disdain for and destructive attacks on others, and even on the treatment process itself. These may include self-destructive or destructive behaviors that represent unconscious efforts on the patient's part to paralyze the therapist in order to protect the identification with the illusory grandiose self.

Case Example: Rebecca—Low Borderline NPD (BPD/NPD)

Although there are many indications of borderline pathology in the description of Rebecca offered in Chapter 1, including self-destructiveness and identity diffusion, among others, narcissistic features were evident in her sense of self as special and unique as a result of parental overvaluation, her ambitions that outstripped her actual potential, and her preoccupation with envy. From a structural object relations point of view, Rebecca showed much evidence of NPD organized at a low borderline level with moderate to severe levels of impairment in the realms of identity, self-direction, empathy, and intimacy.

- *Identity.* Split in her self-concept between an inflated sense of self as her father's pet, "the pretty one," and a deflated sense of self as ugly and impaired; inordinate reliance on valuations of others for self-definition and self-esteem regulation.
- *Self-direction.* Goals and accomplishments were not so much self-generated as a means of gaining parental attention and approval; unrealistic set of personal standards not in line with actual potential.
- *Empathy.* Difficulty identifying and considering the feelings of others; unable to consider the perspectives of others.

- *Intimacy.* Few friends and no enduring, intimate relationships. Capacity for intimacy limited by expectation of abandonment or rejection; exhibits belligerent behavior in the face of criticism or threats to self-esteem with little awarenss of the impact of this on others.
- *Ethical functioning.* Ethical standards highly inconsistent; either unduly harsh or lax.

The Syndrome of Malignant Narcissism

Individuals with the most severe narcissistic pathology present with a grandiose self that is infused with ego-syntonic aggression, paranoia, and antisocial features, leading to the syndrome of malignant narcissism (see Chapter 10 for a full description of this more severe form of pathological narcissism). In such a configuration, ego-syntonic aggression can be a source of empowerment and may be directed to antisocial behavior towards others or to attacks on the self in the form of self-mutilation or self-destruction. Individuals with malignant narcissism show little concern with pain and death, and in fact may take proud pleasure in self-destructiveness and destructiveness. Although their capacity for relatedness is limited, they maintain some capacity for emotional investment in a relationship with significant others, and some capacity for experiencing guilt and concern that protects them from total submission to destructive forces (Kernberg, 1984, 2004). They also have some capacity for forming attachment bonds with others, although their attachment status is usually insecure and disorganized. With regard to the latter etiology, the constitutionally given dominance of negative affective systems typical for all severe personality disorders applies to those with malignant narcissism, and these are often linked to frustrating and traumatic experiences in infancy and/or childhood. One frequently finds a predominance of negative environmental features, including parental absence or sadistic parental behavior, physical or sexual abuse, or chronic witnessing of physical or sexual abuse.

Case Example: Michael—Malignant Narcissism

The confluence of grandiosity, aggression, and antisocial features in Michael's presentation (see Chapter 1) indicated that he was characterized by a more severe form of narcissistic pathology: the syndrome of malignant narcissism as articulated above (Kernberg, 1984, 1986, 2007).

- *Identity.* Self-concept is highly unstable and contradictory. Oscillations between the conviction of greatness (gifted artist who

awaited discovery) and feelings of self-loathing and worthlessness expressed in chronic self-defeating and self-destructive behaviors (binge drinking and self-mutilation). Sense of emptiness and futility. Excessive reference to others for self-definition and self-esteem regulation; poor boundary between self and others.

- *Self-direction*. Severe difficulty establishing or adhering to personal goals; life experienced as meaningless or dangerous; unrealistic set of personal standards; severe limitations in capacity to reflect on mental states.
- *Empathy*. Impaired emotional empathy and cognitive empathy; confusion and lack of awareness of impact of actions on others; projects destructive motivations onto others; unable to consider alternative perspectives or understand behavior in terms of underlying mental states. Feels bewildered or threatened since destructive motivations are frequently attributed to others.
- *Intimacy*. Relationships based on beliefs about expectation of abandonment or abuse; desperate attempts to connect through aggressive behaviors toward others. Relationships are exploitative and based on personal gain. Minimal capacity for mutuality; paranoid constructions of motivations of others.
- *Ethical functioning*. Lack of a clear consistent set of ethical standards guiding behavior; antisocial features. Need to triumph over others and may engage in self-destructive and destructive actions as a means of getting back at others for past injuries, real and imagined. Idealization of harsh and exacting ethical standards or disregard of moral codes out of self-interest.

In subsequent chapters (7–10), we discuss the trajectory of TFP-N therapy for these three patients.

Thick-Skinned (Grandiose) and Thin-Skinned (Vulnerable) Narcissism

As introduced in Chapter 1, there is now a consensus that those with pathological narcissism may have quite varied presentations, the most typical of which are grandiose or thick-skinned or vulnerable or thin-skinned (Bateman, 1998; Rosenfeld, 1971, 1987). Grandiose narcissists are self-aggrandizing, socially dominant, entitled, and exploitative of others for whom they lack empathy, often conforming in their surface presentation to the categorical diagnostic criteria of NPD in DSM-5 (American Psychiatric Association, 2013). But although they have an

exaggerated sense of self-importance, they may also be charming, successful, articulate, and socially adept. They often function well at work, sometimes attaining leadership positions in government or the professions, and may have some stable enduring relationships, but these are usually superficial and shallow in nature. They may have relationships that give the appearance of intimacy, but the capacity to love deeply, which involves integrating sexual passion with emotional intimacy in a relationship with another who is valued for his or her own unique qualities separate from the projections and needs of the self, is usually truncated. Thick-skinned individuals regulate self-esteem through gleaning the admiration, loyalty, and submission of others, whom they seek to dominate and control, although they may be prone to functional collapse, becoming symptomatic usually only if their sense of supremacy or self-importance is threatened or challenged. Their relationships are based on ideals of perfection, physical attraction, status, and control.

Vulnerable or thin-skinned narcissistic individuals, by contrast, are inhibited, self-effacing, prone to states of shame and feelings of inferiority that they cope with through retreat to a grandiose illusory world that insulates them from others, from limitations in the self, and even in more extreme cases from aspects of reality itself. They are highly reactive to criticism and failure, and their sense of specialness often stems from annexing themselves to others they perceive as special and unique, or through glorification of their distress and suffering. Self-esteem is regulated not so much through dominance and control of others as through surrender to idealized others, although their capacity for object relations in depth and empathy for others is limited since they tend to avoid sustained, in-depth contact with the world (Bach, 1985). Thin-skinned or vulnerable individuals may tend to show more overt fragility and psychological distress and to be more symptomatic (Wright & Edershile, 2018), so that they are often diagnosed with depression, anxiety disorders, and the pathological narcissism is missed. Often when they fail to respond to treatment for such conditions do clinicians consider that narcissistic pathology may be a prominent part of the presentation. This is because thick-skinned narcissists manifest their grandiosity more overtly in behavior while the narcissistic pathology of thin-skinned narcissists may be diffident and unassuming in their external life but expressed in internal beliefs.

The distinction tracks fairly closely to the Shedler–Westen Assessment Procedure–II (SWAP-II) categories of "grandiose/malignant" and "fragile" (Russ et al., 2008; described in greater detail in the following section), respectively, and largely along the externalizing versus internalizing spectrum that organizes the PDM-2 (PDM Task Force, 2006). The distinction between the grandiose and vulnerable, thick- and thin-skinned

presentations has to do with the rigidity or permeability of the primitive defensive boundary between the idealized and devalued aspects of self. For thick-skinned individuals, defenses are more rigid and effective in shoring up their grandiosity and establishing a beachhead of dominance and supremacy that keeps devalued aspects of the self at bay. However, if the entitled expectations, exaggerated sense of self-importance, or need for admiration of those with grandiose or thick-skinned narcissistic presentation are threatened, the individual may experience severe dysregulation at the affective, cognitive, or even behavioral level.

Individuals with vulnerable or thin-skinned presentation are more susceptible to defensive fluctuations and even collapse of their grandiosity, which is often illusory and covert, whereupon they are likely to develop often disabling symptoms of depression, anxiety, substance abuse or suicidality. Higher-functioning narcissistic individuals are likely to be more grandiose and thick-skinned, as illustrated by the case of Mark, introduced in Chapter 1 and discussed above. Those at the borderline level are likely to be more vulnerable and thin-skinned, as illustrated by the case of Rebecca in Chapter 1. However, the two presentations of narcissistic pathology may characterize those at all levels of personality organization (Rosenfeld, 1971). We might say that grandiose/thick-skinned and vulnerable/thin-skinned presentation is a dimension that is orthogonal to the level of personality organization, and both must be considered in understanding patients with pathological narcissism. Thus, it is possible to have thick- and thin-skinned higher-functioning individuals, as well as those organized at the lower borderline level, or characterized by malignant narcissism.

In conceptualizing narcissistic pathology, it is important not to think in terms of a simple dichotomy between thick- and thin-skinned, grandiose or vulnerable, narcissistic disorders. Each narcissistic patient is often a unique admixture of grandiosity and vulnerability. In individuals with pathological narcissism, one state may be more prominent than others, but they will usually oscillate between grandiosity and vulnerability over the course of treatment, depending on the dynamics of the individual patient. In addition, as clinicians we encounter a number of patients in whom pathological narcissism comingles with other characterological features that lead to more complex finely graded and varied presentations, as described earlier. The following two case examples illustrate these points.

Case Example: Laura—Thin-Skinned Narcissism

An aspiring female writer in her late 20s, whom we call Laura, sought TFP treatment because she was involved in a problematic relationship

with an older well-known writer to whom she subordinated herself, caring for his every need and neglecting her own career. She derived great gratification from his success, accompanying him to his readings and book parties, and often spending inordinate amounts of time editing his manuscripts while neglecting her own work. She tolerated considerable verbal and emotional abuse, as when frustrated in his work, he would berate her, on one occasion in the midst of a rage, casting her out of the apartment without her coat during a snowstorm. He also took her ideas for stories, at one point finishing a story she had begun and publishing it under his own name.

In the initial sessions, Laura told the therapist a compelling narrative of persecution and survival. Her parents, both teachers, were forced to emigrate to the United States when they were stripped of their positions in their country of origin and briefly incarcerated. Unable to work at their profession in the United States, her parents took menial jobs and focused on educating their only child, working several jobs each in order to provide her with every opportunity that they lacked. Her father became severely depressed and was hospitalized several times during her childhood. Laura was "discovered" as a promising writer when she won a prize for the best short story in her high school, which was later published in a major literary magazine in an issue devoted to student writings.

During her childhood and adolescence, Laura's parents controlled her social life and pushed her relentlessly to achieve, while at the same time resenting her for their own lost opportunities. She was offered a scholarship at an elite prep school where she excelled academically, but made few friends, instead focusing single-mindedly on developing her writing skills. She reported that her fellow students were envious of her literary success, but she also experienced paralyzing states of despair, sometimes lasting for days, when she received criticism for her work. In the summers, she attended programs for gifted writers winning more prizes and another scholarship to a college known for its writing program. There she excelled but no matter how much recognition she garnered, she would become overwhelmed with envy of other students, especially those who achieved recognition for their writing.

The gap between who Laura was and who she aspired to be continued to widen, causing her continual emotional torment. She dealt with this by seeking relationships with powerful, prominent writers that would begin as intense infatuations, but then would be followed by devastating disappointment when she failed to obtain their total devotion. Her most recent relationship had begun several years previously when she had attended a reading of her lover's novel where she fell in love at first sight. At first their relationship had seemed like an ideal one

where love and work blissfully converged, but the infatuation waned and her boyfriend, who drank excessively, became increasingly controlling and abusive, both physically as well as psychologically. Laura had been offered a book contract by a major publisher, but she found herself unable to make progress on it. Six months before she began TFP-N, she withdrew from friends and family, refusing to respond to e-mails or phone calls. She continued to see her lover intermittently, and she continued to write desultorily in isolation, often imagining that as she was doing so, she would become a world-famous novelist whose reclusiveness would only enhance her fame.

It was clear that Laura had organized around a difficult individual and family narrative, which along with her literary talent, gave her a sense of specialness and uniqueness, and that she had little investment in change. A previous treatment of several years duration had ended unsatisfactorily, and she related the following anecdote about how her previous therapy had ended. After she had been in analysis with a well-known expert clinician for several years, her mother attended a session with her. The therapist, who was familiar with the mother's native tongue, quoted from a prominent literary text, and the mother had corrected his pronunciation in a demeaning. imperious manner in front of her daughter. Laura said that she was never able to view the therapist in the same positive light that she had before this incident, and although she said he had been devoted to her as a patient, he had not helped her to change. When in subsequent sessions, the previous therapist attempted to interpret how her identification with her mother interfered with her establishing any viable life and relationships of her own, she terminated her treatment. A year later, when she found herself increasingly paralyzed in making progress on her novel and unable to leave the abusive relationship, she returned to her former therapist for consultation and was referred for TFP-N.

Thin-skinned individuals with vulnerable narcissistic presentation, such as Laura, whose defensive functioning is more variable and tenuous, may present as inhibited and self-effacing. Their pathological narcissism is expressed primarily in grandiose cognitions, beliefs, and fantasies that are often hidden from others, for fear of shameful exposure of the gap between their aspirations and accomplishements. They react to threats to self esteem or unrealistically inflated self image not with aggression or animosity, but with withdrawal, contemptuous dismissal, or attempts to control others (Akhtar, 2009). This necessarily limits the depth of their relations with others, although they may annex themselves to others whom they perceive as special and/or worthy of admiration that they themselves crave but cannot attain. The more vulnerable narcissistic patients may ceaselessly unload experiences of maltreatment or

self-loathing in the form of rigid or fixed narratives that make them feel special and unique but that leave little space for the therapist to clarify or understand. Thus, although patients with vulnerable or thin-skinned narcissism are more likely to seek and value therapy because they are more overtly symptomatic and distressed, they may also be quite difficult to engage in treatment, albeit in subtler ways. Such vulernable or thin-skinned individuals have often suffered traumatic experiences or injuries to their self-regard, which they counter through compensatory narcissistic fantasies and preoccupations that severely curtail their investment in the therapy and in a relationship with the therapist. The therapist who serves such a "lavatory" function (Rosenfeld, 1971) is expected to contain the load of clinical material but attempts on his or her part to understand or go deeper into the material is summarily dismissed or stonewalled. Rather, patients have an investment in protecting their narrative, which is not only rigid but fixed and unyielding in ways that may make the therapist feel defeated and dismissed.

Rosenfeld (1971) and others have noted how thin-skinned individuals may have often triumphed over extraordinary travails and have established what seems like an adequate adaptation in love and work as a result of unusual intellectual capacities or talents, but they may be chronically self-defeating, which in more extreme cases may be expressed in extreme self-destructiveness or destructive behavior (P. F. Kernberg, 1989). However, in either case, they remain so bound up internally with identifications with ideal, if punitive, internal objects and/or with longings for approval from such figures that they have little investment in current relationships, including the relationship with the therapist, as the foregoing example indicates.

Case Example: Thomas—Grandiose or Thick-Skinned Narcissism

A higher-functioning narcissistic patient, an investment banker in his late 30s, whom we call Thomas, sought therapy because although he became infatuated with women repeatedly, he was unable to fall and remain in love. He spoke proudly about his sexual prowess and capacity to attract any woman he desired but said that he felt increasingly out of sync with his peers, many of whom were getting married and starting families. He came to treatment to try to understand why he could not fall and remain in love, although he reported that the main motivation for wanting to understand this was fear of being perceived as inadequate by his peers, and also fear of losing a long-term on-again, off-again, relationship with a woman whom he relied on for comfort and companionship, but did not love. In fact, he said he had had a number of relationships, but had never been in love. During Thomas's yearlong

involvement with this woman, he had continually pursued liaisons with other women, without any of the women being aware of one another's existence. When his girlfriend became distressed by his lack of commitment to her and ended the relationship, he found himself, much to his surprise, quite distressed and humiliated, although he insisted he did not love her enough to make the commitment she requested.

Exploration of Thomas's history had revealed that his parents had been divorced when he was 10 years old, and that he was thrust prematurely into the role of being the man of the house, caring for a depressed, dependent mother who doted on him and made him the center of her emotional life in ways that made him feel both empowered and also resentful and deprived. His father was a distant figure who showed little interest in Thomas outside of his achievements in school and later in his striking success in work, which Thomas believed had finally earned him his father's respect and attention. When he consulted the TFP therapist, he requested right off the bat that the therapist charge him her lowest fee (although he could well afford her highest fee), with the idea that he would give her a large bonus after 6 months—a payment policy that was used by his company—if he achieved his goal of falling and remaining in love. When the therapist stated that such a plan was unacceptable, Thomas disparaged her as having no imagination and refusing to think "outside the box," focusing on how his own success was due to his ability to think in original and nonformulaic ways. Hence, his grandiose narrative, as one who could control others and control the duration of the relationship if it displeased him, came immediately into the transference during the contracting sessions. The therapist told him that while she was optimistic that they could work together to address his inability to fall and remain in love, she could not guarantee that this could be changed in 6 months. Furthermore, she said that his difficulties with doing so stemmed from issues and conflicts of which he might be unaware, and that might take a substantial amount of time to change. Thomas's response was to denigrate the therapist for what he saw as her lack of optimism and skill in treating him and wondered whether he should consider another therapist and type of treatment. The therapist responded that his inability to commit to the therapy may reflect his difficulties with tolerating the inevitable uncertainties and the give-and-take of a genuine investment in relationships and/or also reflected fears of committing to any relationship, particularly without a steadfast guarantee that he could remain in control and dictate the terms of the relationship. She also observed that it was difficult to imagine a relationship in which one person was not being exploited by another and thus he was attempting to find a way to ensure that his needs would be met before he would commit to treatment.

This vignette illustrates how at the beginning of therapy, there may be a reactive heightening of grandiosity in those with thick-skinned narcissistic presentation, which in Thomas took the form of demands for special treatment and a rejection of the frame, as a defense against the fears of dependency and exposure of underlying vulnerability.

Commentary

The two patients described above initially presented quite differently. Laura, the thin-skinned patient, presented with a narrative of vulnerability that focused on self-abasement, self-doubt, and mistreatment, whereas Thomas, the thick-skinned patient, organized around a grandiose narrative that focused on the self as superior, socially dominant, and supremely self-sustaining. In both cases, however, grandiose and vulnerable features comingled in subtle ways. For Laura, the clinical presentation of vulnerability masked the grandiose beliefs about her special gifts as a writer who could only ally herself with well-known accomplished writers whose success she experienced vicariously. For Thomas, the grandiose features of self-centeredness, social ease, and exploitativeness masked his underlying sense of unworthiness and insecurity that interfered with his capacity for mutuality and commitment. In addition, in both individuals, we see a sense of entitlement, exploitativeness, self-preoccupation, and interpersonal antagonism that are thought to be at the core of narcissistic pathology (Miller et al., 2016). This was evident in the fear of dependency that brought them into contact with vulnerable or flawed aspects of self, and that precluded genuine engagement with others in depth, including the therapist. In addition, both organized at least initially around a rigid, fixed narrative that sequestered them from exploring with the therapist aspects of their internal worlds that threatened their illusory self-sufficiency forged out of identification with powerful, idealized internal objects.

Research Support for the Grandiose/Vulnerable Distinction

There is a growing body of clinical and empirical studies identifying the characteristics and specific correlates of the two variants of pathological narcissism. A review by Cain et al. (2008) identified over 50 labels delineating variations in the phenotypic presentations of narcissism, including thick-skinned and thin-skinned narcissists (Rosenfeld, 1987), the oblivious and hypervigilant (Gabbard, 1989), overt and covert (Cooper, 1998), arrogant/entitled and depressed/depleted (manipulative and phallic, or craving and paranoid; Bursten, 1973), and arrogant/overt and shy/

covert (Akhtar, 1989)—most of which are subsumed under the grandiose/vulnerable distinction. The grandiose and vulnerable dimensions of the disorder have also been affirmed in a growing number of empirical studies that have identified specific traits and characteristics with each group, including differences in treatment utilization, social cognition, facial recognition, attachment status, interpersonal functioning, coping strategies, and rejection sensitivity (Dickinson & Pincus, 2003; Diamond, Clarkin, et al., 2014; Ellison et al., 2013; Hendin & Cheek, 1997; Miller et al., 2018; Rathvon & Holmstrom, 1996; Rose, 2002; Wink, 1991, 1992, 1996; Russ et al., 2008; see Miller et al., 2017, and Wright & Edershile, 2018, for reviews). While comprehensive coverage of this research is beyond the scope of this chapter, it is important to mention that the two manifestations of narcissistic pathology have somewhat different personality trait profiles, although they are linked by core characteristics indicating that they are variants of the same disorder. Both grandiose and vulnerable narcissism are associated with entitlement and exploitativeness although these may be expressed quite differently in overt exhibitionistic or covert vulnerable ways depending on whether they are in a vulnerable or grandiose state of mind (Wright & Edershile, 2018; Edershile et al., 2019). Both narcissistic presentations have been linked to interpersonal antagonism (e.g., disagreeableness, callousness; Miller et al., 2017). But while grandiose narcissism has been associated with social dominance, agentic extraversion and low neuroticism, vulnerable narcissism has been associated with interpersonal distrust and high levels of neuroticism (e.g., affective instability, impulsivity, and the tendency to experience negative emotions, such as anger, anxiety, or depression; Miller & Campbell, 2008; Miller & Maples, 2011; Miller et al., 2017).

Interestingly, grandiosity and vulnerability have been found to be largely distinct and unrelated in individuals who show lower levels of these traits. Indeed, one study that examined both vulnerable and grandiose narcissism and different domains of dysfunction showed that in nonclinical groups, grandiose narcissism, when decoupled from vulnerability, was associated primarily with difficulties in the interpersonal realm, with otherwise good functioning in other domains. Those with vulnerable narcissistic presentation are more likely to function poorly across multiple domains with not only difficulties in interpersonal relationships but also with more overt symptomatology, and lower overall life satisfaction and difficulties in daily functioning, typical of those who function on an overt borderline level (Wright & Edershile, 2018). Significantly, when grandiosity exists at high levels, it is also associated with vulnerable features of fear, negative affect, and depression (Jauk & Kaufman, 2018).

These findings affirm that pathological narcissism exists on a continuum, and those who have higher levels of grandiosity also have vulnerable characteristics as well. Since those with grandiose and vulnerable narcissism have been found to be characterized by the core personality features of exploitativeness, self-centeredness, entitlement, and interpersonal antagonism, individuals with one or both manifestations of the disorder may meet criteria for pathological narcissism or narcissistic disorder if they experience high enough levels of functional distress and impairment (Miller et al., 2016, 2017). The personality trait profiles found in the studies of those with narcissistic traits suggest that pathological narcissism may exist at different levels of organization. Those with a higher-functioning grandiose profile report less subjective distress, but induce more distress in others (Miller et al., 2007), and those with a more vulnerable profile show more emotional distress, fragility, and overt symptomatology with overlaps with personality pathology in general and BPD in particular. However, the majority of these studies have been cross-sectional in nature so that they while they provide a snapshot of the mental states of vulnerability or grandiosity at one point in time, they do not assess for how these states may fluctuate within the individual over time. Furthermore, they are conducted with nonclinical as well as clinical groups with different self-report measures of narcissism (e.g., NPI, Raskin & Hall, 1981; Pathological Narcissism Inventory [PNI], Pincus et al., 2009; Five-Factor Narcissism Inventory [FFNI], Glover, Miller, Lynam, Crego, & Widiger, 2012) that may assess for narcissistic traits rather than for the underlying structural characteristics of pathological narcissism. Although more research is necessary to understand the different presentations of pathological narcissism, it is clear that pathological narcissism is a heterogeneous disorder with both grandiose and vulnerable, and covert and overt manifestations, which may coexist, or oscillate within the individual concurrently or over time, depending on shifts in mental state or life circumstances (Cain et al., 2008; Gore & Widiger, 2016). Most recently personality rearchers have followed psychodynamic clinicians in calling for more attention to the dynamic processes that give rise to the disparate observable features of those with pathological narcissism.

In fact, such investigations have confirmed what clinicians have long observed in their intensive work with those with narcissistic disorder: that the surface observable appearance of grandiose narcissism may mask a covert world of suffering and distress involving difficulties with self and affect regulation. Individuals may be shame ridden and full of self-doubt when grandiose expectations fail to materialize, may withdraw from others socially to avoid exposure of weakness or vulnerability, and may show symptomatic distress and even suicidal ideation in the

face of actual or anticipated humiliation and failure—all of which are consistent with different narcissistic presentations (Bach, 1985; Kernberg, 2007; Ronningstam, 2009, 2011). Even when one presentation is dominant, which is often the case in those with narcissistic disorder, individuals may oscillate between grandiose and vulnerable states of mind over time or over the course of therapy. However, the extent to which these two faces of narcissism are distinct phenotypical presentations of the disorder versus mental states that may fluctuate differentially in response to context remains a matter of considerable debate and requires further research to determine. The timing, frequency, and duration of such oscillations, as well as the extent to which they involve shifts in self-esteem, affect regulation, and interpersonal functioning that may differ depending on the underlying level of personality organization and surface presentation of pathological narcissism, have only begun to be delineated (Perry, 2014).

While the plethora of clinical and research investigations has substantially broadened our understanding of the variable presentation of narcissistic disorder, there are few overarching theories that would explain why those with pathological narcissism may have such variable presentations and why these dimensions of narcissistic disorder may coexist within the same individual. In addition, in these studies there has not been much consideration of how grandiosity and vulnerability might be expressed differentially depending on the underlying level of organization of the individual—that is, whether he or she is functioning at a higher (neurotic) level, borderline, or even psychotic level of functioning, and further, what might be the implications of the relationship between surface presentation and underlying personality organization for the therapeutic process and relationship. That narcissistic pathology exists at different levels of organization and encompasses a range of presentations beyond grandiosity and vulnerability is at the core of our object relations model that is further delineated in the following sections.

Some Further Characterological Presentations of Narcissistic Pathology

Other indications of narcissistic pathology include the following.

Narcissistic–Masochistic Presentation

Individuals with a narcissistic–masochistic presentation are those in which the self-regard is organized around a sense of being special in the magnitude and nature of their suffering. They are often highly

self-critical as a response to anxiety about attacks from idealized internal objects or actual others, to whom they submit masochistically (Cooper, 1989). They often present as beyond help in a way that may conceal a covert unconscious grandiosity, including the conviction that they are so special that no one, including the therapist, will ever be able to comprehend or change the depth of their suffering. These individuals often evoke strong feelings of compassion in the therapist in response to their self-loathing and overwhelming suffering that may be linked to credible narratives of abuse, trauma, neglect, misfortune, or maltreatment. However, over time it becomes apparent that the therapist's efforts to understand the impact of these experiences on the patient's actual functioning and inner representational world is experienced as a threat to a covert grandiosity that combines superior with degraded self-representations. They may harbor a secret contempt for others for not knowing how inferior they are and for having the moral weakness to make the compromises that are necessary to make life tolerable: The narcissistic–masochistic personality lives with a sense of moral superiority as the core of their grandiose self that is difficult to challenge and that may induce a sense of futility in the therapist.

Narcissistic–Histrionic Presentation

Those with a narcissistic–histrionic presentation are often more overtly attention seeking, dependent, and more related to others. They are often more exhibitionistic and self-dramatizing around sexual issues, although the latter realm is conflictual and often acted out in dissociated ways (Bakkevig & Karterud, 2010), and they use their narcissistic features to advance ambition. This profile of pathological narcissism is similar to the high-functioning/exhibitionistic group identified by Russ et al. (2008) who are articulate, extroverted, and competitive. Narcissistic personalities with histrionic features also may present with hypochondriasis and somatic preoccupations that severely limit their functioning and lead to a constriction of affect and even of thought processes that are narrowly focused on bodily symptoms and preoccupations.

The Dead-Mother Syndrome

These individuals present with negative narcissism in which there is a preoccupation with nonbeing and inertness that severely limits the capacity to engage in any relationship or endeavor, including the therapeutic one, as articulated by Green (2002). Negative narcissism in Green's view involves self-contempt, renunciation, emptiness, pervasive sense of superiority in the context of self-depreciation, and impoverishment of speech

and communication, all of which should be understood fundamentally as not only masochistic but as reflecting a disinvestment in the self, so profound that it leads to a desire for nonbeing. In such cases, the patient may present a truncated, constricted, and unformulated narrative. The task for the therapist with those with this syndrome is to remain in emotional contact with them while the patient does almost everything to push the therapist away in his or her frozen, deadened stance. The therapist's capacity to tolerate this, to see it as the most extreme form of resistance to the treatment and to life itself, while at the same time recognizing both the disparate representations of self and other that underlie both the profound despair and grandiosity that fuel such self-experience and behavior, is crucial to help bring the person into greater emotional contact with self and others. We theorize that features distinguishing a presentation of negative narcissism from a major affective disorder include: (a) a sense of emptiness and futility, (b) a pervasive sense of superiority and contempt for others, and (c) gratification in splendid isolation.

Narcissistic Depression

It is essential to distinguish depressive affect stemming from narcissistic dynamics from a major affective disorder. Particularly in the current climate of short treatments for depression based on medication, there can be great risk in missing a characterological narcissistic depression. The depressive narcissist suffers from incessant harsh judgments against him- or herself that are generally refractory to positive feedback, encouragement, or reassurance. Even very successful individuals, whose accomplishments may be partly attributable to their harsh demands on themselves, may experience a chronic sense of failure and inadequacy. When confronted by the therapist about the discrepancy between their considerable achievements and their often catastrophic sense of self-doubt and inadequacy, they often respond that only they are smart enough to know how they fall short. As stated above, they do not tolerate a gap between their self and their ideal self or ego ideal; if they do not measure up at every minute to an unrealistic ideal, they experience themselves as a failure. An object relation of an unworthy self and a harsh, judgmental other (with the self and other experienced alternately in either role) may dominate the treatment of such patients initially and for an extended period of time. These introjective features of perfectionism and self-critical or self-punitive attitudes are often a particular expression of the narcissistic organization. Blatt (2008) describes such individuals as excessively invested and organized along the introjective line of development, in which self-reliance, self-worth, and accomplishment are valorized at the expense of relatedness to others who are experienced as props

for the self. In individuals with introjective psychopathology, "interest is directed to things rather than to people, and heightened emphasis is placed on thoughts and accomplishments (deeds) rather than on feelings and interpersonal relations" (Blatt, 2008, p. 181). This renders individuals prone to depression when they fail to meet their exacting standards or feel unworthy of admiration. However, in such individuals, depression is felt not as guilt or sadness but as a state of emptiness and depletion.

These observations have been affirmed by recent research. In one study of over 200 university students assessed over 8 weeks, pathological narcissism was intially positively associated with anhedonic loss of interest over time (Dawood, Schroder, Donnellan, & Pincus, 2018). Furthermore, in a study of out-patients that compared two groups with depressive symptoms with and without NPD, the NPD group was characterized by negatively toned (hostile and accusatory) ruminations about others, but not the self, along with greater fluctuations in depressive symptoms and suicidal ideation and behaviors, and a lack of neurovegetative symptoms (e.g., insomnia with early wakening, loss of appetite, and weight loss; Fjermestad-Noll, Ronningstam, Bach, Rosenbaum, & Simonsen, 2019). We might surmise that in narcissistic individuals grandiosity might be protection from harsh self-criticism often noted in depression (Blatt, 2008), with hostility turned against others not the self.

The Pathological Grandiose Self

What ties together the surface presentations of those with pathological narcissism is a core structural feature—the grandiose self—that organizes the experience of self and others and underlies the difficulties in self and interpersonal functioning (Kernberg, 1984, 2018; Diamond, Yeomans, et al., 2011; Stern, Yeomans, Diamond, & Kernberg, 2011). In the grandiose self, ideal images of self and others are condensed with realistic, positive self-representations into an inflated, idealized concept of self, while negative aspects of self are rigidly projected onto others who are then seen in the light of devalued, depreciated, inferior aspects of self. This is a way of defending against unintegrated, intolerable, or negative aspects of self and associated unmastered and unintegrated negative affects of fear, anger, or envy. The incorporation of ideal aspects of self and others into the real self buttresses the unstable unintegrated self, providing a semblance of stability and coherence to a fragmented unintegrated internal world. But this is at the cost of much greater rigidity, reduced capacity for relatedness to others, and distortions in reality testing in that realities that do not conform to narcissistic illusions about the self may be rejected. This also necessarily involves a lack of integration

in the concept of significant others who become the repository for devalued as well as idealized aspects of the self, but in either case are not seen as totally separate from the self. See Figure 2.3 for an illustration of the pathological grandiose self.

In contrast to individuals with BPD, whose split, polarized internal world predisposes them to extreme and unpredictable shifts in self and object representations, the grandiose self-structure of those with narcissistic personality provides a surface facsimile of integration that covers over deeper internal fragmentation and that lends a higher degree of rigidity and stability to the personality even in those with lower borderline organization. This is due to the rigidity and effectiveness of the intersecting introjective and projective mechanisms; not only are negative, disavowed aspects of self systematically attributed to others but the ideal and positive aspects of others tend to be appropriated to the self. The grandiose self is a defensive structure that protects the individual against dependency, short-circuiting any need that might evoke complex feelings of love, hatred, or envy of the other. In Joseph's (1959) view, those with narcissistic disorder introject the idealized aspects of the others, not out of love and gratitude for the other but in order to avoid any sense of need for or dependency on the other. In so doing, they maintain control over their objects, denying the idealized object any separate existence either internally or externally. Consequently, the "object cannot be taken in as good, and therefore no really secure core to the ego and no internal stability and self-confidence can be established, so that behind the narcissism and the omnipotent superiority always lurk great doubts about the self and its essential goodness" (Joseph, 1959, p. 218). The sense of disquiet described by Joseph (1959) may also be due to the fact that the idealized aspect of the other taken in by the narcissistic person

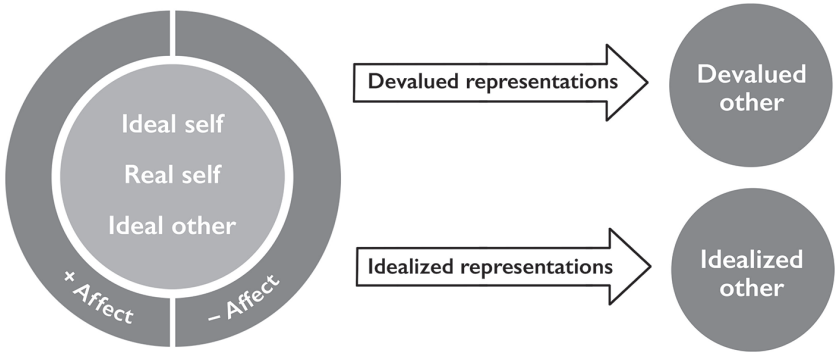


FIGURE 2.3. The pathological grandiose self in relation to others.

replaces any core, stable libidinal investment in the self, and therefore results in a sense of hollowness and insecurity.

Such an inflated self-structure in which the self is identified with the ideal internal object may, however, preserve some capacity for love—this is especially the case in higher-functioning narcissistic persons, because investment in the fantasy of an ideal other to some extent attenuates feelings of hatred and persecution that may more easily overwhelm those functioning at a lower borderline level of personality organization. However, the full experience of loving another, which presupposes the capacity to depend on an external good and valued object seen as unique and separate from the self, as well as the capacity to tolerate ambivalence and the risk of loss, is precluded. Furthermore, negative feelings are always an aspect of the grandiose self since both positive and negative and loving and hateful feelings are inevitably, in the course of development, invested in the self as well as others (Rosenfeld, 1964, 1987; Green, 2002). In addition, in those with pathological narcissism, punitive representations of others, which comprise the earliest layers of the superego, may infiltrate the ego ideal with the development of inexorable demands for perfection (Kernberg, 2018). As a result, the tendency to seek out relationships that combine admiring and punitive aspects, often characterizes those with narcissistic pathology with the severity of aggression more common among more disturbed/lower BPO patients.

The level of organization of the grandiose self, and the extent to which it can protect the individual from negative aspects of self, including fear, weakness, vulnerability, and envy through projective mechanisms, determines the extent to which the underlying split structure is contained or breaks though. When the grandiose self is functioning effectively as a *defensive structure*, as is the case with higher-functioning narcissists, the individual with narcissistic disorder can appear quite stable in his or her identity, sometimes even presenting the illusion of a coherent and integrated self-concept. However, when the idealized self is infused with aggression, the individual may be extremely brittle and prone to collapse if omnipotent defenses against human fallibility should cease to be effective (Rosenfeld, 1971). For individuals with narcissistic disorder organized at a borderline level, the grandiose self functions less well as a barrier between the idealized and devalued aspects of the self, although the rigidity of the grandiose self-structure leads to a less chaotic self-experience than is the case with those organized at a borderline organization without narcissistic pathology.

Kernberg (1975, 1984) initially observed that higher-functioning narcissistic individuals characterized by a relatively stable grandiose self may present the appearance of smooth and effective social functioning, but at the base there is a fragility and rigidity that does not allow for a

deeper level of contact with others or reality. The seemingly intact identity and surface functioning of those with pathological narcissism often masks the more severe disturbances in the object relating characteristics of those with a grandiose self. The distorting prism of the grandiose self refracts reality to ensure a view of self as superior and exceptional. This leads to rejection of any alternative realities about the self and even, in some cases, to gross distortions in social reality testing itself. In sum, by warding off any experience of the self that is negatively valenced, including feelings of inferiority, incompetence, vulnerability, aggression, and envy, the pathological grandiose self provides a superficial veneer of stability, but at a reduced capacity for differentiated relatedness with others. Relationships with others, including individuals, groups, and institutions, are either used to bolster the self, if idealized, or to serve as a repository of all that is unacceptable and disavowed in the self, so that inevitably there is an incomplete differentiation of representations of self and others.

The grandiose self is a structure, as is the case with internalized object relations in general, representing a set of processes and functions that organize subjective experience in a way to buttress and protect the self. Such processes and functions cannot be directly observed, although they are based on observable phenomena (Kernberg & Caligor, 2005; Caligor & Clarkin, 2010). Indeed, the manifestations of the grandiose self in characteristic traits and relational patterns of those with narcissistic disorder (grandiosity in behavior or fantasy, exploitativeness, entitlement, aggressive responses to threat, the quest for admiration, preoccupation with fantasies of unlimited success) have now been documented in a number of cross-sectional empirical studies (Cain et al., 2008). They have also been observed in the constellation of transferences, defenses, and resistances that individuals with pathological narcissism develop in the course of psychodynamic therapy (Diamond et al., 2013; Diamond, Clarkin, et al., 2014; Kernberg, 1975, 2007; Kohut, 1971, 1977; Ronningstam, 2009, 2011, 2014).[1]

Observations of the grandiose self as the underlying structure of narcissistic disorders are not limited to the consulting room, nor is the grandiose self solely a by-product of contemporary society. Indeed, pathological narcissism has long figured in the fictional portraits of characters in literary works, including ancient myths, particularly the myth of "Echo and Narcissus" (Hughes, 1997), as described in the Preface of

[1]Both Kohut and Kernberg postulate a grandiose self-structure in narcissistic individuals but while Kohut views it as a normative structure that results from developmental arrest at a normal infantile stage of development, Kernberg links it to a pathological structure that reflects developmental distortions.

this book, as well as in novels in which main characters have served as prototypes for healthy and pathological narcissism (Symington, 1993) long before narcissistic pathology was conceptualized as part of a clinical syndrome.

The following passage from Henry James's novel *The Portrait of a Lady*, for example, presents a consummate account of the grandiose self in one of its characters, Gilbert Osmond, who had ensnared the young heiress Isabel Archer in an exploitative marriage, using her money to create a platform for his own grandiosity. In this passage, Isabel Archer muses about her recognition that her husband (Gilbert Osmond), whom she had initially idealized as a bohemian aesthete devoted only to what was true and beautiful, was in fact a narcissistic person who cared only for the admiration of the world that he disdained:

> She was to think of him as he thought of himself—as the first gentleman in Europe. So it was that she had thought of him at first, and that indeed was the reason she had married him. But when she began to see what it implied she drew back; there was more in the bond than she had meant to put her name to. It implied a sovereign contempt for every one but some three or four very exalted people whom he envied, and for everything in the world but half a dozen ideas of his own. That was very well; she would have gone with him even there a long distance; for *he pointed out to her so much of the baseness and shabbiness of life, opened her eyes so wide to the stupidity, the depravity, the ignorance of mankind, that she had been properly impressed with the infinite vulgarity of things and of the virtue of keeping one's self unspotted by it. But this base, ignoble world, it appeared, was after all what one was to live for; one was to keep it forever in one's eye, in order not to enlighten or convert or redeem it, but to extract from it some recognition of one's own superiority.* (James, 1881/2003, p. 479, emphasis added)

This passage captures the contradictions inherent in the grandiose self, including the sense of lofty superiority sustained through ruthless devaluation of others coupled with the insatiable need for admiration from such denigrated others. Such a grandiose self-experience tends to be activated in specific situations and evokes primitive defenses, particularly, as the above example indicates, omnipotence and devaluation/idealization in which the individual projects all that is dreaded or unacceptable in the self onto others who are then treated with contempt or indifference. Or the individual sees others through the distorting prism of his or her own sense of specialness and superiority, expecting such idealized others, not seen as separate from one's own needs and imperatives, to provide endless admiration and mirroring (e.g., he had "a

sovereign contempt for every one but some three or four very exalted people whom he envied and for everything in the world but half a dozen ideas of his own"). In the latter situation, the self may be devalued while the other is idealized for the emotional sustenance and admiration he or she might provide, thus inverting the typical narcissistic dyad often with a comparable degree of rigidity and distortion. Over time, paradoxically, those with narcissistic pathology often need to concurrently devalue those same individuals they idealize in order to stave off the awareness of any humiliating deficiency in the grandiose self, as well as feelings of envy. The impulse to destroy the good perceived in others, that the patient feels he himself or she herself does not possess, to deem as worthless what is received from others, leads the individual with narcissistic pathology to experience a sense of inner emptiness and impoverishment (Kernberg, 1984; Rosenfeld, 1964). This lack of a core sustaining autonomous self leaves narcissistic individuals vulnerable to experiences of doubt and insecurity that "disrupt their sense of grandiosity or specialness" (Kernberg, 2009, p. 106).

By contrast, in *The Beast in the Jungle*, James (1903/2005) provides a character portrait of a vulnerable narcissistic person, John Marcher, who is shy, inhibited, diffident, and fearful of intimacy, but superficially charming and decorous. Although outwardly self-effacing, he was consumed by covert fantasies that he was destined for unique greatness that he revealed only to a female friend with whom he shared his life, but whom he persistently devalued as not special enough to marry. Remarking on his grandiose illusions, she said, "You said you had had from your earliest time, as the deepest thing within you, the sense of being kept for something rare and strange . . . that was sooner or later to happen to you" (James, 1903, p. 7). Indeed, convinced that "something or other lay in wait for him among the twists and turns of the months and the years like a crouching Beast in the Jungle" (p. 11), John Marcher lived winningly on the surface of life, without any deep emotional investments. It was only after his devoted female friend died that he realized that his narcissistic illusions of grandeur to come clouded his capacity to love and live authentically in the present.

Empirical Research on Grandiosity

Grandiosity has emerged from both clinical and empirical studies as a key distinguishing feature of pathological narcissism and NPD, and one that discriminates it from other personality disorders. Ronningstam and Gunderson (1991) found that grandiosity distinguished those with NPD from their near personality-disordered cousins with BPD. Similarly, fantasies of unlimited success and the sense of self as special and unique, supported by excessive admiration from others, also distinguished those

with NPD from those with antisocial personality disorder (ASPD), although exploitativeness toward others and lack of empathy did not (Holdwick, Hilsenroth, Castlebury, & Blais, 1998). In sum, the criteria that best distinguished those with NPD from those with BPD and ASPD were a grandiose sense of self-importance, a belief that one is special and unique, a sense of entitlement, and arrogant and haughty attitudes and behaviors, as well as fantasies of unlimited success. Subsequent research has affirmed that grandiosity may be expressed either overtly in arrogant, entitled attitudes and/or exploitative behaviors, or covertly in the individual's mind, as in preoccupation with grandiose fantasy or cognitions (feelings, motives, beliefs; Pincus & Lukowitsky, 2010; Wright, 2016). It is important to note that grandiose fantasies that center around preoccupation with illusions of unlimited success and power (American Psychiatric Association, 2013) do not necessarily imply openness to fantasy per se, which involves receptivity to the unconscious and to the "inner world of the imagination" (quoted in Glover et al., 2012, p. 508), but rather function as an inner preserve of narcissistic chimeras. While these studies affirm grandiosity as a key distinguishing characteristic of narcissistic disorder, grandiosity was found to be unstable and to fluctuate over time (Ronningstam et al., 1995).

This has led a number of theorists to stipulate that grandiosity is not the key distinguishing feature of pathological narcissism and NPD, but that it is instead a by-product of self-esteem and affect dysregulation that characterizes this disorder. Grandiosity bespeaks a maladaptation or failure of normative strivings for self-esteem enhancement, recognition, and validation from others (Wright, 2016; Pincus & Roche, 2011). Investigation of real-time monitoring of states of mind in individuals with narcissistic pathology are consistent with aspects of the object relations view of narcissistic pathology. First, studies based on ecological momentary assessment (EMA) of states of mind in those with narcissistic pathology indicate that they are highly reactive to different social stimuli, and are dependent on admiration and validation from others, with self-experience fluctuating according to the vicissitudes of interpersonal interactions. In studies where those with narcissistic pathology filled out daily diary cards about their state of mind after social interactions (Roche et al., 2013), it was found that pathological narcissism predicted fluctuations within the individual of grandiose and vulnerable self-states and strategies to navigate social interaction over the course of a week. When the individual with whom they were interacting was perceived as friendly and supportive (Roche et al., 2013), or when they experienced a sense of power or agency in relation to others, their grandiosity remained robust. But if they perceived the other as detached, uncaring, or unfriendly, they reported experiencing vulnerable and distressing states of mind. Negative or unsupportive interpersonal interactions appear to activate mental

representations of self as vulnerable or inferior, and the other as superior or rejecting, in contrast to a sense of self as superior and empowered, and the other as admiring and perhaps inferior, which is activated in more positive interactions. Second, the EMA studies cited above affirm the idea that those who present with or initially may be identified by the grandiose criteria of pathological narcissism and NPD are likely to have vulnerable manifestations of the disorder as well. A recent ecological experience sampling study indicated that those with dispositional grandiose narcissism generally tend to perceive others as colder and behave in a cold and dominant manner, whereas momentary grandiosity was associated with acting in a warm but dominant fashion in relation to others who are perceived as warmer and more submissive. On the other hand, those who are higher on dispositional vulnerable narcissism in general are more likely to perceive others as cold and aloof, which was only intensified in the moment (Edershile & Wright, 2019). This clearly has implications for the therapeutic relationship in that those with vulnerable narcissism may have more fixed, negative mental representations of self and others. These studies support the idea of pathological narcissism as a disorder of maladaptive ways of representing self and others, but also indicate that these distortions in the internal world may manifest themselves differently in interpersonal interactions depending on whether the vulnerable or grandiose aspects of pathological narcissism predominate and/or are activated in the moment. These studies support further investigation of the dynamic processes that underlie the interplay between grandiosity and vulnerability.

Indeed, it is precisely the two facets of the disorder and the oscillation between them that renders the treatment of those with pathological narcissism and NPD so challenging and complex. On the one hand, grandiosity involving exploitative, entitled, and in more severe cases, malevolent or vindictive interpersonal behavior may obscure the profound shame, self-criticism, and insecurity that constrict and distort the lives of those with pathological narcissism and NPD. This leads at times to paralyzing self-doubt and, in the worst cases, suicidal collapse (Ronningstam, Weinberg, & Maltsberger, 2008). On the other hand, since most individuals with pathological narcissism and NPD present for therapy in states of narcissistic vulnerability that may mask the core grandiose aspects of the disorder, the full extent of the self-aggrandizing behaviors and attitudes may be obscured. The latter may account for why contemporary clinical views of narcissistic disorder may be skewed in favor of vulnerability, while grandiose and even destructive behavior on the part of individuals who never see the light of the consulting room is rampant. This has led Miller and colleagues (Miller et al., 2016) to call for more studies of those with pathological narcissism in the community or nonclinical groups in whom social dominance, callousness,

and self-aggrandizing behaviors and attitudes may in fact be the predominant presentation of the disorder.

Indeed, in a recent study that asked both academic and clinical psychologists to assess grandiose and vulnerable features in narcissistic individuals, clinical psychologists were more likely to identify more vulnerable features in grandiose narcissists than were academic psychologists, and further to see these features as having been present for a longer time (Gore & Widiger, 2016). Furthermore, another study that compared assessments of narcissitic pathology in clinical and social-personality psychologists also revealed consensus between the two groups that grandiose features were central to the construct, but clinical psychologists perceived vulnerable features as being more salient than did the social personality group (Ackerman et al., 2017). Clearly, further empirical investigations are warranted of the relationship between grandiosity and vulnerability, the extent to which the two dimensions coexist, predominate, and/or fluctuate in those with narcissistic disorder by both clinicians who treat narcissistic individuals intensively over time in the consulting room, and researchers who look at them at one point in time in the community.

Disturbances in Identity in Narcissistic Disorder

While we see disturbances of identity as a concomitant of a pathological grandiose self-formation with the fluctuations between grandiose and vulnerable states, this "ideal self" may provide a semblance of identity integration in those with pathological narcissism and NPD. Consequently, identity disturbances may not be as obvious as is the case with other personality disorders in that there may not be the drastic fluctuations in goals, friendship patterns, or moment-to-moment continuity in a sense of self that one sees in those with BPD. But this stability does not imply an integrated identity because it is defensive and involves distortions and/or disturbances in the concept of self and others and in subjective experience. With more severe narcissistic pathology, when defenses falter or fail, the individual may experience a catastrophic loss of self-definition and identity. This is because the chronic devaluation and idealization of others characteristic of those with pathological narcissism evacuates the self (and internalized others) of key attributes—so that they cannot be integrated into a richly nuanced identity. With idealization the other is described in hallowed terms without complexity, while the self is depicted as either similarly unrealistically positive or inadequate. Identity integration involving a sense of self and others as stable, realistic, valued, and complex (admixture of positive and negative) is thereby precluded.

With higher-functioning narcissistic individuals—that is, those who have attained stable and even productive work lives and have more capacity to sustain relationships, however superficial—more subtle identity disturbances may surface over time, such as the subjective experience of never having felt authentic, and a sense of artificiality or futility of all endeavors. They may seek therapy because they have never felt fully invested in, grounded, or satisfied with their work, professional lives, or relationships (Bach, 1985; Kernberg, 1975, 2014; Kohut, 1971; Fairbairn, 1952; Ronningstam, 2011). Such a sense of futility or inauthenticity may also emerge in treatment in patients feeling that therapy is a futile enterprise that cannot lead to any lasting change, in the sense that they must do it all themselves, and in the conviction that the therapist might be exploitative or unhelpful. In short, disturbances of identity in those with narcissistic disorder take a particular form, involving vacillation between inflated or deflated self-appraisals and excessive reference to others for self-definition and self-esteem regulation (American Psychiatric Association, 2013). The extent of such vacillations is determined by the level of organization of the individual. Individuals with pathological narcissism and NPD who present at a borderline level or with comorbid BPD on the surface may show less fluctuation in goals and functioning, and less severe identity diffusion than do those with borderline organization without pathological narcissism because the grandiose self may confer an illusory identity to which the individual clings.

The characteristics of contemporary industrial and postindustrial societies may be thought to constitute a cultural predisposition to the identity disturbances in narcissistic disorder, which, as indicated in Chapter 1, may be on the rise. Examples include social and geographic mobility, the bifurcation of home and family life from the spheres of work and production, the breakdown of family and community structures that leave the individual more susceptible to the homogenizing influence of mass culture with its array of stereotyped images and plots, and the allure of social media, which may foster both the aggrandizement of self, and a protean sense of self and relationships without depth, complexity, and continuity (Diamond, 2006; Lasch, 1979; Paris, 2014).

It is noteworthy that the concepts of identity and narcissism have been intertwined since their inception (Lunbeck, 2014). A number of theorists, including Erikson (1950, 1956, 1959, 1975; Federn, 1926; Fenichel, 2014), saw identity and narcissism as yoked concepts, stipulating that behind the disorder of narcissism are difficulties with the development and maintenance of a sense of identity (Erikson, 1950, 1975). "We begin to conceptualize matters of identity at the very time in history when they become a problem," wrote Erikson (1950, p. 229), recognizing that this link between narcissism and identity may stem from the fact

that both are problematic arenas in contemporary society. In fact, Federn (1955), who developed the concept of healthy narcissism, linked it to identity and defined it as "the sensation, constantly present, of one's own person—the ego's perception of itself." He wrote that healthy narcissism was essential not only for creative work but also for "full mutuality in mature object relations" (p. 34), a point that has been further elaborated by Kernberg (1975, 2018). In other words, disturbances in the sense of identity are linked to the distortions in object relations for those with NPD or predominant narcissistic traits.

While Erikson (1959) and Kernberg (2006) have further elaborated on the sense of identity as the subjective experience of the "persistent sense of sameness within the self" and a "sharing of some kind of essential character with others" (Erikson, 1956, p. 57), others like Volkan (2009) and Akhtar (Akhtar & Samuel, 1996) have suggested that individual identity is also shaped by large-group identity, such as national, religious, or ethnic identity. Identity formation is based on the integration of self and object representations as self and other become increasingly differentiated from the other through the oedipal phase, but also involves the consolidation of identifications with groups and institutions throughout life (Jacobson, 1964). The entrance of the child into the wider world and/or social realities beyond the sphere of the family, in the view of Chasseguet-Smirgel (1985) and others, is an aspect of oedipal resolution beyond the intrafamilial identifications. Perhaps this is what Erikson (1968) meant when he wrote, "identity formation begins when the usefulness of identification ends" (p. 159). In fact, another crucial aspect of the individual's identity, as described by Akhtar (1996) and others, refers to a link between one's personal identity and large-group identity, with this link established at the oedipal level when the superego, which encompasses group as well as individual values and mores, is consolidated. Disturbances in identity in those with narcissistic disorder may also be linked to distortions in large-group identities involving overidentification of disidentification with cultural, ethnic, or religious group, or conflict between the mores of the dominant culture and subculture to which the individual belongs.

Defense Use in Narcissistic Disorder

Disturbances in identity in patients with narcissistic pathology are intertwined with reliance on both minor and major image-distorting defenses. In psychodynamic thought, defenses are mental mechanisms through which the individual manages negative affects (anxiety, fear, envy, anger, guilt, and shame), impulses (aggression and sexual desires)

and the associated conflicts (Freud, 1914/1953, 1914/1957, 1937/1964). From contemporary object relations perspective, such conflicts are embedded in opposing or contradictory self and object representations, with one self–object dyad defending against an opposing anxiety-provoking self–object dyad (Kernberg, 1993, 2008a). Further defenses are best understood in terms of such conflictual object relations as they are activated in the here and now of the relationship with the therapist. While defenses by definition involve some alterations in the sense of reality, for those with pathological narcissism, in particular, they often serve the function of preserving an unrealistically inflated sense of self, stabilizing self-esteem, and of managing anxieties related to unwanted and/or unacceptable thoughts about the self. For those with pathological narcissism, defenses function to ward off any feelings, memories, fantasies, thoughts, or fears that might expose the individual to vulnerability, need, or imperfection and to block out alternative realities that pose a threat to the grandiose self-concept.

Individuals with narcissistic pathology, particularly those who are operating at a higher level of functioning and for whom grandiose self-organization confers some stability, have been observed to mobilize primarily the so-called minor image-distorting defenses (Perry, Presniak, & Olson, 2013; Clemence et al., 2009; Kernberg, 1975).[2] The minor image-distorting defenses include the following:

- *Devaluation*. The need to denigrate or diminish others in order to elevate the self.
- *Idealization*. The tendency to describe the other in hallowed, unrealistic terms that eliminates the complexity of positive or negative.
- *Omnipotence*. The tendency to present oneself as superior and as having special powers or abilities.
- *Introjection*. The process by which aspects of the other are incorporated into the self such that the boundary between self and other is denied.
- *Projection*. The attribution to others of the qualities one deplores in oneself.

[2]In these studies, defense use was rated from videotapes of interviews with patients in residential treatment (Austen Riggs Center, Stockbridge, MA) using the Defense Mechanism Rating Scale (Perry, 1990), an instrument that reflects many of the defenses listed as part of the Defensive Functioning Scale featured in the appendix of DSM-IV to provide working clinicians with descriptions of defenses. The diagnosis of NPD and the presence of NPD traits were rated by expert clinicians from an intake-guided clinical interview or from videotaped diagnostic interviews of patients.

More disturbed narcissistic persons tend to rely on major image-distorting defenses, including:

- *Projective identification.* Whereby intolerable aspects of the self are projected onto others, who are then maneuvered to behave in accordance with the disowned aspects. The other must be omnipotently controlled, lest the disavowed aspects are returned to the self.
- *Splitting.* The process by which contradictory experiences, affects, and representations with which they are linked are kept rigidly apart.

Illustrations of how these defenses manifest themselves clinically are shown in the following case examples.

Case Example: Tina— Defenses in Higher-Functioning Narcissism

Tina, a 35-year-old interior designer, had struggled for almost 10 years to make her own design consultancy profitable, ultimately failing. She finally accepted a job offer at a building services firm managing contractors conducting template apartment renovations. Tina devalued the "cookie-cutter" work of supervising template renovations, with little room for her individual input, feeling that her true calling was the higher art of design. She prided herself on what she felt to be her unique sense of style and taste. In her consultancy, Tina had experienced repeated conflicts with clients over matters of taste and style, and would become withdrawn and passive–aggressive with clients who disagreed with her recommendations for reasons of taste or budget, stalling projects, retreating from timely communication in ways that resulted in missed deadlines, cost overruns, and in more than one instance, legal action taken against her. Despite these difficulties, many clients would return to Tina. She was charming, and her work, when finished, was often very much to her client's liking.

Tina's use of omnipotence and devaluation was significant: It was *her* vision, hers was the "right" vision, and there could be no deviation. The behavioral control exerted around this feeling of omnipotence—the stalling, delays, endless drafts, and diagrams to impress her point on the client, the refusal to entertain an alternate view—were destructive professionally. However, these mechanisms allowed Tina to ward off and devalue any idea that was *not hers*, any idea that might threaten her ownership of a grand vision in which she was highly invested. It was not "the project" that was threatened, but her fragile self-esteem and sense

of self. Tina's subjective experience of these defensive processes was a feeling of superiority, intense frustration that her talents were insufficiently recognized, and contempt for those who accepted less than what was perfect and desirable aesthetically. These defenses made it difficult for her to acknowledge that she contributed to her work difficulties, such as her difficulty accepting help from others, even the help of collaboration among equals; her withdrawing in hurt and rage when others disagreed with her; and her hypersensitivity to rejection. The experience of her defensive process *by others* was profound. Although her actual talent in her field and her overall personability and charm made her attractive to others, her clients were victimized by her rageful withdrawal and the ensuing delays. The experience of "if only she would listen to me" (i.e., not reject my ideas, me) was also a profound theme expressed by Tina, who at the same time was completely rejecting in her behavior toward anyone who did not mirror precisely what she desired.

Such minor image-distorting defenses can have profound effects on the real experience of the self and can lead to distrust and further devaluation of others, or the tendency to spoil or denigrate what is good in others in order to enhance self-esteem and buttress the grandiose self (Clemence et al., 2009; Perry & Perry, 1996; Perry et al., 2013). Higher-functioning narcissistic individuals have also been found to rely on disavowal defenses, such as denial of any problem or failure, or retreat to autistic fantasy, which allows for narcissistic encapsulation and withdrawal in the face of disappointing life events (Cramer, 1999).

Those with a more vulnerable thin-skinned narcissistic presentation or those who are organized at a lower borderline level have been found to be more inclined to rely on the major image-distorting defenses of splitting and projective identification.

Case Example: Jason—Lower-Functioning (Borderline) NPD

Jason, a 20-something, aspiring musician, struggled during his graduate school years to contain his devaluation and contempt for his musician classmates who were more artistically and socially successful. His envious feelings led him to routinely devalue their accomplishments in these arenas, while concurrently condemning with true self-loathing and contempt his own perceived lack of accomplishment. He expressed indignation at the harsh world that had unfairly deprived him of "just a fair chance" to prove himself. In fact, he had been offered many opportunities to demonstrate his talents, each of which were compromised by his inconsistent participation—his withdrawal from fear, or his overenthusiastic embrace of the opportunity. He never felt his enthusiasm was sufficiently matched by those who enlisted him, and thus served as proof

of their "lukewarm" feelings about him. His response was often then angry, dejected, and sour.

These defensive patterns of idealization, devaluation, and contempt resulted in a subjective experience of Jason as a righteous victim, mistreated by the world, a state reinforced by the cycles and effects created by his defensive process. The subjective experience of those in his environment likely mirrored that of the therapist, who frequently felt guilty for never providing enough support, encouragement, and admiration for Jason's work or social efforts. Despite recognizing these feelings as a powerful and informative countertransference, the therapist, as is common with such patients, experienced the emotional drain associated with Jason's omnipotence, projection, and idealization/devaluation.

Thus, individuals with narcissistic pathology alone (such as Tina) may show somewhat different defensive profiles compared to those with comorbid narcissistic and borderline pathology (such as Jason). Those with comorbid BPD/NPD show a preponderance of major image-distorting defenses—primarily splitting between the positive and negative sectors of experience—and projective identification, as well as dissociation (Perry et al., 2013; Clemence et al., 2009). Projective identification leads the individual to locate these feelings in the other as he or she subtly manages to evoke the feelings in the other, sometimes leading to a paranoid orientation toward the therapist and others. Such a paranoid orientation may be evident not only in the cycles of idealization, projection, and devaluation to which these individuals are prone but also in withdrawal (autistic fantasy). The extent to which primitive defensive mechanisms buttress or fail to protect the grandiose self is one indication of the dominant presentation and underlying level of organization of narcissistic disorder—that is, whether they present as thick-skinned and grandiose, or thin-skinned and more vulnerable and masochistic (Rosenfeld, 1987; Stern et al., 2011).

Thus, the effectiveness or rigidity of defenses may also provide some indication of the underlying level of organization of the individual with narcissistic disorder. Interestingly, individuals with BPD have been observed to exhibit the primarily primitive defenses of splitting and projective identification, but they did not exhibit the narcissistic defenses of omnipotence, devaluation, and primitive idealization (Perry & Cooper, 1989; Zanarini, Weingeroff, & Frankenburg, 2009; Cramer, 1999). Perhaps this is because for individuals with narcissistic pathology who are functioning at a lower borderline level, idealization is brittle and rigid and inevitably breaks down, leading to intense negative feelings of rage, envy, mistrust, and hatred.

It is important to note that while most primitive defenses discussed above are splitting-based defenses, splitting may take a particular

configuration in those with narcissistic disorder at all levels of organization. The split in those with pathological narcissism is between a grandiose superior self-concept (linked to an idealized object) and an inferior, vulnerable self-concept (linked to a devalued and/or rejecting internal object). For the most part, the conscious identification of narcissistic people with a grandiose self-representation remains relatively stable, particularly in those who are higher functioning. This is in contrast to those with BPD, who oscillate chaotically between extreme positive and negative feelings about self and others, reflecting quite directly the split, polarized aspects of the representational world. In individuals with pathological narcissism, particularly those with a higher level of organization and/or grandiose presentation, splitting mechanisms may be eclipsed by the rigidity of the grandiose self-structure whereby states of inferiority and vulnerability are not only projected onto others but also kept implacably at bay. In addition, the reliance on omnipotence and projective identification protects the individual from awareness of conflictual motives that are less likely to be enacted or experienced in violently opposing, intense loving and hateful feelings, with a dismissive attitude toward others more common than a preoccupied one. Consequently, *initially* it may be more difficult to identify splitting mechanisms in those with narcissistic disorder. Identification of split-off negative aspects as part of the self may take significantly longer to identify and interpret than is the case in those with other personality disorders, such as BPD (Blais, Hilsenroth, & Castlebury, 1997; Clemence et al., 2009; Kernberg, 1975; Lingiardi et al., 1999; Perry & Perry, 2004; Perry et al., 2013).

Countertransference as an Information Source about Narcissistic Defenses

The difficulty in engaging and treating individuals with pathological narcissism and NPD is often attributed to either the rigidity of their negative projections onto others, which engages the "vulnerabilities toward projective identification in the therapist" (Perry & Perry, 2004, p. 318), or to their encapsulation in autistic fantasy, which is experienced as their refusal to project any aspect of the internal world into the therapeutic arena. The latter is a subtler type of devaluation that leaves the therapist feeling ignored, disengaged, and hopeless. Over time, however, as aspects of the grandiose self, both idealized and devalued, emerge in the relationship to the therapist as idealizing and devaluing transferences, the defensive splitting of negative self attributes from omnipotent idealized ones becomes more evident and available for interpretation.

It should be noted that research with a sample of individuals with BPD that included 70% who had co-occurring BPD/NPD found

improved defensive functioning after 1 year in TFP where interpreting defenses is a major aspect of technique (Levine, 2002). Examples of improved defenses included less reliance on primitive defenses, such as splitting in the context of increases in the use of more mature defenses, such as anticipation and repression, both within and across sessions (Levine, 2002; see also Perry et al., 2013; Zanarini et al., 2009). The increase in the use of more mature defenses over the course of treatment was accompanied by improvements in mentalization, attachment security, identity diffusion, and, most significantly, the developmental quality of self and object representations (Levine, 2002).

Since the complexity of the defensive processes may be masked by the omnipotence of the grandiose self in individuals with narcissistic structure, therapists should be carefully attuned to their countertransference reactions to help identify defenses at work. By reflecting on their internal experience of the patient, they may be able to determine which reactions are provoked by the patient in a way that represents projective processes (i.e., the activation within the therapist of an affect that the patient has difficulty accepting within him- or herself). For example, the therapist with a full and successful practice, who feels deskilled and incompetent with a narcissistic patient, may heighten his or her attunement to the areas of weakness or helplessness that the patient may be papering over by his or her defensive devaluation of others. In addition, omnipotent control and autistic fantasy may lead the individual with narcissistic pathology to hold the therapist at arm's length, rendering him or her as a shadowy, unreal figure compared to the patient's often vivid, if rigid, ideal internalized objects. This is often experienced in the countertransference as feelings of boredom, frustration, or insignificance on the part of the therapist. Hence, an understanding of these countertransference reactions can become a valuable source of information about and a conduit into the patient's internal world and interpersonal functioning.

Clinical Examples of Narcissistic Defenses

Some typical narcissistic defenses can be categorized as shown in the following examples:

> *Omnipotence.* "I speak to a lot of people but the only person whose judgment I trust is my own. I don't think anybody else really understands the economic situation we are facing. . . ." or "I just have to rely on my instincts."
>
> *Devaluation.* "He can go ahead and marry her but it's like he's consorting with a witch. She's repulsive and disgusting. . . ."

Idealization. "People adore me. Everywhere I go people adore me. It's one of the ingredients of my success." or "My mother is one of the most beautiful women you will ever see. Even though I'm 25 and she is 45, I know she's more attractive and looks younger than I do."

Denial. "I know he's broken up with me but I know it's temporary and he'll come back. I know he still loves me. After all, look at everything my family did for him. He'll never find anyone as caring, attractive, and devoted as I am to him. . . . I know he says I should stop texting him but I want to remind him about the love we shared. . . . "

Projection. "I can't believe you [therapist] are terminating with me because you have to go on internship. I know that you detest me."

Introjection. [In response to an interpretation made by the therapist]: "Didn't I just tell you that last week?" or "That shows some insight on your part, but I already knew that."

Autistic fantasy. "When I was upset I would go into a room and pretend to be a teacher like my father, passing out books and lecturing to imaginary students. I would spend hours pretending to deliver lectures so I didn't feel so alone. . . . "

Projective identification. "My professor in business school hates me and I know he wants to see me fail. He says that I'm not serious about getting my MBA because I have missed a third of the classes so far and I didn't hand in my midterm take-home exam on time. He is completely unaware of how demeaning and provocative he is to me. Of course, in the last class I quoted someone from the real estate world who directly contradicted what he had told us that week, and he was very sharp with me in front of the other students. I felt really embarrassed and I would really like to see him fired. I would drop the class, but for some reason I keep going."

Splitting. "I am furious at you for wasting my time by talking about the bill. You are greedy and untrustworthy . . . like the doctor of a friend who took her insurance but later sued her for the difference between what the insurance would pay and the regular fee . . . but there are very few people I trust, and I don't want to eliminate you. . . . " [Statement showing split view of the therapist.]

Self-Esteem Regulation and Narcissistic Disorder

The range of defenses described above function for those with pathological narcissism to protect the individual from threats to self-esteem. Indeed, pathological narcissism has been described as a disorder of

dysregulated self-esteem (Ronningstam, 2018), and has long been linked to fluctuations between inflated and deflated self-esteem in both clinical and laboratory situations (Cain et al., 2008; Kernberg, 1975, 1976, 2007; Kohut, 1977; Pincus et al., 2009; Vater et al., 2013; Morf & Rhodewalt, 2001). According to Kernberg (1970, 1975, 1976) and others, those with pathological narcissism may present with inflated self-esteem in order to defend against an inner sense of unworthiness or self-loathing, and where self-esteem is fragile but preserved may appear with smooth and effective social functioning. However, in the face of theats to self-esteem in the form of loss of relatonships or career, disappointments or demotions, or aging or illness, narcissistic individuals may become severely symptomatic or even suicidal. Building on these often-noted contradictions, social psychologists Morf and Rhodewalt have developed a dynamic, self-regulatory model of pathological narcissism that focuses on the self-regulatory and interpersonal strategies used to regulate self-esteem. In their model, the core of narcissistic pathology stems from high positive self-regard at the explicit, conscious level, masking vulnerability and impaired self-esteem at the implicit, unconscious level, leading to instability in self-esteem. Such fluctuations could be seen as deriving from a failure of individuals to meet their own exacting standards in the context of an implicit sense of self as special, privileged, or entitled—a discrepancy that highlights the gap between the real and ideal self-representations, the latter sometimes referred to as the "ego ideal." At the core of this model is the idea that the distinctive signature of narcissistic pathology is the "'in your face' determination to build, buttress, defend and enhance a grandiose self" (Morf et al., 2011, p. 56).

While the self-regulation strategies of people with pathological narcissism may be effective in eradicating anything that interferes with the preservation of their self-esteem, in the end their view of self may be so unrealistic and inflated that their relentless attempts to seek confirmation for it from themselves or from others are doomed to fail. Their ceaseless efforts to incorporate all that is desirable and good in the self and to deflect all that is negative, weak, or shameful onto others renders a sense of self inherently unstable. The attempts to make use of others, seen in the negative light of their own projections, remains perpetually unsatisfying. The dynamic self-regulatory model thus suggests that self-esteem dysregulation in narcissistic individuals has a relational as well as a self-component, in that they use both intrapersonal (grandiose fantasies about the perfection and superiority of the self) and interpersonal strategies (attempting to elicit admiration from idealized others and to denigrate devalued others who are not totally diffentiated from the self intrapsychically) to regulate self-esteem. However, the empirical research in support of this clinically derived "masked model" at present

is uneven because the validity and reliability of a measure of implicit self-esteem has not been established (Bosson, Lakey, Campbell, Zeigler-Hill, Jordan, & Kernis, 2008). By contrast, the explicit model of self-esteem, which stipulates that narcissistic individuals are characterized by high self-esteem (by self-report), has received empirical support primarily for those who have a grandiose but not vulnerable presentation (Miller et al., 2017). On the other hand, Vater et al. (2013) explored patterns of self-esteem regulation in a sample of inpatients with NPD and BPD compared to nonclinical controls; they showed that individuals with pathological narcissism and NPD had higher levels of implicit and explicit self-esteem than did those with BPD, but lower levels of both compared to nonclinical controls. These findings suggest that although high explicit self-esteem may be a distinguishing factor in grandiose narcissism, fluctuations in self-esteem may be a distinguishing feature of those with more severe narcissistic pathology in particular.

Shame in Narcissistic Disorder

Related to self-esteem dysregulation in those with narcissistic pathology is the proclivity to experience debilitating states of shame. This is thought to be a ubiquitous aspect of narcissistic disorders, although empirically linked primarily to vulnerable narcissistic states of mind (Di Sarno, Zimmerman, Madeddu, Casini, & Di Pierro, 2020). Clinicians have noted that individuals with pathological narcissism resort to a number of interpersonal and intrapersonal mechanisms, including aggression, anger, compensatory fantasy, and deflection of attention away from the self to diminish feelings of shame. A recent study has shown that, while both grandiose and vulnerable narcissistic individuals respond with anger to threats to self-esteem or self-images, those with vulnerable narcissism are also likely to respond with sadness and shame (Hyatt, Sleep, Lynam, Widiger, Campbell, & Miller, 2018). Research has also affirmed the link between narcissistic disorder and shame at both the explicit or conscious and implicit or unconscious levels (Ritter et al., 2014). Ritter and colleagues assessed shame in individuals with pathological narcissism and NPD with and without BPD, and nonclinical controls. They found that both explicit (consciously experienced) states of shame and implicit (unconscious) shame proneness were significantly higher in patients with NPD compared with clinical controls but were significantly lower compared to those with BPD. In comparison with both patients with BPD and nonclinical controls, those with NPD had the highest level of implicit self-states related to shame, indicating that those with narcissistic pathology may be characterized by feelings

of shame at an implicit level of which they may be unaware, but that may drive their defensive behavior and even self-destructiveness.

Suicidality in NPD Linked to Self-Esteem Dysregulation

When individuals are confronted with the discrepancy between the ideal and real self-representations, along with the realization that they cannot meet their ideal standards, this may be the catalyst for seeking treatment. In some cases, it may even precipitate a breakdown or sudden plunges into suicidal states or behaviors, leading to hospitalization (Ronningstam et al., 2008; Blasco-Fontecilla et al., 2009; Ansell et al., 2015; Ritter et al., 2014). In such cases when narcissistic illusions shatter, self-exaltation may spiral rapidly into self-destruction as in the myth of Narcissus. Several clinical and empirical investigations have identified narcissistic disorder both in adolescence (Apter et al., 1993) and late life (Heisel, Links, Conn, van Reekum, & Flett, 2007) as a suicide risk factor. This is because narcissistic adolescents may be overwhelmed by shame if they are unable to live up to their perfectionistic standards of achievement, while older adults may not be able to tolerate the narcissistic injuries of loss of attractiveness, health, and mental or physical capacitites that ensue with old age. In one longitudinal study that followed individuals with personality disorders over a 5-year period, those with NPD were more likely to make a lethal suicide attempt, while those with BPD were more likely to make impulsive suicidal gestures (Blasco-Fontecilla et al., 2009).

Ronningstam and others (Ronningstam et al., 2008; Apter et al., 1993) have identified a number of risk factors for suicidal collapse in those with pathological narcissism, even those who were functioning at a high level, including intolerance of feelings of shame and humiliation; affective dysregulation in the face of loss of ideal self-states; the acting out of vengeful anger toward others who inflict narcissistic injuries turned against the self; and a sudden breakdown of primitive defenses, particularly the failure to project intolerable feelings onto others to maintain the illusion of superiority and self-sufficiency. Any treatment tailored for those with pathological narcissism and NPD must consider the particular ways in which suicidality may manifest itself differentially, including the often-apparent lack of impulsivity and parasuicidality usually seen in patients with BPD. In addition, the narcissistic patient's conviction that he or she reserves the right to self-destruction means that suicidal acts may occur with little forewarning to the therapist. Extremely lethal and planful suicide attempts may seem to come out of the blue, although therapeutic exploration may reveal that the individual has long harbored

a secret suicide plan should intolerable life events, such as loss of status or job, financial reversals, relationship losses or disruptions, or health crises, occur. Paradoxically, suicidal fantasies may actually serve a protective role for individuals with pathological narcissism and NPD by preserving their sense of agency and control (Ronningstam et al., 2018).

For example, one patient, a high-functioning advertising executive in his late 40s, made a serious suicide attempt when he unexpectedly lost his job because of the downsizing of his firm during a recession. Overwhelmed with feelings of humiliation, he was unable to face his wife and children, and instead crashed his car into a guardrail on his way home from the meeting, thereby endangering others as well as himself. Although he had been in therapy for 6 months, he did not divulge his suicidal feelings to his therapist because he felt that they were not relevant to the treatment that focused on marital issues. He initially refused a contract that would make him take responsibility for his own safety, instead reserving the right to kill himself if the treatment "did not work." He agreed to the treatment contract only after extensive exploration of how his refusal was linked to a sense of omnipotent control that reversed his sense of humiliation and helplessness that led to the vengeful suicide attempt.

In those with malignant narcissism, suicide may function as an omnipotent attempt to transcend limitations and triumph over or defeat others, including the therapist. On the other hand, with less-disturbed narcissistic individuals, a sense of self-importance, fantasies of perfection and invincibility, and the characteristic defenses of omnipotent control and devaluation may actually function as deterrents to suicide (Coleman, Pincus, & Smyth, 2019; Ronningstam, Weinberg, Goldblatt, Schechter, & Herbstman, 2018).

It should be noted that pathological narcissism (both narcissistic grandiosity and vulnerability) have been associated not only with suicidality but with nonsuicidal self-injury including biting, burning, cutting, head banging, skin picking, hair pulling, and scratching in a sample of university students even when BPD features are accounted for (Dawood, Schroder, Donellan, & Pincus, 2018). Thus, it is important that therapy for those with pathological narcissism include techniques to address and contain the underlying precipitants and motivations for self-harm and suicidality. In TFP-N, these include (1) the formation of treatment contracts that spell out the individual's role in reporting and taking responsibility for containing suicidal feelings; (2) the exploration of the origins of suicidal impulses in states of narcissistic rage, shame, and humiliation (see Chapter 6 on the contract); and (3) the linking of such extreme emotional states to specific mental representations of self and others, including an intolerable concept of a sense of self as vulnerable, powerless, and humiliated in relation to a humiliating, superior other.

Summary and Conclusion

Individuals with pathological narcissism and NPD are characterized by a grandiose self that organizes the experience of self and others, and that distinguishes NPD from other personality disorders. The sense of self is often unrealistic and inflated, but it confers a sense of stability so that many narcissistic persons maintain smooth and stable social functioning and show less volatile fluctuations in self-states than in other personality disorders. In short, the grandiose self is a compensatory structure that serves to employ defensive functions against the underlying anxieties associated with vulnerability, weakness, and inadequacy, which are systematically and rigidly projected onto others. Relationships serve to support the sense of self, and hence the experience of others is often shadowy, vague, superficial, and colored by one's need to maintain a desirable view of the self. When such external validation fails in the face of threats to self-esteem, disappointments, or failures, narcissistic persons are vulnerable to sudden discontinuities and/or a breakdown in a sense of self, including overwhelming states of despair and self-destructiveness.

CHAPTER 3

Interpersonal Functioning in Pathological Narcissism

The deficits in self-functioning of those with pathological narcissism delineated in the previous chapter may not be immediately evident, since narcissistic individuals often maintain a façade of smooth and effective social and occupational functioning despite fluctuations between grandiose and vulnerable states of mind. Significant difficulties related to the dynamics of narcissism, however, may manifest more readily in the interpersonal realm. It is sometimes only when faced with the interpersonal challenges of relating at a deeper and more intimate level that individuals with pathological narcissism may reveal the full extent of their detachment from others with exploitative attitudes toward them, emotional reactivity in the face of personal slights or challenges, a tendency to blame others for their own limitations, antagonism, and in the case of vulnerable narcissistic presentation, social withdrawal and insularity. In this chapter, we focus on interpersonal functioning in those with narcissistic disorder, particularly on difficulties with intimacy and lack of empathy, and on the insecure attachment representations and deficits with mentalization that may underlie them. We begin with the developmental origins of narcissistic disorder in early childhood and family experience.

Parent–Child Interaction and the Development of Narcissistic Pathology

It is not clear exactly at what point in development the distortions in mental representations of self and others characteristic of the narcissistic

self-organization become consolidated, but its roots likely develop in early childhood experiences and relationships, when positive and negative aspects of the representational world are beginning to be consolidated into specific concepts of the self and other. The kernel of narcissistic pathology is often found in the actual talents or abilities of the child that draw parents' admiration and attention to begin with. In such situations, the parents substitute adulation of the child's special qualities for genuine love of the child as a unique, separate individual. Parents may overvalue such special qualities to buttress to their own narcissism or to compensate for their own narcissistic deficits or disappointments. This then leads the child to internalize an idealized, often unrealistically inflated concept of him- or herself as reflected by the parents, and to merge them with idealized views of the parents themselves that camouflage the parents' exploitativeness or other negative traits (Kernberg, 1975, 2007).

In the case of those who develop narcissistic pathology, parents may treat the child as an extension of the self, valorizing the child's real talents, while neglecting his or her needs for dependency and nurturance. Children destined to develop narcissistic pathology may suffer from emotional neglect by parents who may be emotionally distant or unavailable, or they may be overcontrolled by parents who are intrusive or overinvolved. In the face of either maladaptive parenting pattern, the child may turn away from the caregiver, defensively substituting idealized images of the self and the parent's perfection for actual dependency upon, and relatedness in depth with, parental figures. This may also occur when there is sudden de-idealization of the parents by the child as a result of exposure to unethical behavior on the part of parents or overt parental psychopathology.

Such a developmental pathway to narcissistic idealization and inflated views of self and other inevitably also evokes the child's anger and frustration, which must be split off and denied in the interests of preserving the idealized relationship. The child may develop a premature, precipitous sense of autonomy, turning away from disappointing, exploitative, or intrusive parental figures, in some cases developing a "false self" (Winnicott, 1971)—that is, a self that is adapted superficially, and sometimes even consummately, to parental needs and imperatives but that fails to develop a spontaneous, authentic existence.[1] As

[1]Winnicott (1960), for example, in his paper, wrote that ego distortion in terms of true and false self identifies a group of patients who may initially present as perfectionistic, high functioning, often extremely successful, and seemingly related and motivated for treatment, but who over time are felt to be inauthentic and remote, hold themselves superior to the therapist, and are inaccessible to deeper contact

a result, real dependency and mutuality—involving the give and take of both loving and aggressive interactions that enables the child to feel genuinely accepted and connected to others—is prematurely interrupted and denied. Instead, there is precipitous internalization of parental functions and the fusion of defensively idealized parental images with idealized images of the self. Primitive, distorted, or disappointing experiences and images of the parents are split off, repressed, or denied. These negative images of parental objects may be reprojected, leading to pervasive devaluation of external objects, and hostility and distrust toward them. There may be fear of others now perceived as denigrated or even harmful, or a wish to merge with venerated others as a result of projections of aspects of their own internal object world.

There are several retrospective and longitudinal empirical studies that have lent support to the parent–child relational patterns that clinicians have observed in their treatment of individuals with narcissistic disorder (Brummelman et al., 2015; Brummelman, Nelemans, Thomaes, & Orobio de Castro, 2017; Kernberg, 1975; Kohut, 1971, 1977; Shaw, 2010; Symington, 1993). For example, in retrospective accounts of parental attitudes of individuals rated high on both grandiose and vulnerable narcissism, individuals were found to recollect their parents as overvaluing and unduly praising (Brummelman et al., 2015; Imbessi, 1999; Millon, 1981); as cold and indifferent or neglectful (Otway & Vignoles, 2006); and/or as controlling, nonempathic, or overly permissive (Horton, Bleau, & Drwecki, 2006). Parents who are overinvolved and unrealistically validating have been observed in one longitudinal study to foster an unrealistic sense of superiority and entitlement in children with high self-esteem (Brummelman et al., 2017), but significant, excessive parental praise of children may also lead to deflated self-esteem. In fact, such particular patterns of parenting are among the antecedents of child fluctuations in self-esteem and emotional

---

with him or her. Winnicott hypothesizes that such individuals have developed a false self-organization, in response to a maternal figure who was not able to match the spontaneous gestures of her child in a way that crushed the child's illusions of omnipotence rather than ameliorating them, thereby setting the stage for their persistence into adulthood. Such infants may respond with compliance or aligning the self in the direction of the mother's internal reality rather than developing their own living reality of something that can be symbolized. Winnicott makes the point that where the mother's adaptation to the infant is good enough, the reality of the self and others is introduced in ways that are tolerable and that allow for the infant to gradually relinquish omnipotence while still recognizing and enjoying the illusory element of omnipotence as it comes into play with imagining and creativity. Where this is not the case, the infant's omnipotence persists in unmodulated form. The latter overlaps with the omnipotence of narcissistic disorder.

dysregulation, which may predispose children to the development of narcissistic disorder. In situations where parents overvalue or inflate the child's accomplishments to aggrandize themselves, while rejecting the child's real needs for nurturance, the child may develop a bifurcated sense of self as superior on the one hand, but worthless and devalued on the other. In such individuals, the surface grandiosity that puts forth a positive explicit self-concept masks an implicit sense of self as depleted or empty (Kernberg, 1975, 1979; Vater et al., 2014) since the parents' overvaluation of the child often does not correspond to the child's real potential or accomplishments (Millon, 1981). In either case, the child organizes around a grandiose sense of self (Bardenstein, 2009; P. F. Kernberg, 1989; Thomaes, Brummelman, Reijntjes, & Bushman, 2013). Stechler (2014) reminds us that "overregulation and underregulation of developmental functions can both lead to psychological aberrations" (p. 385). Paradoxically, then, narcissistic pathology may result from the parents being unduly gratifying and indulgent, as well as unduly depriving and exploitative, fueling, rather than modulating, the child's fantasies of omnipotence. The end result is that the child retreats defensively from genuine intersubjective relatedness to others into an ideal internal world where inflated views of the self and ideal concepts of others reign.

The nascent narcissistic organization has been observed by expert clinicians and developmental researchers to have consequences for peer relationships and academic performance, along with self and affect regulation, and the development of ethical values (P.F. Kernberg, 1989; Thomaes, Brummelman, Reijntjes, et al., 2013). Based on extensive clinical investigation of children's play, peer relationships, and familial relationships, child psychiatrist Paulina Kernberg (1989) identified a profile of narcissistic disturbances in childhood characterized by grandiosity and arrogance, preoccupation with success, inability to tolerate failure, and an inability to learn new skills and maintain consistent academic effort, or to persevere on tasks on which they are not immediately successful. There was also failure to acknowledge flaws and difficulties in the self, all stemming from an idealized sense of self and deficiencies in the development of conscience and superego. Children who present as narcissistic also show a lack of empathy or guilt over aggression toward others whom they exploit and devalue, leading to poor peer relationships—all of which are evident in play and school performance. They may also show antisocial traits and paranoid anxieties about others.

Paulina Kernberg (1989) identified several family risk profiles for the development of such manifestations of narcissistic pathology in children, including (1) families where successful and highly accomplished parents convey to the child that he or she must reflect and augment parental accomplishments and talents, and who may suffer narcissistic

injury if he or she has lesser abilities; (2) families where affluent parents convey to their children a sense of entitlement, where control and exploitation of others is accepted as the norm; (3) families of divorce where the children are enlisted in cyclical dynamics of devaluation and idealization of one parent at the expense of the other in order to empower themselves and express distress; and finally (4) families where parents who adopt children attempt to compensate for the adoption with excessive praise and valorization of the child disproportionate to his or her real abilities.

Empirical Research on Parent–Child Interaction Patterns

Several cross-sectional studies of children with narcissistic disorder have identified certain temperamental factors that appear to lead to narcissistic pathology. These include a high proclivity for approaching others and an early hypersensitivity to rewards on the part of the child. Such characteristics may contribute to the formation of narcissistic traits (Thomaes, Brummelman, Reijntjes, et al., 2013) and potentially to the development of narcissistic organization (e.g., the grandiose self) in childhood. It is possible that in a climate of parental overvaluation and/or aloofness, children with such an approach temperament may become unduly dependent on parental praise and attention (Elliot & Thrash, 2002; Thomaes, Brummelman, Reijntjes, et al., 2013).

By midchildhood, children rated high on narcissism as assessed on self-report measures are highly reactive to the valuations of others and show a high degree of oscillation between positive (e.g., euphoria, pride) and negative (e.g., shame, anger) emotions. In addition, as is the case with narcissistic adults, such fluctuations are more likely to occur when grandiose self-views are threatened. In sum, the antecedents of grandiosity, and perhaps narcissistic disorder, may be observed by middle childhood in the child's preoccupation with his or her value and worth (Barry, Frick, & Killian, 2003; Thomaes, Stegge, Bushman, Olthof, & Denissen, 2008; Thomaes, Bushman, Orobio de Castro, Cohen, & Denissen, 2009); in fluctuations in self-esteem; in the tendency to react aggressively when his or her inflated self-view is imperiled, particularly in those with high self-esteem; and in the exacerbation of social anxiety, particularly in those with low self-esteem (Thomaes, Brummelman, Reijntjes, et al., 2013).

Based on these and other findings, there is a consensus (Thomaes, Brummelman, Reijntjes, & Bushman, 2013; Ronningstam, 2013) that a stress-diathesis model accounts for the development of narcissistic pathology. Intrapersonal and intrafamilial factors identified earlier may intersect with sociocultural factors, such as the overemphasis on

self-esteem independent of actual accomplishment in the educational system and the image veil offered by social media that allows for the promotion of unrealistic, inflated views of the self. These factors may trigger a latent temperamental oversensitivity to reward and/or difficulty with managing negative self-concepts and negative affect, leading to the later development of pathological forms of narcissism.

It is important to take into consideration that there may be genetic and biological, as well as psychological and social factors, that might contribute to the development of NPD. In a number of studies, NPD has been found to have a moderate to high level of heritability, although the rates vary considerably in clinical versus nonclinical groups. Twins diagnosed with NPD have a heritability rate of 25% (in nonclinical groups) to 79% (in clinical groups; Jang et al., 1996; Torgersen et al., 2000, 2012), but the exact nature of the temperamental or trait aspects that might genetically predispose a child to develop pathological narcissism or NPD in adulthood necessitates further research. The extent of the contributions of genetics and temperament versus intergenerational patterns of parent–child and family interaction in the development of narcissistic disorder remains in question and may vary according to the specific case (Ronningstam, 2013).

Empirical Research on Temperament, Gender, and Developmental Factors

Among the temperamental factors that may predispose the development of narcissistic disorder are difficulties with the management of negative affect and aggression, which in those with narcissistic disorder may be activated in the face of self-esteem threats, criticism, or rejection. This is particularly the case with male children and adolescents who show grandiose manifestations of narcissism, and who react to self-esteem threats with aggressive behaviors or attitudes. Girls, on the other hand, are more likely to exhibit vulnerable manifestations of narcissism, and are likely to be self-absorbed and self-centered, withdrawing from others rather than lashing out in the face of self-esteem threats or rejection (Thomaes et al., 2009). However, even when male adolescents with prominent narcissistic traits appear to be socially confident and contained on the surface, they may experience visceral distress in response to social exclusion, responses of which they are unaware. Neurocognitive researchers Cascio and colleagues (2015) found that adolescent males who showed high levels of self-reported narcissistic traits did not report significant levels of threat or distress following simulated experiences of social exclusion. They did, however, show higher levels of activation in the

neural regions associated with social pain (the anterior insula, dorsal anterior cingulate cortex, and subungual anterior cingulate cortex), as measured by activation of neural responses during the Cyberball task,[2] as assessed by functional magnetic resonance imaging (fMRI; Cascio et al., 2015). These findings again point toward a split in the self between a consciously held grandiose self-concept and a vulnerable sense of self, prone to distress and rejection sensitivity, that is dissociated from conscious awareness.

While the cross-sectional studies discussed above provide some evidence for the characteristics that might predispose children and adolescents to develop narcissistic disorder, further longitudinal research is called for to identify the intrafamilial and developmental trajectories of normal and pathological in childhood and adolescence. In a prospective longitudinal study, Carlson and Gjerde (2009) traced the development of narcissistic traits from preschool through adolescence and young adulthood. Using a preestablished measure of personality and social characteristics of preschoolers (the California Child Q-Set [CCQ]; Block & Block, 1980), Carlson and Gjerde identified five preschool characteristics that were likely to foretell the development of narcissistic personality features, including (1) interpersonal antagonism, (2) histrionic tendencies, (3) high activity level, (4) the desire to be the center of attention, and (5) undercontrol of impulse. The participants were evaluated again at ages 14, 18, and 23 by independent observers, using the California Adult Q-Set (CAQ; Block, 1961), which is based on in-depth clinical interviews and experimental laboratory-based tasks. Based on these observations, the researchers derived narcissism scales that included the items of the CAQ that correlated most highly with the NPI (Raskin & Hall, 1981), including items such as "expresses hostile feelings directly," "unaware of self-concern," "feels satisfied with self," or "characteristically pushes limits."

All of the preschool scales predicted the development of narcissistic traits at ages 14, 18, and 23 (with the exception of Interpersonal Antagonism, at age 23 only). Interestingly, males overall were found to be more narcissistic than females, which may be due to differences in socialization for males and females, such that males are generally expected to be more hostile, impulsive, and self-centered. In addition, there were

[2]The Cyberball is an ostensibly online ball-tossing game that is often used to assess for reactions to social exclusion, rejection, or discrimination. The participants are led to believe that they are playing freely with two or three others, but the game is regulated by the programmer so that the participants are excluded more than they are included.

significant increases in narcissism from age 14 to 18, which Carlson and Gjerde (2009) observe is consistent with previous research (Twenge et al., 2008a, 2008b). This suggests that there is a rise in narcissism in adolescence and young adulthood for college students in the historical cohort to which the participants belonged. Based on their findings, Carlson and Gjerde suggest that the characteristics observed in preschoolers who later are rated high on narcissism in adolescence and young adulthood—including inflated self, antagonism toward others, and inadequate impulse control—interact in a dynamic development process that may spiral toward narcissistic pathology over time. They also found that primitive defenses play a role in the continuity of these precursors identified in early childhood. In such a spiral, a grandiose but fragile sense of self fuels an exploitative interpersonal approach to relationships in order to elicit admiration from others, which if disrupted, may evoke hostility and lead to projection of negative aspects of self and affects onto others to shore up self-esteem. Primitive defenses and impulsivity short-circuits self-reflection and results in defensive exclusion of difficult realities, including alternative views of the self (Carlson & Gjerde, 2009).

The above research and clinical investigations suggest that there are forerunners of pathological narcissism underpinned by a grandiose self-organization in childhood and adolescence that may coexist with vulnerability to feelings of inadequacy, inferiority, and social exclusion. These oscillations in self-experience not only disrupt a sense of continuity in identity at a point at which identity is being consolidated but also contribute to disturbances in interpersonal functioning, including withdrawal from close relationships and aggression toward others. The findings also point toward difficulties with tolerating feelings of anger and aggression in social relationships, leading to an isolating sense of self-sufficiency and independence and/or to hypersensitivity to exclusion by or criticism from others. Based on these studies, we might conclude that those with narcissistic disorder have split multiple models of self in relation to others at the representational level. At the conscious level, a concept of self as superior, self-sufficient, and invulnerable (in relation to an inferior other) vies with a self-concept as vulnerable and inferior (in relation to another perceived as rejecting and superior) that is split off from conscious awareness. This pattern may be reversed in those with vulnerable presentation, who see themselves as inferior to and fear rejection from idealized others on whom they project their own covert grandiosity. Any effective and comprehensive treatment for narcissistic disorder ought to consider such multiple conflicting mental representations that underlie the overt symptoms of narcissistic pathology.

The Role of Childhood Neglect and Abuse in the Development of Narcissistic Disorder

Adverse childhood experiences, such as verbal, physical, and sexual abuse, and emotional neglect and abuse, have been well documented as risk factors for the development of both clinical and subclinical symptoms of borderline and narcissistic personality disorders by both clinicians and researchers (Afifi et al., 2011; Bowlby, 1977, 1978, 1980; Ensink et al., 2017; Cohen et al., 2014; Grover et al., 2007; Shaw, 2013; Zanarini et al., 1998). The linkage between narcissistic pathology in adults and experiences of childhood maltreatment (reported retrospectively or gleaned through case records) has now been established in a number of studies (Cater et al., 2011; Cohen et al., 2014; Ensink et al., 2017; Johnson, Smailes, Cohen, Brown, & Bernstein, 2000; Maxwell & Huprich, 2014). Interestingly several studies have shown specific associations between verbal and emotional abuse and neglect and the development of narcissistic pathology in adolescence and young adulthood (Afifi et al., 2011; Hengartner, Ajdacic-Gross, Rodgers, Müller, & Rössler, 2013), with the strong link between NPD and verbal abuse in one study found to be independent of temperament, physical or sexual abuse, neglect, or co-occurring disorders (Johnson, Cohen, Smailes, Skodol, Brown, & Oldham, 2001). Those with vulnerable narcissistic presentation, characterized by deflated self-esteem, hypersensitivity to criticism and rejection, and shame proclivity may be particularly susceptible to the impact of verbal and emotional abuse. Shaw (2013) sees individuals with vulnerable narcissism as having endured a type of cumulative relational trauma in which their self-esteem was damaged and their subjectivity quashed by narcissistic parents who consistently demeaned and humiliated them when they failed to meet often inflated, unrealistic expectations. Indeed, the quality of the parent–child bond, and particularly the traumatic experiences of abuse and neglect, have been found to be significant mediating factors between the development of pathological narcissism and self-esteem regulation (Maxwell & Huprich, 2014).

It is important to note that just as childhood maltreatment (emotional and physical abuse and neglect) may predispose the individual to develop pathological narcissism and/or NPD, so may narcissistic personality pathology predispose the individual to become symptomatic in adulthood in the face of later trauma. In one study, it was found that individuals with vulnerable narcissism who experienced sexual abuse in childhood were more likely to develop posttraumatic stress disorder (PTSD) symptoms later in life if they experienced traumatic events, such as war trauma, terror, torture, and so forth (Bachar, Hadar, Barcuh,

Dor, & Freeman, 2015). Similarly, individuals with pathological narcissism were more likely to experience acute anxiety symptoms when exposed to life-threatening mass trauma than were those without narcissistic personality pathology (Besser, Zeigler-Hill, Pincus, & Neria, 2013). Furthermore, individuals may develop transient narcissistic symptoms in the face of intolerable feelings of humiliation, rage, and shame when they are faced with failures and overwhelming losses (of status, employment, or relationships; Ronningstam, 2018). These studies suggest that (1) there is a relationship between trauma and the development of narcissistic personality pathology and symptomatology throughout the life cycle; and (2) that when considering this relationship, one must take into account the individual characteristics and inner subjective experience (e.g., the meaning and internal psychological construction of the trauma for the individual), as well as the objective severity of the traumatic event.

Speaking to gender differences in the impact of trauma and abuse in the development of narcissistic disorder, Ensink, Chretien, and colleagues (2017) found that adolescent females were more likely to show high levels of internalizing disorders (e.g., depression, social inhibition) or externalizing behaviors in the context of an abuse history if they rated themselves high on pathological narcissism (as assessed by the PNI; Pincus et al., 2009). Interestingly enough, they found that both vulnerable and grandiose narcissistic traits tended to diminish for adolescent girls from ages 14 to 21, while they increased for adolescent boys. Ensink, Chretien, and colleagues (2017) concluded that "girls may be more vulnerable than boys in terms of neglect and abuse affecting core self-processes and undermining the development of healthy narcissism that can facilitate self-regulation . . . and increasing the underlying risk for vulnerable and grandiose narcissism" (p. 310). These findings suggest that there may be somewhat different developmental pathways toward pathological narcissism and NPD in adulthood.

Adolescent Development and Narcissism

Adolescence may be a particularly high-risk period for the development of pathological narcissism, as indicated earlier. Adolescence is a fluid time when there are transient regressions and symptoms as the adolescent defines a self that is uniquely separate and autonomous in what Blos (1967) has called the second individuation. As adolescents consolidate identifications with different attachment figures, they may swing between idealization and devaluation of them, even as they relinquish their dependency on them. It is noteworthy that adolescents appear to

be more dismissing of attachment in this period and that narcissistic traits or features tend to become more pronounced. As they separate from parents and move toward increasing autonomy and consolidation of identity, adolescents may present as supremely self-sufficient and dismissive of ties to parental figures, which may account for higher rates of dismissing attachment in this group (Ammaniti & Sergi, 2003). There may be transient idealization of aspects of the self and preoccupation with appearance and the body, as well as discontinuities in the sense of self as a result of bodily and cognitive changes—all of which may involve periods of identity confusion.

For this reason, it can be difficult to differentiate between those adolescents who have major identity disturbances in the form of lack of self-cohesion, goal directedness, and a chronic sense of emptiness, and those who have transient narcissistic traits that may be typical of adolescents. In addition, as Stern (1979) points out, the heightened narcissism of adolescents, which to some extent reflects the collective narcissism of our society, makes it increasingly difficult for adolescents to develop the capacity to integrate tenderness and erotic desire in love relations, so that sexual performance takes precedence over emotional closeness.

Assessment of narcissism over the life cycle has revealed more pronounced narcissistic traits in adolescents with levels diminishing in the later stages of the life cycle (Foster et al., 2003). This increase in the narcissistic traits of self-absorption and compulsive self-reliance (often alternating with the need for reassurance from and connection with attachment figures) may be normative and even adaptive for adolescents. Avoidance of dependency and repudiation of closeness with attachment figures is usually relinquished as the individual matures and identity is consolidated. This is not the case for those adolescents who consolidate their identity by organizing around a grandiose self-structure, and who may later develop narcissistic pathology.

Pathological narcissism may develop in situations where adolescents (1) rely too heavily on parental approval and adulation for self-esteem regulation, particularly when they have been the repository of their parents' frustrated desires and wishes for narcissistic fulfillment; (2) dismiss or devalue parental figures in attempts to counter their own disavowed wishes for protection, care, or dependency; and (3) develop insecure/disorganized attachment representations in the context of trauma or maltreatment. The latter is not limited to childhood experience but may develop when adolescents have experienced sudden disillusionment in regard to parental figures (Ammaniti & Sergi, 2003). Speaking to this point, Maldonado (2006) discussed the onset of narcissistic pathology in an adolescent male because of sudden and massive de-idealization of the parents who had previously served as the embodiment of his ideals and

values. The de-idealization stemmed from a confrontation with parental transgressive behaviors, including infidelity and psychopathic behavior.

The rise of narcissistic pathology in adolescence may also sometimes result from an intensification of parental overinvolvement and investment in the child's accomplishments as he or she begins to individuate and separate. This is evident in the contemporary phenomenon of "helicopter parenting," a pattern in which parents overinvest in the child and unduly involve themselves in his or her life after it is developmentally optimal, often writing or editing papers for them in high school or college, or maintaining near constant contact with their child through texting and tweeting and other forms of social media. In addition, parents may abrogate their authority, treating their offspring as peers, even giving the child a voice equal to or greater than that of the parents in making crucial family decisions, and generally expecting their adolescent child to augment their own unfulfilled lives and aspirations. In contrast to previous generations where adolescents modeled themselves on their parents, many parents today ape adolescent culture by adopting the dress, speech patterns, and culture of their offspring. The result is that midlife, rather than being a time of reflection, deepening maturation, and generativity, has become for many individuals in our society a time of emotional impoverishment and regression. (Hence, the increasing number of individuals with narcissistic pathology who present for treatment in midlife.) Adolescents who are made by parents to experience the self as the center of the universe may come to feel entitled and narcissistic (Thomaes, Brummelman, Reijntjes, et al., 2013; P. F. Kernberg, 1989). Consequently, they may experience a devastating sense of failure if they do not live up to parental expectations. At the Personality Disorders Institute (PDI), we see many cases of narcissistic collapse in adolescents and young adults who are characterized by the syndrome of failure to launch. In our view, this is often an external manifestation of narcissistic pathology in which individuals opt to withdraw from life's challenges and do nothing, rather than forthrightly testing and accepting their actual capacities, and working to develop them to the best of their ability. Such developmental paralysis may also be the sequalae of insecure attachment relationships to significant others that interfere with phase-appropriate separation and autonomous functioning.

Attachment, Mentalization, and Narcissistic Disorder

Attachment theory and research have provided a new theoretical framework and research foundation in support of object relations formulations of narcissistic disorder. The development of insecure and disorganized

attachment patterns have been found to be a major factor in the development of personality disorders, including NPD. John Bowlby (1977), the chief architect of attachment theory and originally an object relations theorist, was the first person to explore the origins of personality disorders, including narcissistic disorder in early parent–infant attachment processes. He stipulated that the quality of childhood attachment experiences determined the "later capacity to make affectional bonds as well as the whole range of adult dysfunctions," including "neurotic symptoms and personality disorders" (p. 206). Connecting pathological narcissism with specific patterns of attachment, Bowlby (1988) observed that individuals with avoidant–dismissing internal working models of attachment were likely to have been continually rebuffed by attachment figures when seeking comfort and protection, and consequently might "attempt to live life without the love and support of others . . . to become emotionally self-sufficient . . . and may later be diagnosed as narcissistic or as having a false self of the type described by Winnicott (1960)" (p. 125). Bowlby's (1988) clinical observations have been affirmed by subsequent research indicating that individuals with narcissistic pathology are likely to be classified with insecure/disorganized states of mind with respect to early attachment experiences and relationships. Dismissing individuals often show a cool, contemptuous attitude toward attachment figures whom they describe as laughable, foolish, disappointing, and/or inferior, and tend to speak about their attachment figures with active derogation or brittle idealization. These devaluing or idealizing states of mind with respect to attachment are accompanied by self-enhancing statements that valorize personal strengths and autonomy and disparage any need for or dependency on attachment figures (Main & Goldwyn, 1998; Diamond, Clarkin, et al., 2014; Meyer & Pilkonis, 2011).

Subsequent research has linked narcissistic disorder with a wider range of attachment representations, including secure, insecure (dismissing, preoccupied), and disorganized (Diamond, Levy, et al., 2014; Diamond, Clarkin, et al., 2014; see Meyer & Pilkonis, 2011, for a review). Nonetheless, Bowlby's (1977, 1988) original observations of the dismissing detachment of narcissistic patients resonates with those of contemporary clinicians, who have noted the "detachment of narcissistic patients serves to protect them from needing any relationship at all" (Bromberg, 1993, p. 159), "the state of non-relatedness in which there is 'a fundamental inability to take anything in' " (Modell, 1980, p. 259), or "the defense of avoidance of true dependency . . . often matched by the complementary defense of omnipotent control" (Kernberg, 1984, p. 627). As Steele, Steele, and Murphy (2009) point out, knowing the current state of mind with respect to attachment may enable the clinician to understand how individuals' "probable attachment history has

shaped their emotional regulation strategies and accompanying mental representations" (p. 635).

Internal Working Models of Attachment and Mental Representations

Bowlby (1988) and followers (Main et al., 1985) hypothesized that a significant aspect of the cognitive/affective structures that comprise the representational world are internal working models of attachment—that is, the child's specific representations of early attachment-related transactions, particularly how the caregiver responded to him or her in times of loss, threat, or danger, when the attachment system is likely to be activated (Bowlby, 1982b). The history of parent–child attachment transactions become encoded in internal working models of self in relation to others that encompass not an objective picture of the parent per se but rather the "history of the caregiver's responses to the infant's actions or intended actions with/towards the attachment figure" (Main et al., 1985, p. 85). Internal working models of attachment are akin to self and object representations in that they both index cognitive-affective schemas that include conscious and unconscious dimensions, and tend to be quite stable (Hamilton, 2000; Waters, Merrick, et al., 2000; Weinfield, Sroufe, & Egeland, 2000). However, like self and object representations, internal working models of attachment are subject to revision with changes in life circumstances, such as abuse, illness, divorce, or death of a parent, which may precipitate shifts from security to insecurity (Lewis, Feiring, & Rosenthal, 2000; Waters, Merrick, et al., 2000). On the other hand, internal working models can also become more secure with the development of new relationships in which the patient has experienced *contra* insecure attachment models, such as the therapeutic one, which may catalyze shifts from insecurity to security (Buchheim et al., 2017; Levy et al., 2006). In addition, genetic factors that become more salient in adolescence when the shared environment of the family becomes less central for the child's development (Barbaro, Boutwell, Barnes, & Shackelford, 2017) may also lead to shifts in internal working models (see Raby, Cicchetti, Carlson, Egeland, & Collins, 2013, for a review).

We have hypothesized that TFP, with its focus on the patient's moment-to-moment experience with the therapist in the here and now of the transference relationship, will activate the attachment system with its dominant internal working models of attachment. In TFP we focus on the gradual building up, modulation, and integration of object relations linked to different behavioral and motivational systems, including attachment, as they emerge in the clinical material and are manifest in the transference to the therapist. TFP is based on the idea that mental representations that organize the transference are linked to a broad

array of motivational systems, including the erotic system (sexuality/libido), the play bonding system, the fight flight system (aggression), and attachment-separation system among others (Panksepp, 2003, 2008; Panksepp & Biven, 2012; Diamond & Blatt, 1994; Auerbach & Diamond, 2017; Kernberg, 2018). From a contemporary object relations perspective repetitive experiences of self in relation to others, linked to different behavioral systems, themselves organized and activated by particular brain structures and neurotransmitters and by primary inborn affect dispositions (e.g., joy, rage, surprise, fear, disgust, sadness, excitement), are gradually internalized as dyadic structures of self and object representations, forming the basic building blocks of the mind. Contemporary attachment theorists have focused on how such motivational structures comprise internal working models of self in relation to an attachment figure shaped in part by the caregiver's responses to the child's inborn proclivity to seek care and comfort in time of danger or threat when the attachment system is activated. However, from a contemporary object relations perspective such internal working models are also intertwined with representations of self in relation to other based on other motivational systems. For example, the caregiver who soothes and comforts the child who seeks proximity in times of danger or stress is also the caregiver who inevitably stimulates him or her sensually in the course of ordinary interactions, even if in a bounded way. Bringing attachment theory in line with object relations theory, Kernberg (2018, p. 38) has observed that "it was the effort to objectify in behavioral terms these assumed intra-psychic structures that led Bowlby and Ainsworth to develop contemporary attachment theory as the behavioral correspondence of the internalized object relations set up under the influence of early mother infant interactions (Diamond & Blatt, 2007)."

The close linkage between attachment and object relations theory is evident in Bowlby's (1973) statement that "the concept of working models is no more than a way of describing in terms compatible with systems theory, ideas traditionally described in such terms as 'introjection' of an object (good or bad) and 'self-image' " (p. 204). In keeping with object relations theory, Bowlby (1982b, 1988) proposed multiple systems that interact with the attachment system (e.g., sexual/reproductive, aggression, play/exploration), but he believed that "in keeping them distinct, we are able to study not only the ways that they differ, but also the ways that they overlap and interact with each other—as it has long been evident they do" (Bowlby, 1988, p. 6). By late childhood and into adolescence and adulthood, attachment patterns congeal into secure or insecure internal working models but later on, influences from other behavioral and motivational systems continue to shape representational models of self in relation to others in increasingly complex hierarchically organized dyadic and triadic structures.

Adult Attachment Interview (AAI)

This linkage between infant attachment patterns evident in parent–child behaviors in the Ainsworth Strange Situation (Ainsworth et al., 1978) and parents' current attachment representations was established by Main and her colleagues through research with the Adult Attachment Interview (AAI; George et al., 1996). The development of the AAI, which assessed patterns of narrative discourse in adults, catalyzed a "move to the level of representation" in attachment research (Main et al., 1985), which has also served to study the transmission of attachment patterns from parent to child and across generations (Main et al., 1985; Main, Hesse, & Goldwyn, 2008). The AAI (George et al., 1996) is designed to elicit thoughts, feelings, and memories about early attachment relationships in a semistructured interview format. It is designed to "surprise the unconscious" (George et al., 1996, p. 3) by introducing attachment-related stress with concrete probes designed to elicit episodic memories. The AAI is further described in Chapter 9. Since the AAI has been shown in numerous studies to be a reliable and valid method to assess current mental representations of attachment (Main et al., 2008), we have used it as one method of assessing mental representations and the way they change over the course of TFP treatment.

AAI Classification System

Main and Goldwyn (1998) identified three major *organized* patterns of attachment: (1) secure/autonomous, characterized by open, seemingly honest, coherent discourse that gives the impression that individuals value the significance of attachment relationships; (2) dismissing, characterized by a constricted and distant discourse style in which all relationships are either idealized or devalued, a valorization of the self as independent and autonomous, with a denial of feelings of "needing or depending on others" (Main et al., 2008, p. 57); and (3) preoccupied, characterized by a confused discourse style that suggests that individuals remain either passively or angrily entangled in attachment relationships. Subsequent research in both adults and infants in high-risk groups identified two *disorganized* categories that bespeak of lack of integration in the attachment representations: (1) "cannot classify," characterized by lack of any consistent discourse style or shift in discourse strategy in the course of the interview or by two diverse but distinct discourse patterns; and (2) "unresolved for loss and trauma," characterized by a breakdown or lapses in the monitoring of reasoning and discourse in response to specific questions about loss and abuse (Hesse & Main, 2000; Hesse, 1996; see Steele & Steele, 2008, for a full discussion of the AAI classifications and their clinical applications). Individuals with disorganized

attachment are usually characterized by a lower level of personality organization and less adequate psychological structure. Those with disorganized attachment (either unresolved or cannot classify) tend to show multiple unintegrated and contradictory states of mind with respect to attachment, and thus, not surprisingly, are more highly represented in clinical groups and particularly in those with personality disorders (Bakermans-Kranenburg & van IJzendoorn, 2009).

The Convergence of Object Relations Concepts of Pathological Narcissism with Attachment Theory and Research

There are several major trends that have brought concepts of attachment and narcissism increasingly into alignment, setting the stage for investigations of how they interconnect and diverge with other motivational systems. First, as discussed in Chapters 1 and 2, it is increasingly recognized that pathological narcissism is a heterogeneous disorder that manifests itself with different presentations at different levels of organization. This suggests that there may be underlying differences in aspects of the internal world of mental representations, including attachment representations and/or disorganization in the attachment system that necessitates further exploration. Second, as psychoanalytic theory has become increasingly focused on the nature and quality of mental representations of self and others as the core of pathology, there has also been increasing interest in how such representations might have roots in early parent–child transactions. This has led psychoanalytic clinicians to interest themselves in laboratory-based assessment of parent–child attachment patterns, and in research instruments, such as the AAI, that link such infant attachment patterns with attachment representations in adults. The central tenet of object relations theory is that infants are inherently object seeking or oriented to seek relationships with significant others from birth on, and further that these early interactions are internalized as dyadic memory units of representations of self and others (object) infused with positive or negative primary affects. The nature of these bonds contribute to the way they come to be internalized and represented, and converges with the emphasis among attachment theorists and researchers that infants have a psychobiological proclivity to form attachment relationships, which as Fonagy and Campbell (2015) point out, function as "the driver of intentional (thought and feeling-driven) action and the primary underpinning for mental development" (p. 231). Beginning with Fairbairn (1952) narcissism has been defined as a state in which the ego is identified with objects and involves the investment of loving libidinal feelings and aggression or hate in the self. The idea that pathological narcissism is linked to the vicissitudes of both self and object relations (Green, 2002; Kernberg, 1984, 2018) suggests that

narcissism and attachment may be inextricably intertwined from the dawn of life. In fact, object relations theorists have redefined the concept of narcissism as due to excessive drive investment in internalized objects as a defense against disappointments as a result of disappointment in or frustration with primary object relating (Fairbairn, 1952; Green, 2002; Kernberg, 1984, 2015, 2018). In the previous two chapters, we presented the idea that narcissism involves a state of detachment from the object, involving a split in the self, where one part of the self remains in contact with the attachment object, who is the source of emotional life and ensures survival, but where the core self in the face of disappointment and frustration prematurely internalizes the object, which is now idealized and melded with the self. Attachment theorists and researchers have provided close systematic observations of the conditions under which such a turning away from object relating, so fundamental to those with narcissistic pathology, might occur and how this is represented in insecure and or disorganized internal working models of attachment.

For example, processes of idealization, which characterize both those with narcissistic pathology and those with dismissing attachment, involves not knowing aspects of one's internal and external psychological reality, and results in distortions of concepts of self and others in favor of illusory rather than actual aspects. Both avoidant infants and dismissing adults have been shown to suppress emotions in a number of studies (as indicated by their facial expressions, electrodermal responses, and self-reported feelings during the AAI (Sroufe & Waters, 1977; Dozier & Kobak, 1992). These findings suggest that those with avoidant–dismissing states of mind are more prone to defensive exclusion of negative thoughts and feelings, particularly in situations that activate the attachment system. In a similar fashion, Mikulincer, Shaver, Cassidy, and Berant (2009), in a series of laboratory-based investigations, found that those with avoidant–dismissing attachment tend to show self-enhancement and self-aggrandizement in the face of any experience that would activate the attachment system (physical or psychological threat, feelings of vulnerability, limitation), including lack of conscious awareness of negative aspects of the self and rigid adherence to fixed negative views of relationship partners that correspond to unwanted aspects of self with no conscious recognition of this fact. There is thus a tendency in those with dismissing attachment to rely on image-distorting defenses of projection and devaluation, as well as the tendency to divert attention away from the positive traits, intentions, and relational behaviors of others that causes them to miss signals of support and love—all of which are also typical of those with narcissistic disorders.

Understanding the propensity of those with avoidant–dismissing attachment representations to minimize affect, but to experience anger and distress at the visceral unconscious and/or nonverbal level evident

in their psychophysiological reactivity (accelerated heart rate or cortisol level; Sroufe & Waters, 1977; Dozier & Kobak, 1992), alerts the clinician to the subtler signs of distress in bodily gestures, expressions, or other nonverbal communications. Hence, attachment research provides evidence for the split in the representational world of the individual with dismissing attachment representations, such that a dominant model of an idealized self in relation to a devalued other coexists with a model of an unworthy self in relation to an inconsistently loving and abandoning other based on experiences of rejection or lack of care that is not consciously available or dissociated (Main & Goldwyn, 1998).

Bowlby's (1977, 1988) expectation that the contributions of attachment theory and research would provide a lens to amplify and sharpen our understanding of personality disorders, including NPD, by linking them to another theoretical frame of reference, is now solidly backed by empirical research. A number of studies have now affirmed that insecure and/or disorganized internal working models of attachment and deficits in reflective functioning (RF) underlie the difficulties with self-regulation and interpersonal functioning that make narcissistic personalities so challenging to treat (Fonagy et al., 2002; Levy et al., 2006; see Meyer & Pilkonis, 2011 for a review).

The dismissing detachment of narcissistically disturbed patients observed by expert clinicians (Bromberg, 1983; Kernberg, 2018) has been affirmed by research studies linking narcissistic pathology with dismissing–avoidant attachment in studies that have used both in-interview measures of attachment (e.g., the AAI; Rosenstein & Horowitz, 1996; Diamond, Levy, et al., 2014; Barone, 2003; Bakermans-Kranenburg & van IJzendoorn, 2009) and self-report attachment measures (Blatt & Levy, 2003; Westen, Nakash, Thomas, & Bradley, 2006; see Meyer & Pilkonis, 2011, for a review) in both clinical (Bender, Farber, & Geller, 2001; Barone, 2003; Levy et al., 2006) and nonclinical groups (Dickinson & Pincus, 2003; Smolewska & Dion, 2005; Otway & Vignoles, 2006). But one of the major contributions of attachment theory and research with individuals with personality disorders is the finding that they present with a range of insecure and disorganized attachment states of mind. Although dismissing states of mind are predominant in clinical observation of narcissistic patients, as well as research, individuals with narcissistic disorder have also been characterized by preoccupied (anxious) attachment in which the individual alternates between extreme positive and negative valuations of attachment figures with whom he or she remains angrily or passively enmeshed (Kealy, Ogrodniczuk, Joyce, Steinberg, & Piper, 2015), and in a minority of cases, by disorganized (cannot classify or unresolved), fearful, or even secure attachment status (Diamond, Levy, et al., 2014;

Meyer & Pilkonis, 2011). It should be noted that by contrast, individuals with BPD have been found to have primarily preoccupied and/or unresolved attachment status (Patrick, Hobson, Castle, Howard, & Maughan, 1994; Fonagy et al., 1996; Rosenstein & Horowitz, 1996; Diamond, Stovall-McClough, Clarkin, & Levy, 2003; Levy et al., 2006; Bakermans-Kranenburg & van IJzendoorn, 2009), although a few studies show that some are characterized by dismissing attachment status as well (Barone, 2003; Levy et al., 2006).

In keeping with the investigations of both clinicians and researchers, in those with NPD, grandiose self-states may co-occur and oscillate with vulnerable self-states (Horowitz, 2009; Kernberg, 2009, 2018; Ronningstam, 2018; Pincus et al., 2014). Other studies have examined the attachment correlates of such states. While those with grandiose presentation have been found to have dismissing attachment, those with vulnerable narcissism were high on attachment anxiety (Smolewska & Dion, 2005; Otway & Vignoles, 2006; Dickinson & Pincus, 2003). Fossati, Feeney, Pincus, Borroni, and Maffei (2015) found that adolescents assessed for pathological narcissism with the PNI (Pincus et al., 2009), which identifies both grandiose and vulnerable features, showed that high levels of pathological narcissism are characterized by both dismissing and preoccupied attachment. Based on these findings, Fossati et al. (2015) concluded that those with pathological narcissism "experience both intense anxious attachment needs and deep fears of trusting and depending on others" (p. 420). These findings affirm that the grandiosity of those with narcissistic pathology in fact may coexist with and defend against vulnerable self-states. Indeed, in one study of the self–other representations of those with different attachment styles, Kealy and colleagues (Kealy et al., 2015) found that "narcissistic grandiosity is associated with the representations of self as inadequate, fearful and sensitive to rejection" (p. 402).

In aggregate, these findings suggest that individuals with pathological narcissism and NPD have diverse and contradictory internal working models of attachment, both dismissing and preoccupied, that have been linked respectively to the mental states of grandiosity and vulnerability (Cain et al., 2008; Meyer & Pilkonis, 2011). In other words, a representation of self as superior in relation to a devalued or idealized other (grandiose mental state), most characteristic of thick-skinned, grandiose narcissistic individuals, may be linked to dismissing or avoidant attachment, while a representation of the self as unworthy, inferior, and hypersensitive to rejection in relation to superior, rejecting, or an abandoning other, characteristic of those with vulnerable or thin-skinned narcissistic presentation, has been linked to preoccupied attachment. Some individuals with pathological narcissism and NPD have been found to oscillate

between dismissing and preoccupied representational states with respect to attachment, which bespeaks disorganization in the attachment system in that they cannot rely on any consistent attachment strategy to manage anxieties and affects in relation to attachment figures, instead ricocheting among different attachment states of mind (Diamond, Levy, et al., 2014; Diamond, Clarkin, et al., 2014). The discovery that, in fact, narcissistic disorders across the spectrum of severity are linked to divergent, insecure, and/or disorganized states of mind with respect to attachment figures, has given empirical support to the complexity and multiplicity of the internal world of those with narcissistic disturbances.

The question of course is: What does attachment status tell us that is of clinical utility in therapy with patients with narcissistic disorder? Since individuals with personality disorders, including those with narcissistic pathology, are likely to have multiple, contradictory states of mind with respect to attachment, and since these are likely to change over the course of dynamic therapy, understanding the attachment representational states helps the working clinician to listen for different attachment states of mind as they emerge in the clinical situation. These include:

1. The constricted, idealizing state of mind of the dismissive patient who creates an arid wall shielding him- or herself from pain that cannot be experienced or articulated.
2. The confused and conflicted state of mind of the patient with a preoccupied attachment state of mind who gets lost or overwhelmed in inchoate anger at attachment figures, necessitating both containment and confrontation.
3. The fragmented state of mind of the unresolved patient, who may appear intact but might enter a frozen state or become fearful and dissociated as he or she experiences past loss or trauma in ways that compromise, even momentarily, his or her reflective and cognitive capacities (Lichtenberg, 2003).
4. The oscillating cannot classify state of mind in which the patient veers from dismissing to preoccupied tactics, moving in a confused fashion between emotional withdrawal and aloofness that minimizes expressions of attachment while maximizing expressions of attachment with inordinate demands for attention, reassurance, caregiving, and so forth.

In Chapter 9, we provide a clinical example of how these attachment states of mind are evident in a TFP session with a patient with combined narcissistic and borderline pathology.

Deficits in Mentalization in Pathological Narcissism

Narcissistic pathology has been associated not only with insecure and/or disorganized working models of attachment but also with deficits in mentalization that are thought to be at the core of insecure attachment (Fonagy et al., 2002). Fonagy and colleagues have suggested that mentalization is "intrinsically linked to the development of the self, to its gradually elaborated inner organization, and to its participation in . . . a network of human relationships with other beings who share this unique capacity" (p. 3). The rigid organization around a grandiose self makes it difficult for those with pathological narcissism to access mental states (feelings, beliefs, desires) beyond the grandiose state of mind, or to comprehend that the grandiose self is a mental state among other mental states that are part of the self. Similarly, it is often difficult for those with narcissistic pathology to understand another's state of mind separate from his or her own needs or projections. Many patients with personality disorders, including narcissistic disorder, show extreme sensitivity to anticipated and/or actual humiliation that causes them to misread the intentions of others, and to misread their own self-states, particularly those that are experienced as intolerable (Bateman & Fonagy, 2004; Fonagy, Luyten, Allison, & Campbell, 2017b).

Deficits in mentalization in those with pathological narcissism may be traced to the lack of congruent, marked mirroring in which the caregiver's emotional state, facial expressions, and gestures are contingently calibrated to not only accurately mirror but also symbolically represent (mark) those of the infant (Fonagy et al., 2002). In patterns of incongruent mirroring, ostensive cues, such as facial expressions, voice tone, or gestures, are either over- or undercontingently calibrated to the infant's states so that he or she fails to develop a second-order representation of such states as symbolic mental states. This interferes with the development of epistemic trust or the capacity to take in and rely on interpersonally transmitted knowledge or social learning not only from attachment figures but from the wider social and cultural groups beyond the family (Fonagy et al., 2017a, 2017b).

Given the accumulating evidence presented earlier that many narcissistic individuals have suffered experiences of parental misattunement, maltreatment, or abuse, a consideration of these points is in order. Epistemic mistrust may have been an adaption to a less than optimal environment for those with narcissistic pathology who have been observed to turn away from the caregiver in childhood, seeking the "narcissistic solution" (Symington, 1993) in response to disappointment or disillusionment with attachment figures. As Bateman and Fonagy (2012, 2016) point out, in such situations, the individual may become hyperaware of

the feelings, thoughts, and needs of others in order to ensure psychic survival. But the individual may use the capacity for mentalization in self-serving ways, either to control, seduce, or coerce others by anticipating their needs, or to avoid painful realities that do not converge with their distorted self concept. Bateman and Fonagy (2016) termed this process as *hyper-* or *overactive mentalization*. These formulations about hypermentalization add another dimension to understanding narcissistic individuals' often preternatural capacity to read the mind of the other, while remaining obtuse to their own mental states. Hypermentalization is evident in the tendency toward concrete and/or rigid thought processes that make the individual impervious to new information; in the tendency to engage in overly elaborated reasoning about mental states that reflect the individual's own wishes and needs more than actual experience or external reality; and in the tendency to construct complex psychological explanations for interpersonal events that contradict or exceed reasonable inferences gleaned from the observation of others' actions or behavior (Unruh & Drozek, 2020).

Deficits in mentalizing have also been linked with identity diffusion (De Meulemeester, Lowyck, Vermote, Verhaest, & Luyten, 2017). Improvements in the capacity for mentalization are thought to go hand in hand with the capacity to develop fuller and more elaborated representations of the self and internal objects (Fonagy, 1999; Fonagy & Campbell, 2015; Kernberg, 2004). As Fonagy has observed, "enhanced reflective capacity allows patients to integrate split off parts of the self and create representations with complex thoughts, mixed emotions and differentiated desires" (1999, p. 16). Thus, the concept of mentalization as developed by Fonagy and colleagues is complementary to the object relations foundations of TFP (Kernberg, 2004). From the perspective of TFP, the interpretation of the split, polarized, internal world of the borderline individual helps to consolidate an integrated sense of identity that in turn improves mentalization. Indeed, integration of mental representations, identity consolidation, and affect regulation are intertwined in a complex spiral both in treatment and in early development, the exact strands of which are not always so easily differentiated. As noted earlier, Bateman and Fonagy (2012, 2016) and Jurist (2018) have defined full mentalization as the integration of cognition and affect. Hence, one of the goals of treatment of those with pathological narcissism and NPD would be to reinstate mentalization in the full sense, perhaps building on the observed capacity for cognitive empathy (intellectual understanding of emotions) and even hypermentalization in some narcissistic individuals as a bridge to the capacity for emotional empathy or the capacity to identify and share the feelings of others (see discussion of empathy below).

The Reflective Functioning Scale

There are now a number of methods to assess mentalization, but in our research we used the Reflective Functioning Scale (RFS; Fonagy et al., 1998), designed to capture "the psychological processes underlying the capacity to mentalize" in the context of attachment relationships (Fonagy et al., 1998). Designed for use with the AAI, the RFS is an 11-point scale that assesses individual differences in the capacity to mentalize (Fonagy et al., 1998) along a number of dimensions, including the developmental trajectory of mental states and the extent to which they can change with maturation and new life circumstances, the extent to which mental states are relatively opaque and subject to limitations, the extent to which there are explicit efforts to tease out mental states underlying behavior, and the extent to which the individual is aware of mental states in relation to the therapist or interviewer. Those with personality disorders are typically characterized by a simplistic, naïve, and formulated view of mental states, most typical of those with BPD, or by hyperactive or overanalytic RF, most typical of those with NPD. In the latter, the individual claims infallible knowledge of mental states with little evidence to corroborate this conviction and have a lack of awareness of the separateness of the mind of self and others (Fonagy et al., 1998).

Research on Change in Attachment and Mentalization in Patients with Pathological Narcissism

If we think of attachment and mentalization as fundamental ways to understand the mental states and representations of individuals with personality disorders, we would expect them to change over the course of treatment. That attachment status shifts in the course of intensive psychodynamic therapy has been affirmed in a number of studies on change in attachment classification from insecure to secure, and from disorganized to organized after 1 year of psychodynamic treatment, including TFP (Buchheim et al., 2017; Diamond et al., 2003; Levy et al., 2006), and psychodynamic treatment (Gerber, 2005) of those with personality disorders. Several studies have indicated that the AAI is a useful measure to track shifts in representational states with respect to attachment that may occur in the course of therapy.[3] In addition, just

[3]That dismissing attachment is in part defensive is evident from one study (Gerber, 2005) that used the AAI to track the shifts in attachment representations over 18 months of treatment of 25 young adults with severe personality disorders sequentially assigned to psychoanalysis, or psychodynamic psychotherapy at the Anna Freud Centre in London. Gerber found in successful treatments that those who were

as deficits in mentalization are fundamental to severe personality disorders, several studies have shown that improvements in mentalization, as measured by the RFS, which assesses mentalization in attachment relationships, are a key mechanism of change in psychodynamic psychotherapy with such patients (Bateman & Fonagy, 2004; Fischer-Kern et al., 2015; Levy et al., 2006; Rudden, Milrod, Target, Ackerman, & Graf, 2006).

In our research at the PDI, we investigated attachment and mentalization in patients with co-occurring NPD/BPD, and how they change over the course of 1 year of TFP. The data from two randomized clinical trials (New York–Cornell RCT, Clarkin et al., 2007; Vienna–Munich RCT, Doering et al., 2010) with a sample of patients with BPD with and without comorbid NPD allowed for a more fine-grained exploration of the attachment representations of those with combined narcissistic and borderline pathology, how they might differ from patients with BPD without narcissistic pathology, and how they might change over the course of TFP. In this study, attachment was assessed, and for measures used, we refer the reader to previous publications (Diamond, Levy, et al., 2014; Diamond, Clarkin, et al., 2014; Levy et al., 2006). Here we offer a brief summary of this study and its clinical implications for TFP-N with narcissistic patients. As expected, we found that those with combined NPD/BPD and those with borderline pathology showed deficits in mentalization and insecure/disorganized attachment representations. Both groups were characterized by a simplistic, one-dimensional, naïve concept of mental states, or by hyperactive mentalization (e.g., overelaborated and unconvincing interpretation of motivations, feelings, and beliefs of self and others; an RFS score of ≤3).

However, the two groups showed somewhat divergent representational states with respect to attachment. Patients with combined NPD/BPD were significantly more likely to be classified as dismissing (characterized by idealization/devaluation of others and dismissal of need for closeness) or cannot classify (e.g., characterized by oscillation between angry preoccupation or passive enmeshment with attachment figures and dismissing devaluation of them) than was the BPD group. The BPD group was significantly more likely to be classified as preoccupied (angrily or passively enmeshed with attachment figures) or unresolved (breakdown of discourse and reasoning in response to questions about

---

classified as secure or dismissing on the AAI at admission showed a shift to preoccupied attachment representational states in the midphases of therapy, characterized by anger toward attachment figures, with another shift toward secure attachment states of mind at the end of treatment (which spanned 6 months to 8 years).

loss and trauma) than was the NPD/BPD group (Diamond, Clarkin, et al., 2014; Diamond, Levy, et al., 2014). Thus, both clinical groups showed evidence of a lack of integration in attachment representations, but in the NPD/BPD group this took the form of oscillation between opposing states of mind with respect to attachment relationships—most typically between dismissing and preoccupied discourse strategies throughout the interview. By contrast, in the BPD group there was a more severe but focal breakdown in the monitoring of reasoning and discourse in response to specific questions about loss and abuse, indicating the intrusion of "peculiar and even partially dissociated states of mind" (Hesse, 2008, p. 570) associated with traumatic experiences of loss or abuse in childhood.

Interestingly enough for the patients from the New York–Cornell RCT we found no differences between the patients with NPD/BPD and patients with BPD on retrospective self-reports of the frequency of childhood trauma, including emotional, sexual, and physical abuse, and neglect (Diamond et al., 2013). Yet, the individuals with BPD were significantly more likely to have a lack of resolution with respect to these experiences of loss or trauma than were those with combined NPD/BPD (Diamond, Levy, et al., 2014). These findings raise the question of whether the NPD/BPD group was *better defended* and/or *more reflective about* childhood loss and trauma. The comparable amounts of childhood trauma in the context of low mentalization (RF) is paired in those with NPD/BPD with dismissing or combined dismissing and preoccupied mechanisms (cannot classify). This suggests that the individuals with NPD/BPD may have the capacity to mobilize working, if rigid and primitive, defenses to contain and limit disorganization in the face of childhood trauma. Thus, our findings to date suggest that narcissistic pathology may have a stabilizing effect in the context of borderline organization. Also relevant to this point is that individuals with NPD/BPD were found to drop out significantly less often from all outpatient treatments in both studies; had fewer hospitalizations or inpatient days, as well as less unresolved childhood trauma on the AAI; and in the New York–Cornell study, less self-harming behavior (Diamond et al., 2013). In sum, our findings suggest that narcissistic pathology in the context of BPD may be a protective factor that helps narcissistic patients to stay in treatment, limit self-destructiveness, and contain disorganization in the face of early traumatic attachment experiences (Hörz-Sagstetter et al., 2018; Diamond et al., 2013; Simonsen & Simonsen, 2012).

These findings, along with the findings from the other studies discussed above, have important clinical implications in that they imply specific types of attachment representations and defensive use in

individuals with narcissistic pathology that have implications for aspects of the clinical process, including transference and countertransference. Although dismissing mechanisms may predominate in some patients in which the "linguistic focus is continuously *away* from past attachment relationships and their influences," in others these dismissing mechanisms may alternate with preoccupied strategies in which the focus is "persistently, although confusedly so, strongly oriented *toward* attachment relationships and experiences" (Hesse, 2008, p. 556). In any case, as Perry (2014) points out, the study is consistent with clinical observations that narcissistic personalities are characterized by detachment and failures in relating deeply and empathically to others, while those with borderline pathology without pathological narcissism are more likely to consistently show an angry and fearful preoccupation with attachment figures (Kernberg, 2014). Hence, the study confirms the importance of assessing attachment representations and the capacity for mentalization in individuals with personality disorders, including NPD, "opening the way for examining potential etiological factors leading to both the attachment style and the disorders in question" (Perry, 2014, p. 446).

Disturbances in Interpersonal Relating, Intimacy, and Empathy in Narcissistic Disorder

The distortions in the internal world of object relations in those with narcissistic pathology, including insecure/disorganized attachment representations, are mediating factors between early problematic experience and current difficulties in interpersonal functioning, particularly with intimacy and empathy. The capacity to value and cherish others for their unique qualities; to depend on others not as a source of narcissistic supplies but in a mature, reciprocal manner; to respond empathically to others separate from one's own needs and perspectives; and to invest in and sustain intimate love relationships over time, even in the face of loss or illness, is often truncated. The need of those with narcissistic pathology to cling to ideal images of self and others means they do not have access to the dependent aspects of self, and cannot tolerate dependency in others, which is seen as a sign of weakness. The tendency of those with narcissistic disorder to project such dreaded aspects of self onto others leads them to keep such denigrated others at a distance, but also paradoxically binds them to others who contain devalued aspects of self (Kernberg, 2018; Konrath, Bushman, & Grove, 2009). Conversely, those with narcissistic disorder may create a retinue of idealized others seen through the illusory halo of their own grandiosity. This process of

experiencing positive or idealized aspects of others as an aspect of the self serves to defend against the envy of others and may lead to unrealistic expectations of what others can provide. In fact, such processes of devaluation and idealization lead to compromised reality testing about self and relationships, even though the corporal or physical boundaries between self and others are well delineated in all but the most severe cases.

In those with narcissistic pathology in the higher range of functioning, enduring relationships may be maintained but they often are superficial and shallow, organized primarily to buttress self-esteem and protect the grandiose self (Kernberg, 1975, 2007, 2010; Morf & Rhodewalt, 1993, 2001). In lower-functioning individuals with pathological narcissism and NPD, there may be a near total defensive withdrawal from intimate relationships. In even more severe cases, there is a withdrawal from reality itself, alternating with chaotic relationships laden with aggression and conflict. Although individuals at the more severe end of the spectrum present as dependent on others, in fact the dependence is shorn of any mutuality or reciprocity—rather, it is based unilaterally on the need for the admiration and adoration of others, which may be an organizing relational force.

Recall James's (1881/2003) depiction in *The Portrait of a Lady* of the young American heiress, Isabel Archer, and her dawning recognition that her husband, Gilbert Osmond, lacked an authentic sense of self or identity separate from the admiration received from others. This lack severely limited his capacity for genuine engagement with her and others. Under normal conditions, individuals have an integrated concept of self relating to an integrated concept of significant others that contributes to a stable, coherent identity on the one hand and to object relations in depth on the other. James understood this in his description of Isabel Archer: "For herself, avowedly, the world had always interested her, and the study of her fellow creatures had been her constant passion" (p. 479). For those with pathological narcissism, a sense of unity is maintained through the appropriation of admiration from others to create a nucleus of an idealized self that eclipses realistic mental representations of self and others, providing an illusion of "a grand indifference, an exquisite independence. . . . *But indifference was really the last of his qualities; she had never seen anyone who thought so much of others. . . . He was unable to live without it [society], and she saw that he had never really done so; he had looked at it out of his window even when he appeared to be most detached from it"* (emphasis added; p. 479). Early on, Isabel Archer recognizes that her husband's narcissistic equilibrium demands that she who "had been so free of step" had to accept his "conscious

calculated attitude" as her own, that "the real offense as she ultimately perceived, was having a mind of her own at all. Her mind was to be his—attached to his own like a small garden plot to a deer-park . . ." (pp. 480–481).

James's (1881/2003) depiction highlights aspects of interpersonal functioning in those with pathological narcissism: the conscious attitude of haughty indifference to others combined with insatiable and even slavish cravings for admiration from them; the proclivity to induce suffering and distress in others by denying them separateness (Ronningstam, 2009; Kernberg, 2007, 2018); and the tendency to retreat to fantasies of unique greatness often realized through annexing the self to others deemed superior. These aspects do not prevent those with pathological narcissism from experiencing states of suffering themselves. Indeed, as clinicians we observe the excruciating states of isolation, loneliness, emptiness, humiliation, rejection, and shame experienced by narcissistically disturbed patients, but these are often difficult for the individual to articulate or even acknowledge for fear of exposure.

A number of research investigations on the interpersonal functioning of individuals with pronounced narcissistic traits, as well as full narcissistic disorder, have revealed core dysfunctions in interpersonal relating. A full discussion of these findings is beyond the scope of this chapter, but they include (1) denigration of others for not meeting entitled expectations; (2) social withdrawal and detachment from others; (3) devaluation of others who threaten superiority; (4) callous, hostile, domineering, and intrusive behavior toward others who are exploited to protect self-esteem; (5) arrogant dismissal of others in the face of shame and humiliation; and (6) hypersensitivity to criticism and social rejection (Campbell & Baumeister, 2006; Cain et al., 2008; Dimaggio et al., 2002; Dickinson & Pincus, 2003; Konrath et al., 2009; Kealy & Ogrodniczuk, 2011).

Interestingly, these studies do not encompass the profile of interpersonal functioning of the individual with vulnerable presentation. Rather than being oblivious to internal suffering, those with thin-skinned or vulnerable narcissistic pathology often present as shy and self-effacing, and needy and distressed, showing a tendency to repeatedly incur disappointment and humiliation, and even to sanctify misery. In such individuals, as Cooper (1989) points out, there is a wish to continually recreate an object tie to a powerful idealized internalized parental figure experienced as rejecting and/or cruel. These formulations suggest how important it is to go beyond the surface of interpersonal behavior and presentation of those with narcissistic disorder to explore the contradictory, multiple representations of self and others, both conscious and

unconscious, and both grandiose and vulnerable, that characterize the disorder.

The maladaptive interpersonal patterns have been linked empirically not only with insecure and disorganized internal working models of attachment but also with specific distortions in self and object representations. Those with narcissistic pathology tend to show a lower level of object relations development characterized by lack of self–other differentiation and inner impoverishment, which leads to intense reactivity to separation and attachment anxiety. In narcissistic patients, these reactions may be linked to fear of loss of narcissistic supplies or sustaining a merger with idealized others (Kealy et al., 2015). Interestingly, these object relations features were found to characterize those with both grandiose and vulnerable narcissism, indicating the defensive nature of grandiosity.

Psychoanalytic theorists have postulated that pathological narcissism is associated with impairments in self and other representations at the intrapsychic level (Blatt, 2008; Kernberg, 1975, 2007). The representational characteristics of those with narcissistic pathology, including the condensation between ideal and real aspects of self and other, and the inability to differentiate between them (Kernberg, 1975, 1984, 2018), were found in one study to have a correspondence at the perceptual level. Through a unique facial-morphing task that uses facial stimuli to assess self–other differentiation or the ability to distinguish between self and others at the perceptual level, individuals with pathological narcissism, and vulnerable narcissism in particular, were found to be more likely to rate others' faces as *more* like oneself or to show more "sensitivity" to perceive the self in others' faces (Karan, Diamond, Grinband, & Fertuck, 2019). However, they did not have more difficulty in discriminating themselves versus others on the facial morphs. Thus, in individuals with pathological narcissism, reality testing remains largely intact, but there are milder or transient impairments in distinguishing between self and other. This likely reflects confusion between aspects of the self and other at the intrapsychic or representational level that contribute to the difficulties with self and interpersonal functioning observed in individuals with pathological narcissism. This lack of self–other differentiation may contribute to deficits in empathy and impairments in facial emotional recognition, one of the primary ways in which affect is communicated to and read by others. This also accounts for the frequently noted transference disposition of narcissistic patients to perceive the therapist as a mirror image or more similar to the self than they are (Bradley, Heim, & Westen, 2005; Kohut, 1971). Hence, this study suggests a perceptual base for the combination of self-preoccupation and

indifference to others, and hypersensitivity to social exclusion and seeking of mirroring/merger experiences noted in individuals with pathological narcissism.

Also, unique to those with vulnerable narcissism is a self-representation of malignant self-regard (Huprich, Lengu, & Evich, 2016) associated with social inhibition, submission to others, and excessive self-sacrifice. Such a negative self-concept affects aspects of both self-functioning in that it involves problems in the assertion of healthy narcissism, and interpersonal functioning in that overaccommodation to the needs of others limits mutuality in intimate relationships. Interestingly, individuals with vulnerable narcissism and malignant self-regard tended to respond with anger and anxiety to negative feedback, but to attack the self rather than the other (Huprich et al., 2016).[4]

Additional support for an object relations conceptualization that underlying narcissistic disorders are distortions in the internal world of self and object representations can be found in cognitive-behavioral literature that has identified the maladaptive cognitive schemas of self in interactions with others associated with aspects of pathological narcissism (Beck et al., 2004; Zeigler-Hill, Myers, & Clark, 2010; Morf & Rhodewalt, 2001; Young & Flanagan, 1998; Zeigler-Hill et al., 2011). More specifically, the internal world of those high on grandiose narcissistic traits appears to be characterized by entitlement schemas involving the belief that one should be able to do or have whatever one wants regardless of the consequences for self and others (Zeigler-Hill et al., 2010), as well as schemas identified as "self-aggrandizer" and "bully and attack" (Lobbestael et al., 2008). Narcissistic traits and schemas characterized by a sense of vulnerability to harm, emotional inhibition, and insufficient self-control have also been found in those whose presentation is characterized by the vulnerable presentation of the disorder (Zeigler-Hill et al., 2010).

Along the same lines, Besser and Priel (2010) found that those individuals who were rated with high levels of grandiose narcissism were particularly emotionally reactive in the face of achievement failures, while those with high levels of vulnerable narcissism were more likely to feel threatened by interpersonal rejection. Further investigations on social rejection and rejection sensitivity in those with narcissistic pathology were conducted by Twenge and Campbell (2003) in a series of laboratory-based studies. These involved simulations of social rejection and monitoring of the affective and behavioral reactions. In

[4]This finding corresponds to the idea that narcissism involves the investment of aggression as well as positive libidinal feelings in the self, and that these feelings may be turned inward.

these studies, those high in narcissism (grandiose) were more likely to report feeling angry and aggressive, rather than feeling shame and distress, than those whose self-report was low on narcissism. In addition, those high on narcissism tended to behave with more direct aggression toward others than did the non-narcissistic individuals (controls) after being subjected to experiences of social rejection *in vivo* (Twenge & Campbell, 2003).

However, such study findings must be taken with caution since they are based on self-report ratings of narcissistic traits, both adaptive and maladaptive, that tend to deal with consciously held and reported attitudes and behaviors. Other studies, such as that by Cascio and colleagues (2015) cited earlier, reveal contradictory reactions to social exclusion. While those with narcissistic features do not report distress on self-report measures after experiences of rejection, they do show high reactivity in neural networks seen on fMRI typically activated in situations of social pain. These findings suggest that narcissistic pathology is complex and contradictory in that conscious attitudes of indifference to others and isolating self-sufficiency may mask hypersensitivity to social exclusion and criticism. In fact, the latter may fuel the detachment observed in narcissistic personalities from others, and a concern with power and social dominance that may be as much self-protective as self-aggrandizing.

Taken together, these findings suggest that narcissistic aloofness and detachment may indeed mask a profound need for and fear of social connection and acceptance. This suggests a split in the internal world between a consciously held self-sufficient and autonomous self that spurns connection with devalued others, and a needy, dependent self that craves connection but fears rejection and the emotional and physical dysregulation that it evokes. The latter self–object dyad operates outside of conscious awareness. Again, an object relations perspective that considers such splits between opposing dyads that defend against each other helps us to understand the complex dynamics of those with narcissistic disorder and offers the possibility of a comprehensive treatment that recognizes both the longing and the aggression tied up with this bifurcated representational world.

Empathy and Pathological Narcissism

Deficits in empathy, or the "natural capacity to share, understand and respond with care to the affective states of others" (Decety, 2012, loc. 87), while still being able to maintain a clear differentiation between them, contribute to the problems in interpersonal functioning in those with narcissistic pathology. These deficits include difficulties in sharing the emotions of others in a differentiated way, as well as deficits

in the social-cognitive understanding of the intentional mental states (thoughts, feelings, motivations) underlying their actions.

A lack of empathy has historically been identified as a major characteristic of individuals with narcissistic pathology based on clinical observation and expert consensus (Kernberg, 1975; Kohut, 1966; Akhtar & Thomson, 1982; Millon, 1996). In fact, deficits in empathy have figured as a criterion in the diagnosis of NPD from its appearance in DSM-III in 1980. Its association to NPD has been backed up by considerable research (Watson, Grisham, Trotter, & Biderman, 1984; Watson & Morris, 1991), although some studies show that lack of empathy is inconsistently associated with NPD and is characteristic of other personality disorders as well (Morey, 1985; Gunderson & Ronningstam, 2001). The exploitation of others to gratify self-interest and the instrumental use of others as sources of admiration to preserve a positive, superior sense of self preclude empathic resonance with the feelings and needs of depreciated others. However, those with narcissistic disorder have also been observed to present as socially adept, charming, and seemingly engaged with others, if somewhat facile. They are often quite skilled at imagining the thoughts and feelings of others, although this capacity may be used to garner recognition, attention, and appreciation from others, including the therapist, and may dissipate if this is not forthcoming. For example, one patient with pathological narcissism was most attentive to a business associate and friend, whose favor she had long cultivated, and with whom she shared many professional interests and contacts. In fact, her friend often told her that no one had been more insightful or helped her as much in understanding the minds of her coworkers and strategizing about her role in the company. However, when her friend became wrapped up in the excitement of preparing for her son's wedding, she was not able to maintain their frequent discussions or follow through on some shared projects. Subsequently, the patient scheduled a business trip at the same time as the wedding and showed little awareness of why this might have created a breach in the relationship, or of the envy that motivated her actions.

Thus, the relationship between empathy and narcissism is complex and somewhat paradoxical. On the one hand, individuals with narcissistic pathology may be indifferent or unempathic to the needs and feelings of others and may be dismissive or even contemptuous toward others when they express their concerns or problems. On the other hand, they are inordinately dependent on others for admiration and affirmation, and hence, are often particularly gifted in imagining the mental states of others, although they may sometimes use this capacity to exploit and manipulate others (Baskin-Sommers, Krusemark, & Ronningstam, 2014; Gunderson & Ronningstam, 2001; Ritter et al., 2011).

The deficits in empathy in individuals with narcissistic disorder often contribute to the dismantling of relationships, stemming from the need to control the other to function as, or contain, the split-off negative self-representation. Hence, empathy can extend only as far as the needed arrangement. The individual with narcissistic pathology cannot tolerate being empathic to the other and will turn away if (1) the individual fails to fit the projection, because that interferes with the imperative to see the self in certain ways; or (2) the empathic attention to the other takes away from the narcissistic needs of the self. For example, a high-functioning academic who was the head of his department and renowned in his field entered therapy at age 40 after his wife asked for a divorce. He had long been much admired and revered by his students and junior colleagues, whom he mentored in a dedicated fashion. In fact, he was known to generously promote the careers of his mentees and also to offer emotional and even financial assistance when they had professional or personal problems. However, when his wife miscarried in the second trimester of pregnancy after trying to conceive for over 10 years, he behaved callously, not returning from an international conference to be by her side and showing little tolerance for her grief, instead insisting that they attempt to try for another child as soon as possible. This heartless attitude contrasted with the anger and distress about the miscarriage that he had difficulty expressing in therapy, instead focusing on what his wife might have done to prevent the miscarriage. In this situation, his wife failed to act in accord with his own need to see himself as invulnerable and successful in all endeavors, including becoming a parent. Furthermore, her overwhelming grief and vulnerability made him all too acutely aware of his own, and he had to distance from her for fear that he would also be overwhelmed and incapacitated. In sum, clinical experience with individuals with narcissistic disorder reveals that empathic responsiveness may vary according to the person, situation, and/or motivational factors (see Baskin-Sommers et al., 2014).

The variability of narcissistic individuals' empathic capacities across the spectrum of functioning suggests that significant dynamic factors may be at work involving the activation of defenses and/or distorted or maladaptive mental representations of self and others. Dynamic conflicts around dependency, autonomy and separateness, intrusiveness, social exclusion, and social avoidance—all aspects of the problems in interpersonal functioning characteristic of those with pathological narcissism—may be activated in situations where empathy is elicited (Kealy & Ogrodniczuk, 2014). Because they are inordinately dependent on the admiration of others to regulate self-esteem, those with narcissistic pathology are likely to be hyperattuned to the mental states of others from whom they seek affirmation, but at the same time may be

oblivious to the emotional reality and needs of others because of their relentless self-focus and grandiosity.

Research on Narcissism and Empathy

There is now a wealth of research from multiple levels of analysis, including social cognition and clinical neuropsychology, that enhance our understanding of the variable and somewhat paradoxical relationship to empathy in those with narcissistic pathology. A full presentation and discussion of these studies is beyond the scope of this text, but we highlight here the most important findings and their clinical relevance for understanding and treating those with narcissistic disorder. It should be noted that by social cognition we are referring to the mental processes that underlie social interactions, such as the capacity to recognize and understand the emotions, intentions, and attitudes of others by reading social cues.

Recent laboratory studies in social cognition and empathy have identified a multidimensional model of empathy, including:

1. *Emotional empathy.* The sharing of affect states with others that involves not only resonance with the basic emotions and sensations of others (pleasure, fear, lust, pain, and anger, as well as the more complex emotions of guilt, shame, and love) but also the capacity to imagine these feelings of others, to re-create them internally in the self, and to distinguish them from feelings originating in the self (Singer, 2006; Decety & Jackson, 2004).
2. *Cognitive empathy.* The capacity to understand and reflect on the thoughts, intentions, desires, and feelings of self and others that overlaps but is not synonymous with the concepts of theory of mind or mentalization (Frith & Frith, 2003).
3. *Motor empathy.* The imitation and mirroring of others at the level of motor action that begins in earliest infancy, and forms the basis for the so-called action–perception model of empathy (Gallese, 2003; Preston & de Waal, 2002).

Empathy thus involves the capacity to develop shared representations of the mental states of others—that is, to resonate affectively with what others feel, and also the mental flexibility to reflect on those mental states and to see them as separate from one's own, while still being able to take the perspective of the other (Baron-Cohen & Wheelwright, 2004; Decety & Jackson, 2004; Decety & Lamm, 2006).

Individuals with pathological narcissism have been found to show impairments in emotional empathy involving affective resonance with or

responsivity to others, while their capacity for cognitive empathy, or the capacity to understand and imagine the emotions of others, remained relatively intact (Ritter et al., 2011). Compared to individuals with BPD and healthy controls, those with NPD showed an impaired capacity for mature empathic concern and shared experience of the emotions of others, but they maintained a capacity for cognitive empathy or the ability to infer the goals and intensions of others and to take their perspective.[5]

Interestingly, those individuals rated as high on narcissistic pathology showed little awareness of their lack of emotional empathy. Instead, consistent with the tendency toward self-aggrandizement and an inflated, unrealistic sense of their own capacities, those with NPD assessed themselves on self-report measures of empathy (e.g., the Interpersonal Reactivity Index [IRI]; Davis, 1983) to have intact emotional empathy. It should be noted that the failure to detect differences in self-reported empathy between those with narcissistic disorder and normal controls was also found in other studies (Marissen et al., 2012; Schulze et al., 2013). While those with NPD show little awareness of their lack of emotional empathy, they do acknowledge some deficits in their motivation to exercise their capacity for cognitive empathy as indicated by their negative responses to questions such as "When I'm upset with others I usually try to put myself in their shoes for a while" (Ritter et al., 2011).

These studies suggest a motivational dimension to deficits in empathy for those with narcissistic disorders, in that individuals may be motivated in some circumstances to exercise their capacity for cognitive empathy intuiting the mental states of others in order to garner the admiration and adulation that they rely on to enhance or bolster self-esteem. By contrast, in situations of greater intimacy when a deeper level of affective resonance with others and a sharing of mental states at the affective as well as cognitive level is called for, they may shut down their empathic capacities without much awareness of the defensive nature of their actions. It should be noted that individuals with pathological narcissism and NPD show deficits in affective empathy and motivational deficits in cognitive empathy, while those with comorbid NPD/BPD and BPD show deficits in both cognitive and affective dimensions of empathy (Ritter et al., 2011), a finding that affirms the importance of assessing narcissistic disorder dimensionally. Deficits in cognitive empathy noted in those with comorbid NPD/BPD overlap with other findings on deficits

[5]It should be noted that this study used not only self-reports of empathy, which are unreliable because narcissistic individuals tend to present the self with socially desirable traits like empathy, but also measures based on perception or behavior (e.g., the Multifaceted Empathy Test (MET; Dziobek et al., 2008) or the Movie for the Assessment of Social Cognition (MASC; Dziobek et al., 2006).

in mentalization for the more severely disturbed narcissistic patients (e.g., those with NPD/BPD; see Fonagy et al., 1996, 2002; Diamond, Levy, et al., 2014).

In fact, a discrepancy between two types of empathy may be a hallmark of those with pathological narcissism, albeit one that often is confusing to the clinician. Those with narcissistic disorder often quite accurately hypothesize about the motivations, thoughts, and feelings of others, but are not able to emotionally resonate with others, particularly around experiences of loss, failure, defeat, sadness, and death, experiences that involve the recognition of limitations. For example, a patient who lost her daughter under tragic circumstances could comprehend and discuss with the therapist what the loss meant to her and her husband, and could talk cogently and empathically about what the loss meant for her life given a history of parental illness and death in her childhood. However, after the death of their child, she distanced from her husband and became increasingly estranged from him. Although she told the therapist that he was the only person who could comprehend the depth of her feelings of loss, she was unable to mourn with him for fear of losing her sense of narcissistic equilibrium that depended on a sense of invulnerability that was incompatible with sharing her grief.

The above example suggests that there may be differences in empathic responsiveness between more vulnerably presenting narcissistic individuals and those who are more grandiose. While the individual may be unempathic, and unmotivated to mentalize in a grandiose state of mind, he or she may feel overly identified with the distress of others while in a vulnerable state. In fact, one recent study (Luchner & Tantleff-Dunn, 2016) showed that deficits in both cognitive and affective empathy (as measured on the IRI; Davis, 1983) were associated with vulnerable narcissism (as assessed on the PNI; Pincus et al., 2009), while deficits in emotional empathy, but not cognitive empathy, characterized those with grandiose narcissism. These findings should be considered in the light of a recent meta-analytic review that showed that both grandiose narcissism and vulnerable narcissism are negatively associated with both self-reported cognitive and affective empathy (Urbonaviciute & Hepper, 2020).

Facial Recognition, Empathy, and Narcissistic Disorder

The limits in empathy in individuals with pathological narcissism with both vulnerable and grandiose presentation may be linked to difficulties in the capacity to identify and read emotions from facial expressions. The recognition of facial emotions is among the most important mechanisms by which one understands and intuits others' internal states and is

thought to be one of the correlates and roots of empathy (Besel & Yuille, 2010). Although individuals with pathological narcissism and NPD rate themselves as sensitive to the feelings of others, in fact they show an impaired capacity to recognize the basic emotions of fear, anger, disgust, happiness, sadness, and neutral expressions compared with healthy controls (Marissen et al., 2012).[6] Individuals with antisocial personality and psychopathy also show impairment in recognition of fear (Blair et al., 2004), sadness (Dolan & Fullam, 2006), and disgust (Kosson, Suchy, Mayer, & Libby, 2002). Interestingly, De Panfilis and colleagues (2019) found that individuals with narcissistic features (as assessed on the Structured Clinical Interview for DSM-IV Personality Disorders [SCID–II]; First, Gibbon, Spitzer, Williams, & Benjamin, 1997) could more accurately and quickly recognize subtle signs of negative emotions at low intensity, but not at full intensity, in facial expressions than did those without narcissistic features, and they were also more likely to recognize neutral expressions (De Panfilis et al., 2019). These findings suggest greater sensitivity to nascent signs of disapproval or rejection, but not full rejection sensitivity in those high on narcissistic features. Thus, those with narcissistic pathology may more readily than non-narcissistic individuals recognize neutral and low-intensity negative facial expressions despite their seeming indifference to others.

The foregoing findings are consistent with the idea that in the face of the happiness of others, or in response to even subtle signs of rejection or lack of social approval by them, those with pathological narcissism or NPD or prominent narcissistic features might experience negative emotions of anger, anxiety, and envy, and/or heightened arousal at the neurophysiological level. These findings provide evidence for the particular ways in which those with narcissistic pathology process social information and provide data for understanding the underlying view of self and others that might fuel impairments in empathy and interpersonal functioning generally. It may be that such heightened affects and arousal might occur in situations that pose a threat or challenge to the dominant dyad of the superior grandiose self–devalued other dyad, which defends against an opposing dyad of an unworthy or vulnerable self in relation to a superior, rejecting, or indifferent other. The coexistence of detachment from others in the context of extreme reactivity to even subtle signs of interpersonal threat or rejection observed in those with pathological

[6]While Marissen and colleagues (2012) interpret their findings as evidence for deficits in the cognitive dimension of empathy, it is likely that they also speak to the lack of automatic empathic responsivity that originates in recognition and imitation of facial expressions between mother and caregiver in infancy (see Beebe & Lachmann, 2013; Fonagy et al., 2002).

narcissism are explained by these contradictory, unintegrated object relational dyads that may exist in opposition to each other.

In sum, for those with narcissistic pathology, impairments in emotional empathy in the context of relatively intact cognitive empathy may be understood as a type of dissociation between two types of empathy, and two extreme contradictory object relational dyads. Those with narcissistic pathology are often exquisitely sensitive to the reactions and valuations of others, leading in certain circumstances to the capacity to accurately read others' emotions and the intentional mental states behind them in order to protect the self and/or garner narcissistic nutriment or ward off rejection or disapproval, a hypothesis backed up by the finding that narcissistic exploitativeness has been associated with emotional reading capacities in those high on narcissistic traits (Konrath, Corneille, Bushman, & Luminet, 2014). However, the capacity to develop shared representations of the feelings of others and to experience internally and identify with what others feel (affective empathy) while remaining differentiated from them, is impaired. Nonetheless, in one study (Hepper, Hart, & Sedikides, 2014) the capacity for perspective taking was found to temper the negative links between pathological narcissism and self-reported empathy and autonomic (heart rate) arousal. This suggests that strengthening the capacity for cognitive empathy in treatment may be a bridge to improving not only understanding but sharing the experience of complex emotions of others in those with NPD.

Empathy and Neurological Correlates of NPD

Recent investigations have established a possible neurobiological substrate to the deficits in empathy in those with narcissistic disorder. For example, the deficits in the capacity to recognize and respond empathically to the facial expressions of others may have correlates at the neurobiological level. In a brain imaging study of individuals with pathological narcissism and NPD, adolescents with conduct disorders, and nonclinical controls, Schulze and colleagues (2013) found reduced grey matter volume in the left anterior insula in patients with NPD, the right anterior cingulate (trend), and the rostral and medial cingulate cortex, as well as the dorsolateral and medial regions of the prefrontal cortex—all areas of the brain implicated in the neural circuitry associated with empathy and emotional regulation (Fan et al., 2011). In addition, there was a marginal association between grey matter volume and emotional empathy (as measured by the self-report IRI; Davis, 1983; German version, Paulus, 2009) regardless of diagnostic group. The latter finding suggests that adolescents with conduct disorders, as well as those with narcissistic traits who do not meet full criteria for NPD, may also show

neurobiological markers related to narcissistic disorder. This finding strengthens the association between those with antisocial personality or psychopathy and NPD.

The foregoing findings were confirmed by Nenadic, Smesny, Schlosser, Sauer, and Gaser (2010), who in comparing the neural anatomy and circuitry in those with NPD and nonclinical controls, similarly found the deficits in grey matter in the prefrontal areas (although not the insula), as well as alterations in white matter in the prefrontal area.[7] Impairments in these regions are thought to affect not only empathic processing but also perhaps provide a neurobiological substrate for the self-referential processing bias seen in narcissistic disorder (Schulze et al., 2013). In a study by Chester and colleagues (2016), those high on narcissistic traits (as assessed on the NPI) showed impairments in the structural connectivity between brain regions that process self-relevant stimuli (i.e., the medial prefontal cortex) and reward (i.e., the ventral striatum) compared to those who did not show such traits. More specifically, those with grandiose narcissistic traits were found to have reduced white matter integrity and less frontostriatal connectivity between brain regions that are related to self-esteem and reward. Chester and colleagues proposed that their findings of deficits in the frontostriatal pathways connecting self and reward point toward the idea that there is more of a discrepancy between baseline and wished-for reward in those who score high on grandiose narcissism (a finding that hearkens back to the findings on sensitivity to reward stimuli in children identified with narcissistic traits). Furthermore, they surmise that the exhibitionism and grandiosity in those with narcissistic traits may be a "regulatory strategy to compensate for their internal deficit in self-reward connectivity" (Chester et al., 2016, p. 1036). It should be noted that some of these studies were conducted with normal subjects with marked narcissistic traits (e.g., Fan et al., 2011), as well as with those with a full DSM-IV/DSM-5 NPD diagnosis (e.g., Chester et al., 2016; Schulze et al., 2013). The similarity in the results, including neurostructural deficits (left and right anterior insula) and lack of integrity in neural pathways, suggests that both groups show impairments in the neurobiological structures and processes related to emotional empathy, self-esteem, and affect regulation. This suggests continuity at least at the neurological level between those with a full NPD diagnosis and those with narcissistic traits.

---

[7]White matter is the brain tissue that conducts and coordinates messages between different areas of the brain, whereas grey matter is the tissue that is associated with processing and cognition. While grey matter is made up primarily of cell bodies with few myelinated axons, white matter is made up primary of long-range myelinated axon tracts with few cell bodies (Kolb & Whishaw, 2003).

The above studies are consistent with previous research with both clinical and nonclinical groups that have established links between neural structure and functioning in insular brain regions and empathy (Decety, 2012), and in prefrontal regions (dorsolateral prefrontal and anterior cingulate cortices) and emotion regulation (Ochsner & Gross, 2005; Northoff et al., 2006). For example, in a magnetic resonance imaging (MRI) study, nonclinical subjects with marked narcissistic personality traits (as assessed by the NPI) who scored high on narcissism showed abnormal functioning in the right anterior insula when they were asked to empathize with others based on responses to pictures depicting various emotions (Fan et al., 2011). In the studies reported above, the abnormalities in grey and white matter structure and functioning may begin to trace the imprint at the neural level of both the impaired emotion regulation and empathic processes in those with narcissistic disorder (traits and diagnoses), particularly under conditions of threat, as well as the structural differences in the way the experience of self is structured or represented. These studies, although rudimentary, have begun to forge links between mind and brain in narcissistic disorder (Ronningstam, 2016), and provide evidence at the neural level for NPD as a disorder of self and interpersonal functioning, self-esteem, and affect dysregulation.

Ethical Values and Reality Testing

The object relational configuration of the grandiose self along with deficits in interpersonal relating, empathy, and self- and affect regulation may also involve distortions of the individual's ethical functioning. While those who function at a higher level generally adhere to a set of ethical standards that reflects an individualized value system in line with a universally agreed-upon moral code, they may also show variability in moral standards from rigid moral superiority and perfectionism to moral compromises. Persons with more severe narcissistic pathology often show significant ethical lapses and even contemptuous disregard for a just value system. The condensation of the real self with the ideal self and *ideal other representations* leads to the sequestering of idealized images in the sense of self, rather than their integration into a set of ethical values and standards (e.g., the superego) that make up the ideals to which one can realistically aspire as a humane and compassionate individual. This merging of the ideal and real aspects of self results in a version of superiority and perfection that repeatedly runs into conflict with reality. The chronic overestimation of the self in turn leads to transient warps in social reality testing, which may be glaring or subtle, but which often also may lead to social blunders at best and transgressive behaviors of dishonesty or disregarding rules at worst. Individuals with narcissistic

disorder often overvalue themselves and/or perceive themselves as more valued in the eyes of others than is the case, as well as misread social cues and situations because of lack of empathy with others and lack of understanding of the intentional states that underlie behavior.

In those with narcissistic pathology, the realm of the ego ideal, the repository of goals, ambitions, fantasies of achievement, and perfection, fails to be adequately differentiated from the real accomplishments of the self. In such situations, the individual does not tolerate a gap between the ego ideal and the ego, or the ideal self and the real self. Hence, the self incorporates everything good, not leaving the ideal self or other as structures external to the ego. Because ideal aspects of self and others have been highjacked to the needs of the grandiose self where they exist in unmodulated form, they fail to contribute to the development of a consistent depersonified and realistic set of guiding moral values. Such values function to guide and structure behavior, bringing them into alignment with both social reality and the needs and aspirations of the self. Driven by grandiosity, perfectionism, and/or a sense of underlying unworthiness, those with narcissistic pathology make unrealistic demands on the self that are not always consonant with their real abilities and capacities. In such situations, there are no protective checks and balances on self and interpersonal functioning separate from the needs and demands of an inflated self-concept that includes the idealized images of self and others that form the basis for the ego ideal. In addition, since there is less integration of early harsh superego forerunners (personified and exaggerated parental prohibitions) with what Schafer (1960) called the "loving and beloved" aspects of the superego (the legacy of the ego ideal), ethical values may be unduly rigid or overly lax.

In sum, in those with narcissistic disorder, the persecutory, forbidding, punitive aspects of the superego tend to be experienced as internal demands for perfection and projected in a way that interferes with the development of an integrated superego. That integration would combine positive (protective) and negative (prohibitive) aspects, and a personal set of moral values that reflect both realistic prohibitions and personal ideals and goals. The lack of protective aspects in the superego puts the narcissistic individual at risk for the influx of aggression, particularly in situations of early abuse or maltreatment, leading to the development of self-punishing, perfectionistic behaviors and/or antisocial or psychopathic behaviors. The substantial degree of covariance between NPD and ASPD (Gunderson & Ronningstam, 2001; Gunderson, Ronningstam, & Smith, 1991; Hart & Hare, 1998; Widiger & Corbitt, 1993) provide some support for the deficits in moral values deriving from specific structural aspects of those with narcissistic pathology, as discussed above. On the other hand, other studies show

discriminant validity between NPD and ASPD. In one study, in those with NPD, six out of nine criteria differentiated them from those with ASPD (Gunderson & Ronningstam, 2001). This is not surprising since individuals with pathological narcissism and NPD may be characterized by none to mild to moderate antisocial features, ranging from overly lax moral standards to subtle indications of rule breaking to overt criminal behavior. In those with higher-functioning NPD, there may be considerable conflict about antisocial activities, with individuals acknowledging that their need for self-aggrandizement overrides their judgment and sense of morality, but little understanding of why this would be the case. Ronningstam (2010) points out that whereas in those with antisocial personality proper there are often conscious recurrent criminal activities, in those with pathological narcissism exploitative and transgressive behaviors may be unconsciously motivated. Often, such behaviors devolve from the quest for power, status, admiration, and/or an underlying need for reassurance, which is the vulnerable underside of the grandiose self that is often split off or disavowed. This accounts for the lack of awareness on the part of those with pathological narcissism and NPD of the motivations behind the potentially risky or transgressive aspects of their behaviors.

Individuals with pathological narcissism often do not recognize the problematic nature of their behaviors, which are often so ego-syntonic that they show little guilt or motivation to change. For example, a high-functioning 35-year-old research physician who was very creative and productive in his work life was living with one woman in a long-term relationship while carrying on an affair with another woman, a resident in his lab (which was against the regulations of his department). He supported both women financially and neither knew about the other. His long-term girlfriend was pressuring him to get married and adopt a child because she was unable to conceive. She had begun to plan a wedding and had initiated adoption procedures, and even though he was unclear about the depth of his commitment to her, he had given his consent to proceed with these plans. While he recognized that both women would be devastated if they knew the real situation, he justified his behavior on the grounds that they both needed him emotionally and financially, and that they would be destitute should he leave them, downplaying the destructive impact of his chronic deception and the potentially problematic consequences for his career, social standing, and friendships should his duplicity be discovered. He professed to care deeply about both women and had convinced himself that the financial and emotional support he provided to both justified the arrangement. Also, he could not tolerate the excruciating sense of loss he felt when he contemplated ending one or both relationships, which revealed an

underlying sense of vulnerability and fear of abandonment. In this case, his self-inflated views obscured the socially challenging and questionable behaviors, fueled his sense that he was indispensible to both, and justified his chronic deception. It was only after exploring how his grandiose view of his own self-importance blinded him to the full extent of the exploitative nature of his relationships with both women that he began to experience guilt and remorse about his behavior and to work toward changing the arrangement.

Summary and Conclusion

In sum, in Chapters 2 and 3 we have drawn on findings from neurocognitive science and attachment research and theory to develop a multifaceted, dynamic view of the self and interpersonal processes that contribute to pathological narcissism and NPD. Central to our object relations model is the idea that those with narcissistic pathology are characterized by distortions in the representational world characterized by a hypothesized self-structure, the pathological grandiose self, in which there is identification between the self and idealized objects, internalized to avoid any feelings of dependency, need, or weakness. The latter negative aspects of self-experience are systematically and rigidly projected onto others, leaving the self inflated but impoverished of sustaining, valued realistic representations of self and others, and thus dependent on others for self-regulation in the form of admiration or approval. When such needs are not met, the narcissistic individual may react with antagonism or emotional withdrawal, which causes others to retreat and withdraw positive regard or submit slavishly to the other's need for approval, but in either situation, there is a breakdown of mutuality and authenticity in interpersonal relating. Not surprisingly, with such a self-structure, the narcissistic individual is also prone to vulnerable states of experiencing the self as depleted, impoverished, and inferior in relation to others seen as ideal and superior in the light of the projected grandiosity. This core self-structure provides an explanatory mechanism for the somewhat contradictory aspects of self and interpersonal functioning in those with narcissistic disorder.

Viewing narcissistic grandiosity and vulnerability that characterize the grandiose self through the lens of attachment theory helps to illuminate the personality dysfunction and interpersonal impairments seen in those with narcissistic disorders. Individuals with pathological narcissism have been found to have multiple insecure attachment representations, both dismissing and preoccupied, and/or show disorganization in the attachment system that leads them to oscillate between these

two attachment strategies in interpersonal functioning. This means that a person with pathological narcissism may alternate between a supreme sense of self-sufficiency and dismissing devaluation of attachment relationships and feelings, and anxious compulsive seeking of reassurance or mirroring from others. Although grandiose narcissism, which involves a representation of self as superior, self-sufficient, and invincible in relation to devalued or idealized others, primarily has been linked to dismissing avoidant attachment, both grandiose and vulnerable narcissism have also been linked to anxious, preoccupied attachment with representations of the self as inadequate, fearful of abandonment, and/or angrily or passively enmeshed with others who are experienced as inconsistently available (Diamond, Levy, et al., 2014; Kealy et al., 2015; Meyer & Pilkonis, 2011). These findings support our object relations model that sees pathological narcissism in dynamic terms as involving fluctuations between grandiose and vulnerable states of mind and between dismissing detachment and fearful preoccupation with others, in different admixtures depending on the individual. These insecure attachment patterns have also been associated with deficits in the capacity to reflect on one's mental states, including the often intense or painful affects that infuse them (Diamond, Levy, et al., 2014; Diamond, Clarkin, et al., 2014). Dismissing individuals who minimize affect, may experience anger and distress in relation to attachment figures at the visceral unconscious and/or nonverbal level, while for those with anxious/preoccupied attachment affects may be heightened to elicit reassurance and stave off abandonment fears. In either case, lack of epistemic trust that is a sequalae of insecure attachment limits the capacity not only to bond to, but also to learn from, attachment figures who are the conduit for the mores of a larger social/community network, perhaps leading to deficits in social-cognitive processes, such as mentalization and facial recognition. Such deficits in social-cognitive processes have also been linked to some of the other contradictory aspects of interpersonal functioning in individuals with pathological narcissism and NPD, including:

- The uncanny ability to detect subtle signs of disapproval or negative emotions in the facial expressions of others combined with a seeming indifference to others.
- The lack of emotional empathy or the inability to share the emotions of others while still distinguishing between the feelings of self and other combined with an intact capacity for cognitive empathy, or the ability to infer the perspectives and mental states, with the latter subject to motivational deficits.

- The overadaptation to aspects of social reality, evident in good surface social functioning, combined with striking deficits in reading social cues and particularly facial expressions.
- The fluctuation between overly lax and unduly rigid ethical functioning.

All of these contradictory and perplexing features of narcissistic pathology, which have long vexed clinicians who treat those with pathological narcissism, may exist in different admixtures and at different levels of severity, depending on the individual patient. In these initial chapters, we attempted to illustrate how research findings on pathological narcissism and NPD in clinical groups, as well as narcissistic personality traits in the general nonclinical population, may amplify our clinical investigations into a complex, dynamic understanding of the self and the interpersonal processes that characterize the disorder. Our model of treatment follows organically from this contemporary object relations model that encompasses attachment and social cognitive findings since it is designed to work with all aspects of the representational world, both dismissing/grandiose and anxious/preoccupied/vulnerable, that we encounter with narcissistic patients.

In the following chapters, we present the model of assessment and therapy for TFP-N that considers the complexity and variable presentation of narcissistic disorder at different levels of organization.

CHAPTER 4

An Overview of TFP-N

In this chapter, we present our model of TFP for persons with pathological narcissism and NPD (TFP-N), a psychodynamic object relations approach that has been empirically validated for patients with borderline pathology (BPD), including many patients with pathological narcissism and NPD as well. Based on contemporary object relations theory and informed by neurocognitive and attachment research, as outlined in Chapter 2, our object relations model has been tailored to identify and address the totality of the special challenges that these patients bring to treatment. These include high rates of dropout (over 60%; Hilsenroth et al., 1998); fluctuating mental states of grandiosity and vulnerability, and arrogance and depletion; seamless or striking external presentation that belies the internal suffering and painful, distressing, or even fearsome internal experiences; dread of dependency and fear of exposure that limits access to internal states; and the tendency to react aggressively or to collapse in the face of criticism or threat to grandiose illusions about the self, and rejection of the realities that challenge these beliefs. These features may characterize narcissistic persons with different admixtures and levels of severity depending on the level of personality organization.

TFP-N combines elements of standard psychodynamic technique—particularly, attention to unconscious processes and motivations; free association and the therapist's evenly hovering attention, interpretation, and analysis of the transference in the here and now; and use of the therapist's countertransference to understand aspects of the patient's internal experience—with a higher level of therapist activity and focus on

external experience than is usually found in psychodynamic therapies. The primary goal of TFP is to reduce symptomatology, self-destructive and maladaptive behaviors, and internal distress and impairment that usually characterizes those with personality disorders, but which may not be so evident in those with pathological narcissism. It takes place within a set of mutually agreed-upon behavioral parameters designed to limit maladaptive behaviors and promote the unfolding of the patient's full emotional experience in the treatment setting. The treatment contract functions as an ongoing agreement between patient and therapist and establishes the framework in which exploratory dynamic therapy can best proceed (see Chapter 6 for a fuller discussion of the contract-setting process).

The focus of the therapy is primarily on identifying and modulating the maladaptive representations of self and others, and the extreme affects linked to them as they are experienced and enacted in the relationship with the therapist. From an object relations perspective, it is the underlying distorted mental representations in those with personality disorders that fuel the internal distress; chronic conflict; broken, turbulent, or unsatisfying relationships; and alterations in reality testing that characterize those with pathological narcissism and NPD. These representations constitute the lens through which an individual interprets what he or she is experiencing. Internal objects are hypothetical constructions made by the therapist for the patient to anchor the relational patterns being evoked in the therapeutic situation. These maladaptive psychological structures are identified and understood as they unfold in the transference, or in the relationship with the therapist as it is experienced by the patient. Transference mobilizes mental representations of self, object (other), and a linking affect as they are actualized in verbal and nonverbal interactions with the therapist. Although the internal representations of self in relation to significant others may reproduce a specific relationship from the past, they may also represent an internal construction, the ultimate origin of which cannot be identified precisely. Therefore, in TFP-N, our focus is current psychic reality, particularly on the dominant object relation that is alive in the moment, how it is structured, and how it can be modified.

It should be noted that internal objects are enduring organizations that are evident only in their derivatives—that is, how they define the contours of self-experience and experience of others. We focus on the transference as a way of understanding how the patient's internal reality is enacted in his or her relationships with others, including the therapist. The experienced transference most often involves the repetition of internalized relationship patterns and underlying object relations (Freud,

1914/1953; Kernberg, 2018) and/or internal working models of attachment linked to them. As disparate aspects of the representational world are actualized in the transference, they may be explored, understood, and integrated. The combination of understanding in the context of immediate affective experience in the transference relationship with the therapist leads to the modulation and integration of the extreme, distorted representations. Primitive, split self-representations are transformed into more mature, integrated object relations that enable the individual to more adaptively tolerate negative affects (anger, envy, anxiety). This in turns helps the person to develop and sustain more meaningful engagements in interpersonal relationships and work. The movement toward more integrated representations of self and other goes hand in hand with an improved capacity to accurately understand the behavior of self and other in terms of intentional mental states—in current terminology, to "mentalize."

Also related to the improvement in mentalization is the fact that TFP-N, like classical TFP for BPD, aims to foster increased attachment security. As described in Chapter 3, representations of self in relation to primary attachment figures are inevitably activated in relation to the therapist (Parish & Eagle, 2003). In the case of individuals with narcissistic pathology, attachment representations are likely to be insecure (dismissing or preoccupied) or disorganized (particularly cannot classify with oscillations between dismissing and preoccupied) in nature (Diamond, Levy, et al., 2014; Diamond et al., 2013; Meyer & Pilkonis, 2011; Main, 1999). TFP, with its focus on the patient's moment-to-moment experience with the therapist in the here and now of the transference relationship, activates the attachment system—that is, the expectations for help or care from attachment figures in a situation of separation, loss, or threat, along with the capacity to reflect on the caregiver's motivations, thoughts, and feelings. In TFP we focus on the gradual building up, modulation, and integration of object relations linked to disparate behavioral and motivational systems as they emerge in the clinical material and are manifest in the transference to the therapist. Significant change in attachment representations from insecure to secure and disorganized to organized, as well as improvement in mentalization (reflective function) in patients with BPD with narcissistic pathology have now been found in three case studies (see Chapter 1 for a summary). Attachment representations, however, are only one set of the panoply of self–object dyads that comprise the internal world of our patients. Beyond infancy they are of course colored by representational states stemming from other motivational systems (libidinal/erotic, aggressive/fight-flight, play, affiliation, etc.; Panksepp & Biven, 2012).

It is this substrate of self–object dyads deriving from different motivational systems we track as they unfold in the treatment process of TFP-N. The internal array of these self–object dyads—which ones are more prominent, affectively charged, and which are defended against—shapes both the structure (integrated, flexible vs. split, rigid) and outward expression of the individual's personality as revealed to us in the transference relationship. The hypothesized mechanism of change in TFP is the reactivation of the panoply of internalized object relations and associated affects that have been split off or repressed in the safe and controlled therapeutic setting where they can be cognitively contained and explored. By interpretation we mean identifying and exploring dominant representations of self and others, and the defenses that sustain them that contribute to repetitive problematic behaviors in the patient's life *as they are activated in the here and now of the relationship with the therapist*. It is only after these maladaptive object relational dyads and habitual problematic modes of relating to which they give rise and have been repeatedly experienced and interpreted in the arena of the transference that we offer hypotheses about their origins in the unconscious conflicts, motivations, and anxieties arising from early conflictual relationships and experiences (Caligor, Diamond, Yeomans, & Kernberg, 2009). As they are evoked in the relationship with the therapist, patients learn to identify, tolerate, modify, and integrate their disparate positively and negatively toned object relational dyads through the therapist's ability to contain, reflect upon, and interpret these states.

Through recognizing and reflecting on such repetitions and distortions as they arise in the patient's experience of the therapist, the therapist and patient come to constitute a third position (Kernberg, 2018; Ogden, 1994, 2004), defined as an intersubjective field consisting of the patient's dominant object relational dyads as they are alternately enacted and projected in relation to the therapist, and their reverberations in the therapist's own internal world. By recognizing and reflecting on repetitive relational patterns as they emerge in the transference and understanding their origins in maladaptive mental representations, the patient comes to experience relationships, including the relationship with the therapist, differently. The goal is to replace the patient's polarized (highly idealized or devalued), unrealistic mental representations of self and others linked to ideal, and in more extreme cases, punitive internal objects, with more flexible, mature, realistic mental representations that are integrated over the course of treatment into a coherent, and stable, if multifaceted, sense of identity. This in turn enables the individual to reflect on and regulate intense affects, particularly negative affects, and to "put momentary

affect arousal . . . into a more benign integration of emotion, thought and behavior" (Clarkin, Cain, & Lenzenweger, 2018, p. 80).

In sum, in TFP-N we strive to develop integrated representations of self and others, modify primitive defenses, and resolve identity diffusion through the experience and interpretation of the transference. The major goals of TFP-N are to foster not only symptom change but also enduring change in personality structure and organization. For many narcissistic individuals this may be reflected in the movement from:

- Shallow and exploitative relationships that serve largely to amplify self-assurance to more emotionally sustaining and mutually gratifying friendships and intimate relationships.
- Work lives organized around superficial markers of success or punishing, perfectionistic standards that the individual can never meet to deeper and more satisfying engagement in work and creative pursuits that reflect interests and commitments not bound to narcissistic self-aggrandizement.
- Transient infatuations with others experienced as idealized but shadowy and superficial to the capacity to enduring love relations with others, seen as separate from the self and valued in their own right.
- A value system based on personal gain to one that transcends their own existence and needs.
- The rejection of alternative realities that do not affirm the grandiose self to the experience of more vibrant and authentic modes of being and relating to others that ensue once the individual is freed from the maladaptive repetitive relational cycles that ensue from distorted idealized and devalued object relations.

The nature of narcissistic and borderline pathology creates a pressure that necessitates projective processes emanating from our patients. Although we only have anecdotal evidence from our ongoing supervision groups (which many TFP therapists at the PDI participate in for both research and clinical purposes) to support the following, we assume therapists trained in TFP are significantly freer from such pressures. This is because TFP therapists are strongly encouraged to participate in their own personal therapy and have theoretical knowledge, clinical experience, and ongoing supervisory processes as a consistent and routine part of their practice. These help us to monitor our personal influence on the treatment frame and to be better able to observe the effects of the same. Thus, we see a decided asymmetry in the contributions to the canvas of the treatment process, one that we as therapists, most of the time, are in a more solid position to observe and diagnose.

Stages of Therapy

As with any psychodynamic therapy, it is difficult to delineate stages of therapy precisely, especially since each therapist assimilates the broad strokes of TFP-N techniques to his or her own style, and to the unique challenges presented by particular patients. Nevertheless, TFP-N can roughly be thought of as progressing from the pretreatment assessment and contracting stage, which is the most clearly delineated, to the early, middle, late, and termination stages.

Pretreatment Assessment and Contracting Stage

Before engaging the patient in the therapy per se, we first do an assessment through the structural interview (Kernberg, 1984), as described more fully in Chapter 5. In accordance with our object relations approach, the heart of the structural interview involves asking patients to describe themselves and significant others in depth in order to obtain a sense of the extent to which their self and other concepts are integrated, coherent, multifaceted, and complex, versus one-dimensional, superficial, polarized, or vague. To this end, the structural interview combines elements of a standard clinical assessment using diagnostic criteria for the dimensional/categorical hybrid DSM-5 described earlier, with more open-ended questions designed to explore the patient's history, current symptoms, relational patterns, understanding of his or her problems, and what he or she expects from therapy. We also discuss with the patient and, if appropriate, the family, our impression of the individual's problems, define the goals and objectives for therapy, and set in place a structure that creates a frame for the therapy, which is tailored to the individual patient and his or her difficulties. It is important to keep in mind that while explicit discussion of the BPD diagnosis is endorsed in all of the empirically validated treatments for that disorder, there is not at present a similar consensus about sharing with patients the clinician's impression about narcissistic pathology because of some of the negative connotations of the diagnosis. It may be useful for the clinician to use laymen's language about personality disorder symptoms, and to describe elements of personality organization more generally, including (1) the severe gap between what the individual aspires to and what he or she has accomplished; (2) oscillations between feelings of superiority and/or inferiority, and how these shape self-experience and interpersonal functioning; and (3) fears of dependency and humiliation that limit genuine connection to others and aspects of the internal world. That said, there may be situations when patients come to clinicians already familiar with and wondering about narcissistic traits, or even having already

diagnosed themselves as narcissistic personalities—in these cases, it may be useful to speak openly about what is currently understood about pathological narcissism and the NPD diagnosis and the limits of the current DSM-5 nosology. Furthermore, clarification that narcissistic disorder often involves a range of external presentations that may be discrepant with the person's internal experience is also in order. In sum, setting up the therapy generally involves two to three assessment sessions (as described in Chapter 5) and two to three, or possibly more, contracting sessions (as described in Chapter 6).

The treatment contract defines the perimeters of the therapeutic relationship by clarifying the patient's and the therapist's roles and responsibilities, and sets guidelines for how to deal with self-destructive and destructive behaviors. In this regard, the contract challenges an oft-noted tendency for narcissistic patients to imagine either that they can do therapy on their own with the therapist as a sounding board (in those with grandiose narcissism), or that the therapist can omnisciently understand and solve their difficulties (in those with vulnerable narcissism). The therapeutic framework serves both practical and psychodynamic functions in that it brings to light almost immediately aspects of the patient's internal world, particularly the dominant grandiose and devalued concepts of self and other and how these might shape the response to limitations and boundaries.

With higher-functioning narcissistic patients, who maintain smooth and effective social functioning, the major difficulties may revolve around speaking freely during therapy since their communications often function to manage the therapist's impression of them and to conceal rather than reveal internal distress and conflict. The therapist and patient should agree on the broad outlines of the contract, setting up the minimum necessary requirements for effective treatment with the idea that they will work toward a resolution of areas that the patient may find intolerable (e.g., following up with a medical evaluation for past substance abuse to determine whether there is any cognitive impairment, which may be humiliating for the patient to tolerate at the beginning of therapy). However, with more severely disturbed narcissistic patients, especially those with antisocial features, it is essential to set a firm contract from the beginning of treatment to forestall destructive or self-destructive acting out and to address the problem of dishonest communication. This is because with more severely disturbed narcissistic patients, self-destructiveness may function as a way of maintaining their superiority and as a vehicle to triumph over the therapist. The contract is also important in situations where patients' behaviors or attitudes have posed threats to their occupational or social status, which they deny (e.g., tweeting inappropriately, using office computer for Internet

liaisons or to view pornography). Certain requirements of the contract, such as the necessity for a certain degree of productive activity, challenge the "secondary gain" or lifestyle of depending on others for support and maintenance, rather than realistically testing their own capabilities in the world.

Regardless of level of disturbance, the contract requires the delineation of self-destructive behaviors that are often not as glaring or obvious as is the case with patients with BPD, and/or are not clearly either recognized or disclosed by patients in the beginning of treatment. This is because reliance on image-distorting defenses on the part of those with narcissistic pathology may in fact obfuscate subtler forms of acting out, such as bending rules at work, financial irresponsibility, reckless driving, chronic dishonesty, or other ego-syntonic behaviors. These behaviors or traits may have caused difficulties in the patient's life, but he or she may deny their severity and not share them with the therapist in the initial stage of treatment. As a result of the narcissistic individual's intolerance for imperfection, a particularly important aspect of the contract-setting process is enlisting the patient's active participation in setting treatment goals, which enhances their sense of agency and control. In emphasizing the need for patient commitment and responsibility in the joint effort of treatment, the contract can function as a secure base in the treatment from which exploratory work that involves reflection on problematic aspects of the patient's life can be launched.

The Early Stage of Therapy

Therapy per se begins when the therapist and patient have arrived at a good enough consensus about the conditions of therapy. At that point, the interaction shifts from a focus on the assessment and contracting process to the process of exploring the patient's internal world and external reality through free association; that is, the individual is asked to report what goes on in his or her mind—the flow of thoughts, feelings, dreams, memories—as they occur to him or her regardless of how trivial or significant, shameful or self-enhancing. With narcissistic patients, early challenges to the frame often are evident not in acting out but in distortions in the process of free association. They will mobilize defenses to protect against anxieties about being seen (fears of humiliation, exposure of underlying vulnerabilities) or truly seeing the other, which evokes feelings of envy (Steiner, 2011; Kernberg, 2014). Denial of the therapist's skills and devaluation of the relationship and competitiveness with the therapist, long stints of intellectual self-analysis (as if the therapist was not in the room), and the tendency to idealize the therapist, who is used as an audience, mirror, or sounding board, are typical. Higher-functioning

narcissistic individuals may present eloquent, even scintillating, narratives that appear to give a colorful, textured picture of their lives. Over time it may become clear that these narratives skim the surface of their experience in ways that are designed to conceal as much as to reveal and to manage the therapist's impressions of and responses to them. It is important to remember that such challenges to the treatment frame are generally motivated by anxieties and other intense affects involved in the exploratory process. Exploration of an individual's defenses inevitably provokes anxiety since these mechanisms, however maladaptive, have been the patient's principle way of coping with internal or interpersonal conflicts or split-off and disowned aspects of the self.

We pay particular attention in the early phase of treatment to nonverbal and countertransference channels since they communicate more about what is split off from the patient's conscious experience and not communicated verbally. Although in our object relations model we focus on the psychological structures that organize the experience of self and others, and self and interpersonal functioning, we focus initially on how the narcissistic patient interacts with the therapist and others, with particular attention to the expressions of the grandiose self in the person's way of interacting and relating. Although we focus on the manifestations of grandiosity as a core trait of narcissistic patients, we do not necessarily confront grandiosity per se in the early stages. Rather, we focus on identifying the shifts between self-aggrandizing and self-denigrating mental states, the specific contexts in which they are activated, and exploring their impact on the individual's subjective experience.

In contrast to borderline patients, who characteristically experience rapid shifts in their experience of the moment based on different internal relational dyads being activated one after another, narcissistic patients tend to remain rigidly stuck in the "grandiose–devalued" dyad. In this initial stage, the major task is to identify which aspects of the self are tenaciously held to and which must be forcefully disavowed and relentlessly projected to sustain the narcissistic structure, and then to contain the anxieties evoked through tactful confrontation with and exploration of these unacceptable parts of the self as they emerge fleetingly in the session. In any case, we avoid moving too quickly to foster integration. As Feldman (2007) observes, the therapist must rely on skill, tact, and sensitivity to determine the extent and tenacity of the splitting and projective mechanisms that sustain such bifurcation among grandiose and devalued aspects of the self, since premature interpretations of disowned parts of the self may cause a retrenchment of such defensive processes.

The question is often raised of how we engage narcissistic patients who often present a seamless veneer, despite their having sought

treatment. Initially, we promote patients' active engagement in identifying problematic behaviors and attitudes, areas of dissatisfaction or distress. We do this by making the setting of personal goals and treatment goals an essential aspect of the treatment contact process, which engages the patient in developing a shared focus for the therapy. This gives narcissistic patients a sense of agency and control that protects against the fear of dependency and humiliation that fuels the grandiose insularity of patients with pathological narcissism. Defining goals that are realistic and attainable as opposed to grandiose and unfeasible is often a protracted process with narcissistic patients and may be defined as a problem to be addressed (Ronningstam, 2018). We encourage the patient to speak freely, particularly about the problem areas as defined in the initial assessment and contract-setting phases. Often these problematic issues are more productively explored as they emerge in the description of extratransferential relationships since transference work is more difficult for those with pathological narcissism. We do this before we begin to interpret how these problem areas are related to the narcissistic organization per se. This is because the process of recognizing relational patterns and identifying affects, and the object relational dyads with which they are linked, simply takes more time and requires more patience in those with narcissistic pathology.

Such patterns may be identified initially through an exploration of the costs and consequences of grandiosity on the individual's relationships and work life. Identifying how the patterns of self-promotion, self-aggrandizement, and/or emotional detachment may be self-defeating, as well as self-enhancing, is important in the early stages. For example, an architect spoke off the record to a reporter who guaranteed him anonymity about his firm's designs for a major and long-anticipated memorial monument that involved complex, confidential negotiations between public and private funders. He felt puzzled about why he had agreed to have this conversation in which he divulged confidential information about the monument that thus far had only been shared with the planning department of the city whose responsibility it was to communicate with the press. The architect feared that he would be exposed even though the reporter had guaranteed his anonymity. When the therapist explored with him his reasons for doing this, he said that being courted by the press made him feel like a celebrity and that his egotism overcame his judgment. Further exploration of his motivations led to his acknowledging his need to feel empowered and special, although he felt self-disgust about his breach of trust in the aftermath. His distress about this event provided an opening to raise his curiosity and reflectiveness about the costs and consequences of indulging his momentary need for approval or affirmation. We see here how clarification and confrontation

can be used to foster reflection on perplexing or ambiguous behaviors or attitudes.

Thus, we first identify problematic, contradictory aspects of the experience of self and others that are characteristic of those with narcissistic pathology. In the case of the grandiose thick-skinned narcissistic individual, this would involve the contradiction between the seeming disinterest in others and the emotional overreaction to criticism or rejection. In the case of the vulnerable or thin-skinned individual, this would involve the tendency to avoid social interactions to preserve a hidden sense of specialness and out of fear of rejection or exposure, while at the same time seeking reassurance from others. Identifying and confronting the individual about such contradictory aspects, if well done in a timely and tactful manner, is often the first step to raising the person's curiosity about the underlying conflicts and motivations behind the narcissistic organization. It is important to remember that individuals with pathological narcissism are subject to sometimes surprising affect dysregulation evident in sudden withdrawal, flashes of anger, somatic reactions (e.g., bodily discomfort or pain), or other indications of vulnerability when self-regulatory mechanisms falter in the face of ego threats, failures, or other challenges to their narcissistic equilibrium. Because the latter are often evident only fleetingly in the early stages of treatment, we pay particular attention to the realms of action and nonverbal communication with the goal of identifying the split-off vulnerable aspects of the self, including negative feelings of hostility, fear, envy, or shame that the narcissistic structure defends against. With narcissistic patients in particular, it is important for the therapist to empathize with all sides of the patient's self-states and to keep his or her own mind in motion to create a reflective space or "analytic third" through which to understand and put into words dissociated aspects of the patient's subjective experience.

As the therapist tracks what is affectively dominant in sessions, including expanding his or her understanding of it through clarification and confrontation, the salient self and object dyads that shape the patient's experience in the transference and extratransferential relationships come into focus. It is only by clarifying what is happening in the session that the therapist learns what aspects of the patient's internal world (e.g., representations and associated feelings) can be tolerated as his or her own and which the patient needs to disown and project. In this early stage, the therapist interprets in the projection—that is, the therapist clarifies the patient's experience in the moment, with the goal of identifying the intolerable aspects of the patient's internal world as they are projected onto the therapist or others, without necessarily linking

them to the patient's own representational world, history, conflicts, or defenses.

As the therapist explores with the patient the implications of his or her attitudes, actions, and the underlying conflictual motivations, as in the example given above, the therapist implicitly provides evidence of genuine interest in and curiosity about the patient. This begins to activate the patient's attachment system that most typically is insecure or disorganized and makes it difficult for the individual to express vulnerability, disclose emotions, or talk about distressing experiences. Consequently, in the initial stages the therapist should recognize and expand on any attempts to establish an emotional connection or share internal states.

The Middle Stage of Therapy

In the middle stage of TFP-N, the therapist repeatedly calls attention to the relationship being activated in the transference and how the same relational pattern tends to get activated with role reversals. The patient will alternately cast the therapist in the role of the self or the object of a dominant object relation while enacting (often without awareness) the opposite pole (Kernberg et al., 2008; Yeomans et al., 2015). Examples would be the dismissing patient who is uncomfortable with dependency and closeness yet complains that others, including the therapist, are not sufficiently attentive; or the vulnerable patient who holds a conscious identification with a self-representation as shamed by critical judgments of the therapist while insisting that the therapist is inadequate to help him or her, thereby enacting the role of critical judge that he or she had previously attributed to the therapist.

In this process of identifying role reversals, the therapist must show what Sandler (1976) refers to as "role responsiveness": This involves the therapist accepting the experience of being cast into a role from the patient's internal world, while maintaining the capacity to reflect on this experience internally and/or with the patient. Repetition and reflection on such role reversals, which alter the positions from which patient and therapist interact in the session, begin to break down the patient's dismissing and insular stance, enlarging the individual's awareness of split-off aspects of the self of which he or she has been unaware. For example, a self-sufficient, perfectionistic patient who casts the therapist into the role of the incompetent treater, may become increasingly aware that he or she at times feels worthless and incompetent in relation to the therapist, who is perceived as superior and skilled. This stage may be protracted in individuals with pathological narcissism because it is often difficult for them to consciously recognize their identification with both

the self and object poles of a predominant grandiose–devalued scenario. Repeated activation of these dyads through role reversals in the transference expands the awareness of aspects of the self beyond the narcissistic organization, tempering grandiosity and integrating split-off vulnerability. However, the narcissistic person may continue to defensively cast the therapist as a player in one side of the grandiose–devalued scenario, even while his or her awareness of self-experiences beyond this scenario is emerging.

To move beyond such prescribed roles, the therapist optimally uses his or her experience to explore and reflect with the patient on the relational scenario being actualized in the moment, with the aim of raising the patient's curiosity about its functions and the limits it imposes. For example, a patient with an exaggerated sense of self-importance may begin to recognize that the wholesale devaluation of the therapist stems from an underlying sense of inadequacy and fears of humiliation, and to become aware of how this limits his or her engagement in therapy. Working with role reversals that engage the patient in reflecting on shifting experiences and views of the therapist and the self is an essential step in the dissolution of the grandiose self. Identification of role reversals promotes the patient's awareness of the complex and contradictory array of representations of self and other that the grandiose self defends against; that is, the unsettling lack of unity is at the base of the patient's feeling both ideal and superior on the one hand, and inferior and without value on the other hand. Nonetheless, the patient's shame proneness in the face of perceived inadequacies and/or self-critical, perfectionistic attitudes may prolong his or her dismissing or devaluing stance, sometimes preventing the therapist from becoming aware of when changes begin to occur.

As the individual experiences the various aspects of his or her mental representations of self and others in the transference, alternately enacting and projecting onto the therapist the self and object aspects of the dominant object relational dyad, the insularity of the grandiose self begins to break down. Working with role reversals by definition expands the relational sphere by drawing attention to how processes of introjection (taking everything good into the self) and projection (ridding the self of intolerable aspects by attributing them to others) sustain the grandiose self, thereby catalyzing reflection on its component aspects. As the defensive functions of the grandiose self are identified and observed *in vivo*, detachment is also diminished, often indicated in the capacity to experience others, including the therapist, with greater depth and dimensionality. A richer and more complex array of transferences develop, reflecting aspects of the representational world beyond

the configuration of idealized and devalued representations, signaling the dissolution of the grandiose self. The livelier unconstrained use of language and the development of shared metaphors between patient and therapist promotes spontaneity and playfulness that emerge in the sessions. At this stage, the capacity to depend on the therapist emerges, although this may take an instrumental form. This is because dependent wishes and longings may have been denied and truncated by what Symington (1993) calls seeking the "narcissistic solution" of turning away from the caregiver and instead relying on the grandiose self with its sustaining mélange of idealized representations of self and others.

In the middle stage of therapy, the individual begins to experience dependent wishes and longings for a gratifying relationship with a caregiver that previously evoked intolerable feelings of vulnerability, resulting from disappointing experiences with unresponsive or inadequate caregivers. The emergence of a positively toned idealized dyad may appear early in therapy in the form of the therapist as imagined "magician," who is equal to the patient in his or her perfection. This transference is usually short-lived since the therapist usually does not meet the expectation of a quick fix. Later on in the therapy, evidence of a positively valenced dyad of gratified self and caring other generally emerges gradually. At this stage, the therapist must assess the degree to which such a positive dyad is in fact idealized in nature or represents a realistic appreciation of the therapist's emotional investment in and capacity to help the patient. The advanced middle stage often sees an alternation between an expectation of perfect care and a realistic appreciation of what the therapist has to offer in the way of genuine concern and understanding. In sum, in the middle stage of therapy the patient increasingly experiences the therapist as a secure base to explore difficult aspects of both the internal and external worlds—however, genuine emotional engagement with the therapist may activate intense feelings of love and hatred, and longing and sadness, leading at times to defensive retreat to the grandiose self.

The Late Stage of Therapy

The late stage of treatment includes interpretation of the underlying conflicts and anxieties that catalyzed the formation of the grandiose self-structure and that at times continue to make it difficult for the patient to move beyond it. These principally include a reluctance to give up belief in the ideal object and difficulty in experiencing the remorse that accompanies awareness of negative aspects of the self that the patient has avoided by projection. The individual must work through painful

feelings, including guilt over damage done to significant others and sadness about recognition of limitations and losses. Reliance on narcissistic illusions about the self tied to archaic idealized representations of others are gradually relinquished, which allows the patient to be able to appreciate and accept the more complex and ultimately more satisfying intrapsychic and interpersonal reality. This is often evident in the capacity to invest and take pleasure in seeking the ideal in the arts, music, creative, or spiritual pursuits that are not bound to self-enhancement. There are also losses inherent in relinquishing narcissistic illusions of omnipotence and perfection, but this is balanced by a deepening of the capacity for relatedness to others, including the therapist, now seen as complex and valued, if imperfect, along with a more realistic assessment of and tolerance for his or her positive and negative qualities. This stage may in fact be quite protracted for narcissistic patients because the insularity of the grandiose self has protected them from the experiences of need, desire, loss, or mourning for the other, plunging them into realities they have eschewed in the retreat to grandiosity. A shift from defenses based on splitting and projection to higher-level defenses opens the patient up to new realities, including an expanded range of feelings, thoughts, hopes, and satisfactions—that although exhilarating, may also be foreign to the patient and at times difficult to tolerate.

As the patient achieves greater integration of disparate split-off and projected aspects of the self, he or she becomes able to recognize and reflect on polarized and contradictory feelings as mental states, rather than invariant, dreaded realities. At this stage, the therapist's role shifts from clarification, confrontation, and interpretation to witnessing the patient's newfound capacity to reflect on prior choices, validating that the situations and emotions the patient experiences are indeed complex. They may have no ideal solution and may require the patient to decide what is the best, if not ideal, way for him or her to resolve major life conflicts and choices. This is particularly evident when the patient begins to review life choices that he or she made at a time when his or her psychological functioning was driven by seeking relationships or work arrangements that maintained the narcissistic equilibrium. A typical example is a woman who chose to marry a man because he was adoring and masochistically willing to accept her chronic devaluation of him. In her more integrated state, she wished for a spouse whose emotional complexity and maturity were more at the level of what she had achieved. However, she also began to recognize that her valuing of admiration over mutuality had curtailed her intimacy with her husband, and she began to work with him to deepen their intimacy, both sexual and emotional. At this advanced stage of therapy, the therapist's role was to maintain neutrality about the question of whether she should stay in the relationship, and to

continue to help the patient to find her own solution to a situation that had no perfect one.

At this late stage, there is a shift in the transference from the therapist as an ideal or devalued object to the therapist as a new object who forms a bridge to the development of an internalized good object (Loewald, 1979). This forms the foundation for the anchoring of a secure internal working model of attachment (Main, 1991; Levy et al., 2006). As the patient's capacity to reflect increases, the therapist's role gradually shifts from more active clarification, confrontation, and interpretation to the role of an observer who witnesses the patient's process of reflection and, when appropriate, validates the patient's newfound capacities. Nonetheless, as the next section on termination illustrates, there continues to be times for the therapist to engage in the interpretive process.

The Termination Stage

The termination stage may proceed differently for patients with different presentations of narcissistic pathology. Ideally, we like to work until there is evidence that the patient is capable of genuine mutuality and satisfaction in love relations and investments in occupational, cultural, and creative activities. Although such an outcome is possible, some patients achieve a state of integration that contains residues of the narcissistic gratifications and illusions, albeit now integrated into a more complex, nuanced view of self, others, and reality. An example is a film producer who had never experienced intimacy in a relationship, but instead had only casual, transient affairs. At the end of therapy, he had settled into a relationship that seemed to include genuine love and mutuality. However, his fiancée was a much younger woman whose intense admiration of him as powerful and brilliant was crucial to him, although he recognized this narcissistic element and attempted to curb it. Other patients may achieve the ability to experience gratification in work and deep friendships, but many continue to find an enduring intimate relationship elusive. In such a case, the therapist might recommend continued work, but such patients may feel that they have arrived at an adequate adjustment in their lives.

It is important to note that structural change in psychodynamic therapy does not necessarily involve "total destruction of old object relationships that fuel the transference" (Westen & Gabbard, 2002, p. 91) but rather these shrink in salience as they are supplanted by new mental representations based on the new models of relatedness that develop in the treatment.

The process of termination may provoke anxiety and involve regression. The most typical types of regression around termination are either

the paranoid or the depressive. The more primitive, paranoid type involves a regression to the feeling that "my therapist really doesn't like me or care about me and wants to get rid of me," even though the termination was agreed upon by both patient and therapist. This reaction may recapitulate the narcissistic path of turning away from the caregiver, perceived as inadequate or rejecting, in the face of loss or disappointment (Symington, 1993). This process, which may have been explored earlier in the therapy, may surface again in the termination process in a more mature form where the individual takes more responsibility for his or her role in seeking the narcissistic solution of turning away from others into self-encapsulation. Or the acknowledgment of limitations in the work may provoke a depressive reaction to the inevitable imperfections and losses inherent in all human relationships and endeavors. The depressive regression may involve a feeling that the therapist has been devoted and concerned, but that the person's needs are too much for anyone to bear. This reaction may involve a final stage in the process of mourning the loss of the ideal object (which may have been set up prematurely in the internal world to deal with early frustrations and disappointments; Kernberg, 1975). There may be attempts to terminate prematurely in order to avoid the pain of facing the loss of the therapist and anger about the limitations of what therapy provided. In these cases, narcissistic persons may revert to a stance that they must and can do it all themselves, or even mobilize the manic defenses of contempt, triumph, and denial of the importance of the treatment. This may account for some of the abrupt and premature terminations observed in patients with pathological narcissism. Listening for and exploring these reactions in the termination stage is crucial.

Whatever the precise state of the patient at the time of termination, we end with the understanding that the patient may return in the future for consultation should his or her psychological state or situation in life call for it.

Major Techniques of TFP

The main techniques in TFP-N are those defined by Fenichel (1941), Gill (1954, 2013), Kernberg (2010, 2014), Joseph (1989), and others as the essential features of psychodynamic treatment—free association and evenly hovering attention, attention to unconscious processes, interpretation, transference analysis, countertransference utilization, and technical neutrality—all of which have been adapted for work with more severely disturbed patients (Caligor, Kernberg, & Clarkin, 2007; Kernberg, 2010; Yeomans et al., 2015).

Free Association

In TFP-N, as in most psychoanalytic approaches, patients are encouraged to talk freely about what is on their mind—that is, to free associate—so that the therapist can track the patient's internal reality while being mindful of what is happening in the patient's external life. More specifically, patients are encouraged to report their thoughts, feelings, fantasies, daydreams, and dreams as they emerge without any attempt to judge or order the contents of their mind. Recent developments in psychoanalytic thought privilege the analysis of the experience of the transference as a new relationship in the here and now as a route to therapeutic change; the therapist is therefore particularly alert to transference references in the free associations, even when manifest material is not explicitly about the transference (Gill, 1954; Aron, 1990). The therapist is also keenly attentive to the tone or atmosphere of the session, which communicates a great deal about the patient's experience of the therapist. In line with object relations perspectives, when patients talk freely about their relationships to others, they are often talking about aspects of the self and their immediate experience of the therapist (Bollas, 2008; Joseph, 1989). Hence, the therapist listens particularly to descriptions of self, others, and their relationship that are embedded in the flow of associations, along with the ways that free associations are designed to have an impact on the therapist, and to elicit a particular reaction from him or her. As Heimann (1956) stated, when the patient free associates, we must ask, "Who is speaking, to whom, about what and why now?" (p. 303).

Free association in TFP-N is thus viewed as an organizing framework for exploring the patient's internal world, including aspects of the self and the representational world, unconscious conflicts and motivations, external reality, and past and present, in the context of the relationship with the therapist. The centrality of free association in the treatment is included when the therapist discusses the patient's responsibilities in the therapy in setting up the treatment contract. The method of free association paradoxically demands that the therapist remain open to comprehending what is fresh, surprising, or unexpected in each session, without undue influence of prior knowledge or theoretical models. "The method presupposes that all the patient says can be meaningfully tied together and shown to belong to the continuum of psychic life," as Aron (1990, p. 458) reminds us. Yet, "the continuum of psychic life" is lacking in patients with personality disorders, including narcissistic disorder, whose experience of self and others is more or less distorted by minor or major image-distorting defenses that alter the experience of both inner and outer reality.

TFP-N includes a modification of the general rule of free association: The patient is instructed to speak freely in relation to the problems that brought him or her to therapy. Without this modification, the image-distorting defenses of patients with personality disorders can lead to free association that consistently excludes information regarding unintegrated or dissociated parts of the internal world.

Free association is particularly problematic for narcissistic patients. First, they are invested in maintaining a grandiose narrative about the self, which sustains their sense of superiority and often obfuscates significant sectors of reality. Second, they are invested in maintaining a detached relational stance that insulates them from deeper contact both with others, including the therapist, and with their own internal world, although this stance is defensive and often obscures vulnerable self-states linked to inner relational experiences not easily accessible. Individuals with pathological narcissism and NPD will leave out, distort, or deny important aspects of both internal and external reality in the interests of preserving their grandiose sense of self (Kernberg, 2015). With narcissistic patients whose attachment states of mind are often dismissing or disorganized (Diamond, Levy, et al., 2014; Meyer & Pilkonis, 2011), it is important to work with this more focal definition of free association. Verbal expression may be truncated and denuded of affect, often severely constricted, subject to repression, or denied. In such situations, it is particularly important to pay attention to the here and now—that is, to what is going on in the interaction between patient and therapist—in particular, the "immediate, often concrete reality of the individual's experience" (Joseph, 2013, p. 2) at the verbal and nonverbal levels, as well as to the reverberations in the therapist's countertransference.

Technical Neutrality

Technical neutrality is an essential aspect of TFP-N and is particularly essential for patients with narcissistic spectrum disorders, in part because of the intense countertransference pressures. The TFP therapist remains emotionally engaged and responsive, but he or she does not side with any aspect of the patient's internal object world over the other—rather, the therapist maintains an equidistant position between mutually split-off opposing positively and negatively valenced object relational dyads. The therapeutic rationale for neutrality is to keep the focus on conflicts as they are experienced within the patient rather than to collude with unconscious defensive attempts to resolve internal conflicts by the externalization of one element. The therapist's role is to maintain the stance of observer or a reflective third who can empathize with all sides of the patient's object relational world—even those that are the most extreme

or aggression laden—and to try to engage and strengthen the patient's observing ego in relation to conflictual or opposing aspects of his or her object relational world. For example, a seemingly high-functioning and successful male patient who presents himself initially as trapped in a marriage with a fragile, self-denigrating spouse who was alternately servile and harshly critical in relation to him, may initially present as frustrated and unhappy with the arrangement. Over time, however, he may recognize that he projects weak, inferior, and shameful aspects of himself onto the partner to whom he is then bound by the psychic filaments of projective identification. His disavowed sense of inferiority was safely contained in the dyad with his partner, who was both controlled and disparaged.

In such situations, the therapist must listen in an equipoised manner to all aspects of the patient's self and other descriptions as they emerge, identifying the internal, opposing object relations in which conflicts are embedded, rather than be swayed by the patient's surface presentation of the problem. In a similar fashion, the therapist remains open to listening for and exploring material from spheres of self (ego), moral values (superego), and the unconscious (sexual and aggressive impulses) with evenly hovering attention (Freud, 1914/1953) but without undue interest or investment in any one of these spheres. In other words, the therapist avoids siding with any aspect of the patient's internal or external life, including impulses, drives, defenses, object relations, or conflicts. The latter, from an object relations perspective, are embedded in self and object representations in mutual conflict. Individuals with personality disorders usually project such conflictual opposing object relations onto the screen of the session more readily and intensely than does the therapist who, as a result of supervision, personal analysis, and ongoing self-analysis, is continually monitoring his or her countertransference response to the patient's material. Remaining neutral, then, requires careful attention to countertransference. Rather than being in neutrality we are always working toward it and monitoring our proximity to it.

While the commitment to technical neutrality described above is generally in place, the therapist may appropriately shift away from a neutral position under certain circumstances. In situations when dangerous or self-destructive acting out threatens the patient, others, or the therapy, and cannot be contained with confrontation and interpretation, the therapist deviates from neutrality. He or she sets limits and perimeters for the patient to curtail the problematic behaviors and ensure the continuity of the therapy. However, after doing so, it is important that the therapist acknowledge and address the meaning of stepping out of neutrality to intervene directly in the patient's life by giving advice, setting limits, and so forth. After the risk has been contained, the therapist

explores the transferential repercussions of what transpired, such as the wish for a caregiver who will rescue the patient from dangerous behaviors.

Interpretation

Interpretation in TFP-N is conceptualized as a process that moves from clarification of the patient's affect states and mental representations with which they are linked, moving only in the later stages to exploration of unconscious conflicts. Confrontation about contradictions or inconsistencies in the patient's experience of self and others are revealed through discrepancies in the different channels of communications and interpretation per se as a hypothesis about the unconscious meanings and conflicts that relate these aspects of the patient's communications. Interpretation in TFP-N focuses initially on clarifying the patient's immediate experience of the therapist as a route to identifying the dominant self and object representations that emerge in the clinical situation. We move only gradually in the later stages of treatment into an exploration of the underlying unconscious conflicts, anxieties, fantasies, and motivations, and their connection with past experiences and relationships. In this process, the therapist relies on three channels of communication: (1) what patients tell us verbally through words and narrative in the free association; (2) what they tell us nonverbally through gesture, expression, or eye contact, or lack of it, and the atmosphere in the session; and (3) what they tell us by projectively communicating intense feelings that they cannot tolerate, or of which they are often unaware, through projection of such feelings onto the therapist who experiences them in the countertransference. From an object relations perspective, each self–object dyad is bound by a linking affect, and thus identifying the dominant affects is the conduit to identifying the dominant mental representations of self and others that are coloring the patient's experience in the moment. Working with what is affectively dominant in TFP-N is based on the idea that affects are primary motivators, and that there is a correspondence between affects and objects, since objects are first constructed in mental life by early affective experiences (Kernberg, 2007; Sandler, 1987). Interpretation as a hypothesis about the meanings and nature of the patient's unconscious, split-off, or dissociated aspects is focused in TFP-N on unconscious meanings in the here and now (Sandler & Sandler, 1987), as facets of the self and object world and their linking affects emerge in the moment-to-moment relationship with the therapist.

As the dominant affects that infuse mental representations of self and object representations are identified through clarification and

confrontation, the therapist forges linkages between the conscious, split representations of self and other that fuel his or her contradictory behaviors, and the unconscious conflicts and motivations that are hypothesized to affect the patient's motivations and functioning. Although we believe that interpretations are an essential ingredient for integration of the internal world, we also hold that interpretation of any sort for patients with personality disorders must deal with conscious or preconscious self-states that are experienced sequentially in a fragmented or dissociated way. Furthermore, a major goal of interpretation in TFP-N is the capacity to understand the concepts of self and significant others, and the thoughts and feelings with which they are linked as mental states that are subject to change and fluctuation depending on context, or the capacity to mentalize (Bateman & Fonagy, 2016). It is only in the late stage of treatment, when the patient's defensive system has shifted from primitive to more mature, that interpretations address how split, polarized, or distorted representations of self and others are linked to unconscious conflicts in relation to atavistic internalized objects and/or past experiences.

The focus in TFP-N on interpretation in the here and now in the arena of the transference follows Sandler and Sandler's (1987) idea that "the present unconscious" is a conduit to the "the past unconscious." The interpretations of split-off self-states and affects, as they emerge in the arena of the transference, ensure that they will have immediate affective resonance and that the treatment will not be derailed into intellectual speculation or the patient's pseudocollaboration. Both are obstacles to treatment for patients with narcissistic pathology. This focus on the identification and interpretation of affect as a link to underlying object relations is particularly important for those with narcissistic disorder because the primary function of the pathological grandiose self is to ward off intolerable affect states. Narcissistic patients, who are organized around a compensatory grandiose self, tend to project negative self-representations and associated affects onto others, and so these patients are prone to extreme reactivity (anger and antagonism) toward others who embody negative aspects of the self and if they fail to serve self-regulatory functions.

The narcissistic self-structure, however, protects against the underlying split in the self between extreme grandiosity and extreme vulnerability, manifesting itself in the transference initially and often for long periods as detached, distant, and emotionally unavailable.

How then do we use interpretation in TFP-N? Psychoanalytical clinicians have long recognized the need to modify "standard" psychoanalytical technique to meet the needs of narcissistic patients (Bromberg,

1983; Coen, 2010; Kernberg, 1975; Mitchell, 1986; Steiner, 2006), although they all recognize the importance of an interpretive approach for those with pathological narcissism.

Interpretation as conceived in TFP-N is our effort to observe with the patient the object relations dyads enacted in the treatment and their defensive function with respect to dissociated or projected experiences of the self and other. Although it serves as the primary technical strategy in TFP-N, interpretive work constitutes a major challenge to our narcissistic patients' protective armor. The rejection of the therapist's interpretive efforts involves the patient's experience of another's viewpoint, independence, and creativity as overwhelming and/or annihilating, a view discussed by several of the modern Kleinian writers (Bion, 1959; Britton, 2004b). Perhaps the therapist's interpretations evoke the specter of an earlier figure in the patient's life who embodied a threatening failure of containment in that there was both lack of empathy or concern for the patient, and misrecognition of his or her feelings. Perhaps the therapist's flexibility and openness, embodied through interpretive creativity, represents his or her freedom from the fixed roles assigned to him or her by the patient—that is, the therapist, in interpreting tactfully, assumes neither the role of validating, gratifying idealized object, nor the familiar, albeit negatively valenced role of a withholding, critical, rejecting, or otherwise disappointing object.

Interpretations that emanate from the therapist's independent functioning and reflect his or her *not* getting caught up in the enactment of *either* of the typical roles assigned by the patient can provoke intense anxiety, creating a situation in which the patient cannot locate him- or herself in the familiar role either in identification with, or opposition to, his or her internal objects. Alternatively, the therapist's interpretive work may signal an independence from the patient that threatens the patient's wish for unity of thought between patient and therapist, of the sought-after validation of the patient's dominant self and object worldview. The latter in fantasy allows the patient to avoid the reality of imperfect objects, and of the experience of feelings of deprivation, humiliation, badness, persecution, and guilt (Steiner, 2006).

Said plainly, when a dominant and cherished self–object configuration, however maladaptive for the patient, is challenged, the patient may feel under attack. The wished-for good connection with an object can be threatened, significantly raising anxiety and shutting down the creative interplay between patient and therapist. At such moments, the intensity of a dyadic treatment process can become overwhelming. LaFarge (1997) has argued that when the patient's mode of experience is characterized by paranoid–schizoid functioning in which a sense of danger and unsafety prevail, the therapist's communications should focus on

the containment of affect, rather than on more cognitively elaborated interpretations in the traditional sense. Indeed, at such moments, any but the mildest interpretative stance—anything beyond simple clarification, in the language of TFP—may be experienced by the patient as an enactment, one that has the effect of confirming the persecutory internal experience that the patient was attempting to manage through projection (Feldman, 2007).

The Interpretive Process in TFP-N

To avoid the above pitfalls, we refined the interpretive process in TFP-N for the spectrum of patients with pathological narcissism. A fuller description of this process, as it works in the service of addressing the individual's narcissistic structure and changing it, is provided in the following case example and in the chapters that follow. Here we provide just a brief sketch focusing on the initial stages. We recognize that each patient–therapist dyad has its own unique characteristics, and hence, it is difficult to talk about a sequence of interpretations that fits all patients or all patient–therapist pairs. However, through videotaping and studying transcripts of many patients over the course of three major research studies, we identified a sequence of interpretation that engages patients with more severe personality pathology in dynamic therapy, which leads to integration and changes in the developmental level of self and object representations, attachment security, mentalization, and self-destructive symptoms (Clarkin et al., 2007; Caligor et al., 2009; Yeomans & Diamond, 2010; Levy et al., 2006; Kernberg et al., 2008; Yeomans et al., 2015).

Clarification and Confrontation: Preparing the Ground for Interpretation

Through clarification and confrontation, the therapist prepares the ground for interpretations. Clarification involves inviting the patient to talk about what he or she is experiencing in the moment in an effort to expand on his or her current feeling states or other material. The goal is to make the individual more aware of self-states and the perceptions of others in the moment, and/or to elaborate on the aspects of his or her experience that may appear as vague, perplexing, or ambiguous. In other words, clarification may focus on external reality; internal reality, including mental states, thoughts, feelings; and/or the immediate experience of the therapist. The latter is particularly useful in that it fosters joint exploration of a shared experience when the affect is immediately

accessible to verbal expression. We use clarification, including direct inquiry about any of the foregoing aspects of the patient's presentation, in the interest of expanding our understanding of the patient's communications, thoughts, and feelings as they emerge in sessions, since lack of clarity may signal the presence of conflicts and defenses against them.

As contradictions or inconsistencies emerge through clarification, we try to bring them to conscious awareness where they can be explored by confronting the patient about them. The process of confrontation is an inquiry that calls attention to contradictions or inconsistencies in the patient's communications, either within verbal communications, such as a focus on contradictory information presented at two points in time, or between the verbal and nonverbal channels of communication at one moment in time. As such, through confrontation we bring to the fore preconscious levels of experience as a way of accessing split-off aspects of the self that are being enacted in various ways that are in contradiction with what is being verbalized. An example would be the therapist commenting, "You're saying that you're very sorry and concerned that your colleague was fired. However, the smile with which you're communicating that information suggests there may be more to the picture than that," or "You are telling me that I understand you the way no therapist ever has, but you have missed two sessions in a row, and that suggests that may not be the only thing in your mind right now." The therapist engages the patient in reflecting on these inconsistencies and their meaning, mobilizing the issues that underlie such discrepancies and preparing the way for interpretations that explore the dissociated aspects of experience and unconscious conflicts, underlying wishes, fears, and anxieties. As therapy advances, we also explore their antecedents in early experiences and relationships.

Clarification and confrontation pose particular challenges with individuals in the narcissistic spectrum. In the early stages of treatment, such individuals often present a grandiose narrative that clearly does not correspond to the reality of the patient's life circumstances or possibilities, that distorts crucial aspects of his or her functioning, or that justifies behaviors that can be quite profoundly self-defeating or self-destructive. Seeking clarification or confrontation in the face of such narratives may be met with indifference with the patient calmly accepting them and denying that they require any further exploration, or may elicit strong reactions from the patient. For example, one patient, who was being supported by her parents while living in an ashram and writing a screenplay about it, disdained any formal professional training, insisting that when she decided to start a career, she would quite quickly become a successful film director. Clarification of the route she intended to take to realize this ambition—to apprentice herself to a family friend

of her parents, an amateur filmmaker himself, and to eschew any formal training—revealed a striking lack of reality orientation about how to attain her goal. At first, she responded to the therapist's inquiries with devaluing comments about the therapist's ignorance of what it takes to make it in the film world today. But further clarification and confrontation about the discrepancy between her strong investment in this potential career and the vagueness of her plans to meet her aspirations led to exploration of the shame and self-criticism that underlay her rigid adherence to an unrealistic plan.

Clarification and confrontation are particularly important in the early stages of TFP-N because they slow down the dominant pathological grandiose narrative and encourage the individual to reflect on the minute, fleeting shifts in representational states from grandiose to vulnerable as they play out in the patient's life and therapy. These techniques bring attention to the costs of the patient's retreat to grandiosity in the form of relational disruptions, interferences with functioning and productivity at work, and emotional isolation, and help them address the strong and seemingly impenetrable defensive barrier (Diamond & Meehan, 2013; Kernberg, 2007; Rosenfeld, 1971).

By challenging defenses and revealing inconsistencies, clarification and confrontation lay the groundwork for interpretation, while we are mindful of how all of these interventions may make the narcissistic individual feel unmasked or exposed, thereby incurring shame and humiliation. The therapist should be sensitive to these reactions, but in our view, confronting and interpreting grandiose defenses is essential for personality change to occur, particularly with those with thick-skinned narcissistic disorder, a view shared by Rosenfeld (1971) and others. Even those in the relational school (e.g., Bromberg, 1983; Mitchell, 1986) stress the importance of confronting and interpreting aspects of the grandiose self and the narcissistic illusions that stem from it, particularly the extent to which grandiose fantasies linked to idealized objects constrict and truncate engagement with others in depth. As poignantly expressed by Mitchell, our work involves steering "a narrow passage between the contrasting dangers of complicity and challenge . . . reflecting an acceptance of the importance of the narcissistic integration as a special and favored mode of relation yet questioning why it must be this and only this way" (p. 126). Such a stance blends curiosity and analytic inquiry into the meaning and functions of the narcissistic self-structure, while exercising the tact and respect required in our efforts to help loosen the defensive net that entraps our narcissistic patients, yet in which they are often willfully ensnared.

Thus, as a therapist first explores the narcissistic defenses and resistances through clarification, confrontation, and interpretation, he or she

should do so in the context of an empathic understanding of how the grandiose self defends against underlying anxieties. Narcissistic patients use elaborate and sometimes extreme and seductive maneuvers to stabilize their sense of self and keep feelings of inferiority at bay, to guard against fears of humiliation and attack. Furthermore, the narcissistic organization insulates them against intolerable anxieties about dependency on others by offering the illusion of self-sufficiency and omnipotence. The therapist should be mindful of how the patient has relied on the chimeral world of the grandiose self to stabilize an underlying fractured sense of self (Kernberg, 2015), to preserve ties to idealized objects (Fairbairn, 1952), and to sustain a narcissistic solution that may have been adaptive at an earlier point in development.

While the focus of the interpretive process in TFP-N is on the dominant object relations that organize the experience of self and others, with narcissistic individuals the grandiose self may obscure the specific object relational dyads. Hence, in TFP-N we focus on the psychological structures/processes underlying the grandiose self. First we explore the manifestations of the grandiose self through clarification and confrontation—the first steps in an interpretive process that focuses on identifying and understanding the distorted, unrealistic mental representations that comprise it. From there, the interpretative process consists of the following phases:

1. *Interpreting in the projection*. The focus is on *identifying and containing the affect* that is evoked in the patient's immediate experience of the therapist or others without challenging it or linking it to anything in the patient's history, motivations, or aspects of the internal world. This involves recognizing and containing the intolerable aspects of the patient's internal world as they are projected onto the therapist or others, as a first step in defining the dominant mental representations that comprise the internal world.

2. *Defining the dominant object relational dyad*. The therapist identifies the representations of self and other, and their characteristic interaction, hence completing the interpretation of the projection identified in phase 1. The focus is on naming the internalized, split-off object relational dyads, particularly those associated with grandiosity and vulnerability, which are activated in interpersonal interactions in the here and now. For those with pathological narcissism, this often involves identifying the patient's experience of the therapist as incompetent or ineffectual with the self as superior, or the therapist as superior and omniscient with the self as inadequate and devalued.

3. *Working with role reversals within the dyad*. The focus is on tracking how the patient repeatedly activates the dominant object

relation with role reversals. The therapist calls attention to how the patient may shift the position from which he or she interacts with the therapist, alternately identifying with one side of the dominant object relation while enacting the other, reversing polarities within the self, often without awareness. A dismissing patient who is uncomfortable with dependency initially may perceive the therapist as devalued and irrelevant, but then, as he or she shifts identifications from the self-sufficient, exceptional self to the previously-projected devalued aspect, may feel neglected and needy, complaining that the therapist, now seen as superior, is not sufficiently attentive. Such role reversals challenge the ideal self, thereby tempering grandiosity.

4. *Identifying how split-off object relational dyads with opposing positive and negative affective valences may oscillate and/or defend against each other.* The focus is on identifying and reflecting on how dyads on the surface, such as the superior self in relation to a devalued other, may defensively dissociate against split-off and devalued longings for a gratifying caregiver in relation to a dependent self, with the goal of developing more multifaceted, realistic representations of self and significant others that integrate positive and negative affect states as the grandiose self dissolves.

5. *Integration of dissociated, projected, and split-off aspects of the patient's internal world.* The focus is on interpreting the patient's unconscious anxieties, impulses, and conflicts. Then, as these feelings retreat to the grandiose self and the primitive defensive operations that sustain it, focus shifts to exploring the origins of this self-structure in the developmental past.

The interpretative process and the stages of TFP-N are described more fully in Chapters 7 and 8.

Transference Analysis

Interpretations in TFP-N may address a number of aspects of patients' self- and interpersonal functioning, including the dynamics of extratransferential relationships, conflictual feelings, motivations, and anxieties and defenses against them, but they are largely focused on our understanding and experience of the patient as it emerges in the transference. Through systematic attention to the transference, there is a "gradual transformation of pathological character patterns into an emotional experience and self-reflection in the transference" (Kernberg, 2004, p. 126).

The importance of transference interpretation for successfully treating individuals with narcissistic spectrum disorders has been affirmed by

prominent clinicians from different theoretical orientations (Bach, 1985; Bromberg, 1983; Kohut, 1977; Kernberg, 1984, 2007, 2014, 2015; Maldonado, 1999; Steiner, 2011; Rosenfeld, 1971). This is because in those with personality disorders, including narcissistic disorder, bifurcated, polarized aspects of the internal world predispose the patient to develop more rapid, intense, and powerful transferences than to defend intractably against them. In those with narcissistic pathology, these transferences are often less evident and take longer to emerge than is the case with patients with other personality disorders. In fact, the grandiose self-organization of those with narcissistic pathology often first manifests itself in a subtle sense of distance and unavailability in the transference.

Several psychoanalytic clinicians from different theoretical vantage points have noted such detachment and disengagement on the part of those with narcissistic disorder (Kernberg, 1984; Bromberg, 1993; Modell, 1975). Kernberg, for example, has written that with narcissistic individuals, "the defense of avoidance of true dependency is often matched by the complementary defense of omnipotent control, which is a conscious effort on the patient's part to avoid . . . the emergence of feelings of inferiority stemming from the newness of recognized importance of what the analyst is saying. . . ." (p. 627). Bromberg, from a relational standpoint, comments on the detachment of narcissistic patients, which serves to protect them from "needing any relationship at all; that is from the experience of inadequacy that accompanies any evidence that the patient lacks anything he cannot give himself" (p. 162). In a similar fashion, Modell (1980) has observed that narcissistic patients are characterized at best by a state of nonrelatedness, in which there is "a fundamental inability to take anything in," or in the most extreme cases by a "cocoon-like state of non-communication," that functions "as a prison from which they cannot escape" (p. 261). The foregoing statements describe the manifestations of the defensive compensatory self-structure and insecure (dismissing) attachment patterns of individuals with narcissistic disorder as they emerge in relation to the therapist. Hence, with narcissistic patients it is important to clarify the dynamics of the way the patient is relating to the therapist before focusing on content. It may take a very long time before narcissistic patients may experience or own any feelings in relation to the therapist.

In TFP-N, the face-to-face interaction and emphasis on the here and now counterbalances the detachment and emotional disengagement frequently observed in persons with pathological narcissism. The process of clarifying, confronting, and interpreting aspects of the grandiose self as they emerge in the therapeutic arena is the first step in helping the patient to move beyond the narcissistic structure and insecure (dismissing and anxious) attachment states of mind to develop and sustain

mutual, complex, multifaceted, enduring relationships. Transference analysis is often based on reflecting on the patient's way of being with us, or mode of relating, more than on the content of what the patient is saying to us. The therapist's primary task is to identify the dominant object relation from what Joseph (1985) and others would call "the total transference situation" (p. 447). This is the idea that the patient's predominant object relations come alive in the transference along with the defenses used to maintain psychic equilibrium. From an object relations perspective, defenses and impulses are embodied in object relations dyads (Kernberg & Caligor, 2005). For example, in those with narcissistic pathology, a grandiose self in interaction with an inferior, deficient other infused with affects of contempt or anger may defend against a dependent self, perceived as weak, in relation to a powerful, rejecting other linked to feelings of longing, loving, and sadness. In this view, the transference may represent the repetition of an object relationship and defenses against it.

With narcissistic patients, often the transference first appears in reports of the conflictual or problematic dynamics in extratransferential relationships, termed *lateral* or *displaced transference* (Busch, 2013). Dealing with the transference dynamics first through an exploration of extratransferential relationships, as Busch points out, functions to build a bridge to the transference until it is closer to the patient's conscious awareness or emotional "neighborhood" (p. 106). Thus, while initially the therapist may focus on material outside of the transference, particularly when it is affectively dominant, in TFP-N the therapist usually privileges material reflecting the dominant object relations dyad in the context of the patient's experience of the therapist. At the same time, the therapist needs to be aware of how the content of such indirect or lateral transferences reflects the interaction with the therapist, even if it deals with things outside the session. For example, a patient who presented for treatment while in the midst of a bankruptcy caused by fiscal irresponsibility and a divorce precipitated by multiple infidelities told the therapist in one of the first sessions about being swindled out of his inheritance by his uncle who was made his guardian after his father died in an event that was never openly acknowledged in the family, although everyone was aware of it. The patient reported this traumatic experience in a bland manner despite his describing it as nodal event that affected his life in multiple negative ways both emotionally and financially. Interestingly, he stopped during his narrative to wonder whether he was telling the therapist this to enlist the therapist's compassion because he feared the therapist's negative judgments about his past irresponsible behavior. Here the dominant object relation was not the victimized patient in relation to an exploitative other but an unworthy, transgressive self in relation to a critical other, whose negative judgment he feared.

Transference may also represent the emotional climate or feeling tone of early inchoate experience not necessarily linked to a specific, identifiable mental representation, as a result of defensive processes or impoverishment of an internal world of object relations (Busch, 2013; Kernberg, 2018). In those with pathological narcissism, this may be evident in a remote, aloof presentation, in a subtle sense of disengagement and emotional unavailability that conveys the impression that there is no transference. This can make the therapist feel ignored or discounted and may lead him or her to make premature transference interpretations that are experienced by the patient as baffling, off-putting, or intrusive. In such cases, it is important to pay attention to all the material that emerges in the session, not only the content of what the patient is saying but also his or her attitude toward the therapist, and his or her wishes that the therapist respond in a particular way.

Comparison of the View of Transference and Countertransference in TFP and Neo-Kleinian and Relational Approaches

The focus on transference analysis begins early in treatment and emphasizes the repetition of earlier relationships or defenses against them as they emerge in the therapeutic relationship. This makes TFP-N consonant with neo-Kleinian approaches, wherein the therapist is acutely attuned to the ways in which he or she is being figured, used as a person onto whom the patient's projections of split-off or unacceptable self- and object representations can rest (Joseph, 1985). Our approach is also to some extent consistent with relational theorists (see Bromberg, 1983; Mitchell, 1986), who express great interest in how the patient's experience of the therapist's actual characteristics influence the therapist's perception of "transference." Relational theorists discuss transference as the patient's need to approach and make use of the therapist (Mitchell, 1999) in a particular way, and the therapist's interest in the same.

It is now generally accepted in contemporary psychoanalytic thought that transference and countertransference can be mirror images of each other in that they are mutually influential, and that both patient and therapist contribute to the transference–countertransference matrix in ways that are unique to each patient–therapist dyad. However, we do not ascribe to the idea that all transferences are inevitably, or in equal measure, co-constructed. Following from our model that those with narcissistic pathology often rely on splitting-based defenses—but that splitting is often obscured by the rigidity of the projection of negative aspects—it is our view that the patient needs to communicate via projection onto others in his or her interpersonal field. The analysis of these defensive maneuvers when emanating predominantly from the patient

ought to be the focus of our attention. We believe that the nature of what is projected in the case of narcissistic patients, as discussed above, is somewhat consistent in form and content. Not surprisingly then, we observe a trajectory of transferences across the specific patient–therapist dyads in individuals with narcissistic disorder, although they manifest somewhat differently across the spectrum of those with narcissistic pathology. In addition, we do not adhere to the idea that there are no set techniques or psychoanalytic guidelines or methods that are applicable across individual cases, even though we respect the idea that such techniques should be applied flexibly according to each patient–therapist dyad. The idea that there are standard techniques that can be learned, taught, and practiced effectively for particular groups of patients has been challenged by a number of contemporary analysts. However, such critiques ignore a great deal of research on the efficacy and effectiveness of different forms of psychoanalytic therapy for different groups (see Graf & Diamond, 2018, for a review).

Despite these differences, in TFP-N we are constantly attentive to how the patient is viewing and experiencing the therapist in the treatment process. The primary lens through which we identify the specific object relation that is active at any moment in the treatment process is that which is *affectively dominant* in the moment. In working with more severely disturbed patients with personality disorders, the therapist almost immediately is caught up in a maelstrom of projections and enactments necessitating more direct transference work. However, such patients are often consciously or unconsciously fearful of the transference feelings. Thus it is essential first to help the patient understand the specific dynamics involved in his or her mode of relating to the therapist in the moment by staying close to the surface and to what the patient can consciously tolerate (Busch, 2013; Yeomans et al., 2015). This focus on the here and now is a way of particularizing the affects and fantasies of the object relation being expressed in the transference and of learning what aspect of the patient's internal world he or she can tolerate and elaborate on. This paves the way for a deeper exploration and reflection on the more varied primitive and advanced transferences delineated in the next section. Occasionally, however, with patients with personality disorders, including narcissistic disorder, for whom primitive transferences may be evoked from the beginning of treatment, an early, deep transference interpretation is called for when patients are at risk for self-destructive acting out, or premature dropout with little awareness of what is motivating their behavior.

Like many neo-Kleinian approaches, we privilege what is affectively dominant in the session and the relationship with the therapist as a conduit to unconscious conflicts and fantasies around internalized object

relations, but in contrast, the TFP-N therapist weaves back and forth between the analysis of the transference and the exploration of external reality. Specifically, the therapist obtains the most complete picture possible of the patient's actual functioning in the spheres of work and interpersonal relationships, as well as the destructive and self-destructive symptomatology characteristic of patients with severe personality disorders, including narcissistic disorder (Bender, Morey, & Skodol, 2011; Kernberg, 2014, 2015; Yeomans et al., 2015). In this sense, the therapist is not only the transference figure but also the historian and conveyor of reality. Thus, in TFP-N the concept of the total transference (Joseph, 1985) has been expanded to include aspects of external reality that are often dissociated or in the arena of enactment with more disturbed patients. The importance of this "dual focus" is further discussed later in this section.

The focus on the transference is particularly important for patients across the spectrum of narcissistic pathology in that the face-to-face interaction and emphasis on the transference, as it is activated in the here and now, counterbalances the detachment and emotional disengagement of those with pathological narcissism. Furthermore, the focus on the transference activates the dissociated or projected mental representations of self and others linked to grandiose and vulnerable self-states so that the patient can experience and observe them *in vivo* in the therapeutic arena. This leads to an increased acceptance and awareness of disowned and projected aspects of self and others. The latter are often first evident through the countertransference experience of the therapist.

Overview of the Main Transference and Countertransference Patterns

Individuals with pathological narcissism have been found in both clinical and empirical investigations to precipitate particularly challenging patterns of transference and countertransference that may account for the high rates of treatment failure and dropout. Although we recognize that the experience of patient and therapist in the transference and countertransference is "complex, deep and ephemeral beyond words" (Tuckett, 2019, p. 852), we have compiled a profile of general transference themes that we have observed in TFP-N. While to some extent this profile follows our clinical theory of narcissism, it has been affirmed in independent empirical investigations with clinicians with varied clinical orientations that are described later in this section (Bradley et al., 2005). These transference dimensions are the baseline on which other more fleeting moment-to-moment transferences may be experienced. While

the following transference dimensions reflect the fundamental underlying structure in narcissistic patients, their sequence and expression vary depending on the presentation (thick- or thin-skinned) and organizational level of the patient (Kernberg, 2019).

The Narcissistic Transference

The narcissistic transference has a unique status because it can be present in conjunction with the others while serving to obfuscate them. We have described narcissistic transferences above as a lack of relationship based on an incapacity to depend on an other or a fear of dependency that results in a dismissive attitude toward the therapist. The dismissive attitude often covers over fears of rejection or dependent longings. It can include a devaluing of the therapist as a means of eliminating him or her as a significant object who, if valued, could be envied, could be capable of hurting the patient emotionally, and could be longed for. Thus, narcissistic transferences may create the appearance that there is "no transference," that the therapist does not matter enough for the patient to care about. Or the narcissistic transference can take the form of a grandiose self in relation to an idealized other who is experienced as omnipotent, sagacious, and all-knowing.

What appears to be a nonrelation in the early stages is actually a relation between parts of the [grandiose] self projected onto the other in self–self transference (Kernberg, 1975). Hence, the narcissistic transference may take different forms depending on which aspects of the grandiose self are experienced or projected. Rooted in the structure of the compensatory grandiose self, narcissistic transferences generally serve a defensive purpose against the experience of an underlying paranoid, sadomasochistic, dependent, or depressive transference.

The Antisocial/Psychopathic Transference

This involves a fundamentally exploitative attitude toward the other and the assumption that others share that exploitative attitude. An antisocial/psychopathic transference may be present in patients with narcissistic pathology organized at a low level who are not totally lacking in the desire for a relationship. We should clarify that although the terms *antisocial* and *psychopathic* are sometimes used interchangeably, they have distinct meanings, and both apply in looking at transferences of those with narcissistic pathology. Psychopathic refers to individuals who are cold, callous, controlling, and exploitative toward others, while antisocial refers to specific behaviors, such as lying, stealing, and so forth

(Hare, 1996, 1999). Antisocial/psychopathic transferences in narcissistic patients are marked by the assumption that everyone is dishonest and only looking out for themselves, that relationships are based on control and manipulation, and that the possibility of genuine love and concern is nil. Since most narcissistic patients are not full-blown cases of ASPD, they may harbor an underlying wish that some such positive connection could exist.

The Paranoid/Narcissistic Transference

Paranoid transferences may manifest as fear of harm from the therapist or as masochistic or sadomasochistic transferences, in which the experience of suffering at the hands of the therapist, and making the therapist suffer, are enacted in subtle ways. In general, paranoid transferences may sometimes defend against the idealized libidinal object relations that constitute the other side of the split internal world and against the depressive transferences described below. The narcissistic transference may also sometimes defend against a paranoid transference: If the other does not matter, the other cannot hurt the patient. Importantly, in the case of the patient with pathological narcissism, the harm that the other is capable of can involve representing a reality that does not support the grandiose self. In this sense, the threat that the therapist/other presents is not necessarily as direct a threat of harm as the patient fears—of being attacked or abandoned—but the indirect threat of the annihilation of the sense of self. While the typical fear of the patient with BPD, of being rejected or hurt, is intense, the fear of the narcissistic patient, of the annihilation or collapse of his or her sense of self, is much more so. In the narcissistic/masochistic transference, there is a split object relation of a helpless victimized self and a defeated therapist with a focus on self-condemnation and sanctification of suffering that serves to thwart the therapist's efforts. In the sadomasochist transference, there is a split object relation characterized by a dominant controlling sadistic other in interaction with a subjugated self.

The Eroticized Transference

The eroticized transference, evident in more disturbed narcissistic individuals, is characterized by aggression masked by a libidinal façade. It may take the form of overt seductiveness and even attempts to physically seduce the therapist on the part of the patient in the interests of turning the tables and omnipotently controlling the therapist. In the eroticized transference, there is an underlying split object relation in which sexual excitement is used in the interests of control, hostility, and manipulation.

The Dependent Transference

The dependent transference involves a partially integrated object relation of a dependent, vulnerable self in relation to a gratifying, protective idealized caregiver. It often emerges in tandem with the paranoid transference in that dependent longings on the part of those with narcissistic pathology challenge the narcissistic illusion of self-sufficiency, leaving the individual prone to fears of exposure of underlying fragility or vulnerability.

The Depressive Transference

As narcissistic patients work through various experiences with the therapist in TFP-N, with new awareness they learn to temper and tolerate idealized and devalued, positively and negatively toned presentations of self and others at the same time. The move toward integration, or the approach to the depressive position, puts the patient in touch with affects that were previously inaccessible and managed largely through projection. This dawning awareness that the same person representing a deeply feared relationship is also the person to whom one directs powerful longings for love and connection, leads to the eventual breakdown of the belief in an ideal object. Others are now seen in a more complex nuanced fashion as an amalgam of positive and negative aspects, with idealization tempered by the recognition of human imperfection and limitations. This capacity to tolerate ambivalent reactions toward others, to bear envy and experience gratitude, and to take responsibility for one's own aggression, combined with impulses to repair previously damaged relationships, constitutes the movement from the paranoid–schizoid to the depressive position (Klein, 1946, 1957). The final stages of therapy involve the resolution of the concerns and anxieties of the depressive transference: these involve guilt that comes with increased awareness of the individual's capacity to hurt the other; fear of loss of the others now valued as separate vulnerable individuals; relinquishment of ties to ideal introjects; and acceptance of more real internal representations of self and others. In sum, the depressive transference involves an object relation of a grateful, remorseful self in relation to a valued, if imperfect, therapist seen as a whole, complex other separate from one's own needs.

The Erotic Transference

The erotic transference may take different forms in those with narcissistic disorders depending on their presentation and level of organization. Particularly in those individuals who are higher functioning, or

thick-skinned, erotic transferences emerge slowly since they involve acknowledging longing and desire for the therapist, and thus recognizing the value of the other despite imperfections and/or unattainability. As treatment evolves and those with narcissistic pathology overcome their sense of omnipotence and self-sufficiency, patients may begin to experience and acknowledge longing and desire for the therapist. Toward the later stages of therapy, erotic transferences come to involve a blend of loving, tender, and erotic feelings toward the therapist. In general, it is also in these advanced stages of treatment that transferences become specific to the individual person of the therapist. Thus, the erotic transference involves a fairly well-integrated object relation of an erotically stimulated self in relation to a valued, desired, but unattainable, therapist. Erotic transference usually develops in tandem with triangular (oedipal) transferences and themes in the therapy involving the patient's conflicts around competition and rivalry in triangular relationships. They often emerge in the form of increased curiosity about and jealousy in regard to the therapist's personal life and relationships, or as representations of a competitive, vanquished self in relation to an ideal rival. Table 4.1 provides a summary of the major transference

TABLE 4.1. Transferences of Narcissistic Patients

- Narcissistic transference
  - Self–self transference—a grandiose self obscures specific object relations; other seen in the light of idealized or devalued aspects of self
  - Grandiose self–devalued other
  - Grandiose self–idealized other sharing in patient's uniqueness
- Narcissistic/paranoid transference
  - Grandiose, devaluing ideal self in relation to a harmful or persecutory other
- Antisocial/psychopathic transference
  - Exploited self in relation to an exploitative other or vice versa
- Dependent transference
  - Dependent, vulnerable self in relation to a gratifying protective idealized caregiver
- Depressive transference
  - Grateful/reflective worthy self in relation to a valued, imperfect other seen realistically as an amalgam of positive and negative qualities
  - Guilty/remorseful self in relation to a uniquely valued/vulnerable other
- Erotic/oedipal transference
  - Competitive self in relation to an ideal rival
  - Desirable, erotically stimulated self in relation to an unattainable, desired other

patterns that evolve from the early to the later stages of TFP-N with those with narcissistic disorder.

Countertransference Use in TFP-N

With narcissistic personalities we often experience in therapy the countertransference aspects of the patient that are split off, unconscious, enacted, and thus not available to verbal expression or other modes of communication (Bion, 1957; Kernberg, 2010; Joseph, 1989; Steiner, 2011; Yeomans et al., 2015). Primitive aspects of the patient's internal world are encapsulated in dissociated self-states that may shift drastically and tend to be expressed either in actions or through inducing powerful feelings in others (Kernberg, 2010; Joseph, 1989). For this reason, our countertransference reactions with individuals with personality disorders are often disturbing and intense. Countertransference may be defined as an unconscious reaction in the therapist to what is unconscious, split off, or enacted by the patient (Kernberg, 2018). It involves the therapist's reactions to the patient's transference that may derive from either specific idiosyncratic issues or conflicts activated in the therapist in response to the patient's material, or from the intensity of the patient's need to project unacceptable aspects of his or her internal world onto the therapist, or both. In TFP-N, we attend continually to the countertransference to learn more about our patients and what is going on in the clinical process, as well as to minimize the risk of the therapist's acting out, even in subtle ways. Although our ability to attune to transference distortions is invariably compromised by our own defensive needs and pressures, we nevertheless position our role as observing and working to familiarize our patients with, and return to them, as tactfully as possible, the parts of themselves that they have been managing by projection.

As the narcissistic mode of relating comes alive in the transference, the therapist may find his or her interventions ignored or disregarded, or his or her attempts to make linkages between conscious and unconscious experience disparaged, denied, or appropriated immediately as originating in the patient. The patient may omnipotently attempt to control the therapy, so that the therapist may in fact feel not only devalued, but also actually deskilled and even obliterated as a real person in the room. Conversely, the therapist may find him- or herself massively idealized, with interpretations and expertise valorized as if oracles from Delphi that may be incorporated immediately as one's own but without genuine exploration or genuine reflection. In such situations, the narcissistic individual has projected aspects of his or her own ideal self onto the therapist, including the therapist in his or her sense of omnipotence and omniscience, but in the process, the therapist is obliterated as

a separate, real person with knowledge and skills that could enhance self-understanding, or on whom the patient may depend.

Each of the transferences described above poses powerful countertransference challenges to the therapist. Feeling ignored and rejected, feeling pressured to mirror and match elitist, grandiose experiences of the self, while feeling the anxiety associated with the inevitable need to show the patient something new or different from his or her dominant, most ego-syntonic self-view—all of these pressures are intense and consequential. There is always a risk that a therapist who has unresolved narcissistic conflicts may derive too much gratification from the patient's idealization, may encourage such narcissistic illusions about the therapy in subtle ways, or may abruptly deflate them without adequate exploration of their meaning or function. It is essential to monitor countertransference for the therapist's own issues around narcissistic conflicts or illusions in order to sustain a stance of open exploration about the meanings and functions of the narcissistic organization and modes of relatedness, while offering alternatives to it. Failure to acknowledge and understand such feelings puts us at risk of acting them out in either a chronic (supporting or validating narcissistic illusions) or acute (making the patient feel special by stretching the boundaries of the treatment in various ways) manner. Nonetheless, following the idea that therapist and patient constitute a "bipersonal field" (Baranger & Baranger, 2008), it is important for the therapist to allow the self to feel in the countertransference what the patient makes him or her into. We use our various countertransference responses first to imagine the patient's experience, and second to share with the patient a hypothesis about his or her experience—this indeed represents so much of what we struggle with in implementing TFP-N with our patients.

The affect(s) evoked in the countertransference may be so intolerable to the patient's consciousness that the therapist may need to "hold" or "contain" them for a considerable period of time before speculating with the patient as to whether those feelings have any connection to the patient's internal world. The therapist subjected to relentless devaluation may find it difficult to not react defensively, or to use interpretive methods prematurely. However, such a reaction may perpetuate the patient's view that all relations are based on each person vying to assert his or her superiority over the other. Such countertransference reactions may be termed *complimentary identifications in the countertransference* in that they involve an identification with an internal object or aspect of self that the patient cannot tolerate (Racker, 2018). The therapist's ability to contain the projection of the devalued (or idealized) object—combined with the use of interpretations that focus on how these projected aspects

of the self contour his or her experience without necessarily locating these aspects within the patient (e.g., interpreting in the projection)—creates a situation in which the patient may be helped to observe and reflect on a phenomenon that does not fit into his or her worldview. Such a process often involves the therapist's *concordant identification in the countertransference*, which involves the therapist's emotional identification with the patient's subjective experience. Such an identification on the part of the therapist may help him or her to better understand such experiences and help the patient to observe how he or she emanates, at least in part, from the self.

With narcissistic individuals it is often more tolerable and egosyntonic for the therapist to identify with the objects of the patient, who may be bearing the brunt (as the therapist often does) of the patient's narcissistic pathology in the form of devaluation or exploitation. It is less palatable for many therapists to recognize their concordant countertransference with the narcissistic patient, which may tap into their own unresolved narcissistic conflicts and residues of infantile narcissism or lure of narcissistic gratifications. Such an approach to transference interpretation and countertransference analysis takes into account not just the flow and content of transferences and countertransferences but resistances and defenses against them in both patient and therapist. Table 4.2 outlines the major countertransference reactions that evolve in therapists conducting TFP-N with individuals with narcissistic pathology. Although these are the typical countertransference reactions we have observed, they may not unfold in sequence, but evolve out of the

TABLE 4.2. Typical Countertransference Reactions to Narcissistic Patients

- Devalued/sense of inadequacy
- Admiration/idealization of the patient
- A sense of twinship with the patient
- Feeling rejected/deskilled/incompetent
- Boredom/detachment
- Fear of angry, contemptuous attacks
- Anger/an urge to defend self/retaliate
- Being impressed by the patient's wealth, power/prestige, talent, or beauty
- Sexual seduction
- Sadness/hopelessness
- Rescue fantasies
- Wish to please/gain approval

matrix of the patient–therapist interaction and depend to some extent on the patient's level of organization and dominant presentation (i.e., thick- or thin-skinned).

Empirical Research on Transference and Countertransference Patterns in NPD

A recent review of transference and countertransference patterns in the therapeutic relationship, based on self-reports of expert clinicians from a variety of theoretical orientations, revealed a profile of transference processes of patients with pathological narcissism and NPD that converge with the clinical profiles of transference patterns presented above (Bradley et al., 2005). Indeed, clinicians from a variety of theoretical orientations reported that patients with narcissistic pathology show the following behaviors and attitudes in relationship with the therapist: (1) detachment, (2) entitlement and demands for special privileges, (3) critical attitudes toward the therapist, (4) playing the therapist off against other treaters, (5) feeling mistreated and abused by the therapist, (6) showing sadistic behaviors and attitudes toward the therapist, (7) envy of the therapist, (8) excessive need for admiration from the therapist, and (9) tendency to think of the therapist as a mirror image of or more similar to the self than he or she is (Bradley et al., 2005; Tanzilli, Colli, Muzi, & Lingiardi, 2015). These patterns were also found to be related to countertransference experiences.

In a separate independent study, experienced clinicians reported countertransference reactions with narcissistic patients that corresponded remarkably to the prototypical transferences described above, including (1) emotional disengagement and hopelessness, (2) wishes to terminate the treatment, (3) hostility/resentment, (4) feeling rejected/deskilled, (5) lack of empathy; urge to defend the self and retaliate, (6) fear of the patient's hostility and sadistic attacks, (7) fear of the patient's envious attacks, (8) admiration/idealization of the patient, and (9) feeling invaded or controlled (Betan et al., 2005; Tanzilli et al., 2015). These empirical findings suggest that patients with pathological narcissism are likely to develop transferences that are narcissistic (devaluing and idealizing) and paranoid (fear of harm or maltreatment) in nature, especially during the first months of therapy. Thus, narcissistic patients often induce powerful, unsettling, and often distressing feelings in their therapists, just as they do in others (Miller et al., 2010). As listed in Table 4.3, there is a welter of contradictory feelings that can arise in therapists' countertransference with narcissistic patients.

The transferences and countertransferences with narcissistic patients, reported by clinicians, may be thought of as co-constructed

TABLE 4.3. Clinician Self-Report of Transference/Countertransference with Narcissistic Patients

Transference	Countertransference
Emotional detachment	Emotional disengagement; wishes to end treatment; interpretive enactments to make presence felt in room
Entitlement and demands for special privileges	Hostility, resentment
Critical attitudes toward the therapist	Feeling rejected/deskilled
Playing the therapist off other treaters	Urge to defend the self and retaliate
Feeling mistreated and abused by the therapist	Lack of empathy
Showing sadistic behaviors and attitudes toward the therapist	Fear of the patient's hostility and sadistic attack
Envy of the therapist	Fear of patient's envious attacks, or envy of the patient
Excessive need for admiration from the therapist	Admiration/idealization of the patient
Tendency to think of the therapist as a mirror image of the self or more similar to the self than he or she is	Feels a twinship with the patient, or feels invaded and controlled

Note. Based on Bradley, Heim, and Westen (2005) and Tanzilli, Colli, Muzi, and Lingiardi (2015).

resistances that may sometimes be unconsciously negotiated by patient and therapist (Boesky, 1990; Wilson, 2003). Interestingly enough, these findings reflect the narcissistic (idealizing and devaluing) and paranoid transferences that we observe in the earlier stages of TFP-N, but not the more advanced transferences observed in the later stages that indicate a deepening of the therapeutic relationship and increased integration and maturation of self and object representations. As such, these findings affirm the focus in TFP-N on the importance of ongoing transference analysis and countertransference awareness in the treatment of those with pathological narcissism. The array of transferences we observe clinically over the course of TFP-N treatment indicates that as the

relationship with the therapist evolves, a richer, more nuanced array of self and object representations emerges in the transference and countertransference that bespeak a shift from a narcissistic mode of relating toward a greater mutuality and integration in object relations.

Clinical Illustration

The following clinical example illustrates how these transferences manifest themselves in therapy, as well as the process of interpretation.

Case Example: Olivia

A relatively high-functioning 30-year-old female patient, whom we refer to as Olivia, sought TFP after several unproductive treatments had not helped her to resolve her conflicts about her stalled career as a musician, nor her unsatisfying romantic life. Despite her stated goal of wanting a stable, long-term relationship, Olivia had difficulty forming an enduring commitment to any relationship. She had an on-again, off-again relationship with a fellow music student with whom she shared many interests but whom she devalued as not accomplished or prominent enough. Beautiful and talented, Olivia had no difficulty attracting prominent, affluent, and powerful men, who would become initially enamored of her but then put off by her emotional and financial demands. When she entered TFP she was reeling from a recent breakup with a prominent conductor who had precipitously ended their relationship when he learned that she had charged a large amount of luxury items to his credit card, leaving her feeling devastated, ashamed, unlovable, and unattractive. She had attempted to reconcile with her former boyfriend, but he also was questioning his commitment to the relationship. Like many patients with pathological narcissism, Olivia presented with grandiose features, including some DSM-5 criteria for NPD in that she craved admiration, felt entitled to a certain lifestyle, and exploited her parents and boyfriends to obtain it with minimal recognition or gratitude for what she received. But she also showed characteristics of vulnerable narcissism, evident in her deep sense of shame about her stalled career, her inordinate reactivity to criticism and rejection, her sometimes crippling sense of inadequacy, and her rigid narrative of self-sacrifice and self-defeat.

Olivia was a talented musician who aspired to be a concert pianist, but she found her teachers at the rigorous training program in a prestigious music academy where she studied too critical, the training program too demanding, and the opportunities for performing not commensurate with her ambitions and fantasies of obtaining international acclaim. Her mother was a frustrated musician who had become

a prominent music critic for an international publication and became the sole support of the family when Olivia's father was forced to shut down his business for financial malfeasance and declared bankruptcy. After that, he worked only intermittently, and the family's economic status declined precipitously. Olivia's mother alternately promoted and sabotaged Olivia's career. Olivia both resented her mother for her pressuring attitudes and emotional neglect, but identified with her and felt that she would inevitably wind up re-creating her mother's life. Her sense of angry enmeshment was evident in her statement that the more difficult the mother, the harder to separate from her. By contrast, her dismissing devaluation of her father was evident in Olivia's statement that if her father died, she wouldn't grieve but laugh out loud.

The Early Stage of TFP-N

When Olivia decided to begin TFP, after consulting the therapist several times over a few years, she expressed the desire to understand her failure to have a long-term relationship and to pursue her career in a more sustained manner. The therapist explicated the diagnosis of NPD by suggesting that a chronic pattern of fluctuating between overestimation of her abilities and collapsing into paralysis when she could not meet her own perfectionistic standards, and alternating between idealizing and devaluing others, underpinned her difficulties with work and relationships. Even after agreeing to the contract that stipulated both that she resume some productive activity and speak freely about the problems that brought her to treatment, Olivia continued to wonder whether the treatment would be worth the sacrifice in time and money. She kept the therapist at a distance by focusing on her accomplishments, her active social life, and the various men who pursued her. At other times, she beseeched the therapist to solve immediate problems, seeing her as an idealized figure who could provide magical solutions to her dilemmas. She would often become quite critical or angry when responses to her requests were not forthcoming, and when the therapist interpreted the unmet dependency needs and anger behind such requests, she reacted with contemptuous dismissal, insisting that therapy was a waste of time. It was only after the therapist repeatedly focused on her immediate experience of Olivia as needing to present herself as superior and self-sufficient, and the therapist as inadequate, that the dominant object relational dyad of a grandiose self-sufficient self in relation to an inadequate, rejecting parental figure emerged. As she put it, she had a mother who never told her anything about what to look for in her life or relationships. She felt that she had to raise herself. She then went on to talk about her prowess as a musician and aspirations to become a

renowned soloist and her impatience with her professors who were not acknowledging or helping her to develop the full scope of her talent. The therapist wondered whether this was a transference referent about Olivia's view that the therapist would not be able to help her. The therapist pointed out the contradiction between these two views of herself as emotionally deserted in relation to a neglectful, inattentive other, and as a self-sufficient person needing nothing in relation to an admiring, but distant, other (audience).

Individuals with pathological narcissism often organize in the early stages of therapy around a set narrative that they rigidly control that involves flat, rigid views of self and others and that does not allow for any reflection or alternative point of view. Such narrative may not correspond to the individual's life circumstances or potential, and that distorts crucial aspects of functioning, and/or justifies behaviors that are profoundly self-defeating or self-destructive. In individuals who are thick-skinned, the narcissistic narrative may focus on the specialness of their talents and accomplishments, while in the thin-skinned group, it may take the form of the relentlessness and uniqueness of their suffering. Both were evident in Olivia, who emphasized her self-sufficiency and success, but also her sense of utter despair that she could or would ever be helped. Clarification and confrontation offer the first interpretive route to challenge such repetitions by challenging the rigid set beliefs about the self and others, and to bring into focus the impoverished and distorted representations of self and others that underlie them.

In the early stages of treatment, the TFP therapist routinely identifies a patient's pattern of ascribing to the therapist negative qualities that may, in fact, reflect some distressing aspect of the patient's own history or internal world. The therapist's goal here is to clarify and understand how the patient is experiencing her in the moment, and to respond in a way that makes the patient feel understood and contained (Steiner, 1994). This process of "interpreting the projection" is designed to highlight the patient's immediate experience of the therapist, avoiding a challenge to the patient's vulnerability, and therefore avoiding a provocation of the patient's shame, humiliation, or rage. The TFP therapist strives to keep in mind the psychic pain that may be behind the patient's defensive devaluation.

In the initial month of TFP, Olivia began the session by saying that she had been eager to talk to the therapist about her distress that her former boyfriend, with whom she had hoped to reconcile, was questioning his commitment to her. She requested an additional session that week, but then canceled it at the last minute because it conflicted with a social commitment. When she requested another specific session time, the therapist was unable to accommodate it but offered her another time on the same day, which Olivia rejected. In the following scheduled session,

Olivia expressed considerable anger at the therapist. The therapist wondered whether she was feeling uncared for and abandoned by her, as she was by her boyfriend. She observed that it must be difficult to have a therapist who was unable to adjust her schedule to meet her needs, focusing on the feelings of anger, disappointment, and abandonment that this entailed. Olivia then stated that she had actually called another therapist who agreed to see her at the time she requested, only to cancel that appointment because she decided that any therapist who could see her on such short notice would not be worthy. The therapist also wondered whether Olivia's turning to another therapist just as they had begun their work, as well as her rejection of the session time offered by the new therapist, functioned to limit her investment in the therapy, and thus her capacity to get help, which she had sought intermittently and unsuccessfully for a long time. Perhaps to ask for help was to expose her sense of vulnerability to another who she feared would reject her or not meet her needs as her boyfriend was at that time. The patient's response was to say that as much as she wanted help from the therapist, she kept her at a distance for fear of being disappointed or dismissed. She stated that in painful times she pushes people away and that she wanted her therapist to be like her tax attorney: always there to give her advice, but not that involved. At that point in the countertransference the therapist alternated between feeling aware of the pain that fueled the patient's devaluing attitudes as evidenced by her assertion that when in pain she pushed people away, and feeling frustrated by her pattern of seeking emotional connection and then resisting it. The therapist's focus was on clarifying the feelings of anger, despair, and futility that the patient was experiencing in relation to the therapist and others without linking this to any aspect of the patient's history or internal world.

In the session material above we see how articulating the patient's experience of the therapist (or "interpreting the projection") can function as a first step in defining the specific object relational dyads taking shape in the transference; this process can therefore serve to promote the patient's capacity to see the therapist beyond the projection of the superior self–devalued other dyad. In fact, initially the grandiose self can obscure specific object relational dyads that will later emerge in the transference. In the early stages, the predominant transference relationship is often between a grandiose, superior self and a devalued other (therapist), with the latter reflecting the denigrated and disavowed aspect of the patient's pathological grandiose self, or the therapist is seen as an aspect of the idealized, special self. As these dominant object relational dyads were alternatively evoked in relation to the therapist, Olivia began to recognize her identification with both the grandiose and devalued aspects of the self. She talked about how her mother made her feel special, like she should have whatever she wanted, but also made her

feel like she was not allowed to be successful. This led to an exploration of the real-life costs and consequences of her grandiose beliefs about herself and her persistent self-depreciation. The former was evident in the sense that she was destined for unique greatness and that the professional opportunities available to her were not good enough, and the latter in the sense that she was destined to fail and be left with nothing and no one. The ways these divergent internalized object relational dyads of superior self–inferior other and inadequate self–superior other organized her expectations, and her experiences were clarified and explored.

The Middle Stage of TFP-N: Working with Role Reversals

After several months of TFP and in line with the treatment contract, Olivia resumed her studies and began to perform again, going on concert tours every several months. The therapist accommodated her schedule by conducting sessions electronically when she was out of town but held fast to their agreement that Olivia attend in person the bulk of the twice-weekly sessions when she was not traveling. She said therapy was taking away time from her class and practice schedule even though the therapist made every attempt to accommodate her demanding schedule. At one point, Olivia called the therapist during her session time from a taxi, suggesting they have a session then and there because she had had to schedule another rehearsal for a concert. When the therapist suggested that this was not an optimal condition for a session because there was no confidentiality and offered her another session time the following day, Olivia accused the therapist of trying to control her, of considering only her own schedule, and not the patient's needs.

In the following session, the therapist offered the interpretation that while Olivia saw the therapist as controlling her and herself as subservient, perhaps she herself was attempting to control the treatment by asking for something outside of the agreed-upon structure—to have her session on the phone in a taxi—which she felt was merited despite the contract. Olivia then told the therapist that her boyfriend had recently said that one of the reasons he was questioning his commitment to her was her sense of entitlement, and that he thought no matter what he gave her, she would never be satisfied. Bringing this into the transference, the therapist observed that while Olivia saw her as controlling and rejecting, and herself as deprived and subservient, she herself could behave in a controlling manner, demanding that the therapist submit to her wish to have the session when and where it suited her. The therapist also speculated that perhaps she had been keeping her dependent wishes at bay by not attending sessions in person. Further exploration led to Olivia revealing her wish that the therapist would put her (the patient's) needs

above all else. She became tearful and stated, "I guess my expectation is that others will put me at the center of their world because I never had that—I had the illusion of being special but in reality, I was always left out in the cold."

Olivia then talked about a memory at age 9—when she already showed promise as a musician—of being made to perform for her mother's guests at a luncheon, but then being locked in her room alone for many hours by her mother while she continued to entertain her friends. The therapist then observed that it was no wonder she wanted to control how she had sessions since had been subjected to arbitrary and capricious control of others in the past. This response illustrates how working with role reversals enlarges the person's awareness of split-off aspects of the self and their own experience of which they had been unaware, revealing how unconscious beliefs and representations configure current experience. The recognition that negative qualities that the patient had attributed to others might actually be an aspect of the self is an essential part of the dismantling of the tightly bound knot of idealized and devalued self–other representations that comprise the grandiose self. The sense of vulnerability that emerges as the grandiose self collapses may evoke particular countertransference feelings in the therapist. At this point, the therapist struggled to maintain a stance of concerned neutrality as she alternated between feeling protective of Olivia and pulled to indulge her wish to bend the frame in order to make her feel special, and feeling frustrated by Olivia's capricious, denigrating behavior. With narcissistic patients, maintaining neutrality often means steering a course between confronting grandiosity prematurely or colluding with narcissistic illusions.

As the narcissistic patient begins to appreciate the maladaptive aspects of long-employed narcissistic defenses, there is an increasing awareness of yearnings for a gratifying other on whom the individual can depend—also evident in the foregoing session material. Earlier in therapy, such a positive dyad takes the form of seeing the therapist as a treater who is equal to the patient in his or her perfection and who can magically fix the patient. Such a positive relationship, however, usually cannot contain any longings for dependency or genuine emotional connection, since dependent wishes threaten the narcissistic illusion of omnipotent self-sufficiency. For example, Olivia stated that part of her was scared to get into a relationship with anyone because she had been lied to and betrayed so much. She expressed fears of becoming dependent on the therapist and worried about developing an unhealthy dependency. At this stage, the narcissistic transference evident in the patient's instrumental attitude toward treatment may evolve into a dependent transference characterized by a partially integrated object relation of a

dependent, weak, or devalued self in relation to a gratifying, protective, idealized caregiver. These may alternate with paranoid transferences in which the patient imagines that the therapist or the treatment could be harmful or exploitative.

The Later Stages of Therapy

In the later stages of therapy, Olivia not only attended sessions regularly and on time but also began to come early to sessions, sitting in the therapist's waiting room, using the time to write in her journal. Olivia stated that writing in the journal was her way of bringing the broken part of herself to therapy. She said, "the damaged part of myself doesn't go away completely, but I don't want to be carrying it around anymore. In the past I gave it to my boyfriend, and it was destructive. . . . I exploited him. . . . And the important thing is not to let it be destructive again." The therapist interpreted this behavior as an indication of the emergence of a positive dyad of a nurtured, gratified self in interaction with an accepting, caring parental figure, who fosters rather than impedes her creativity (journal writing). Recording her thoughts and feelings in her journal in the therapist's waiting room also represented an identification with the therapist's reflective capacity, which in turn enhanced the ability to reflect on her own mental states, including those that are destructive and/or painful. As the therapist becomes more securely anchored as a good internal object, strong ties to archaic, powerful, punitive objects are lessened. At this stage, dependency is ameliorated as a more secure attachment state of mind is consolidated. As Olivia put it, "I don't feel like I'm coming from such a needy place because it's been here in therapy, that needy part." She stated that she had become aware of "a lot of feelings" she had not been able to express before, and that seeing the therapist made her feel "calm and secure." Her capacity to more openly disclose and explore her dependent longings, sense of vulnerability, and inner distress are all indicative of more secure states of mind (Bowlby, 1982b; Talia, Miller-Bottome, & Daniel, 2017).

In the later stages of therapy, as narcissistic defenses and conflicts are worked through, persons with pathological narcissism cycle toward genuine, if need-based, intimacy and defensive withdrawal to the grandiose self. For example, as Olivia began to explore her own sense of self as impaired or inadequate, she said that she did not know what to do with the broken part of herself, or how to integrate it. She said, "I know I have to talk more about it, but part of me wants to be taken care of here, and part of me doesn't . . . part of me wants you to know me and part of me doesn't." The therapist wondered whether Olivia feared that talking about the damaged (angry, destructive, envious) part of her might also

harm the therapist whom she had come to value. As the grandiose self is analyzed into its component aspects of devalued and ideal self–other configurations, the narcissistic patient presents as more multifaceted and more related. There is not only a greater tolerance for dependency but also an increased capacity to use the therapist as a secure base to explore problematic aspects of the internal and external world. At this stage, the patient becomes increasingly aware of conflictual aspects of early relationships with primary attachment figures who may have been previously defensively idealized or devalued, which led him or her to take refuge in the grandiose self. Such was the case with Olivia, who recognized how seeking the ideal in herself and others had eclipsed her capacity to develop her own real talents and make deeper, more enduring commitments, which she eventually did. She realized that she had been hampered by her loyalty to a maternal figure, described as like one of the three witches in Macbeth, who tormented her with dark prophesies that she would never be both happy and successful, or if successful, that no man would want her, even while she was made to feel special. She recognized that her previous relationships had been problematic in part because she denigrated and exploited her partners (and the therapist) out of an unconscious identification with a powerful, self-centered, malevolent maternal figure—a central organizer of her grandiose self.

As a more integrated, psychological organization emerges in the later stages of therapy, narcissistic and paranoid transferences give way to dependent and depressive transferences. The latter involve the modulation and integration of loving and hateful feelings toward the therapist and others as whole, complex objects with whom the person is now deeply engaged. The loss of the ideal states previously imagined in the self and others are gradually mourned and relinquished as they are replaced by more realistic, sustaining, and integrated representations of self and others. The guilt associated with gaining awareness of the hostility behind the exploitative and entitled attitudes toward others, including the therapist, must also be acknowledged and worked through. This stage may in fact be quite protracted for narcissistic patients because the insularity of the grandiose self has previously protected them from the experiences of need, desire, loss, or mourning, plunging them into the realities they have avoided in the retreat to grandiosity. Such a depressive transference is indicated in Olivia's gratitude toward the therapist for providing a safe place to express and work through the needy, dependent, angry part of her, but also in her acknowledgment of how destructive this part of her had been in previous relationships and her sense that the important thing was not to let it be destructive again. Hence, in the later stages, we see depressive transferences that center on painful feelings of guilt, remorse, and reparative strivings to redress past

damage to self and others. Depressive transferences are characterized by an object relation of a worthy, gratified self in relation to a caring, if imperfect, object infused with predominantly positive, affiliative, and loving affects that modulate hostile, ambivalent feelings that are modulated, better tolerated, and contained.

The Role of the Relationship Itself

Beyond the activation and enactment of internal object relational dyads in the transference, a real relation develops between patient and therapist. This is sometimes referred to as the therapeutic or helping alliance (Helping Alliance Quesionnaire [HAQ]; Hatcher & Barends, 1996), which involves an emotional bond between patient and therapist based in part on the shared collaborative focus for their work. The therapeutic alliance enlists the observing part of the patient that understands and accepts the reality of the therapeutic situation, and involves accepting help from another, who with further insight and understanding of the conflicts underlying the patient's difficulties, provides his or her best efforts to work toward jointly agreed-upon therapeutic goals. Ronningstam (2012, 2018) has emphasized the importance of building and maintaining such an alliance through collaborative goal setting, which gives the patient a sense of mastery and agency, thereby lessening the shame proneness, fear of dependency, and exposure that makes narcissistic patients so difficult to treat. Also contributing to the difficulties with developing and maintaining an alliance with patients with narcissistic pathology are the patterns of insecure (particularly dismissing) and disorganized attachment.

It has been proposed that another aspect of the real relationship between patient and therapist is the attachment bond that develops when a person seeks help from a concerned and caring person who provides a secure base for exploration of the internal and external world (Diamond et al., 2003; Eagle, 2003; Parish & Eagle, 2003; Mitchell, 1986). Attachment dynamics have now been associated with a number of aspects of the psychotherapy process, including the therapeutic alliance (Kanninen, Salo, & Punamäki, 2000), transference–countertransference dynamics (Diamond et al., 2013), and aspects of the in-session discourse characteristics, including the ways that patients regulate and express affect and emotional proximity in relation to the therapist (Talia et al., 2017). But in those with personality disorders, including narcissistic disorder, ordinary communication is distorted and there are difficulties using the therapist as a secure base. Narcissistic patients who are "difficult to reach" and treatment resistant may actually be relying on learned insecure and disorganized attachment patterns that have helped them to preserve a sense of safety and agency (see Chapters 3 and 9, respectively,

for descriptions of attachment classifications and the way they manifest themselves in therapy). Therapists should be alert to the ways these maladaptive attachment patterns function to maintain proximity to others often in the face of rejection, loss, or inconsistent caregiving. In patients with personality disorders, including NPD, these attachment patterns are often multiple and likely to change in the course of treatment—thus, therapists should be attuned to the way these patterns manifest themselves in the ebb and flow of a patient's discourse and the patient–therapist interaction, and adjust their interventions accordingly.

Combining TFP-N with Other Interventions

TFP-N has the ultimate goal of achieving internal psychological change, with related changes in mood stability, object relations, interpersonal functioning in work/career, and capacity to love. While elements of TFP, such as the treatment contract, have a powerful impact on helping the patient control problematic behaviors and symptoms, there may be particular situations in which TFP-N can be combined with supportive, informative, and skill-enhancing treatment approaches carried out by auxiliary therapists (see Koenigsberg et al., 2000, chapter 13). Specific examples of appropriate ancillary treatments include substance abuse and/or 12-Step programs, nutritional counseling, medical consultation, skills training, couple treatment, and for the lowest-functioning patients, day hospital programs. In our experience, it is increasingly common that patients who have benefited symptomatically from more cognitive-behavioral treatments, such as dialectical behavior therapy (DBT), may then seek TFP-N to explore their identity, interpersonal issues, and engagement in life issues in more depth. Conversely, more severely disturbed narcissistic patients who have not been able to benefit from DBT or cognitive-behavioral therapy (CBT), even though they might benefit from techniques for symptom control or life skills training because they find such concrete approaches to be beneath them, may be able to avail themselves of such approaches, if needed, after a course of TFP-N.

TFP-N can be combined with other treatment interventions, including medication for specific symptom constellations and more behavioral treatments for specific symptomatic behaviors, such as eating disorders, substance abuse, or skill deficits (see Koenigsberg et al., 2000, chapter 13). The issue of medicating individuals with pathological narcissism and NPD is based on clinical experience and on applying what has been learned from research on the "near-neighbor" condition of BPD. Certain principles and cautions apply: First, a focus on medication can distract from psychological exploration. Second, the patient's

reaction to the medication can have psychological meanings. In "split treatments," the psychopharmacologist may be viewed by the patient as the "good object" who offers potential, and perhaps magical, help, with fewer expectations of the patient. In other patients with a more paranoid disposition, the medication may be experienced as a harmful substance that has injured the patient or as a means of control. Third, the therapist's attitude toward medication may reflect countertransference developments. The therapist may turn to medication if he or she is feeling hopeless or impotent in relation to the patient. Or, in other cases, the therapist may turn to medication if he or she is feeling uncomfortable with the level of affective intensity in the sessions or feels a need to control the patient.

Summary and Conclusion

The techniques of free association, interpretation, transference analysis, countertransference use, technical neutrality, attention to both the internal world and the external reality, and the role of the attachment relationship between the patient and the therapist, as described above, are crucial tactics and techniques of TFP-N that have been tailored to meet the challenges of working with individuals with a spectrum of narcissistic pathology. In previous manuals (Yeomans et al., 2015; Caligor et al., 2018), the specific tactics and techniques of TFP that are applicable to those with a range of personality disorders are spelled out in greater detail. Here we focused on how we adapted these tactics and techniques in TFP-N for those with pathological narcissism across the spectrum of functioning.

PART II

TREATING PATHOLOGICAL NARCISSISM

CHAPTER 5

The Assessment and Diagnosis of Pathological Narcissism and NPD

The growing interest in the conceptualization and treatment of pathological narcissism has spawned a myriad of assessment tools focused on narcissism and NPD (Campbell, Bonacci, Shelton, Exline, & Bushman, 2004; Hendin & Cheek, 1997; Pincus et al., 2009; Raskin & Terry, 1988; Rosenthal, Hooley, & Steshenko, 2007; Westen & Shedler, 1999). In this chapter, we address the clinical manifestations of the disturbances of self- and object representations that we view as central to the diagnosis of NPD, what we typically see in the assessment process that confirms our sense of the diagnosis, and what questions we might ask to reveal the same. Although there have been several reviews related but not limited to the assessment of pathological narcissism (e.g., Caligor, Levy, & Yeomans, 2015; PDM Task Force, 2006; Pincus & Lukowitsky, 2010), our goal in this chapter is to provide an overview of our method for the assessment of the descriptive and structural features of pathological narcissism in the clinical situation.

Structural Diagnosis and the Clinical Assessment of Pathological Narcissism and NPD

The clinical assessment of pathological narcissism and NPD involves a careful assessment of DSM-5 criteria for NPD, with particular attention to the core features and manifestations of narcissistic grandiosity. In addition to a determination of the patient's phenomenological diagnosis,

the therapist must assess the patient's overall level of personality organization and, if the patient is in the borderline range, determine the level of narcissistic pathology within the borderline range. Extending this point, we note at the start of this chapter what one might term the *two poles of narcissism*—that is, core vulnerability, masked or expressed through defensive grandiosity, and the associated complication of assessing narcissism in both of these manifestations. Thus, in discussing the clinical assessment of narcissistic pathology, one must be careful to clarify the boundaries of the term (pathological or healthy narcissism, covert or overt manifestations, etc.) and the ways in which each assessment tool covers the breadth of the narcissism construct as defined (Wright, 2016).

In our general clinical approach, we explicitly orient all patients who come for treatment to the assessment/consultation process, noting that we spend one or two sessions on the objective of learning, in as much detail as possible, the nature of the patient's difficulties (cognitive, emotional, and interpersonal) and the patient's sense of what he or she would like to accomplish in the treatment. We inform patients that after these initial sessions, we will spend an additional session in which our clinical findings, our diagnosis, and understanding of the patient's difficulties are shared. We also note that at this time, after our diagnosis and formulation are shared, we discuss with the patient the type of therapy we recommend and the conditions under which we feel the treatment needs to be structured to allow for a successful process to unfold (see Chapter 6 on the treatment contract).

Our assessment approach combines evaluation of both DSM-5 diagnostic categorical and dimensional criteria, and exploration of the patient's level of organization. This approach has been elaborated over several decades and is based on the "structural interview," a clinical interviewing procedure that assesses the patient's symptomatology and cognitive and emotional functioning in the broader context of personality (i.e., developmental level of representations of self and other, quality of object relations, defensive functioning, ethical values, and functional capacity/impairment; Kernberg, 1984). The interview was designed to provide a full evaluation of these features while also providing an *in vivo* test of the patient's object relations, defensive system, and reality testing, achieved by confrontations of patient responses that appear to contradict other parts of the interview or are internally inconsistent, and/or trial interpretations. To this end, clinicians first ask patients to describe why they are seeking therapy, how they understand their problems, and what they expect from therapy. However, the heart of the interview involves asking patients to describe themselves and significant others in their lives in order to obtain a sense of the extent to which they can provide a view of the self and others that is integrated, coherent, multifaceted, and complex versus one-dimensional, polarized, fragmented, or vague.

Patients at lower levels of personality organization may respond to these challenges with anxiety, hostility, paranoia, and/or a marked decrease in the coherence of their narrative, suggesting either deficits in reality testing or, in less severe instances, anxiety that emerges as splitting processes are challenged. Katie, a 28-year-old woman with narcissistic pathology organized at the borderline level, first described her boyfriend of 4 years as the "love of my life" in idealized terms, listing his great looks, sense of style, and confidence. When pressed to elaborate further as to what she loved about him, what made him special to her, and unique among other attractive, confident men in the world, Katie was at a loss. She quickly became enraged with the interviewer, accusing him not only of doubting her love for her boyfriend but of attempting to demean her by even asking the question. In contrast, patients at higher levels within the narcissistic range respond to such challenges perhaps with anxiety, but while retaining the ability to think and respond to the interviewer in ways that either express curiosity about their response or clarify and elaborate the conflictual material being discussed. For example, when Tom, a 44-year-old man, presented for individual treatment pursuant to having been caught having an affair, he had great difficulty describing any significant person in his life, including his spouse, or lover, with any depth or nuance. This was all the more surprising given his report of a troubled marriage, one that contributed to his pursuing an affair! When his difficulty thinking and talking about others was raised with him, Tom was at a loss, curious as to why this was such a struggle for him. This confrontation, rather than serving as a point of disjuncture in the early treatment process, became the starting point for an extended discussion of Tom's defensive style, his experience of significant others, and the relation of these two features to his broader difficulties.

The narcissistic structure and the operation of primitive defenses makes it very difficult to obtain a clear picture of the patient's self- and object representations, and of the quality of his or her engagement with external reality (work, recreation, romantic/sexual relations). One often must obtain detailed information from collateral sources (individuals close to the patient, the referral source) to begin to clarify the extent of the patient's entitlement; need for admiration, attention, and special treatment; the patient's difficulties with empathy; and the experience others have of the patient in the interpersonal context. When such information is not readily available, one often develops a sense during the evaluation process and through the developing patient–therapist relationship of these same features becoming manifest, whether it takes the form of requests for unusual, idiosyncratic changes to the standard evaluation process (session length, payment procedures), difficulty answering the therapist's questions or allowing the therapist to direct the interview,

or contemptuous or denigrating responses to the therapist's conduct of the evaluation. At this early stage, the therapist may sense being under the hostile, omnipotent control of someone who is threatened by close contact with an independent other (such as the therapist), one who if not controlled could, in the patient's view, evoke the negative affects or experiences of self, such as devaluation, aggression, or envy, that the patient is denying and projecting. One can also begin to formulate a diagnosis as, throughout the interview procedure, one notices descriptions of self and others that are overly idealized or depreciatory, lacking depth and differentiation, or largely self-referential.

Assessment of NPD through the Structured Interview of Personality Organization—Revised

The Structured Interview of Personality Organization—Revised (STIPO-R; Clarkin, Caligor, Stern, & Kernberg, 2016; Stern et al., 2010), was developed for research and clinical purposes to provide an organized and structured format for the content typically covered in Kernberg's (1984) structural interview. The STIPO-R, which is accessible in full at *ISTFP.org*, is scored based on interviewer ratings of each of 55 items, and scale scores can be computed for each of five content domains:

1. Identity (subscales: sense of self, capacity to invest)
2. Quality of Object Relations (subscales: friendships, narcissistic object relations, sexual and intimate relations)
3. Defenses (subscales: primitive defenses, coping and rigidity)
4. Aggression (subscales: self-directed, other-directed)
5. Moral Values

A Narcissism scale can be computed from various items in the Identity and Quality of Object Relations domains, and from items elsewhere in the interview, including items related to sense of self, grandiosity, capacity to invest the self in work and recreation, self-esteem regulation, and narcissistic object relations. These are the items that are essential for making the diagnosis of NPD.

Assessment of Identity

The STIPO-R begins with an assessment of two aspects of identity—namely, (1) the patient's capacity to invest in his or her primary role (work/school), and in a serious way in recreational pursuits; and (2) the patient's sense of self. The assessment of the patient's capacity to invest in his or her primary role and recreational pursuits is clearly external,

as opposed to intrapsychic. Manifestation of narcissistic pathology reflects the patient's need to use the activities primarily for narcissistic self-enhancement, rather than the ability to derive intrinsic gratification from the activity, which would correlate with more stable, rich, and gratifying involvement with work and recreation over time. Starting the interview with the assessment of these concrete features of the patient's life allows for a softer approach to more sensitive internal experiences, while also tapping a sphere of life that for many of our patients is not entirely "conflict-free," thus pointing directly to the concerns about identity and defensive style that follow.

The assessment of the *capacity to invest* includes questions related to the patient's effectiveness in his or her primary role, and the consistency in his or her engagement in the same over time. The specific question that is scored on the STIPO-R Narcissism scale pertains to the patient's capacity to derive internal satisfaction from engagement in his or her primary role:

Sample Narcissism Item 1: Primary Role Satisfaction

Do you enjoy working?
Does working give you a sense of satisfaction and accomplishment?
Does working give you a sense of pride and satisfaction by just "doing a job well"?

Upon completion of these items, the interviewer introduces a series of questions related to the patient's *sense of self*. This section of the interview beings with a series of probes designed to elicit an open-ended self-description. The probes are as follows:

Tell me about yourself. What are you like as a person? Let's say that you wanted me to get to know you as quickly as possible, in just a few minutes—how would you describe yourself to me so that I get a full of picture of the kind of person you are?
Is there anything else you can tell me about what is most characteristic of you, about your essence as a person?

If respondent provides . . .

1. A *list of adjectives*, respond thus: "You've used several adjectives to describe yourself. I'm wondering whether you could fill in your description a bit, perhaps bringing it to life with an example or story that illustrates some of those qualities." [Note: Elicit *two* examples, as needed.]
2. A *superficial description*, inquire about one or more of the

adjectives or qualities offered, and ask whether the respondent can describe those qualities in greater detail.

The goal of this exercise is to get a sense of whether the description is (1) realistic, having depth and nuance, as opposed to offering a superficial, caricatured description; and (2) integrated—that is, whether the patient can present a balanced view of his or her positive and negative qualities, or whether the description tends toward split idealization or devaluation. Indeed, one can often begin to formulate a diagnosis as one notices descriptions of self and others that are overly idealized or depreciatory, lacking depth and differentiation, or largely self-referential, with themes of self in relation to others across relationships that are typical of narcissistic personalities (i.e., others who frequently disappoint or fall short of expectations, or of others who fail to sufficiently admire or accommodate the patient). At a later point in the interview, an item with the identical probes but for a different target—namely, the "most important person in the patient's life at present"—is administered. This item can be repeated for other individuals in the patient's life according to the interviewer's sense of the need to further clarify the quality of the patient's object relations. Notable with narcissistic patients is the discrepancy between the reports of self and other. Although many patients in the borderline range *without* narcissistic pathology provide superficial, split, caricatured descriptions of self *and* significant others, it is not uncommon for narcissistic patients to describe the self in ways that, albeit somewhat idealized, appear to reflect some consolidated sense of self and perhaps even some reflective capacity. In contrast, however, descriptions of others, for many narcissistic patients, lack depth, nuance, and a realistic sense of integration.

After finishing this item, the interviewer proceeds to further assess the patient's sense of self with a series of questions related to the sense of coherence and continuity in the patient's sense of self. Questions assessing the stability in the sense of self across various people in the patient's life and across various situations; consistency in tastes, opinions, and preferences; and the stability in the patient's sense of self in the context of intimate relations are included in the general assessment of identity, but are *not* included in the Narcissism scale. These standard questions for the assessment of the patient's sense of self and identity (i.e., consistency in the sense of self over time and across situations, stability in self-representations, open-ended descriptions of one's sense of self) may prove less useful in the assessment of narcissistic pathology, as the grandiose self frequently functions as an intact shell covering the fragmented self underneath. The exceptions to this are some cases of NPD in which the patient is functioning at an overtly borderline level

(i.e., poor anxiety tolerance, poor impulse control, and compromised capacity for sublimating impulses into prosocial activity). In such cases, it is not uncommon for patients to respond to these items concerning the sense of self in ways that reflect the fragmentation and instability that a more robust narcissistic system protects against.

Specific items keyed to the Narcissism scale included in the assessment of the patient's sense of self include the patient's need for admiration, stability, and valence of self-esteem:

Sample Narcissism Item 2: Need for Admiration

Although everyone appreciates being admired, would you say that other people's approval, admiration, and positive attention is very important to you?

Do you find yourself actively seeking out such admiration or attention?

Do you find yourself feeling empty or down when you are not receiving that attention or admiration?

Sample Narcissism Item 3: Self-Esteem

Would you say that there are significant swings or shifts in your sense of self-esteem, or that it is pretty stable?

Would you say that your self-esteem alternates between at times seeing yourself as special and wonderful, and at other times as small and defective?

Does your self-esteem depend a lot on how you are seen by others, or would you say that your self-esteem comes from inside of you?

Do you compare yourself to others a lot?

Assessment of Narcissistic Object Relations

In addition to the items listed above that assess narcissism primarily in terms of the sense of self, the STIPO-R includes several items that assess narcissism through the lens of the patient's object relations (i.e., how the patient relates to others as a means of narcissistic self-regulation). Included among these items are the following:

Sample Narcissism Item 4: Self-Centeredness

Have others told you that you are very difficult to have a conversation with, that you do not appear to be listening to them, or that you seem to be paying attention to something else?

Have people told you that in conversations you tend to turn things around to discuss only yourself, or what *you* are interested in?

Sample Narcissism Item 5: Envy

When others succeed or accomplish something, do you tend to feel badly about yourself?

Is it painful to you when others have thoughts or ideas you feel *you* should have thought or expressed first?

Are there some people who are bothersome to you because even though they are not better or worse than you, or perhaps someone who is not as good as you, they are more successful, admired, or prominent?

Do you find yourself sometimes secretly wishing for certain people close to you to fail, even though you may feel guilty about it, because their success would be painful to you?

Sample Narcissism Item 6: Scorekeeping

In close relationships, do you find yourself "keeping score," thinking about how many times your friend or partner did this or that thing, or figuring out whose turn it is to do this or that?

Do you spend a lot of time thinking about or trying to figure out who is getting more out of the relationship?

Do you frequently have the experience of feeling taken advantage of, like you are not getting enough back from your friends and/or romantic partners?

Is it important to you that things be equal in your friendships and romantic relationships, or perhaps that you are getting more out of the relationship than your partner?

Sample Narcissism Item 7: Empathy

Do you find it difficult to be supportive, or to comfort friends or family members who are in emotional pain?

Do people close to you complain that they cannot count on you to be supportive, or that you do not seem to understand what they are going through?

Sample Narcissism Item 8: Idealization/Devaluation

Does it happen to you that you idealize extraordinary people you know, putting them up on a pedestal or expecting a lot from them, only to realize or feel, after a while, very disappointed or let down, finding fault with or criticizing them?

In relationships *other than* your romantic relationships, do you tend to look up to people, to put them on a pedestal?

Are there people who you would say that you idealize, who you hold in an unrealistically high regard?
And does the same pattern apply, where you eventually become quite disappointed in, or critical of them?

Sample Narcissism Item 9: Boredom

Do you find that your relationships become boring in the long run, or that you tend to lose interest in your friends or romantic partners over time?
Are you the kind of person who goes out with someone for a while, loses interest in him or her, and then moves on to the next person, only to have the same thing happen again?

In making a diagnosis of NPD based on the foregoing items related to sense of self, the capacity to invest the self in work and recreation, grandiosity, and narcissistic object relations, there are two concluding points worth noting. First, in reviewing the totality of items from the STIPO-R that contribute to a diagnosis of NPD, one must take into account that this universe of items, like the DSM criteria set, is essentially polythetic (i.e., not every person diagnosed with NPD needs to respond affirmatively to every item in order to receive the diagnosis). As a result, patterns of scores from these items can be used to identify subgroups of narcissistic patients—for example: high-functioning narcissistic patients demonstrating a capacity to invest in work and recreation, and a stable sense of self, along with profound deficits in their representation of others, in the nature of their object relations, and in their intimate and sexual relations. In addition, narcissistic patients functioning on an overtly borderline level, with significant instability in their experience of self over time, have difficulties investing in their primary role and recreation, as well as significant difficulties in their object relations. Second, in addition to these items, which form the core of the NPD diagnosis, other items from the Aggression and Moral Values domains, items related to object-related aggression, exploitation of others, lying and criminality, and the capacity for guilt and remorse, speak to the severity of pathology within the spectrum of NPD.

The Structural Interview: Case Examples

The following case examples summarize the results of structural interviews with two different types of narcissistic personalities.

Case Example 1

Suzie is a 28-year-old aspiring pastry chef. She was referred to me (B.L.S.) by her therapist of 2 years who had discharged Suzie due to the therapist's sense that she could not effectively manage the patient's increasing demands for time, attention, and extra session contact, nor help the patient to contain or understand her rage when the patient's demands were not met. At our first meeting, Suzie described a profound sense of emptiness and fears of abandonment, and a recent history of impulsive self-injury, drug abuse, and hypersexuality. She had engaged in destructive responses to a variety of disappointments, including feelings of inadequacy in relation to her peers who had jobs and boyfriends; unreciprocated social overtures with her friends; rejection by men in whom she had expressed interest; and feelings of rejection and shame resulting from what she experienced as failed bids for emotional support from her father.

Although Suzie described having a consistent interest over several years in the food industry, and in the pastry arts more specifically, her ability to persist and perform in school and in jobs was severely compromised, as she failed to complete tasks and avoided work that she experienced as challenging or that involved necessary skills at which she was less adept. These difficulties resulted in poor grades, an inability to find paid work in her field, and her complete inability to direct herself on her own initiative to grow her skills. Suzie described no significant activities or interest outside of her work. Her pattern of avoidance and inconsistent engagement in school and work suggested a significant gap between her ego ideal and her actual accomplishments, as well as her efforts to avoid acknowledgment of this gap and to get others to collude in doing the same. Furthermore, the difficulties investing the self in one's primary role and in recreation, as in the case of Suzie, suggest difficulties in *valuing* (i.e., things and activities; people are valued only for the short-term gratifications they provide, rather than for the more sustained, intrinsic pleasures and value they provide for a more established sense of self).

Suzie's sense of self was unintegrated and superficial. When asked to describe herself, she had great difficulty with the open-ended nature of the question, stating that she needed me to direct her. When prompted, her description focused on attributes related to her appearance and the self-image she aspired to (glamorous, accomplished, admired), while both recognizing and dismissing her awareness that her accomplishments did not live up to this desired self. When prompted to explain adjectives such as "funny in a different kind of way" and "angry," she had difficulty elaborating with examples, or in any way that gave a realistic sense of how these qualities applied to her. The patient's description

of her best friend was similarly superficial: "He's rich, successful, he's a doctor, he has his shit together and I like that in a person . . . and he is really nice to me, he cares about me." This description, and her failure to elaborate on these qualities and why they were meaningful to her, highlight that she was drawn to this friend because of idealized qualities that he embodied (his wealth, profession), and the self-referential nature of her object relations—that is, that others are valued because of the narcissistic needs they fulfill (he is really nice, he cares).

In describing her sense of self, Suzie described quite explicitly craving admiration from others, and working to present a self-image that would attract the attention and admiration of others, while fully realizing that the image was superficial and hollow. Suzie deliberately hid areas of deficit and weakness from her mentors while in training to enhance her image in the eyes of others who are competent and skilled, much more so than she knew she was. She described the "high" she got when more well-established colleagues spoke to her vaguely of glamorous opportunities for work in the culinary centers of Europe, or of the enlivened, thrilling feeling she got when a man demonstrated sexual interest in her. These fulfilled feelings associated with admiration received or promised, oscillated with powerful feelings of aloneness and inferiority when the man's attention waned after a brief sexual encounter, or the promised professional opportunities never came to pass.

Similarly, Suzie described the fragility of her feeling competent and special in her talents, how those feelings collapsed quickly when she struggled to complete work on her own, or experienced rejection from a job that she already felt to be unsuitably beneath the level of her self-perceived ability, and how she frequently would retreat to these grandiose fantasies in the face of such disappointments. She described fantasies of success and wealth, in which she would earn the admiration of those who doubted her, and of others who, with lesser talent, have nevertheless exceeded her in accomplishment. For example, while working on an unpaid internship, Suzie would often express her frustration: "I'm stuck doing minor decorations, simple birthday cakes. . . . I'll spend a whole day doing these minor tasks, while others are developing their own recipes, innovating, catering large affairs with diverse offerings; it's bullshit, I'm so much more than that!" Or, "Today I spent the whole day researching high-end brands of icing; *really*??!! I'm sorry, that's just not what I should be doing. I should just go to Paris; there they know how to recognize what people can do, I wouldn't have to do all this shit work . . . they wouldn't know me, and I could get a fresh start where they would appreciate my work and ability." Suzie had become enraged at her previous therapist when she had gently raised the fact that Suzie had not actually completed the final project for her advanced course of

study, nor had she earned any significant jobs or opportunities of her own to date. In raising this, I, too, was seen as a critical, unsupportive, cruel doubter, as yet another who failed to recognize her talent.

In response to feeling so demeaned and unappreciated, Suzie would often express, with a blithe superiority, her fantasy of triumph. One day, after a weekend of long, hard work, which she experienced as demeaning, Suzie began a session stating, "I think I'm going to move to Paris at the end of next month. I can get hired by a pastry shop there. Things are not as formal there . . . it's so hierarchical here in New York, you've got to do the grunt work for so long before they let you do anything . . . and I know that when Jane [her boss] sees my work when she visits Paris, she'll be shocked at what I can do . . . and she'll just have missed out." In this example, we can see the shift to grandiosity as a means of denying reality and as a related defense against feelings of inferiority, shame, and humiliation.

Suzie described "never catching a break," feeling that others received mentorship and attention that she too deserved, and a sense that her past suffering in a broken family entitled her to special treatment and accommodation in her present life. She was quick to feel insulted and angry with me as she was when her previous therapist had questioned this narrative, noting her father's long-standing and unquestioning financial support of her, her rejection of the offers of support and mentorship from instructors and supervisors, and her destruction of other opportunities—for example, by routinely having missed classes and failing to accept additional opportunities to work when those opportunities did not feel sufficiently glamorous, high-end, or narcissistically rewarding. Suzie's *lack of concern* over her own negligence, her mistreatment of others and opportunities, and her financial exploitation of her father, while shirking the responsibility associated with her educational opportunities, are evidence of deficits in her moral functioning consistent with narcissistic pathology. Further evidence of compromised superego functioning was expressed through her unabashed use of threats of hurting herself to express her anger and to control friends and romantic/sexual partners, and the complete absence of guilt related to these behaviors.

Suzie's experience of romantic and sexual relationships was characterized by superficial encounters in which she reported hiding her true self, fearing that her partners would discover her lack of talent and accomplishment, her neediness, and her rage. She reported a series of relationships ending with her being left by her partners, her rage at them for abandoning her, and protracted periods of intense volatility and conflict involved with the breakup, including explicit attempts to hurt her partners by lying about them to others, and threatening to hurt herself in response to their rejection of her. Similarly, Suzie described idealizing

those who held out the promise of support, caring, and attention, and her rage and desire to strike back at those who withheld, who frustrated her wish for the same. Suzie's sense of concern about her aggression, along with any sense of remorse or regret over her behavior, was markedly absent. Her difficulties in social and intimate relationships stem from her difficulty in bridging the gap between her idealized self-image and the reality of her life (i.e., paucity of relationships and accomplishments). As a result, relationships came to serve the function of supporting the defensive processes associated with managing that gap (i.e., providing affirmation of her good qualities and brittle optimism about her prospects, while fulfilling her need to minimize all that was difficult about or deficient in her). When partners failed to fulfill her needs in this way, the volatility and desperate drama involving abandonment and rage ensued. More succinctly, Suzie's narcissistic needs related to a diminished self-representation and compensatory grandiosity, resulting in severe limitations in her capacity to participate in reciprocal, loving, intimate relationships.

Last, Suzie was, at the time of the evaluation, being supported by her retiree father. She had, despite her father's urging, not looked for part-time work to help defray her expenses and balked at my suggestion that her getting some paid work, even if the work was not in her field or below the level of her aspiration, would be an essential part of our treatment agreement. In sum, Suzie was diagnosed with NPD, with functioning at an overt borderline level and with prominent ego-syntonic aggression directed toward self and others, and compromised superego functioning. For individuals such as Suzie, the establishment of a clearly delineated treatment contract is particularly important in order to minimize the risks to both patient and therapist, and to ensure the conditions for a viable treatment. Procedures for treating contracting in cases such as these are reviewed in Chapter 6.

Case Example 2

Greg is a 42-year-old man referred to me (B.L.S.) by his couple therapist. Greg and his girlfriend of 13 years presented for couple treatment pursuant to it having been revealed that Greg had for some time been soliciting sexual encounters via the Internet with various women while he was away on business trips. The couple therapist referred Greg to me to address long-standing difficulties related to self-esteem and motivation that were also manifesting in Greg's professional life. Upon evaluation, Greg stated that he loved his girlfriend and that he was afraid to lose the relationship because of his behavior. He described quite clearly the inflated sense of self-worth he derived from his brief sexual encounters

with the random women he solicited online. He stated that these encounters provided one place in his life where he can still "feel like a good guy" and "exceed expectations." This last phrase was explained by his stating that someone of his intelligence, bearing, and academic background exceeded the expectations of these women, whom he imagined were used to being solicited by men of lower social status who treated them with less respect. Greg was able to recognize, in response to my confrontation, the irony in this belief—namely, that the sexual activities involved with these women were explicitly demeaning toward them, as was the betrayal of the woman he genuinely understood, valued, and cared for. His ability to acknowledge and work with the contradiction between his own demeaning behaviors toward women and his critique of other men for their demeaning attitudes was a good prognostic sign, as it showed some capacity to reflect on his own attitudes and behaviors, and to acknowledge and work with conflict.

Greg stated, about his life in general, "I constantly get in my own way, never seem to be able to finish things, commit to things . . . I'm always keeping my options open, and never end up getting anywhere, only feeling frustrated and with others feeling frustrated with me." Greg was trained as a structural engineer and for several years attempted to run his own firm in which he consulted on construction projects. He reported several semesters in graduate school during which he functioned extremely well, was motivated, creative, and generative, each followed by periods of withdrawal and stagnancy, leaving himself feeling depressed and his mentors puzzled and frustrated by his inconsistent engagement. As a professional, he described each project as a tortured experience, delaying decisions, avoiding deadlines until confronted by clients or contractors, delaying billing as a result of the subsequent guilt, and fearing the anger of clients and the sense of disappointment resulting from what was a rushed and poorly elaborated project. Despite this experience and these fears, Greg reported receiving positive feedback from his clients, who reported satisfaction with the work and referred him to other clients. The patient even reported feeling pride and satisfaction on several of the projects, while noting wistfully what they could have been had he been more focused and disciplined. Although Greg's area of interest and investment was stable, his functioning, satisfaction, and ambivalence about his professional goals were all demonstrations of his difficulty investing himself in his chosen profession.

Greg's interest outside of work involved the restoration of antique mechanical devices—for example, clocks and cars. He was fascinated with things that were old and discarded, and would lavish upon them significant care and attention. Greg acknowledged a personal sense of sadness, along with deep pleasure, associated with this interest, and linked

these feelings to aspects of his own history, which involved a family legacy of faded glory and an ambivalently held wish of his to restore a legacy that was tarnished, along with a sense of being woefully inadequate for the task. Greg's self-description was nuanced, reflective, and balanced between realistic appraisals of his positive qualities and strengths, as well as his limitations, which dominated his conscious experience. Likewise, his description of his girlfriend reflected an appreciation and understanding of her, a seemingly realistic appraisal of her positive and negative qualities, and deep understanding, when prompted, of how her limitations resulted in an experience in which critical needs of his were not met (i.e., narcissistic needs to be admired, inflated, and motivated by someone else's high expectations, confidence, and accomplishments). In contrast, when asked to describe his closest friend, someone with whom he had grown up and continued to be in contact, Greg struggled: "He's great, the kind of guy, you know, we just have so much history." After a pause, he continued: "It's hard to describe someone you've known your whole life, he'd always be there for you, the kind of guy that everyone loves. I don't know what else to say." When I asked if there was anything else he wanted to share about his friend, or a specific example that could bring the description to life, Greg was able to provide a few more adjectives to describe him, but could not summon an example or shared experience that might characterize his experience of his friend.

Although Greg's descriptions of self and other appeared to be integrated and nuanced, Greg clearly described his painful experience of the gap between his appraisal of who he ought to be and his actual level of accomplishment. Furthermore, alongside what seemed to be a genuine knowledge and deep appreciation of his girlfriend, he also acknowledged his frustration with the fact that she did not help him to bridge the gap between the self he wanted to be (accomplished, artistic, bold, creative) and the shame-prone, avoidant, depleted self that was his conscious self-experience. Said differently, his girlfriend, seen realistically for who she was, frustrated his desire to be with an idealized partner, who, by association, would help him to feel elevated and fill him with the drive to accomplish and succeed to the level of his grandiose aspiration.

Greg's subjective sense of self-esteem was largely low; he was highly self-critical, and self-loathing dominated his daily self-experience. At the same time, he had a high opinion of his capability, and looked down upon engineers functioning as technocrats—those who simply used automated programs to sketch—without considering the artistry of a problem creatively solved. At times Greg was quite explicit that his disdain for the technical aspects, and his lack of facility with the same, resulted in avoidance of his failure to take on work that might advance his career. He described losing interest in projects once the work came

to involve details, issues about construction aspects of the work that he devalued in relation to the more visionary, larger-scale design elements. Although he did not fantasize about success, wealth, or power, he felt himself capable and destined for greater accomplishments, although he feared his capacity to undermine those possibilities, as well as the realistic limitations of his talent.

Greg did not explicitly endorse a need to be admired, or feelings of deflation, when his desire to be admired or respected went unfulfilled. He did, however, acknowledge this desire in the domain of his illicit sexual activities, noting a feeling of inflated esteem while noticing himself behaving "like a gentleman" with the women whom he solicited—again, his sense that he was "giving" these women a better experience than someone else would, and gaining their admiration in doing so. This suggested to me a covert desire for greater recognition and admiration, despite feeling at another level that such admiration had not been earned. Greg admitted to feeling fraudulent and linked his fear of being discovered as incompetent and unskilled to his avoidance of more ambitious work projects.

Greg held numerous grudges and grievances; he felt mistreated by those who in business seek a financial edge and had a sense that his "good-natured" and collaborative spirit resulted in him getting taken advantage of by others. It seemed to me that this reflected a masochistic and narcissistic tendency, and he was nursing a grievance initiated by his own behavior, one function of which was to give voice to his sense of being special and to be recognized. Greg did not describe feelings of entitlement or envy, nor did he elaborate a sense of keeping score in his relationships, a tendency to value others according to what they gave him, or a tendency to withhold when he felt he was not getting his due. He reported several long-lasting friendships that appeared to have considerable depth and value to him. Greg's issues related to self-esteem and his explicit desire for a partner whose presence would allow him to feel better about himself, which resulted in profound difficulties in his romantic and sexual relationship, particularly in his lack of sexual fulfillment and desire with his partner. These deficits in self- and interpersonal functioning led to a primary diagnosis of NPD. Although Greg's narcissistic difficulties were in the context of self and object representations that were to a large extent integrated and differentiated, the management of his self-esteem was also characterized by primitive defensive operations, resulting in some split-off self-representations and powerful wishes that were consciously denied while unconsciously influencing his behavior. The absence of significant overt aggression directed at self and others, and the presence of some concern over his aggression and the damage it was causing to himself and his relationships, were positive prognostic signs as we agreed to begin TFP.

Assessment Considerations Related to Levels of Narcissistic Psychopathology

As noted in Chapters 1 and 2, narcissistic disorder encompasses a spectrum of pathology from higher functioning to lower (borderline) levels of functioning, with narcissistic features manifesting in different ways across this spectrum. Furthermore, patients with prominent narcissistic features can also be found in the neurotic range—even though these patients do not meet criteria for NPD proper, they also warrant discussion insofar as they demonstrate impairments in functioning for which they present to treatment. Assessment of the higher-level patient with NPD can be misleading for the evaluating clinician insofar as the patient's social ease and grandiosity—the expression of which may be more carefully monitored so as to appear socially appropriate—may give the appearance of a successful individual with focal difficulties in the context of a secure and integrated identity. It is when the patient's relationships with others are assessed, however, that the vagueness, superficiality, and need-fulfilling and often devaluing quality of the relationships is observed, suggesting the operation of defenses, such as idealization/devaluation and projective identification, which result in a distortion of the patient's representation of the self (biased toward the positive) and other.

It is only with more subtle evaluation that evidence of the narcissistic pathology becomes manifest in the sense of self as well. The narcissistic patient's need for ever-increasing admiration; the benchmarking of his or her achievements in relation to those of others in order to evaluate his or her worth; the fluctuations in self-esteem with the corresponding exposure of the self to the patient's harsh superego; and in what often manifests as abrupt and extreme responses to disappointments, setbacks, and negative feedback all reveal that what on the surface may appear to be integrated, strong, and secure covers a more fragile sense of self.

Manifestations of aggression in higher-level patients also tend to be more subtle, well integrated, and revealed only upon more nuanced assessment, in contrast to the lower-level narcissistic personalities described below. Less prominent manifestations of aggression and superego pathology that are perhaps more in line with what is considered normative in the culture, although still potentially quite destructive, may include neglect of their health, breaking of company rules (e.g., the use of office computers for nonauthorized purposes), having unprotected sex with random partners, and illicit drug use.

In contrast, individuals who fall in the borderline range of functioning pose other types of difficulties with the assessment process. In such patients, there is a lower tolerance for anxiety, poorer impulse control, and less adaptive coping. In addition, relationships in the borderline

range tend to be more superficial and unstable since representations of self and others tend to be more polarized and less integrated. In short, narcissistic patients high in the borderline range may appear socially adept and engaging and, to varying degrees, successful professionally. Notable, however, is a quality of superficiality that characterizes their social and particularly intimate relationships, an instrumental quality of relatedness with a focus on scorekeeping, on who is getting more out of the relationship, and an explicit focus on quid pro quo, as well as frustrations in relationships related to a sense of what one is owed, feeling deprived, or mistreated.

Two key features that require assessment in all patients, but are of particular significance for patients in the lower levels of the borderline range for determining suitability for treatment and prognosis, are the nature and quality of the patient's aggression and superego functioning. As part of the assessment process it is important to query the patient about the range of self-directed aggression, including behaviors such as minor skin picking, neglect of health (failing to attend regular appointments or obtain recommended screenings or tests), and various forms of self-injury (cutting, burning, irresponsible use of drugs or alcohol). The assessment of suicidality in such patients is critical: Have they made prior suicide gestures or attempts, how severe have these efforts been, and by what method? The thorough assessment of suicidality, and the subsequent establishment of parameters related to severe self-injury and suicidality through the treatment contracting process is essential to establishing a viable TFP treatment with narcissistic patients operating in the low borderline range (see Chapter 6, as well as Yeomans et al., 2015, for a more detailed discussion of our approach to the management of self-injury and suicidality). Aggression directed at others should also be assessed, including the extent of the patient's verbal dyscontrol, tendencies toward physical violence, and aggression manifest in sexual encounters.

Assessment of the patient's concern related to aggression directed at others provides a bridge to the assessment of the patient's superego pathology. Does the patient express feelings of remorse or guilt over the hurt/damage inflicted physically or emotionally on others? We also inquire as to whether the patient is law-abiding or has a criminal record—the circumstances surrounding any criminal activity, as well as the patient's feelings about the same, are explored. Although difficult to query directly because of social undesirability, the patient's proclivity toward exploitation of others for the patient's material or emotional gain is important to evaluate. Similarly, in assessing these areas, the clinician should note the patient's tendency to rationalize his or her amoral or aggressive behaviors and the ability generally to take personal responsibility for his or her behaviors, or the lack of this capacity.

As discussed in Chapter 2, at the lowest levels within the borderline range one tends to find individuals characterized by extensive hostility directed at others, with little sense of guilt or remorse that would mitigate this expressed aggression. The addition of frank antisocial tendencies, even the mere tendency to be untruthful, can entirely undermine the treatment and should increase the evaluator's level of concern and attention to the details of the treatment contracting process.

Differential Diagnosis of Malignant Narcissism

As described earlier, the syndrome of malignant narcissism is characterized by the diagnosis of NPD, with associated features of paranoia, egosyntonic aggression, and antisocial features. Some patients who present with the syndrome of malignant narcissism may be helped by TFP-N. The most important diagnostic and prognostic consideration in these cases involves the differentiation between the syndrome of malignant narcissism and ASPD proper. Patients with the syndrome of malignant narcissism may maintain the capacity for emotional investment in a relationship with significant others (Kernberg, 1984, 1992, 2004). Idealization of harsh and exacting ethical standards indicates a certain capacity for honesty and a capability for experiencing guilt and concern that may become activated in the transference in the course of the therapeutic engagement. They may evince a capacity of concern for themselves, and of a potential future for their lives that protects them against total submission to their own self-destructive forces. The presence of these capacities differentiates these patients from those with ASPD per se, who present a total absence of the capacity for any nonexploitive investment, a lack of the capacity for any sense of concern or guilt, and a lack of the capacity for any vision of the future beyond their immediate survival.

The differential diagnosis between the syndrome of malignant narcissism and the severe regression of patients with BPD is facilitated by the lack of predominant antisocial behavior in the latter; their capacity for intense, clinging, and dependent relationships split off from opposite, paranoid reactions to others; and a major capability for maintaining intense nonexploitive relations with others in spite of their ambivalence and negative emotional dispositions. The differential diagnosis between malignant narcissism syndrome and ASPD requires intense exploratory work as part of the initial diagnostic evaluation, and a search for the differential diagnostic elements throughout the entire past history of the patient's development. Initial psychological evaluations, including psychological testing, often fail to provide the definite diagnostic differentiation that may require a more extended evaluation than what would usually be the case for severe personality disorders. A thorough

structural interview (Kernberg, 1984) is the most efficacious means of establishing the diagnosis, although even this may be foiled by dishonest communication and require information from third parties. Indications of an authentic capacity for object relations include a history of a significant past object relationship, the capacity to fall in love, or a deep disappointment for lost love, the investment in the survival and good care of a pet, and the capacity to convey their honesty in sharing with the diagnostician a concern over their violent, chaotic, aggressive behavior. Discovery and straightforward discussion of such experiences stand in sharp contrast to the permanent emotional unavailability to a therapeutic relationship of antisocial personalities proper.

The differential diagnosis between less severe NPDs and malignant narcissists may be determined by the quality of the antisocial features. In the former, the antisocial behavior is restricted, mostly of the passive–exploitive kind, there are no or only very mild paranoid tendencies, and there is a clear capacity for investment in others, in spite of the typical narcissistic limitation of their capacity for love relations and intimacy.

Instruments for the Assessment of NPD

Over the past several years numerous self-report measures have been developed, primarily for the purposes of conducting empirical research on NPD. We include here those measures that have items that might be of interest to clinicians in assessing and treating individuals with narcissistic disorder. Although a full description of the aspects of narcissism covered by each, the dimensions of narcissism derived from each through data-reduction methods, and the psychometric properties of each instrument is beyond the scope of this chapter, a brief summary of some of the more widely used measures provides a useful overview of the dimensions of narcissism that are currently being studied empirically, which in turn can inform our understanding of the manifest features of NPD that we observe in the clinical situation.

There are two noteworthy, broad-based, multidimensional, self-report measures of narcissism: the Narcissistic Personality Inventory (NPI; Raskin & Hall, 1979, 1981; Raskin & Terry, 1988) and the Pathological Narcissism Inventory (PNI; Pincus et al., 2009). Although originally designed to assess NPD, the NPI is widely viewed as an instrument that assesses a more adaptive, hardy, and extraverted form of narcissism (Miller & Campbell, 2008). The 40-item version of the scale forces respondents to choose between a narcissistic and non-narcissistic choice for items on each of several dimensions (e.g., entitlement, attention seeking). The factor structure of the measure is unclear, with various studies

reporting different factor structures. One criticism of the measure is that it neglects the assessment of the vulnerable forms of narcissism described above.

In contrast, the PNI (Pincus et al., 2009) assesses both grandiose and vulnerable narcissism. The 52-item, Likert-type scale assesses seven dimensions of pathological narcissism: contingent self-esteem, exploitativeness, self-sacrificing self-enhancement, hiding the self, grandiose fantasy, devaluing, and entitlement rage. Several studies have demonstrated that these seven subscales group into the higher-order dimensions of narcissistic grandiosity (entitlement rage, exploitativeness, grandiose fantasy, self-sacrificing self-enhancement) and narcissistic vulnerability (contingent self-esteem, hiding the self, devaluing; Tritt, Ryder, Ring, & Pincus, 2010; Wright, Lukowitsky, Pincus, & Conroy, 2010). The PNI has been shown to have good reliability properties and expected correlations with near-neighbor constructs and measures, thus demonstrating sound construct validity. Further detail as to the research supporting the PNI, as well as the types of questions contained in the measure, can be obtained from clinical psychologist Aaron Pincus at the Penn State Personality Psychopathology Laboratory (*http://psupersonalitylab.weebly.com/pathological-narcissism.html*).

The Shedler–Westen Assessment Procedure–II (SWAP-II; Westen, Shedler, & Bradley, 2006) is a comprehensive personality disorder assessment tool consisting of 200 personality-descriptive statements, each of which may describe a given patient well, somewhat, or not at all. The statements are rated by clinicians, not patients themselves, and the clinicians then sort the rated statements into eight categories, from "least descriptive of the patient" to "most descriptive of the patient." Analyses of SWAP-II items related to NPD have revealed three subtypes of NPD, which the authors labeled "grandiose/malignant," "fragile," and "high functioning/exhibitionistic." Among the central features of NPD identified by the SWAP-II are interpersonal vulnerability, underlying emotional distress and anger, difficulties with affect regulation, and interpersonal competitiveness. The fact that several of these features are not included in the DSM depiction of NPD underscores yet again the need for assessment tools, like the PNI and SWAP-II that provide a more complete and clinically resonant characterization of the disorder. Although the SWAP-II has been studied extensively, and was used to inform the work of the DSM-5 personality disorders work group, the procedure has also been criticized for the paucity of published interrater reliability involving the prototype-matching procedure (Samuel, Lynam, Widiger, & Ball, 2012). Concerns about the SWAP's validity include reductions in comorbidity that are forced by the Q-sort procedure that may not accurately reflect the patient's symptomatology and

may provide inadequate diagnostic coverage for targeted patients (Block, 2008; Wood, Garb, Nezworski, & Koren, 2007; Widiger, 2007). Further information related to the SWAP, including the instrument itself, can be obtained at *www.swapassessment.org*.

In sum, although none of these diverse measures individually assesses all of these qualities, they collectively cover the full range of presentations of narcissistic pathology in our model of NPD outlined above, including difficulties with self- and affect regulation, self-esteem regulation, identity, quality of object relations, and moral functioning. For the greatest coverage of these areas of content and the closest convergence on our model of narcissistic pathology, we refer the reader to the second edition of the *Psychodynamic Diagnostic Manual* (PDM-2), and even more closely tied to our particular model, the STIPO-R.

Summary and Conclusion

As described above, the structural assessment of narcissistic personality requires a nuanced and multidimensional assessment of features, some overt and frankly related to what we think of as classically "narcissistic"—that is, grandiosity (entitlement, the need for admiration, score-keeping in relationships), envy, idealization/devaluation, and instability in self-esteem—as well as features more obliquely related to narcissistic pathology, such as boredom in relationships, difficulty deriving intrinsic satisfaction from one's primary role (work or study), and a spectrum of superego deficits that speak to the level of narcissistic pathology within the borderline range. Whereas with phenotypically grandiose narcissistic patients many of these features can be quite easily ascertained, diagnosis of more vulnerable or covert narcissistic patients is considerably more challenging, requiring greater patience and tact to help reveal these features, perhaps over a more extended time.

CHAPTER 6

The Treatment Contract and Frame

Establishing and maintaining the treatment structure is among the most challenging aspects of therapy with individuals with pathological narcissism and NPD. Before entering into the work of therapy per se, it is essential to establish the conditions of treatment that are necessary to adequately address the patient's pathology. We do an assessment, as described in the previous chapter, discuss our impression of the nature of the patient's problems with him or her (and, if appropriate, the family), discuss the goals and objectives of therapy, and set in place a structure that creates a frame for the therapy that is tailored to the individual patient and his or her difficulties. Preparing the appropriate setting for therapy generally involves two to three assessment sessions (as described in Chapter 5) and possibly more, as well as the contracting sessions to carry out the tasks just mentioned.

The discussion of the treatment contract establishes the frame for the treatment that follows. The frame serves both practical and psychodynamic functions that are further elaborated in this chapter. We begin with a review of the basic principles of contracting with a focus on how this pretreatment contracting stage of TFP-N poses unique challenges with patients with narcissistic pathology. A fuller discussion of the functions and process of treatment contracting is elaborated in our earlier writings on the treatment of BPD (Yeomans, Selzer, & Clarkin, 1992, Yeomans et al., 2015).

In TFP-N, the major mechanism for establishing a treatment alliance and minimizing the risk of dropout or more subtle forms of undermining therapy is through establishing the treatment contract. Introducing the contract as a collaborative agreement early in therapy functions is an alliance-building experience. This can be especially important in

the case of patients with severe narcissistic pathology who have a combination of phobic avoidance of dependence and a dismissive attitude toward others. The contract fosters mutuality and creates a shared focus for treatment by establishing a mutual understanding of the patient's problems and goals. It articulates the patient's and therapist's respective roles and responsibilities, and creates limits and structure to contain destructive and self-destructive behaviors. In so doing, the therapist attempts to create an atmosphere of tentative trust and collaboration from which further therapeutic operations can be launched, including confrontation of narcissistic defenses and resistances, and exploration of internal object relations. Most importantly, in terms of the patient's internal dynamics, the contract-setting process activates the dominant relational dyads of narcissistic patients. This may involve the superior, controlling self in relation to the inferior, subservient other, or an idealized self in relation to an equally idealized other that can be observed in the clinical situation and taken into consideration by the therapist as he or she proceeds with the contracting process. Full exploration of this, or any dyad, is generally reserved for the therapy that begins after the contract is in place.

Thus, in TFP-N the treatment contract acts as a platform for the consolidation of the alliance and for the activation of the mental representations of self and significant others. Indeed, the contract poses an initial challenge to the patient's grandiosity in that it establishes therapy as a *collaborative* enterprise that requires their active participation and commitment, as well as input from and structuring by the therapist, from the beginning. The setting up and adherence to a treatment contract may be particularly difficult for higher-functioning narcissistic patients or those with a high-profile status who are used to surrounding themselves with service professionals (e.g., personal assistants, publicists, nannies, housekeepers, and even physicians) who are at their beck and call and who may actually bend their usual boundaries and procedures to curry favor or to bask in the reflected glory of such individuals. For example, one "very important" patient who had been in analysis for over 20 years paid for four sessions a week, but rarely attended more than one or two sessions and would, at times, miss all of them. The lack of attention to the attendance parameter of the therapeutic frame in that case created a situation in which the prolonged analysis was more a chronic acting out of the patient's internal world—in this case, the control and avoidance, and subtle devaluation of the other—than a therapeutic endeavor.

Hence, treatment contracting with narcissistic patients is associated with various complications pursuant to the patient's grandiosity. While grandiosity is compensatory, some patients nevertheless present considerable strengths, such as intelligence, verbal fluency, superficial social competence, and a dependency–avoidant self-reliance. These strengths

obscure their difficulties in acknowledging internal distress, hypersensitivity, fluctuations in self-esteem, and feelings of worthlessness—all aspects of the generally split-off vulnerable self-states to which these patients are prone. Although narcissistic patients are most likely to present for treatment when they are in a vulnerable self-state, paradoxically, they may be unable to express or explore these internal painful states as a result of their image-distorting defenses of omnipotence, projection, idealization, and devaluation (Clemence et al., 2009). This poses particular challenges for therapy and for contract setting in particular. Furthermore, the relentless devaluation and/or idealization that characterize the narcissistic patient's initial presentation results in part from defensive processes in which the therapist is experienced as a repository of dreaded aspects of the self and hence as not separate from the self. This leads to difficulties in establishing and managing boundaries with narcissistic patients that may be even more subtle and challenging than is the case with patients with BPD (Luchner, 2013).

The grandiose narcissistic patient generally attacks the therapist's attempts to set up parameters of treatment, or devalues his or her ability to be of help. Another possibility is that the patient may include the therapist in his or her own grandiose self-presentation, idealizing the therapist in extreme ways that may challenge the therapist's narcissism, either confronting the therapist with his or her limits in being able to help such patients, or evoking negative countertransference feelings of wanting to get rid of a patient who is likely to be a source of undue pressure to provide an imagined magical cure (Betan et al., 2005). By contrast, the narcissistic patient with a predominantly vulnerable presentation who idealizes the therapist may draw the therapist away from the exploratory goal of therapy by eliciting advice, soothing, caregiving, or overinvolvement from the therapist in ways that may compromise boundaries between patient and therapist (Luchner, 2013).

Nevertheless, for patients presenting with narcissistic pathology, we emphasize the importance of the contracting process. Any attempt to begin therapy without a clear frame in place creates the risk of a chaotic process and renders transference analysis more difficult since transference is often most effectively perceived by noting the ways in which a patient deviates from the established treatment frame.

Patients May Ask: Is the Commitment to TFP-N Worth It?

Working in TFP involves a major effort on the part of the patient, and that effort may extend beyond the therapy sessions themselves. Given the choice of therapies for patients with personality disorders, patients sometimes ask why TFP would be preferable to other approaches. The

recommendation for TFP-N is based on the belief that the most complete resolution of the patient's problems will come from addressing the psychological makeup that underlies the patient's specific symptoms. In addition, this psychological exploration must be accompanied by attention to how the patient is living his or her life. Therapeutic work that combines a focus on the patient's internal world with a focus on the external reality of his or her life is most likely to achieve a satisfying level of functioning in the areas of self-regard, affect regulation, work, love, interpersonal relations, and creative and leisure activities.

Although the contracting process precedes the therapy per se, it can activate intense affects and forces that must be contained in the process of contracting, and then worked with in the therapy to follow. Therapists engaging in this work should, therefore, be comfortable enough with narcissistic pathology to be able to carry out the contracting process without feeling intimidated or deskilled.

The Multiple Functions of the Treatment Contract

Table 6.1 provides a full list of the functions of the contract (see also Yeomans et al., 2015).

From the point of view of an object relations–based therapy, the most important function of the contract is that it defines the reality of the therapeutic relationship by describing the respective roles and

TABLE 6.1. Functions of the Treatment Contract

1. Establish a mutual understanding of problem(s) to address in treatment.
2. Define the reality of the treatment relationship by clarifying the patient and therapist roles and responsibilities in the treatment.
3. Protect the patient, the therapist, and the therapy, including protecting the therapist's ability to think clearly.
4. Minimize the secondary gains of illness.
5. Provide a safe place for the patient's affects to be experienced and dynamics to unfold.
6. Set the stage for interpreting the meaning of deviations from the contract/treatment frame.
7. Provide an organizing therapeutic frame that permits therapy to become an anchor in the patient's life; the internalization of the contract discussions often becomes the first internal link with the therapist.
8. Begin to define the patient's choices; the discussion of possible life activities begins to clarify elements of identity and conflicts therein.

responsibilities of patient and therapist. Subsequently, observing the patient's response to this reality provides a first window into the internal world of mental representations as they get played out in the patient's way of responding to and, often, deviating from, the roles as defined. The patient's role is to report, without censoring, anything that comes to mind in relation to the problems that brought him or her to therapy. The therapist's role is to listen and comment whenever he or she has anything to say that might advance the patient's understanding (implicit in the listening is the therapist's capacity to contain intense affective experiences). In the course of the therapy, these roles are generally challenged by the patient as his or her internal object relations are acted out in the patient's interaction with the therapist. In short, the patient's way of interacting with the therapist may communicate more about the makeup of his or her internal world than the verbal content of the patient's discourse. Contrasting the relationship that the patient creates in therapy with the relationship as it was defined by the contract helps the therapist gain insight into the patient's internal world.

Case Example 1: The Function of the Frame in Advancing the Exploratory Process

A middle-age man came to therapy because his wife threatened to leave him if he did not do so and because of depressive feelings in response to what he saw as his wife's irrational discontent with him. He began every Monday session by asking his male therapist, with a very friendly tone, "Did you play any rounds of golf over the weekend?" The therapist was struck by the fact that the question, on the surface at least, seemed removed from the problems that brought the patient to therapy. Therefore, the therapist tactfully pointed out that it might be interesting to reflect on why the patient began sessions this way rather than talk about the problems he was dealing with. This perspective provided a window into the patient's narcissistic dynamics, including his inability to tolerate a difference in status in a relationship and his need to set the tone and control interactions, eventually leading back to the difficulties in his relationship with his wife.

Case Example 2: Also Showing the Function of the Frame in the Exploratory Process

A patient was very attentive to the time in each session and always stopped talking a few minutes before the session was scheduled to end. While this may seem to be a minor event on the surface, it is a deviation from the initial agreement established in the contract that one of the

patient's responsibilities in therapy is to freely report his or her thoughts or feelings throughout each session. The patient's stopping speaking is a form of "acting in" that should be explored in terms of the underlying dyad—the underlying experience of self and other—that motivates it (controller/controlee, in this case). The therapist must always be mindful that a surface behavior may have different underlying motivations. This patient's silence could represent an anxiety that the therapist would interrupt him when it came time to end the session. The patient may prefer to manage the end time either to assert his control or for fear of the anger he might experience if the therapist interrupted him. The therapist's job is to reflect with the patient on the divergence from the initial agreement with the goal of understanding the internal experience that motivated that behavior.

Such subtle distortions in the treatment frame are particularly evident in individuals with narcissistic pathology. Compared with patients with BPD who challenge the frame in often flagrant ways by intruding on the therapist with repeated phone calls between sessions, demanding extra sessions, acting out in sessions through affect storms, and so forth, the ways in which narcissistic patients challenge the frame are more subtle, creating a particular challenge for the therapist who is trying to help the patient perceive meaning in his or her actions. When the therapist calls attention to a subtle deviation from the patient's defined role, a frequent response is that the therapist is "talking like a shrink" or "making a mountain out of a molehill." The patient who responds this way is enacting the dyad of grandiose self in relation to the devalued other while resisting the therapist's invitation to explore possible meaning. Narcissistic patients may be glibly facile in their attempts to feel special and in control of the treatment in a way that challenges the exploration of their psychological structure.

Thus, the need for the treatment contract establishing the frame is crucial with narcissistic patients whose need, and at times demand, to feel special and obtain narcissistic supplies in the form of special treatment places tremendous strain on the therapist. This may result in the therapist losing his or her sense of how to proceed at any point in a session once treatment proper has begun (e.g., "Should I give advice at this point, or remain neutral? Would I be cold and depriving if I don't offer an opinion?"). At such moments, the therapist can refer internally to the treatment relationship as it was established by the contract in order to regain his or her orientation as to what part of the interaction corresponds to "objective reality" and what part corresponds to personae in the patient's internal world. In other words, the frame provides reference points that help the therapist distinguish between the reality of the

treatment relation on the one hand, and the deviations from this clearly defined relation that are manifestations of transference and countertransference on the other hand. The patient's inner world of object relations will be evident in the countertransference pressures to deviate from the therapeutic relationship as established in the contract. Such pressure may be direct or may be subtle; the therapist must have the reality of the relation anchored in his or her mind as the reference point against which any pressure to confer special status or privileges or otherwise deviate from the frame may be understood.

To elaborate further on the general principles of contracting:

1. The therapist must feel safe enough in sessions to be able to think clearly and not experience a level of anxiety that would interfere with his or her reflective process. While it is expected that the therapist will experience intense emotions, including anxiety and anger, in working with patients with severe personality disorders, including narcissistic disorder, he or she must be able to maintain the capacity to reflect on these emotional responses. This is no small matter in the treatment of patients whose intensely devaluing transference, or even dangerous acting out, may lead therapists to internally retreat from what is going on in the therapy.
2. The parameters of treatment should limit the patient's secondary gain of illness, such as using symptoms to obtain extra benefits, including more access to the therapist, medical disability, or unending family financial support that eliminates the need to act productively in life. Secondary gain may play an especially important role in narcissistic patients whose retreat into the pathological grandiose self leads them to withdraw from life activities since the latter inevitably challenge their unrealistic grandiose internal image and structure.

Information from Prior Therapies

It is important for a therapist who is evaluating a patient for treatment to speak with the therapist(s) who treated the patient before. Narcissistic defenses protect patients from any sense of responsibility for any therapeutic endeavor that did not succeed. Therefore, they may speak with disdain of prior therapists who were attempting to provide appropriate therapy. Another possibility is that the narcissistic patient resisted a prior therapist's efforts to question and reflect upon the patient's narcissistic structure, leading to a stalemate in the therapy. In either case, speaking to prior therapists provides useful information for a therapist

who is in the process of developing a treatment for a narcissistic patient. The information obtained may alert the therapist to ways the patient's defensive structure interfered with therapy and may help the therapist create an optimal set of treatment parameters for the therapy to proceed.

Discussing Diagnostic Impressions

In the transition between the assessment phase and the contracting phase, the therapist should offer the patient a summary of his or her diagnostic impression in layperson's language since the conditions of treatment that are recommended in the contract discussion are based on the understanding of the patient's difficulties. This may be especially true in the case of narcissistic patients who have been treated for long periods for diagnoses that imply a more biological or somatic understanding of the condition. The dysphoria of narcissistic patients whose defenses fail is often misunderstood as major depression. Many patients have been in multiple treatments that did not address the narcissistic pathology, often including medications for depression and anxiety, transcranial magnetic stimulation, and even electroconvulsive therapy. While there is no question that the patient may experience depressive affect, the etiology of the depressive feelings in narcissistic patients often stems primarily from their psychological structure and thus is more characterological than biological. Therefore, patients who may have been treated for years (Magnavita, Levy, Critchfield, & Lebow, 2010) with somatic treatments should be introduced to the idea that their dysphoric moods may stem from psychological sources and specifically from a personality disorder.

An example of this is the case of a lawyer who had been asked to take early retirement from his prestigious firm when they were cutting back during an economic recession. He developed multiple somatic complaints in the context of a severe depression that only partially remitted with medication. A course of cognitive-behavioral therapy (CBT) was unsuccessful in alleviating his symptoms. He was referred for TFP by his psychopharmacologist but found it difficult to accept that his difficulties, particularly the somatic complaints, might have a psychological root and might stem from his distress and feelings of humiliation at being laid off at such an advanced phase of his career, as well as other disappointments, including being rejected by a girlfriend whom he had hoped to marry in late middle age, his sense of diminishing physical powers, and other narcissistic injuries. He acknowledged that this may be the case, but through the initial phases of treatment, he continued to focus on the somatic complaints and on the ways that he had been disappointed, exploited, and even persecuted by his partners, his family, and

former girlfriends. This focus continued until he himself, while researching personality disorders on the Internet, came across the criteria for NPD and then proudly announced to the therapist that these criteria fit his situation—even though his therapist had discussed this diagnostic impression with him in the initial sessions. The question remains of how to enter into this discussion with a patient initially and how to keep this dialogue open until the patient is ready to understand and accept the nature of his or her difficulties.

In our work with patients with BPD, open discussion of the diagnosis is recommended—that is, naming the diagnosis and offering a layperson's explanation of how to understand it (Yeomans et al., 2015). This discussion is often reassuring to a patient who does not understand the source of the chaos in his or her life. Although the extent to which clinicians should share diagnostic formulations involving personality disorders with patients has been controversial, one study found that patients with BPD do better if the therapist openly discusses the diagnosis with the patient (Lequesne & Hersh, 2004). The issue is more complicated for patients with narcissistic pathology. While the term *borderline* is gradually shifting from a highly stigmatized and misunderstood term to one whose clinical complexity is appreciated, the diagnosis "narcissistic personality disorder" carries the negative connotation of the term *narcissistic*, as it is used in everyday discourse. Therefore, the therapist who has made the diagnosis may choose to name the diagnosis directly with an explanation that the term has a more specific and less pejorative meaning in the field of mental health than in colloquial terms, or may choose to provide a layperson's description of the dynamics involved without naming the diagnosis. The direction of the discussion may vary according to whether the patient suffers more from dysphoric moods, interpersonal difficulties, or failure in accomplishments. This discussion would go along the following lines:

> "It seems from what I have learned over the last sessions that one way to understand the problems you've been describing is to think of them in relation to certain ways your mind works. It seems that in your depressive states, no matter what you do, you find yourself [and/or, if it is the case, find others] inadequate and even worthless. You experience this as a reality. However, things you have said [refer to statements of grandiose expectations/demands] suggest that there is something about how you judge yourself [and/or others], internally and automatically, that could be a major part of the problem. This involves the impossibility you've experienced in satisfying some of the basic needs/wishes of life: to feel good about yourself, to find intimacy with others, to find satisfaction in your activities,

and so on. It is hard to achieve these things if there is an imbalance of the forces at play in your mind: the forces that represent your wishes and define your goals and the way you evaluate and judge yourself and others. There seems to be a large gap between what you aspire to—that is, your goals and ideals—and what you can actually accomplish, which leaves you chronically dissatisfied, no matter what you do. [Or, if the person is relatively accomplished and successful: You've reached the pinnacle of success but never feel like it's enough or sufficient to make you happy.]

"Our work here, if you are interested in pursuing it, would involve getting to know aspects of your thinking and feeling that may be working in you at a level you're not aware of; you may confuse aspects of what you think and feel with elements of the objective world around you. If I'm right, these forces are very ingrained in you and it will take time to 'feel' them in our work here, and to feel their impact on your subjective states. If getting better were simply a matter of cognitive understanding, an extended version of this discussion might be enough to help you change. However, what we would be trying to explore here is deeply embedded in you—it defines your 'internal reality'—and it will take time to have this internal reality unfold between the two of us in a way that will permit us to experience it, to observe it, and to reflect on it."

If the patient agrees that this approach makes sense, the therapist goes on to discuss the treatment contract, making clear that the conditions of treatment described in the contract discussion are recommended because they enhance the type of psychotherapeutic work that addresses the problems as they were conceptualized in the discussion above. The contracting process is not the imposition of a set of behavioral rules on the patient but rather a description of conditions that are necessary to carry out the treatment that is developed collaboratively between patient and therapist. Our formulation of contracting has been informed by accumulated clinical experience that has taught us what conditions foster successful treatment and what conditions disable treatment, or derail the exploratory goal. One study with patients with BPD has supported the correlation between the clinician's skill in setting up the contract and the continuation of therapy in contrast to dropout (Yeomans, Gutfreund, Selzer, Clarkin, Hull, et al., 1994). In discussing the conditions of therapy with the patient, the therapist makes clear that he or she is proposing an intensive, in-depth therapy because it stands to offer more important gains than a more supportive, less structured form of therapy. But the therapist also makes clear that the patient is free to either choose

this treatment or request a referral to a treatment that does not require the same conditions.

The Contents of the Contract

The therapist's discussion of the contract with the patient is not a unilateral listing of treatment conditions but rather a dialogue in which the therapist proceeds by presenting and discussing each part of the contract and the reasons for it, and then pays careful attention to and works with the patient's response. If the patient does not agree to an essential element of the treatment, such as making the commitment to attend regularly scheduled sessions (some patients with pathological narcissism feel their other activities are too important to allow for this), the therapist should enter into a discussion of the divergent points of view to see whether they can come to an agreement. While an agreement to perfect adherence to the conditions of treatment is not necessary, it is essential to at least have an agreement to try on the part of the patient ("I'll do my best").

When the therapist enters the discussion of the contract, he or she must first address those parameters of treatment that apply to all cases in psychodynamic therapy (see Table 6.2), and then address any specific form(s) of behavioral resistance to introspection that the therapist learned of during the assessment (Yeomans, Selzer, & Clarkin, 1992; Yeomans et al., 2015; Yeomans, Delaney, & Levy, 2017).

TABLE 6.2. Essential Elements in Treatment Contracting

- The patient's responsibilities:
  - Attendance and participation in treatment.
  - Free association around the problems that brought the patient to treatment (making the effort to report thoughts and feelings freely, without censoring).
  - Paying the fee.
  - Making the effort to reflect on what he or she is reporting, on the therapist's comments, and on the interaction.
- The therapist's responsibilities:
  - Attending to the schedule.
  - Paying evenly suspended attention to all aspects of the patient's material.
  - Making every effort to help the patient gain an understanding of him- or herself and the deeper aspects of his or her personality and difficulties.
  - Clarifying the limits of his or her involvement.

Attending to Countertransference during the Contracting

It is important that the therapist not agree to treatment arrangements that require unusual efforts or heroic measures. The temptation to provide heroic treatment offers a clue to the beginning of countertransference difficulties. When the therapist accepts more responsibility for the patient than would be reasonable in the average therapeutic treatment, the result could be an increased risk of destructive acting out as the therapist assumes responsibilities that should lie with the patient, as well as an increase in the likelihood of unmanageable countertransference developments as the therapist becomes exhausted, overwhelmed, and/or harassed (Carsky & Yeomans, 2012). The therapist should keep in mind what the "good-enough" therapist would be likely to do and, if there is an inner compulsion to go beyond that, to examine his or her motives.

Patient Responsibilities

The patient's responsibilities include regular attendance, participation in the form of reporting thoughts and feelings without censoring, and paying the fee. Although every type of treatment requires some form of patient participation and the responsibilities just mentioned seem straightforward, the idea of having responsibilities at all may be alien to patients with narcissistic pathology who, as we have noted, are characterized by a sense of entitlement and, often, exploitation. These characteristics, and the underlying structure of the pathological grandiose self, may have an impact on a patient's reaction to the treatment contract. At the most basic level, the contract represents objective reality in the form of describing the roles of the patient and the therapist, and in defining the goals of therapy. These roles may run counter to the nature of the pathological grandiose self, especially in patients who have withdrawn from meaningful involvement in the world into a narcissistic "bubble." These may be young adults in their 20s or even 30s who have dropped out of school and avoided employment, or they may be middle-aged patients whose attempts to live at the level of their grandiose expectations have failed and who withdraw into lives of isolation.

In setting up the contract, we represent reality not only in the form of clarifying responsibilities within the treatment relationship but also in the form of making it clear that participating in therapy sessions without an active engagement in some life activity is a sterile enterprise, akin to trying to treat a patient with substance abuse with exploratory therapy before the substance abuse has been dealt with practically. Talking about narcissistic vulnerabilities and defensive grandiosity in treatment is often a mere intellectual exercise if patients are allowed to avoid facing those

same vulnerabilities by opting out of all real-world commitments. Our clinical experience is that patients with personality disorders, including NPD, are generally capable of both a higher level of activity than is often assumed, and that to approach them with this understanding is beneficial for progress in therapy and appeals to the part of the patient that could find satisfaction in functioning.

Because of the above, the discussion of the contract may be the therapist's first challenge to the grandiose self. Therefore, the therapist must take on the task of balancing the need for structure in the treatment with the brittleness of the pathological grandiose self. It is at this point that a difference between patients with NPD and patients with BPD should be considered. Patients with BPD, who appear more disturbed on the surface because of the obvious chaos in their internal world, tend to more readily accept the structure described in the contract. While they may not *welcome* it, they more readily accept the fact that they are troubled and need help than is the case with patients with NPD, precisely *because* patients with NPD are in touch with their internal chaos. The patient with NPD, because of the more stable projection of all that is devalued, tends to respond to the conditions of the contract with an attitude including elements of "Why should I bother?"; "Who are you to ask things of me?"; "Can you prove this will work?"; and "This isn't important enough for me to go out of my way." The therapist may understandably find these attitudes off-putting but should remember that they are part of the narcissistic defensive system that protects against feelings of worthlessness, nothingness, or being defenseless in the face of a perceived critical attack. With this understanding of the nature of narcissistic defenses, the therapist may choose to be more flexible with a narcissistic patient than he or she would be with a patient with BPD when setting up the contract. This flexibility may allow the patient to enter into the structure without a sudden collapse or rigidification of his or her defensive system.

Frequency of Sessions

The recommended frequency of sessions in TFP-N is twice weekly. This allows time to explore the patient's internal world and also to keep informed about what is going on in the patient's external life. It is also important because of attachment issues. Patients with pathological narcissism and NPD almost always present with insecure internal models of attachment, in keeping with their involvement with exaggerated and unrealistic internal representations of others. Fewer than two sessions per week makes it much more difficult for the therapist's interventions and presence as a concerned other to have an impact on the power of the

patient's internal representations in determining his or her experience of others and ultimately of self. Having said this, while the therapist's concerned presence is an important factor in therapy, clinical practice has repeatedly shown that well-intentioned therapists who do not actively interpret how paranoid elements in the patient's internal world defend against attachment have little impact on modifying how the patient experiences others.

However, we should add that many clinical settings do not allow for more than once-weekly sessions. In an effort to make TFP-N accessible to patients who would not otherwise have access to it, we have supervised cases that took place on a once-weekly basis and are in the process of writing guidelines for delivering TFP-N with that frequency. However, a twice-weekly schedule corresponds to standard TFP.

If the patient objects to the expectations concerning attendance, the therapist makes it clear that he or she is not making an arbitrary recommendation but simply referring to the reality that treatment requires regular attendance. While we emphasize the need for discussion in order to understand the patient's position in relation to the conditions of treatment, the basic parameters are essential for this type of therapy. Therefore, should the patient object, the therapist notes the objection and points out that understanding the patient's objection might lead to important insights (around resistance) in the course of the therapy. If the patient does not eventually agree to a twice-weekly schedule in a setting where it is administratively possible, the therapist should make clear that the patient is undermining the usefulness of the therapy and the therapist should consider referral to a supportive therapy with an acknowledgment of the more modest treatment goals implied in that choice.

Case Example 3: Balancing the Need for Structure with the Brittleness of the Pathological Grandiose Self

A patient in her early 30s who had been treated since her late teens for "treatment-refractory depression" was rediagnosed as NPD. While somewhat ambivalent about the new diagnosis, she understood the reasoning behind it and agreed to discuss the conditions for TFP. The therapist started by discussing the frequency of sessions. As soon as he had said that they should set up two regular session times a week, the patient responded, "Why should I commit to coming here twice a week when I have no guarantee this would help? This could turn out to be a waste of my time, like the other therapies I've had."

The therapist, sensing a combination of phobia of dependency and a struggle as to which of the two of them would have the superior "power position," decided to allow for some leeway around this parameter,

hoping that they would be able to get to the point of having a regular twice-a-week schedule but also realizing that the patient's narcissistic defenses could block the attempt to set up an adequate treatment structure in the short term. Time would tell. He said, "Okay, then let's see if we can find a time later this week to meet so that we can continue then."

The patient agreed. For the next month, at the end of each session, the therapist and patient took out their calendars to schedule the next session. During all this time they were meeting twice weekly. At the end of the month, the therapist said, matter-of-factly, "We've been taking up some time at the end of each session to schedule the next one. Maybe it would be simpler to schedule two regular times." The patient agreed.

The therapist's management of this situation reflected a consideration for the patient's narcissistic defenses: her view that accepting the therapist's conditions right away represented submission to his superior position, the related "need" to devalue the therapist, and her deeper and less conscious concern that agreeing to commit to therapy would acknowledge his potential role as an important person in her life, touching on dependency issues.

Fees

An important parameter of treatment is dealing with the fee for therapy. Paying for therapy is part of the therapeutic relationship. As with any parameter, it may have a particular significance for patients with NPD. Most directly, it is a sign that what the therapist has to offer is worth something, which may be difficult for a narcissistic patient to acknowledge.

Case Example 4: An Enactment around the Fee

In the contracting phase, a patient with a well-paying job told the therapist that her fee was more than he could afford so she lowered her fee by 20% [the therapist should have explored this more before taking action]. After a year of therapy, the patient made a reference to the cost of therapy and then added in an off-handed way, "But it doesn't matter because my insurance pays 100%." At that point, the therapist reminded the patient of their initial discussion of the fee and discovered that, although he did not have to pay anything out of pocket for the therapy, the patient responded as he did in the initial discussion of the fee because it just seemed to him like "more than the therapy [therapist] was worth."

The discussion of the fee should be direct and open. It may be less of an issue with patients who see therapists in a clinic or who are on an insurance plan, in contrast to patients in a private practice setting, but

even in the former cases the patient has the responsibility to take care of paperwork and co-pays in a timely fashion. In the case of private practice, the patient and therapist must agree upon the fee, when the bill should be paid, and the policy on payment for missed appointments. Many therapists work on a sliding scale. The discussion of the patient's finances should be open and direct; even in cases where adjusting the fee is not an issue, the therapist should learn of the patient's financial reality as part of knowing about the state of his or her life. It is interesting that many patients who talk relatively freely about other areas of their life are reluctant to openly discuss their finances. Their concerns about this become another area of exploration in the therapy.

The Patient's Role in the Sessions

While every type of treatment requires some form of patient participation, often patients approach treatment in the hope of a "quick fix" with the idea that the therapist has ready-made answers to the dilemmas that they are presenting with. This may be particularly true of patients with pathological narcissism and NPD in a way that touches on both poles of the "grandiose–devalued" dyad. On the one hand, discussing the patient's role in therapy may run counter to a narcissistic patient's grandiose expectation that he or she should receive special caregiving from the therapist that often takes the form of requests to modify the fee (even though they can afford it), reschedule sessions frequently or do them on the phone, and other special arrangements. On the other hand, the projection of the ideal object on the therapist can foster a belief that the therapist should be able to magically cure the patient. The contracting discussion of patient and therapist responsibilities is an implicit challenge to these aspects of the patient's internal world. Therefore, pointing out to the patient the need to participate in his or her own therapy and describing the therapist's role in concrete terms touches on many themes common in patients with NPD, including grandiose expectations of self and other, and anxiety about dependency.

TFP-N modifies the general psychoanalytical rule of free association by asking the person to report everything that comes to mind—all thoughts and feelings—*that ultimately have some connection to the problems that brought him or her to treatment*. While the rule of free association is generally followed in TFP-N, we also have noticed that the associative process can be used as a defense against access to uncomfortable dissociated material: Associations may dwell endlessly on an aspect of the patient's life or thought process that supports the grandiose self and avoids discussion of a split-off area of difficulty, such as behaving at work in a way that creates the risk of being fired. Thus, the therapist

must continually pay attention to the patient's affect and behavior, as well as the content of his or her discourse. A discourse lacking in affect, accompanied by boredom in the countertransference, is a sign that the associative process may be being used defensively. In contrast to using the process of free association as a form of resistance through trivialization of discourse, some patients with pathological narcissism and NPD manifest resistance by failing to follow the principle of speaking freely (Kernberg, 2015). This may lead the therapist to engage in a back-and-forth discussion that becomes more of an interview or conversation, rather than an exploration at a deep level; the form of interchange then becomes an enactment of an aspect of the patient's character pathology (e.g., to be controlling or avoidant).

A typical initial statement about the patient's role in treatment might be:

> "Your role in therapy is to speak freely about whatever is on your mind, particularly in relation to the main problems that brought you here, with the goal of understanding the unknown motivations for your thoughts, emotions, and behavior. Although at times it may feel difficult for you to speak freely, it is important to try not to censor yourself. What you say can include thoughts, feelings, dreams, fantasies, and so on.
>
> "Beyond the general rule of speaking freely in session, if something is happening in your life where you run the risk of harming yourself either directly or indirectly, such as by engaging in activities that might threaten your job or that might harm others or that might affect the continuity of the treatment, then you should bring that issue up before anything else. For example, if it looked like you might lose your job and it would be hard to pay for therapy, it would be important to bring that up for discussion before talking about whatever else might come into your mind."

Therapist Responsibilities

The therapist's central responsibility is helping the patient achieve more understanding about him- or herself and the internal conflicts underlying the patient's difficulties or symptoms. The therapist's other responsibilities have to do with the scheduling of regular appointments, being open to and accepting of the intensity of the patient's affects, limiting his or her involvement with the patient to the work of exploratory therapy, and maintaining confidentiality. Some therapists feel that the effort to explore and understand is not enough to offer patients with severe personality disorders whose suffering is intense and whose lives may be at

risk. Our belief is that the therapist is likely to be the only person in the patient's life who is willing to immerse him- or herself in the intensity of the patient's internal world and that the therapist should appreciate the importance of this unique role.

One of the aims of the discussion of the contract is to educate the patient about the nature of the therapy being recommended. The therapist's description of his or her role may be along the following lines:

> "My responsibility is to listen as attentively as I can to what you are saying and to make comments when I feel they might be helpful to furthering our understanding of you. There may be times when you will ask questions that I may not answer or there may be times when you want me to speak and I may not have anything to say at the moment. Whatever the situation, I will always be interested in your experience of what is going on. There may well be times when you want me to give advice or tell you what to do. The form of therapy I'm recommending for you is meant to foster your own ability to reflect on yourself, on interactions, and on situations. It is also meant to foster your autonomy and independent functioning. Therefore, in most cases, my providing you with direct answers or advice—as though I had all the answers—would not be as useful as my helping you arrive at your own decisions and understand the underlying issues that have traditionally made it hard for you to do so."

Confidentiality

With regard to confidentiality, the therapist explains in most cases that the patient has a right to full confidentiality, except in instances in which there is an immediate risk to the safety of the patient or others. However, the therapist may discuss treatment parameters that involve communication with third parties as part of the treatment plan in cases with antisocial features and a history of dishonesty. This situation is discussed below in the section on modifications of the contract for lower-level narcissistic patients with antisocial features.

Limiting Communication between Sessions and Understanding the Possible Need for Additional Support When the Grandiose Self Begins to Give Way to Underlying Structures

The therapist's description of his or her availability is important both to establish what the patient can realistically expect and to provide a model of measured consistency, as opposed to impulse-driven erratic contact or entitled expectation of constant availability. In this treatment where

communication is usually limited to the sessions, patients may complain that the therapist offers them nothing to help with their distress between sessions. However, one of the objectives of the therapy is to help the patient develop the capacity to maintain a consistent internal image of and sense of attachment to the other, as the interpretive process helps the patient understand how internal affect states tend to diminish or destroy that capacity and such internal images.

The position the therapist takes regarding patient phone calls may vary according to the dynamics of the situation. In early treatment, calls are often motivated by the secondary gain of extra contact with the therapist, which may feel gratifying but does not help the process of change. In this phase, calls are appropriate for practical purposes, such as rescheduling, or in cases of emergency (defined as an unpredicted major stressful event, in contrast to chronic states of distress or anxiety). However, as therapy proceeds and enters the midphase, one type of emergency typical of narcissistic patients occurs when a patient begins to experience distress and anxiety as the therapy challenges his or her characterological defensive structure. A patient whose internal structure is based on the pathological grandiose self, supported by consistent devaluing and dismissing of others, generally experiences distress when a sense of initially covert dependency begins to emerge into consciousness in relation to the therapist. This initially inchoate sense of dependency challenges the patient's feelings of grandiosity, leaving the patient's sense of self endangered both because its self-sufficiency is questioned and because the dependency is generally accompanied by paranoid anxieties. This development in treatment may lead to pushing at the boundaries of therapy, including requests for extra sessions, long and/or frequent phone messages, or expectations that the therapist read written material or e-mails, or may lead to the sudden emergence of interruptions to the treatment, including frequent trips, urges to drop out, or even to suicidal ideation.

In a situation like this, the therapist may take an active role by (1) communicating his or her empathy for the acute difficulty the patient is experiencing; (2) confirming that this difficulty may be necessary for meaningful change to take place; and (3) letting the patient know that during this time of anxiety, because of the "unknown territory" the patient is entering into, he or she can call the therapist if the urge to end the therapy becomes strong or if anxiety or depression become intense. The therapist's consistent presence is an element of the treatment situation that provides "scaffolding" that makes the peeking out beyond the grandiose self less terrifying; it counters the patient's internal conviction that his or her grandiosity is a protection against collapse, annihilation, abandonment, or destruction by a malevolent other. The therapist may also offer an additional session at times to work on the anxiety aroused

by the developments in the treatment. While this message may appear to contradict the general policy that phone calls are appropriate only in times of emergency events, it reflects the fact that, as the therapy develops, emergencies can occur in the transference as the patient's defensive structure changes and the dismantling of the grandiose self must be followed by the strengthening of a more integrated ego, including more split off, and therefore initially-frightening, aspects of the sel .

Behavioral Resistances to the Exploratory Process

A patient's resistances stem from the fact that exploratory therapy threatens a fragile homeostasis that exists within his or her psychological structure. The pathological grandiose self that defines the internal structure of patients with pathological narcissism and NPD does not provide successful adaptation to life, but it *does* provide a certain stability in relation to intense affects and internal conflicts. The internal split at the core of NPD involves the appropriation of all that is positive and ideal, and the projection of all negative aspects of self on others, who are then experienced as a combination of inferior, persecutory, and/or exploitative—in short, a version of the split paranoid–schizoid defensive structure. Conversely, as discussed in chapter 2, the individual with narcissistic pathology may experience the self as negative, vulnerable, and defective with a grandiosity organized around the specialness of his or her suffering and covert fantasies of special treatment to which he or she should be entitled from exalted others who are inordinately idealized (Cooper, 1988). While in either case, this structure does not provide for a good (flexible, adequate) adaptation to the complexities of life, it does provide some relief from anxiety insofar as it orders the patient's world around the externalization of negative and intolerable aspects of the self via projection, or around undue reliance on idealized others who are the repository of the ideal aspects of self. Any attempt to connect the disavowed aspects of the patient's mind to his or her conscious awareness provokes anxiety, an anxiety that is inevitable until, with the help of therapy, the patient begins to integrate the fended-off material. Hence, patients often manifest resistance to exploration that may take behavioral forms.

Examples of behavioral resistances are substance abuse, disordered eating behaviors, chaotic promiscuous sex, and, in the most extreme cases, self-injury and even suicide,[1] among others. Behavioral resistances may require the establishment of specific conditions of treatment beyond the universal parameters that are specific to the individual patient. For

[1]While self-injury and suicidal behaviors are more common in BPD than in NPD, the collapse of the grandiose self in patients with NPD can, on rare occasions, lead to sudden and potentially lethal suicide attempts.

example, in the assessment process, the therapist may have noticed a pattern in which a patient frantically sought casual sex on Internet sites whenever the work in her previous therapy caused her to doubt her superiority in relation to others. Her therapist at that time would then switch into a role that seemed more like that of a counselor, as though the patient did not have the intelligence to know the inherent risk in such behaviors. The therapist assessing the patient for TFP explained that this pattern of behavior seemed to derail the exploratory therapeutic work and asked the patient whether, in the course of their work together, she could take responsibility for refraining from this reaction to the anxiety precipitated in therapy. The therapist empathized with the distress caused by any challenge to the patient's internal system but emphasized the need to reflect on it without being distracted by dangerous behaviors.

In another example, a patient with anorexic tendencies may be asked, as part of the treatment arrangement, to consult with a nutritionist to establish the minimum weight considered medically safe and to be weighed on a weekly basis until stability of this symptom is achieved. In practical terms, the therapist must avoid getting involved in a therapy where his or her role becomes that of either repeatedly "putting out the brush fires" that the patient creates by acting out, in the case of more disturbed narcissistic patients, or of attempting to provide a "quick fix" or offering advice to try to provide the type of magical solutions for urgent problems in love or work that higher-functioning narcissistic patients may demand. It is important to convey to the patient that the task of the therapist is not to provide immediate "solutions" to difficulties—that role would distract and perhaps overwhelm the therapist's capacity to reflect and to engage the patient in reflecting on his or her inner world. The therapist can explain to the patient that the patient can choose between a therapy in which the therapist's role is that of a supportive case manager and a therapy in which the therapist assists the patient in deep psychological exploration, but that the two are mutually exclusive.

The therapist should not feel an obligation to work with a particular patient if that patient does not accept the fundamental aspects of the treatment. It is the therapist's job to make sure that he or she is providing psychotherapy that addresses the underlying nature of the patient's difficulties. Patients who are not interested in working at this level may be referred to supportive therapy (or the therapist may choose to provide supportive therapy to the patient with the understanding that it does not have the same ambitious goals as exploratory therapy). Once the therapy has started, the work of therapy often involves referring back to the contract and sometimes involves revising it or adding to it during the course of treatment.

Narcissistic patients spontaneously and unconsciously employ resistances to any psychological exploration that questions the pathological

TABLE 6.3. Examples of Resistances/Threats to the Treatment

1. Suicidal and major self-destructive behaviors.
2. Homicidal impulses or actions, including threatening the therapist.
3. Lying or withholding of information.
4. Substance dependence and substance abuse.
5. Eating disorder behaviors.
6. Minor self-destructive behaviors (scratching, in contrast to cutting).
7. Poor attendance.
8. Excessive phone calls or other intrusions into the therapist's life.
9. Not paying the fee or creating circumstances that interfere with payment.
10. Creating problems outside of therapy that interfere with continuing therapy.
11. Maintaining a chronically passive lifestyle, favoring secondary gain of illness.

grandiose self as a valid and accurate stance in relation to the world; the resistance could be in the form of derailing the therapy from the exploratory task or ending it outright. As we emphasized above, resistances can be present in behavioral forms, as well as in psychological attitudes. The list of possible resistances to the treatment in Table 6.3 is somewhat homologous to the hierarchy of priorities that the therapist is instructed to address in TFP (see Yeomans et al., 2015, chapter 7). The first issues to address in a session, should they be present, are issues that might threaten the treatment. In setting up the contract around the specific threats to the treatment that are the manifestations of resistance, the clinician must be alert to the wide range of acting-out behaviors—the most common forms of these in patients with BPD are overt behaviors, such as cutting and overdosing. Patients with pathological narcissism and NPD often express their resistance in more chronic behaviors, such as retreating into isolation from life's challenges, chronically bending the rules as though they do not apply to them (e.g., using the office computer to look at pornography even though it is against company policy), or distorting or withholding crucial information from the therapist in order to make a good impression.

Evaluating the Patient's Acceptance of the Contract

A measured acceptance of the contract, with some reservations by the patient, is the most assuring response. An immediate but unthoughtful

acceptance may reflect a cavalier "lip service" attitude. On the other end of the spectrum is outright rejection of the contract. This is more common with thick-skinned narcissistic patients who find the very idea of a contract intolerable, as it confronts their usually defensive sense of entitlement and their being exempt from the rules. It is at this point that the therapist must decide on the degree of flexibility versus structure to adopt, as we discussed above. In general, more flexibility is indicated for patients with higher-level NPD, whose brittle grandiose self is prone to abrupt collapse, and more structure is indicated for lower-level patients with NPD with higher levels of ego-syntonic aggression, directed at self or others, and/or antisocial traits that make them prone to destructive acting out. Another consideration is how essential the parameter in question is to have in place from the beginning of therapy.

An example of flexibility is with a relatively high-functioning narcissistic patient who was extremely perfectionistic with regard to her work life and physical appearance, who had been physically abused by her father throughout her childhood and continued to resort to head banging when she was under stress. Initially, the therapist suggested that she tell her general doctor about this behavior (which she had not done) and that she ought to have an exam by a neurologist to ensure that she had not incurred any brain damage as a result of this activity. She refused the recommendation because her sense that she might be damaged or imperfect was intolerable—an aspect of her severe narcissistic pathology—but after several contract-setting sessions, she agreed to tell her general physician, with the idea that she and her therapist would work toward her capacity to have the neurological exam. After several months of working on the self–other dyads that underlay this perfectionism (harsh critical other/defective self), the patient agreed to go ahead with the neurological exam in which there were no significant findings. Her resistance to this seemed to have been based on a retreat into the grandiose idea that her aggression could not hurt her because she was too strong to be damaged.

Addressing Secondary Gain in the Contracting Phase

One aspect of behavioral resistance we frequently encounter with more disturbed narcissistic patients is the pattern of having established secondary gains of illness that support the sense of grandiosity and also buffer the patient from contact with aspects of living in the real world. The secondary gain may be in the form of obtaining financial support from family or from government benefits that enable the patient to live, often in an isolated state, without being involved in a productive activity. Addressing such a system of dysfunctional secondary gains that support

the pathological psychological structure is one of the tasks of the first phase of treatment (Yeomans et al., 2015). In the case of patients with NPD, it may be a source of great stress and treatment noncompliance: Taking a job is not just taking a job; rather, it involves taking a step out of the splendid isolation that grandiosity promises to make contact with a dreaded reality.

One patient isolated in her apartment for years while planning to write the music that she felt was in her. In the course of those years, she had had a number of hospitalizations for depression. The diagnosis of depression was based on her anhedonia and isolation. When the patient's diagnosis was changed from recurrent major depression to NPD, her new therapist discussed the need to become involved in a structured activity that involved others. The patient initially responded that she would be active by finally getting down to writing her music. Her therapist pointed out that this would not address the problem of her isolation or lack of structure. As they discussed the issue, it became clear that the patient feared that others would not see her great talent if she actually produced something. The therapist, who was not sure of the patient's level of talent, empathized with the patient's fear that contact with others might take her a step away from the dream that she felt was the only thing that gave her life meaning. However, he also remained convinced of the value of simultaneously getting her out of her isolation and exploring her experience of that process, with the anxieties associated with challenges to her grandiose self. His clinical judgment was that working on her narcissistic structure would help her find meaning in life beyond her grandiose fantasy. The clinical challenge was that the dismantling of the narcissistic structure and shift to a more integrated structure allowing in-depth relations with others can, as the process is happening, leave the patient feeling highly vulnerable. In this case, the eventual choice of activity was for the patient to begin a master's program in teaching. While this did not immediately resolve the problems created by experiencing relationships in terms of a grandiose–inferior dyad, it provided enough healthy narcissistic gratification, side-by-side with the anxieties of interacting with others, to be a first step out of the patient's isolation.

It is important to keep in mind that the treatment plan is predicated on an adequate diagnostic evaluation. The therapist should be comfortable that the patient's primary diagnosis is that of a personality disorder and not another major pathology, such as a major depressive episode or psychotic illness. If the therapist begins to set up the contract with a patient and then begins to change course because of emerging concerns that the patient may be experiencing a depressive episode or be psychotic, the therapist must establish whether his or her doubts about the

diagnosis are grounded in reality or whether it is a countertransference issue (e.g., Is the patient eliciting doubt in the therapist involving guilt that he or she is asking too much of the patient?).

In the example described above of the patient who isolated in her apartment with the plan to write music, in the course of discussing the therapist's proposal to get involved in a structured activity out in the world, the patient argued that this plan would precipitate her relapse into a severe depressive episode. The therapist's internal response was to experience a transient countertransference guilt that his position may damage the patient until he reassured himself that evidence provided by his assessment of her pointed to a diagnosis of NPD rather than a major depressive episode that might involve objective impediments to functioning.

Patients with Antisocial Features

Patients with antisocial features often show stronger resistance to engaging in a life activity because of issues of concrete secondary gain (e.g., parental financial support). Because of that, this phase of the contract setting is particularly challenging with these patients. An unempathic rigid position on the part of the therapist could contribute to patient dropout while too lenient a position could perpetuate a pathological system. In general, the presence of antisocial features calls for more structure in the treatment contract. An example is that of a 25-year-old patient referred to therapy by his parents after he had failed out of college for the second time. At the time of referral, the patient was spending his days in his apartment smoking marijuana and playing video games. He himself had no complaints. His parents paid all of his living expenses. Because of his lack of motivation and the fact that he had not communicated honestly with a previous therapist, the treatment contract required a high level of structure, including (1) a channel of communication between the therapist and the patient's parents for corroboration of what he was reporting in sessions, (2) random drug screens as part of a plan to help him stop his marijuana use, and (3) a gradual decrease in the parents' financial support to decrease the secondary gain of illness and increase his motivation to take on an outside activity.

A second example is that of a 30-year-old male patient with NPD with antisocial traits who sought therapy for symptoms of depression that rendered him unable to work or function. In the assessment, the patient's mood did not appear depressed: There was a gap between his description of his mood as depressed and a presentation that seemed euthymic to the therapist doing the assessment. Yet the patient described states of immobility at home where he "could not get out of bed all

day to get a glass of water." When the therapist asked what was on his mind at those times, the patient described distress that his talents as a writer were unrecognized. In his view, he had a right to be famous and successful—"the next F. Scott Fitzgerald." The therapist saw this presentation as more compatible with the internal structure of NPD than with a major depressive disorder. Indeed, exploration revealed antisocial features in that the patient exaggerated his symptoms of depression in order to receive medical disability payments for "treatment-refractory depression."

The patient was willing to consider the new diagnosis of personality disorder and readily agreed to the general conditions of treatment (attending, free association, etc.). However, when the therapist discussed the need to engage in some form of work or study, the patient responded, in a vaguely threatening way, that any such activity would cause him to relapse to a life-threatening state of depression. The therapist's initial internal response was to feel a combination of anxiety and guilt for proposing a harmful element of treatment (countertransference reactions are important to track in the contracting phase, as in all phases of treatment). However, as the therapist internally reviewed her clinical impression and the reasons for it, her affective state became calmer and she returned to the question of the structure of the treatment and what parameters around engaging in a structured activity would be appropriate. She felt the patient's passive antisocial features motivated his allegation that this parameter of treatment would harm him. In returning to the discussion, she reminded the patient that the decision to enter into this treatment was solely in his hands and the option of a more overly supportive treatment was available but would be less likely to result in major change.

Seeming to accept this parameter of treatment, the patient's first proposal of what activity he could engage in was to finally sit down and write the novel that he had been putting off for a long time. The therapist tactfully responded that that would not include the level of structure or contact with others that would be part of the therapeutic purpose of engaging in an activity. The patient next proposed that he go to the local elementary school one afternoon each week to "toss balls around with the kids trying out for the basketball team." While this would involve some structure and contact with others, the therapist felt this was a minimal engagement for a man who was intelligent and articulate and whose life would be better if he could support himself financially. In addition, the therapist thought that interacting with children would support the patient's need to be "the big man" without helping resolve that narcissistic issue. She proposed that he call a temp agency

and find part-time secretarial work or data entry work. He objected vigorously, angrily stating that he would never do that because "bosses tell those people what to do!" He said this so affirmatively that the therapist accepted this objection, only realizing later that his objection was a fact that applied to most people's jobs (her immediate reaction was a concordant countertransference that corresponded to the patient's internal reality). The session ended at that point.

In the next session, the patient said he had not been able to come up with any further ideas. The therapist brought up the possibility of going back to finish college (the patient had completed 2 years in his early 20s). The patient said he had tried that and could not tolerate sitting in the same room with "arrogant kids 10 years younger who liked to show how smart they are." At that point the therapist felt that this might be one of the rare cases where a consensus could not be reached with a patient. She said, "This is our third session discussing this topic and it may be that our views about what would be therapeutic are too different for us to embark on a therapy together." The patient seemed surprised that the therapist was not continuing to try to find a way to work together and that he could not control her, and he became concerned that he might not be able to continue in the therapy. He said, "I've noticed the people who draw the blood when I've been in the hospital. It looks like it could be kind of interesting. I actually looked into it a little and found out that you can get trained to do that in 3 months. Is that the kind of thing you had in mind?"

The patient's idea was more concrete and practical than the therapist would have imagined, but the fact that the patient, when confronted with the possibility of being referred to a "chronic, maintenance treatment," agreed to a realistically therapeutic treatment plan supported the therapist's diagnostic impression and view that the patient's initial protests against involvement in an activity served a defensive function and did not represent the true state of his potential. As the case progressed, the patient pursued and eventually finished training as a phlebotomist, but it took him more than 6 months, as his insecurities and interpersonal difficulties created obstacles at many points along the way. These problems were discussed in sessions.

This example illustrates how, according to the specific case, one element of the contracting process can require an extended discussion. It also illustrates that the TFP-N therapist, who generally maintains a position of therapeutic neutrality in the course of therapy, is *not* neutral in the process of setting up the frame of treatment. The therapist, at that stage, allies him- or herself with the part of the patient that is seeking a better, more autonomous, productive, and satisfying life. Clinical

experience shows that a clear discussion of the difference between an intensive therapy with ambitious goals and a chronic, maintenance therapy can appeal to the positive health and reality-seeking side of the patient. It may also activate a patient's narcissistic issues in a helpful way, appealing to the possibility of a healthy narcissism based on functioning in the world.

Case Example 5

A 28-year-old patient was referred for evaluation because his condition was not improving in his current therapy. He had been diagnosed with refractory depression and, as can happen in these cases, his prior treatments had included electroconvulsive therapy (ECT). In addition to somatic treatments for depression, the patient had also been in an "insight-oriented therapy" for a number of years. It was a therapy that did not include a clearly defined set of treatment parameters. One of the overall goals of treatment was for the patient, who had never succeeded in keeping a job, to be steadily employed. However, he said that on many mornings it was impossible for him to get out of bed. This led not only to failing at jobs but also to a high frequency (20–30%) of missed sessions in his therapy. His therapist's stance had been to look for the roots of the patient's difficulties in a developmental history that included a demanding father and an often-depressed and bedridden mother. While those elements of the patient's life likely had a relation to his difficulties, a focus on them in therapy was not leading to change and possibly served as an excuse for the patient's low level of functioning. When the therapist providing "insight-oriented therapy" began to experience an impasse in the treatment and began to consider the possibility of an underlying personality disorder, he referred the patient to a TFP therapist.

The patient came to the first assessment session eager to have the opinion of an "expert" in the field (the patient had a history of turning to experts to solve any problem he encountered, such as tutoring for school, and plastic surgery because of discontent with certain facial features). He was surprised when, after completing the assessment and discussing his diagnostic impression with him, the therapist discussed a treatment contract with him that included getting out of bed to go to whatever commitment (work, therapy) that he had on a given day. The patient's response included saying (1) that he had expected the therapist, who had been described as an expert in dealing with his type of problem, would offer him a "cure"; and (2) that his prior therapist took the position of praising him if he was able to get out of bed at all and accomplish one errand on those difficult days.

The new therapist proceeded along the lines discussed above: He clarified that he could not determine the patient's capacity to get out of bed but could only describe to him what the therapy required and that the alternative was a more chronic, maintenance treatment. The patient was dismayed to hear this but agreed to do his best to comply with the treatment. In the first 6 months of therapy, he was able to find meaningful employment and, although it was a struggle, to maintain the job. Part of the message in the contracting phase is that the therapist is aware that functioning, particularly in situations involving others, leads to stress but that part of the work of therapy can be to explore specific experiences the patient is having at work or school to better understand the factors (psychological and practical) that contribute to the negative affects that are aroused on the job. The therapist empathizes with the fact that interacting with people can be difficult but adds that the patient's way of perceiving others may add significantly to the problem.

The above example also illustrates how a discussion of the contract can challenge a patient's projection of the ideal helping object onto the therapist ("I thought you would offer me a cure"). A therapist's honest communication of what he or she can offer is a demonstration of an object who continues to function to the best of his or her capacities, even if not in a state of perfection.

NPD Patients at the Lower Level of Borderline Organization

At the lower level of the NPD spectrum, patients with a more solidly entrenched pathological grandiose self with antisocial features may need the most intense level of structure in the contract and frame. As seen in the example of the 25 year-old patient with antisocial features described above, this is often the case with adolescents and young adults characterized by dishonesty and exploitatively obtaining secondary gain from their parents. Because of the dishonesty, the treatment frame should include a channel of communication to a third party, or third parties (e.g., the parents, a substance abuse counselor, or a school administrator), to have a means of verifying the patient's communication. Even adding this extra parameter does not guarantee that a patient with antisocial features will engage in therapy in a meaningful way. In the most severe cases of antisocial features, the only meaningful therapeutic work that can be done may be with the family or social system rather than with the patient directly. In these cases, the therapist's principle role may be to help the family set realistic limits with the patient in order to try to channel the patient into a more productive life and in order to minimize the damage done to the family.

The Option of a Family Meeting

If the patient does not function autonomously and depends on the family for aspects of survival, the contracting process should include a meeting with the patient's parents, spouse, or partner. The session always includes the patient. The therapist can use the session to discuss his or her impression of the patient's condition and the nature and limits of the therapy. This session is important when families do not understand either the nature of the disorder or the fact that the treatment offers no guarantee that a self-destructive patient will get better. Establishing the latter understanding relieves the therapist of unrealistic pressure to perform. With regard to discussing the nature of the disorder, family members often have the misunderstanding that the patient is suffering from an exclusively biological condition that should respond to somatic treatments. That understanding can leave families with the impression that the patient is incapable of functioning and that nothing can be expected of him or her. This is very different from the view that illness is based on a psychological structure that leads to retreating from the world and that involvement in a life activity, though difficult, is both possible and therapeutic. Thus, families are often relieved to learn about the nature of a personality disorder and the possibility of effective therapy in cases where the patient has been misdiagnosed for years and lived in circumstances where the family felt obliged to provide for the patient because of the belief that he or she was incapable of functioning.

Case Example 6

A 23-year-old man was referred for therapy by his parents. He had no interest in treatment. His parents, however, were concerned about his dangerous, dishonest, and illegal behaviors. He had recently returned from another state where he was supposed to have been in college. Instead of attending college, he spent his time and his parents' money doing drugs, gambling, and womanizing. He also wrote bad checks and misused credit cards. He would have been arrested if his parents had not intervened. The diagnosis was NPD with serious antisocial features, with the possibility that, with time, the patient might meet criteria for a full diagnosis of ASPD. It became clear to the therapist that it would be naïve to depend on the patient as the only source of information. While the therapist respected the traditional rules of confidentiality vis-à-vis the patient, he proposed a system in which each parent had the right to call the therapist any time the parent had a concern about the patient's behavior. In addition, the therapist proposed that in the first phase of therapy, the parents should attend one session each month. The therapist

believed that these conditions represented the only hope of getting accurate information about the patient at this stage of the treatment. A case such as this represents the limits of treatability, as we discuss elsewhere.

Moving from the Contract to Therapy

The therapist must decide when he or she and the patient have achieved a good-enough agreement to end the discussion of the conditions of treatment and move on to the therapy. At that point, the therapist offers a simple statement, such as "It seems we have enough of an understanding about working together to begin the work. At this stage, if you do not have any more questions, let's move on to the therapy and turn to what thoughts and feelings come to your mind."

The Contracting Process in the Course of Therapy

The therapist may have to return to contracting issues in the course of the therapy. Sometimes a new problem arises that was not present at the beginning of treatment and sometimes the patient does not adhere to the conditions of treatment established in the initial contract discussion. In the first case, the therapist should feel free to take time to address the need for new parameters: "Since we have this new problem in front of us (e.g., missing sessions, drinking), we should discuss what conditions of therapy would help both deal with the problem and avoid its taking us away from our work in therapy."

If the problem is nonadherence to the contract that was initially agreed upon, the therapist works both to reestablish the parameter of treatment that has been breached and to interpret the meaning of the breach. Since patients with pathological narcissism and NPD have difficulty with accepting rules and with dependency issues that may develop, the therapist avoids giving ultimata. However, the therapist has to alert the patient that he or she has created a situation that puts the treatment at risk. An appropriate attitude on the part of the therapist is not "If you go on one more drinking binge, that will be the end of the therapy" but rather "We've discussed both the problem of your binges and ways to address it. Nevertheless, you've now reported two episodes in the past month. We can continue to try to work on this problem as we've set out to do, but if there is no improvement over the next 2 or 3 months, then we should revisit our treatment plan to see if this is the best type of treatment for you or if some alternative might be more effective." The point is that the therapist is doing what he or she can to maintain the treatment

but that unabated acting out on the part of the patient may undermine any chance of the treatment succeeding.

Summary and Conclusion

In TFP, the treatment contract is essential to carry out effective therapy. In the early years of TFP, the role of the contract was seen mainly as a means to create enough of a sense of order and security to diminish the chaos and anxiety that could interfere with efforts to treat patients with BPD (Yeomans et al., 1992). With increasing clinical experience, it has become clear that the contract serves an equally important role in establishing a sense of the reality of the therapeutic relationship and viewing deviations, on the part of either the patient or the therapist, as opportunities to better understand the internal world of the patient as it is enacted in relation to the established frame of treatment. The particular challenges of patients with narcissistic pathology have an impact on the contracting process of which the therapist must be aware.

CHAPTER 7

TFP-N Early Phases

In this chapter, we describe the early stages in TFP-N that are designed to address the multifaceted nature of narcissistic disorder. Because TFP-N emphasizes the individual's identification with both the self and object poles of the mental representations that comprise the internal world (that is, the grandiose self/devalued other; vulnerable self/idealized other), it is uniquely suited for narcissistic individuals with different phenotypic presentations, fluctuating mental states, and diverse contradictory or compartmentalized patterns. As described in Chapter 4, in TFP-N we first focus on the various manifestations of the grandiose self as they emerge in the patient–therapist interaction, as a route to identifying the specific internalized object relational dyads that comprise the grandiose self. Although we identify grandiosity as a core trait of narcissistic patients from early on, whether expressed overtly in socially dominant, arrogant attitudes and behaviors, or covertly in the form of preoccupation with convictions about greatness expressed in grandiose beliefs, cognitions, and fantasies, we do not necessarily confront grandiosity per se in the early stages of therapy. Rather, we focus on identifying the contradictory aspects of the grandiose self, such as the entitled and exploitative attitudes and behaviors that coexist with intolerable states of vulnerability, emptiness, inadequacy, and shame, as they are evoked in the moment-to-moment interaction with and experience of the therapist. We also focus on the costs and benefits of grandiosity, including the sense of specialness and uniqueness it confers, and the admiration or even adulation it may evoke in the interests of defining the grandiose presentation of the self-concept, a fundamental feature of pathological narcissism and NPD more clearly. There is also a sense of being left behind in developmental tasks because of a lack of investment in relationships, or in more mature

patients, the often subtle sense of futility, inauthenticity or meaninglessness despite considerable satisfaction in professional achievements or social endeavors. This is because the grandiose self is organized around external admiration and not an internalized world of deep and meaningful relationships, so that the individual may have the sense that nothing memorable or sustaining has happened and may wake up in midlife with "a desperate sense of years lost" (Kernberg, 2008b, p. 301). In the early stages, it is most important to understand and empathize with how the grandiose self defends against the underlying anxieties about dependency, relying on others, fears of humiliation, and exposure that may be evoked in the therapeutic situation.

Initially, it is particularly important for the therapist to contain the projection of the narcissistic person's intolerable devalued, vulnerable aspects of self as a first step to interpreting their defensive functions. Interpreting in the projection involves clarifying the patient's experience of the therapist, or others in the moment, without challenging it or suggesting it has anything to do with the patient's motives or internal world. This allows the therapist to gauge which aspects of his or her experience the narcissistic person can own and which he or she must disown to avoid intolerable pain and anxiety, fostering the capacity to reflect on why this is the case. Holding or containing the projections while gradually introducing an alternative perspective lessens their toxicity and allows them to be reassimilated and understood. Only gradually, as the patient can tolerate it, do we introduce alternative perspectives that foster a more integrated sense of self and others. Thus, with narcissistic patients in the early stages of treatment and beyond we continually balance containment and interpretation. This is done initially through interpreting in the projection, which is designed to bypass the patient's sense of vulnerability, thereby avoiding the provocation of shame or humiliation that those with pathological narcissism often experience.

The importance of interpretation and containment in the treatment of narcissistic disorder is consistent with other, quite divergent theoretical perspectives, although the balance between the two techniques may vary according to the theoretical orientation of the clinician. Joseph (1959), from the neo-Kleinian perspective, talks about the necessity of the therapist containing the projection of "dependent and needy," as well as the "envious and hating" (p. 216) parts of the patient's self. Britton (2003), following Bion (1962, 1994), emphasizes the central role of the therapist in the "transformation of experience through the process of containment" (p. 21).

Those working from within the relational/interpersonal school advocate a shifting empathy–anxiety balance in the therapist's technique,

adjusted to what the narcissistic individual can tolerate at different stages of the therapy. Bromberg (1983) writes, "Interpretive work of a certain kind must be attempted right from the beginning if 'empathy' is to have any meaning beyond a quasi-artificial technical maneuver" (p. 378). Mitchell (1986) also describes work with narcissistic patients as a balancing act between accepting and clarifying the nature and function of the patient's grandiose illusions while interpreting how they are linked to atavistic object ties and have functioned as routes to preserve them.

Although much has been written about both the pitfalls and necessity of interpretation with patients who are narcissistically disturbed, there have been few attempts to delineate the specific steps in the interpretive process that might enable the working clinician to use an interpretive psychodynamic approach successfully with narcissistic patients. In the following section, we outline the object relations–based interpretive process in TFP-N, as it has been adapted for the treatment of patients with pathological narcissism. Although each patient–therapist dyad has its own unique treatment trajectory, we found that the guidelines delineated below have been helpful in engaging patients with narcissistic disorder and in working productively in the transference to explore the difficulties that bring them to therapy, as well as in expanding their capacity to work through these difficulties. We describe these as 'phases' of intervention but would like to emphasize that the therapist will repeat cycles of these phases numerous times in the course of each therapy.

More specifically, we focus on:

Interpreting in the projection (phase 1). The first phase of TFP-N involves clarifying and containing the dominant affects or lack of them that color the experience of the therapist and others in the moment. Containment often occurs through holding the projection about the therapist and others—that is, focusing on the patient's immediate experience without challenging it or linking it to the patient's history or aspects of the internal world.

Defining the dominant internalized object relation (phase 2). Phase 2 involves identifying the ideal self and object representations that defensively exaggerate and preserve the individual's grandiose self-concept. We also identify the vulnerable or intolerable aspects of self that are often defensively and rigidly projected onto devalued others with the goal of helping the patient to recognize his or her identification with both aspects of the object relation. Identifying the costs and benefits of a grandiose self-concept is often a crucial step in identifying the dominant object relation.

Working with role reversals (phase 3). The dominant object relation is repeatedly activated with role reversals, wherein the patient may alternately enact the self-concept and project the concept of the object onto the therapist, or other; or alternatively identifies with the internalized object and treats the therapist as though he or she were the self, often without awareness.

Identifying object relational dyads with opposing affective valence that defend against each other (phase 4). For example, for many narcissistic patients the experience of a superior, omnipotent self in relation to a devalued other for whom one has contempt may defend against the experience of a gratified, dependent self in relation to a responsive, caring other for whom one feels love and gratitude. This positive dyad must be defended against because the narcissistic patient's internal experience prohibits or bars any trust/belief in it. The emergence of dyads with such a positive affective valence reflects the emergence of deeper layers of the object relational world beyond the superior/devalued dyads that comprise the grandiose self and hearken its dissolution. This paves the way for the emergence of a more varied, complex, and nuanced experience of self and others reflected in the multifaceted transferences that develop in the later stages of therapy.

Interpreting the unconscious conflicts, wishes, fantasies, and motivations underlying the grandiose self, along with their connection to past relationships and experiences (phase 5). It is through interpretation of the unconscious conflicts, wishes, and fantasies, and the split-off object relational dyads in which they are embedded, as they are lived out in the transference, that the patients eventually come to tolerate an *integrated representation*, one that is no longer as threatening to them, and thus does not require the splitting-based defenses that drive so much of the phenomenology of pathological narcissism and NPD (Caligor et al., 2009; Diamond & Hersh, 2020; Kernberg et al., 2008; Yeomans & Diamond, 2010).

Shifts in the patient's representational world evolve through a process whereby the therapist tolerates and contains the projections of needy, dependent, despised, hateful, sad, and envious feelings through the process of projective identification. Interpreting such dreaded aspects of the self, as they are experienced in the transference, leads to their modulation and integration, and ultimately the internalization of the therapist as an object capable of understanding, reflecting on, and integrating multiple contradictory aspects of the self and others (Bion, 1959; Britton, 2004b; Loewald, 1960; Blatt & Behrends, 1987; Kernberg, 2015).

In this chapter, we focus on the first three phases of our object relations interpretive approach with illustrative case examples. Chapter 8 focuses on phases 4 and 5.

What Makes Interpretations Effective?

Crucial in each phase of the interpretive process is recognizing, empathizing with, and containing the anxieties around dependency, fear of exposure, vulnerability, or aggression emanating from self or others that may have catalyzed the development of pathological narcissism to begin with. The organization around a grandiose self in many cases is predicated on the denial and omnipotent control, not only of others but also especially in more disturbed narcissistic individuals, of aspects of internal and external reality. Therefore, interpretations at any stage of the process must be delivered with tact, empathy, and meticulous attention to the patient's response. Interpretations convey not only the therapist's understanding, observations, and hypotheses about the split-off or unconscious aspects of the patient but also the therapist's entire emotional attitude toward the patient (Rycroft, 1956; Kernberg, 2014, 2015). Because narcissistic patients may experience interpretations as critical judgments, interpretations are most successful if they function to help the patient to . . .

> give voice to subtle personal experiences that the patient is struggling to articulate. For example, to state that the patient is being "resistant" in dealing with an issue is an interpretation from an external perspective. But to note how difficult it is for the patient to discuss a topic can capture the patient's feelings and struggles in a way that enables the patient to feel understood rather than being judged or criticized. (Blatt & Behrends, 1987, p. 287).

Interpretations, if accurate and well-timed—that is, offered on the cusp of the patient's evolving self-understanding—foster the patient's sense of making emotional contact with the therapist, who understands the patient's conflictual feelings, wishes, needs, and motivations as intelligible and coherent (Blatt & Behrends, 1987; Kernberg, 2015; Rycroft, 1956). We recognize that change does not take place only through an interpretative process (clarification, confrontation, and interpretation) but through the internalization of aspects of the therapeutic relationship. These include the therapist's interpretative function; concerned neutral stance, which involves empathy with myriad aspects of the patient's conflicts and object world; attunement to the patient's affective state; and

the capacity to hold in mind a view of the patient's current characteristics including non-narcissistic ones, and possibilities for growth and change beyond the narcissistic pathology. These all foster the patient's capacity to reflect on and integrate extreme, disparate, and split-off aspects of his or her experience.

Dealing with Narcissistic Resistances

Either because of constitutional factors or in response to early disappointments in significant others, many individuals with narcissistic pathology have withdrawn from the wear and tear of reciprocal relationships. They have retreated to an internal fortress, comprising combined ideal self and object images, that insulates them from genuine emotional investment and engagement with others, and the full experience of recognizing the loved and hated, gratifying and frustrating aspects of objects, both internal and external. As Joseph (1989, 2013) points out, as long as projections of aspects of the self are spread over a number of people, overwhelming affects and anxieties are avoided, and overt distress curtailed. However, in the intense dyadic arena of psychotherapy, particularly with treatments like TFP-N that focus on the transference, anxieties are likely to be activated around feelings of love and dependency, and envy and hatred that may have catalyzed the retreat to the illusory grandiose self to begin with. Not surprisingly, then, initially transference interpretations in particular are likely to be disparaged, ignored, denied, discounted, or appropriated. In the latter situation, the narcissistic individual may acknowledge the value of the therapist's observations but will immediately appropriate the interpretation as stemming from the self, saying things such as "Well, that makes sense but you're not telling me anything I didn't already know," or "What you just said indicates progress on our part, well maybe on your part, but I already knew that." In such situations, the therapist's contributions to understanding the problematic aspects of self and relationships may be valued and recognized, but they are almost immediately incorporated as aspects originating in the self. Such introjective mechanisms serve to reinstate the illusion of self-sufficiency, deny separateness between the self and other (Joseph, 1989), and short-circuit dependency.

Another form of incorporation of the therapist's interpretation involves "intellectual learning." This is the process by which the patient purports to learn from the therapist, recognizing and commenting on what he or she believes are the theories behind interpretations, or continually evaluating the therapist's contributions (that was good and insightful, or bad and unhelpful), but without any genuine reflection on the

content or meaning of the interpretation or any acknowledgment of its resonance in the emotional life of the patient.[1] Narcissistic individuals' difficulty tolerating interpretations stems from their inability to accept evidence that anything good or valuable in the therapist is separate from them, which in turn protects them from seeing the therapist as a separate person. If they did, they would risk exposure of disowned aspects of the self and/or experience envy of the therapist's expertise and capacity to understand (Joseph, 1989; Kernberg, 2015). Thus, for individuals with narcissistic disorder, there is a fear of the independent, creative mind of the therapist, working to link aspects of the patient's experience that he or she is invested in keeping apart (Britton, 2004b).

Individuals with narcissistic disorder may present with a pressing need and desire to be understood, but they also may "appear to hate the very idea of being understood and in fact will try to disavow it and get rid of all meaningful contact" (Steiner, 2011, p. 372). Thus, the paradox of interpretive work with narcissistic patients is that they may have a hunger for the therapist to register, contain, mark, and understand their experience. Yet interpretations about the meaning of such experiences, and particularly interpretations that call attention to the transference or that challenge defensively held, or inflated (or deflated) self-representations, activate the individual's shame response, evoking overt hostility and contempt, and/or increased projective maneuvers. In addition, such interpretations may also activate fears of dependency and attachment. Consequently, the therapist's interpretive efforts are often met with knee-jerk rejection, arrogant dismissal, or retreats into pseudostupidity. Even when there is superficial or facile agreement with the therapist, or effusive praise for the therapist's acumen or intellect, the narcissistic individual often does nothing to advance the interpretation, thereby depreciating the content and the messenger.

These characteristic responses to interpretation may stem from the fact that for narcissistic patients, objects are kept "internally paralyzed," often in an immature ideal state, and if the therapist is felt to be too separate or too perceptive about the patient, it can precipitate antagonism or even destructive attacks (Feldman & Spillius, 2003, p. 561). The interpretive process in TFP-N is a stepwise process designed to bypass the activation of such formidable resistances, which lead to dropout,

[1]This risk is indirectly supported by Kanninen and colleagues' (2000) research showing that therapists tend to shift to cognitive interventions when working with patients with a dismissive internal working model of attachment. In addition, an fMRI study revealed the activation of a social aversion network and low levels of friendliness in the listener in response to a dismissing narrative (but not a secure or preoccupied narrative; Krause et al., 2016).

stalemates, or unproductive endless treatments. Identifying, addressing, and working with the self and object representations in which the conflicts and anxieties that the grandiose self defends against are embedded is the primary way of circumventing narcissistic resistances. It is an essential aspect of work with individuals with narcissistic disorder and constitutes a major aspect of TFP-N at every stage of the treatment process. Interpretation disrupts the narcissistic illusions of omnipotence and total self-sufficiency and helps the individual to understand how representations of actual, wished-for, feared gratifications, and/or frustrations with primary objects are encapsulated in such illusions. It is only by interpreting the latter as they are reexperienced in the transference that the narcissistic structure can successfully be addressed so that the patient can experience the self and therapist as separate beings increasingly free of distortion by narcissistic needs and projections.

For example, a patient whose self-concept as superior and invincible was melded with an idealized concept of his mother as beautiful and invulnerable saw any expression of need or dependency on the therapist as weak and deplorable. The patient imagined that the therapist would experience him as whining and complaining if he talked about what was on his mind, which limited his capacity to speak freely about the difficulties that brought him to therapy, including his dissatisfaction and sense of isolation in his current relationship, and his anxieties about performance at work. He could begin to experience genuine dependency on the therapist only after the ways in which his illusion of being perfectly self-sustaining and invincible was interpreted as it occurred in his retreat from emotional contact with the therapist in the moment-to-moment flow of the session. This illusion was tied up with his wishes to be like the mother he experienced as desirable but distant and this was being lived out in the relationship with the therapist. The patient's identification with such an emotionally unavailable and unattuned maternal figure, along with frustrated wishes for caregiving, rendered him emotionally unavailable in the transference (narcissistic transference).

Navigating through Resistance

How do we respond to patients who are internally highly reactive, and who tend to meet our interventions with challenging responses? What appear to be narcissistic resistances on the surface may often stem from identification with powerful, idealized, critical, or even punitive objects that have been merged with the individual's sense of self and with whom the individual remains internally bound up. Recognizing how interactions with the therapist might be shaped in part by object relational patterns comprised by the grandiose self is particularly difficult in the initial stages. Consequently, we have identified several strategies that

have helped us to understand our patients' difficulties engaging collaboratively with our interpretative efforts, and to understand how these difficulties follow from an internal structure characterized by the pathological grandiose self. These strategies affect a good balance between the need to push forward with painful understanding and insight, necessary in the service of growth, and containment, a pulling back to help regulate the intensity of the patient's affect in the midst of this interpretive work.

Calling Attention

Simply drawing the patient's attention to a pattern in the process—for example, to the patient's knee-jerk rejection of the therapist's communications—can be helpful in raising the patient's curiosity about the process and in sustaining contact through difficult exchanges, such as "This seems to be one of those instances when it's difficult to hear me"; or "I think it's happening again, where you move away"; or "Oh, here we go again, I see we're arguing with one another—we are caught in it now. Can we think for a minute together about what might be going on?" These process-level interventions mark recurring relational patterns, noting, for example, a patient's defensive shift away from affectively laden material, or noting that an exchange between the patient and therapist has become tense, hostile, or otherwise nonproductive, impeding more reflective communication. These interventions of calling attention to what is happening in the dyad also support the construction of a shared experience. It helps get the patient–therapist dyad back on a reflective track in the moment, opening a possibility for mutual observation and understanding of something important taking place in the transference.

Postponing Interpretation

Standard TFP technique encourages the tactful but prompt presentation to the patient of contradictions in his or her emergent narrative (i.e., confrontation or "bids for reflection"). With patients with narcissistic pathology, such an approach is often met with rageful, contemptuous, paranoid, or otherwise dismissing responses that shut down the opportunity for productive exploration. An alternative approach would involve more extended periods of clarification as we "lie in wait." Kernberg (1975) suggested that our interpretive efforts may land more effectively when patients have "achieved some distance" from the immediacy of the patient–therapist exchange, and display their own "spontaneous curiosity" about the same.

Invariably and specifically with narcissistic patients more prone to transient defeats, depressive affects, and narcissistic vulnerability,

there are shifts away from the grandiosity to a self-state characterized by failure and self-loathing, as the patient's defensive processes break down, resulting in a momentary return of that which had been projected. These windows into our patient's vulnerability arise organically, without any active interpretive effort, and present valuable opportunities for us to support the patient's awareness of the otherwise dissociated self-representations and to begin to raise his or her curiosity about the divergent ways he or she experiences the self and others. With this "postponing" strategy we can momentarily disarm the typical defensive posture of dismissal and devaluation or idealization long enough to capture some reflective space in the patient's mind, so that the work of clarifying the internal object world and how it plays out in work and relationships, and in the transference, can be more effectively examined. among narcissistic patients whose defensive barrier is stronger, where primitive defenses work more effectively to avoid any sense of fragility or weakness (Rosenfeld's [1987] "thick-skinned" patients), such a strategy may not be so effective, and a more direct interpretative approach is called for. In contrast, with "thin-skinned" narcissistic patients, whose defensive barrier is less robust, the oscillation or cycling between the poles of the patient's sense of self—strong and self-sufficient versus fragile and helpless—will invariably occur without our active interpretive effort but simply as a function of the patient's response to challenges in his or her daily living.

Spontaneity and Humor

The nonformulaic, spontaneous use of language that emerges in the treatment context can be used to address negative qualities in the self usually managed through denial or projection, thus facilitating a link between these tendencies and various object relations, and their oscillation in and outside of the transference. The introduction and mindful use of "play," as well as humor and creativity between patient and therapist, can help the patient observe and take ownership of, for example, his or her aggressive and dependent selves, bringing closer to his or her self-experience that is often projected outward, and with a diminished sense of its danger and toxicity (Coen, 2003, 2005).

A young man with a particularly voracious need for recognition, validation, and acceptance would invariably, at moments of great anger or frustration with the therapist, remind the therapist of how much money he had earned from the patient over the course of the several years of treatment. The patient's need to distance himself from his use of others for narcissistic supply was facilitated by his projection of his greed, manifested by his sense of being exploited. The patient would

gleefully intone "cher-ching" (the sound of a cash register) at moments when his vulnerability and dependency upon the therapist would dissolve in a regression to a psychopathic transference in which everyone is out to get what he or she can from another (Kernberg, 1992). The therapist began using the expression, "cher-ching," such that it eventually became part of the common language in the treatment, shorthand for the emergence of a particular object relations dyad, along with an understanding, born out of the interpretive work of its defensive functions.

Such an approach does not suppress the negative transference, but rather can allow its expression to be tolerated in a workable form in the moment. Furthermore, the therapist's ability to "play" with such language in an interpretive and challenging context conveys implicitly that the therapist can survive the patient's attacks, in fact using them to facilitate integration and sublimation of the same (i.e., engaging the patient's destructiveness in the service of growth; Coen, 2003). Furthermore, using language that has evolved in the shared treatment context helps the patient draw upon the experience in the treatment of a successful and productive exploration of a specific dyad, as well as its oscillation and defensive function. Doing so facilitates the patient's ability to persist in working with the dyad in its current expression in the transference, with the attendant aggression and frustration, thus promoting a richer understanding of the material, and ultimately an integration of previously split-off affects and experiences of self and object.

In TFP-N, then, our primary technique for dealing with narcissistic resistances is to identify such surface behaviors and attitudes as they emerge in the interactions with the therapist and then to translate them into specific object relational patterns that comprise the grandiose self. The rest of this chapter and the next focus on the application of the interpretive process with illustrative case examples to demonstrate the various phases already delineated in Chapter 4.

Phases 1 and 2: Clarifying and Containing the Immediate Affective Experience and Defining the Dominant Object Relations Activated in the Moment

The first step in the interpretive process is defining the immediate affective experience of self in interaction with the other as a first step toward defining the dominant object relation activated in the moment. As the therapist listens to the content and structure of the patient's discourse, including the patient's thoughts and feelings about the therapist and others, he or she begins to formulate the dominant representations of self in

relation to others that are being evoked. When the therapist has a clear picture of such a self–object dyad, he or she names the actors in the most prominent object relation(s) along with the linking affects, and assesses the patient's reactions. The dominant affects in the session (or lack of them) are often the conduit to the most salient object relational dyad operative at that moment. Hence, the first step is to clarify and name the affect or lack of it. With narcissistic patients, the dominant dyad is often more evident in how the patient is interacting with the therapist, with the affect tone of the words used, rather than the verbal content of the material the patient is reporting. Is the patient treating the therapist as a passive audience to the patient's productions, as a mirror to affirm the patient's sense of specialness, as a virtuoso who can magically solve the patient's difficulties, as a service provider to do the patient's bidding or solve immediate problems, or as a psychic twin who is perceived as more similar to the patient than he or she actually is? Is the dominant affect one of dismissing devaluation, unrealistic idealization, or total negation of the therapist's presence and contributions in the session? All of these stances provide clues to the nature of the patient's object relational world.

For many individuals with pathological narcissism across the spectrum, the initial object relational dyad that emerges is that of the grandiose self and ideal object, constituted by projecting the ideal self onto the other much like a shadow, not quite separate from the self. The individual may appear to be hermetically sealed in such an ideal self–ideal object configuration, like Narcissus entranced by the image of the perfect other, and impervious to the reality of others beyond their ability to echo or reflect ideal aspects of the self. Initially, this ideal self–ideal other dyad renders others, including the therapist, as shadowy and unreal. The patient may perceive the therapist, like many other objects in the patient's life, as there to bolster and support the individual's exaggerated sense of self-importance, to confirm that he or she is at the center of the patient's grandiose narrative, the hero or heroine of the story so to speak, or to function as an ideal superior figure who counters the patient's sense of inferiority and affirms his or her covert grandiose fantasies. But it is important to note that as in the case of Narcissus, this quest for the perfect mirroring other is a chimeral one that is fraught with misrecognition of the true nature of self and other. The optimal stance for the therapist at this point is not necessarily to mirror grandiosity but to maintain a spirit of curiosity and reflectiveness as the grandiose narrative unfolds, clarifying what function this serves for the patient, and how it protects but limits the experience of self and others so that the costs and consequences of the grandiose self-functioning come increasingly into focus. These may include broken ties with partners, coworkers, friends,

or children; or shallow relationships that become less sustaining as the individual matures; an inner sense of hollowness or futility; and sometimes a lack of pleasure in the larger realm of culture, art, or spirituality since all that is ideal is centered in the self and its own gratifications. The therapist also notes any contradictory aspects of experience or shifts of mental states outside the grandiose narrative (e.g., experiences of doubt, vulnerability, confusion).

In those with thick-skinned narcissistic organization, the grandiose narrative often involves a valorization of accomplishments, successes, and talents. The suffering that brought them to therapy is initially obscured in an effort to procure admiration and adulation from the therapist to maintain the self-equilibrium. In the case of those with a more vulnerable or thin-skinned narcissism, the initial narrative may involve adulation and admiration of others, including the therapist, with the presentation of self as denigrated, weak, damaged, or vulnerable. The focus may be on experiences of deprivation and maltreatment that center around the specialness, uniqueness of the patient's suffering, and/or the patient's self-loathing and sense of futility to effect any change (Cooper, 1998). The therapist is enlisted to bear witness to such rigid and fixed narratives that center on defeat and helplessness, but not to explore. Attempts to understand, clarify, or explore the meanings and functions of the unrelenting focus on suffering may be experienced as an attack. In such cases, where grandiosity is expressed covertly as fixed, immutable disparagement of self, the therapist who often initially empathizes or is deeply moved by the patient's suffering may feel increasingly constricted in his or her capacity to interpret, explore, or even think about the intrapsychic or interpersonal dimensions of the patient's self-critical attitudes since they are often readily projected onto the therapist (Symington, 1993).

Inherent in either position is the negation of the therapist as a real, separate, valued person, but many patients with narcissistic pathology across the spectrum have no conscious awareness of the superior self–devalued other or devalued self–superior other internal representations that underlie their initial stance of nonrelatedness. Kernberg (1975) refers to these transferences as self–self transferences since there is no specific object relation involved, but only the projection of aspects of the self (idealized or devalued) onto the other, who is then seen as an amalgam of aspects of the grandiose self. In sum, the narcissistic organization may initially configure the specific object relational dyads taking shape in the transference, interfering with the patient's ability to see the therapist as a separate person or to invest in the relationship with the therapist. In such cases, the absence of transference *is the transference*.

It is important for the therapist to keep in mind that the narcissistic individual is caught in the paradoxical situation where the illusions of omnipotence and self-sufficiency, and repudiation of dependence on the other, can only be sustained "when they are established not autonomously but interactively with the object" (Maldonado, 1999, p. 1132). As Money-Kyrle (1965) points out, "This form of projective identification by which one person manages to dispose of his inferior ego by putting it in someone else, and causing this other to actually feel inferior, can be brought about in ways so subtle that not even the aggressor, let alone his victim, is consciously aware of what is being done" (p. 149). For therapists who are sidelined or relegated to such a marginal role early in the therapy, it can be difficult to understand that this is the first step in the emergence of an interpersonal relationship in the therapeutic dyad. Thus, with patients with pathological narcissism across the spectrum, the therapist must tolerate a sense of being devalued in the patient's experience and resist the impulse to respond reactively. Accepting criticism and being devalued is an important therapeutic intervention. It is what the patient cannot do since he or she feels any imperfection is tantamount to collapse.

In the context of devaluation and attack, what is important at this early stage of therapy is to bring to light and build on nascent capacities for dependency, love, and object investment along with realistic aspects of the self-representation that may be evoked by the therapeutic situation. These often coexist with the grandiose self in varying degrees depending on the level of pathology (as discussed in Chapter 2). Examples of such nascent desire for help would be regular attendance in therapy and flashes of appreciation at the verbal or nonverbal level in response to the therapist's interventions, even if they are later negated. Nondefensive moments of eye contact between patient and therapist may also coexist with the climate of devaluation. Examples of the nascent capacity for investment in others would be devotion to a child, even if the child is seen primarily in the light of the patient's needs and projections, or dedication to caring for an impaired parent or spouse. Any traces of the capacity for love along with a realistic desire to be helped by the therapist may form the bedrock of a working alliance and a positive transference (Ronningstam, 2012). This allows for the analysis of the grandiose self, forming a bulwark for both therapist and patient to withstand and explore the defensive aspects of the contempt and devaluation toward others. Eventually they will explore its roots in early frustrations, disappointments, and traumas with early attachment figures, the full dimensions of which are usually opaque in the initial phases of therapy.

With clarification and confrontation of this initial dyad, more specific scenarios emerge, including the idealized grandiose self in relation

to a devalued therapist, or conversely, the superior therapist in relation to a devalued, inadequate patient. In either case, what we are seeing is an implicit object relation, rather than a fully developed object relation, representing an internalization of a relationship with a significant other. The most typical dyad is the grandiose self related to a depreciated object with the latter corresponding to the patient's own projected, depreciated self that cannot be tolerated or integrated. Indeed, with some patients this dyad emerges initially, and of course there are echoes of it in the patient's inclusion of the therapist in the circle of the ideal that deprives the therapist of any autonomy or individuality, annexing him or her to the position of mirror or observer.

Even when the devalued self-representation predominates, as in the case with those with vulnerable or thin-skinned narcissistic presentation, the good qualities of the other are often quickly appropriated to the self (Joseph, 1989, 2013). In fact, thin-skinned individuals entering therapy may resuscitate latent grandiose fantasies and wishes to be admired as the pinnacle of interpersonal relating, even though these fantasies may have been dormant prior to the beginning of therapy (Mitchell, 1986). This tendency toward rapid-fire appropriation of the valuable and ideal aspects of the other, rather than enhancing object relations, in fact denudes and depletes them. It interferes with the internalization of any aspects of the object that are not refracted through the prism of the grandiose self and immediately assimilated to it. Thus, the split, polarized primitive transferences of those with narcissistic disorder do not readily evolve in TFP-N as they do with patients with BPD.

At the most severe pole of the narcissistic spectrum, and particularly with those individuals who are organized at the lower borderline level, patients are likely to experience the therapist as voyeuristic or exploitative. In cases of malignant narcissism, the therapist may be seen as a dangerous or persecutory figure who must be defeated or destroyed (paranoid transference). Whatever the level of organization, these initial object relational scenarios are particularly tenacious and may predominate for long periods of time.

The tendency toward immediate appropriation of the positive aspects of the therapist, along with the tenacity of the projection of negative aspects of self, may prolong the patient's state of nonrelatedness to the therapist in the initial stages of treatment. Such projections inevitably create anxieties about the object, which show up initially in the denial of the therapist's importance, leading the therapist to feel ineffective or bored at best, or at worst, angry or frustrated and despairing. This is juxtaposed with relentless attempts to induce in the therapist feeling states that are intolerable to the patient's self. Alternatively, the patient may experience the therapist within the orbit of the grandiose

self, as omnipotent and omniscient, negating any attempts by the therapist to explore how clinging to such an ideal image serves the defensive function of not engaging more fully with others in external reality. In either case, in patients for whom the grandiose self-structure protects against intolerable anxieties around dependency or inadequacy, containment of these anxieties is essential to anchor the person to therapy. Such containment involves the therapist initially holding what is intolerable or unknowable to the patient, representing and reflecting on it in his or her own mind, and translating it into "a message that is digestible to the patient" (Busch, 2013, p. 116). In so doing, the therapist creates an atmosphere of safety in the treatment.

In these early stages of the interpretive process, containing and holding the projection should not be confused with direct mirroring of the patient's affect (Kohut, 1971). Rather, affective containment involves not only the process of accepting and tolerating the responses that the patient attempts to elicit in the therapist through projection but also the process by which, in a trial identification, the therapist actually tries on the complementary role in which the patient has cast him or her. This can provide a clue to the role in which the patient has cast the *self*, and to the dominant object relation being activated in the moment (Sandler, 1976). In so doing, the therapist embodies the role without enacting it, thus preserving the capacity to reflect on the object relation the patient is living out with the therapist and inviting the patient to do the same. Thus, although the therapist lends him- or herself to such trial identifications, he or she also functions as "an excluded third" who reflects on how the patient's different self-states linked to different object relational dyads operate in relation to each other and structure the patient's reality.

In the interpretative process outlined above, it is essential that the therapist pay close attention to the elemental aspects of good interpretation, such as timing, tact, clarity, relevance, and succinctness that most psychodynamic clinicians are well versed in (Busch, 2013; Yeomans et al., 2015). Optimally, interpretations emerge organically from the ongoing clinical process and engagement of the patient and therapist in the mutual exploration of the patient's internal world and external reality. In addition, however, it is essential, as Feldman (2007) points out, to maintain a balance between the conviction with which therapists formulate interpretations of their patients' communications, and a state of doubt that prevents interpretations from becoming "overvalued ideas, as opposed to a useful way of organizing and integrating their observations for the time being" (p. 374).

The ongoing dialectic between doubt and conviction in Feldman's (2007) view reflects the therapist's capacity to experience what is going

on in the sessions, while at the same time actively formulating an understanding of the patient's communications, including the communications expressed through projection into the therapist. If the therapist becomes fixed on a particular interpretation or fact, he or she may miss the ongoing flux, movement, and flow; this may include shifts in the introjective and projective processes between patient and therapist, and the glimmers of both positive and negative aspects of representations of self and object as they emerge. The therapist's flexibility and openness to such minute relational shifts in the session, including shifts in the way the patient is representing the therapist, counters the inflexibility of the patient's grandiose self-organization. Interpreting with an optimal balance of doubt and conviction—a balance that may shift depending on the nature of the patient's material, stage of therapy, and the state of the transference and countertransference—helps the patient to recognize that interpretations are hypotheses that may be confirmed or disconfirmed only through further collaborative work. The therapist's capacity to remain open and receptive to the panoply of feelings and the mental representations to which they are linked—both negative and positive—sets the groundwork for the dissolution of the compensatory grandiose self into its component aspects of idealized and devalued self and object representations. These may emerge in fleeting, inchoate ways as therapy progresses.

Identifying the panoply of mental representations that underlie the grandiose self is gradual and problematic because the defensive organization around an inflated sense of self-importance leaves little room for exploring alternative realities about the self (e.g., vulnerability, self-depreciation, humiliation, dependency, or envy). As a result, clarification of the patient's experience of self and other in the moment, the first stage of the interpretive process in TFP-N, is also more difficult because the affects of humiliation, envy, fear, and dread, particularly of imperfection and dependency that devolve from the grandiose self, are defended against.

With narcissistic individuals in the early stages of TFP-N, we focus to a greater extent on the patient's immediate experience of the therapist, encouraging him or her to put the immediate, concrete affective experience of the other into words. We do this without suggesting that it has anything to do with the maladaptive, distorted object relational dyads that structure the patient's interpersonal experience, nor do we make any linkages between this experience and the patient's history or conflicts (Steiner, 1994; Caligor et al., 2009; Diamond, Yeomans, et al., 2011; Diamond & Hirsch, 2020). We move only gradually—as the patient can tolerate them—into interpretations that link the patient's experience of

the self to mental representations that encapsulate unconscious conflicts, impulses, and motivations. Observing the part of the patient's internal world that is disavowed and deposited in the other, including the therapist, is less threatening to the patient's rigid and fragile defensive system (depending on the level of organization).

For example, a successful entrepreneur with a history of rejection from an emotionally abusive and relentlessly critical father entered treatment when a business he had backed financially collapsed, leaving him facing a potential bankruptcy. Initially, he belittled the therapist for his inability to adequately understand the intricacies of this catastrophic financial situation. The therapist did not immediately link this demeaning attitude to an object relational dyad of rejecting, critical other and an unworthy self but instead observed how disappointing it must be to have a therapist who might not fully comprehend the financial complexities of the situation, and thus might not be able to understand his emotional devastation. Such interpretations are designed to provide cognitive containment of concretely experienced affect states or to identify what is the dominant overriding representation in the patient's mind. This approach of interpreting in the projection is consistent with what Steiner (1994) calls "therapist-centered" interpretation, but it focuses not only on projection of aspects of self onto the therapist but also onto others, along with the defensive maneuvers, fantasies, and affects involved. Interpreting in the projection is designed to bypass, at least at the outset, intolerable feelings and self-states, such as the patient's sense of weakness, inadequacy, and confusion. Clarifying and accepting for the moment the patient's experience of the other, including the therapist (i.e., the object representation), without calling attention to the way it may be defensively distorted, helps the patient to feel understood and contained. In contrast, *patient*-centered interpretations focus on clarifying and exploring the patient's feelings, thoughts, and behaviors along with the motives and anxieties behind them.

Interpreting in the Projection

Once the patient's immediate experience of the therapist has been defined through clarification and confrontation, we work on interpreting in the projection with the understanding that projection and projective identification are always dyadic experiences that may catalyze shifts in both the patient's and therapist's experience. In other words, the therapist may experience the self in accord with the disowned pole of the object relation. This identification and its containment help the therapist to identify the dominant object relational dyad in his or her mind, as a

prelude to interpreting it. Even after a dominant object relational dyad has been recognized, it is more difficult for individuals with narcissistic disorder to recognize their identification with the defended-against pole of the dyad than is the case with other personality disorders.

Thus, the goal is to put the emphasis on defining the patient's immediate affective experience of the therapist, using his or her own experience of the patient as a guide, as a way of mobilizing, observing, and discussing the dominant object relations as they take shape in the transference. This is in contrast to interpreting a dominant object relation right off the bat before the patient has a full affective experience of it, or capacity to tolerate his or her identification with both aspects of the dyad. For those across the spectrum of narcissistic pathology, such an affective experience is truncated, since affect is inherently object seeking (Fairbairn, 1952; Kernberg, 2015)—that is, it involves acknowledging not only the need and desire for something in relation to the other but also the other as an autonomous being separate from the self. Hence, defining the patient's dominant affective experience of the therapist (or absence of it) is the first step toward activating and defining the total object relation activated in the transference. The latter is more effective once there is a reciprocal interchange of self and object representations in the transference. This may proceed more slowly with more rigidly organized, aloof narcissistic individuals who refuse to project or experience aspects of their internal world, beyond a grandiose sense of superiority and/or the unilateral devaluation in relation to the therapist out of fears of dependency and/or exposure.

The focus on the patient's experience of the other, including the therapist, temporarily bypasses the provocation of the narcissistic patient's shame, humiliation, or rage, since it is more tolerable for him or her to see flaws in the other than it is in the self. For example, a patient who had seemed to be progressing in treatment flew into a rage at the therapist and threatened to quit therapy because she felt that in the previous session the therapist had missed a cue about how she was feeling rejected by her boyfriend. She had actually only made passing reference at the end of the session about how her boyfriend had not returned her call, focusing primarily on how perfect the relationship was for her. In such a situation, interpreting in the projection initially would involve a statement of how difficult it must be to have a therapist who did not understand the painful experience of rejection that she had had difficulty conveying; only later would the therapist explore the patient's motivations for withholding information (e.g., fear of exposure, envy of the therapist) that might have been humiliating to reveal (patient-centered interpretation).

Insofar as the narcissistic patient is intolerant of perceiving flaws in the self, one aspect of technique is to focus more on negative feelings, such as humiliation, weakness, or shame *as they are projected onto the other or the therapist*, or on positive idealizing experiences of the other, including the therapist. Interpreting in the projection is particularly important with narcissistic patients who cannot tolerate seeing imperfections in the self but may be able to observe them in the therapist and thus can reflect on what it is to have limitations. An implicit function of interpreting in the projection is thus to communicate that a person (the therapist) can be imperfect and still continue to exist and function without the total collapse the narcissistic patient imagines is the alternative to perfection. At the same time, the therapist must convey to the patient the firm conviction that he or she has the skills and knowledge to help the patient to safely explore his or her experience, even dreaded and disowned aspects, and to work through the difficulties that brought the patient to therapy. Negative projections are contained but not accepted as an objective reality.

The capacity to work with such projections with tact and authenticity requires that the therapist tolerate the ferocity and toxicity of the patient's intolerable feelings and self states without returning them to the patient prematurely (Steiner, 2003; Joseph, 1989, 2013). This requires that the therapist has done the work of taking in and bearing "the truth of what he or she has become in the patient's psychic reality" (Steyn, 2013, p. 1094). Of course, this necessitates that the therapist contain the feelings that are evoked in him or her, as opposed to discharging them in interpretations that might be premature (Steiner, 2006). Rather, the therapist contains and processes the experience of the patient's projections in the interests of ultimately using these as building blocks for interpretations not only of the process by which the patient projects affects and representations onto the therapist but also his or her identification with these projected aspects (projective identification).

In sum, the goals in the initial phases of treatment are (1) to identify affect while containing it; (2) to identify the constellation of feelings and representations that have been activated through everyday experiences, including the experience in the session with the therapist *as a first, more palatable conduit into the internal object world of the individual, in particular, the idealized and devalued representations that comprise the grandiose self*; and (3) to help the patient to tolerate imperfection by seeing the dreaded aspects of his or her self and internal objects as they are projected onto the other or therapist, who first contains them and then, through his or her interpretive activity, helps the patient to explore, understand, reflect on them, and ultimately reintegrate them as an aspect of the self.

Case Example: Early Phases of Interpretation in TFP-N for NPD

The treatment of Mark, a relatively high-functioning narcissistic individual (discussed in Chapters 1 and 2), illustrates the early phases of interpretation in TFP-N. Mark, an accomplished architectural engineer and a partner in a leading international firm, entered therapy because of symptoms of anxiety and depression. These symptoms were related to conflicts with his long-term relationship partner. He had been with his girlfriend on and off for the past 10 years, 5 of which they had lived together. They were both in their mid-30s, and she was pressuring him to commit to having a child with her while she was still able to do so, or to go their separate ways. He had broken up with her several times to pursue other relationships, only to return to her when it looked like he might permanently lose her to someone else. Although she was an extremely accomplished interior decorator whose work had been featured in major magazines, he worried that interior decorating was an inferior occupation and that his associates might judge her as not worthy of him. Initial assessment also revealed that there were unsatisfactory and even punishing aspects of the relationship. Although his girlfriend professed to adore him, she made him feel inadequate for not making more money, constantly comparing him to the more affluent partners of her friends, and expecting him to provide a lifestyle comparable to her privileged upbringing. Although he often rejected her sexually, he also wondered whether his sexual performance was adequate since although she desired him, she also made denigrating remarks about their sex life. Relationships with significant others who create exacting standards and inordinate demands to which the individual submits masochistically often go hand in hand with exploitation and entitlement in those with narcissistic pathology. The sensitivity to critical others, even if devalued, often derives from identification with idealized and persecutory internal objects that make impossible demands on the self.

Mark had not been able to resolve his conflict about the relationship in several former treatments and began dealing with his ambivalence by secretly seeking Internet liaisons. An accomplished architectural engineer who had been promoted to partner at an unusually young age, he was coasting on his past accomplishments at work, and was brought up short when he was told that his compensation would be cut unless he developed new business for the firm. This was expected of him as a partner and was necessary in a recession. A previous CBT treatment had helped Mark with his performance anxiety at work, but not with his relationship issues. He had been able to advance in his career, but the underlying narcissistic structure had not been resolved. This was evident

in his torturous self-doubt that was obscured by his grandiose sense of superiority, by his tendency to find gratification in ideal fantasy objects, and the harsh critical elements of his internal world that were not in sync with realistic self-appraisal. Mark also displayed a pattern of experiencing interest and excitement in superficial contacts but then anxiety when he began to enter into deeper, more complex reality-based relationships.

Before starting the current TFP-N treatment, Mark had consulted with several well-known clinicians but found none suitable. They were described in derogatory terms: They did not understand him, they were of no help to him, or they simply reflected what he told them. In the initial consultations, the therapist suggested that a narcissistic disorder—which involved, among other characteristics, a chronic pattern of fluctuation between overestimation of his capacities and collapse into states of self-doubt and paralysis—seemed to underlie his difficulties with work and relationships.

Mark accepted the diagnostic feedback and in fact said that none of the therapists he had consulted had understood the nature of his difficulties in those terms. In the initial stages, there were flashes of the ideal self–ideal other dyad, although these were also shorn of any genuine sense of relatedness or dependency, and often quickly devolved into their opposite sense of devaluation. For example, in the initial sessions, Mark balked at certain parameters of the treatment contract, including the therapist's insistence that most sessions take place in person and in her office rather than on the phone, which he felt was unreasonable given his complex travel schedule. The therapist called attention to the implicit devaluation in this stance along with the ways this might function to keep her at a distance, putting a "screen" between them, as well as enabling him to remain in control by relegating the therapist to the role of spectator. She explained the necessity for in-person sessions, as well as other parameters of therapy by describing the contract as a foundation for the therapy, just as an engineer would require a solid foundation when building a bridge or other structure, using a metaphor related to his profession that she thought might bring the point home. His initial response was a scornful repudiation of any analogy between her work and his. He finally agreed to these and other conditions of the contract only after he looked the therapist up on the Internet and found that she had written a book on sexuality in the era of the Internet that he believed made her uniquely suited to treat him, in the same breath dismissing this as a "pop psychology topic." This statement epitomized the striking mélange of idealization and devaluation that characterized the initial treatment phases.

Once therapy began, Mark filled the sessions with torturous ruminations about his relationship, which took the form of a litany of faults

that he found intolerable in his girlfriend. He talked about how it was unimaginable to him to have a child who would bind him to her further. He focused on his longings for an ideal partner, whose current form took the shape of a woman he had met on the Internet. Initially he attempted to direct the sessions, often beginning with a list of topics to discuss, organizing the communications to present a ready-made product to the therapist in lieu of free associations that could be productively explored. Attempts by the therapist to clarify or expand the range of the discussion were routinely blocked by statements like "We will get to that later, but didn't I just tell you that I wanted to focus on [X] topic?" Or in response to the therapist's observations, he would say, "That makes sense now, but I'll forget it as soon as I leave here because I never remember or think about anything my therapists say. . . . When I leave the session, it is as though it never happened." He also expressed feelings of futility and hopelessness that he would ever be able to truly fall and remain in love, and compared himself to peers who were in committed relationships and who were starting families. The therapist observed in the early sessions that just as Mark had not been able to commit to a relationship, it had been difficult to commit to the treatment contract that stipulated roles and responsibilities for both patient and therapist. She also observed that now that therapy had begun, it seemed that there was only space for one person in the room. He readily replied that he had not learned anything from any of his previous treaters that he did not already know and expected that it would be the same in the current therapy.

Initially Mark filled the sessions with material about immediate crises at work or in the relationship, sometimes wanting solutions to immediate problems, focusing on whether he should leave his girlfriend or not. The therapist reminded him that the goal of his therapy was exploration of his characteristic patterns that were contributing to his paralysis and dissatisfaction in his work and relational life, rather than providing solutions to immediate problems. He responded either by devaluing the therapist's contributions or by affirming them in a superficial fashion without much elaboration, reflection, or expansion. He evaluated sessions as to whether they had an optimal balance among "exploration, listening, and interpretation," which he had learned from previous treatments and from reading about psychodynamic techniques. Such aspects of discourse, although indicative of narcissistic resistances, may in fact also represent the first glimmer of making sustained contact with the therapist as a real, if devalued, object. The therapist observed that while this constant evaluation of the therapist's contributions kept him in control of the session and affirmed his sense of superiority, it also kept him from getting the help he needed. This brought the costs and consequences of his grandiosity into the transference.

The focus in the initial sessions was on helping Mark to articulate his own experience of the therapist that in the first months took the form of seeing her as an ideal therapist who might help him finally fall and remain in love since she had written about sexuality and attachment in the era of the Internet. The therapist attempted to clarify why he had been unable to leave his current relationship. She suggested that his girlfriend provided something he needed, and wondered what it was? He talked about how her adoration made him feel secure, but that she also made him feel inadequate for not making more money, constantly comparing him to the more affluent partners of her friends. Thus, through clarification, more vulnerable aspects of the self emerged.

Although, at times, Mark saw the therapist as an idealized figure who might solve his dilemma, more often, he saw her as flawed, inadequate, and unhelpful, and himself as the expert. His own sense of fallibility was intolerable, and his immediate need was that these negative aspects of self be contained by the therapist and understood in its projected state. In the initial stages of treatment, this stance that therapy is useless and all is futile, is often an indication of the depth of the patient's withdrawal from more authentic relatedness. Although in most cases, it might represent the concrete manifestation of the superior self–devalued other dyad in the transference, it also provides a window into the patient's painful feelings of emotional isolation.

Even though the devalued other also represents an aspect of the grandiose self that must be split off, it can be worked with productively because it involves an acknowledgment, if only through projection, of something beyond the ideal. However, with Mark, as with many narcissistic patients, initial attempts to call attention to his belittling, critical attitude were often deflected or ignored. An example of such an intervention might be: "I wonder if you are aware of the belittling tone of voice in which you made that comment. I have noticed that this tone escalates when certain issues come up here [giving examples from previous sessions]. Perhaps you are trying to put me at a distance in order to ward off exploration of the painful issues and anxieties that brought you to therapy." Such attempts to interpret the defensive manifestations of the ideal grandiose self–devalued other dyad in the transference by observing how it protected Mark from delving deeper into the conflicts that brought him to therapy received superficial acknowledgment. It was only after an interpretation by the therapist evoked a constellation of intense feelings that he could experience his identification with both sides of the ideal–devalued dyad, as the following clinical material shows.

In the third month of therapy, in a session in which Mark obsessed about his conflict between his girlfriend and his Internet partners, the

therapist observed that the choice seemed to be not between two sexual relationships but between investing in an actual relationship versus remaining sequestered in his fantasy world of Internet relationships where he could control and watch anonymous individuals play out his scenarios of dominance and submission. Mark arrived 10 minutes early for the next session and knocked insistently on the door 5 minutes before his session was scheduled to begin. When the therapist opened the door for him at the appointed time, he presented with rigid body posture and a tortured expression, stating immediately that he was thinking of leaving treatment.

MARK: I don't know why I came this morning. I am feeling very adversarial, and this feeling has grown—not diminished.

THERAPIST: It seemed very important for you to come in and tell me how adversarial you are because you came 5 minutes early. So perhaps you have some positive feelings as well, although you are considering leaving therapy.

MARK: Well, I did feel that we made some progress last week when you said to me that I am having trouble having a real relationship versus investing in a fantasy world; that the real conflict was not between my girlfriend and the women I meet on the Internet dating site, but between investing in an actual relationship versus remaining sequestered in a fantasy world. That was progress on our part, well maybe on your part—but I already knew that. And you misunderstood something crucial in the last session—I was talking about my girlfriend, and you failed to pick up on my statement about how she is feeling increasingly hopeless about our relationship and is not encouraging toward me this week. When we talked about my repairing the relationship with her it was totally wrong—rather than encourage me, she mocks me. And you were disturbingly incorrect when you asked about repairing our relationship; how could you not see that she has pushed me away? But then I guess I couldn't really tell you what was going on. This week she even talked about a separation.

THERAPIST: It sounds like I really disappointed you because I didn't understand intuitively that she was pushing you away even though you were having trouble telling me this directly. Maybe it's difficult to speak freely here when you feel like you are being put down or rejected.

MARK: Well, last week you ended the session just as I was in the midst of talking about this. In fact, you cut me off, and I am not one to be cut off.

THERAPIST: It sounds like you are experiencing me as a woman who will cut off and push you away like your girlfriend, and who will fail to care for you and meet your needs. That makes you fearful of speaking freely here and makes you want to leave therapy.

[The interpretation centers on the patient's experience of the therapist, interpreting in the projection, instead of the therapist observing that Mark is the one who cuts off by threatening to leave therapy and refusing to commit to his girlfriend.]

MARK: Yes, my girlfriend has been pulling away. She is away visiting her family and very monosyllabic on the phone with me. She said she doesn't know if she wants to go on with the relationship. I keep coming back to your comment about my fantasy life and how it sustains me, and how after a while I withdraw from all relationships.

THERAPIST: Perhaps that is happening here as well, because you started out saying we had made some progress, but then backtracked and said it was only I who had made progress, or that you knew this all along about yourself anyway, as though it was not something that had come out of our work together.

MARK: Well, she [the girlfriend] treats me like I'm a pathetic creature; she is not empathic to me anymore. She ridicules me for my paralysis and inability to stop going on the Internet. I don't respect her, but I'm not sure I can live without her.

THERAPIST: It's possible that you did tell me about this and I missed it, but it's also possible that you were so upset by her withdrawal that you couldn't tell me directly what was happening. Today I think you were experiencing me that way as well—from the start you saw me as someone who couldn't understand and empathize, who would judge you harshly. It must be difficult to have a therapist who doesn't understand your anguish. But maybe it's also threatening to have a therapist who understands you. I'm thinking about your appreciation of my comment in the last session.

MARK: *(After a few moments of silence)* I . . . I'm very anxious today because I have to do a presentation for a new office complex in Asia, and I am very worried that they won't like it and will think I'm incompetent. I wanted to talk about that but then I couldn't. [*Manifestation of identification with the devalued object pole of the dominant object relational dyad.*]

THERAPIST: I guess you were worried about telling me this for fear that I wouldn't understand how fearful you can be, particularly when you feel inadequate, or that I would judge you as you judge yourself for having these feelings.

MARK: Yes, that's it. I used to be paralyzed with anxiety in situations like this, and then if the client didn't like the model, I'd get depressed and withdraw or lash out. My former therapist helped me to control my performance anxiety and my anger, but I still fear that it will come back—this feeling of incompetence, this paralysis, and that I'll get angry and be humiliated.

THERAPIST: Maybe that accounts for why you were so insistent on having this session, knocking on my door 5 minutes early, because you needed to talk about these feelings, but then when you saw me, you feared that I would humiliate you as well. It is as though someone always has to humiliate or be humiliated. Today you fear that that someone will be you—it sounds like this fear comes up not just at work but with your colleagues and clients, your girlfriend, and with me as well. [*Defining the dominant object relation as it is emerging in the transference.*]

MARK: Yes, that sums it up. I think I can face the presentation now. [*At the door he turns to say "thanks."*]

The foregoing session material illustrates the gradual process by which Mark came to recognize his identification with both sides of the dominant dyad as they emerged directly in the transference. For example, it is clear from the above session material that there is an evolving representation of the therapist as someone who could help him develop greater understanding of conflictual and confusing aspects of his experience (i.e., "we made some progress"). But no sooner had this positive representation of the therapist emerge, Mark immediately backtracked, insisting that he had known that all along. Thus, he attempted to reinstate his psychic equilibrium through devaluing the therapist and retreating into grandiosity ("I already knew that"). Here we see that the feelings of being helped by the therapist elicited feelings of humiliation and rage and activated narcissistic resistances that functioned to deny dependency on the therapist and eradicate her capacity to help him. This was evident in his scornful repudiation of the value of the insight and in his threats to quit. He dealt with these negative feelings by immediately appropriating her insights and disparaging her skills, focusing not on what she had observed correctly but on what she had missed. We see in this case example the complex projective and introjective mechanisms that shore up the grandiose self.

The above clinical material also illustrates the value of interpreting in the projection—that is, focusing on the patient's immediate experience of the therapist (in this case, the sense of the therapist as inadequate or "woefully incorrect") without immediately linking it to the part of

the patient he was projecting (the self as inadequate), or to the patient's current conflicts or history (Feldman, 2007; Steiner, 1994). Interpreting in the projection in this instance helped to contain the intolerable anxieties about exposure and dependency that were propelling Mark to leave therapy. Such interpretations can move the clinical process forward by helping the patient to reflect on the anxieties evoked by the therapist's understanding. We see in this session material the oscillation between a narcissistic refuge and emergence from it in the form of a realistic appreciation of the therapist's efforts to understand him. We also see a movement from hyperindependence to wanting to be understood in allowing himself to reveal defended-against aspects of his inner world (e.g., his fear of humiliation, his dread of dependency). This was because the therapist first contained his anger and anxiety through clarification of these affect states, and then through interpretation of the object relation taking shape in the (negative) transference the patient was helped to face and tolerate the split-off aspects of self.

However, it should be noted that the recognition of the therapist as a separate person with the capacity to understand aspects of Mark's experience was only momentary and then felt to be intolerable and quickly retracted. The foregoing case example highlights how interpreting in the projection may function as the first step in identifying feelings of weakness and inadequacy—the denied and projected pole of the object relational dyad. In the first part of the session, the therapist's provisional acceptance of the projection of Mark's devalued aspects of self allowed him to begin to reflect on and talk about how these were tied to painful anxieties, more specifically his own fear of humiliation lest the deficiencies of his designs be seen by his colleagues. The therapist then connected this with his fear of being judged by her.

Interpreting in the projection involves the therapist containing the full force of the intolerable anxieties or feelings that are being projected. It is important to remember that projective identification is at core a dyadic event. It may result initially from seismic disturbances in the patient but is also felt as aftershocks in the therapist who is the recipient of the projection (Feldman, 2007; Joseph, 1985). Initially the therapist was taken aback by Mark's assertions that he planned to leave therapy since he had expressed appreciation for their work in the previous session. She had a momentary experience of confusion and disappointment, but after reflecting on the process she was able to see that the intensity of his reaction and his threats to leave therapy might have been related to anxieties about feeling exposed by the therapist's interpretation. But at this stage, the interpretation, although it evoked strong feelings of anxiety and anger, helped the patient move toward a deepening of insight, but also to a deepening of the relationship with the therapist. Thus, this

interchange illustrates that holding or containment is a process that includes both cognitive and affective components in that it involves both concern for the patient and an openness to tolerate and explore intense and sometimes unknown or poorly understood affects as they emerge in the dyad.

The therapist's interpretation to the effect that Mark's major conflict was between investing in a real relationship or fantasized ones, had jolted him out of a long-standing retreat to fantasy where his sense of omnipotence was assured. The interpretation had breached his grandiose defenses and exposed his inability to tolerate real intimacy and the narcissistic nature of the transference—for example, using the therapist as a sounding board in ways that left little room for an appreciation of her interventions. Overwhelmed by anxiety that the therapist might have understood something about him of which he was unaware, Mark had arrived early for the session, and intruded on the therapist's space in order to regain his sense of omnipotent control. She reminded him of his recent positive statements about treatment at the end of the last session and wondered whether her interpretation had made him feel understood, but then exposed because it introduced something that he did not already know about himself, which he found destabilizing, if helpful. He concurred that it was unusual for him to acknowledge that he benefited from therapy and talked about how he had deliberately held the therapist at arm's length because he did not want to get close to her or be infantilized by her.

This case example illustrates how it is essential, as Kernberg (2014) has observed, for the therapist to sustain his or her emotional commitment to the patient in order to maintain his or her wish to help and understand in the face of the patient's expressions of arrogance and devaluation. Such resiliency and steadfastness on the part of the therapist protects the therapist from "falling into the trap of reactive devaluation of the patient in response to his or her contempt" (Kernberg, 2018, p. 190). It also models for the narcissistic individual that negative experiences and limitations can be tolerated and explored in the interests of further understanding.

Indeed, once the therapist recognized the feelings of humiliation, fears of dependency, and loss of control behind Mark's threats to quit, she could contain the countertransference feelings of being disappointed and devalued, recognizing them in part as reverberations of what the patient was experiencing. She was then able to translate her own countertransference reactions and metabolize them into insight into and understanding of the patient's experience. This allowed the work to proceed with further revelations, elaboration, and reflection on his part on how his own concepts of self and others might be structuring his experience

of the therapist and others. In addition, the therapist's acknowledging that her lack of attention to his fear that his girlfriend might actually end their relationship devolved from her own less than perfect ability to understand his communications, which modeled the capacity for reflective activity and tolerance for imperfection or fallibility without masochistic self-flagellation or sadistic retaliation. The latter, in turn, helped Mark to reveal and reflect on his own anxieties about imperfection as they were activated by his upcoming presentation to a major client.

It should be noted that in this session there were indications of role reversals in the patient's focus on the therapist cutting him off, even while he himself was enacting this by threatening to leave therapy, and hence cutting off the therapist. In each phase of the interpretive process, we see emerging evidence of the next phase that will then be more fully elaborated as the therapy advances. We see examples in the clinical material above of the early phases of the interpretive process in TFP-N: defining the dominant object relations, identifying with both poles of the dyad, and the emergence of role reversals in the transference. This session material also provides an illustration of the ways that an accurate characterization of the dominant object relationship may lead to an intensification of the pattern in the transference, with an interchange of roles.

Phase 3: Working with Role Reversals

In TFP-N, the therapist initially identifies the predominant object relational dyads of the idealized self–devalued other or idealized other–ideal self that comprise the grandiose self-structure and then works with how the individual oscillates in identifying with both self and object poles so that he or she may alternately enact one role, while projecting the other in role reversals as they occur in the transference (Yeomans et al., 2015). This process entails bringing the patient's attention to how aspects of self-experience that are systematically projected into the therapist or others in the moment may then be enacted by the patient, often without awareness. Due to the rigidity of the narcissistic organization and the individual's difficulty with acknowledging and identifying with negative aspects of the self, the recognition of role reversals may be protracted (Diamond & Hersh, 2020; Yeomans et al., 2015).

By repeatedly calling attention to how the patient's negative experience of the therapist may represent in part an aspect of self, the therapist models for the patient how to step back and reflect on identification with both poles of an object relational dyad as they unfold over time. Such an observing stance introduces what Kernberg (2014) and others

have identified as a third term or triangular process into the therapeutic mix, in that it invites patients to transcend their immediate experience of the moment and to reflect on how their identifications may shift according to fluctuations in their relationships with others (the therapist) and to fluctuating states of mind. In terms of psychological structure, it is the beginning of building a foundation from which the patient can contextualize—can observe and assess—the immediate experience of the moment. As patients form links with aspects of experience that have been spilt off, dissociated, or enacted, they can begin to recognize that their behavior may actually stem in part from a representation of a mental model or working model of relationships in their minds that contribute to shaping their experience of the current interpersonal interaction (Caligor et al., 2009; Diamond, Yeomans, et al., 2011; Kernberg et al., 2008; Yeomans et al., 2015). Through focusing on role reversals, the initial rigid ideal/devalued configuration is gradually translated into a specific object relation; a model of self in relation to others; and the fears, conflicts, and affects that infuse them.

Case Example: Role Reversals

Over time, Mark's narrative of exalted professional success and attractiveness to women was punctuated with moments of insecurity and torturous self-doubt, but it was difficult for him to recognize how these were being lived out in the transference relationship. The latter only came clearly into focus around a crisis about paying the therapy bill, which Mark had let go for several months. He began the session by talking about his distress that his girlfriend was threatening to end the relationship after discovering his Internet activities. Quite quickly he shifted gears and said in a somewhat cavalier tone that he recollected he had ended the last session with his promising to pay the bill. He said that he had tried to reach his insurance company to find out why they were not paying but had been put on hold and then hung up because he had a pressing meeting. The therapist wondered, given how desperate he was about his work and relational life, why he was jeopardizing his therapy by not paying the bill for several months. When she called attention to the somewhat dismissive attitude and tone of voice that Mark used in responding to her inquiries about the bill, he stated, "Now over 10 minutes of the session have gone by and you are imposing your agenda on me . . . you're like a mother telling me what I have to do. You're not my mother, but like a mother." The therapist observed, "It sounds as though you are experiencing me as a demanding but neglectful maternal figure who imposes her agenda and never has your needs in mind, and yourself as an angry, resentful child who has to do her bidding (paying the bill)

in order to win attention or affection but who, instead, resentfully withholds payment in order to remain in control." He replied, "No, I experience you as a mother type, not my mother." He clarified this statement by saying that he expected special treatment from the therapist, who he believed should treat him pro bono—something he knew his own mother who "taxed every gift at every gate" would never do. The therapist proposed that, without any awareness, he might be the one who was imposing his agenda—to attend therapy without paying—and thus be enacting a reversal of the relationship. While Mark viewed the therapist as powerful and controlling, and himself as vulnerable, simultaneously, the therapist saw in this exchange the emergence of dependent wishes and the hope and desire for a perfectly gratifying object, who would see him pro bono. Each phase of the interpretive process leads to the emergence of saplings of what will ensue and be worked on in the next phase.

This led to an exploration of Mark's identification with a controlling and withholding maternal figure who was being projected onto the therapist. The therapist pointed out that by not paying the bill he was attempting to control the therapist, to act in a way that turned the tables of power and control, and thereby avoid any sense of dependency or need. She observed that he talked about driving away significant others, but perhaps it was also her that he feared driving away with his demands for special treatment. Paying the bill meant acknowledging his need for and dependence on the therapist, as well as the limitations of what she could offer him. He stated that he saw therapy as a lab experiment to try to have a different kind of relationship, but said, "I always ask for too much and wind up with nothing." Now he feared that he might have driven away his girlfriend with his demands for total devotion in the context of his not making an enduring commitment to her, and also feared he may have alienated the therapist by not paying the bill. Mark ended the session by asking the therapist, "How much do I owe you?"

In a subsequent session, Mark interrupted the session abruptly twice by taking work-related phone calls or writing texts that he claimed were urgent. The therapist interpreted these behaviors as devaluing of the therapy but also as another manifestation of his wish that she be an indulgent maternal figure who tolerates behaviors that bend the therapy frame. Mark responded that he knew that his actions were a "misjudgment and not good." The therapist stated that by interrupting the sessions by taking phone calls and not paying the bill, perhaps he wanted to let her know what it felt like to feel demeaned and controlled, feelings that he had acknowledged were evoked by having to pay the bill, and otherwise observe the therapeutic frame. Mark observed that he demeaned the therapist as a "leitmotif." He said that he wanted the therapist to indulge him and make him feel special to counter his self-disgust. He

said, "I have self-disgust all the way through; I am self-judging all the way through. Part of my self-disgust is I can't be proud of my girlfriend. When we are alone I value the time together and appreciate her insights about people and we talk about art and architecture, but then when we are with others, I worry that she is second rate—that she is not brilliant enough, or beautiful enough, and I feel humiliated. I acquiesce to keep myself in this debased state, where I can't be with her or without her."

The therapist interpreted these statements and recent actions in sessions as revealing two dominant representations of the self: (1) the entitled, all-powerful self who engages in treatment-disrupting behavior with impunity linked to an indulgent maternal figure with contempt as the linking affect; and (2) the devalued, dreaded image of the self as one who cannot measure up linked to a controlling, rejecting maternal figure, with anger and fear as the linking affect. In his actions—not paying the bill, interrupting the sessions—Mark alternately enacted and projected both representations of self and others, oscillating between the two, sometimes projecting one aspect onto the therapist and at the other time experiencing it himself.

These examples illustrate how when working with role reversals in the transference, an apt interpretation may first *intensify* the dynamic between patient and therapist, resulting in a rapid interchange of aspects of self and object poles of the dyad. In this case, devaluation of the therapist decreased over the next few sessions as the patient's awareness of it developed, leading to the emergence of more positive feelings of the sense of self as dependent and inadequate, longing for an all-giving, caring maternal figure—that had been previously defensively split off for fear of exposing his vulnerability. The capacity to hold in mind this defended-against dyad as it oscillated with the previously dominant grandiose self–devalued other dyad showed some partial integration in the internal world. This was reflected in the evolution in the transference from narcissistic and paranoid to a dependent transference. Here we also see the emergence of insight in which the patient shows some understanding that the object relational scenarios that were being lived out with the therapist in fact characterized Mark's other relationships as well.

Consistent interpretation of Mark's attempt to control the other he experienced as controlling and/or rejecting led to an awareness not only of the rejecting critical elements in himself but also the dependent longings, sense of weakness and humiliation, and associated fear and rage from which the grandiose self protected him. The recognition of his identification with both poles of the grandiose devalued dyad as it was being lived out with the therapist was the first step toward taking back the projection and experiencing the full weight of the anger, longing, and

fear of humiliation that led to the critical devaluation of others, including the therapist, clearing the way for a more positively valenced experience of the self in relation to the therapist and others to emerge. The product of work on role reversals is the recognition and tolerance for the contradictory positively (idealized) and negatively valanced (negative, judgmental) concepts of self and other at the same time, and the capacity to acknowledge that these are in part aspects of one's own mind as they emerge in the transference. Working with role reversals fosters recognition and reflection on the way that negative characteristics or modes of acting and being that the individual deplores in the therapist and others might actually represent aspects of the self and identification with negative aspects of internal objects. In so doing, this tempers the narcissistic person's grandiose sense of perfection and infallibility.

For those with narcissistic disorder, consistent work with role reversals in the transference leads to an expanded understanding of the underlying conflictual object relations with both an aggressive and libidinal, positive and negative cast that the grandiose self defends against. Work with role reversals in the transference enables the patient to understand *in vivo* how the narcissistic organization constricts his or her relations with others—that is, how it functions to camouflage and protect the individual from the full experience of longing for connection with wished-for others and abject feelings of dependency on all gratifying others (e.g., Mark's wish that the therapist treat him for free), as well as hateful feelings in relation to others experienced as fearsome or critical. In terms of the latter, one aspect of the grandiose self that was activated in the transference for Mark involved feelings of rage at a depriving, critical maternal figure that surfaced in frequent references to a TV series in which a therapist was both threatened and protected by her patient, a Mafia boss. The therapist understood, explored, and interpreted these paranoid aspects of the transference as indications of an investment in a sense of omnipotence and destructiveness, as well as a fear of merger with a maternal figure that would lead to annihilation of self. Such fantasies of harm and incorporation were also indicative of projective defenses that served to protect the self from persecutory, harmful internal objects.

Summary and Conclusion

The session material presented above shows that as the rigid, consciously held dyads of inferior/superior and idealized/devalued are explored through role reversals in the transference of other, positively and negatively toned object relational dyads that previously had been defensively

dissociated now begin to emerge. The rapid alternation of idealized and devalued representations of self and others evident in the session material presented above allows for reflection on and interpretation of the grandiose self into its component aspects that foreshadow its dissolution. At the same time, we see how the interpretive process in TFP-N is not primarily an intellectual endeavor but one that blends affect and cognition in ways that invite the patient into a new and richer mode of relating, and that puts the patient in deeper contact with a range of self and object representations beyond the sphere of the grandiose self.

In the following chapter, we present the latter phases of the interpretative process in TFP-N with case examples to illustrate how, with the resolution of the grandiose self, more complex, integrated object relations emerge, which in turn are reflected in more varied and advanced transferences.

CHAPTER 8

TFP-N Later Phases

In this chapter, we discuss the later stages of treatment with narcissistic patients. Interestingly enough, there is a dearth of literature on the later stages of treatment with persons with pathological narcissism and NPD because many of these patients drop out in the early or middle stages and never get to the later stages of treatment. This is because of the countertransference pressures these patients induce in therapists that lead to premature or precipitous terminations, treatment stalemates, or endless prolongation of treatments with little improvement. Patients and therapists may remain mired in narcissistic transferences and countertransferences reflecting rigid and intractable projections of ideal and devalued aspects of the grandiose self. In this chapter, we focus on the advanced stages of therapy in which the narcissistic patient develops more complex and multifaceted transferences, including dependent, erotic, and depressive transferences. These transferences reflect a deepening of the relationship with the therapist, and an expanded capacity to consider alternative perspectives beyond the grandiose self that includes deeper layers of the object relational world and a more integrated sense of self and other.

Phase 4: Identifying Object Relations Dyads with Opposing Affective Valence That Defend against Each Other

In the previous chapter, we presented the case of Mark, a man struggling at the intersection of increasingly conscious feelings of dependency and a grandiose self-structure. The therapist had explored the dominant object relation of grandiose self–devalued other behind Mark's actions of not paying the fee and disrupting sessions with phone calls. As Mark

recognized that these behaviors actually stemmed from conflicts between the grandiose self and the projected devalued aspects of the self in the transference relationship, another opposing dimension of the object relational world began to emerge. In his expectation of a fee waiver from the therapist, what might at first blush be interpreted as a devaluing stance, there was also yearning for total acceptance from a caring, valued other. This signals the emergence of a split-off, positively valenced dyad of an all-giving maternal figure in relation to a needy, dependent self. The surfacing of longings for a responsive, caring other in relation to a gratified self signals the emergence of an object relational dyad of opposite affective valence (positive as opposed to negative in tone and feeling). The surfacing of such an opposing object dyad suggests that one of the functions of the surface dyad of superior–devalued was to defend against the intense fears of disappointment associated with an admired, loved object who may have a great deal to give, but who withholds or disappoints. As Mark put it, "I always ask for too much and wind up with nothing." In other words, Mark's defensive engagement with the superior–devalued dyad protected him from the full experience of longing for an ideal caregiver, as well as from the assumed disappointment in such a needed, desired object (Coen, 2003).

As treatment progresses, it becomes clear that the object relational scenarios on the surface (superior self–devalued other) defend against *positive representations* of self and others linked by loving, gratifying feelings that were fearfully avoided. The goal at this stage of therapy is to make the person aware that the bifurcated way of experiencing others, including the therapist, may stem in part from a fundamental division in his or her internal world between the positive and negative sectors of experience, and to identify and interpret the mechanisms by which these sectors are kept separate (splitting, projection, idealization, devaluation, etc.). In other words, in this phase, the narcissistic person becomes more aware of how grandiose defenses (devaluation, autistic fantasy, and omnipotent control) have protected the individual from the full experience of longing for an ideal caregiver, and/or the fear, disappointment, and sense of humiliation when these longings are not fulfilled. The latter is thought to be one factor that catalyzes the person's sometimes intractable reversion to the grandiose and devaluing side of the split self (Coen, 2003; Kernberg, 2007; Joseph, 1989, 2003; Steiner, 2011). Unlike the borderline patient for whom such disappointments are readily expressed and experienced, for the narcissistic individual they may be long buried under layers of idealization and devaluation and may be inaccessible until the later stages of therapy.

It is important to note that the now emerging "idealized" sector, the longed-for gratifying self–other relationship, also reflects a distortion in

what could reasonably be expected from a relationship (e.g., *all* loving, *all* giving, *never* disappointing). Increasingly, the individual's awareness of the extreme, unrealistic nature of these longed-for relationships promotes a deepened and expanded, if more troubling, experience of self and his or her own history—a step toward greater complexity and ultimately integration in the internal world.

Thus, as therapy progresses beyond the initial stages, the TFP-N therapist's interventions can expand the focus toward the wished-for, longed-for relationships that the patient's grandiose self *defends against*. Initially, such split-off positive dyads may be expressed primarily in the realms of action, gesture, voice tone, fantasy, or dream images. As treatment progresses, such inchoate affects may crystallize around an explicit, positively valenced dyad involving overpowering longings for an all-gratifying parental figure in relation to a dependent, perfectly nurtured self (dependent transference). We should note that the emergence of wished-for, positively valenced dyads in narcissistic patients is often halting and slow. They signal the existence of the patient's need for something possessed by someone else not under the control of the self (i.e., the approach of true dependency on another). Indeed, the grandiose, dismissive stance of narcissistic individuals more consistently wards off awareness of dependent longings than is the case with the chaotic, preoccupied stance of borderline patients. More flagrant manifestations of defensive maneuvers to rid the self of any affects or experience related to dependency on or need for another is seen in narcissistic patients at the more severe end of the spectrum. Those individuals with insecure/disorganized attachment may go to great lengths to avoid dependent feelings, attempting to exercise omnipotent control of the therapist through self-injury, suicidal, and even homicidal ideation. For such patients, dependent longings may evoke intolerable feelings of vulnerability; fears of humiliation, shame, and guilt about aggression; or fear of attacks (see Chapters 9 and 10 for case examples of TFP-N with a more severely disturbed narcissistic patient).

The grandiose self–devalued other relational scenario that characterizes many narcissistic patients, even those who are higher functioning, is often shorn of any semblance of genuine human connection, dependency, or need. What is split off from the grandiose self are not only the negative aspects of self (e.g., the sense of inadequacy, vulnerability, envy, hatred) but also the dependent self with needs for human contact and connection in mutual relation to a caring, concerned other. The latter initially may be experienced as part of the vulnerable, weak, and inadequate self.

For narcissistic patients, the dependent self in relation to a gratifying caregiver often emerges as a mere glimmer that occasionally breaks

through the profound sense of detachment reflected in the prevalence of dismissing and cannot classify (e.g., oscillating dismissing and preoccupied, attachment) in this patient group (Diamond, Levy, et al., 2014; Meyer & Pilkonis, 2011). Such dismissing detachment may have functioned historically to protect the individual from painful affects, such as dread and fear associated with longing for connection and care from attachment figures who were experienced as rejecting or neglecting (Ainsworth, 1985; Ainsworth et al., 1978; Bowlby, 1988; Main et al., 1985). Its current function is to protect the integrity of the grandiose self from the deeper anxieties about needing and relying on others, fears of humiliation, and from the intensity of experiencing loving and hateful feelings that may be evoked in relation to the therapist.

Even when the narcissistic illusion that there is nothing desirable outside the self begins to crumble and the individual becomes more aware of dependent longings, he or she will still have difficulty developing a deeper emotional bond with others, including the therapist, even though the individual may deem others as admirable and valued. As one patient put it, even though he recognized that the therapist was a caring and concerned person, and valued her as such in the midphases of therapy, he still *preferred* to think of her as somewhat aloof and self-contained, since these were qualities he valued in himself. As he put it, "I fear more warmth from you, but I want more warmth . . . but I'm afraid it would infantilize me." When the therapist commented on how terrifying his wishes to be dependent on her were, he said, "Yes, a fearsome dependence." It was only in the later stages of therapy that he talked about valuing her wisdom, warmth, and concern.

It is important not to confuse the dyad of the dependent self in relation to the ideal nurturing other with a bona fide, healthy dependency, in which the other is valued as a separate and unique person, and the self is seen as worthy of care, concern, and understanding. The latter implies a realistic, full appraisal of self and other, which many of our narcissistic patients at this stage cannot tolerate. It is as if they would say, "I can now rely on 'you,' but that 'you' is not a realistic 'you,' but an ideal 'you,' who won't ever frustrate or disappoint; if you can be that perfect, then and only then can I depend upon you." Here the therapist becomes more than an extension of the self and its emotional needs in that the patient sees him or her outside the prism of the grandiose self as someone who has something to offer in the way of understanding and concern. But the patient still may be fearful of dependency on another who may be flawed and disappoint. Clearly, for patients at this phase of the treatment, dependency without such an illusion of perfect responsivity is too fraught. So, what results is a compromise, with patients slowly

testing the waters of genuine object relatedness, resulting in brittle idealizations as a proxy for connectedness, breaches, or ruptures of the idealized object to live up to expectations, the intensification of emotional closeness with the therapist, and then retreating intermittently to the safer waters of self-sufficient grandiosity.

A great deal of the narcissistic individual's pathology stems from the persistence of unrealistic, ideal views of the self, and the insistence that others affirm such a venerated self-concept. Therapists may be less sensitive to this than to the more overt damage and volatility stemming from an internal negative/aggressive object(s) that becomes enacted between the patient and those in his or her surround. Early in treatment it is sometimes essential to mirror the patient's narcissistic illusions, including the sense of uniqueness, invincibility, and self-sufficiency to consolidate the therapeutic alliance (Kohut, 1971; Mitchell, 1986). Premature interpretation of the defensive functions of the grandiose self, and the conflict it protects against between a self-sufficient, omnipotent self and a devalued, inferior, dependent aspect of the self might confront the patient with an intolerable sense of vulnerability and exposure. However, lack of awareness of the impact of the internal ideal object can lead therapists down a dangerous path of trying to comply with unrealistic demands and justifying such behavior to the extent that it is conscious, as entering fully into the patient's narcissistic illusions in order to foster a helping alliance.

As longing for caring and approval from the therapist emerges, it does so in conjunction with a renewed dread of dependency and may be defended against in various ways, including valorization of the therapist's contributions at an intellectual level (which may have previously been devalued or ignored), but without genuine emotional resonance with them. Thus, positive elements at the cognitive level are included in the representational world before affectively charged and experienced ones.

As described, the grandiose self tends to be rigidly organized around a configuration of idealized self and object representations that preclude any real dependency on and need for the other. Gradually as therapy progresses, glimmers of a wished-for relationship between a dependent self evolving outside the grandiose organization and a gratifying, if still highly idealized, other begins to take shape. Initially, the positive dyad of a dependent self in relation to a perfect other is as extreme and distorted as the negative "devaluing" dyad. The self and other representations that constitute it are immature, unrealistic, and inseparable from the grandiose self-concept. However, in the midstages of therapy the patient begins to experience dependent longings and feelings in relation

to the therapist, increasingly recognized as a valued, if imperfect, other who can provide, but who still may be held at arm's length and/or experienced as a remote, if admired, and concerned figure. As an ideal other–dependent self emerges in the transference, the ideal elements still retain a level of detachment and even at times harshness insofar as they are related to demands for perfection.

In narcissistic individuals organized at a lower borderline level, particularly those with malignant narcissism where the self-structure is more infiltrated by aggression, there may be *the threat of hostility if the therapist is perceived as having failed to meet the patients' needs or failed to live up to the idealized representation*. In such cases, the idealization functions as an expression of splitting, protecting the individual from negative feelings. Even with higher-functioning narcissistic individuals where ambivalence is better tolerated, there is always an admixture of positive and negative, loving and hostile feelings that are invested in the grandiose self (Rosenfeld, 1971; Green, 2002). This is evident in the somewhat puzzling tendency of narcissistic individuals, even those organized at a higher level, to seek out and hold fast to relationships that combine admiring and punitive aspects. With higher-functioning narcissistic patients, however, the positive, libidinal aspects of the grandiose self outweigh the aggressive aspects, but for most narcissistic patients, the idealized view of the therapist remains *prone to collapse if defenses against the recognition of limitation, loss, and human fallibility should cease to be effective* (Rosenfeld, 1971).

It is only with the developing attachment to the therapist that the ideal concepts of self and others in the narcissistic individual evolve beyond the static, unrelated, mirroring ideal (epitomized by Narcissus fixated on an ideal image that can only reflect, but never engage or interact) to incorporate deeper aspects of dependency and mutuality. Initially, dependency is seen as a negative or unacceptable aspect of self, associated with weakness and vulnerability. In this respect, the capacity to see the therapist as a separate and valued other on whom one can genuinely depend may hearken a movement from the paranoid–schizoid position in which self organization mirrors a split, internal world of unrealistic highly idealized and devalued representations of self and others to a depressive organization in which there is increased integration and tempering of intense feelings of love and hatred toward imperfect but good objects in relation to a dependent, grateful self (characteristic of the depressive position; Klein, 1946, 1957). It is only with the developing attachment to the therapist that a tolerance for genuine dependency shorn of its associations with weakness and vulnerability emerges. The following case example illustrates these points.

Case Example: Object Relational Dyads That Defend against Each Other

Mark's view of the therapist as inadequate and incompetent vied in the first 6 months of therapy with his view of her as self-sufficient and self-contained, highly accomplished, and socially adept—the type of person, as he put it, who was fulfilled both personally and professionally. All of this he aspired to, but had not been able to attain. When the dyad of an idealized, perfect therapist emerges in the therapeutic relationship, the patient relates to the therapist not as a separate valued individual but as an object who becomes the repository of wished-for aspects of the self as totally self-sufficient and self-contained. Only fools depend on others, in the view of those with narcissistic disorder; if the therapist is seen as one who tolerates dependency in self or others, he or she will be diminished in the patient's eyes.

It should be remembered that idealization is a brittle defensive process that is inherently unstable in that it covers over rather than integrates negative feelings; hence, the idealized object inevitably disappoints and falls short of exaggerated expectations for perfect care, mirroring affirmation, or admiration. What is invested in the ideal self–ideal object dyad is not just pleasurable or loving feelings, but also negative or hostile ones. That accounts for the severely critical and even rageful attacks of narcissistic patients on self and others in the face of disappointments, ego threats, or narcissistic injuries (Rosenfeld, 1971, 1987; Kernberg, 1984, 2007, 2010). The following clinical process material illustrates how the opposing split-off idealized and devalued dyads that defend against each other emerge and oscillate in the transference as the treatment unfolds.

MARK: There are a bunch of things on my mind today, but mostly the issue of pride and humiliation. I have twin emotions of being angry with you but also wanting to come here and talk to you. . . .

THERAPIST: It sounds like you have both positive and negative feelings about me that you want to talk about. Could you say more?

MARK: I'm bothered again by the way you ended our last session, just when we were getting somewhere. I was eager to come because I appreciate your perspective, your concern, and your challenging of me, but now that's all I can think about. I know we were out of time, but it's like you think you're the professor and I'm the 22-year-old graduate student focused on romance every second.

THERAPIST: It sounds like you felt that you were not important to me because I ended the session on time. I am struck by your comment

about feeling like a 22-year-old romantic student and me being the professor because on the one hand you have some good feelings about coming but they are canceled out by the sense that I'm the professor who has the authority to end the session on time. This sounds like a terrible dilemma in that you see me as someone who is competent, and might be able to help you, but I wonder if this doesn't also make you feel some resentment and perhaps envy.

MARK: Yes, I think of you in very different ways—sometimes I see you as off-the-spectrum brilliant, or as really old and haggard, and sometimes as an attractive woman with a confidence that comes from that, or somewhere in between—I see you as both.

THERAPIST: Well, it sounds like I can't be brilliant for too long before you see me as old and haggard. Maybe there is something threatening about seeing me as brilliant, so you revert to the worst view of me. This is not unlike the divided way that you experience yourself as assertive, successful, and competent on the one hand, and as flawed and miserable on the other.

MARK: This is a theme that always comes up for me—that of pride and humiliation and not being able to deal with conflicts about vanity. I think this was basically resolved at work, but not in my relationships. I keep looking for that ideal woman who is going to cure me—particularly P [a woman on the Internet who was the main focus of his fantasies at that time]—but I found flaws with her, too. I guess what I feel about her parallels what I feel here—the horrible duality of being successful and attractive, but also flawed and repellent. That's why I stopped responding to her e-mails. By the way, my girlfriend is actually seeing a new therapist and he is helping her—I am noticing some changes.

THERAPIST: It is interesting that you are telling me that your girlfriend is back in treatment, since every time I raised questions about this you insisted she was beyond help. Yet you have described her as formerly very attractive and desirable, but now overweight and at times almost repulsive to you. This is similar to the divided way that you see me and yourself.

MARK: I don't think you've been able to appreciate my attachment to my girlfriend. I don't think I've conveyed to you the pull as well as the push of that relationship. The pull is the comfort and beauty of the apartment she has decorated for us, but I live in a halfway house—with therapy, with my girlfriend—none of them gets all of me.

THERAPIST: I'm struck by your comment, "I live in a halfway house—with therapy, with my girlfriend, none of them gets all of me." The

part of you that is always attacking your self-worth attacks others as well, making it hard to experience and connect with the good parts—making it hard for you to accept or even acknowledge to me and yourself your attachment to your girlfriend, and perhaps others as well. I think I may also be talking about how difficult it is to talk about your attachment to me.

MARK: Yes. Dependency is fearsome for me. I worry that it will infantilize me.

THERAPIST: At the beginning of the session you said you had been looking forward to coming to session and that you valued my perspective, but then you got sidetracked by focusing on your negative feelings about how I ended the previous session. Maybe it was threatening to have such positive feelings, so you go back to the halfway house where you can be in control, feel superior, and judge the flaws of others from a distance. You remain insulated from others, including me.

MARK: Yes, I judge her [girlfriend] and withhold—all things you experience with me here.

In the foregoing dialogue, we see Mark's identification not only with both aspects of the dominant object relational dyads that were on the surface (e.g., idealized self–devalued other, or inferior, flawed self–superior other) but also the emergence of a sense of a positive self, deserving of nurturance in relation to a gratifying, valued object both in the transference and extratransferential relationships. He emphasized his attachment to his girlfriend, talked about the interests and pleasures they shared, and focused on how she had "solidified" him, and made him feel secure and competent. He also began to feel more genuine compassion for her as a person separate from his own needs and expressed remorse for his ongoing criticism of her profession as trivial and shallow, and recognized that his criticism of her intellect as second rate reflected his own fears about himself. At the same time, Mark began to recognize that there were major gaps in values and goals between them and some of his sexual difficulties with her might stem from his feelings of being relentlessly criticized by her for not being more financially successful. He began to realize that what he had previously referred to as the "punishment of this relationship" (e.g., the ways that his girlfriend made him feel inadequate, especially as a financial provider) devolved from an internal scenario involving the scathing critical part of himself, with his girlfriend being sometimes the recipient via projection and sometimes the critical judge of the denigrated, devalued aspect of himself. He began to reflect on his choice of being in a relationship that led to his feeling at

once superior and inadequate, realizing that it had to do with a harsh, critical aspect of his own mind that could be directed toward his girlfriend or toward himself.

The above session material also shows the typical oscillating pattern of genuine emotional engagement and retreat back to the illusory realm of the grandiose self that often characterizes the middle phase of work with narcissistic patients. Through these cycles between seeking emotional connection and retreat from it, the narcissistic person regulates the level and depth of emotional contact and attunement he or she can tolerate at any given moment. The retreat often involves fears of being disappointed or not attended to. This is evident in Mark's experience of feeling cut off and rejected by the therapist at the end of the last session that interfered with his positive longings to be helped by her ("I was eager to come because I appreciate your perspective, your concern, and your challenging of me, but now that's all I can think about"). Such patterns of engagement and retreat may also stem from defensive withdrawal from the welter of loving and hostile feelings that may be evoked as relationships intensify, or from the painful feelings of envy that are stirred up by seeing the other, including the therapist, as a separate and valued person with much to offer in the way of understanding, compassion, and commitment to the patient and his or her growth. As extreme feelings of fear, anger, sadness, and envy are evoked and diminished through the therapist's acceptance, containment, and interpretation of them, they lose some of their toxicity and thus can be better tolerated by the patient as an aspect of the authentic self-experience.

At this point when there is less need to project negative aspects of the self and more tolerance for limitation, we also see the projection of positive aspects of the self onto the therapist beyond the static and rigid idealized views. This is evident in Mark's rueful recognition of how his perfectionism and harsh judgments about others were in part a projection of his painful sense of his own imperfections, which had limited his involvement with others, including the therapist. He could recognize how the oscillations between seeing others, including the therapist, as idealized ("off-the-spectrum brilliant") and as devalued and imperfect, even repellent ("old and haggard"), reflected the split ways that he experienced himself. Furthermore, he was able to reflect on how these extreme views and associated contradictory feelings about self and others had led him to retreat to what he referred to as an emotional "halfway house" that truncated the depth of his relatedness to and investment in others. This case example illustrates that through identifying, reflecting on, and interpreting the polarized representations of self and others as they emerged in the lived reality of the transference, the therapist helped Mark to tolerate and reflect on these disparate negatively and

positively toned aspects of the internal world. More importantly, Mark recognized how negative representations of the critical self–inadequate other served to defensively dissociate experiencing the therapist and others as attachment figures who might be attuned to him and might gratify and care for him. Here we see an expanded capacity for mentalization evident in his recognition of how his own mental and representational states have influenced how he has experienced and behaved toward others, including the therapist.

In this clinical material, we also see the emergence of Mark's appreciation for, engagement with, and gratitude toward his objects (others), including the therapist as one who can help him understand his conflictual motivations about seeking emotional contact and retreating, including his oscillations between idealization and devaluation of self and others and his dread of dependency. These positive feelings vied with anger at the therapist for the feelings of inadequacy and powerlessness that she evoked in him when she ended the session in what he perceived as an abrupt manner. His distress about the ending of the session was also a sign of emerging dependency, since previously he denied feelings about breaks and separations. In addition, when he talked about the therapist not being able to appreciate his "attachment" to his girlfriend, the therapist saw an indirect transference referent to her not appreciating his "attachment" to her, which he affirmed. Thus, the emergence of dependent yearnings toward the therapist and particularly Mark's capacity to see and appreciate her knowledge and capacity to help are clear in the session material, but they in turn catalyzed feelings of humiliation (presents with an "issue of pride and humiliation") and envy ("you're the professor and I'm the 22-year-old graduate student"), which then caused him to retreat to grandiose self-sufficiency and detachment ("none of them gets all of me").

There are also glimmers of an erotic transference here in the representation of self as a graduate student focused on romance every second, although there was little awareness on Mark's part that these comments might be related to such feelings about the therapist. Narcissistic patients usually deny erotic feelings toward their therapists because they evoke longing for a valued but unattainable other and stir up feelings related to envy and dependency that are experienced as intolerable and humiliating. Erotic transferences that blend love, longing, and desire may not emerge until relatively late in the therapy with narcissistic patients because they are predicated on the capacity to love, and presuppose a relatively integrated object relation of an erotically stimulated self and a valued, desired therapist. The erotic transference should not be confused with the erotized transference in which sexual feelings are used in the interests of control, hostility, and manipulation.

More disturbed narcissistic individuals, particularly male patients, may erotize the therapeutic situation, particularly with female therapists as a way of exercising omnipotent control (Diamond, 1993). They may do so either through attempts at overt seduction, or more indirectly through the recounting of sexual exploits in a provocative manner that attempts to position the therapist as an admiring witness to their sexual prowess. In the erotized transference there may be periods of overt seductiveness, crude eroticization, or severe inhibition of all libidinal feelings in the transference that the therapist tolerates and explores without withdrawing or retaliating. He or she must remain capable of empathizing with the hatred and envy, as well as the fear and longing behind the more disturbed patient's aggressive erotic advances, as well as the emotional detachment and painful incapacity to blend erotic with loving feelings that is behind the subtler devaluation and lack of emotional investment of the higher-functioning narcissistic patient.

In sum, for those with narcissistic pathology, positive sectors of the object relational world emerge more slowly in the transference. They evolve from highly idealized representations of the therapist that are extreme and one-dimensional to a view of the therapist as a valued and separate, but remote and unattainable figure, to more multifaceted, positively valenced representations of the therapist as a desirable, caring individual with expertise to understand and help the patient. Genuine emotional engagement with the therapist in the midphase may activate intense feelings of love and hatred that characterize all deep object relationships and may be punctuated by defensive reversion to the illusory, insular world of the grandiose self. Disparate, contradictory aspects of the internal world emerge in the transference as the narcissistic defenses against and conflicts around dependency and envy are worked through.

Working through: The Dissolution of the Grandiose Self

For those with narcissistic pathology, repeated cycles of engagement with the therapist and wider aspects of their own internal world, alternating with periodic retreats to the insular fortress of the grandiose self, define in large part the process of working through. As the therapist repeatedly observes, confronts, and interprets the manifestations of the grandiose self, along with the defensive illusions that sustain it, more complex, nuanced, and varied mental representations begin to emerge both in and out of the transference. Conversely, as a richer and more complex array of contradictory and polarized mental representations of self and others emerge, a wider range of transference manifestations develops, including paranoid, erotic, and depressive transferences. As these more specific, varied aspects of the representational world emerge, the work in therapy

begins to include more material in the form of memories, dreams, and fantasies.

At this point, the working through of the relationship with the therapist as a new object may take center stage. The periods of engagement and retreat from relatedness described above persist but lessen with an increased capacity for dependency and increased tolerance for the exploration of unconscious fears, wishes, and motivations, along with the atavistic anxieties and disappointments in primary objects that led the individual to develop the narcissistic organization to begin with. As Symington (1993) points out, at some point in development as a result of inordinate frustration or disappointment at the hands of primary objects, the individual may take a narcissistic path, turning away from the primary object by creating a split in the self whereby one part of the self continues to relate to others while the intentional or real self remains sequestered in grandiose self-organization. The narcissistic mode of functioning, including "the grandiosity and disowning of parts of the self—is a defensive procedure" (Symington, 1993, p. 73) and the movement out of it is fiercely resisted. "It is resisted with a desperation that is far stronger—violent even—than any other obstacle . . . a whole life's orientation is at stake, and the collapse of such an edifice may seem like the end of the world" (Symington, 1993, pp. 86–87).

Case Example: Emergence of a More Varied and Specific Transference

As the narcissistic structure begins to dissolve, both the tenacity of the resistances and increased access to the complex, multiple representations were evident in material that Mark brought to a subsequent session. He talked about attending an art opening for a former client of his mother's (who was a retired art dealer). The opening was in a major gallery, and he felt that neither he nor his girlfriend were able to hold their own in the discussions about the exhibit and about the art world. After introducing him to her friends and associates, his mother ignored them, focusing on her other prominent guests who were notable figures in the arts and in publishing. He felt socially awkward, and embarrassed by his girlfriend, whom he saw as not in the same league as the majority of his mother's guests. He felt that he had made a major *faux pas* when talking with his mother and the featured artist about a work that took its inspiration from a book that was also made into a major film (*On the Beach*). He had compared a figure in the painting to a character in the film, only to have his comment dismissed by the artist, with his mother agreeing with the artist. Mark related that at that moment he felt that he and his girlfriend were alone in the world, like two characters at the apocalyptic

end of the film *On the Beach*, who were facing the impending annihilation of the human race after a nuclear accident.

The therapist observed that Mark's experience of being criticized by his mother was so toxic that it activated anxieties and fears about annihilation. She wondered how it felt to tell the therapist about it, to which he replied that he was concerned that she would see him as inadequate as well. In exploring these feelings, the therapist referred to the scene at the end of the film, and Mark said quite imperiously that by referring to the film version she was assuming he had not read the book. He then expressed contempt for the therapist's reference to the film version rather than the book. The therapist pointed out that he was re-creating with her his experience of feeling criticized and judged by a disapproving and critical maternal figure (one who assumes he has not read the book). He was assuming the role of a critical parental figure and placing the therapist in the role of the inadequate, ignorant, inferior one (role reversal). It was only after exploring this role reversal in the transference that Mark acknowledged that this experience had resuscitated devastating feelings of inadequacy in relation to an accomplished, revered maternal figure.

Mark then talked about thoughts of wanting to lie down on the therapist's couch, wishing she would cover him with a blanket, as he had been tucked in by his mother in childhood before she would go out for the evening with his father, or when she checked on him in his sleep upon her return. The therapist commented that this fantasy of lying on her couch and being covered by her represented both longings to be cared for and protected by a good maternal figure, along with fears of rejection, so that such feelings must be covered up. She observed that such longings must be experienced as frightening, in that they involve need and desire in relation to someone who might expose or denigrate him. She observed how difficult it was for him to express such feelings directly, rather than covering them up. She then inquired further about his apocalyptic thoughts and fantasies involving end-of-the-world scenarios. Mark clarified in this and subsequent sessions that in such scenarios he was sustained by fantasies of being guarded by mythical and superhuman figures, such as the goddess Athena, who would protect him from harm and confer her wisdom on him. He was able to recognize that such an idealized female figure represented an aspect of the therapist that related to a persistent oedipal illusion of a woman who was at once desired and unattainable.

Inherent in the foregoing material is the emergence of layers of idealized and persecutory representations of self and others that emerge as the grandiose self dissolves. Profound and persecutory anxieties may be precipitated about exposure, inadequacy, and even annihilation. Increasingly complex and multifaceted, if still polarized, views of the

therapist emerged, including the therapist as a rejecting maternal figure (the disapproving, rejecting mother of the reception) with a corresponding view of self as flawed and vulnerable (the inadequate self who does not understand the iconography of the painting); the therapist as a devalued or inferior figure (who refers to the movie, not the book) with the self having superior knowledge (refers to the book, not the movie); and the therapist as an ideal object with magical powers ("Athena") with the sense of self as the special one who will be rescued from annihilation by a mythical figure. However, we also see in this session the representation of the therapist as a tender, nurturing maternal figure who is attuned to Mark's sense of vulnerability and need for ordinary good care (being covered up by a maternal figure).

What is evident in this material is how unrealistic and phantasmagoric versions of the ideal—the therapist as a deity with superhuman powers who will rescue the individual from annihilation—continue to coexist with more realistic representations of the therapist seen as a caring maternal figure who is attuned to Mark's needs for protection and care. Also evident in this material is the emergence of erotic feelings toward the therapist, which bespeak a lessening of devaluation and idealization. As noted, erotic transferences emerge slowly for patients with narcissistic pathology since they involve acknowledgment of longing and desire for the therapist, and thus a capacity to experience such feelings toward another, despite their imperfections. Such feelings might vie with reversions to grandiosity and to fantasies of an ideal relation as well—that is, the therapist as a revered, unattainable object.

The above clinical material also highlights the collapse of the idealized concept of the self and the ways that this insulated Mark from longings for approval and love from a venerated maternal figure who might rescue him, along with hatred and dread of a forbidding and ridiculing maternal figure who might reject him. Both of these self and object representations and their linking affects converged in the transference at this point.

Dealing with Aggression in the Transference

In the clinical material presented above, we see awareness and tolerance for negative feelings, particularly anger and aggression, that coexist with an ongoing sense of vulnerability and fear of attacks from others—the flip side of the projection of aggression. In the later stages of therapy, representations of self and others as flawed, rageful, and dangerous appear. These aggression-laden representations may be linked to the fear that the self will destroy what is good and needed, now that the individual is increasingly aware of something good outside the self.

As the narcissistic person becomes more aware of the need for the therapist/maternal figure as both loved and admired, as well as hated and denigrated, he or she becomes better able to tolerate and experience aggression and anger, as well as feelings of guilt and remorse about fantasied attacks. For example, in this mid to later phase of therapy, Mark talked about fantasies in which a terrorist kidnapped women and held them hostage, threatening the community. He acted the role of protector, only to have it discovered that he himself was the terrorist, indicating his identification with a hostile introject. He also talked about fantasies of wanting to kidnap his girlfriend and hold her hostage so she could never leave him and would remain under his control. Exploration of these fantasies revealed that they were responses to feelings of inadequacy since he was frail and slight as a young man, and in fact wished he could bulk up like his best friend who teased him, and who was much more successful with women. In fact, in his fantasy world he was sometimes the victim of a terrorist. The therapist commented that these fantasies represented the two sides of him: the gentle, vulnerable man who tries to protect others from danger, and the man who is so violently angry and desperately needy that he kidnaps women but tries to hide this. The oscillation between feeling omnipotent and powerless was striking. These two aspects of self had been so conflictual that Mark avoided intimate relationships in depth. He then acknowledged that in fact he engaged in compulsive masturbation and Internet liaisons not only as a source of pleasure and stimulation but as a protection from becoming too attached to or reliant on any woman, including his girlfriend. These Internet encounters also protected her from his aggressive fantasies that he could enact in virtual reality without consequences. Fantasies and dreams that blended hostility and eroticism in relation to women, including the therapist, emerged at this point. In the later stages, as the panoply of contradictory and conflictual self and object relations that the grandiose self defends against are clarified and interpreted, the therapist begins to link aspects of the patient's current functioning to their antecedents in early experiences and past relationships.

The Oscillation between Thick- (Grandiose) and Thin-Skinned (Vulnerable) Narcissistic Positions

The clinical material above not only illustrates the emergence in the transference of the patient's split polarized aspects of his internal world but his identification with the disparate elements of it that emerge as the grandiose self begins to dissolve into its component aspects that are linked to archaic introjects often rooted in traumatic past experiences. As the kaleidoscopic aspects of the idealized and devalued aspects of self

are experienced and explored in relation to the therapist, the underlying inadequacy, fear, and rage come increasingly into focus as aspects of the real self. There may also be an oscillation between thick- and thin-skinned, or grandiose and vulnerable, respectively, narcissistic positions (Rosenfeld, 1964, 1987; Bateman, 1998). Clinical investigation with patients with severe narcissistic pathology reveals that as thicker-skinned, more rejecting, dismissing, and counterdependent narcissistic patients start to face what had been their unconscious psychic reality, they may indeed undergo what looks like a collapse, and begin to look more "borderline" and preoccupied, with an increasing awareness of feelings of chaos and panic related to a sense of inner emptiness, vulnerability, distress, and/or pain. While they may look more vulnerable and symptomatic, individuals at the higher levels of functioning may not experience the severe identity diffusion and impulsivity of those organized at a borderline level when narcissistic defenses falter. Instead, higher-functioning narcissistic patients, many of whom have stable jobs, careers, friends, and even enduring, if troubled, relationships, may experience a profound sense of emptiness, depletion, and even destabilization. This destabilization is related to the dissolution of the grandiose self and consequent diminished investment in and gratification from narcissistic defenses and modes of relating that may cause profound disruptions in the sense of identity short of more pervasive identity diffusion.

The relinquishing of the idealization of self and others is a double-edged sword in that it clears the way for further growth and individuation, but also allows immature, shameful, and negative aspects of the self, previously repressed or split off, to emerge into conscious awareness. Also, as the essential hollowness and impoverishment of the idealized internal world that the grandiose self defends against is confronted, states of anxiety, depression, and grief may emerge. This often involves mourning for the loss of previously impermeable self-sustaining narcissistic illusions. It should be noted that there may be increasing oscillation between thick- and thin-skinned presentations of narcissistic pathology as treatment progresses. In fact, as Bateman (1998) has observed, interpretations may be most effective when the individual is in transition between the two positions.

About 12 months into treatment, Mark acknowledged that the real reason he had not left his girlfriend or been able to truly commit to her was because of a "black hole" in himself, which his girlfriend had filled with her devotion. This black hole was associated with terrifying fears of aloneness, emptiness, and inadequacy. Therapeutic exploration of this image helped him to understand that one aspect of this black hole was his longing for and dread of women, which drove his quest for perfect sexual partners, and limited his capacity to commit to them. At

this point he talked about an "unquenchable neediness" that he tried to fill with "a compensatory fantasy life." It should be noted that as the narcissistic individual emerges from the hard shell of the grandiose self, and before mature, integrated object relations are firmly anchored in the self, he or she may experience devastating fears of annihilation and emptiness as the quest to procure narcissistic supplies fails to sustain and is relinquished. As this point, those with thick-skinned or grandiose narcissistic presentation may look more thin-skinned, vulnerable, needy, hypersensitive to rejection, shame ridden, and demanding of the therapist. Mark, for example, became preoccupied with fantasies that he was the therapist's only patient. He wondered how much she thought about him between the sessions and asked whether he could increase the number of sessions per week. At this point, Mark's experience of the transference was dominated by that of a helpless, inadequate self, seeking care from a giving maternal figure, with emerging tolerance for genuine dependency and the need for others, including the therapist (the typical dyad of the dependent transference).

In the later stages of therapy then, dependency is uncoupled from devalued aspects of the self, understood as an acceptable human need rather than a humiliation, and integrated with positive, benevolent, realistic views of self and others. With individuals with narcissistic disorder, whose dependency needs may have gone underground, the emergence of dependency will be a slower process and may manifest itself indirectly and intermittently. It should be noted that in therapy, as in early development, dependency—seeking of contact, attention, and caring from a significant other—anticipates and overlaps with, but is not necessarily synonymous with, the development of an attachment relationship per se. The latter is a more specific enduring affectional bond with a specific other experienced as complex, unique, and not interchangeable with others (Ainsworth, 1985; Bowlby, 1988).

Movement into the Depressive Position and the Emergence of Depressive Transferences

As narcissistic defenses against dependency are worked through, what appeared to be an absence of object relations in fact masks a world of distorted object relations. These often involve internalized object relations comprising a rejected, persecuted self in relation to a cruel, exploitative, cold parental figure or of an overstimulated, excited self in relation to a tantalizing, seductive other (Fairbairn, 1952). The full recognition of such a split internal world has been obscured by the ideal representations that comprise the grandiose self. With the increasing recognition of the distinction between real and ideal aspects of self and object, and

of the costs and consequences of the narcissistic organization in the form of emotional isolation, estrangement from others, and aspects of the self, profound depressive feelings (the black hole) may emerge.

Such depressive feelings may be particularly painful and agonizing, as the patient confronts and mourns the damage done by the self to relationships, of time lost, and opportunities squandered because of the failure to live fully in reality rather than in the illusory world of the grandiose self. The individual's recognition of the ways he or she has chronically devalued and even mistreated others, including the therapist, is often an indication of the emergence of depressive transferences. Depressive transferences are characterized by feelings of guilt and remorse for the ways the individual may have damaged his or her objects, now perceived as whole and separate from the self; there is often the wish to restore objects, reinstating as valued previously devalued internal and external objects, and repair relationships with them, rather than denigrating them or denying their existence (Klein, 1946, 1957). The projective and introjective mechanisms that sustain the grandiose self protect the individual from loving others as separate and valued beings, and thus ameliorate the risks of loss and depression.

Feldman (2007) observes that as individuals move into the depressive position and narcissistic omnipotence and idealization are diminished, they may experience others with greater depth and complexity, along with positive feelings of elation and gratitude toward them, but such feelings may also catalyze profound anxieties around guilt or destructiveness as the narcissistic organization dissolves. At such moments, it becomes possible to interpret at a deeper level the anxieties that have maintained the retreat into grandiosity, including anxieties about annihilation, humiliation, aggression toward or from others, abandonment, and insignificance (e.g., the black hole). They are the consequence of relinquishing narcissistic omnipotence, allowing dependency and engagement with the therapist and the individual's own internal world.

The therapist's receptivity and acceptance of these intolerable states of mind provide temporary containment of such anxieties until there is a mental structure beyond the grandiose self-organization to deal with and integrate such states (Bion, 1959, 1962; Britton, 2003, 2014). The latter is predicated on the extent to which the patient has been able to internalize the therapist as a good-enough (Winnicott, 1971), rather than ideal object, as well as the patient's reflective capacity to recognize that these disparate mental states do not threaten annihilation of the self. The processes of mourning for damage done to significant others, as well as the loss of ideal representations of self and others—that are part and parcel of the depressive position—contribute to the development and anchoring

of a more mature, differentiated, and integrated level of mental representations of self and other.[1]

Phase 5: Integration

In the late stages of therapy, the experience and interpretation of the contradictory (ideal and devalued, positive and persecutory) object relational dyads as they emerge in the here and now of transference pave the way for their integration and transformation. More complex, specific transferences emerge that reflect myriad aspects of the representational world. This signals the dissolution of the narcissistic organization, specifically the shift from polarized, immature, and unrealistic representations of self and other to mental representations that are not only more differentiated and integrated but are also more realistic, complex, stable, and positively valenced, reflected in increased vibrancy, mutuality, and aliveness in relationships. As the therapist repeatedly interprets alternating devaluation and idealization of him or her, the individual begins to recognize how these projections protect the individual from the pain of facing (1) anxieties about genuine connection to and attachment to others, along with the limitations of what they can provide and the intense feelings of love and hatred that such attachments may entail; and (2) the self as imperfect and the sense of humiliation and shame that such feelings evoke. Both aspects of the grandiose self—an exaggerated sense of self-importance and perfectionism along with relentless attacks on the self (and others) for imperfections—preclude genuine connection to others and truncate the capacity to experience pleasure and gratification in what the self can realistically achieve, or what others can realistically offer.

In the course of treatment, the therapist addresses the way that idealized internal objects have functioned to protect the self against pernicious, persecutory aspects of the internal representational world, and the disappointing or difficult experiences that may have given rise to them. The individual recognizes that the allegiance to and identification with ideal objects has created a straitjacket in that ideal objects demand an ideal response (e.g., one must behave perfectly to an ideal object or be angrily rejected; Riviere, 1936). The narcissistic individual may remain bound up in struggles with such ideal internal objects in a

[1]Not all narcissistic patients are able to experience and tolerate the anxieties inherent in the depressive position. Also, while they do not necessarily revert to paranoid–schizoid mechanisms to stabilize the self, they may short-circuit this phase of treatment.

way that limits engagement with and dependence on others in the immediate interpersonal sphere.

For example, Mark talked over several sessions about his girlfriend's lack of interest and support for his work on a project for which he had received an award. She had focused on the limited amount of money awarded to him for the prize. He then castigated himself for getting the "childish and narcissistic pleasure of whining" to his therapist about his hurt feelings. The therapist commented on his experience of self as pathetic and needy in relation to an indifferent other, or, should the other (therapist) express the concern and caring he sought, he often reverted to an experience of self as supremely self-reliant and the other as infantilizing, criticizing the therapist for indulging his "childish pleasure." He acknowledged that this was the case and reflected that he was beginning to feel that it was "okay to be needy in a regular human way." He recollected situations in which his emotional turmoil or need was not understood or registered. For example, in childhood, Mark regularly broke out into hives before birthday parties of his peers; he now understood that the hives were in part brought on by fear of seeing a boy who bullied him. His parents belittled him and insisted he attend. He attributes their exacting behavior to their conviction that weakness, fear, or failure was "not part of existence," and hence, they did not inquire beyond the surface about the reasons for his party refusal, focusing instead only on the somatic symptoms. They took him to a number of allergy specialists who found no organic reasons for his skin eruptions and put it down to stress, but did not take the next step of investigating the origins of such a stress reaction. This clinical material illustrates how in the later stages, interpretation involves exploring the unconscious conflicts, wishes, fantasies, and motivations that gave rise to the narcissistic organization and their connection with past relationships and experiences.

At this point, genuine dependency on the therapist begins to emerge along with deepening appreciation and gratitude for the therapist's capacity to maintain ongoing concern for, commitment to, and compassion for the patient even in the face of the previous pervasive, and still intermittent, devaluation. Gratitude for the therapist's contributions to self-understanding and identification with the therapist's reflective capacity, along with remorse for previous devaluation of the therapist emerges. More complex multifaceted transferences, including erotic and depressive transferences, begin to emerge, gradually supplanting the narcissistic and paranoid transferences that may have previously prevailed.

The latter go hand in hand with the consolidation of an attachment relationship with the therapist seen more realistically, rather than through the distorting prism of the patient's projections. The consolidation of a more secure attachment relationship to the therapist is evident in the increase of the capacity to seek and maintain emotional contact

with the therapist, to share affects, both positive and distressing, and to engage in mutual, collaborative exploration of the patient's internal world and external reality. It is also evident in the diminution of demanding, instrumental aspects of dependency (the desire to be the sole patient of the therapist or the requests for more frequent sessions); a decrease in the tendency to revert to grandiose defenses of devaluation or autistic fantasy in the face of injury or disappointment; and an increased curiosity about and appreciation of others, including the therapist as a separate unique individual with his or her own life, as well as in increasing reliance on the patient's own reflective capacities, using the therapist as a resource.

As treatment progresses and the grandiose self dissolves, attachment based on realistic aspects of relationships and those based on unconscious fantasy become more aligned, although there always remains a tension between the two spheres. The development of an attachment to the therapist goes hand in hand with the capacity to explore and ultimately relinquish narcissistic illusions tied to archaic, idealized representations of others. This allows for the emergence of flexible, multifaceted representations of significant others, and to integrate them into a social context and imaginatively, but more realistically, reflect on the self and affect states that are linked to them—all of which catalyzes the new levels of vibrancy and aliveness in relationships.

The development of more mature integrated representations of self and other also improve mentalization or the capacity to connect internal experiences (thoughts and feelings) with visceral reactions (e.g., hives), as well as to understand how the subjective experience of others may in fact diverge from others' thoughts, motivations, and feelings. This allows the individual to modulate affect states, particularly those that are intense and negative, as they are integrated with more benign emotions, thoughts, and behaviors. The relinquishment of ties to idealized and/or powerful, punitive mental representations of others paves the way for the development of more secure as opposed to insecure/disorganized attachments that in turn allow the patient to appreciate and accept a more complex and satisfying intrapsychic and interpersonal reality.

The development of a secure attachment to the therapist is an essential step because, while it poses a challenge to the hegemony and omnipotence of the grandiose self, it also provides the patient with a new and sustaining relational platform that allows for the claims of the grandiose self to diminish as a more authentically related self emerges. As stated earlier, narcissistic patients may continue to oscillate between emotional engagement with the therapist and reversions to self-contained grandiosity as narcissistic defenses and conflicts around attachment, dependency, and envy are worked through. Indeed, those with narcissistic disorder may continue to revert defensively to the illusory world of the grandiose

self even into the later stages of therapy, although the type of unbridled grandiosity that characterizes earlier phases of therapy becomes increasingly rare as a secure attachment relationship between patient and therapist is consolidated.

In the later stages of treatment, this defensive retreat can sometimes take the form of the patient provocatively asserting that nothing has changed, that the therapy has been worthless, or more often that the substantial changes in their lives are their own doing and would have happened without treatment. At the same time, such recurring provocation of the therapist functions to reassure the patient that the therapist will maintain a steadfast commitment to him or her even in the face of such resurgences of withdrawal or devaluation, as the patient comes to grips in the termination stage with the limitations as well as the positive gains of treatment. Usually at this stage the narcissistic patient is able to reflect on the breakthroughs of grandiosity and devaluation because of the internalization of aspects of the therapist's reflective capacity. The latter fosters the capacity to reflect on grandiosity as a mental state rather than an absolute reality.

Ongoing systematic working through of component disparate, polarized, and primitive object relations, as they are alternately projected and enacted in the transference, in turn diminish the extent to which the grandiose self organizes the internal and external world of the patient. At this stage, there is a weakening and separating of claims of idealized representations of self and others (the grandiose self) over the needs of the now more robust real self. This leads to decreased valorization of the narcissistic dimension, increased capacity to reflect on the costs and consequences of grandiosity and the conflicts that have motivated it, and to invest more deeply and genuinely in relationships, including the relationship with the therapist. In the later stages of therapy, one patient, after feeling he had been unduly self-aggrandizing and self-promoting in a meeting at the expense of his colleagues, said, "I had intense self-disgust about a narcissistic display last week at a presentation . . . on the surface not much happened but I think I talked too much and behaved obnoxiously, acting like a repulsive, narcissistic, self-aggrandizing child and wanting all eyes to be on me . . . part of me wants you to say it's not that bad . . . but I know I have to dial it down" [*self-reflection on grandiosity*].

Statements such as the following are also indicative of movement into the later phases of treatment:

> "Something happened yesterday and I thought we might make sense of it together, rather than my just exhibiting my psyche to you." [*recognition of therapist's authentic interest and capacity to help*]

"I was eager to come because I appreciate your perspective and your challenging of me—not just my instinct to talk about my problems and need for relief." [*expression of gratitude*]

"My girlfriend is attuned to what I am thinking and feeling. This is a failure because I am not totally attuned to her. So, I've failed her. . . ." [*recognition of limitation; failure, mourning*]

"I respected you, but I always thought my understanding came from me until you said I needed my girlfriend as a psychic twin. Now I realize that that's not love, although I still care about her. I feel gratitude and something more—an intense feeling about you, but it's not scary like it used to be." [*tolerance for intimacy, consolidation of attachment to the therapist, and gratitude for her contributions*]

Case Example: Increased Reflection, Tolerance for Envy, and Dissolution of the Grandiose Self

The following session material highlights changes in the later phases of treatment:

MARK: This morning I realized that at some point my therapy is going to end and I won't be coming here anymore.

THERAPIST: It sounds like termination may be on the horizon.

MARK: I don't know if I'm ready to stop because I have finally realized that I need you and that it's okay to be needy in a regular human way—I guess we all have such needs.

THERAPIST: It has been very difficult to acknowledge those needs, especially here.

MARK: Well, I have always been afraid of them [needs] because I felt like I had such a gaping hole in me.

THERAPIST: That hole left you feeling empty and insecure and fearful of being taken over by others, including me. So, you kept me at a distance.

MARK: Now I am reaching out in a modest way. I feel a need for you to help with my problems—that's different from neediness. Now I see that it's more human than insecure to feel a need for others; I kept you at an emotional arm's length for such a long time.

THERAPIST: And now?

MARK: I'm really having a reaction to the idea of not coming here, of losing you. It's not like I need you to help me solve day-to-day problems

or issues—so much has been resolved in my work life, and I may be in love for the first time ever—but I will miss the relationship.

THERAPIST: You are talking about losing me in the future, but we haven't even set a termination date. Perhaps you need help with something today but are holding me at arm's length still by having trouble articulating what it is about. Can you be more specific?

MARK: Well, something came up this week that I need help understanding. I don't like how I treated my brother when he and my family attended my niece's wedding. It was a 3-day affair and he expected that we would spend time together. He and I were the only ones without children or partners. I barely interacted with him, instead hanging out with other friends and relatives. I feel particularly bad about it because he is recently unemployed—his start-up business failed, and he was really looking forward to spending time with me, maybe in hopes I might help him figure out the next step. He was always the black sheep and I was always the star, and he was ostracized. But everyone was asking about the breakup with my former girlfriend and about my new relationship and I felt sort of exposed.

THERAPIST: I wonder if you were also feeling isolated and deficient like your brother because you don't have a family and had difficulty with those feelings. Your own conflicts about getting married and having children have been a major focus of our work, and even though you are in a new relationship, you're still uncertain about its future. Is it possible you were feeling envious of your sister who had a daughter who was getting married, and others in the family, and so devalued your brother and had to distance from him as a way of coping with these feelings?

MARK: Envy is so humiliating. I hate to admit that I'm so competitive. . . .

THERAPIST: It's been very difficult for you to tolerate envy. In fact, in the past you've kept things out of the session because you couldn't admit here that you had such feelings. In fact, in the past we've talked about how difficult it was to talk about your friends or siblings with children.

MARK: Well, that brings up another matter. Recently I looked you up on the Internet and I learned that you are married and have a child.

THERAPIST: It sounds like finding out those things intensified your feelings of envy and inadequacy that were activated on the family trip, so we are talking about your feelings about me, not just your sister.

MARK: Yes, that is much harder to admit, but it's true. I want to think I have an exclusive relationship with you, but I realize that's not the

case. You will go on seeing other patients who I see in the waiting room but not me.

THERAPIST: I wonder if that doesn't make you angry as well as sad, and that trying to find out more about my life through the Internet doesn't also involve a wish to control me, as well as a way of holding onto me to cope with the separation when you went away for the week.

The session material presented above shows that as therapy proceeds, there is ready recognition of how the devalued, needy self has been projected onto the other, and with that the reintegration of these aspects of self into a realistic, mature self that can tolerate dependence, vulnerability, loss, and imperfection, as well as guilt and envy. The latter is evident in Mark's acknowledgment that his rejection of his brother in fact stemmed from his own sense of inadequacy in relation to his other siblings and the therapist. In the later stages of treatment, as others are seen as separate individuals with their own needs and realities, envy is inevitably evoked. But as Jacobson (1964) has observed, envy can also be a catalyst to further growth in that it involves wanting to have as one's own what is envied in the other, and hence, can function as a step toward identification with a valued other.

In narcissistic patients, the acknowledgment and tolerance of envy heralds the shedding of the husk of the grandiose self that previously encased the individual, insulating him or her from perceiving and recognizing certain disconcerting realities about self and others. Interestingly, at this point, Mark began to question his continual postponement of committing to his girlfriend and having a child. He had rationalized this because he believed that having a child would make too great demands on his time and limit his investment in work and travel. Now he began to realize that his fantasy of having children at some indeterminate future date while not making space for them in his life represented an inability to acknowledge the implications of the passage of time (Kernberg, 2014).

With many narcissistic persons there is the sense that one is not bound by the strictures of time and that decisions and commitments can be postponed indefinitely without consequences for self or other. The lessening of his omnipotent attitude toward time led Mark to recognize that while he cared for his girlfriend and appreciated many of her positive qualities, he was not in love with her, now that he had a better sense of what it meant to be in love, and that to continue the relationship would be exploitative and self-defeating. While his search for information about the therapist on the Internet showed vestiges of omnipotent control, it also showed a recognition of her as a separate person with her own unique identity and reality, an authentic curiosity about her as

a person separate from his own needs and desires, and some capacity to acknowledge aspects of her life that were lacking in his own, even while this knowledge evoked painful feelings of exclusion and envy. Previously genuine interest in and curiosity about the other, including recognition of positive aspects of the lives of others, including the therapist, were not possible because they evoked intense feelings of vulnerability, envy, shame, and competitiveness.

Dealing with Loss, Limitation, and De-Idealization

We have found that a crucial aspect of the movement toward integration for narcissistic patients involves the recognition and full experience of the individual's rage and destructiveness about limitations, losses, and imperfection; this is an aspect of the depressive position. The development of a more integrated self in which vulnerability, weakness, and dependency are better tolerated and grandiosity tempered, continues to vie in the later phases of therapy with a destructive, envious omnipotent aspect of the self that periodically resists genuine therapeutic engagement and exploration. Such narcissistic omnipotence is only overcome with the attainment of the depressive position in which love and gratitude toward the internal and external good objects, now experienced in all their complexity as separate from the self, counter and contain hatred and envy. In the depressive position, negative feelings are tolerated, ameliorated, and contained within the self rather than projected; hence, the predominant anxieties are no longer about fear of annihilation from the other, but about fears of loss of love as a result of one's own aggression and the damage done to good objects, both internal and external (Klein, 1946; Steiner, 2003, 2011; Kernberg, 2014; Joseph, 1989; 2013).

However, the working through of such depressive anxieties may be complex for those with narcissistic disorder. The dissolution of the narcissistic structure may leave the individual prone to the full experience of intense negative feelings of hatred and envy of objects, which have been denied and deflected, preventing the patient from taking in and experiencing love uncontaminated by negative feelings. The experience of such negative feelings toward others, whose value and autonomy are recognized, now may plunge the individual into depressive anxieties involving guilt and fear of his or her own destructiveness. This can lead to feelings of desolation and despair. Mark's apocalyptic fantasies of destruction and annihilation of self and others are examples of such depressive anxieties that can emerge in the later phases of therapy, as the individual realizes that those he loves and cherishes can also be the objects of his hatred, resentment, and malevolence.

Such anxieties and the retreat from them may contribute to the profound depression these patients can experience in the later stages of

therapy. There are several reasons why the depressive position may be particularly difficult or prolonged in those with narcissistic pathology. The dissociation between affective and cognitive empathy noted earlier means that those with narcissistic disorder may be unable to share or appreciate the emotional states of others, although they may have an intellectual understanding of the other's feelings. Unable to share what others feel, those with pathological narcissism may experience little empathy for those they exploit and therefore have little guilt (Leunissen, Sedikides, & Wildschut, 2017). As the capacity for emotional empathy expands with treatment, the individual may be able to understand on an emotional level the experience of others whom they might have previously exploited, taken for granted, or otherwise injured. With increasing recognition of their own imperfections, flaws, and shortcomings that were previously denied, along with the destructiveness of their entitled and exploitative attitudes toward others, those with narcissistic pathology may experience considerable and sometimes prolonged and excruciating guilt. Reparation then has a particular meaning in that it involves attempts to restore bonds with others and repair broken relationships, or if this is not possible, to mourn and take responsibility for how relationships may have been damaged by their own aggression that previously was projected onto others.

Some patients may not be able to tolerate such depressive anxieties and may terminate before experiencing their full scope. It is possible that the increasing recognition of the need for and dependency on others may lead to a flight to manic mechanisms of omnipotence, contempt, and triumph over others, including the therapist. These mechanisms may take the form of insistence that the individual does not need the therapist anymore, that no lasting change is possible, or if change is acknowledged, that he or she accomplished it on his or her own. Or the individual may develop into a full-fledged depression involving a profound sense of futility, loss of meaning, and even suicidality that comes with the recognition that the past cannot be redone.

In our experience, a depression that develops in the course of treatment has a better prognosis than if the individual presents initially with severe depression in midlife or on the cusp of old age in the context of a narcissistic disorder. Treatment in such cases ought to be multimodal, involving psychopharmacological intervention and even hospitalization if the depression does not remit in outpatient TFP-N, with therapy continuing after the most severe symptoms of the depression have been ameliorated. Sometimes a supportive treatment may be the treatment of choice, at least initially until the depression is less severe and can be worked with dynamically. However, in our view, an essential aspect of therapy in such cases involves mourning the losses (of significant others, family ties, occupational opportunities, and status as well as the

ideal sense of self that might have previously sustained and insulated the individual from such painful realities that contribute to or catalyze depressive states) and acknowledging and working through the sense of narcissistic injury that interferes with fully grieving such losses. In our experience, the inability to work on these issues dynamically may lead to other complications, such as substance abuse or a retreat from others into splendid isolation. In such cases, a TFP-N approach that focuses on the dominant dyad of a damaged self-concept in interaction with a harsh, critical other that is often at the base of such severe depressions (Blatt, 2004, 2008) is warranted.

One aspect of the depressive position functioning that is particularly difficult is mourning for former states of grandiosity and omnipotence that insulated the individual from painful realities. In the later stages of therapy, for example, Mark began to relinquish grandiose illusions of perfection and acknowledge the ways they constricted his experience of self and others. This was evident in rueful acknowledgment that he did not feel as important, powerful, or invulnerable as he had previously while recognizing that this shift in self-concept was more authentic and reflective of his capacities and life stage. He also recognized that he had reached a stalemate in his current relationship and that he had constructed through it a relational vise that kept him from moving forward to find a deeper, more enduring love and commitment or moving away for fear of the pain of the loss. After some months of exploring these feelings, he ended the relationship and began to date, not only women who were either hopelessly out of his league or women he could control and devalue, but women he respected as equals. In the course of a year, Mark met a number of women and was able to tolerate both the experience of being rejected and the experience of rejecting others. After several failed relationships, he was able to fall and remain in love with a colleague whom he met while working on a project. The relationship blends friendship based on respect, caring, and shared interests and values with sexual passion that has endured. Tellingly, he talked about how he realized his new love object was not perfect but relished introducing her to friends and family so that others might appreciate the qualities that made him fall in love with her. This is in striking contrast to his previous relationship, where he was inordinately sensitive to the faults of the loved object, imagining that others would see him as lesser because of her imagined shortcomings.

Another aspect of working through depressive anxieties involves relinquishing the reliance on idealized objects that previously sustained, if isolated, him. As Mark worked through cycles of idealization and devaluation in the transference, he began to understand how

his idealization of others, including the therapist and his parents, had grossly exaggerated and distorted their good qualities. Previously, he had described them as superhuman, almost mythic figures—successful, accomplished, and indomitable—who had created the perfect family and had a perfect relationship that he felt he could never match. Over the course of therapy, he recognized that this idealization had functioned to prevent acknowledgment of their negative qualities, including their limitations and frailties. For example, Mark realized that his parents excluded aspects of their children's lives that did not fit with their ideal scenario so that his brother had been ostracized by his mother who saw him as far less than her other children. Feelings of failure, rejection, fear, and severe upset on the part of the children were ignored or dismissed because they were not part of the existence of a family organized around fantasies of invincibility and success. However, loyalty to and protection of idealized internal objects, along with difficulties in relinquishing their ideal demands, may prolong and complicate this process for those with narcissistic pathology.

How an Object Relations Approach Fosters Integration and Reflection

Necessary for personality reorganization is an interpretive approach that balances interpretation and containment, and reflective curiosity about the functions and costs of the narcissistic organization with exploration of its developmental antecedents in the experience of early attachment relationships as they are lived out in the moment-to-moment relationship with the therapist. Although transference interpretations may provoke feelings of anger, envy, and hatred, as well as gratitude, appreciation, and affection in the later stages of treatment, these affective reactions may in fact signal an expanded capacity for involvement in depth with the therapist. This is evident in the lessening of unrealistic idealization or scathing devaluation in the transference, and increased playfulness and moments of genuine emotional connection and attunement along with expressions of gratitude for the ways that the therapist's interpretations may have promoted the patient's self-understanding and growth. The patient will remember and repeat such interpretations and use them as a springboard to further self-exploration, often over many sessions.

In the later stages of TFP-N, as the patient reintegrates aspects of the self previously split off, denied, or projected, the therapist may more productively explore the unconscious conflicts, motivations, and meanings of the patient's myriad experiences in the transference, along with a more sustained exploration of their connection to early experience and

relationships. For example, in the later stages of the therapy, as Mark was able to form a relationship with a woman for whom he felt both tenderness and sustained erotic desire, a blend he had predicted at the outset of therapy that he would never achieve, there was a resurgence of oedipal issues and conflicts. Desires to possess his mother and guilt over rivalrous feelings toward his father, who had died suddenly of a stroke when Mark was 12 years old, came into focus. He recalled his mother as an ideal figure, but as tantalizing and teasing, emotionally remote and unattainable. These conflictual feelings interfered with his capacity to love fully—that is, to engage in a relationship with deep complementarity and reciprocity in which sexual passion could be experienced freely without undue guilt or aggression.

The Working through of Oedipal Issues in Narcissistic Disorder

The attachment to ideal internal objects described above in the case with Mark is a complicating factor in the working through of oedipal issues in those with narcissistic disorder. With the increased capacity to value and love the other as objectively perceived, the individual will relinquish the need to control the other. He or she will be able to acknowledge and tolerate the significance and exclusivity of the object's relationships with others from which he or she may be excluded. In this process the individual moves from early, archaic, oedipal configurations in which he or she forms an identification with a parental couple that may be seen as highly idealized or destructive (Britton, 2003, 2014) to a representation of a parental couple as creative and productive. This happens even while the individual works through the feelings of envy, exclusion, rivalry, and hatred that are evoked. The latter was evident in Mark's capacity to tolerate and explore his painful feelings of envy and sense of exclusion at learning more about the therapist's life. This in turn allows patients to better tolerate and benefit from the therapist's creative interpretive capacity and allow him or her "a mind of one's own."

With narcissistic patients, envy, or the desire to appropriate the characteristics of the other, may color and even intensify oedipal issues or the difficulties with competition, rivalry, exclusion, and inclusion in triangular relationships. Early in the treatment of those with pathological narcissism, envy is usually displaced over a number of individuals in the patient's life and there may be enormous difficulty in acknowledging envy of the therapist. This usually emerges directly in the more advanced stages of treatment as devaluation and other defenses against envy diminish. As envy of the therapist is experienced and worked through, issues around competition, rivalry, and exclusion in relation to others, no longer either highly idealized or devalued, may be better tolerated.

In general, in work with narcissistic patients, the manifestations of triangular oedipal issues and conflicts in patients with pathological narcissism have been underemphasized in part because of the centrality of dyadic issues around dependency, envy, and self-regulation. However, we have found in our work with both higher- and lower-functioning narcissistic patients that oedipal issues are evident throughout the course of therapy. Although conflicts around triangular relationships may emerge at any phase of the therapy, they increasingly come to the fore in the advanced stages of treatment as omnipotence lessens, envy diminishes, and depressive anxieties stemming from increased tolerance for and recognition of ambivalent feelings toward others in all their complexity, including the therapist, become predominant. They may first manifest themselves in archaic issues about intolerance for being excluded from the intimacy of the parental couple, which in more severe cases may extend to intolerance for any type of thirdness that affirms the autonomy and separateness of the other. This includes intolerance for the therapist's creative interpretive efforts that may be linked to an imagined exclusive internal dialogue between parental figures (Britton, 1989). More differentiated manifestations of oedipal dynamics evolve over the course of treatment, as the individual becomes better able to tolerate separateness, envy, competition, or rivalry. As Britton (1989) puts it, "this third position provides us with a capacity for seeing ourselves in interaction with others and for entertaining another point of view whilst retaining our own, for reflecting on ourselves whilst being ourselves" (p. 87). The oedipal stage may be seen as the "culminating stage in a continuum of triadic experiences" (p. 85) in which the child's oedipal wishes and fantasies make him or her a witness of the exclusive link between the parents, and an object in the observations of a third, as well as a participant in dyadic links with each parent.

The individual with narcissistic pathology is unable to tolerate these multiple psychic positions involving disparate representations of parents as loved attachment figures, desired oedipal objects, and dreaded rivals, and the disparate representations of self that are linked with them. The experiences of such variable perspectives on self and other foster the integration of the representations of the parents perceived in love and hate. The working through of oedipal issues evokes depressive anxieties about the impact of rivalrous and destructive feelings toward parental figures, but it also may evoke the desire to make reparation to the parental couple, now seen more realistically (Britton, 1989, 2004a, 2004b, 2014). Hence, depressive and erotic/oedipal transferences may be closely intertwined. In the case of individuals with severe narcissistic pathology, such integration is not possible because the grandiose self and the concomitant superego lacunae may limit the capacity for such reparative

strivings. Instead, the narcissistic individual strives to remain forever at the apex of the oedipal triangle, perpetually connected with conflicted desire and longing for exclusive possession of the idealized, unattainable oedipal figure, and/or perpetually seeking admiration from an idealized, if hated, rival/competitor.

The intolerance for being the excluded third other may evolve into several different types of oedipal configurations. There may be intolerance for triangular relationships, or there may be a persistence of fixations on intensely idealized, but unattainable, objects that sustain the individual because they both feed self-sufficient grandiosity while also fulfilling the need for punishment based on a covert sense of inadequacy. Such oedipal fixations and illusions may coexist sometimes for years with a seemingly stable, if somewhat shallow, committed relationship. In such cases, the intractable fixations on significant others who are idealized, but unattainable, may sustain the individual, but their sudden collapse may lead to destructive or self-destructive acts.

For example, a seemingly well-adjusted, successful female associate in a law firm developed a fixation on her mentor, the senior managing partner in her firm, convinced that he not only recognized her superior talents, but that he also shared her passion. Although he had always behaved in an appropriate but generative fashion toward her, she believed that when she made partner and was his equal, he would declare his love for her and leave his wife. When she learned that his wife was pregnant and he was a taking a paternity leave, she made a serious suicide attempt, signaling the sudden collapse of her oedipal illusion.

Another example is a relatively high-functioning physician who remained fixated on a relationship with a colleague whom she had met at a conference in Europe 8 years previously and with whom she had an intense, brief affair. The relationship continued without a definitive commitment, and although she saw him only intermittently, she persisted in the illusion that he was the only man she could love, that this feeling was reciprocated, and that he would leave his wife and marry her. The therapist's attempts to explore the tenacious nature of these illusions and how they in fact curtailed her potential to form a relationship in the present that might in fact lead to an enduring commitment were met with scathing attacks that the therapist did not understand the depth of her passion for such a special love object. It was only when the therapist began to explore how this illusory relationship in fact also limited her engagement in therapy that she began to understand the meaning of this oedipal illusion. Far from expressing a great passion for another, it was fueled by narcissistic fantasies of perfection, self-sufficiency, and emotional remoteness that in fact curtailed and constricted her relational possibilities.

Another typical manifestation of oedipal conflicts characteristic of narcissistic pathology involves the pattern termed *reverse triangulation* in which the narcissistic individual splits the image of the other into the parental (maternal or paternal) nonerotic partner and the devalued, erotically exciting lover, thereby preserving his or her position as the child in a triangular relationship where he or she remains the object of adoration, desire, and competition between the two parents. Such a scenario differs from the typical oedipal one not only in the depth of the split between the valorized but asexual and the devalued but erotically exciting objects, but also in the infantile, dependent nature of the relationship with both love objects. The individual is unable to sustain a long-term commitment or invest in a love relation with either in depth, and this is reflected in the attitudes of entitlement and exploitativeness toward both figures (Kernberg, 1995). Often it is in the later or even termination phase of the work that the full extent of triangular oedipal conflicts emerge and are worked through, as the individual begins to experience the whole constellation of rivalry, longing, desire, and renunciation in relation to the therapist, now seen more realistically as a valued, if limited, other worthy of intense feelings and fantasies.

The above examples illustrate how triangular oedipal conflicts in narcissistic personalities can be interwoven with dyadic conflicts around fears of dependency and attachment, as well as with regressive pulls toward retreat to grandiosity and symbiotic union. Kernberg (1999) has advocated careful assessment of sexual functioning in narcissistic patients, especially in the later phases of treatment in order to gauge the extent of change in object relations, and as a conduit to unresolved oedipal conflicts. Kernberg provides a case of sexual inhibition in a high-functioning narcissistic male patient that emerged in the termination phase and was linked to the resurgence of archaic oedipal conflicts of being "the third excluded other" in relation to a revered male analyst from whom he feared both separation and retaliation. All of these foregoing examples show the persistence of narcissistic illusions. This involves an inability to acknowledge and accept the limitations of the self or the realities of the object, including the "continuity of its existence in time and space" (Britton, 2003, p. 33), along with the realities of the object's other affectional bonds. The foregoing are all aspects of a lack of oedipal resolution and intolerance for triangulation that are major but undertheorized aspects of narcissistic pathology. Without the relinquishment of such oedipal illusions that are linked to a sense of narcissistic omnipotence and completeness (idealized aspects of the grandiose self), the depressive position cannot be worked through (Britton, 2003; Kernberg, 2014). In fact, essential aspects of the depressive position involve relinquishment of the control of significant others; tolerance for their

intimate relationships with others beyond the self; and acknowledgment of the need and desire for others, as well as empathy for them; along with a capacity to perceive and love others objectively, separate from one's own needs and projections (Segal & Bell, 1991). Hence, the full experience of oedipal desire, rivalry, and relinquishment of illusory oedipal relationships, as well as mourning for the loss of such illusions, may be inextricable aspects of the working through and resolution of narcissistic pathology (Britton, 2003, 2014; Kernberg, 1999).

Termination

As is the case with most patients, termination with those with pathological narcissism is often a time of mourning, regression, and consolidation of gains made during the treatment. As is the case with most psychoanalytic therapies, the criteria for termination include the capacity to love, greater integration in the internal object world between positive and negative aspects of experience, strengthening of reality adaptation, greater productivity and creativity at work, and a more realistic sense of self (Klein, 1950). As discussed earlier, mourning takes place not only for the end of treatment and the loss of the therapist but also for the loss of narcissistic illusions. In the process of exploring again the patient's anguish about the emotional unavailability of caregivers, experienced as cold, unattuned, and exploitative, the patient may become quite acutely symptomatic, either depressed as indicated above or severely anxious. Furthermore, with termination approaching, the transference may intensify as experiences of depression and anxiety are linked to early experiences of separation or loss. Not surprisingly, given the tendency of those with narcissistic pathology to hold the therapist at arm's length and defensively devalue the therapy, conflicts around dependency and attachment that may not have been fully explored or worked through may take center stage in the patient–therapist interaction in the termination phase.

For example, Mark became briefly but quite acutely symptomatic during a 2-week break in the therapy during the termination phase. He experienced anxiety that escalated on several occasions into a full-blown panic attack. The covering therapist prescribed low-dose benzodiazepines, which Mark took briefly, as he was unable to sleep. Upon the therapist's return, she explored with Mark how these symptoms might be related to the termination of therapy. Mark acknowledged that the roots of the panic attacks lay in his dread of and anger about the impending separation from the therapist, and with further exploration of how this was evoking anxieties about previous separations from parents, and

their emotional unavailability, the full emotional significance of which he had previously evaded, the symptoms resolved.

Therapists at this point may have doubts about whether termination should proceed in the face of the flaring up of symptoms. However, unless the patient shifts from being transiently symptomatic to being chronically symptomatic, it is important to see this as part of the response to termination in patients who are no longer protected by a grandiose self-structure. Therapists should accept and explore the outpouring of sadness, anger, anxiety, fear, grief, and longing that may come forth as the end of treatment approaches, and allow sufficient time for the patient to mourn for the impending loss of the therapist. As is the case with most patients, the ending of the therapeutic relationship may also evoke earlier losses, but with narcissistic patients it may evoke not only past losses of significant relationships but also the losses involved in their own narcissistic solution of turning away from emotional engagement with others. Tolerance for intense feelings of love and hatred toward the therapist in the termination process may actually bespeak a lessening of such narcissistic disengagement and a growing integration and strengthening of the patient's internal world as ideal representations of self and other evolve into more realistic, mature ones. As the patient comes to grips with all the various positive and negative aspects of the therapist and the self, and the gains as well as the disappointments in the treatment, the therapist becomes more securely anchored as a good, realistic, if imperfect, as opposed to an ideal or devalued, object in the patient's mind.

In order for the work of termination to proceed, there are certain pitfalls that the therapist must be aware of lest he or she prolongs the treatment either because of unresolved transference and countertransference dynamics involving perfectionism, or an inability to accept the limitations of treatment. With patients with pathological narcissism, therapists may be tempted to collude with the patient in a mutual idealization of the treatment and what it can and has accomplished. If the tendency toward idealization that often supplants devaluation with narcissistic individuals in the later stages of treatment is not interpreted and worked through, the patient may not be able to accept the limitations of the therapy, the therapist, or the self. In such situations, the work of termination will be short-circuited and therapy prolonged even in the face of major gains in coping and object relations, work, and love.

The integration of the patient's internal world, with its more complex and realistic experiences of self and other, can bring up conflicts around choices made earlier in life. A patient who married in accordance with the need to rid himself of any sense of a negative trait may no longer get the same gratification he once obtained from being married to a

woman he found easy to devalue. A patient with grandiose narcissism who was chief executive officer of a hedge fund started a foundation after he had been in therapy for several years and then decided to teach math in an inner-city high school because he had lost a sense of meaning and purpose in life. A woman who had chosen to work at a routine job despite having several advanced degrees, because of the sense of superiority she obtained from that work, may now question that choice and wonder about changing to a career and a more challenging job in which she would use her graduate training. In situations such as these, earlier life choices fit less well with an integrated view of self and others. The role of the therapist is to maintain neutrality, to help the patient review the various sides of the conflict (e.g., the promise of a more satisfying relation vs. the disruption to the family that would result from divorce), and come to his or her own decision of how to resolve the conflict. The patient's ability to do so is enhanced by the newly developed ability to accept limitations and understand that there is no perfect solution to many situations in life.

Summary and Conclusion

In TFP-N, the therapist moves from (1) clarifying and containing the patient's affects through interpreting in the projection; to (2) defining the dominant object relations that comprise the grandiose self and the patient's identification with both self and object poles of the dominant dyad; to (3) working with role reversals as the self and object poles of the object relational dyads are alternately enacted and projected in the transference (which catalyzes change by challenging the ideal sense of self); to (4) identifying and working with object relational dyads with opposing affects that defend against each other (which fosters the resolution of the grandiose self); and then to (5) exploring hypotheses about why such divisions have occurred, linking primitive defensive operations to unconscious conflicts, wishes, fears, and motivations, and their historical antecedents. With the increasing integration of the representational world, the array of defense use broadens to include higher-level defensive operations and a lessening of primitive defenses (Levine, 2002). This broader range in turn enables the individual to reflect on emotional states that were previously projected, acted on, or experienced as somatic symptoms. The person's understanding of the repertoire of self and object images that populate his or her internal world (and their impact on his or her experience of self and other in the here and now of the transference) in turn fosters the development of more differentiated and mature levels of self and object representations, and a more integrated, complex,

and realistic identity. For narcissistic individuals, such a more integrated identity includes:

1. The gradual relinquishment of grandiose illusions about the self and the capacity to understand and reflect on the defensive function of such illusions and their developmental antecedents.
2. The understanding of how the grandiose self has served a defensive function by distorting views of others in an attempt to manage idealized and devalued or aggressively laden internal objects, and how this has isolated the individual and undermined involvement with others as separate and valued persons.
3. The development of an expanded repertoire of relatedness outside of the narcissistic organization (even while aspects of it may endure). This would be evident in the capacity for committed, enduring friendships and the capacity to fall and remain in love, with the idealization of the loved object sufficiently in line with the individual's real qualities; these are valued and cherished while recognizing limitations and flaws.
4. The internalization of aspects of the therapist who comes to form a new object with both positive and negative traits.

Instead of immediate incorporation, devaluation, or idealization of the therapist's interventions, the patient gradually assimilates them, letting them germinate internally and using them as a jumping-off point to creatively elaborate on or reflect on aspects of experience that have been disavowed or split off. All of this implies the internalization of aspects of the relationship, particularly the therapist's analyzing function that enables the patient to tolerate, understand, and reflect on how self-states linked to different object relational dyads might function in relation to each other and to external reality. The gradual integration of disparate, split, polarized self and object representations lead to an overarching stable concept of self and significant others or to improved identity integration. The individual develops a more stable and coherent mental model of self, albeit one that may encompass contradictions and inconsistencies. The individual also develops a stable and consistent model of others, albeit one that may encompass ambivalent feelings of love and hatred, against which momentary mental states, even those that involve extreme affects, can be assessed and evaluated. Increased integration in the internal world of those with significant narcissistic pathology ought to be reflected in the interpersonal sphere in the shift from relationships based on admiration and exploitation to those characterized by rich transformative exchanges with others based on appreciation of the other as valued, separate, and admired for his or her unique

qualities. An enduring intimate love relationship is often the last arena with narcissistic patients because they have destroyed intimate bonds with spouses, partners, and children over time, and also because taking the risk of needing and desiring another inherently involves, as Freud (1914/1953) put it, the submersion of one's own narcissism in the gratifications of loving another deeply, which always incurs the risk of facing limitations, rejection, and/or loss. In our experience, it is lingering difficulties with love relations that sometimes form the impetus for narcissistic patients to seek further treatment or to return to treatment later in life. Limitations in the capacity to love in narcissistic persons is the topic of Chapter 11.

In Chapter 7 and the current one, we presented the whole trajectory of TFP-N from contract setting through termination with a focus on both the more typical and the higher-functioning narcissistic patients. In the following two chapters, we present modifications of TFP-N for patients at the more disturbed end of the narcissistic spectrum—those with comorbid narcissistic and borderline disorders (NPD/BPD; Chapter 9), and those with malignant narcissism organized at a low borderline level (borderline personality organization [BPO]; Chapter 10).

CHAPTER 9

TFP-N for Patients with Combined Narcissistic and Borderline Pathology

In this chapter, we focus on the treatment of more severely disturbed narcissistic patients, those with combined narcissistic and borderline personality disorder (NPD/BPD). Several studies have shown particularly high rates of co-occurrence of NPD with BPD, ranging from 17 to 80% (Clarkin et al., 2007; Diamond, Levy, et al., 2014; Ronningstam, 2009). In the Wave 2 National Epidemiologic Survey on Alcohol and Related Conditions (NESARC) Survey, over 62.9% of those with NPD were found to have a comorbid BPD diagnosis, while 38.9% of those with BPD had a comorbid NPD diagnosis (Stinson et al., 2008). Given the frequencies with which borderline and narcissistic disorders co-occur, it is surprising that there have been relatively few clinical or empirical investigations of the particular characteristics of these patients or the clinical challenges that they present.

As discussed earlier, narcissistic patients organized at a lower borderline level, particularly those with co-occurring BPD, are often characterized by a more thin-skinned or vulnerable narcissistic presentation. In these individuals, the compensatory grandiose self is structurally weaker, more brittle, and aggression laden, so that the individual shifts from states of arrogance, exaggerated self-importance, and superiority to debilitating feelings of inferiority, inadequacy, insecurity, humiliation, and shame, sometimes leading to suicidal tendencies. Because the grandiose self functions less efficiently to maintain the boundary between idealized and devalued aspects of self, the alternations between grandiosity and vulnerability, and inflated and deflated self-concepts, are more extreme. The individual may present with chronic suicidal

tendencies, severe characterological depression (that often does not remit with medication), and marked identity diffusion. In other words, the split in the sense of self is more pronounced, narcissistic defenses are less effective, although often more brittle and intractable, and the sense of identity more tenuous or illusory—all of which lead to greater affective and behavioral dysregulation, rendering these patients among the most difficult to treat. In addition, these patients are likely to have insecure/disorganized attachment that bespeaks a lack of structure for regulating relational and affective experience stemming from multiple, often contradictory representations of attachment figures and/or lack of resolution of trauma and abuse (Fonagy et al., 1996).

As discussed in Chapter 3, attachment representations are an important component of the psychological structure for understanding the development and maintenance of personality disorders, as well as for assessing aspects of the therapeutic process and outcome over the course of therapy. In this chapter, we illustrate how the concepts of attachment and mentalization may help to understand and illuminate the often contradictory transferences, defenses, object relations, and resistances we observe with more severely disturbed narcissistic patients by focusing on a female patient with NPD/BPD during 1 year of TFP-N.

Case Example: Rebecca—NPD/BPD

Rebecca, introduced in Chapters 1 and 2, a single, unemployed Asian woman, was referred for TFP-N at age 27 after several unproductive outpatient treatments and one brief hospitalization for self-destructive behavior. Her condition had worsened to the point where she had spent the 6 months prior to beginning TFP-N isolated in her apartment, lying in bed binge eating, only rarely bathing, with chronic suicidal ideation, but no major attempts or gestures. Like many patients in the borderline range of narcissistic pathology, Rebecca presented in a state of isolation and despair, focusing on herself as an inadequate failure (low explicit or conscious self-esteem) who had felt so chronically mistreated by others that she had no choice but to withdraw from the world and relationships. However, this initial presentation concealed a covert grandiose image of herself as destined for great things and deserving of a prestigious career despite her difficulties with making consistent efforts to realize her goals.

Rebecca had aspired to go to law school, but failed to do well on the entrance exam. She opted for a master's program in legal studies but soon dropped out, feeling it was beneath her. She then held a series of jobs as a legal aide, but repeatedly became embroiled in altercations with her coworkers, often involving her perception of others as critical or rejecting toward her, which led to her either being fired or quitting. Rebecca's sense of entitlement was evident in her feeling that it was

beneath her to work at a job below her imagined capacity. She preferred to be supported by her parents, withdrawing to a world of splendid isolation where she could sustain her illusory sense of specialness that masked her excruciating sense of shame. Her extreme narcissistic vulnerability was evident in her statement, "I always felt pressure to appear smart and put this self-image out because I couldn't just be me, cuz that's not good enough." When she failed to attain her goal of achieving a high-enough score on the law school entrance exam to apply to a good law school, she withdrew from her family and friends, refusing all phone calls and contact, and was furious when her family called 911 to check on her safety. It was at this point that she was referred by her family for TFP-N.

At admission to the study Rebecca met criteria for not only BPD but also NPD and avoidant personality disorder on the International Personality Disorder Examination (IPDE; Loranger, 1999), and she met Structured Clinical Interview for DSM-IV Axis I Disorders (SCID-I; First, Gibbon, Spitzer, Williams, & Benjamin, 1997) criteria for current dysthymia. On the Inventory of Personality Organization (IPO; Clarkin et al., 2001), a self-report measure designed to assess level of personality organization, she scored very high on identity diffusion and primitive defenses and evidenced compromised reality testing and elevated aggression scores. On the Adult Attachment Interview (AAI) given at admission to the study Rebecca showed disorganized states of mind with respect to attachment figures, shifting chaotically between dismissing devaluation and angry preoccupation with parental objects with whom she was emotionally entangled. This led to an attachment classification of cannot classify, with mixed preoccupied/angry and involved, and dismissing/devaluing (CC/E2/D2) states of mind.

Rebecca was the oldest of three children in an upper middle-class family that immigrated to the United States so that her father could pursue a legal education. She described her mother as extremely controlling—the kind of mother who was "always running around with us, taking us to ballet class, and you know, pushing us to the limit." Her first memory of her mother was of being punished by her for taking her younger sister's milk bottle when Rebecca was 4 or 5 years old. She felt chronically rejected by her mother, who was often depressed, suicidal, and unable to care for her children. She felt that her mother gave her "mixed signals": at times clinging to her when she [mother] was lonely and in states of depression, but at other times shutting her out, locking her in a car alone on one occasion and frequently sending her to her room for punishment. Rebecca stated that she often felt that she did not really have a mother. She did not remember "anybody comforting up to age 13—I just remember being disciplined, I don't remember comfort."

Rebecca described her father as emotionally absent and preoccupied with his academic career, but she also experienced him as obsessed with

his children's educational performance to the point of frequently abusing them verbally and sometimes physically if they did not perform up to expectations. Her father had erotized his relationship with her in that he treated her like a surrogate wife, expecting her to cook for him and accompany him on business trips when her mother was incapacitated with depression. However, Rebecca felt that she "lost" her father when she was not doing well in school or not achieving and felt he had abandoned her emotionally when she failed to get a competitive score on the law school entrance exam, dropped out of her legal studies graduate program, and spiraled into a state of despair and paralysis.

There are many indications of borderline pathology in the description of Rebecca, including (1) affective instability and impulsive aggression, evident in her lashing out at coworkers and supervisors; (2) identity diffusion, evident in her inability to imagine anything but an illusory identity as a top-notch lawyer while being chronically under- or unemployed; and (3) self-destructiveness, evident in her intermittent suicidality and binge eating (she was obese when she entered treatment) among other behaviors. However, Rebecca also met criteria for NPD. Narcissistic features were evident in her sense of detachment and antagonism toward others, her self-punitive perfectionism, and her sense of self as special and unique as a result of her father's attention. She described herself as "the pretty one," who was envied by her mother and sisters, even though she also felt herself to be disgusting. She gave "Daddy's girl" as the description of her relationship with her father, but also talked about how she never felt she lived up to his expectations. She similarly described significant others in contradictory ways, insisting during the initial evaluation (the STIPO-R) that her father was "like Stalin," although she countered this by saying that he took good care of her mother when she was depressed. When the therapist pointed out this contradiction, Rebecca insisted that "he was like Stalin and maybe that's why mother was depressed in the first place." Nonetheless, she identified with her father as a powerful, punitive figure whom she also idealized, reporting that when stressed she would retreat to a fantasy world and pretend to be a lawyer, like her father, examining witnesses at an imaginary trial. She would only work at high-prestige law firms, and since she was unable to hold a job in those contexts, she stayed in bed watching reruns of legal programs while being financially supported by her parents.

Characteristics of Individuals with NPD/BPD

Individuals with both borderline and narcissistic disorders, like Rebecca, have been found to be among the most treatment-refractory patients in the personality disorder spectrum because of the rigidity of their

personality structure, the severe pathology of object relations that translates into destructive and self-defeating interpersonal patterns, and the reliance on primitive defenses—a finding that has been confirmed by a number of other clinical investigators (Kernberg, 2007; Stone, 1989, 1990, 2003, 2016; Clemence et al., 2009). In addition, more severely disturbed patients with NPD/BPD often have a history of trauma, including emotional and physical neglect and abuse, sexual abuse, and lack of protection (Diamond et al., 2013; Simon, 2002). They also often sanctify their own suffering because it makes them feel special and unique. Furthermore, although they may report levels of trauma similar to other patients with BPD, the grandiose self-structure makes them more resilient but also more rigidly defended, thus making the trauma more difficult to access and work through (Diamond et al., 2013).

Although individuals with NPD/BPD show the core features of narcissistic pathology, including an exaggerated sense of self-importance, exploitativeness, and entitlement, as well as excessive need for admiration from others, lack of empathy, and excessive envy, they also present with a rigid and fragile grandiose self, severe identity diffusion, lack of anxiety tolerance, poor impulse control, and precarious self-esteem (Vater et al., 2014). All of these factors result in self-defeating and/or tumultuous relationships or near total withdrawal from interpersonal contact and chronic failures in the spheres of love and work (Kernberg, 1997, 2007). In addition, although individuals with NPD/BPD share core structural features, they show some specific variants of those features.

As is the case with higher-functioning narcissistic individuals, the grandiose self that merges ideal and real elements of self and others is a compensatory structure, but with individuals with NPD/BPD, it is also particularly rigid, inflexible, and illusory. The idealized elements may eclipse the real aspects of self, so that these individuals' aspirations are often so out of line with their real capacities that they are bound to fail or collapse, as did Rebecca. To the extent that the pathological grandiose self protects against an intolerable sense of inferiority, it may lend the illusion of stability to those with NPD/BPD, particularly in the case of the overt or thick-skinned narcissist (Bateman, 1998; Rosenfeld, 1964, 1971). If more thick-skinned, these patients may initially present as higher functioning, and the personality pathology is missed, whereas if thin-skinned, they may present with not only borderline pathology but also symptoms of depression, anxiety, eating disorders, and/or substance abuse, and the narcissistic personality element is missed. This was the case with Rebecca, who was diagnosed and treated unsuccessfully for depression by previous therapists.

First, although patients with NPD and NPD/BPD are characterized by disturbances in the realm of identity, those with NPD/BPD are characterized by more severe identity diffusion rather than the milder

disturbances in identity found in higher-functioning individuals with pathological narcissism and NPD, who may retain the capacity to read the subtleties of interpersonal situations in the service of garnering the admiration and support from others that they need to enhance their self-esteem and to realize their goals. For higher-functioning narcissistic individuals, grandiosity, exhibitionism, and exploitativeness in some situations may be adaptive and even confer some benefits, especially if these are based on some capacity for a realistic self-concept existing alongside the grandiose self. However, this is rarely the case for those with NPD/BPD, such as Rebecca. Her grandiose self-perception masked severe self-depreciation that precluded any realistic evaluation of self and others that might lead to the development and realization of her actual capacities or ability to work productively toward long-range goals.

Second, although the pathology of object relations characterizes patients with both BPD and NPD, the extreme devaluation and idealization of self and others characteristic of individuals with NPD/BPD interferes with the internalization of others, leading to deterioration and impoverishment in the internal world of object relations. This also creates a dread of dependency, since how can one depend on a devalued other and/or how could one experience such an other as a sustaining internal figure? Similarly, how can one live up to or be sustained by a highly idealized internal figure with harsh demands for perfection? Consequently, involvement with others both internally and externally may be curtailed. The extreme nature and lack of integration in the internal representations of self and others was evident in Rebecca's contradictory descriptions of her father as "Stalin" on the one hand, and as helpful to her mother on the other, and in her inability to reflect on these contradictions or even to show awareness of them.

Third, with individuals with NPD/BPD, there may be a lack of investment in and capacity for warm, trusting, and enduring relationships with others, which often masks investment in highly idealized powerful, punitive internal objects. Such was the case with Rebecca, who avoided all intimate relationships, instead dwelling in a private world of grandiose fantasy and clinging to an illusory identity linked to an idealized, if fearsome, paternal figure. Despite impairments in all of the foregoing dimensions, narcissistic patients, even those with comorbid borderline pathology (BPD and/or borderline personality organization [BPO]; cf. Kernberg, 1984) may not manifest the acute symptoms of those with borderline disorders without pathological narcissism (Hörz-Sagstetter, Diamond, Clarkin, Levy, et al., 2018)—although they often suffer from depression, anxiety, and even suicidality. They often, however, show a decline toward self-destruction when their unrealistic expectations of

self fail to be met, and their relationships are often eroded by their attitudes of exploitativeness, entitlement, and lack of empathy—leading to deterioration in spheres of both love and work (Ronningstam & Maltsberger, 1998), as the case example illustrates.

Not surprisingly, the co-occurrence of narcissistic and borderline pathology has been shown to be a complicating factor for diagnosis, for developing a treatment alliance, and for working effectively with resistances and transferences (Clemence et al., 2009; Kernberg, 1984; Stone, 1989, 1990; Hilsenroth et al., 1998; Diamond & Yeomans, 2008; Diamond & Hersch, 2020). These individuals tend to provoke or alienate the therapist not only through their alternations between devaluation and idealization of the therapist but also through attempts to omnipotently control the treatment (Kernberg, 2007; Diamond & Hersch, 2020). Most often they present for therapy not because of acute symptomatic distress typical of BPD but because of core difficulties in self and affect regulation, as well as interpersonal functioning when their hostile, controlling behaviors (often masking crippling shame, self-loathing, and hypersensitivity to rejection) lead to a breakdown in their relational and occupational lives.

TFP-N with Patients with NPD/BPD

Rebecca began therapy dramatically. She walked into the therapist's office for the first meeting and, before sitting down, was talking nonstop about someone who had just offended her on the subway. The therapist commented that it must have been difficult to have such an experience on the very day she was to begin treatment, but then had to talk over her to remind her that this was a consultation session. He said that he could not address a specific problem with her yet because he did not have a sense of who she was, how to understand her psychological problems, or what treatment approach to recommend. Rebecca ignored his questions about her symptoms and his attempts to have her describe herself, as well as significant others and their relationships, and instead engaged in a pressured monologue.

After three initial sessions, the therapist sought help from the supervision group. The group's recommendation was that the patient's way of presenting provided the information he needed and that he could proceed to attempt to establish a treatment contract with Rebecca without gathering all the usual history. In the contract sessions, the therapist managed to present his impression that her main problem was a personality disorder with borderline and narcissistic characteristics, and that the depression was a by-product of this, which seemed to offer her some

relief as her many medication trials had been unsuccessful. However, difficulties arose when the therapist stipulated that treatment would require agreement on certain conditions, one of which was that Rebecca would have to engage in a structured, productive activity in addition to attending therapy sessions. The therapist proposed that this be in the form of either studies or some type of work, at least on a part-time basis. Rebecca responded that she could not possibly work and that she could "forget about therapy." The therapist suggested that she start with an unpaid part-time activity and that they discuss it further in the next session. She began the next session by announcing that she had obtained a volunteer job in a legal office on a half-time basis.

Once they began the therapy per se, Rebecca filled every session by flooding the therapist with an endless stream of discourse. The therapist perceived this behavior as reflecting a strong need to control every interaction. In a setting where the "basic rule" is for the patient to associate freely, there was the risk that this patient would continue talking indefinitely, without ever changing. The therapist felt the most effective therapeutic intervention would be to address the representations of self and other that he saw in her mode of interaction and nonverbal communication, and what he also experienced in the countertransference.

With regard to the content of Rebecca's verbal communication, her main theme was that she was constantly mistreated. She provided endless examples of this and continually made the point that she could never succeed in life because of the animosity others held toward her.

The therapist focused on three things: (1) Rebecca was flooding him with material (an implicit request/demand that he solve all problems and, in short, fix her); (2) every time he spoke up, Rebecca disregarded him and spoke over him (suggesting that narcissistic issues—the need to be in control and be right—predominated over dependency issues, at least for the moment); and (3) his experience was of being mistreated, devalued, and disparaged, just as Rebecca complained she was treated by others.

Employing the treatment strategies of TFP-N, the therapist began to name the "actors" or the dominant self and object dyads that emerged in the interaction. He asked Rebecca to reflect on the nature of her interaction with him. She said that she was just doing what he asked her to do. He agreed, but wondered whether she was doing it in a way that left him no room to participate and that perhaps her way of interacting exerted control over him. Initially, she gave no indication of reflecting on his comments. Her typical response was to escalate her examples of how people mistreated her and to suggest that he was just like everyone else because he was not listening to her (thus maintaining the projection). Because she kept talking over him, he pursued the strategy that, rather than address the content of what Rebecca was saying, he had to focus

solely on her style of interacting and the defense it represented. He made the following interpretation:

THERAPIST: You know, I find it hard to focus on what you say, and, in fact, I think there is more important information being communicated in *how* you are saying it—how you are interacting with me. Could we agree that you're talking nonstop?

REBECCA: Yes, you told me to say what's on my mind.

THERAPIST: Yes, but you are doing it in a particular way and I also said that my role was to offer questions and comments from time to time. Your way of speaking kind of keeps me "pinned to the wall" here; immobilized, so to speak. That's okay if you want to control me, but it might help to try to understand why you feel the need to do that. Do you have any thoughts about that?

Rebecca then angrily repeated that she was just doing what he had asked her to do—to say whatever came to mind, and that he was contradicting himself and demonstrating what she already knew—that he was not really interested in what she had to say.

THERAPIST: We might have the answer right there in what you're saying. [*In other words, looking for the motivation of the behavior.*] You're convinced that I'm not interested in you, that I don't care. Someone who believes that might want to control the interaction for fear that if she were not in control, she would lose the other person.

REBECCA: Of course I'd lose you if I didn't control you. [*Bursts into tears.*] Even my parents weren't interested in me, so why should you be?

Rebecca's direct expression of affect marked a beginning of communicating with her therapist with a sense of authenticity. He pointed out that Rebecca was not even allowing herself the chance to find out whether he was interested in her. By controlling the interaction to create a *semblance* of interest in her—she maintained indefinitely the belief that he was not *really* interested in her. And beyond that, she remained distant from him, since her monopolizing the interaction did not even allow him to be in the room as a full-bodied person. He pointed out that she related to him as though he were a two-dimensional cardboard cutout.

For the first time, the therapist felt that Rebecca was able to engage in a mutual dialogue with him. She became calmer and could acknowledge that she did not allow him to "exist" in the room and that this was

because of her fear that he would abandon her if she did not attempt to constantly control him. As the discussion continued, it became clear that this strategy left her feeling alone in relation to others, a condition for which she had blamed others. After this positive session, Rebecca returned to treatment with much the same mode of interaction that she had presented with from the beginning. The therapist had to return again and again to the interpretation of her omnipotent control and the fear it represented. It was only after repeating this interpretation many times, in the context of the therapeutic relationship where Rebecca allowed herself to sense his interest in her, that her behavior began to change. Her capacity to "let him exist," and to engage in a dialogue in which she revealed and explored her inner experience, increased.

The AAI: Clinical Implications

The work described above took place mostly in the first year of therapy. It should be noted that Rebecca was given the AAI (George et al., 1996) at the beginning of the study and after 1 year of TFP-N, as was the case with all patients in the research study. In brief, on the AAI, individuals are asked to give five adjectives to describe their childhood relationship with each parent and then to provide memories or experiences that illustrate and support these descriptors. There are also specific questions on experiences of childhood loss or abuse (e.g., physical, sexual, or other traumatic experiences that might have adversely affected the individual's development). Finally, there are questions that ask individuals to reflect on their experience, including how their childhood experience has affected their adult personality and relationships, and why they think their parents acted as they did. The questions are designed to assess metacognition and reflective functioning (see Chapter 3 for a description of the research study and measures).

After the treatment year ended, the therapist looked at the AAI data gathered by the research team in order to see whether it might help him understand aspects of the transference and countertransference. For instance, on the AAI at Time 1, before the therapy, the first adjective that Rebecca used to describe her mother was *controlling.* Other adjectives were equally negative (e.g., *emotionally abandoning*, *confusing* and *sad*, *weak bond*). These adjectives predicted the initial transference themes. As mentioned above, the focus of the first interpretations in the therapy were of the patient's need to control without being fully aware of it. Her enactment of the behavior she had been subjected to fit with the object relations concept of dyads of internalized self- and object representations that exchange characteristics, as the person alternately identifies with

one side of the dyad and then the other. Behind Rebecca's omnipotent control, evident in angry resistance to the therapist's attempts to explore their interaction or her internal world, was her conviction that she would be again neglected and abandoned. It is noteworthy that to illustrate the adjectives *emotionally abandoning*, she gave an early childhood memory of her mother leaving her alone in a locked car.

Addressing Rebecca's use of omnipotent control in the first year of TFP in the here and now of interactions with the therapist seemed to help her to free herself from this behavior and enter more deeply into other themes. On a more superficial level, Rebecca began to focus on the theme of being criticized and attacked by others. Interestingly in this regard, the first adjective that she used to describe her mother in the AAI after 1 year of TFP was *critical*—now *controlling* was no longer on the list. With the therapeutic dialogue more established, Rebecca could begin to see what the therapist had tried to point out at the beginning: that she sometimes treated others as she complained they treated her. She became more aware of an angry, critical part of herself that (1) she experienced as coming from others; but which (2) she actually directed toward others; and (3) she also directed toward herself; which (4) defended against longings for emotional connection.

On a deeper level than that of the projected critical part, Rebecca's discourse and behavior in therapy began to give evidence of positive, loving feelings in that she described her mother at 1 year not only as critical but also at times *protective*. This adjective showed that attention to the negative aspects of attachment were diminished, and her tolerance for ambivalence toward attachment figures was improved after 1 year of TFP. The therapist observed that Rebecca's projection of an internal critical presence in her mind—a part that could be aimed at herself or at others—had defended against longings for a dependent relationship with a caring, protective other. Over the course of the treatment year, her complaints of mistreatment decreased as she was able to see her anger and hostility in relations with others and in relation to herself. Subsequently, she began to report less anxiety in relation to others and more positive interactions in her work setting. She was offered a paid position at work and began to make efforts to develop friendships and intimate relationships. Let us further examine how these changes occurred.

Positive, loving feelings had heretofore been slow to emerge in the therapy or in the AAI—with one exception. To return to Rebecca's initial responses when asked to describe her parents on the AAI, the first adjective that she gave, and then immediately retracted, was that her parents were *loving*:

REBECCA: So, you want me to say adjectives. That's not a fair question, because you know, I mean of course I'm going to say loving and kind, because I love them, I mean, I did—

THERAPIST: Do you want to say that, then?

REBECCA: Well, I want to say it, because, I mean, does—is there anyone who doesn't feel love for their parent? Or you know I mean you feel something, you know? I mean, I did love them, you know? I mean I—but—you want to know how. . . .

THERAPIST: Well, I'd like it if you could just give me five words or phrases that describe your relationship.

REBECCA: You know what? You better erase loving, then.

THERAPIST: Excuse me?

REBECCA: You better erase loving. Cause I don't remember that. I just know I loved her, but I don't remember that, being in a loving relationship. It was more like "I'm the teacher, you're the student, do as I say"; that type of thing. It was controlling, I would say. So, you erased loving, right?

The above response for adjectives to describe her mother on the AAI shows Rebecca's disorganized state of mind with respect to attachment. She shifted from first describing her mother as "loving and kind" to describing her as "controlling," ending with dismissing derogation of their relationship ("I'm the teacher, you're the student"). While the latter is indicative of a dismissing attachment state of mind, the fluctuations between positive and negative attitudes toward her mother, and confusing, incoherent discourse are indicative of those with preoccupied states of mind. So, on the AAI at the beginning of treatment there were indications of oscillatory evaluations of parental figures, with devaluation and distancing alternating with elements of struggle, ambivalence and overinvolvement (Main et al., 2008) that is typical of individuals with disorganized attachment where there is a lack of integration in attachment representations and an inability to use any consistent attachment strategy to manage the affects that are aroused in being asked about the person's attachment history.

In the coding of the AAI, disorganization is observed in two ways: (1) as a lack of resolution with regard to loss or abuse indicated by lapses in reasoning or discourse on questions related to traumatic experiences leading to assignment of "unresolved" (U) and (2) as an inability to use any consistent discourse strategy, oscillating between mixed preoccupied and dismissing states of mind leading to assignment of "cannot classify" (CC). Both the unresolved and CC states of mind suggest the

presence of multiple contradictory representational states with respect to attachment and a breakdown in the organized strategies (dismissing or preoccupied) for managing the anxiety and affect aroused in the interview. Considered in terms of defenses, these may reflect the dissociative defenses characteristic of individuals organized at the borderline level (Liotti, 2004). Rebecca was in fact assigned a CC classification, with secondary classifications of dismissing/devaluing (D2) and preoccupied/angry and involved (E2), evidenced in the alternations between D2 and overwhelming, uncontained anger at attachment figures with whom she remained emotionally entangled.

These first associations to the request for adjectives was split-off material—loving and positive feelings about Rebecca's mother that were immediately replaced by negatively valenced words: *controlling*, *emotionally abandoning*, *confusing*, and *sad and weak bond*. Her elaboration on "weak bond" illustrates the discourse characteristics of dismissing speakers—in particular, the cool, offhand devaluation of those who tend to discount the significance of attachment-related memories and experiences that they have difficulty recalling.

> "I only say it's a weak bond because when you're asking me about remembering about my mother, it's difficult for me to remember . . . because I don't remember bonding with my mother. . . . I don't remember doing things with my mother that children do for fun. . . . You know being with my parents, I swear to God it was like being with a schoolmaster. That's what it feels like."

In the above passage, we see that the lack of recall serves to limit access to painful memories and experiences, but in Rebecca's case, these dismissing defenses quickly broke down. Interestingly enough she gave a similar adjective to describe her father: *nonexistent bond*. But although she began her elaboration of these words with dismissing mechanisms, she quite quickly reverted to a preoccupied state of mind characterized by overwhelming, involving anger and enmeshed, entangled states of mind as the following passage indicates.

> "I would say practically a nonexistent bond. I didn't feel a bond. I don't know, I didn't feel bonded to him. And um, it was such a weak bond, I swear to God, I can't even think of three other adjectives to describe it. Um, I don't remember my father. I swear to God I don't. I don't remember him. . . . I remember what my mother has told me but I don't remember him. . . . I remember he kind of joked around with us. Sort of, but—you know, especially with me, and I know that—I don't know if this makes sense, okay but—just tell me

> if it makes sense. Like, you know when you're a child you still don't know the relationship, you still—you—you, I don't know like the concept of dad and father. As a very small child, you don't understand what that means. And my father would like play with me, or tickle us, tickle me, whatever. And play with me. And I would run away scared, but I felt some sort of a sexual thing—I know it sounds weird, I know. But it is what I felt. And it's also, I know I felt this feeling of wanting to masturbate—this is the truth, and I told this to Dr. K. [former therapist] at a really really really really really early age. Um, so I felt this sort of sexual thing with my father, and I was scared of him."

The above passage begins with derogation, and continues with run-on sentences and violations of relevance as Rebecca becomes fearfully confused and overwhelmed by her conflicted feelings about her father. This passage also shows the tendency to get caught up in episodic memories and the disturbing feelings they evoke, such that she is unable to provide an objective or coherent elaboration of such memories at the semantic level.

One can also see how Rebecca shifts between two contradictory discourse strategies that are associated with two divergent states of mind with respect to attachment: a dismissing state that is linked to an internal working model of a controlling and intrusive other (the schoolmaster) and an overcontrolled, rejected self, and a preoccupied state that is linked to an internal working model of an overstimulating but neglecting object, and an excited, frightened self.

We see the origins, as well, of Rebecca's suicidal and parasuicidal behaviors in an unconscious identification with a powerful, destructive representation of a paternal figure. Such negative or destructive attachments—linked to wished-for and feared states related to early experiences of felt security, or lack of it in relation to early attachment figures—may be characterized by "a negative bond" that bespeaks an insecure and disorganized state of mind. This is evident in the following passage from the AAI, in which Rebecca talks about her father punishing her when she forged his signature on her report card.

> "I remember that he called me into his room . . . and I remember this—he was like, are you ever going to do it again? And he kept slapping me. And I was like hiding. And he just kept slapping me. And he kept saying—are you ever going to do it—What are you going to—if somebody keeps slapping you and beating you, and they say are you ever going to do it—you're not going to say yes. Of course, you're going to say no. That was—I—that was a bond. I

mean, I'm joking in a way, but that was a bond. But a very destructive bond. . . . It was a negative bond. Because you learn to fear that person."

Rebecca went on to talk about her fantasies that her father might have drowned her, stating, "I wish he had done it. I would have rather been like those children in China when they drowned them. Yeah. Because that's how he treats us." In the above passages, we see her preoccupation with traumatic or frightening events that overwhelmed her capacity for coherent "discussion of relationships required by the immediate discourse context" (Main & Goldwyn, 1998, p. 174). From an object relations point of view, this passage also shows her propensity to remain loyal to bad internal objects despite their destructiveness. Indeed, the origins of Rebecca's self-destructive and destructive actions may be seen in her unconscious identification with such a powerful yet hostile object. As Sandler (2003) put it, "we can speak of attachment between a person and a fantasy object just as we can speak of attachment to the real external objects in the person's life" (p. 16). However, perhaps attachment concepts alone cannot explain the origin or tenacity of negative bonds without reference to object relations understanding of the identification with such powerful, punitive internalized parental figures. Indeed, attachment theory and research have been critiqued for underestimating the role of anger and aggression in the formation and maintenance of attachment bonds—that is, the ways that aggression may be both reactive to trauma, loss and separation, and in its more modulated forms, "enlivening and vitalizing" (Eagle, 2003, p. 31).

Reflective Function Ratings

Not surprisingly, given Rebecca's representation of her father's intentional mental states in relation to her as aggressive and fear provoking ("I would run away scared"), her reflective functioning (RF) score on the AAI prior to the initiation of treatment was the lowest possible rating (–1), indicative of repudiation of reflection on mental states and included bizarre/inappropriate statements in the face of questions designed to elicit reflection. It should be noted that the RF Scale (RFS) ranges from –1 (negative RF in which interviews are antireflective, totally barren of mentalization, or grossly distorting of the mental states of self and others) to 9 (exceptional RF in which interviews show unusually complex, elaborate, or original reasoning about mental states; Fonagy et al., 1998). For example, Rebecca responded to the question "How do you think your childhood experiences have affected your adult personality?" by saying:

> "I mean, I think that if somebody had been in my room, put me to sleep, and assured me that there was nothing bad going to happen, I think a lot of this stuff would have gone alright, I don't think I would be talking about all of this. . . . You know, like even to the point that um—my dad and I, we went to a movie once . . . and it was a movie about this woman who was I'll tell you what it was about, actually, it was—it was a 1977 movie, and um, the woman that I remembered upset me a lot. And I was about, maybe 3rd or 4th grade then. It was about this woman—she's married, and she goes to this party, and um, her friend—and—and her husband doesn't come with her to the party. And her friend says, well you know, like this guy, he'll take me home, you know, so you [laughs] and so, the guy takes her home. He doesn't exactly take her home. He rapes her."

The above passage shows the confusion and disorganization that ensues when the interviewer attempts to initiate reflection so that one is "almost 'shocked' by the utterances" that are inexplicable and bizarre or inappropriate (Fonagy et al., 1998, p. 43).

It should be noted, however, that on the AAI, the first association to the request for adjectives was split-off material—about love—that did not emerge in the therapeutic dialogue directly for about 6 months. In a session that took place 6 months into the first year of therapy, Rebecca showed an evolution from the emotional unavailability of a narcissistic patient that is a sequelae of the grandiose self-organization to an increased capacity to work productively in the transference as the dominant self and object representations of the grandiose self–inferior therapist and defective self–superior therapist were alternately enacted and projected in the therapeutic interaction. It also shows the evolving capacity to reflect on the interaction and consider other perspectives even as, at times, she reverted to her hostile, controlling stance, both in fantasy and reality.

Attachment Dynamics in the Clinical Process

The following clinical material from a session 6 months into TFP-N reveals the moment-by-moment shifts in narrative organization and interaction between patient and therapist that show the impact of the disorganized (CC) attachment state of mind in the clinical situation.

In addition to applying the clinical lens of TFP-N, this session was rated with the Patient Attachment Coding System (PACS; Talia et al., 2017) to further clarify how Rebecca's state of mind with regard to attachment shaped the way she communicated in session, and

particularly the ways that she regulated her emotional proximity to the therapist. Recently, attachment researchers have catalyzed a move to the level of the relation (Talia, Daniel, Miller-Bottome, Brambilla, Miccoli, et al., 2014) in which they demonstrated that patients' states of mind with respect to attachment on the AAI are predictive of distinct relational patterns of interacting in sessions. The key realization that led to introducing the PACS is that discourse can be analyzed as a means of regulating emotional proximity—for example, when we disclose feeling hurt or we report a distressing narrative, this can be seen as serving an attachment function. When discourse is analyzed in this way—rather than for its form or content alone—it is possible to predict the recurrence of certain discourse patterns with distinct interpersonal functions based on patients' pretreatment AAI classification. Bowlby (1988) prefigured this current move to the level of relation that emphasizes the study of attachment dynamics in the therapeutic relationship and process itself when he wrote, "I want to emphasize that . . . the focus of therapy must always be on the interactions of the patient and therapist in the here and now . . . " (p. 141).

With the PACS, Talia and colleagues (Talia et al., 2014; Talia et al., 2017) have reliably identified in-session behavioral and relational correlates of the attachment classifications through a fine-grained analysis of in-session discourse in a relatively large sample of psychotherapy patients independently classified for attachment on the AAI (N = 160).

The PACS yields an overall classification of balanced (corresponding with secure–autonomous), resistant (corresponding with preoccupied), avoidant (corresponding with dismissing), or disorganized (corresponding to CC and unresolved) based on scales that rate the frequency of speech acts regulating proximity. Patients who are assigned a balanced classification use discourse markers that openly disclose vulnerability, take into account different perspectives, and share the impact that the therapist has on the patient. Patients who are assigned a resistant classification display high levels of direct resistance (e.g., overriding the therapists' interventions), enlisting agreement of the therapist, and/or merging (e.g., vague speech, sudden change of topic). Patients who are assigned an avoidant classification, finally, tend not to report narratives and internal states, instead minimizing or normalizing their feelings and evaluations.

The disorganized classification, which was recently developed by Talia, Taubner, Hauschild, Duschinsky, and Volkert (2018), is assigned based on the rating of six additional subscales: collapsing, freezing, disowning, disrupting, frightening, and isolating. Collapsing is evident when the patient completely fails to respond to the therapist's inquiry, forgets the question, and is unable to answer. Freezing is coded when the

patient describes a frightening experience without convincingly describing how it was resolved. Disowning is coded when the patient describes an objectively distressing event but does not state the impact it had on him or her, even in a downplaying or normalizing tone. Disrupting is coded when the patient actively refuses to collaborate with the therapist. Frightening is coded when the patient presents information or describes a situation without any regard for the feelings of the other people involved (e.g., describing inflicting harm on someone with a callous lack of empathy). Finally, isolating is assigned when the patient makes assumptions about other people's internal state without giving convincing reasons for those attributions.

Initially in the following session, Rebecca was angry and suicidal in anticipation of her therapist's vacation. The therapist suggested it may be humiliating to care so much about him when she felt he did not care about her. When coded with the PACS, the patient was assigned a disorganized classification of a dominant/resistant subtype (D/C1, which corresponds with CC/E2 on the AAI).

THERAPIST: But, everything you're saying today, I think, is geared to have an impact on your relationship with me. [*Directing attention to the transference.*]

REBECCA: Really? How? I don't know how. [PACS disrupting]

THERAPIST: Well, you probably don't because I don't think you're stopping to think about it. [*Encouraging reflection.*]

REBECCA: Well, I'd like to talk about something. [PACS disrupting] Why did you say that if I already have a therapist, why would I want to look for one? This is kind of like people in a group supporting me. [PACS involving; enlisting therapist] I don't really want it. I know what those people are like. [PACS involving; entitled criticism] You sit there and talk about your feelings. There's no way. Why am I supposed to come and say I hate people? I'm not going to say that. [PACS involving; enlisting therapist] You know I don't believe in group therapy, chatting with people. . . . You know, I mean I can never . . . I don't believe in group therapy, I don't believe in it. I don't believe in this support thing. I really don't.

THERAPIST: Well, let's put this to the side and talk about . . .

REBECCA: Well, I don't know. . . . Maybe you believe in it. My boss thought I was depressed . . . I didn't want to tell him I have a personality disorder.

THERAPIST: I feel it's important for me to point out . . .

REBECCA: How do you know I'm not just depressed? [PACS disrupting]

THERAPIST: I think what you're doing here is kind of eliminating me more and more, in different ways telling me I'm not important, that I'm not helpful. . . . [*Addressing the overall pattern of interaction of grandiose self–inadequate therapist; narcissistic transference*.]

REBECCA: You're not. . . . You're not helpful. You don't hear what I'm saying. [PACS disrupting] [*Dismissing devaluation of therapist and example of role reversal*.]

THERAPIST: I'd like to try to understand why you have escalated that position so much, because in the past 2 days . . . [*Calls attention to the preoccupied, hyperactivating dynamics; e.g., suicidality, exaggerated expressions of anger and distress*.]

REBECCA: It's not related to your going away, it has nothing to do with that. [PACS disrupting] [*The patient denies any connection between feelings about therapist's vacation and her suicidality; dismissal and deactivation of attachment-related feelings in the face of separation*.]

THERAPIST: But . . .

REBECCA: It might have a little bit . . . but yesterday I was drawing a picture at work: I drew a gun and I drew like a head, and that's what I feel like doing. [PACS frightening] [*Angry resistance expressed through suicidality*.]

THERAPIST: I know. Every which way, you're sort of waving flags at me telling me that you're worse, that I'm useless, that I'm not helping you, that this is useless . . . [*Calls attention to the overall pattern of dismissing devaluation*.]

REBECCA: Well . . .

THERAPIST: And you're interrupting me incessantly. So, I think it's a combination of rage at me that I am not providing you with the kind of total caregiving that you wish for, and my going away underlines that. [*Interpreting a defended-against underlying dyad—the wish for the perfect caregiver*.]

REBECCA: I was suicidal before I knew you were going away. I was suicidal that day when I was at my cousin's birthday. [PACS frightening] [*Dismissing devaluation of the importance of the relationship*.]

THERAPIST: Well, you can continue to reject everything I say but it won't give . . .

REBECCA: Well, then stop relating everything to your going away, cause I'm suicidal even when you're here. [PACS disrupting]

THERAPIST: If you'd just look at today's session and yesterday, and do a graph, it's gone up like this. . . . [*Therapist gestures with his hand*.]

REBECCA: What?

THERAPIST: Your talk about suicide.

REBECCA: Yeah, so I talked about it before your going away. [PACS direct resistance] I felt suicidal that day when I felt bad about my cousin. [PACS frightening]

THERAPIST: It's important to make the point—whether you hear it or not is up to you—because you're continuing to push me away. [*Calls attention to the dismissal of emotional connection with therapist.*]

REBECCA: I'm hearing everything you say. It just isn't helping me.

THERAPIST: This is a real dramatic reaction to feeling like you're getting attached to someone, and this is apparently what happens when that process begins. [*Beginning to interpret the motivation for staying in the negative transference; i.e., beginning to interpret the deeper longings for connection; attention to the competing deactivating and hyperactivating dynamics in the face of the growing attachment to the therapist.*]

REBECCA: Why should I swallow what you're saying? [PACS disrupting; angry resistance]

THERAPIST: It's for you to decide what to do with it. But you seem to be rejecting it, without even thinking about it. [*Calls attention to dismissing devaluation.*]

REBECCA: Well, you don't know me, and you come up with this stupid theory and I'm supposed to swallow it. You don't know me, why should I? Especially when you don't know me. You say the same thing to every one of your patients. I'm not like all of them. [PACS disrupting]

THERAPIST: I think that sentence . . . is another reflection of your feeling hurt, your feeling like you're not getting the kind of caring that you'd like, that that type of caring is impossible. You feel like you're one of an assembly line. [*Continuing to interpret the motivation for avoiding the positive transference; therapist repeated a phrase patient had used earlier in the session.*]

REBECCA [with sudden change of affect, from angry to sad]: I don't feel that I deserve to be here . . . I don't know. [*Patient covers her face with her hands.*] I just feel badly that I have to walk around with other human beings. [PACS proximity seeking; present relational wishes and unmet needs] I just don't feel like. . . . ? [*Collapse of the grandiose self; rapid shift from devaluation of therapist to negative valuation of self; role reversal in the transference.*]

THERAPIST: I think that's more to the point. . . . Why is it hard for you to show your face when you say that?

REBECCA: I don't know what else to do. I just don't want you to see me. [PACS proximity seeking; present interpersonal wants]

THERAPIST: I think you don't want me to see you in the longing that you feel. You don't mind if I see you in your anger and your rejection. You don't want me to see the longing, because you think I'll just use that to humiliate you, by rejecting, by turning away from you. [*Continuing to interpret the motivation for avoiding the positive transference; addresses fears of dependency; paranoid transference.*]

REBECCA: And how does that translate into how I take care of myself? I didn't take a bath, I didn't brush my teeth, I spend thousands of dollars on my teeth and you know, and how is it translated into. . . . [PACS disrupting]

THERAPIST: It translates into how you *don't* take care of yourself. [*Interpreting how the patient presents herself shabbily as a way to feel she's in control of the rejection she anticipates, and provokes.*]

REBECCA: Yeah . . . I just feel like the tragedy of everything, of all of this, is that I have help available, I have people like my boss . . . he's really caring . . . but he doesn't understand anything. [PACS involving; entitled criticism] He felt that everything is overanalyzed . . . we analyze everything. And I kind of disagree with it because this is not, this is what I feel, maybe I should just try, maybe I'm just saying it and I don't know—maybe it *is* an analytical thing, but this therapy is different from other therapy—it's not just insight oriented. You're actually working with me, with tangible things, things that I've told of that actually happened. . . . My boss just thinks I'm depressed. He's wrong. It has nothing to do with being depressed. He sees me laughing and joking at the office, and he says, "Oh, I see you talking to the other people," and, you know, I say, "You just see the outside. I feel that these people I'm talking to really hate me." [PACS involving chronicle or a narrative that serves to enlist the listener's agreement with the patient's stance] [*Shift from devaluation to idealization; patient shows gratitude toward the therapist; shows emergence of epistemic trust.*]

THERAPIST: Your whole tone changed here a few minutes ago. Did you notice that? [*Bringing the focus to the here and now.*]

REBECCA: Yes.

THERAPIST: How did you understand that?

REBECCA: You mean, now I'm talking with more of a dialogue?

THERAPIST: Yes, less angry.

REBECCA: Well, in the waiting room, my fantasy was that the room had all your patients, and that you came out and said "Hi" to me and I just ignored you and then you said, "Didn't you hear me say 'Hi'?" And then I said, "Yes," and I was just very cold to you and then everyone else was staring at me, like angry with me and I just stared right back at them, I didn't care. And then one person said, "That's not nice to do of you," and I told them to shut up. [PACS isolating] [*A defensive return to the negative, angry feelings, but with greater reflection indicated in the capacity to articulate the fantasy of humiliating the therapist rather than acting it out.*]

THERAPIST: So, your fantasy was . . .

REBECCA : . . . humiliating you. [*Role reversal.*]

THERAPIST : . . . humiliating me.

REBECCA: I want that fantasy to happen. [PACS frightening]

THERAPIST: Why would you want to humiliate me?

REBECCA: Why not? [PACS disrupting] I was humiliated, why shouldn't I humiliate you? [PACS involving; enlisting therapist] I have to deal with it all my life, every single day. . . . It's true.

THERAPIST: I think we should look at something a little closer to home right now, I think my going away may be the humiliation you're experiencing right now. [*Bringing the focus to the here and now.*]

REBECCA: No . . . yes, I guess there's a longing in a way, because I did come today on time, I didn't really want to come, in a way, but I do long to come here, in a way, I guess I do. [PACS contact maintaining] And it's not the same thing between seeing Dr. Jones [the colleague who covered for the therapist when he was away] and you. I am more attached to you than I was to him. He was, but it's not the same. It just isn't. It's who you are, I like you more. . . . I remember this boy in high school. There weren't many students from my background there. . . . I was one of the only ones. I used to look at him. I never thought he'd speak to me. One day . . . [*Here patient is able to bring in genuinely affect-laden material from the past; acknowledges erotic transference.*]

In the above session material, we see the way that insecure/disorganized attachment representational states are associated with different ways of regulating affect and emotional closeness or distance

to the therapist. In this case, the therapist may hear reverberations of (1) the constricted, idealizing, or devaluing state of mind of the narcissistic patient that creates an arid wall shielding the patient from pain that cannot be experienced or articulated, necessitating clarification and exploration of affective experience; (2) the confused and conflicted state of mind of the preoccupied patient who shows angry resistance, necessitating both containment and confrontation; and (3) the fragmented state of mind of the disorganized person who may become fearful and dissociated as he or she recollects loss or trauma in ways that compromise reflective and cognitive capacities, and defends against this with hostile, controlling interpersonal behaviors. Rebecca was observed to ricochet among the different discourse strategies.

With the PACS, the primary classification was disorganized (dominant) and resistant (angry/conflicted; J. Bate, personal communication, June 18, 2020). The two disorganized subscales that were rated most highly were disruption and frightening. These markers suggest an interpersonal process characterized by the patient's active refusal to collaborate with the therapist.[1] Markers associated with the frightening subscale are seen in the patient's fantasy of humiliating the therapist, and in the descriptions of suicidality alongside a reluctance to explore the motivations for her suicidality. Throughout, preoccupied strategies were evident in the angry resistance to accepting help, alternating with maximizing distress through the suicidal ideation related to the therapist's upcoming vacation. These alternations between devaluing/derogating/disrupting and preoccupied strategies also mirrored fluctuations between grandiose and vulnerable self-states (e.g., her desire to humiliate the therapist while feeling humiliated by him), deactivation of attachment, and hyperactivation of attachment, seeking emotional connection with the therapist and recoiling from it.

However, there is also a revelatory moment in the middle of the session, when the therapist repeatedly interprets the patient's "hurt" and feeling like she is "one of an assembly line" (J. Bate, personal communication, June 18, 2020). Rebecca's affect suddenly changes and she covers her face, which could be seen as avoiding or withdrawing, but she actually makes a proximity-seeking utterance, stating, "I just feel badly that I have to walk around with other human beings." She also goes on to

[1]Markers associated with the disruptive subscale were seen in material that traditionally might have been considered pervasive, dismissing devaluation of the therapist and the treatment. However, rather than functioning to protect the patient's image/representation of the parent, these markers suggest an interpersonal process characterized by the patient's active refusal to collaborate with the therapist.

say, "I just don't want you to see me." In these two lines we see a brief glimpse of secure communication, where Rebecca is able to articulate her relational difficulties and her interpersonal wants in the present relationship with the therapist, albeit stated in the negative.

This case example illustrates how attachment classification offers a route to understanding the multiple, often contradictory, internal working models of self and significant figures in more severely disturbed narcissistic patients. The attachment classifications, which identify discourse and narrative characteristics of different attachment representational states, do not represent static or fixed entities, but rather alert the clinician to dynamic ways of regulating affect and attunement in interpersonal relationships, as well as to the defensive processes as they manifest themselves and fluctuate in the moment-to-moment flow of transferences and resistances in the dialogical clinical situation (Eagle, 2003; Slade, 2008; Talia et al., 2017).

The shifts in attachment representational states just described were also manifested in the fluctuations in transference dispositions. In terms of the evolution of transferences, there are manifestations of a narcissistic transference in which the patient's grandiose state of mind sometimes eclipses the reality of the therapist and his interventions, which are systematically and at times scathingly devalued; a paranoid transference evident in Rebecca's fears that the therapist will harm her by misdiagnosing her or not helping her; a dependent transference indicated in the sense of self as inadequate and needy, and longing for help from a caring therapist; a depressive transference in her sense of self as worthy, but guilty and remorseful for having wanted to humiliate the therapist who she acknowledges has helped her significantly; and finally, an emergent erotic transference evident in the awakening of desire and longing in relation to a former wished-for object who also valued and noticed her, a proxy for the therapist.

Toward the end of the session, a nascent secure state of mind in relation to the therapist emerges, evident in Rebecca's acknowledgment of her attachment to the therapist, in her gratitude toward him, and her expression of positive feelings about his commitment to and concern for her. She states that the therapy is "not just insight oriented" but that he is "actually working" with her in the here and now of the relationship. Along with the diminution of negative (paranoid and narcissistic) transferences and the emergence of dependent and depressive transference themes, we see an increase in reflectiveness about mental states in her statements that the therapist understands and is attuned to her and that he is a reliable interpreter of her subjectivity ("You're actually working with me"). Although Rebecca fantasizes about humiliating the therapist in front of his other patients, she also sees him as a trustworthy

touchstone for receiving and learning "from the social knowledge that has the potential to change one's perception of oneself and the social world" (Fonagy et al., 2017b, p. 9)—the basis of epistemic trust. Indeed, at the end of the session, she recollects a pivotal experience of being liked, chosen, noticed, and understood, and even desired.

The dialogue between Rebecca and the therapist also shows the beginning of the dissolution of the grandiose self, as the component aspects of it emerge in the transference (i.e., superior self–humiliated therapist, and devalued self–rejecting therapist). This collapse is evident in the session in her turning away and hiding her face from the therapist as she shifts the focus from criticizing the therapist for not helping her to talking about her painful self-loathing reflected in lack of self-care, and the tragedy of her inability to accept the help that is offered. Here she may also be expressing a deep sense of shame and remorse for how she has treated the therapist, showing the beginning of the depressive anxieties around guilt and responsibility for destructiveness toward valued others. However, the session material also indicates how the rigid organization around the dyad of grandiose self–devalued other renders the interpretation of the process by which the patient alternately identifies with the self or object poles of the object relations dyad more difficult and protracted. This is because as the grandiose self becomes less effective as a defensive structure, underlying anxieties about fragmentation, annihilation, and dependency emerge.

As the disparate self–object dyads associated with grandiose and vulnerable self-states are evoked and interpreted in the transference, the patient's capacity to reflect on and contain these anxieties begins to improve. This was evident in the diminution of the PACS markers of resisting and disrupting, which shows Rebecca actively blocking collaboration in the session. She presented with suicidal ideation and initially resisted any exploration of its meaning. The therapist first clarified how this ideation might have been fueled by a powerful split-off affect (anger at the therapist, who is perceived as uncaring for going away) that she initially brought up by negation but then denied. The therapist then worked with Rebecca to clarify and reflect on these feelings, linking them with the emergence of dependent longings in relation to the therapist that she had difficulty acknowledging. In this process, Rebecca recognized that the therapist might have a different state of mind separate from her attributions—this promoted differentiation of her own mental states from that of the therapist.

The recognition that Rebecca's experience of self and others was shaped in part by myriad, disparate mental models of self in relation to others in her mind was catalyzed in part by the lived experience of role reversals in the transference. In this session, the therapist interpreted the

process by which Rebecca alternately identified with the self and object poles of a specific object relation by projecting or enacting each pole (i.e., moving from feeling humiliated to trying to humiliate the therapist). Recognizing role reversals both tempered her grandiosity and fostered reflection by helping her to recognize the symbolic and constructed nature of the transference and how it replicated aspects of her own internal world. In addition, Rebecca showed an increased capacity to access the split-off loving feelings linked to a positive, if idealized, object relational dyad of cared-for child and gratifying attachment figures that she had initially disowned.

In the session above, this opposing dyad emerges in Rebecca's recollection of a wished-for connection with an idealized figure from her past—a boy she desired but felt was out of her reach, but who finally noticed and valued her. The full emergence of idealized and persecutory object relations in the transference (i.e., he is the longed-for but unattainable fantasy figure from the past and also the person who treats her as one on an assembly line) functions as a regression on the way to integration (Kernberg, 2007; LaFarge, 1997). Such micro-regressions in patients with severe narcissistic pathology may be related to the deepening of the transference as the patient begins to experience real dependency, gratitude, and fears of losing the therapist. Subsequently, more modulated, realistic positive representations emerged of the therapist as one who was a genuinely concerned, caring, if imperfect, figure. The presence of secure markers in the PACS speaks to this point. In particular, after Rebecca articulated a fantasy that elaborated one of her dominant object relations scenarios of humiliator/humiliated, the therapist brought her back to the here and now of the upcoming separation, prompting her to express her deep longing and positive feelings toward the therapist. This is noted as contact maintaining in the PACS.

It is interesting to note that in line with the session, the patient's responses on the AAI pre- to post-TFP-N shifted from a disorganized (CC) attachment state of mind with chaotic alternation between insecure attachment strategies (dismissing and preoccupied) to an organized, if still insecure, attachment classification (preoccupied). At the end of treatment on the AAI there was no longer the chaotic alternations between cool devaluation of, and angry preoccupation with, early attachment figures, or between lack of memory for early attachment experiences and a tendency to be caught up in and overwhelmed by traumatic memories. Rather, although Rebecca still presented with involving anger at attachment figures, she was able to articulate her negative feelings in a more organized way without defensively retreating from them through lack of recall or defensive devaluation. In addition, her

propensity to become disorganized in the face of past malignant experiences was diminished. The movement from dismissing to preoccupied attachment states of mind has been noted by Gerber (2005) to be an important intermediate stance on the trajectory toward attachment security in therapy with severely disturbed patients with personality disorder. The shift from dismissing to preoccupied and from disorganized to organized attachment status may involve an increased capacity to tolerate and express both positive and negative feelings in a more coherent or organized manner, and a greater capacity to bear ambivalence without defensive turning attention away from attachment-related memories and relationships.

There are indications that the identification with the therapist and an internalization of his interpretive, reflective capacity (the analytic third) formed the bridge to changes toward improved organization. Indeed, Rebecca's responses on the AAI reveal an identification with the therapist. She referred many times to now being treated as and seeing others as "three-dimensional" rather than "two-dimensional," the terminology that the therapist had used earlier in his interpretation of the effect of her use of omnipotent control. Both reflective function or mentalization in TFP-N and attachment security are thought to be enhanced through the building up, modulation, and integration of disparate, opposing object relational dyads as they emerge in the transference. Rebecca's capacity to reflect flexibly and accurately on her own mind and the minds of others at 1 year is evident not only in her shift to organized, if still insecure, attachment but in the more dramatic improvement in her RF rating from –1 (active repudiation of RF) to 6 (above-average understanding of mental states), as the following passage from her AAI after 1 year of TFP-N indicates.

> "I've learned that the world is not as hostile as I thought it was. I've learned that people are not two-dimensional, they're three-dimensional, and they want to be treated the way I want to be treated three-dimensionally. I've learned that um . . . that's a huge one that I've learned the world isn't as hostile, um, even though this is a bad week coming in. Um, I've learned that . . . people don't necessarily abandon you. . . . People can be mad with you for a while, but that doesn't mean that they're going to abandon you . . . it's possible for people to be mad with you and still like you."

This passage shows an improvement in a number of aspects of mentalization, including the recognition that mental states evolve and change over the course of time, that mental states are separate from

overt behavior and even momentary shifts in affect states ("it's possible for people to be mad with you and still like you"), as well as a clear differentiation between Rebecca's own mental states and those of others.

In sum, understanding the discourse characteristics associated with different attachment states of mind and levels of RF may help the working clinician to understand the dynamic processes and representational states that may underlie the often drastic shifts between grandiosity and vulnerability in narcissistic patients as they ebb and flow in the dialogical clinical interaction. This is particularly important with more severely disturbed narcissistic patients whose attachment representations are likely to be multiple and contradictory, and to change gradually toward increased security over the course of intensive therapy, such as TFP-N. Improvements in RF may function as a mechanism of change toward the reorganization of mental representations of attachment. Indeed, improvements in RF may precede or accompany changes from disorganized to organized or insecure to secure attachment (Diamond & Hörz-Sagstetter, 2012; Levy et al., 2006). It should be noted that after 1 year, Rebecca went back to work and began to develop more enduring friendships and intimate relationships. As was the case for most individuals in the research study of which Rebecca was a participant, there were significant improvements in personality organization, psychosocial functioning, impulsive aggression, and symptomatology, including self-destructive symptomatology after 1 year of TFP-N (Clarkin et al., 2007). As part of her initial contract, Rebecca volunteered in a lawyer's office and was quickly offered a regular job as a paralegal. She began to date and although her initial relationships were somewhat problematic because they were based on idealization, she eventually developed an enduring, loving relationship.

In sum, in TFP-N the emphasis is on transference interpretation and countertransference use as a conduit to understanding and mobilizing the patient's split, polarized mental representations, including attachment representations and associated affects. These aspects of the treatment are thought to counterbalance the detachment and emotional disengagement, and angry resistance that impede the therapeutic process of those with pathological narcissism and NPD. With its focus on face-to-face communication in the here and now, TFP-N activates the attachment system in controlled ways that allow for the mobilization and reworking of insecure/disorganized attachment states of mind as they emerge as transference dispositions—a process that promotes mentalization. The latter is crucial since neurocognitive studies have shown that when the attachment system is kindled, mentalization may be compromised or even disabled (Mayes, 2006; Fonagy, Bateman, & Luyten, 2012). TFP-N may actually be effective in improving mentalization and attachment

security, because it supports the containment of and reflection on affective experience through the identification of dyads that underlie them in the context of structure and limit setting (the contract). This in turn allows for the cognitive appraisal of affect states as mental states that evolve and shift over time, as well as the capacity to distinguish one's own mental states from those of others, including the therapist.

Summary and Conclusion

With its consistent interpretation of split, manifold, rapidly shifting transferences that develop in the course of therapy, TFP-N may provide an effective treatment for patients with NPD/BPD. As the case of Rebecca illustrates, these individuals often present initially with a combination of contemptuous superiority and painful self-loathing and will almost immediately project their internal confusion and conflict about self and others in the transference (Kernberg, 2018; Rosenfeld, 1987). Often inordinately hypersensitive to threat or criticism, containment of the maelstrom of conflictual feelings and representations is necessary before the transference can be approached productively. The identification and clarification of the dominant object relation in the transference "in the projection," enables the patient to tolerate and observe negative feelings in the transference that he or she must initially project, enact, or disown in order to fend off feelings of being humiliated or attacked. Subsequently, interpretation of role reversals, whereby the patient experiences him- or herself as the internal object that had previously been projected onto the therapist, while retaining an identification with the self-pole of the object relation, fosters integration and tempers grandiosity, as concepts of both self and object that had been split off and denied are alternately enacted and projected. This also paves the way for the identification of positive or idealized aspects of the relationship based both on good, benign aspects of the therapeutic relation, as well as on defenses against negative aspects. The interpretation of these disparate transferences that represent facets of the fragile, if rigid, grandiose self, as they emerge in the lived emotional reality of the interaction with the therapist, is particularly crucial for the treatment of more disturbed narcissistic patients with comorbid NPD/BPD.

Attachment theory and research have offered an expanded lens that increases our understanding of the varied transferences and the contradictory representational states of severely disturbed narcissistic individuals. At a conscious level, the dismissing attachment state of mind involves a representation of self as omnipotent and self-sufficient in relation to another who is either an idealized admired complement or devalued

repository of unworthiness and inferiority. At an unconscious level, the individual may harbor a sense of self as unworthy and vulnerable, and the other as rejecting. In the case of individuals with disorganized (CC or unresolved for loss and trauma) attachment status, the person generally alternates between dismissing derogation of the therapist and frail idealization. The person alternates between an attitude of superiority, with the therapist seen as an inadequate figure who must be humiliated, and a pervasive sense of inferiority in relation to the idealized, flawless therapist before whom he or she feels a sense of abject humiliation, as did Rebecca in the clinical examples above.

Therapists who are able to provide a secure base for the exploration of internal and external reality are those who recognize and understand the dynamic processes—that is, modes of defense and affect regulation—behind the patient's attachment state(s) of mind as they emerge in the dyadic clinical situation and adjust their interventions accordingly. This involves identifying and amplifying affects in dismissing or disrupting states of mind; confronting angry resistance and demands for merger in preoccupied states; containing fearful or frightening mental behaviors in disorganized states that may be manifest only fleetingly in words, affect, or behavior; and recognizing and reinforcing proximity-seeking and contact-maintaining strategies in secure states. These involve understanding these attachment states of mind as mental states that function to impede or advance the therapeutic dialogue (Steele & Steele, 2008; Slade, 2008; Talia et al., 2017).

In sum, in this chapter we attempted to demonstrate how attachment theory and research helps to inform the treatment of patients with NPD/BPD by illuminating the variants of insecure and disorganized attachment and deficits in mentalization that underlie grandiose and vulnerable self-states that characterize this group. This in turn sheds light on the heterogeneity among these patients, potentially identifying common developmental risk factors, and helping us to recognize how these manifest themselves in the fluctuating affects, defenses, transferences, and resistances of those with pathological narcissism and NPD.

PART III

PATHOLOGICAL NARCISSISM IN INTIMATE RELATIONSHIPS AND SOCIETY

CHAPTER 10

Malignant Narcissism

Clinical and Social Aspects

The syndrome of malignant narcissism, introduced in prior chapters, has been described in earlier work on pathological narcissism and NPD (Kernberg, 1992, 1994). Recently more interest has developed in this variant of narcissistic pathology (Skodol, Bender, & Morey, 2014; Ronningstam, 2014; Stone, 2016). Malignant narcissism is the most severe clinical syndrome within the general spectrum of pathological narcissism and may constitute the furthest end of the personality spectrum that is amenable to modification and treatment; it is prognostically and therapeutically the most challenging. The syndrome is defined by the presence of a typical narcissistic personality structure that, in addition, includes (1) intense paranoid characterological features, (2) ego-syntonic aggression against others and/or self with an emphasis on defeating others, and (3) antisocial behavior.

Despite these common features, individuals with malignant narcissism may present with variable levels of clinical severity, with some who are so severely disturbed, impulsive, and dangerous to self and others as to require hospitalization, and others who function relatively well in work, social, and relational life. Two factors determine the difference between these two groups. The first is the extent to which the pathological grandiose self is infused with aggression; the higher the level of aggression in the personality, the more likely the individual will develop malignant narcissism. The second is the extent to which the pathological grandiose self permits an integration of a kind of ego functioning. In other words, where the pathological grandiose self is sufficiently stable, where it functions well as a pseudo- or false self, the individual with

malignant narcissism may perform adequately in work and relationships, although the latter are usually exploitative and conflictual in nature. In addition, individuals with malignant narcissism may be either thick- or thin-skinned in their presentation. For thick-skinned malignant narcissists, aggression often infiltrates the pathological grandiose self, but not to the extent that it interferes with good overall control that allows them to function well. Thin-skinned malignant narcissists, on the other hand, are usually functioning on an overt borderline level with generalized splitting and lack of control of affect, reacting with chaotic and impulsive aggression to perceived ego threat, often leading to hospitalization in the more severe cases. So, the degree of aggression in these patients varies from dangerous to just clinically evident but controlled. In sum, the extent to which the pathological grandiose self is infiltrated with aggression versus the extent to which it just replaces a normal integrated self determines the level of severity.

The most severely disturbed individuals with malignant narcissism may require treatment in inpatient psychiatric settings. In fact, the high prevalence of patients with this syndrome in inpatient settings constitutes a significant challenge to hospital staff. In addition, with the decrease in availability of long-term inpatient treatment, these patients have become the greatest challenge in outpatient settings. The level of aggression and antisocial features with which they present often require extra parameters in the treatment frame, such as the ability to communicate with third parties to ensure the accuracy of information provided by the patient and to ensure their safety. At this level of severity, patients present with a significant failure in their capacity for work, for intimacy in their sexual life, and for maintaining ordinary social relations. They often tend to be confused with regressed patients presenting with BPD, but the combination of (1) the dominant features of the narcissistic personality, (2) the severe paranoid tendency, and (3) marked aggression, in combination with (4) antisocial behavior, permits the specific diagnosis of the syndrome. This is in contrast to patients with more traditional borderline disorder whose destructive and self-destructive actions are manifestations of unintegrated hostility and aggression that present as part of the confusing array of affects typical of those with a more straightforward core identity diffusion. One might argue that the confusion between patients who have malignant narcissism and patients with more straightforward BPD is one of the factors behind the unfortunate stigma attached to the borderline diagnosis.

At the other extreme—a socially salient polarity of this syndrome—we find individuals who function much better, maintaining the capacity for ordinary social functioning, at least at the surface, and relations with others, although the need to dominate and control

others severely limits intimacy and mutuality. They maintain sufficient control of their paranoid and antisocial tendencies to present a semblance of social adaptation. In these individuals, narcissistic grandiosity and a sense of superiority is often rationalized as an identification with extreme ideologies, including those with prejudices against a specific social minority or out-group. These individuals concentrate their paranoid tendencies in an intense hatred of what is defined as other. Their aggressive behavior is rationalized by an ideology that justifies aggression against the supposedly threatening group. In other words, at this level we find individuals who gravitate toward social subgroups or political movements that provide a sanctioned outlet for paranoid, aggressive, and antisocial behavior directed against a designated out-group. The knowledge that we have of some historical figures with these characteristics would point, for example, to Hitler's and Stalin's personalities as presenting the typical combination of narcissistic grandiosity and superiority, severely paranoid features, antisocial behavior in terms of unmitigated aggression and cruelty, both overt and covert, while maintaining, as much as possible, a benevolent, paternalistic, and patriotic veneer to disguise their true intentions (Kernberg, 2003a, 2003b). In considering this manifestation of malignant narcissism, it becomes clear that this psychological constellation can present a grave danger to society.

In comparison to the extremes of the syndrome of malignant narcissism, we find that narcissistic personalities at an intermediate level of functioning—that is, with the characteristic symptoms of NPD—have a relatively contained aggression in terms of brutality, usually limited to intimate relations, and antisocial behavior limited to betrayal and the undermining of assumed enemies. In keeping with the prominence of antisocial features, the interpersonal world of the malignant narcissist seems to consist of individuals to be used and exploited and/or to be feared. *All* patients with the syndrome of malignant narcissism show a more or less subtle arrogance and lack of tactfulness in social relations that isolates them and evokes negative reactions from others that reinforces paranoid and related antisocial behaviors. Jacobson (1971) first pointed out the intrinsic relation between paranoid tendencies and the urge to betray as a manifestation of aggression.

Their grandiosity and superiority, combined with hypersensitivity to any criticism, occasionally collapses with intense feelings of inferiority, worthlessness, and bitter resentment against the world that has plunged them to this depth of self-devaluation. They make enormous efforts to find culprits outside of their own personality functioning. When not threatened, they may present a surface of security and self-assertion that, in the higher-functioning range, can impress those in their social environment. These individuals sometimes demonstrate a

charm and seductiveness that masks their underlying lack of capacity for deeper relationships, their sense of entitlement, and their ruthless manipulation of others.

The ego-syntonic presence of antisocial behavior may present in two general forms. The "passive–parasitic" polarity of antisocial behavior takes the form of lying, cheating, irresponsibility with money, stealing, and emotional exploitation of others. The "active–aggressive" pole displays inordinate aggression in social behavior, destruction of property, and even assault of others related to narcissistic frustrations or hypersensitivity to criticism. At the most severe level of this syndrome, aggression may take on more primitive characteristics, such as extreme temper tantrums, occasional social violence, and, very significantly, self-directed aggression when frustrated in their need to control and manipulate their environment. In fact, self-directed aggression may become an important feature of the most severe level of this psychopathology and can contribute to the potential diagnostic confusion with patients who are ordinary borderline because of chronic suicidal and parasuicidal tendencies.

There is an important difference, however, between chronic suicidal and parasuicidal behavior of patients with malignant narcissism and similar behaviors in those with BPD. In the latter group, self-destructiveness can be a reaction to extreme emotions, such as rage or despair, an attempt to self-soothe, and/or an action geared to have an impact on others. Patients with the syndrome of malignant narcissism utilize self-destructive, self-mutilating, and suicidal behavior as an assertion of their control over their environment and their sense of superiority. They are not afraid of pain, illness, or death. In fact, in their fantasy, the potential for suicide represents an implicit control over their existence, and if the world is not able to gratify their needs, they are willing to abandon it rather than submit to humiliating limitations or exposure. A subgroup of these patients uses severe chronic self-mutilation as a defiant control of their social environment—an ongoing expression of superiority and revenge against all those who are trying to help them. At a deeper level, it is an expression of envy of all those who do not share the patient's terrible self-experiences. In the treatment setting, this dynamic can also lead to a negative therapeutic reaction based on envy: As the treatment advances with initial apparent improvement, the patient may begin to do worse, harm him- or herself, or even commit suicide as a reaction to envy of the therapist's capacity to help and an ultimate expression of who is stronger and on top.

From a psychodynamic viewpoint, the pathological grandiose self, typical for the structure of pathological narcissism, has been failing to protect the patient against the effects of deep sources of primitive aggression. An aggressive infiltration of the pathological grandiose self

transforms his or her ordinary sense of superiority into an experience of superiority guaranteed by power over others and self, and so the expression of aggression toward others or self maintains an omnipotent control over reality. The projection of primitive, sadistic superego components in the form of paranoid trends contributes to the deterioration of internalized ethical values and fosters antisocial behavior (Jacobson, 1971). These dynamics are typical of rare, extremely severe self-mutilators who may cut fingers or puncture their eyes or produce such lesions in their bodies that medical interventions are required and, in some cases, insufficient.

Clinical illustrations of the most severe manifestation of this syndrome include a hospital patient who was using rat poison to induce internal hemorrhages with life-threatening consequences. Repeated searches in the hospital would fail to discover all the sources and hiding places where this patient strategically hid his supplies of rat poison. This treatment ended with a transfer to internal medicine, where further complications caused by his behavior required major medical interventions. The ultimate destiny of that patient is not known to us. Another case involved a woman who cut her forearm in deep parallel cuts, then, repeating these cuts in a perpendicular direction across the original ones, produced an extended quadriculated pattern of self-mutilation deeply cutting into muscle and nerve branches, to the extent that a reparative surgery was no longer possible. A third patient managed to gradually lose all the fingers of one hand because of self-cutting maneuvers. Such behaviors are not limited to hospitalized patients and indeed may escape detection for long periods of time, as in the case of a physician's assistant nurse who was injecting herself with insulin, and would pass out while on the job, detected only when she was observed doing so on a video camera in the office. In each of these cases, what was most dramatic was the patient's quiet self-assurance and superior demeanor between the self-mutilating episodes, and the denial of any concern over his or her behavior in the periods of apparent "normal" functioning.

Some of these patients may be helped by TFP with refinements to treat narcissistic pathology (TFP-N). In contrast to the less severe NPD patients, in which the pathological grandiose self protects the individual from a conscious experience of aggression, this defensive function fails in the case of malignant narcissism. The pathological grandiose self cannot prevent the individual from feeling under environmental challenge and attack. The intensity of his or her aggressive impulses; the intolerance of the negative, directive, prohibitive aspects of superego development; and the lack of integration of prohibitions that are experienced as sadistic all lead to both projection of aggression and its infiltration into the very structure of the pathological grandiose self. This leads to

a search for power, and the potential for the expression of violence as a guarantee for maintaining the patient's superiority. This aggressive infiltration of the pathological grandiose self also explains the relatively crude and nonsubtle aspects of the challenging arrogance, and the generally negativistic behavior of these patients, lacking, except in certain higher-level cases, the apparent charm of the higher-functioning narcissistic personalities.

The better-functioning patients usually do not come to treatment, except if some specific symptom militates against their sense of superiority and draws them into the search for help. These personalities easily become leaders of extreme political movements, organizations, or political parties, and the particular social environment permits them to rationalize their pathology to fit a paranoid social orientation. The more severe cases experience intolerable tension that may motivate their seeking treatment. In some cases, family members refer the patient to treatment. Obviously, these narcissistic patients, functioning at the severe level of the pathology, with total breakdown in the various areas of personal functioning, do need therapeutic intervention.

As in all cases, treatment has to provide an adequate, firm therapeutic frame that protects the family, the patient, the social environment, and the therapist and the treatment itself from destructive acting out. If such conditions can be achieved, psychotherapeutic help is possible. If these conditions cannot be achieved, the most meaningful therapeutic help may be work with family and others close to the patient to minimize the harm done to them by the patient's condition.

As the transference evolves, individuals with pathological narcissism show a remarkable and quite characteristic frailty of the pathological grandiose self. From sessions in which they tout their superiority, question the therapist's capacity, and devalue the treatment, they may shift into moments of insecurity and despair, desperately asking for help and understanding. It is important to differentiate such honest moments from periods of pretense, a need for help and support masking antisocial behavior or manipulations of the therapist. The therapist has to be able to be both distrustful and yet open to authentic wishes for understanding, for helpful human contact at moments of despair, and the falling apart of the pathological grandiose self.

To a large extent, these developments correspond to what has been described as the "thin-skinned" narcissistic transference (Rosenfeld, 1987). Yet the patient with malignant narcissism can react with aggressive affects and behaviors that are not typical of thin-skinned narcissists who are less severely disturbed. The oscillations between devaluative superiority and desperate clinging may, in turn, alternate with periods of frankly paranoid developments, even micropsychotic paranoid episodes

in the treatment. Patients who are prone to betray friends by whom they feel disappointed may "betray" the therapist, whom, at the same time, they desperately need. The patient may set up hostile family reactions against the therapist outside the sessions, while pretending in session to be on the side of the therapist, defending the treatment against the hostile family. This is a frequent manifestation of combined paranoid and antisocial behavior.

A particularly complex area is the treatment of these patients' sexual difficulties. With some frequency, these patients present a total inhibition of their sexual life,[1] lack of capacity and interest to masturbate, lack of any sensual excitability of their skin, lack of erotic response to any type of erotic stimuli; in short, they exhibit a primary inhibition of sexual life not derived from repressive mechanisms but related to the overwhelming predominance of negative affective systems. Individuals with malignant narcissistic traits may engage in sexual activities only for secondary gain—for example, sexual exploitation or prostitution. Or, if they have attained positions of power in organizations or the media, may actually become sexual predators in the workplace, demanding sexual favors in return for career advancement and enlisting those who work for them to aid and abet these behaviors often kept secret for many years.

These patients, therefore, experience enormous difficulties engaging in intimate sexual relations, which represent one more area of potential inferiority and humiliation that they have to deny by transforming their incapacity into one more area of superiority. These patients, in a traditional patriarchal culture, may express their superiority by not being involved in any "sinful" activity, utilizing religious ideologies to justify their moral superiority in this regard. Practically, it may take a long time before this sexual inhibition is overcome as part of the therapeutic progress. The activation of their sexual life, to begin with, may typically be expressed as sexual fantasies and impulses with severely sadomasochistic features. In fact, however, the appearance of such sadomasochistic features in the emerging sexual life of patients with malignant narcissism may signal the beginning of working through their conflicts around severe aggression and indicate the beginning of "freeing" their sexual life from the dominance of an aggression that did not allow any contact.

Other patients, in contrast, with full development of their capacity for erotic responses, may show a typical dissociation between a severe incapacity of emotional tenderness and depth on the one hand, and

[1]Hull, Clarkin, and Yeomans (1993) did a study of 100 inpatients with BPD and found that 50% had never engaged in sexual activity with another person. Those with malignant narcissism are likely to meet criteria for BPD as well.

impulsively driven sexual activity on the other hand. Here, the expression of their exploitive and paranoid features interferes with the development of more mature integration of erotic and tender impulses, and may be linked, in the exploration of the transference, to the profound condensation of preoedipal and oedipal conflicts that emerge once an authentic world of object relations is reactivated in the transference. This evolution is described more fully in Chapters 7 and 8. The important issue here is the enormous difficulty these patients experience in tolerating the exploration of their world of unconscious fantasy, which can be hidden behind inhibition, as well as acting out of sexual impulses.

To date, the research on malignant narcissism as a clinical syndrome is limited, but social psychologists have identified a comparable variant in their findings that some individuals with grandiose narcissism are also characterized by the dark triad of psychopathy, Machiavellianism, and paranoia (Miller et al., 2010). Such individuals are driven to gain power and control over others through callous manipulation and vengefulness. In prior work, researchers at the PDI have demonstrated that malignant narcissism is associated with lower overall global functioning and slower rates of clinical improvement in TFP (Lenzenweger, Clarkin, Caligor, Cain, & Kernberg, 2018).

Social Implications of Malignant Narcissism

A final concern regarding the social implications of the syndrome of malignant narcissism involves the emergence of socially dangerous political leadership. These considerations extend beyond the therapeutic realm of intervention but may expand the clinical understanding of pathological narcissism to the field of social psychology and social theory. The problem resides in the extent to which individuals with high aspiration for social leadership, who, at the same time, present a severe personality disorder with the features of malignant narcissism, represent a danger to the destiny of the social groups with which they engage. High intelligence, high ambition, and a sense of unusual self-assuredness and superiority, combined with an intense search for social dominance to compensate for an internal world rendered empty by the force of internal aggressive objects, may push such individuals into positions of leadership. In such a position, they show an inordinate need to be both loved and obeyed. Any disagreement or challenge of their views would be perceived as a personal attack and cannot be tolerated.

Volkan (2014) has distinguished between narcissistic leaders of the benign and malignant types. Benign narcissistic leaders encourage group members to identify with and "reflect the leader's shining self-image and be extensions of his or her superiority" (p. 221). They focus on

"chosen glories"—that is, past positive events or group characteristics that enhance self-esteem and bind disparate members of the group into a shared purpose, rather than reactivating "chosen traumas" that might further fuel the group's xenophobia and vengefulness. The ordinary or benign narcissistic leader has an inordinate need for love and approval but can tolerate different views because his or her grandiosity does not depend on a fear of attack presented by alternative positions. In contrast, malignant narcissistic leaders insist upon submissive "yes men" in their immediate circle of followers, and an implicit dictatorship of ideas and persecution of critical voices will follow. Leaders characterized by malignant narcissism foster large group regressions in which ordinary citizens are made to experience past injuries, injustices, and defeats as devalued aspects of the self. To redress such injuries, individuals are conscripted as part of a mass to control, persecute, or even eliminate out-groups (Kernberg, 2003b; Volkan, 2009, 2014). The self-esteem of the leader, who often exists in a "glass bubble," removed from in-depth contact with followers, is preserved through repeated aggressive attacks and often trumped-up triumphs that serve to assuage narcissistic injuries at the individual and group levels.

Although Stern (1979) does not use the term *malignant narcissism*, he is one of the first psychoanalysts and social theorists to link violence with the rise of pathological narcissism in our society. He writes, "the narcissist is a violent man. . . . He uses his power to manipulate others and makes them subservient to him . . . he is not encumbered by a sense of justice . . . he believes he is entitled to whatever he can get" (p. 155). Speaking to the relationship between malignant narcissistic leaders and their followers, Stern warned that "Now there is not enough interdependence to maintain the structure of functioning society" and that "this has left us unprotected from the violence of asocial narcissists" (p. 194).

Malignant narcissistic leaders catalyze primitive narcissistic mechanisms in their followers through demanding the submersion of individual identity in that of the group, the division of both the indigenous and neighboring groups into highly polarized good and evil factions, the undermining of intrafamilial bonds in the interests of group loyalty, and the formation of ideologies that valorize and distort the historical features of the group (Volkan, 2009). Under such social conditions, even ordinary citizens without significant narcissistic pathology remain vulnerable to regression to a core of unmetabolized primitive aggression, primitive (sadomasochistic) object relations, and primitive defenses. These are activated through identifications with narcissistic leaders of the malignant type who promise resolution of individual and historical humiliations through the persecution of devalued out-groups. In such situations, individuals follow the leader, who may "resemble the follower and appear as his enlargement" (Adorno, 1951/1987, p. 127).

Malignant narcissistic leaders accomplish this by mythologizing aspects of the group's history, by excavating hallowed narratives of the group's past defeats and triumphs, and by fostering mechanisms of introjection (of idealized images) and projection (of devalued images) that render group members susceptible to uncritically assimilate political propaganda that demonizes enemies or devalued out-groups. Narcissistic regression in groups under the sway of malignant narcissistic leaders is also characterized by a focus on borders that protect against out-groups depicted as predatory or subhuman, justifying their expulsion or persecution.

This, in fact, is the atmosphere of totalitarian regimes in contrast to ordinary dictatorships. In the case of small social organizations, the overdependency of the organization on the judgment of and submission to the respective leader may become extremely destructive. When the grandiosity of the leader fails for one reason or another, the paranoid, aggressive, and antisocial features of the leader may become more severe and will usually result in an aggressive attack directed toward the outside world. Now the totalitarian leader may authorize socially sanctioned violence against depreciated social subgroups. If the leader's policies fail, his or her moments of despair will coincide with increased paranoid outbursts against the "enemy." In the worst-case scenario, defeat unavoidably may lead to the leader's suicide and/or a final extreme burst of violence as part of an effort to destroy everything else with him or her—a final effort to hold on to the control of his or her destiny. Various features of these mechanisms can be observed in the history of the major totalitarian regimes that originated national and international conflicts in the 20th century.

For the clinician, the syndrome of malignant narcissism represents the challenge of the outer limits of treatability of the structure of severe personality disorders. As mentioned earlier, that limit still excludes, at the present day, the antisocial personality proper (Hare, 1999; Kernberg, 1984, 1992; Stone, 2003).

Malignant Narcissism with Comorbid Borderline Personality Organization

Where narcissistic pathology coexists with borderline organization, the grandiose self can be so brittle that it does not tolerate sustained contact with external reality. Patients who are more thick-skinned at this level demonstrate chronic and ruthless exploitation of others for adulation, while patients who are more thin-skinned tend toward rigid withdrawal from interpersonal relations altogether, which often masks intense involvement with powerful, destructive internal objects. For patients

with malignant narcissism, the narcissistic organization is characterized not only by identifications with idealized representations of self and others but also with omnipotent, destructive parts of the self that are not well differentiated from such powerful internal figures. These destructive aspects of the self shore up their sense of superiority and are directed against any part of the self that experiences the need for or desire to depend on others.

More than other narcissistic patients organized at a borderline level, those with malignant narcissism need to keep others permanently devalued in order to ward off aspects of self that cause pain and anxiety, and this severely limits the extent to which they can engage in genuine, authentic relationships (Kernberg, 1975, 2007, 2010). There is sometimes an attitude of cool indifference to others, so that the omnipotent destructive aspects of self are not immediately evident. However, the experience of others as worthless, punitive, or fearsome is shaped by the persistence of harsh, primitive superego precursors that are systematically projected onto others so that they experience the world as populated by enemies; if their grandiosity is not reflected back to them, the person who fails to reflect it is the enemy. These patients are more likely to experience the therapist's interventions as shaming them or being used to attack them. For more disturbed narcissistic patients operating at a borderline level in the realm of malignant narcissism, interpretations that challenge the omnipotent fictive narrative that is the patient's reality may lead to a hatred of reality itself and an attack on "the mental apparatus as the instrument with which to grasp it" (Etchegoyen, 1999, p. 764; Bion, 1959).

Such individuals are more likely than are higher-functioning narcissistic patients, who are more aloof and detached, to develop paranoid transferences and to experience paranoid regressions in the transference. For such patients, the submission to powerful, punitive internal objects eclipses the possibility of any meaningful link with another. In the treatment situation, this manifests itself in terms of attacks on the frame, the language of the therapist, and every relationship, internal or external, that in any way threatens the patient's omnipotence—all of which disrupts the possibility of productive work. The most severe manifestation of NPD at the borderline level is the syndrome of malignant narcissism, described above. These characteristics can involve a destructiveness to self or others that can include suicide as a form of empowerment of self and triumph over others.

While the prognosis of patients with malignant narcissism is guarded, many individuals organized at the borderline level have been treated successfully with TFP-N. In order for treatment of such patients to be successful, the grandiose self of the patient, and particularly its aggressively laden self and object representations that are not

well differentiated, must be experienced, clarified, and interpreted in the transference. Many of these patients have histories of abuse, either physical and/or sexual, often of a perverse sort that involves a simultaneous elevation and debasement of the patient. With these more disturbed narcissistic patients, the focus on the here-and-now relationship tends to activate an insecure and/or disorganized attachment system in which early attachment relationships have evoked feelings of fright without solution, leading to the shutting down of metacognitive monitoring and reflectiveness about self and relationships. This is because the primary attachment figures, often the first touchstone of reality about self and others, have either in fantasy or actuality become the source of fear and abuse (Hesse & Main, 2000). In such situations, where the patient has a limited capacity to mentalize or think about aspects of his or her behavior and feelings in terms of intentional mental states, the therapist must put some emphasis in the interpretive process on these deficits as well as on understanding the omnipotent destructive aspects of the self and object world, and the disorganized internal working models of attachment as they are mobilized and enacted in the transference (Fonagy et al., 2002; Hesse & Main, 2000; Diamond, Clarkin, Levine, Levy, et al., 1999).

We illustrate these points with an overview of the interpretive process in the treatment of a patient we call Michael, a patient with the syndrome of malignant narcissism with comorbid BPD.

Case Example: Michael

As introduced in Chapter 1, Michael was a married White male in his mid-20s who presented for therapy with the following problems: chronic self-harm, suicide attempts, substance abuse, impulsive aggression, and antisocial features (lying and cheating) since early adolescence. These problems had led to several inpatient hospitalizations and a number of failed outpatient treatments. The fact that Michael's problems were presented as behaviors rather than problems with affect, such as depression or anxiety, reflected his poor capacity for being in touch with his internal states.

Brief Developmental and Family History

Michael was the older of two siblings by 2 years. His father was a successful executive in the entertainment business, although he had an alcohol problem that occasionally interfered with his work, as he would go on drinking binges. When Michael was 5 years old, his younger brother died in a car accident. His mother, who had enjoyed acting in community

theater, sustained serious injuries and became severely depressed. After the accident, she was at times bedridden for reasons that seemed both physical and psychological. She was unable to work subsequently and depended on her husband, and especially her son, for caregiving. Frustrated and embittered by her own injuries, the mother was alternately seductive and envious of her son, at times projecting her own thwarted ambitions onto him, and then when he succeeded, cutting him down. She would expose herself to Michael and invite him to share her bed, at one point masturbating beside him. He also witnessed his father masturbating while watching pornography on several occasions during his adolescence. The father managed to maintain a successful career as an executive in the entertainment industry despite excessive drinking, but traveled extensively and left Michael to care for his mother. When home, the father supported Michael in developing his talents as an artist, musician, and athlete, taking him to games and playing sports with him. But he was sometimes physically abusive to Michael. In addition, Michael witnessed his father killing his pets, at one point watching him strangling his pet birds because their chirping bothered him, depriving Michael of soothing sounds. Perhaps in response to this, Michael developed an interest in animals and volunteered at an animal shelter, reflecting a capacity for positive investment in others. In those with malignant narcissism, there is often a total devaluation of one of the parents, with an ambivalent idealization and paranoid fear of the other one. Such was the case with Michael, who loathed his mother and had contempt for her as weak and inadequate, while he idealized his father as powerful and fearsome.

Michael showed talent as an artist and attended a high school for students gifted in art and music. However, when he entered early adolescence (age 14) he began to self-harm. His self-injury began after an incident of physical abuse where, in the midst of an argument, his father pummeled him with his fists while calling him vile names. Subsequently, Michael began to injure himself in increasingly serious ways: lacerating his arms, legs, and abdomen; and making several suicide attempts (overdosing on over-the-counter medication) that led to several hospitalizations. He also began to lash out impulsively at teammates and coaches, on several occasions physically attacking another player on his basketball team who he believed had made a foul play and gotten away with it.

Michael's self-injury was motivated in part by his wish that the parental abuse would come to light. He also engaged in antisocial activities, like cheating on tests, lying about his grades to please his exacting father, and, at one point, stealing sports equipment even though he had a generous allowance. While he excelled academically in school, he did not develop close friendships, as he anticipated criticism from and

rejection by his peers. He won a scholarship to a prestigious art and design college, but he dropped out during the first year after a suicide attempt precipitated by failing an exam. He returned to college where he met his future wife, an aspiring actress, whose father was a well-known art dealer. She initially saw Michael as talented and charismatic, excusing his erratic behavior as part of an artistic temperament. But his drinking and self-destructiveness led to a brief hospitalization, after which he dropped out of college again. He married at age 23, hoping that his wife would support him and that her father would help promote his career. His wife wanted to have a child, but he felt he was too disturbed to have a family. He continued to paint and compose and had entry-level jobs in several art galleries, but was fired because of his entitled attitude, difficulty accepting direction or criticism, and drinking and substance abuse on the job. His wife supported the couple through her earnings as an actress in a daytime television series, believing that a big break might be around the corner for him since he had had some minor commercial success as an artist and had exhibited in several group shows. Michael was episodically unfaithful to his wife, who, although aware of his infidelities, remained devoted to him. When she expressed her distress, he would become verbally abusive and threaten to leave her.

Brief Treatment History

Michael had been in a series of outpatient therapies since age 5, the most recent of which ended when he began to stalk the therapist, calling her at all hours, stalking her online, writing numerous e-mails with links to his art exhibits, and then becoming enraged when she attempted to limit such extra-session contact. At one point he followed her from her office to her home. The therapist's attempts to set limits on this behavior led to a suicide attempt and a short-term hospitalization. Another previous treatment had ended after he attempted to kiss his therapist at the end of a session.

The confluence between grandiosity, aggression, paranoia, and antisocial features in this patient indicated the syndrome of malignant narcissism. His behavior represented an internalized object relational scenario in which the patient was both victim and abuser (e.g., cutting himself) that was infused with both aggression and libidinal feelings, so that he experienced the self-injury as a highly gratifying "secret pleasure."

Michael was referred for TFP-N at age 24 after a hospitalization for a serious overdose after which the therapist he had been seeing in supportive psychodynamic therapy refused to take him back. The treatment contract included the standard items of regular attendance and reporting

thoughts and feelings freely, with additional responsibilities of attending Alcoholics Anonymous (AA), reporting to the emergency room if he felt he could not contain his destructive and/or self-destructive urges, limiting extra-session contacts with the therapist, and engaging in a productive activity in the form of taking some college courses and doing volunteer work. As is often the case, the setting up of the treatment contract activated an early transference. Michael experienced his new therapist as cold and uncaring, largely because he limited contact outside of sessions in a way that his previous therapist had not. During this initial phase of therapy, Michael's clinical course was tumultuous, with ongoing urges to self-injure, on which he acted on two occasions, but at a level that did not require hospitalization. For the most part, however, he accepted and followed the treatment contract.

Following the contract included 2 occasions, during the first 2 months of therapy, when Michael reported to the emergency room because of suicidal ideation. The first time, he was assessed as not presenting an immediate risk. The second time, he was admitted to the hospital. He stabilized quickly there and was discharged after 3 days. He seemed indifferent to his therapist's attempts to explore what feelings motivated the visits to the emergency room, attributing them to missing his former therapist and the availability she had offered him between sessions. He was not only distrustful but also fearful of his new therapist, whom he saw as uncaring and potentially only interested in him as a subject in a research study with whom he could try out new therapy techniques. The therapist encouraged Michael to express these feelings rather than enacting them, reminding him that his previous treatments, which did not include a contractual agreement about how to manage self-destructive and destructive behaviors, had not been successful in alleviating his symptoms or affording him the space to reflect on what was motivating these behaviors. After this tempestuous beginning, Michael settled into therapy and, while there were milder episodes of acting out (superficial cutting), his flagrant self-destructiveness came under control and he became increasingly involved in the therapy as his relationship with the therapist deepened and his reactions to his therapist became more varied.

In the following section, we focus on the early and midstages of TFP-N with Michael to illustrate how we address the narcissistic organization in a patient with features of malignant narcissism.

Defining the Dominant Object Relations

Not surprisingly, given Michael's history and level of disturbance, the experience of others as potentially harmful and of himself as a potential

victim was quite quickly actualized in one of the first contract-setting sessions.

MICHAEL: I have hidden from people . . . people are dangerous.

THERAPIST: That does sound like the typical way you experience others, and I'm not surprised it's coming up at the beginning of therapy.

MICHAEL: My former therapist gave me her home number for emergencies but then I kept calling her and she referred me to you.

THERAPIST: What was your reaction to that?

MICHAEL: Well, I think that in a way I feel rejected and it makes me angry and suicidal, but being treated nicely makes me feel encouraged, and then it just gets confusing. . . . I mean not that I want to be treated not nicely, but you know what I mean.

THERAPIST: I imagine that is going to be an issue here as well. You see me as dangerous and resent my setting limits, but also fear that things could get out of control here, as they did with your previous therapist when you began to stalk her. Some of the conditions of treatment involve limiting some of the behaviors that led to the end of your last therapy, like frequent calling between sessions unless it's an emergency. I'm wondering if you have feelings about that.

MICHAEL: Feeling miserable . . . like to die.

THERAPIST: That does sound like a chronic feeling in your experience, particularly when you feel like people you rely on aren't totally available to you.

The therapist explored with Michael how, by making such a dramatic statement, he highlighted the depth of his despair and disorganization, but also expressed how powerful he felt when he was suicidal, which accounted for why he said, "like to die." He then told the therapist that when he was suicidal, he would feel his hands twitching, which made him feel both empowered and out of control.

Not surprisingly, the experience of the therapist as a potentially fearful and dangerous object emerged early in the therapy in a session where Michael talked about his parents' lack of concern for him. They had recently told him that they would be spending 6 months in Asia for his father's work and would not be able to see him during that time; nor did they want him to visit them.

THERAPIST: Where do you think I'd fall on the "concerned versus not concerned" spectrum?

MICHAEL: Well, you are probably about where my parents are.

THERAPIST: Your parents who you have said won't visit you?

MICHAEL: Well, my dad recently said to me, "Why don't you just jump off the Empire State Building, 'cause then that would work."

THERAPIST: So, you imagine that I too would just stand by indifferently while you injure yourself, or worse, that I would be a malevolent figure who wished you harm, and you a helpless victim.

The above session material indicates that an object relational pattern of a cold, abusive, rejecting parental figure and an imperiled, abused self infused with feelings of rage and fear was actualized in the transference early in the therapy. This is in contrast to the typical narcissistic transference in higher-functioning narcissistic patients where the idealized–devalued dyad predominates, and the patient is detached and unavailable. The therapist focused on Michael's immediate experience of himself and of their interaction in the moment. The focus on the immediate experience in contrast to a focus on the patient's history reflects our view that a patient's internal representations are generally not literal representations of people in the patient's life but images modified (distorted or exaggerated) by unconscious processes.

During this period, the major interventions involved naming Michael's raw and overwhelming affects and inchoate bodily sensations (e.g., hands twitching), linking them to his experience of the therapist as untrustworthy and potentially dangerous. This represented the first stage in the interpretive process—interpreting in the projection—that emphasized helping Michael to put his subjective experience of himself in relation to the therapist into words, and to connect them with the visceral bodily sensations he experienced. It did not yet link to an overall object relation pattern that shaped his contradictory behaviors and feelings. Interpretations in the projection are designed to provide containment of chaotically experienced affect states and to verbally identify what is in the patient's mind without yet tracing it back to the patient's internal world since the patient may not yet be ready to tolerate that awareness.

In addition, in the first months of therapy, Michael also verbalized, in a coy way that made it hard to find the line between teasing and serious thoughts, impulses to harm the therapist: "not doing anything gory, but perhaps strangling him." In an early session, he said, "I got it [destructiveness and self-destructiveness] out of my system for a while with that last overdose, so *you'll* [italics added] be safe for 6 months or so." At times he would feel as if his hands were twitching but stated that he was not thinking about hurting the therapist (as he sometimes had felt in the past when he felt his hands twitching), but about hurting

himself. Previously when his hands felt this way, he had been thinking about hurting the therapist, too. The therapist commented that perhaps Michael was feeling confused about where the danger was in the room—and where the fear should be. The therapist wondered whether his hands twitching represented his wish to render the therapist helpless or even injure him, so that he could remain powerful and in control of his therapy and life, even at the expense of continuing to cut or injure himself. He also commented that the shift from wanting to hurt the therapist to wanting to hurt himself showed some sense that the therapist would not harm him, although he continued to harbor fears of such.

It should be noted that the therapist's ability to openly discuss these negative feelings and the way they manifest themselves in self-destructive and destructive fantasies and actions is essential in the treatment of patients with malignant narcissism. To avoid or downplay the negative transference would communicate the therapist's discomfort with negative and aggressive affects and difficulty to contain it and reflect on it. In fact, the capacity to accurately identify and understand one's own bodily sensations and to link them with intense emotions is the first step toward mentalization or the capacity to define subjective experience and to differentiate it from that of others, including the therapist.

Subsequently, Michael talked about his fears that the therapist was critical of him, and connected these fears with his overwhelming feelings of shame and humiliation. Not only was he not moving toward his long-term goal of becoming an artist but he was still injuring himself and verbally abusing his wife, behaviors that he acknowledged brought him a sense of satisfaction and empowerment. This led to the exploration of how, when he injures himself or aggresses against others, a sense of self as supreme and in control and powerful takes over (e.g., his hands twitching). However, the origins of this self-experience in his identification with powerful, punitive figures was only fully understood and explored when it was enacted in a role reversal whereby the patient assumed an omnipotent, teasing, and aggressive stance vis-à-vis the therapist and cast him in the role of the helpless victim.

Role Reversals in the Transference

Three months into therapy, after the brief hospitalization mentioned above, Michael talked in a superficial, indifferent manner about the suicidal urge that had led to him going to the emergency room. The therapist wondered whether the suicidal feelings might not be linked to the desire to maintain control of the therapy by not revealing more and thereby titrating the closeness to the therapist. When Michael continued

in his detached stance, the therapist began to be concerned that there was something that was being split off and not being directly discussed in the therapy. Shortly thereafter, Michael announced that he had started an affair with a woman he met in AA, a performance artist who he found more entertaining and captivating than his "boring" wife. The therapist saw this as a challenge not only to his marriage but also to the therapy in that starting an affair shortly after beginning a new treatment often functions to put up a barrier to the emotional intensity and the dyadic intimacy of the therapeutic situation (Symington, 1993).

Michael talked with great excitement about how his new lover showed him her routine that involved ceremonial play with knives that she used in one of her acts and told him that they had toyed with the idea of carrying out a murder–suicide pact in the therapist's waiting room. He described the idea with a level of excitement bordering on glee. The therapist understood this as stemming from Michael's sense of triumph and empowerment that he could make a mockery of the therapist's efforts to help him. This was combined with a sense of gratification in rendering the therapist hurt, humiliated, and even potentially physically harmed. In his explorations of the material, which Michael conveyed as somewhere between a fantasy and a plan, the therapist reflected on the confluence of aggression, eroticism, destructiveness, and pleasure that fueled the idea, and on which part of the picture to bring into the therapeutic dialogue first. The therapist, who was understandably quite alarmed, decided to try to bring Michael's attention to his combination of superficial indifference and underlying teasing, and to his provocative tone that somewhat contradicted the seriousness of his verbal content. In response, Michael protested that the therapist was blowing the discussion out of proportion and admonished him for not understanding that what he was discussing was "all play." At this point, the therapist's anxiety increased; he did not know what to believe anymore and felt that Michael was toying with him, and felt like a mouse to Michael's cat. He shared this thought with the patient and brought attention to his affect, asking how he felt about how the therapist was communicating with him in this moment. His response was to say, tersely, "I don't know what you mean. I'm just telling you the facts." The therapist said that he might be wrong, but that Michael seemed to be finding pleasure in communicating with him in a way that might very well make him anxious and fearful, that might make him squirm. This situation would reverse Michael's sense of being helpless and rejected in relation to another who is experienced as rejecting or uncaring and this reversal could include a certain pleasure in being aggressive.

Here we see the confluence of ego-syntonic aggression, paranoia, antisocial features, and vengefulness that constitutes the grandiose self

of individuals with malignant narcissism. At this point, the dominant object relation shaping the transference was sadomasochistic in nature in that it was organized around themes of power, control, dominance, and submission, with an absence of guilt or remorse on Michael's part.

The exploration of this incident involved naming and linking a series of affects—aggression and pleasure—that Michael had not previously connected, and placing them in the context of this threat of a violent enactment. The therapist observed that it might not be uncommon for people to have negative, angry, or even sadistic feelings from time to time, but he was struck by the difficulty and discomfort Michael had in associating any such feeling with himself, even though they seemed present in his actions. Michael continued to protest that the therapist was accusing him of being sadistic and attacking him but, after some exploration, acknowledged that he did take some gratification in provoking the therapist. This provocative behavior toward the therapist appeared to re-create an object relation of a tantalizing but frustrating parental figure and a victimized, overstimulated self. Increasing Michael's awareness of the anger, hostility, and desire implicit in the dominant concepts of self and other which were actualized in the transference represented a turning point in the therapy. It is noteworthy that after this exploration of a specific enactment of his aggression, Michael did not engage in any overtly destructive or self-destructive actions, such as cutting himself or attempting suicide. He began to attend sessions like clockwork and invest in his treatment, which allowed for further exploration and gradual integration of his internal world.

As the therapist continued to reflect on the situation, he saw an additional level of the enactment. Along with the expression of a sadistic aggression, the therapist saw Michael's enactment as an effort to mobilize him to take care of Michael. It brought into relief both Michael's representation of himself as a neglected, traumatized child in relation to an overstimulating but neglectful caregiver who would stand by and watch him injure himself, and also the increasingly evident self-representation of the avenging angel, referred to above in the discussion of his sadistic side. Challenged to deal with the situation in a way that maintained safety while it advanced understanding, the therapist bypassed therapeutic neutrality to contain the situation by advising Michael to end the destructive relationship, since it was incompatible with their goals of containing and exploring his motivations for acting out, and to open up about the boundary-violating affair in his AA meeting. Although at first Michael was angry and resentful, he did end the relationship. After taking this action, he became very anxious, recognizing that he had in fact behaved in a way that could potentially victimize the therapist just as he had been victimized himself.

In a follow-up session that confirmed the aggressive charge involved, a few weeks after this incident, Michael told the therapist that, after telling the performance artist that he was breaking up with her, he had feared that she would "go after" his therapist in retaliation. Only later, after considerable therapeutic exploration, was Michael able to express distress about how the incident might have affected the therapist and compromised his safety. In this incident, the therapist was able to interpret in an experience-near way how object relational dyads may defensively reverse polarities within themselves, so that while he experienced himself as a victim, he enacted his split-off identification with a tantalizing, cruel other in the murder–suicide plan as it was imagined in the therapist's waiting room. This episode epitomizes how the struggle to control the therapy through threats of aggression or violence represented not only the actualization of a split, polarized internal world but also compromised emotion regulation and empathic ability that contributed to such vengefulness (Ronningstam, 2016).

In severely disturbed patients with a history of physical and sexual abuse, the split between idealized and persecutory aspects of the representational world may take a particular form. In addition to a representation of a victimized self in relation to an abusive other, there is also often a representation of an overstimulated self in relation to a tantalizing other (Fairbairn, 1952) that is sometimes dissociated. Often such dissociated representations emerge in the transference as enactments (although rarely as dramatic as the one described above). These may then be understood, integrated, and mentalized—that is, the experience of an overstimulated self in relation to a persecutory and/or tantalizing, frustrating object that fuels such enactments may be understood as shifting mental states and henceforth will not need to be expressed so readily in actions that may be irreversible.

In this material, we see that Michael realized that some of his mental states regarding the therapist corresponded to states that he had previously perceived in the therapist. This involves the recognition that aspects that he attributed to the object (rejecting, tantalizing) actually represented aspects of the self and his own internal world. At this stage, awareness of the alternation or interchange of roles within the same object relational constellation is crucial. The therapeutic task is to help the patient to understand and tolerate the double identification with self and object poles of the object relations dyads that comprise the internal world. This is done through interpretation of the repetitions and reversals of specific object relational dyads as they emerge in the transferential relationship. The repetitions of the dyad generally move from actions to more symbolic enactments. But, as is the case with patients with narcissistic pathology, particularly with malignant features, change comes slowly.

The Process of Integrating the Persecutory and Idealized Object Relational Dyads That Defend against Each Other

In more severely disturbed narcissistic patients, the persecutory sector of experience may predominate even until the midphases of treatment and defend against other, split-off positive or libidinal object relational dyads. The active dyad on the surface is often of a tormented patient–victim fearful of a therapist perpetrator that may be used to defensively dissociate experiencing the therapist as a caring, concerned individual in relation to a cared-for self, even after the treatment appears to be progressing. In Michael's case, a few months after the interaction described above and at a time when the therapy seemed to be progressing and his life settling down, he started a session by saying, "I have a box cutter in my bag and haven't decided if I should hurt you today." Considering the essential rule of having to work in safe conditions, but also considering Michael's propensity to provocative teasing, the therapist said, "I can't tell from your casual tone of voice if this is real or you are trying to provoke me. If you have a box cutter in your bag, we'll have to end the session and you'll have to leave." Michael took offense and reacted angrily: "This is proof you don't trust me! How could you think I really came with a box cutter? It was just a joke! There's no point in continuing this therapy if you don't trust me!"

In this interaction, we see Michael's playing with aggression as opposed to more direct acting out, although again there is a role reversal in that he is expressing hostility, but then accusing the therapist of not trusting him. This incident reflects the type of negative therapeutic reaction that can be seen in narcissistic patients in general and especially in those with malignant narcissism. When the therapy begins to go well, the patient may experience the therapist's capacity to help as a sign of his or her competence or even superiority, which may evoke intolerable feelings of vulnerability in the patient. To reestablish his sense of superiority and control, Michael attacked the therapist in two ways: first, by reporting he had a box cutter, which could have posed a clear danger to the therapist and the therapy; and second, by accusing the therapist of not trusting him when the therapist's actions to ensure the safety of both of them (i.e., forestalling the murder–suicide pact in the previous example and questioning him about the box cutter in this situation) constituted the only logical response to the situation. The therapist saw this interaction as an indication of the actualization of a dyad where the patient perceived himself as a hapless victim in relation to a punitive therapist, which evoked dysregulating levels of wrath and aggression, and then worked to reverse the situation by attacking the therapist.

In individuals with malignant narcissism, the self may be seen as including a malignant traumatizing other that cannot yet be symbolically represented or mentalized but must be repeatedly externalized onto others in the interest of self-stabilization. Fonagy and colleagues (2002) have called such a bad object representation an "alien self" that is unconnected to the "structures of the constitutional self" (p. 358). While Fonagy and colleagues (Fonagy & Allison, 2014) see this "alien self" as not being part of the self but rather as the colonization of the self by an external element, we see this internal representation as based on the internalization of traumatic experiences that ultimately leave a residue of intense aggressive affects in the self. Commenting on the link between narcissistic organization and traumatic experiences, Symington (1993) has observed, "one of the ways of managing trauma, a sudden onrush of stimulation, is the narcissistic option. To distance himself from what is happening, the person enters the narcissistic way of being . . . in the grandiose state, he or she is able to push away those things that are painful, is able to dispel parts of himself into others, and lives anesthetized against whatever the painful thing might be . . ." (p. 74).

In Michael, we see the traumatized individual achieving mastery through omnipotent control of the other, which both reflects and disowns identification with the traumatizing other. This alien object relation is split off and activated not just as an alien self but as an alien self relating to an alien object or bad internal object. Thus, transference evokes not just an alien self but an alien object relation with a constituent self and self-representations controlled and distorted by severely negative affect. In our view, it is typical for patients with severe narcissistic and borderline disorders to alternately behave as the alien self, or see the therapist as an alien self; whether they enact it or project it, they are in a complementary role. Through our approach, which fosters the bringing together of split-off experiences, the alien self and its corresponding objects are transformed into self and object representations that can be integrated with their opposite. This permits the modulation of affect, and the strengthening of cognitive functions, including the beginning of reflective function. In work with those with malignant narcissism, there may then be long periods in which the therapist must accept, tolerate, and contain the patient's projection of negative feelings and the representations with which they are linked in order to detoxify and ameliorate them. This enables the dismantling of such a pathological grandiose self into constellations of idealized and persecutory aspects, and paves the way for an expanded capacity to tolerate feelings, anxieties, and conflicts linked to more varied object relational scenarios, such as those of a dependent, erotic, and depressive nature.

Subsequently, the work focused on interpreting the splitting of the more apparent aggressive aspects of self from dissociated libidinal longings for connection that are defensively dissociated in the patient's initial presentation because they evoke fears of dependency, helplessness, weakness, or exploitation. The integration and modulation of Michael's own anger and aggression, rooted in his identification with abusing others, then cleared the way for the emergence of more positive, libidinally charged, if still extreme, experiences of the self in relation to the therapist. The integration of split-off defended-against longings for connection with a caring other, previously seen as persecutory, leads to an expanded understanding of the mind of the other, as well as his or her own mind. It enables the patient to recognize that his or her mind is representational in nature—that is, that the experience of self and others is shaped in part by myriad mental models of self in relation to others. Michael's attempt to control the threatening or seductive other led to an awareness of threatening and seductive elements in himself, which he was expressing through the murder–suicide scenario or box-cutter incident. This new awareness of his identification with both poles of the initial persecutory and frustrating representations of self and others, as well as his later recognition of the dependent longings this defended against, enabled him to understand them as mental states in self and others.

In the following dialogue, we see the emergence of an array of positively and negatively charged representations of self and significant others, with a lessening of identifications with omnipotent powerful internal objects, although such identifications persist in somewhat attenuated form.

MICHAEL: Can I ask you a question that is so dumb, I'm afraid to ask. . . . Am I mentally ill? You see my mom just gets a kick out of telling people that I'm mentally ill.

THERAPIST: Were you wondering if I'm critical and dismissive of you? I think you're asking if I could allay some fear that you're having. What is the fear?

MICHAEL: That my mom is right. She's joined some support group [for parents of the mentally ill]. I hate that it's given her something. She's the one who couldn't get out of bed for weeks on end, and my father is the one who killed my pet birds . . . but I'm the mentally ill one!

Subsequently, Michael produced a flood of memories about the bizarre and inappropriate behavior of his parents, but ultimately wound up talking about his fears that he is fated to be like his parents. He talked

about how he could not imagine having a family of his own, although increasingly he felt that he wanted to father a child. The therapist observed that it might be important for him to examine his hopes and what renders them impossible in his mind. He replied, "because I would turn into my father or my mother, maybe." The therapist observed that it sounded like he felt that his parents had become the representative of everything negative he feared in himself: "You might be afraid that you are just like your mother, but you're just desperately trying to avoid thinking about things within you that disturb you . . . it sounds like that is what the question 'Am I mentally ill?' is all about." Michael replied, "I am a lot like my parents—look how I've treated my wife, I've tormented her with my affairs, and neglected her and lived off her for long periods." The therapist then connected his sense of being mentally ill, "like his mother," with his bringing up the murder–suicide pact earlier in the treatment. At this point, the therapist focused on the destructive aspects of his identifications with powerful, punitive internal objects—the seductive, but neglectful maternal figure and the abusive paternal figure. The therapist further explored how the power of these internal representations, and how they were enacted both in Michael's experience of the therapist and his treatment of him—kept him from forming a genuine emotional connection with the therapist that moved beyond the lens of these internal representations and thereby kept him from accepting his help.

Michael then proceeded to describe a recent incident where he had attempted to rescue a wounded squirrel that, in spite of his efforts, had to be put to sleep. The therapist observed, "It's interesting that right after saying you couldn't be a father, you're giving a vignette which describes a great deal of caring, but also the end result is putting some little creature out of its misery . . . which is also interesting in someone who has made serious suicide attempts." He pointed out that his caring for animals reflected an inclination to identify with the therapist and his capacity for caring, but that his underlying belief that relations inevitably turn destructive could make the sense of self as caregiver seem frightening.

Michael then told the therapist that he had tormented his wife by indirect references to his infidelities. He said, "I mean this is probably mean of me—I got tickets for the opera Othello, about a man who suspects his wife of an affair and kills her . . . and I knew that, in a way, it was horrible to my wife to make her go with me . . . I knew it was not really humane to have her watch it with me." The therapist wondered, "Is that a part of what you meant when you said that some of you is 'like your mother'—aggressive without acknowledging it?" Michael then talked about his fear that the therapist would not be able to stand

the onslaught of his anger and aggressive fantasies and would retaliate. The session ended with the patient acknowledging that the therapist had already withstood them and contained them when he dealt with the murder–suicide pact and the box-cutter incident.

In the clinical material above, we can see the patient shifting among various representations of self as victim in thrall to a victimizer, or the self as a helpless and defective creature in need of rescue by an omnipotent savior, but with increasing awareness that he could play both roles in those dyads. As the self and object representations of a dependent self in relation to an omnipotent rescuer emerged in the transference, more primitive and varied internalized object relations emerged along with more genuine longings for connection and dependency. Initially such dependent longings may activate primitive fears of humiliation, intrusion, or vulnerability. For example, after 10 months of therapy, Michael told the therapist, with his voice trembling and with an anguished expression, that he had a vision of the therapist holding his soul in their joined hands. The therapist commented that these bodily images bespoke his two contradictory images of him: one as someone who might let him die, take something out of him, and leave him defenseless and dependent; and the other as someone to whom he is giving the enormous responsibility of holding his soul. The therapist pointed out how, in order to avoid intolerable anxiety, it must be essential for him to sequester his loving feelings toward the therapist on whom he had come to depend to the point where he imagined trusting him with his soul, from the hateful or vengeful feelings toward the therapist who might abandon, humiliate, or even destroy him. Michael said that he trusted the therapist as his soul keeper but also feared what would happen if he did let him hold his soul. He suggested that when these opposing feelings begin to come together it was too threatening, and he feared that expressing such positive feelings would lead to exploitation.

Here we see an example of the advanced stage of interpretation, where the therapist identifies the two contradictory dyads (idealized and persecutory) that have been dissociated and considers the reasons for the defensive dissociation between these dyads. This sets the stage for linking negative–paranoid and idealized transferences but does not yet address the underlying motivations for splitting. For more disturbed narcissistic patients, the shift from enactment to understanding the symbolic, constructed nature of the transference is often more protracted as the foregoing clinical examples indicate. As the patient emerges from the defensive retreat to the grandiose self and relinquishes the sense of omnipotence, he or she may experience extreme feelings of vulnerability, fear, or anxiety. These feelings may emerge in particularly dramatic ways, often expressed in images and metaphors. Noteworthy here is the

hallucinatory vividness of the language in the sessions reported above. Such vivid images as that of the therapist holding the patient's soul in their joined hands is often characteristic of the mid- to later phases of treatment with these patients and stems from the primarily imagistic rather than lexical nature of their representational world. In other words, patients tend to speak in images and such images encapsulate the self–object affect units. Such primitive self and object constellations, which have roots in bodily experience, may also be linked to a nascent symbolic function in these patients. In this material, we also see the beginnings of what Winnicott (1971) calls a potential or transitional space—that is, "an intermediate area of experience unchallenged in respect of its belonging to inner or external (shared) reality" (p. 16)—a realm of "intense experience that belongs to the arts, and to religion and to imaginative living (p. 16)" . . . where inner and outer realities meet, interact, and transform each other.

The development of an increased symbolic function often emerges for more disturbed narcissistic patients out of bodily or visceral sensations and experiences such as those described above. The therapist observed that these visceral bodily images bespoke Michael's two contradictory images of him: someone who might let him die, take something out of him, and leave him defenseless and dependent; and someone to whom he is giving the enormous responsibility of the keeper of his soul.

It was not until the extreme mutually dissociated mental representations of persecutor–victim and ideal savior–rescued were experienced directly in the transference that they could be explored and integrated. The dissolution of the grandiose self, infused with omnipotent aggression, paved the way for the emergence of these more disparate and contradictory representational dyads. These in turn enabled Michael to tolerate experiencing and exploring intrapsychic conflicts and motivations that had catalyzed his retreat to a grandiose self organized around idealized, powerful mental representations of self and other.

Although he continued to periodically have suicidal impulses, Michael ceased acting on them. He began to discuss feelings of anxiety and fear, disappointment and rage, and erotic longings and desires, rather than discharging them in actions. He often requested that the therapist look at images of his paintings and attend his art exhibits, and would become discomfited or even enraged when the therapist encouraged him to talk about what he was trying to convey in his images. He brought in song lyrics, or quotes from literature, to illustrate how he was feeling, and imagined from the therapist's facial expressions that he was familiar with and appreciated some of his literary and artistic references, insisting these were indications that they were soul mates meant to be

together. These themes in the transference were also accompanied by Michael devaluing his wife as uninteresting and not sexually desirable. The therapist consistently explored these requests/demands as indicative of longings for contact, but also as attempts to control him and make him submit to his demands as a way of warding off or protecting himself from imagined attacks. His transference to the therapist at this point involved his dual representation of him as an ideal other who would take care of him and make his life interesting and worthwhile, and also as a corruptible caregiver who would break boundaries and become his friend and even get involved with him sexually. In the latter, we see a split object relation organized around sexual seduction deployed in the interests of aggression and control that characterizes an erotized transference (Kernberg, 2019).

The therapist repeatedly reminded Michael that he was in treatment not to find a more fulfilling life in therapy but to overcome his difficulties in his experience of self and others so that he could establish a more fulfilling life outside of treatment. Michael would reply that the therapist was telling him that he was not worthy of him and he would have to settle for a second-rate existence. In this phase, the therapist had to address an idealization of him that prior manifestations of aggression and envy had defended against. It is important to remember that the ideal segment of the split internal world is as pathological as the persecutory segment in that it is equally unrealistic and it involves an internal representation that neither the self nor any real object could live up to. When the therapist reminded Michael about the contract that defined specific parameters for their relationship, he felt attacked and rejected, and often had the urge to counterattack and leave treatment. However, the therapist brought his attention to elements of the relation that did not fit with his formulation of him as a rejecting figure, including his consistent efforts to help him understand and express his intense feelings, his efforts to persuade him to continue when he wanted to leave treatment, and the moments when their dialogue in sessions had a mutually respectful and positively engaged feeling to it. Gradually, Michael began to relinquish the fantasy that the therapist would abandon his professional boundaries and become his soul mate and friend. Although he continued to threaten to leave the therapy at times because he found it so painful to have his feelings not reciprocated, increasingly he also recognized how these fantasies served to control, dominate, and affirm his superiority vis-à-vis the therapist. Thus, at this stage of the therapy, the integration and modulation of the aggressive and hostile aspects that emerged both in paranoid and erotized aspects of the transference cleared the way for the emergence of a more modulated, realistic experience of the self in relation to the therapist.

This movement was evident in the first 2 years of Michael's treatment. He had returned to college, working toward a degree in studio art. He continued to be preoccupied with inclinations to self-harm, but represented these artistically in drawings depicting self-injury. At the 18-month point, Michael began to talk about the therapist as "a sour lemon" that soothed his throat but could become stuck there and that he "could neither swallow nor spit out," a metaphor that showed his ambivalence about internalizing the therapist. At times he reported thinking obsessively about the therapist, saying things like, "I feel like you're in my head. I talk to you at lot."

Here one can also see some modulation of the image of therapist as the rescuing savior or ideal erotized fantasy figure into the image of the therapist as a soothing maternal presence, and, similarly, a modulation of the representation of the therapist as a threatening, destructive persecutor or corruptible caregiver into a needed, ambivalently held presence. In the countertransference at this point, the therapist experienced himself as a concerned caregiver who at times confronted Michael with aspects of reality that belied his grandiose aspirations—for example, when he decided to quit art school and focus on exhibiting in the major galleries, even though they rejected his work. The therapist worked with Michael on modulating his fantasies of immediate career success, focusing on how difficult it was for him to tolerate the narcissistic injury of taking it step-by-step. The therapist felt that such an approach would enable Michael to test his own capacities gradually and guard against his pattern of setting grandiose goals not in line with his actual possibilities or potential—which in the past had led to collapse and failure. Michael initially experienced his caution as a vote of no confidence and accused the therapist of undermining him and not wanting him to succeed.

At the end of his second year of therapy, Michael completed his BA and applied for a master's degree program. He brought a portfolio that he was planning to submit with his application to a session and requested that the therapist look at it and give him his honest opinion about his prospects as an artist. He wanted to leave the portfolio with the therapist. The therapist analyzed this request as an indication of his anxiety about the application process, and said that, in reality, it would take some time to know what he was capable of artistically. Michael then accused the therapist of being uncaring and of wanting to sabotage his career. In a later session, Michael was convinced that he had left his portfolio with the therapist and the therapist had hidden it and would not return it. He raged at the therapist and threatened to ransack the office and quit therapy unless he returned the portfolio or could prove that he had not taken it. The therapist acknowledged the strength of his conviction about what he saw as his dishonesty but insisted that he did

not have the portfolio. He further stated that it seemed that they were living in incompatible realities that could not be reconciled. Michael's first response was to accuse him of lying, indicating a regression to a paranoid, even psychopathic, transference. The therapist invited him to recognize that they might have divergent perspectives and that they were experiencing two incompatible realities, and that they would have to explore this discrepancy further to understand its meaning. After missing two sessions, Michael was able to acknowledge that he might have been mistaken and that he in fact was feeling extremely envious of the therapist, who had just published a book. Through this projection of his envy, he reexperienced the paranoid anxiety that his therapist would betray him and undermine his own attempts to be successful.

Michael's recognition that he was projecting his intense feelings of competition and envy onto the therapist opened up an awareness of his desire to have a successful career like the therapist and his fear that he would not be able to do so. By the time he found the portfolio in the lost and found of the art school, the regression was resolved. It should be noted that, even as patients move into the advanced stages of treatment, such regressions may still occur. However, at this stage of the treatment, these episodes may be worked through in several sessions or even in one session, often by going through the cycle of interpretation from the beginning and tracing the renewed projection of not fully integrated distrust, anger, and hostility; the process of integration is not linear but is subject to regression under stress.

The Advanced Stages of Treatment

In the above case example, one can see the clinical processes that involve reflection on the patient's experience of the therapist in a way that allows recognition and gradual integration of the split-off elements of self. This process involves a degree of internalization of the therapist's analytic function that allows the patient to move to the advanced stages of treatment.

The persistence of narcissistic defenses and resistances made this patient prone to complications that can emerge in the later stages of treatment. The type of paranoid regression in the transference described above may plunge the patient into quasi-delusional states. The patient can develop specific and circumscribed delusions about the therapist in situations where reality testing is relatively intact and ego functioning is preserved outside the therapy. In fact, because such regressions occur in the context of relatively improved ego functioning and reality testing, they often come as a shock to the therapist, causing him or her to question the solidity of the patient's improvement. Our experience

has shown, however, that such paranoid regressions in the transference, although usually quite intense and dramatic, are also generally brief in duration if handled adeptly. Paranoid regressions usually devolve from profoundly meaningful transferences that may not yet have surfaced in the treatment, and their interpretation and working through often leads one to a deeper understanding of the patient and what is going on in the transference and countertransference. They usually focus on the therapist as the source of the patient's distress, confusion, or anguish. Thus, it is essential for the therapist to both analyze his or her countertransference and to maintain his or her equanimity and neutrality during the episode, "interpreting it and weaving it into the overall treatment, rather than fostering its isolation or trying to bypass it" (Kernberg, Selzer, Koenigsberg, Carr, & Appelbaum, 1989, p. 185).

The resolution of paranoid regressions in the transference enables the patient to understand that the therapist may have a different state of mind than that which the patient attributes to him or her. The improvement in Michael's reflective capacity included the ability to identify and reflect states of visceral or bodily arousal (e.g., hands twitching, sexual feelings) and their connection with internal experiences (e.g., mental representations of interactions between self and others) so that they may be contained and understood. It also involved the ability to recognize that his own perceptions of interpersonal relationships are subjective and do not necessarily correspond to the other's intentions or mental states, or to external reality. This improvement in reflective capacity is crucial in that it diminishes the omnipotent control, devaluation, and omniscience that make narcissistic patients, particularly with malignant features, so difficult to treat. Thus, in the paranoid regression in Michael's treatment described above, the therapeutic task involved helping him to accurately identify the visceral feelings and impulses, and their associated affects that were being experienced in the immediate interaction with the therapist, and to recognize that these are linked to specific representations of self in interaction with others.

The patient moves into the advanced stages of treatment when he or she has achieved sufficient identity integration to tolerate and reflect on persecutory and idealized sectors of his or her internal world without reenacting them in a dissociated fashion. The patient advances when paranoid fears of hostile intentions in the clinical picture are supplanted by the capacity to take responsibility for and feel remorse about one's anger and hostility, rather than unilaterally projecting it onto others—in other words, movement from the paranoid–schizoid to a depressive position. The clinical signs that one will notice as patients, including the more severely disturbed, move into the more advanced stages of treatment include:

1. Decrease in acting out and increase in reliance on language as opposed to other forms of communication of internal states (e.g., gesture, silence, enactment); the patient may talk about acting out (e.g., the desire to injure him- or herself or others) rather than actually doing it.
2. Increase in capacity to anticipate and circumvent maladaptive behaviors.
3. Decrease in the abrupt, violent transitions between one representation and its related affective state and another—there are continuities that can be relied on to bridge affective states and contradictory experiences of the therapist and others.
4. The opening of the transitional space and tolerance of fantasy and free association with increased reliance on transitional phenomena—journal writing, poetry, and art—to help tolerate separation and relinquishment of the quest for the ideal in self and others.
5. Decrease in exhibitionism, exploitation, domination of others, overestimation of abilities, and improbable goals, as well as increase in realistic goals.
6. The resolution of psychopathic and paranoid transferences so that the patient no longer believes that the therapeutic relationship is based on mutual exploitation or attack, and the emergence of dependent and depressive transferences characterized by a more integrated and complex view of self and others in which honest, authentic communication dominates the clinical picture.
7. Increased attachment security and capacity for mentalization (e.g., reflective functioning).

Movement into the Depressive Position

During the final later stages of therapy, Michael completed his master's degree and developed deeper, more genuine friendships. He developed a greater appreciation for his wife and their relationship, felt renewed sexual interest in her, and expressed sadness about the pain he had caused her through his affairs. He began to consider becoming a father, which he had previously thought was impossible for him to manage. He began to focus on his regret for damage done to self and others in the past through his self-destructiveness, his sense of guilt about betraying his wife, and his rageful attacks on his therapist. He recollected times when he had wanted to hurt the therapist, or even attack him physically, and said that he felt remorse for such violent fantasies.

In the later stages, Michael began to have the capacity to work with interpretations linking here-and-now transferences to past experiences

and relationships—the last stage of interpretation in TFP-N. He began to talk about his own former attacks on his younger brother, who he imagined was his mother's favorite and who had been subsequently killed in a car accident. He was able to mourn the loss of his brother and to feel guilt over his previous sense of triumph at surviving the accident that killed his brother and injured his mother. He also felt remorse for his maltreatment of his wife, who had maintained her devotion to him in the face of his humiliating infidelities and financial exploitation of her. At this point, Michael experienced a transient depression, characterized by extreme self-criticism, a profound sense of unworthiness, and despondent mood, as he reflected on the damage he had wreaked on self and others as a result of his self-injurious behaviors and lack of empathy, as well as the traumatic early experiences that his grandiose self defended against.

The development of depression, sometimes quite severe and commensurate with the recognition of the severity of damage done to self and others, at this point in the therapy, may signal an improvement in superego functioning, a more refined moral sense, and the deepening of object relations. Even though this is progress toward achieving an integrated self, it can be a very painful experience. In the most severe cases, these understandable feelings of depression can be accompanied by renewed urges to self-harm or even to commit suicide. In such cases, the therapist explains to the patient that this important step in therapy is generally accompanied by strong depressive feelings but that these can be worked through to achieve a better life. This reconfiguration in his object relational world also led to a consolidation of identity that allowed Michael to set attainable and realistic goals and alleviated his need to retreat into an illusory grandiose identity. In sum, here one can see the differences in technique in the later stages of treatment that increasingly connect the here-and-now transferences between patient and therapist with unconscious motivations and conflicts, and their link with past experiences and relationships.

In the later stages of treatment, Michael showed an increased capacity to tolerate issues of rivalry and competition in triangular relationships. For example, in the last year of therapy he talked about feeling very special because his professor had used one of his most recent paintings in the studio art class to highlight a fine point of technique. He noticed that one of his male classmates had gotten up to go to the bathroom and had looked dismayed, and he imagined that his classmate was painfully envious of him. He then mentioned a champion athlete who was viciously attacked by another athlete to put him out of circulation for competition in the Olympic games. Michael's self-representation continued to show some division, but was beginning to coalesce as the patient could simultaneously represent his former acting-out self (the athlete who attacks

the teammate); the self as a special, achieving one (the athlete favored to win); and the self as helpful, reparative, and concerned (the self who is concerned about the impact of envious attacks on others).

In addition, one can see emerging a more integrated and modulated view of the therapist: the sense of the therapist as one who is not omniscient or omnipotent but who can potentially be hurt and injured, who has limits; one who cannot prevent all damage, but who tolerates damage and aggression without retaliation. Most evident here is the emergence of and tolerance for ambivalence toward the therapist as an important needed object on whom the patient now depended, and the expression of guilt and concern over the way this valued relationship was threatened by previous envious attacks. This capacity for guilt and concern also involved the desire to take reparative action toward the ambivalently loved object and formed the basis for more mature dependency and collaborative work, as well as expansion of this capacity into relationships outside the treatment setting. Hence, we see here the emergence of oedipal/competitive and depressive transference configurations characterized by more integrated, mature object relations (Diamond, 1993; Diamond & Yeomans, 2011).

The depressive position also involves mourning the loss of the idealized self and other representations that comprise the grandiose self, and with that comes a more differentiated sense of self and others, now seen with more dimension and complexity. During the later phases of the therapy, the patient brought in a fantasy that he would paint a portrait of the therapist with his family that would be superimposed on a portrait of him and his parents in the background. Michael said that this idea was inspired by his sense that when he looks at the therapist, he sees a double image. He said, "I see you, but it is my father on my retina. . . ." This fantasy portrait illustrates the patient's capacity to experience the as-if nature of the transference—that is, the understanding that the therapist is like the father even while he knows that he is not—epitomized by his statement that the image of the therapist he saw was configured by the internal representation of a parental figure that was, so to speak, on his "retina." These statements also show the expansion of the patient's capacity to experience relationships differently through the transference relationship with the therapist, which forms a bridge from the fragmented internal world of a distorted, caricatured, and one-dimensional representational world to one that is more integrated, multidimensional, complex, and realistic.

Michael's treatment ended at that point after 4 years. In subsequent years, he consulted with the therapist occasionally when challenging situations arose. Since termination, the patient has continued to make major gains. He has achieved some professional success as an artist and

has had several major exhibitions, while also supporting himself through teaching art. His relationship with his wife has endured and he reports a satisfactory sexual life, and a circle of supportive friends and colleagues. He has also had several children. So, our goals for TFP-N—change in the personality structure, conceptualized as diminution of symptoms and self-destructive behavior; the consolidation of identity; and the evolution of an integrated, differentiated world of internal object representations—were met for this patient. These changes aid in the expansion of fundamental psychological capacities: the capacity to appreciate the complexity and depth of others, and to have more gratifying relationships, including sexual relationships; the capacity to make more realistic and productive use of one's talents and abilities; and the capacity to develop and maintain self-esteem regardless of life's vicissitudes.

These gains in love and work, and in personality organization and functioning, were reflected in the research findings. Michael's clinical improvement was consistent with our research data showing that after 1 year of TFP-N (that part of his therapy that was in the context of a study), Michael showed:

1. Change in attachment (from insecure to secure) and from disorganized (from a primary AAI classification of unresolved for loss and abuse and secondary classification of fearfully preoccupied with trauma at admission to the study) to a primary attachment classification of secure (with a secondary classification of unresolved for loss and trauma).
2. Improvements in RF (mentalization) from low to ordinary (i.e., from a limited capacity to reflect on behavior in terms of intentional mental states) to a clear and explicit and even marked and striking capacity to reflect on mental states characterized by a developmental and an intergenerational perspective (i.e., recognition that his relationship and representation of his parents has changed and evolved with treatment, and appreciation for how his parents' parenting behaviors were distorted by their own early childhood experiences).
3. Substantial improvements in symptomatology, self-destructiveness, and level of personality organization (Clarkin et al., 2001; Diamond et al., 2003).

Summary and Conclusion

As the foregoing case example indicates, patients like Michael, with the syndrome of malignant narcissism, are at the boundary of treatability

but may be helped with TFP-N if a clear framework and structure for the treatment can be established, and if the therapist can hold them in mind as treatable through the vicissitudes of the therapy. Reflecting the paranoid, antisocial, and grandiose elements of his fragmented internal world, Michael oscillated between acting out and verbalizing his erotic and aggressive impulses, on the one hand, and a collaborative stance with respect to the therapist, on the other hand. Erotic feelings were conflated with aggression and both were split off and then enacted through his beginning an affair with a woman he met in AA, and then forming a murder–suicide pact with her that would also endanger the therapist. The therapist's incorruptibility, reliability, and capacity to tolerate and contain Michael's impulsive and dangerous acting out through a firm frame that protected both the patient and the therapist, allowed for the therapeutic relationship to evolve from one characterized by split polarized object relations that were evident in the narcissistic and paranoid/erotized transferences in the earlier and mid stages of treatment to the more integrated object relations that characterize dependent and depressive transferences of the later stages of therapy.

As Michael's extreme affects and conflicts embedded in contradictory, extreme object relational dyads were experienced and interpreted in their full emotional reality in the transference, he was able to join the therapist in reflecting on how they structured his internal and external reality in maladaptive ways. This case example illustrates that in patients with malignant narcissism the capacity for some nonexploitative investment in others, for some idealization of others and capacity for guilt and remorse about the damage they have wreaked on self and others, as well as some aspirations toward a more satisfying and stable life, however intermittent and partial, makes the therapeutic relationship possible (Kernberg, 2018); the latter forms a bridge to experiencing self and others in a more integrated, nuanced, and stable way. It is particularly important in such cases for the therapist to keep such a vision of the patient alive through the vicissitudes of the treatment so that it may "be brought into its own" (Loewald, 1960, p. 18).

CHAPTER 11

Narcissistic Love Relations

In previous chapters, we delineated how we work with conflicts and inhibitions in the capacity to love that we frequently see in those with pathological narcissism. In this chapter, we focus less on how we work with these conflicts in TFP-N, but more about the various ways in which narcissistic conflicts manifest themselves in love relations. Inspired by the myth of Narcissus, Freud (1914/1953), in his seminal essay "On Narcissism," hypothesized that while narcissism involves libidinal cathexis of the self, such cathexis may turn pathogenic if it is excessive or developmentally prolonged as a result of fixation or frustration. He wrote, "We must begin to love in order not to fall ill, and we are bound to fall ill, if, in consequence of frustration, we are unable to love" (p. 88). Although he did not have a clear model of pathological narcissism, Freud linked disorders of narcissism with the inability to love or invest in others in depth, and this led him to explore the balance between narcissism and love in object relating and choice. He stipulated that the individual may choose an object of the anaclitic or attachment type modeled after the nurturing maternal figure or the protective paternal figure. Or an individual may choose objects of a narcissistic type who represent what he or she is, what he or she once was (and no longer is), what he or she aspires to be but cannot attain, or a cherished love object felt to be part of the self.

These two aspects of love relations—the anaclitic and narcissistic—are optimally balanced in healthy love relations and exist in different proportions according to the level of organization of the individual. Kernberg (1975) points out that in intimate relationships, the object is always loved for both its unique otherness and its capacity to enhance the self, such that "the investment of objects and the investment of the self in

the gratifying relations with such objects go hand in hand" (p. 323). As discussed in previous chapters, narcissistic personalities function along a broad spectrum of pathology. At the mildest level, patients with pathological narcissism give a surface presentation of functioning quite appropriately in the social realm; are able to carry out work and professional demands; and can be a partner in a stable, but superficial, love relation. However, in treatment their conflicts show mostly in intimate relations, in the subtle grandiosity and contempt they experience in work relations, and in the endlessly unsatisfied ambitions and sense of emptiness that requires continuous external stimulation. For many, their difficulties can be well disguised through compensatory grandiosity and an external appearance of satisfaction.

In contrast, at the most severe level of narcissistic pathology, we may find a total breakdown in their capacity for work or profession, total incapacity to get involved in any love relation except transient sexual involvements, and chronic symptoms of anxiety and depression. With those who are higher functioning, narcissistic elements may predominate in love relations but they may coexist with some capacity for anaclitic bonds based on love and dependency, although relationships are likely to remain superficial and privilege admiration over love. For those with more severe narcissistic pathology, the narcissistic dimension may largely eclipse the anaclitic dimension altogether so that love relations are based on exploitation to gratify narcissistic needs. At all levels of narcissistic pathology, however, careful evaluation of the individual's love life shows significant problems. The basic dynamics of the narcissistic personality are the struggle with intense conscious and unconscious conflicts around envy, oscillation between grandiosity and thwarted ambitions/feeling devalued, and bouts of feelings of inferiority, all of which are particularly stimulated in intimate relations with a sexual partner.

A mature love relation implies the integration of erotic strivings and tenderness, ongoing mature idealization of the partner, gratitude for love received, interest in the partner's life as a major concern, and the sense of fulfillment of one's need to depend on and to be taken care of, as well as the ability to take care of a beloved being, while sharing a passionate involvement in their sexual encounters. The capacity for idealization of the love object is one of the characteristics of a mature adult passionate love relation. Mature idealization is based on the capacity to maintain an idealized view of the body and person of the other that becomes the repository of the history of their love and sexual passion—one that transcends the inevitable changes of time and aging and thus forms a wellspring for the renewal of the relationship

throughout time. Mature idealization is an aspect of the depressive position and should be differentiated from primitive idealization, which in those with narcissistic pathology forms a defense against devaluation. Mature idealization involves gratitude that the partner who has been able to maintain love despite having been the brunt of aggression, is still able to love and accept the partner. Mature idealization enriches the personality and couplehood, while primitive idealization diminishes it as it defends against aggressive feelings and interferes with the integration of loving and hateful feelings that characterize all love relations. In addition, mature idealization is based on respect and admiration for a partner who shares one's value and ideals, and who embodies esteemed characteristics that one aspires to, thus amplifying and enhancing one's existence. Such an ideal state of relatedness is clearly antithetical to the narcissist's inability to invest deeply in another person, to become emotionally dependent on him or her, and to be grateful for the gratification of dependency and sexual needs.

Individuals with narcissistic pathology suffer from feeling that to depend emotionally on another represents a loss or diminishment of their own greatness. Needing another person generates intense envy and evokes feelings of inferiority and vulnerability to the needed and potentially superior person. As a result, the unconscious tendency to devalue the partner erodes the feelings of love and intimacy, and tends to dissipate the sexual excitement of intimate passionate relations. Unconscious envy leads to defensive spoiling of the "good" that is received from the person who was initially idealized, leaving the patient bored, empty, and resentful.

Typically, narcissists experience a quick intense infatuation with an unrealistically and even wildly idealized other (i.e., the most beautiful, brilliant, interesting, accomplished, etc.) who seems tantalizingly unattainable. The chase is on. If the idealized other responds to the narcissistic person's desire, the idealization may continue during a brief "honeymoon," during which all needs are perfectly met, and both parties constitute a "golden couple." Disaster follows as the reality of the love object's "separateness" and imperfections gradually threaten the grandiose idealization. The excitement of sexual "conquest" is thus spoiled by disappointment and disillusionment over minor flaws—sex becomes routine and something of a chore as the unconscious envy and defensive devaluation of the other interferes with sexual gratification; the patient begins to feel "trapped," and the search for another ideal person to replace the present one begins anew. The sequence of frequent infatuations, intense relationships that rapidly become disappointing, and consequent sexual promiscuity is typical for narcissistic

personalities. Traditionally, this seemed more a feature of male psychology than of female psychology, because a "double standard" gave men within a patriarchal ideology more freedom than women. As women have increasingly achieved social and financial independence, this clinical picture is showing up in female narcissistic patients as well.

Patients with significant masochistic tendencies also may present with a history of frequent unsatisfactory relationships that would seem to reflect a similar dynamic of promiscuity, but that, in fact, has a completely different structure. In the case of masochistic patients, their unconscious guilt over a good relationship tends to induce its dismantling, and, typically, such patients finally get "stabilized" in relationships where they are made to suffer, in which, unconsciously, a heavy price is paid for a gratifying sexual love relation. In contrast, with narcissistic personalities, the search is on for a partner who, in the mind of these patients, is so extraordinarily great and gratifying that he or she will be protected from the apparently unavoidable devaluation that characterizes the love relations of these patients: somebody ideal enough to promise permanent gratification and yet, not so ideal that intense envy would tend to destroy the relation. One such patient, having already ended two marriages, was looking for a relationship with a woman who could be a gratifying partner, but who would be neither as disappointing as his first wife, nor as threatening as his successful second wife who had induced in him chronic feelings of inferiority and resentment as he contrasted himself to her.

Some individuals with pathological narcissism resolve this dilemma by maintaining a relationship with a person who gratifies his or her objective needs for comfort, social standing or admiration, and economic security. This relationship enables a freedom that permits them both transient sexual experiences with others and a complete dedication to a career or artistic field as a means of personal triumph that provides gratification and confirmation of one's superiority. In these cases, the quality of intimacy itself is degraded. Sometimes narcissistic personalities are drawn together because of their shared grandiosity and need for admiration to combine as the "beautiful couple," or "power couple" that affords maximum gratification of their need to be seen as superior. In other cases, a narcissistic person selects a partner with such apparent negative features that the patient's own superiority is highlighted by contrast and thus has a stabilizing effect. Or the couple may exist in a narcissistic bubble in which one person may inordinately and rigidly idealize the partner seen through the halo of his or her own grandiosity, and demand that others see the person in the same light regardless of the individual's actual characteristics (e.g., William Randolph Hearst

and Marion Davies in the film *Citizen Kane*). These dynamics are seen in both heterosexual and homosexual relationships where one or both parties have prominent narcissistic pathology.

At a deeper level, we often find an intense unconscious envy of the other gender, combined with the unconscious desire to be both genders at the same time. Here a combination of narcissistic and oedipal dynamics may be involved as a defense against a highly conflictual, traumatic relation with the unavailable oedipal ideal. An unconscious identification with both parental images constitutes a fantasized, fused, primitive oedipal couple within the patient's pathological grandiose self (Kernberg, 1995).

Narcissistic personalities with significant sexual inhibition related to oedipal conflicts, fears of being rejected, and the risk that such rejections would cause severe feelings of inferiority or failure may defensively stay away from all efforts to establish intimate relations, with a paranoid attitude toward potential partners. Sometimes, a long history of severe sexual inhibition may be followed by a "breakthrough" in a sexual affirmation that then permits the patient to engage in a pattern of "superior" promiscuity in replacement of the previous inhibition. In either case, the fear of injury to one's grandiose self is managed pathologically.

The incapacity to become interested in the life of the other person—typical for narcissistic personalities—usually creates, in the long run, intense negative developments in the life of the couple that threaten the relationship. In general, the absence of the psychological capacity for curiosity and interest in one's partner is one of the most dramatic consequences of narcissistic pathology, with its tendency to take the other for granted, boredom with the subjective experience of the other, and the experience of the relationship as more "transactional" than intersubjective, dominated by concern with "which of the two is getting more from the other." An example is a patient who would get enraged if the woman with whom he was involved seemed to obtain more pleasure in their sexual relations than he did.

A woman in her early 40s, who consulted because of chronic difficulties in maintaining a love relationship, was consciously desirous of establishing a marital relationship, but experienced her life as moving from one unhappy relationship to the next. She presented a combination of markedly narcissistic and masochistic characterological patterns. She could only accept successful and brilliant, good-looking men from her own cultural environment, and was easily attracted to what appeared to be severely narcissistic men, who were unwilling to establish a stable relationship with her on her terms. If, however, men seemed genuinely interested in her, she would rapidly devalue them: How good could they

be if they seemed to need her more than she needed them? By the same token, she was dismissive of men who seemed to be really mature and kind, but without the attitude of being special or unique.

Efforts to exert omnipotent control on the partner, to make sure that the partner does not behave or appear in ways that would make him or her look inferior, while making sure that there was no area in which the partner might appear superior, triumphant over, or controlling toward the patient, may result in chronic tensions and tend to disrupt the relationship—except in the case of narcissistic patients involved with severely masochistic partners. In such cases, a complementary relationship may be established between a narcissistic patient and an adoring, enslaved partner, or a narcissistic dependency on a partner in the sense of exploitation without recognition or gratitude for what is received, and an empty existence living together submerged in a conventional routine and social life sufficiently gratifying so that intimacy can be dispensed with.

Often, narcissistic patients show a complete absence of jealousy, reflecting the lack of normal dominance of oedipal dynamics. In yet other cases, a shared tolerance of a relationship with many sexual partners on the part of both partners of the couple, without jealousy, may also reflect a type of narcissistic equilibrium. Narcissistic personalities may not experience jealousy for various reasons, which include the chronic insularity of the grandiose self, an intolerance for acknowledging one's need of the other, and/or the inordinate gratification associated with seeing one's partner desired by rivals.

The development of triangulations, parallel sexual relationships that dilute the relationship with any one particular partner, may permit a narcissistic patient to sustain a sense of grandiosity, and fulfill greedy impulses for "not missing out on anything," while having a stabilizing effect on the primary relationship between a severely narcissistic personality and a masochistic or naïve partner. One patient considered that he was married to a woman who took excellent care of his home, children, and social engagements, but she seemed to him otherwise uninteresting and boring. After a brief initial period of their marriage, he experienced no further sexual interest in her. This patient was involved with a small group of prostitutes with whom he maintained relatively "stable" relationships over the years, and with whom he was able to experience sexual interests and gratification combined with the sense of superiority and control, and a sufficient maintenance of this sexual interest by "rotating" among that small group. In the course of the treatment, it turned out that his depreciated wife was an intelligent, effective, and highly attractive professional who, for her part, had developed an intense social life, and resigned herself to what she experienced as the emotional limitations of her husband. True to the defensive nature of the pathological

grandiose self, the patient's unconscious feelings of envy and inferiority regarding his wife and, behind that, a rageful hatred toward his mother, combined with an unconscious paranoid fear of women to present a dominant dynamic constellation that needed to be worked through in his psychoanalytic treatment.

The partner selection of those with narcissistic disorders is characteristic for this personality constellation. Their incapacity to evaluate other people in depth determines a basic insecurity regarding their selection of a partner. An incapacity to evaluate and project the impact of the partner's personality on their future interaction, the dominance of superficial selective criteria—such as whether the partner is sufficiently admiring of the patient, or whether the partner is sufficiently attractive to be admired by others—reinforces the grandiosity and superiority of the patient. The possibility of "getting more than having to give" in the overall transactions with the partner are leading considerations in object choice. Superficiality in the service of self and need gratification replaces deeper aspects of more mature love, which are the hallmarks of whole object relations, in which idealization of the other is not excessive or self serving, and there is a mutual appreciation and acceptance involving physical, sexual, ethical, and general personality features. In sharp contrast, narcissistic patients typically carry out a "market analysis" of the pros and cons of a prospective partner's attributes. The lack of the capacity to fall in love is a characteristic symptom of severely narcissistic personalities.

It is important to differentiate the infantile narcissistic features of certain male patients, corresponding to a histrionic or infantile personality, from the narcissistic personality proper in a strict sense. We are referring here to the type of "Don Juan" described by Chasseguet-Smirgel (1985): men who act in an adolescent way and are particularly attractive to women who have unconscious conflicts with and resentment of powerful male images. These men become dependent partners of dominant women and enact the childlike power of a mother's preferred child replacing the absent father. These patients are able to develop clinging, dependent relationships with dominant women, unlike the usual distance and aloofness typical of narcissistic personalities. The infantile type of Don Juan is able to maintain a relationship in which his role is protected and acceptable to the partner, and sexual promiscuity may appear only during the stages of development that precede finding the kind of woman who would be happy with such a man. Those with pathological narcissism or NPD, in contrast, maintain the stable pattern of narcissistic promiscuity without the capacity for dependency. The manifestations of envy at a conscious level are particularly marked in more severe degrees of narcissistic pathology. Better-functioning

narcissistic personalities may suffer less from painful envy, as their envy is compensated for by their sense of superiority, fostered particularly by professional success.

When the incapacity for maintaining any stable couple relation, and, in fact, a lack of interest in such a stable relation, combines with a predominance of distrustful, envious, and paranoid features, the search for dissociated sexual experiences as a source of excitement and a replacement of intimacy may become prominent. Here we find the typical syndrome of narcissistic patients who are socially isolated but pursue an intense involvement in "virtual sexual engagements" through the Internet, spending much of their free time watching pornography, in an interchange of erotic messages, and/or are involved in one-night stands. In other cases, the accommodation to narcissistic pathology takes the form of a life devoid of emotional engagement in a sexless marriage, while intense sexual relationships are split off from the "couplehood" and expressed in temporary engagements with others, thus providing the conditions for the "stability" of the narcissist's grandiosity. Sometimes the emptiness of a marital relation is compensated for by the patient's relationships with his or her children, enjoying the emotional dependency of the children who, at the same time, serve as a source of admiration as long as the real demands and rigors of child rearing are taken up by the spouse. In the case of chronic conflicts around a sense of mutual exploitation or disregard within the couple, children may be engaged as allies or replacement of an unsatisfactory relationship with the spouse, and the patient's envy may extend itself to the envy of the good relation that a partner may have with his or her children.

In milder cases, characterized in part by the capacity to tolerate depressive affects that work against the grandiose self, a wish to have a gratifying love relationship, fear of remaining alone, an experience of wished-for and yet feared dependency may lead the individual to wish to save a relationship in spite of a chronic sense of unhappiness engendered by the narcissistic structure. This may be the first step in the search for change, treatment, or some kind of solution. Guilt feelings over exploitation or aggression may color the relationship with the partner, the experience and awareness, perhaps fleeting, of the spouse's goodness and value, and a sense of concern over the patient's tendency to *devalue* the same, may indicate the development of some degree of awareness and concern related to the patient's narcissistic conflicts.

Sometimes a stable triadic relation that on the surface appears to be conflictual but at a deeper level stabilizes a couple's relationship may characterize the life arrangements of the narcissistic patient. For example, one prominent businessman was married to a distinguished member of the local aristocracy, while maintaining a relationship with a famous

actress. Both relationships were well-known in their social environment, and the family of the wife as well as the friends of the actress were putting pressure on the businessman to make a decision regarding with whom he wanted to stay. Under pressure, he entered treatment, but it became clear, soon enough, that treatment now constituted the alibi that permitted him to maintain the two relationships without any concern about them except to cover himself against attacks from partisans of both women. He decided to end the treatment because it was not helping him, he said, to decide regarding these two women, and he implied that he had to continue looking for other solutions: He did not seem particularly unhappy about maintaining this status quo.

A different situation was that of a married woman who had fallen in love with another man while being unable to end the relationship with her husband. She presented a narcissistic personality with a long history of sexual promiscuity that had now become a serious problem for her. She wanted to find a way to be happy in one stable relationship. The treatment revealed the unconscious, intense hatred and resentment of her husband, who represented a sexually teasing and indifferent father, condensed with the image of an absent, self-centered socialite mother, and the patient's conviction that she could be appreciated for her physical beauty but that she could not expect anybody to be interested in her as a person. These unconscious conflicts were defended against by a narcissistic grandiosity maintained by being admired rather than loved and needing to reactivate the gratification of being admired by new objects, as she was revengefully devaluing men with whom she established a relationship. Here the treatment permitted, first, the activation of intense resentment and conscious hatred of the husband as a father image, and later on, the analysis of her unconscious identification with her self-centered, superior, and indifferent mother as a central organizer of her pathological grandiose self. Eventually, the integration of her capacity to experience both love and hatred toward the analyst as a representative of a combined abandoning parental couple permitted her to deepen her marital relationship and to become able, for the first time in her life, to experience the combination of a tender, idealizing, and sexually gratifying relationship with her husband.

Regardless of the multiplicity of pathogenic object relations from infancy on, the structural development of a pathological grandiose self results in a fairly typical failure to develop mature love relations and a deficit in the psychological capabilities involved in such relations. A mature love relation requires the potential for interest in the life project of the partner; the development of basic trust and the capacity for dependency; the tolerance of ambivalence with its awareness of one's own aggression and the tolerance of the aggression in the partner; the

capacity for gratitude and, in fact, of humility for the love received by a person whom one has learned to appreciate and admire; the development of a joint ego ideal, joint aspirations, and life goals; and above all, the capacity for a mature dependency on the partner, as well as the capacity to permit the partner to depend on oneself (Kernberg, 2012). A mature love relation also involves containment of the paranoid mechanisms that emerge when the tolerance of negative emotional states and conflicts in a couple's life are being tested by adversity. The disappearance of prevalent mutual power struggles as an expression of the decrease of such paranoid tendencies, and, very importantly, the repeated reencounter of sexual passion as a permanent aspect of the relationship, are important indicators of a couple's maturing relation.

All of this requires the resolution of the pathological grandiose self that operates against the tolerance of dependency on one's partner, as well as against the tolerance of being depended upon, which is experienced as exploitativeness by virtue of the projection of the patient's own exploitative tendencies.

The Evaluation of Couples with Narcissistic Conflicts

Although our focus in this book is on the treatment of narcissistic individuals, it is frequently the case that an individual's first presentation to treatment is through the portal of couple therapy. One often hears in the initial couple therapy session how one partner has long requested that the spouse seek individual treatment for his or her rage, controlling tendencies, arrogance, or demeaning behavior. Approaches to the evaluation of couples vary widely across theoretical approaches. For example, systemic and systemic–analytical approaches (Bowen, 1978; Gerson, 2009; Ringstrom, 2012, 2014) often eschew individual evaluation and diagnosis, focusing largely on the couple as a system. In contrast, more purely analytical approaches (e.g., Akhtar, 2009; Dicks, 1967; Scharff & Scharff, 1991) combine a focus, to varying degrees as needed, on the individual with a focus on system dynamics.

The chronic emptiness of a couple's relationship when one or both participants suffer from severe narcissistic pathology is often palpable to the evaluating clinician, and the operation of narcissistic resistances in one or both of the partners can pose a challenge to the evaluation of the couple and their difficulties. Starting from the assumption that couples, often referred by an individual therapist, are being referred for couple treatment, the manifestation of narcissistic resistances in the evaluation process raises several general questions. First, why is the couple consulting now, and what are the actual dynamics that have shifted a previous

equilibrium (often a narcissistic–masochistic or depression equilibrium, or a grandiose–vulnerable narcissistic equilibrium)? Is the couple really interested in treatment or is one person forcing the participation of the partner, and how will that influence a decision regarding treatment? To what extent are the problems created mostly by one person's pathology as opposed to the pathological interaction and its shifts throughout time? This is a particularly controversial question for anyone approaching the couple from a systemic or systemic–analytical point of view, as the focus on one individual's contributions draws the therapist out of a neutral stance, more or less equidistant from the individual and working with the couple system, and *into* the very system being assessed. That being said, there are instances, not infrequent, where one partner's rage, contempt, and devaluation of the spouse and the therapist eviscerate the potential for work as a couple. When one partner's rage and devaluation cannot be stanched by the therapist, thus enacting an ineffective and abusive situation in the consulting room, one might opt to meet with that partner individually to raise the possibility of individual work, or of an evaluation of the nature of the work in which he or she may be already engaged.

Some clinicians, upon meeting intense narcissistic resistances, will engage in a full diagnostic evaluation of the presumptive narcissistic partner. If that evaluation reveals the partner indeed suffers from a severe pathological narcissism or NPD, which contributes significantly to the couple's conflict, the indication would be for the psychodynamic treatment of that partner. If both partners show significant pathology, the question can be raised to what extent a better equilibrium may be provided by couple therapy, or whether both should be referred for individual treatment.

The extent to which there is still a strong attachment within the couple, in contrast to a situation that seems to indicate a process of progressive dissolution, becomes central in the evaluation. Here the study of the nature and gratification in their sexual relationship, of mutual interests and interaction in their daily life, the feelings of love, admiration, respect, and empathy still available, in contrast to resentment, hatred, and feelings for revenge and contempt, have to be evaluated. A final element in the evaluation of the couple is the extent to which a joint superego system in the form of shared values and life goals, and relationship to family and friends, is still viable and can be recruited in the service of the therapy.

An ideal situation may be the decision to treat a member of the couple with the diagnosis of pathological narcissism or NPD in the context of a couple's relation that still seems sufficiently viable to tolerate the lengthy and slow process of treatment. Often a preliminary, time-limited

treatment of the couple may provide the conditions on the basis of which the couple can tolerate the vicissitudes of a lengthy treatment of one of its members.

The history of the couple is important in evaluating whether there ever was closeness, mutual idealization, a mutual sense of gratification, and being in love with each other that may have been lost but can still be remembered. Such a history strengthens the possibility that, beyond their present conflicts, there may be a basic future for them. Sometimes success or failure in work or the profession of one or both participants has changed a viable equilibrium into an intolerable situation, and when such external circumstances—economic, social, familial—prevail, their therapeutic exploration may provide the stability of the frame required for the treatment of the individual patient.

The Narcissistic Dynamics of the Love Relation in the Transference

The typical transference activation of the pathological grandiose self corresponds to the general characteristics of all narcissistic pathology in the analytical situation. The underlying dynamics of envy as a dominant aspect of aggressive conflicts with early parental images needs to be reactivated and explored in the transference. The resolution of narcissistic defenses in the transference typically involves the perception of the relationship with the therapist as always between one superior, grandiose, derogatory person, and one devalued, depreciated, worthless inferior person, reflecting, respectively, the grandiose self and the dissociated, devalued part of the patient's self. As discussed in previous chapters, this dominant transference dyad, with frequent role reversals, needs to be explored extensively, to the point where the pathological grandiose self can be dissolved into its component idealized and sadistic self and object representations.

Gradually, this dominant early transference disposition is replaced by the underlying conflicts, both preoedipal and oedipal, of infancy and early childhood, respectively. Fragmented aspects of the grandiose self in the transference take the shape of the dominant aspects of infantile objects and the conflicts related to them. Now, frustrating and teasing, rejecting and traumatizing representations of parental images and their corresponding self-representations are activated in the transference, and their emergence permits the working through of primitive, mutually split-off idealized and persecutory relations that eventually lead to an integration in the form of depressive transferences—the tolerance of ambivalent and reparative impulses. Parallel to these developments, the

patient's relationship to his or her sexual partner may shift and reveal similar conflicts and become more complex and deepen the patient's understanding of his or her emotional involvement with his or her partner. Optimally, a previously ignored and devalued partner may become an important, interesting and exciting, ambivalently feared, loved, and desired companion.

The long duration of such therapeutic involvements requires both enormous patience on the part of the analyst, and a minimum external viability of the couple that cannot always be maintained. Frequently, in the treatment of a narcissistic patient who consulted because of severe problems in couple or marital relations, the initial relation is replaced by one or several others until the patient reaches the capacity for a more mature love relation. It is very difficult to predict the final outcome in individual cases, but when treatment has been successful, the capacity for sexual love is restored to a point that permits the establishment of a mature love relation that makes a significant difference in a patient's and the couple's life.

The Relation between Preoedipal and Oedipal Conflicts of Narcissistic Personalities

The frequent combination in narcissistic patients of choosing severely narcissistic sexual partners, along with the incapacity of tolerating a good relationship with such a partner, may reflect the condensation of narcissism and yet unconscious guilt over identification with the oedipal couple. Narcissistic patients often do not wish to have children out of unconscious fear of their children's dependency upon them, the fantasy (realistic) of children's greedy demandingness, reflecting, in part, the projection onto the children of the patient's own unconscious feelings of deprivation, along with the attendant expressions of resentment and greed. At the same time, however, the refusal to have children may also reflect unconscious guilt feelings over one's oedipal identification, a masochistic element that needs to be taken into consideration, particularly in advanced stages, of the treatment of narcissistic patients. In that regard, the development of jealous feelings, painful feelings that one's partner may prefer a rival, and the emergence of fears of potential rivals much more able than the patient to gratify his or her partner, may signal a painful but progressive stage of development in the treatment.

Jealousy is a normal aspect of the repertoire of triangular relations geared to protect the boundaries of a love relation against invasion from the outside. Severely narcissistic patients are remarkably free of jealous feelings, reflecting their denial of oedipal wishes. The development of

jealousy, in addition to the working through of envy, marks the strengthening of the capacity to tolerate triangular relations. Narcissistic patients typically attempt to destroy the boundaries of exclusive relations of other couples, enacting both the envious wishes to appropriate for themselves what others have and they lack, and, at a deeper level, the attack on the oedipal couple. The tolerance of jealousy is a symptom of oedipal recovery.

A narcissistic patient's new girlfriend was a strikingly beautiful woman. He felt proud and successful for having established a love relation with her, which was, in fact, one more relationship in a long chain. They were invited to a party with people he was looking forward to being with, primarily for the admiration of this new relationship that he expected from them. He insisted that she would look "smashingly" . . . and she did.

In the next psychotherapeutic session, he complained about her coquettishness, her seductive behavior during that party, and her ignoring him during it. On further inquiry about what changed his feeling of pride and well-being into rage about her, he responded angrily that she became the center of attention of all the men at the party. The therapist reminded him that this fit with his own intentions before that evening.

Now the patient got angry with the therapist. In exploring what made him feel this way, he responded that it was clear that he had become jealous of her friendly reactions to various men, and that the therapist acted as if he could not understand that. The patient went on to say that he had the fantasy that the therapist was so certain of the love of his wife, that he would not have to feel jealous about other men approaching her, and then he remained silent. After a few minutes he said that this was a useless session, one more in which the therapist just showed his limited understanding of him.

The therapist commented that the patient seemed to have reacted with a sense of shame and humiliation because the therapist, in contrast to him, as he saw it, was secure in the love of his wife, implicitly putting him down. This interchange illustrates the shift from the activation of the patient's narcissistic "pride" in presenting his "smashing" girlfriend to the social gathering, in a sudden activation of oedipal competitiveness and fear, replicating the experience in the transference.

Countertransference

The typical picture of narcissistic transferences becomes complicated by the patient's description of the relationship with his or her partner. These partners are described, in turn, as highly desired objects or totally

devalued ones. There can be a devastating shift from the longing of a female patient for an ideal man, whom she describes in idealized fashion over an extended period of time, perceiving him as unavailable, and describing all his marvelous characteristics, to the description of the development of the relationship with that ideal subject, and then to her gradually or precipitously devaluing this man. In fact, in the therapist's subjective effort to empathize with the patient, that previously ideal object may end up appearing to the therapist as a completely worthless, useless individual. Convincingly, the patient points out that it is ridiculous to continue seeing such a poor choice, and to maintain a relationship that she entered in a clearly erroneous way.

What may be remarkable, at this point, is the therapist's identification with this view, unwittingly colluding with the patient's devaluation so that it becomes impossible to confront the patient with the devaluation of the person, who is described as so totally worthless that the therapist experiences himself as an arbitrary judge artificially pumping up the image of that completely deflated object. This process may develop over a sufficiently extended period of time to obscure the process of devaluation sufficiently so that the therapist, in fact, no longer has any basis to question the patient's reactions. It may take several of these developments—radical devaluation of desired objects—before the therapist may become fully alert to the process of devaluation, and more effectively attentive to its early developments and the clear relationship among initial idealization, envy, and devaluation. The attentive exploration of the patient's love relations, even if they appear, at first, as purely sexual engagements without any emotional involvement, may clarify the mechanism of defensive devaluation and the patient's destruction of her own gratification with the relationship that she had established, and become an important aspect of the interpretation of parallel developments in the transference.

Another countertransference with severely disturbed narcissistic patients may be the development of gradually growing, intense negative countertransferences because of the patient's unconsciously teasing the therapist: seducing the therapist with the evident lovely nature of a new partner in the patient's life, only to disappoint him or her sadistically with a following, clearly capricious devalution and contempt toward that new object. A male patient may describe what impresses the analyst as a lovely woman whom the patient mercilessly tears apart, as an expression of a teasing presentation of a good object to the therapist that the patient then dismantles revengefully with the fantasy of destroying an illusion of the therapist. The patient's relentless attack and depreciation of most people in his or her daily life, in a self-assured pseudological way, may become part of the omnipotent control exerted over the therapist and

convey to the therapist a feeling of impotence or incompetence in dealing with the degree of destructiveness in the patient's life.

In all cases, in the advanced stages of the treatment of narcissistic personalities, the tolerance of primitive aggression against needed and frustrating early parental images, and the development of the capacity for concern and guilt feelings over the consequences of these reactions permit the gradual consolidation of an integrated self and integrated representation of significant others, resolution of identity diffusion and the capacity for investment in an object relation in depth, and by the same token, the development of a mature love relation, a primary objective of the treatment.

CHAPTER 12

Epilogue

We have offered a model of psychodynamic psychotherapy with patients with pathological narcissism and NPD in the hope of guiding clinicians toward more precise interventions to effectively and empathically engage this challenging group of patients. A modicum of healthy narcissism is adaptive in that it confers positive self-regard based on a realistic, integrated self-concept, but when there is inordinate investment in an unrealistic, inflated sense of self and extreme, rigid emphasis on self-enhancement to the detriment of relationships with others, narcissism takes a pathological form and often results in disconnection, anguish, loss, distress in self and significant others, and even self-destruction. In short, narcissism in its pathological forms brings pain to those it afflicts and to those who are close to them.

In this book, we have attempted to convey that pathological narcissism is a complex and prevalent psychological disorder related to significant deficits in relationships and work, with ramifications in social, political and organizational domains. Pathological narcissism stems in part from early attachment experiences involving excessive parental deprivation and/or excessive overindulgence, which may severely truncate the capacity to love fully and work effectively. In addition, the strands of pathological narcissism are spun from different aspects of society. These strands come from the fragmentation of the family as a repository of non-narcissistic values of caring and concern for the next generation; from the hegemony of social media, which appears to offer new platforms for social connections and self-expression while fostering social isolation and superficiality; and from the rise of malignant narcissistic leaders, who foster collective narcissism in the citizenry by promising the restoration of a tribal form of supremacy, usually at the

expense of demonized outgroups, rather than promoting a truthful complex view of reality and humanistic ideals of individual autonomy, social responsibility, and equality. However, we do not pessimistically believe that human history is condemned to endless cycles of group regression to a paranoid-schizoid position , which reflects a fantasized perilous, persecutory, and chaotic world. Instead, we believe that education and psychotherapy might serve to counter the escalation of pathological narcissism at both the individual and collective levels. In this book, we have addressed the problem of narcissism on two fronts, through education about the complex social, intrafamilial, and intraindividual factors that predispose individuals to pathological narcissism, and through the development of effective methods of assessment and treatment offered in TFP-N.

TFP-N adapts and extends the techniques of TFP to meet the special clinical challenges posed by patients with pathological narcissism and NPD, whose retreat from reality into an illusory grandiosity makes them particularly difficult to engage in treatment. Narcissistic individuals fall on a spectrum from high functioning on the cusp of neurosis to functioning on an overt borderline level to suffering from the syndrome of malignant narcissism, the most severe variant of the disorder. Some may present as highly functional, accomplished, and self-assured, but harbor deep covert fears of inadequacy. Others may present as socially isolated, self-effacing, and symptomatic with chronic failures in love and work, but be internally preoccupied with grandiose beliefs that they are too shame prone to reveal or test in reality but that offer some semblance of stability. In others where grandiosity is infused with aggression and paranoia, there is the need to feel special and important by triumphing over or subjugating others. In this book, we focused on how to use TFP-N to help patients across this spectrum recognize the dynamics of pathological narcissism, that is, the maladaptive representations of self and others and the defenses that sustain them. By understanding how these dynamics manifest in the here and now of the relationship with the therapist, patients can develop and pursue their deepest aspirations for interpersonal connection.

We base this clinical approach on a contemporary object relations model of assessment and treatment that integrates findings from attachment theory and research, social cognition, neuroscience, and social psychology. In this multidimensional model, the pathological grandiose self of those with narcissistic pathology stems from a confluence of factors, including insecure/disorganized attachment representations; maladaptive, overinvolved, or exploitative patterns of parenting; early traumatic experiences; and/or temperamental or neurocognitive deficits. The grandiose self involves a specific constellation of mental representations of self and others: the pathological grandiose self in which ideal

and real aspects of self are condensed and negative aspects repudiated. Although often brittle and illusory, the pathological grandiose self confers some stability of functioning for narcissistic individuals, but renders them prone to discontinuities in self-states that account for their variable presentations. For example, narcissistic patients may present in a state of self-devaluation and inferiority characteristic of vulnerable or thin-skinned narcissistic persons, in contrast to the more frequent attitude of superiority and grandiosity typical of grandiose or thick-skinned narcissistic personalities—so in applying the technique, we consider the different presentations of narcissistic pathology, as well as the underlying level of personality organization. Whatever the surface presentation, persons with narcissistic pathology possess a heightened vulnerability in the sense of self, for which the typical narcissistic presentation (e.g., grandiosity, exploitativeness, entitlement) compensates.

The narcissistic person's difficulties allowing for a healthy dependency on and attachment to the therapist, as well as problems with tolerating interpretive work that requires a degree of objectivity and reflection, pose significant technical challenges. Understanding pathological narcissism in terms of structural pathology of object relations offers a coherent model that is readily translated into practice. TFP-N offers a well-articulated and unique mechanism of action for pathological narcissism and NPD—namely the resolution of the pathological grandiose self via transference exploration. TFP-N is an effective treatment for the spectrum of narcissistic disorders, because it is designed to identify the totality of the individual's internal dyadic experience (e.g., grandiose self–devalued other, vulnerable self–idealized other) as they emerge in the interpersonal patterns between patient and therapist in the here and now. Thus, it is effective in addressing the different phenotypical presentations, forms of expression, and fluctuating mental states that characterize those with narcissistic pathology across the spectrum of personality functioning.

In writing this book, we integrated the various theoretical and technical influences that have shaped each of us. Although we incorporate aspects of the work of other theoretical traditions, our approach is most reliant on the work of the North American object relations theorists, particularly coauthor Kernberg (1975, 1976, 1984, 2007). This work integrates aspects of contemporary North American and British object relations theory, ego psychology, and contemporary Freudian drive-based perspectives, with attachment research, neurobiology, and social psychology that expands the understanding of fundamental motivational systems that contribute to personality functioning and development.

The goal of TFP-N for narcissistic patients is to resolve the grandiose self that organizes the person's experience of self and others, and to increase the capacity for relatedness through an exploration of

interpersonal patterns between patient and therapist in transference. Through an interpretive process that includes clarification, confrontation, and interpretation, the therapist brings to light the distorted, contradictory, internalized object relations dyads, paying particular attention to those associated with grandiosity and vulnerability that comprise the grandiose self. The patient's involvement with idealized internal objects creates a situation in which what appears to be a relation with another person is a "self–self relation"—that is, the other is experienced, not as a separate being, but in the light of highly idealized or devalued aspects of self.

Such narcissistic transferences may be protracted and require an extended period of working through. Initially, the therapist will either be incorporated into the ideal grandiose self or serve as a repository for the devalued self. Exploring with the narcissistic patient the costs and consequences of grandiosity may be a first step in translating nonspecific narcissistic grandiosity into specific internalized object relations of grandiose self–devalued other or devalued self–superior other. In addition, exploring these manifestations of the grandiose self and the way it defends against intolerable affects, anxieties, and conflicts as they appear in relationships outside of the therapist dyad (with partners, spouses, children, colleagues) provides a bridge to exploring these issues in the transference to the therapist.

Interpreting in the projection or identifying the patient's immediate experience of the therapist and others in the moment without suggesting this has anything to do with the patient's motives or internal world is a first step toward defining the dominant concept of self and others that comprise the grandiose self. By not immediately challenging the patient's view of the therapist, while tactfully introducing alternative perspectives, we foster a more integrative sense of self and others, and invite reflection. Interventions should be delivered with tact and empathy, and calibrated to the patient's capacity to tolerate and integrate them. Working with role reversals in the transference, the process by which the therapist calls attention to how the patient may shift the position from which he or she interacts with the therapist, identifying with one side of the dominant object relation while enacting the other, often without awareness, enlarges the person's awareness of split-off and projected aspects of the self. This focus on perspectival shifts allows for richer elaboration of and reflection on the patient's internal experience beyond the rigid organization around grandiose or devalued self-states. As the components of the grandiose self are experienced, enacted, and projected in relation to the therapist, and its origins in identifications with powerful, idealized internal objects are explored, the grandiose self begins to break down. In the mid to later stages of TFP-N, previously

defended-against aspects of the representational world (involving dependent longings for a benevolent, caring other in relation to a dependent, gratified self) begin to configure the transference relationship. This sets the stage for the emergence of a more complex array of varied transferences (e.g., dependent, depressive, erotic transferences) that provide access to manifold layers of the internal world and reflect the deepening and intensification of the relationship with the therapist.

The dissolution of the grandiose self may be experienced as catastrophic for the narcissistic person as he or she confronts the inner sense of emptiness, vulnerability, isolation, or inadequacy that the grandiose self defended against often through unconscious identification with powerful, idealized, and, in some more severe cases, punitive internal figures. In the later stages, the individual becomes increasingly aware of conflictual aspects of relationships with primary attachment figures who previously may have been defensively idealized or devalued, leading to increased reflection on the origins of the grandiose self in past experience. Mourning for the loss of the ideal self-concept and ideal objects, as well as for the damage that narcissistic patients may have done to relationships, emerges in the later stages of therapy and may be quite painful. However, it is tempered by an expanded capacity to love and invest in relationships, including the relationship with the therapist. As patients emerge from the defensive retreat of the grandiose self, and with greater integration of their disparate self-states, they begin to experience themselves in relationships with increased vibrancy, depth, and connectedness.

Although residues of the narcissistic organization may persist over the course of TFP-N, the person has much greater capacity to reflect on them as mental states that are transient and situation bound, rather than fixed and invariant, and to integrate them into a more complex, nuanced view of self, others, and reality. A crucial factor in this process is the therapist's use of countertransference to identify and empathize with the patient's disparate self-states and to understand their relation to each other, without either retaliating or detaching in the face of the patient's devaluation, or colluding with narcissistic illusions in response to the patient's idealization of the therapist.

Conceptualizing treatment for the spectrum of narcissistic disorders (from low to high functioning) is necessary and timely. This is because we are seeing a growing number of individuals with diagnoses of pathological narcissism and NPD, as well as sub-clinical narcissistic traits, in our society; the comorbidity of NPD with other disorders is rampant; and the pathology manifests in different forms at transitional points in the life cycle, particularly young adulthood and midlife. In general, the development of techniques for treating those with narcissistic pathology

has not kept pace with the proliferation of theories about its etiology, the social factors that might be promoting it, the descriptive criteria through which to identify it, the social-cognitive deficits that characterize it, and the varied presentations of the disorder. More importantly, while there are a number of complex and rich theories of narcissism and narcissistic disorder, it is not always clear how such theories are translated into treatment modalities. Narcissistic patients are heterogeneous, and they pose significant treatment complications, including high dropout rates.

In sum, understanding narcissism through a clinical lens can grant us perspective on collective narcissism, or the proliferation of narcissistic traits as an adaptation to narcissistic aspects of contemporary society. Since the functional impairments in those with narcissistic pathology are sometimes most evident in the sphere of intimate relationships, and in the social and organizational realms, we have focused on this dimension in this final section. There is no doubt that narcissistic personality traits such as self-centeredness, social dominance, independence, and attention-seeking are socio-culturally sanctioned in our time and may lead to self-enhancing and self-promoting behaviors and attitudes in the general population that have some impact on the capacity for relatedness. Yet it is important to maintain the distinction between collective narcissism, on one hand, and maladaptive narcissism, as a specific pathological structuring of the personality, on the other. In other words, we must distinguish between the rise in narcissistic traits evident in the trend toward shallow self-absorption, the restless search for new sensations and experiences, the valorization of virtual reality, the captivation with celebrity and consumption, and the expansion of pathological narcissism characterized by an underlying personality structure involving the condensation of ideal and real self and object representations that may curtail the capacity to love. It may be that those who are destined to develop narcissistic pathology because of certain temperamental, developmental, or familial experiences are more susceptible to the influences of "the culture of narcissism" (Lasch, 1978). Although narcissistic traits may be on the rise, it is not likely that these will fundamentally interfere with the capacity to love reciprocally or to invest in significant relationships in those who do not suffer from pathological narcissism. There is something that happens in a deep relationship with a significant other that brings great satisfaction to the individual. It has been called transcendence, the sense of extending beyond oneself and feeling a sense of unity with all others who have lived and loved and suffered before, whether one's parents or people throughout history. When this sense cannot be attained, the individual feels emptiness and chronic dissatisfaction. People will simply continue to choose patterns that fulfill these needs for mutual affective resonance and intersubjective exchanges,

despite what society promulgates. In a sense, TFP-N serves to foster such innate proclivities by mobilizing aspects of the internal world as they emerge in transference with the goal of alleviating distortions and promoting maturation and integration of mental representations of self and others and the loving and hateful feelings with which they are linked in the interests of resolving the grandiose self and thereby fostering attachment security, the capacity for mentalization, and the capacity to love. This in turn fosters the basic human proclivity toward the integration and modulation of narcissistic strivings in the interests of relatedness to others that optimizes the formation, development, and sustaining of affectional bonds.

We hope this clinical guide equips clinicians to provide effective treatment for narcissistic patients across the spectrum of personality organization and functioning, and with diverse presentations. We also hope it is used to inform further reflection and research on the treatment of individuals with narcissistic disorders across the spectrum.

References

Ackerman, R. A., Hands, A. J., Donnellan, M. B., Hopwood, C. J., & Witt, E. A. (2017). Experts' views regarding the conceptualization of narcissism. *Journal of Personality Disorders, 31*(3), 346–361.

Adorno, T. W. (1987). Freudian theory and the pattern of fascist propaganda. In A. Arato & E. Gebhardt (Eds.), *The essential Frankfurt school reader* (pp. 118–137). New York: Continuum Books. (Original work published 1951)

Afifi, T. O., Mather, A., Boman, J., Fleisher, W., Enns, M. W., MacMillan, H., et al. (2011). Childhood adversity and personality disorders: Results from a nationally representative population-based study. *Journal of Psychiatric Research, 45*(6), 814–822.

Ainsworth, M. D. S. (1985). Attachments across the life span. *Bulletin of the New York Academy of Medicine, 61*(9), 792–811.

Ainsworth, M. D. S., Blehar, M. C., Waters, E., & Wall, S. (1978). *Patterns of attachment: A psychological study of the Strange Situation*. Hillsdale, NJ: Erlbaum.

Akhtar, S. (1989). Narcissistic personality disorder: Descriptive features and differential diagnosis. *Psychiatric Clinics of North America, 12*(3), 505–529.

Akhtar, S. (2009). Love, sex, and marriage in the setting of pathological narcissism. *Psychiatric Annals, 39*(4), 185–191.

Akhtar, S., & Samuel, S. (1996). The concept of identity: Developmental origins, phenomenology, clinical relevance, and measurement. *Harvard Review of Psychiatry, 3*(5), 254–267.

Akhtar, S., & Thomson, J. A. (1982). Overview: Narcissistic personality disorder. *American Journal of Psychiatry, 139*(1), 12–20.

American Psychiatric Association. (2013). *Diagnostic and statistical manual of mental disorders* (5th ed.). Arlington, VA: Author.

Ammaniti, M., & Gallese, V. (2014). *The birth of intersubjectivity: Psychodynamics, neurobiology, and the self*. New York: Norton.

Ammaniti, M., & Sergi, G. (2003). Clinical dynamics during adolescence: Psychoanalytic and attachment perspectives. *Psychoanalytic Inquiry, 23*(1), 54–80.

Ansell, E. B., Wright, A. G., Markowitz, J. C., Sanislow, C. A., Hopwood, C. J., Zanarini, M. C., et al. (2015). Personality disorder risk factors for suicide attempts over 10 years of follow-up. *Personality Disorders: Theory, Research, and Treatment, 6*(2), 161–167.

Apter, A., Bleich, A., King, R. A., Kron, S., Fluch, A., Kotler, M., et al. (1993). Death without warning? A clinical postmortem study of suicide in 43 Israeli adolescent males. *Archives of General Psychiatry, 50*(2), 138–142.

Aron, L. (1990). Free association and changing models of mind. *Journal of the American Academy of Psychoanalysis, 18*(3), 439–459.

Auerbach, J. S., & Diamond, D. (2017). Mental representation in the thought of Sidney Blatt: Developmental processes. *Journal of the American Psychoanalytic Association, 65*(3), 509–523.

Bach, S. (1985). *Narcissistic states and the therapeutic process*. Lanham, MD: Rowman & Littlefield.

Bachar, E., Canetti, L., Hadar, H., Baruch, J., Dor, Y., & Freedman, S. (2015). The role of narcissistic vulnerability in predicting adult posttraumatic symptoms from childhood sexual abuse. *Child Psychiatry and Human Development, 46*(5), 800–809.

Bakermans-Kranenburg, M. J., & van IJzendoorn, M. H. (2009). The first 10,000 Adult Attachment Interviews: Distributions of adult attachment representations in clinical and non-clinical groups. *Attachment and Human Development, 11*(3), 223–263.

Bakkevig, J. F., & Karterud, S. (2010). Is the Diagnostic and Statistical Manual of Mental Disorders, histrionic personality disorder category a valid construct? *Comprehensive Psychiatry, 51*(5), 462–470.

Baranger, M., & Baranger, W. (2008). The analytic situation as a dynamic field. *International Journal of Psychoanalysis, 89*(4), 795–826.

Barbaro, N., Boutwell, B. B., Barnes, J. C., & Shackelford, T. K. (2017). Rethinking the transmission gap: What behavioral genetics and evolutionary psychology mean for attachment theory: A comment on Verhage et al. (2016). *Psychological Bulletin, 143*(1), 107–113.

Bardenstein, K. K. (2009). The cracked mirror: Features of narcissistic personality disorder in children. *Psychiatric Annals, 39*(3), 147–155.

Baron-Cohen, S., & Wheelwright, S. (2004). The empathy quotient: An investigation of adults with Asperger syndrome or high functioning autism, and normal sex differences. *Journal of Autism and Developmental Disorders, 34*(2), 163–175.

Barone, L. (2003). Developmental protective and risk factors in borderline personality disorder: A study using the Adult Attachment Interview. *Attachment and Human Development, 5*(1), 64–77.

Barry, C. T., Frick, P. J., & Killian, A. L. (2003). The relation of narcissism and self-esteem to conduct problems in children: A preliminary investigation. *Journal of Clinical Child and Adolescent Psychology, 32*(1), 139–152.

Baskin-Sommers, A., Krusemark, E., & Ronningstam, E. (2014). Empathy in narcissistic personality disorder: From clinical and empirical perspectives. *Personality Disorders: Theory, Research, and Treatment, 5*(3), 323–333.

Bateman, A. (1998). Thick- and thin-skinned organisations and enactment in borderline and narcissistic disorders. *International Journal of Psycho-Analysis, 79*(Pt. 1), 13–25.

Bateman, A. W., & Fonagy, P. (2004). Mentalization-Based Treatment of BPD. *Journal of Personality Disorders, 18*(1), 36–51.

Bateman, A. W., & Fonagy, P. (Eds.). (2012). *Handbook of mentalizing in mental health practice*. Washington, DC: American Psychiatric Publishing, Inc.

Bateman, A. W., & Fonagy, P. (2016). *Mentalization-based treatment for personality disorders: A practical guide*. Oxford, UK: Oxford University Press.

Beck, A. T., Freeman, A., & Davis, D. D. (2004). *Cognitive therapy of personality disorders*. New York: Guilford Press.

Beebe, B., Jaffe, J., Markese, S., Buck, K., Chen, H., Cohen, P., Bahrick, L., Andrews, H., Feldstein, S. (2010). The origins of 12-month attachment: A microanalysis of 4-month mother-infant interaction. *Attachment and Human Development, 12*(1–2), 3–141.

Beebe, B., & Lachmann, F. M. (2013). *Infant research and adult treatment: Co-constructing interactions*. New York: Routledge.

Beebe, B., Lachmann, F., Markese, S., & Bahrick, L. (2012). On the origins of disorganized attachment and internal working models: Paper I. A dyadic systems approach. *Psychoanalytic Dialogues, 22*(2), 253–272.

Beebe, B., Messinger, D., Bahrick, L. E., Margolis, A., Buck, K. A., & Chen, H. (2016). A systems view of mother–infant face-to-face communication. *Developmental Psychology, 52*(4), 556–571.

Behary, W. T., & Davis, D. D. (2015). Narcissistic personality disorder. In A. T. Beck, D. D. Davis, & A. Freeman (Eds.), *Cognitive therapy of personality disorders* (3rd ed., pp. 299–325). New York: Guilford Press.

Behary, W. T., & Dieckmann, E. (2011). Schema therapy for narcissism. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 445–456). Hoboken, NJ: Wiley.

Bender, D. S., Farber, B. S., & Geller, J. D. (2001). Cluster B personality traits and attachment. *Journal of the American Academy of Psychoanalysis, 29*(4), 551–563.

Bender, D. S., Morey, L. C., & Skodol, A. E. (2011). Toward a model for assessing level of personality functioning in DSM-5, part I: A review of theory and methods. *Journal of Personality Assessment, 93*(4), 332–346.

Bernhardt, B. C., & Singer, T. (2012). The neural basis of empathy. *Annual Review of Neuroscience, 35*, 1–23.

Besel, L. D. S., & Yuille, J. C. (2010). Individual differences in empathy: The role of facial expression recognition. *Personality and Individual Differences, 49*(2), 107–112.

Besser, A., & Priel, B. (2010). Grandiose narcissism versus vulnerable narcissism in threatening situations: Emotional reactions to achievement failure and interpersonal rejection. *Journal of Social and Clinical Psychology, 29*(8), 874–902.

Besser, A., Zeigler-Hill, V., Pincus, A. L., & Neria, Y. (2013). Pathological narcissism and acute anxiety symptoms after trauma: A study of Israeli civilians exposed to war. *Psychiatry, 76*(4), 381–397.

Betan, E., Heim, A. K., Zittel Conklin, C., & Westen, D. (2005). Countertransference phenomena and personality pathology in clinical practice: An empirical investigation. *American Journal of Psychiatry, 162*(5), 890–898.

Bion, W. (1957). Differentiation of the psychotic from the non-psychotic personalities. *International Journal of Psychoanalysis, 38*, 266–275.

Bion, W. R. (1959). Attacks on linking. *International Journal of Psychoanalysis, 40*, 308–315.

Bion, W. R. (1962). A theory of thinking. *International Journal of Psychoanalysis, 43*, 306–310.

Bion, W. R. (1994). *Learning from experience*. Lanham, MD: Jason Aronson.

Blair, R. J. R., Mitchell, D. G. V., Peschardt, K. S., Colledge, E., Leonard, R. A., Shine, J. H., et al. (2004). Reduced sensitivity to others' fearful expressions in psychopathic individuals. *Personality and Individual Differences, 37*(6), 1111–1122.

Blais, M. A., Hilsenroth, M. J., & Castlebury, F. D. (1997). Content validity of the DSM-IV borderline and narcissistic personality disorder criteria sets. *Comprehensive Psychiatry, 38*(1), 31–37.

Blasco-Fontecilla, H., Baca-Garcia, E., Dervic, K., Perez-Rodriguez, M. M., Lopez-Castroman, J., Saiz-Ruiz, J., et al. (2009). Specific features of suicidal behavior in patients with narcissistic personality disorder. *Journal of Clinical Psychiatry, 70*(11), 1583–1587.

Blatt, S. J. (1974). Levels of object representation in anaclitic and introjective depression. *Psychoanalytic Study of the Child, 29*, 107–157.

Blatt, S. J. (1983). Narcissism and egocentrism as concepts in individual and cultural development. *Psychoanalysis and Contemporary Thought, 6*(2), 291–303.

Blatt, S. J. (1990). Interpersonal relatedness and self-definition: Two personality configurations and their implications for psychopathology and psychotherapy. In J. L. Singer (Ed.), *Repression and dissociation: Implications for personality theory, psychopathology & health* (pp. 299–335). Chicago: University of Chicago Press.

Blatt, S. J. (2004). *Experiences of depression: Theoretical, clinical, and research perspectives*. Washington, DC: American Psychological Association.

Blatt, S. J. (2008). *Polarities of experience: Relatedness and self-definition in personality, development, psychopathology, and the therapeutic process*. Washington, DC: American Psychological Association.

Blatt, S. J., Auerbach, J. S., & Behrends, R. S. (2008). Changes in the representation of self and significant others in the treatment process: Links between representation, internalization, and mentalization. In E. L. Jurist, A. Slade, & S. Bergner (Eds.), *Mind to mind: Infant research, neuroscience, and psychoanalysis* (pp. 225–263). New York: Other Press.

Blatt, S. J., & Levy, K. N. (2003). Attachment theory, psychoanalysis, personality development, and psychopathology. *Psychoanalytic Inquiry, 23*(1), 102–150.

Blatt, S. J., & Behrends, R. S. (1987). Internalization, separation–individuation, and the nature of therapeutic action. *International Journal of Psycho-Analysis, 68*(2), 279–297.

Block, J. (1961). *The Q-sort method in personality assessment and psychiatric research* [monograph] Springfield, Illinois: Charles C. Thomas.

Block, J. H., & Block, J. (1980). The role of ego-control and ego-resiliency in the organization of behavior. In W. A. Collins (Ed.), *Minnesota symposia on child psychology* (Vol. 13, pp. 39–101). Hillsdale, NJ: Erlbaum.

Block, J. (2008). *The Q-sort in character appraisal: Encoding subjective impressions of persons quantitatively*. Washington, DC: American Psychological Association.

Blos, P. (1967). The second individuation process of adolescence. *Psychoanalytic Study of the Child, 22*, 162–186.

Bodlund, O., Ekselius, L., & Lindström, E. (1993). Personality traits and disorders among psychiatric outpatients and normal subjects on the basis of the SCID screen questionnaire. *Nordic Journal of Psychiatry, 47*(6), 425–433.

Boesky, D. (1990). The psychoanalytic process and its components. *Psychoanalytic Quarterly, 59*(4), 550–584.

Bollas, C. (2008). *The evocative object world*. New York: Routledge.

Bosson, J. K., Lakey, C. E., Campbell, W. K., Zeigler-Hill, V., Jordan, C. H. & Kernis, M. H. (2008). Untangling the links between narcissism and self-esteem: A theoretical and empirical review. *Social and Personality Psychology Compass, 2*, 1415–1439.

Bowen, M. (1978). *Family therapy in clinical practice*. Northvale, NJ: Jason Aronson.

Bowlby, J. (1973). *Attachment and loss: Vol. 2. Separation*. New York: Basic Books.

Bowlby, J. (1977). The making and breaking of affectional bonds: I. Aetiology and psychopathology in the light of attachment theory. *British Journal of Psychiatry, 130*(3), 201–210.

Bowlby, J. (1978). Attachment theory and its therapeutic implications. *Adolescent Psychiatry, 6*, 5–33.

Bowlby, J. (1980). *Attachment and loss: Vol. 3. Loss: Sadness and depression*. New York: Basic Books.

Bowlby, J. (1982a). Attachment and loss: Retrospect and prospect. *American Journal of Orthopsychiatry, 52*(4), 664–678.

Bowlby, J. (1982b). *Attachment: Vol. 1. Attachment and loss* (2nd ed.). New York: Basic Books.

Bowlby, J. (1988). *A secure base: Parent–child attachment and healthy human development*. New York: Basic Books.

Bradley, R., Heim, A. K., & Westen, D. (2005). Transference patterns in the psychotherapy of personality disorders: Empirical investigation. *British Journal of Psychiatry, 186*(4), 342–349.

Britton, R. (1989). The missing link: Parental sexuality in the Oedipus complex. In M. Feldman & E. O'Shaughnessy (Eds.), *The Oedipus complex today: Clinical implications* (pp. 83–101). London: Karnac Books.

Britton, R. (2003). *Belief and imagination: Explorations in psychoanalysis*. New York: Routledge.

Britton, R. (2004a). Narcissistic disorders in clinical practice. *Journal of Analytical Psychology, 49*(4), 477–490.

Britton, R. (2004b). Subjectivity, objectivity, and triangular space. *Psychoanalytic Quarterly, 73*(1), 47–61.

Britton, R. (2014). The Oedipus situation and the depressive position. In R. Anderson (Ed.), *Clinical lectures on Klein and Bion* (pp. 34–45). New York: Routledge.

Bromberg, P. M. (1983). The mirror and the mask: On narcissism and psychoanalytic growth. *Contemporary Psychoanalysis, 19*(2), 359–387.

Bromberg, P. M. (1993). Shadow and substance: A relational perspective on clinical process. *Psychoanalytic Psychology, 10*(2), 147–168.

Bromberg, P. M. (1996). Standing in the spaces: The multiplicity of self and the psychoanalytic relationship. *Contemporary Psychoanalysis, 32*(4), 509–535.

Brummelman, E., Nelemans, S. A., Thomaes, S., & Orobio de Castro, B. (2017). When parents' praise inflates, children's self-esteem deflates. *Child Development, 88*(6), 1799–1809.

Brummelman, E., Thomaes, S., Nelemans, S. A., De Castro, B. O., Overbeek, G., & Bushman, B. J. (2015). Origins of narcissism in children. *Proceedings of the National Academy of Sciences, 112*(12), 3659–3662.

Buchheim, A., Hörz-Sagstetter, S., Doering, S., Rentrop, M., Schuster, P., et al. (2017). Change of unresolved attachment in borderline personality disorder: RCT study of transference-focused psychotherapy. *Psychotherapy and Psychosomatics, 86*(5), 314–316.

Bursten, B. (1973). Some narcissistic personality types. *International Journal of Psychoanalysis, 54*(3), 287–300.

Busch, F. (2013). *Creating a psychoanalytic mind: A psychoanalytic method and theory*. New York: Routledge.

Cain, N. M., Pincus, A. L., & Ansell, E. B. (2008). Narcissism at the crossroads: Phenotypic description of pathological narcissism across clinical theory, social/personality psychology, and psychiatric diagnosis. *Clinical Psychology Review, 28*(4), 638–656.

Caligor, E., & Clarkin, J. F. (2010). An object relations model of personality and personality pathology. In J. F. Clarkin, P. Fonagy, & G. O. Gabbard (Eds.), *Psychodynamic psychotherapy for personality disorders: A clinical handbook* (pp. 3–35). Washington, DC: American Psychiatric Association.

Caligor, E., Diamond, D., Yeomans, F., & Kernberg, O. F. (2009). The interpretive process in psychoanalytic psychotherapy of borderline pathology. *Journal of the American Psychoanalytic Association, 52*(2), 271–301.

Caligor, E., Kernberg, O. F., & Clarkin, J. F. (2007). *Handbook of dynamic psychotherapy for higher level personality pathology*. American Psychiatric Publishing, Inc.

Caligor, E., Kernberg, O. F., Clarkin, J. F., & Yeomans, F. E. (2018). *Psychodynamic therapy for personality pathology: Treating self and interpersonal functioning*. Washington, DC: American Psychiatric Association.

Caligor, E., Levy, K., & Yeomans, F. E. (2015). Narcissistic personality disorders: Diagnostic and clinical challenges. *American Journal of Psychiatry, 172*(5), 415–422.

Campbell, W. K., & Baumeister, R. F. (2006). Narcissistic personality disorder. In J. E. Fisher & W. T. O'Donohue (Eds.), *Practitioner's guide to evidence-based psychotherapy* (pp. 423–431). Berlin: Springer Science + Business Media.

Campbell, W. K., Bonacci, A. M., Shelton, J., Exline, J. J., & Bushman, B. J. (2004). Psychological entitlement: Interpersonal consequences and validation of a new self-report measure. *Journal of Personality Assessment, 83*(1), 29–45.

Carey, S. (2009). *The origins of concepts*. Oxford, UK: Oxford University Press.

Carlson, K. S., & Gjerde, P. F. (2009). Preschool personality antecedents of narcissism in adolescence and young adulthood: A 20-year longitudinal study. *Journal of Research in Personality, 43*(4), 570–578.

Carsky, M., & Yeomans, F. (2012). Overwhelming patients and overwhelmed therapists. *Psychodynamic Psychiatry, 40*(1), 75–90.

Cascio, C. N., Konrath, S. H., & Falk, E. B. (2015). Narcissists' social pain seen only in the brain. *Social Cognitive and Affective Neuroscience, 10*(3), 335–341.

Cater, T. E., Zeigler-Hill, V., & Vonk, J. (2011). Narcissism and recollections of early life experiences. *Personality and Individual Differences, 51*(8), 935–939.

Chasseguet-Smirgel, J. (1985). *The ego ideal: A psychoanalytic essay on the malady of the ideal*. London: Free Association Books.

Cheng, J. T., Tracy, J. L., & Miller, G. E. (2013). Are narcissists hardy or vulnerable? The role of narcissism in the production of stress-related biomarkers in response to emotional distress. *Emotion, 13*(6), 1004–1011.

Chester, D. S., Lynam, D. R., Powell, D. K., & DeWall, C. N. (2015). Narcissism is associated with weakened frontostriatal connectivity: A DTI study. *Social Cognitive and Affective Neuroscience, 11*(7), 1036–1040.

Clarkin, J. F., Cain, N. M., & Lenzenweger, M. F. (2018). Advances in transference-focused psychotherapy derived from the study of borderline personality disorder: Clinical insights with a focus on mechanism. *Current Opinion in Psychology, 21*, 80–85.

Clarkin, J. F., Caligor, E., & Sowislo, J. F. (2020). An object relations model perspective on the alternative model for personality disorders (DSM-5). *Psychopathology. 53*(3–4), 141–148.

Clarkin, J. F., Caligor, E., Stern, B. L., & Kernberg, O. F. (2016). The Structured Interview of Personality Organization—Revised (STIPO-R). Unpublished manuscript available through the Weill Medical College of Cornell University: White Plains, New York.

Clarkin, J. F., Foelsch, P. A., Levy, K. N., Hull, J. W., Delaney, J. C., & Kernberg, O. F. (2001). The development of a psychodynamic treatment for patients with borderline personality disorder: A preliminary study of behavioral change. *Journal of Personality Disorders, 15*(6), 487–495.

Clarkin, J. F., Levy, K. N., Lenzenweger, M. F., & Kernberg, O. F. (2007). Evaluating three treatments for borderline personality disorder: A multiwave study. *American Journal of Psychiatry, 164*(6), 922–928.

Clemence, A. J., Perry, J. C., & Plakun, E. M. (2009). Narcissistic and borderline personality disorders in a sample of treatment refractory patients. *Psychiatric Annals, 39*(4), 175–184.

Coen, S. J. (2003). The thrall of the negative and how to analyze it. *Journal of the American Psychoanalytic Association, 51*(2), 465–489.

Coen, S. J. (2005). How to play with patients who would rather remain remote. *Journal of the American Psychoanalytic Association, 53*(3), 811–834.

Coen, S. J. (2010). Neediness and narcissistic defensive action. *Psychoanalytic Quarterly, 79*(4), 969–990.

Cohen, L. J., Tanis, T., Bhattacharjee, R., Nesci, C., Halmi, W., & Galynker, I. (2014). Are there differential relationships between different types of childhood maltreatment and different types of adult personality pathology? *Psychiatry Research, 215*(1), 192–201.

Cohn, J. F., & Tronick, E. Z. (1988). Mother–infant face-to-face interaction: Influence is bidirectional and unrelated to periodic cycles in either partner's behavior. *Developmental Psychology, 24*(3), 386–392.

Coleman, S. R. M., Pincus, A. L., & Smyth, J. M. (2019). Narcissism and stress-reactivity: a biobehavioural health perspective. *Health Psychology Review, 13*(1), 35–72.

Cooper, A. M. (1988). The narcissistic–masochistic character. In R. A. Glick & D.

I. Meyers (Eds.), *Masochism: Current psychoanalytic perspectives* (pp. 117–138). New York: Analytic Press.

Cooper, A. M. (1989). Narcissism and masochism: The narcissistic–masochistic character. *Psychiatric Clinics, 12*(3), 541–552.

Cooper, A. M. (1998). Further developments in the clinical diagnosis of narcissistic personality disorder. In E. F. Ronningstam (Ed.), *Disorders of narcissism: Diagnostic, clinical and empirical implications* (pp. 53–74). Washington, DC: American Psychiatric Press.

Cooper, L. D., Balsis, S., & Oltmanns, T. F. (2012). Self- and informant-reported perspectives on symptoms of narcissistic personality disorder. *Personality Disorders: Theory, Research, and Treatment, 3*(2), 140–154.

Cramer, P. (1999). Personality, personality disorders, and defense mechanisms. *Journal of Personality, 67*(3), 535–554.

Cukrowicz, K. C., Poindexter, E. K., & Joiner, T. E., Jr. (2011). Cognitive behavioral approaches to the treatment of narcissistic personality disorder. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 457–464). Hoboken, NJ: Wiley.

Davis, M. H. (1983). Measuring individual differences in empathy: Evidence for a multidimensional approach. *Journal of Personality and Social Psychology, 44*(1), 113–126.

Dawood, S., Schroder, H. S., Donnellan, M. B., & Pincus, A. L. (2018). Pathological narcissism and nonsuicidal self-injury. *Journal of Personality Disorders, 32*(1), 87–108.

Decety, J. (Ed.). (2012). *Empathy: From bench to bedside*. Cambridge, MA: MIT Press.

Decety, J., & Jackson, P. L. (2004). The functional architecture of human empathy. *Behavioral and Cognitive Neuroscience Review, 3*(2), 71–100.

Decety, J., & Lamm, C. (2006). Human empathy through the lens of social neuroscience. *Scientific World Journal, 6*, 1146–1163.

De Meulemeester, C., Lowyck, B., Vermote, R., Verhaest, Y., & Luyten, P. (2017). Mentalizing and interpersonal problems in borderline personality disorder: The mediating role of identity diffusion. *Psychiatry Research, 258*, 141–144.

De Panfilis, C., Antonucci, C., Meehan, K. B., Cain, N. M., Soliani, A., Marchesi, C., et al. (2019). Facial emotion recognition and social-cognitive correlates of narcissistic features. *Journal of Personality Disorders, 33*(4), 433–449.

Dhawan, N., Kunik, M. E., Oldham, J., & Coverdale, J. (2010). Prevalence and treatment of narcissistic personality disorder in the community: A systematic review. *Comprehensive Psychiatry, 51*(4), 333–339.

Diamond, D. (1993). The paternal transference: A bridge to the erotic oedipal transference. *Psychoanalytic Inquiry, 13*(2), 206–225.

Diamond, D. (2006). Social and psychological perspectives on narcissism and its disorders. In O. F. Kernberg & H. P. Hartmann (Eds.), *Narzissmus, grundlagen, storungsbilder, therapie* (pp. 171–204). Stuttgart, Germany: Schattauer.

Diamond, D., & Blatt, S. J. (1994). Internal working models and the representational world in attachment and psychoanalytic theories. In M. B. Sperling & W. H. Berman (Eds.), *Attachment in adults: Clinical and developmental perspectives* (pp. 72–97). New York: Guilford Press.

Diamond, D., Clarkin, J., Levine, H., Levy, K., Foelsch, P., & Yeomans, F. (1999). Borderline conditions and attachment: A preliminary report. *Psychoanalytic Inquiry, 19*(5), 831–884.

Diamond, D., Clarkin, J. F., Levy, K. N., Meehan, K. B., Cain, N. M., Yeomans, F. E., et al. (2014). Change in attachment and reflective function in borderline patients with and without comorbid narcissistic personality disorder in transference focused psychotherapy. *Contemporary Psychoanalysis, 50*(1–2), 175–210.

Diamond, D., & Hersh, R. G. (2020). Transference-focused psychotherapy for narcissistic personality disorder: An object relations approach. *Journal of Personality Disorders, 34*(Suppl.), 159–176.

Diamond, D., & Hörz-Sagstetter, S. (2012, September). Attachment and reflective function in patients with co-morbid narcissistic and borderline personality disorder: Implications for treatment process and outcome. In E. Ronningstam (Chair), *Borderline personality disorder and co-occurring conditions—Impact on course and treatment.* Symposium conducted at 2nd International Congress on Borderline Personality Disorder and Allied Disorders, Amsterdam, the Netherlands.

Diamond, D., Levy, K. N., Clarkin, J. F., Fischer-Kern, M., Cain, N. M., Doering, S., et al. (2014). Attachment and mentalization in female patients with comorbid narcissistic and borderline personality disorder. *Personality Disorders: Theory, Research, and Treatment, 5*(4), 428–433.

Diamond, D., & Meehan, K. B. (2013). Attachment and object relations in patients with narcissistic personality disorder: Implications for therapeutic process and outcome. *Journal of Clinical Psychology, 69*(11), 1148–1159.

Diamond, D., Stovall-McClough, C., Clarkin, J. F., & Levy, K. N. (2003). Patient-therapist attachment in the treatment of borderline personality disorder. *Bulletin of the Menninger Clinic, 67*(3), 227–259.

Diamond, D., Yeomans, F., & Levy, K. N. (2011). Psychodynamic psychotherapy for narcissistic personality disorder. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 423–433). Hoboken, NJ: Wiley.

Diamond, D., & Yeomans, F. E. (2008). Psychopathologies narcissiques et psychothérapie focalisée sur le transfert [Narcissism, its disorders and the role of transference-focused psychotherapy]. *Santé Mentale au Québec, 33*(1), 115–139.

Diamond, D., & Yeomans, F. E. (2011). Oedipal love and conflict in the transference/countertransference matrix: Its impact on attachment security and mentalization. In D. Diamond, J. Lichtenberg, & S. Blatt (Eds.), *Attachment and sexuality* (pp. 219–254). New York: Routledge.

Diamond, D., Yeomans, F. E., Stern, B., Levy, K. N., Hörz, S., Doering, S., et al. (2013). Transference focused psychotherapy for patients with comorbid narcissistic and borderline personality disorder. *Psychoanalytic Inquiry, 33*(6), 527–551.

Dickinson, K. A., & Pincus, A. L. (2003). Interpersonal analysis of grandiose and vulnerable narcissism. *Journal of Personality Disorders, 17*(3), 188–207.

Dicks, H. (1967). *Marital tensions*. London: Karnac Books.

DiGiuseppe, R., Robin, M., Szeszko, P. R., & Primavera, L. H. (1995). Cluster analyses of narcissistic personality disorders on the MCMI-II. *Journal of Personality Disorders, 9*(4), 304–317.

Dimaggio, G., Semeraria, A., Falcone, M., Nicolo, G., Carcione, A., & Procacci, M. (2002). Metacognition, states of mind, cognitive biases, and interpersonal cycles: Proposal for an integrated narcissism model. *Journal of Psychotherapy Integration, 12*(4), 421–451.

Di Pierro, R., Costantini, G., Benzi, I. M. A., Madeddu, F., & Preti, E. (2019). Grandiose and entitled, but still fragile: A network analysis of pathological narcissistic traits. *Personality and Individual Differences, 140*, 15–20.

Di Pierro, R., Di Sarno, M., Preti, E., Di Mattei, V. E., & Madeddu, F. (2017). The role of identity instability in the relationship between narcissism and emotional empathy. *Psychoanalytic Psychology, 35*(2), 237–243.

Di Sarno, M., Zimmermann, J., Madeddu, F., Casini, E., & Di Pierro, R. (2020). Shame behind the corner? A daily diary investigation of pathological narcissism. *Journal of Research in Personality, 85*.

Doering, S., Hörz, S., Rentrop, M., Fischer-Kern, M., Schuster, P., Benecke, C., et al. (2010). Transference-focused psychotherapy v. treatment by community psychotherapists for borderline personality disorder: Randomised controlled trial. *British Journal of Psychiatry, 196*(5), 389–395.

Doidge, N., Simon, B., Brauer, L., Grant, D. C., First, M., Brunshaw, J., et al. (2002). Psychoanalytic patients in the U.S., Canada, and Australia: I. DSM-III-R disorders, indications, previous treatment, medications, and length of treatment. *Journal of the American Psychoanalytic Association, 50*(2), 575–614.

Dolan, M., & Fullam, R. (2006). Face affect recognition deficits in personality-disordered offenders: Association with psychopathy. *Psychological Medicine, 36*(11), 1563–1569.

Dozier, M., & Kobak, R. R. (1992). Psychophysiology in attachment interviews: Converging evidence for deactivating strategies. *Child Development, 63*(6), 1473–1480.

Drozek, R. P., & Unruh, B. T. (2020). Mentalization-based treatment for pathological narcissism. *Journal of Personality Disorders, 34*(Supplement), 177–203.

Dziobek, I., Fleck, S., Kalbe, E., Rogers, K., Hassenstab, J., Brand, M., et al. (2006). Introducing MASC: A movie for the assessment of social cognition. *Journal of Autism and Developmental Disorders, 36*(5), 623–636.

Dziobek, I., Rogers, K., Fleck, S., Bahnemann, M., Heekeren, H. R., Wolf, O. T., et al. (2008). Dissociation of cognitive and emotional empathy in adults with Asperger syndrome using the Multifaceted Empathy Test (MET). *Journal of Autism and Developmental Disorders, 38*(3), 464–473.

Eagle, M. (2003). Clinical implications of attachment theory. *Psychoanalytic Inquiry, 23*(1), 27–53.

Eagle, M. N. (2013). *Attachment and psychoanalysis: Theory, research, and clinical implications*. Guilford Press.

Edelstein, R. S., Yim, I. S., & Quas, J. A. (2010). Narcissism predicts heightened cortisol reactivity to a psychosocial stressor in men. *Journal of Research in Personality, 44*(5), 565–572.

Edershile, E. A., Woods, W. C., Sharpe, B. M., Crowe, M. L., Miller, J. D., & Wright, A. G. C. (2019). A day in the life of Narcissus: Measuring narcissistic

grandiosity and vulnerability in daily life. *Psychological Assessment, 31*(7), 913–924.
Edershile, E. A., & Wright, A. G. (2019). Grandiose and vulnerable narcissistic states in interpersonal situations. *Self and Identity*, 1–17.
Elliot, A. J., & Thrash, T. M. (2002). Approach–avoidance motivation in personality: Approach and avoidance temperaments and goals. *Journal of Personality and Social Psychology, 82*(5), 804–818.
Ellison, W. D., Levy, K. N., Cain, N. M., Ansell, E. B., & Pincus, A. L. (2013). The impact of pathological narcissism on psychotherapy utilization, initial symptom severity, and early-treatment symptom change: A naturalistic investigation. *Journal of Personality Assessment, 95*(3), 291–300.
Ellman, S. J. (2014). Traversing narcissistic pathways: From Freud to present times. *Psychoanalytic Inquiry, 34*(5), 394–407.
Ensink, K., Berthelot, N., Bégin, M., Maheux, J., & Normandin, L. (2017). Dissociation mediates the relationship between sexual abuse and child psychological difficulties. *Child Abuse and Neglect, 69*, 116–124.
Ensink, K., Chretien, S., Normandin, L., Begin, M., Daigle, D., & Fonagy, P. (2017). Pathological narcissism in adolescents: Relationships with childhood maltreatment and internalizing and externalizing difficulties. *Adolescent Psychiatry, 7*(4), 300–314.
Erikson, E. (1950). *Childhood and society*. New York: Norton.
Erikson, E. H. (1956). The problem of ego identity. *Journal of the American Psychoanalytic Association, 4*(1), 56–121.
Erikson, E. H. (1959). *Identity and the life cycle: Selected papers*. New York: International Universities Press. Kindle Edition.
Erikson, E. H. (1968). *Identity: Youth and crisis*. New York: Norton.
Erikson, E. H. (1975). *Life history and the historical moment*. New York: Norton.
Erreich, A. (2015). Unconscious fantasy as a special class of mental representation: A contribution to a model of mind. *Journal of the American Psychoanalytic Association, 63*(2), 247–270.
Etchegoyen, R. H. (1999). *The fundamentals of psychoanalytic technique*. London: Karnac Books.
Fairbairn, W. R. D. (1952). *Psychoanalytic studies of the personality*. New York: Routledge.
Fan, Y., Wonneberger, C., Enzi, B., De Greck, M., Ulrich, C., Tempelmann, C., et al. (2011). The narcissistic self and its psychological and neural correlates: An exploratory fMRI study. *Psychological Medicine, 41*(8), 1641–1650.
Federn, P. (1926). Some variations in ego-feeling. *International Journal of Psycho-Analysis, 7*, 434–444.
Federn, P. (1955). On the distinction between healthy and pathological narcissism. In P. Federn & E. Weiss (Eds.), *Ego psychology and the psychoses* (pp. 323–364). New York: Basic Books.
Feldman, M. (1997). Projective identification: The analyst's involvement. *International Journal of Psychoanalysis, 78*(2), 227–241.
Feldman, M. (2007). Addressing parts of the self. *International Journal of Psychoanalysis, 88*(2), 371–386.
Feldman, M. (2009). *Doubt, conviction, and the analytic process*. New York: Routledge.

Feldman, M., & Spillius, E. B. (2003). *Psychic equilibrium and psychic change: Selected papers of Betty Joseph*. New York: Routledge.

Fenichel, O. (1941). *Problems of psychoanalytic technique*. [Trans. by D. Brunswick]. Psychoanalytic Quarterly, Inc.

Fenichel, O. (2014). *The psychoanalytic theory of neurosis*. New York: Routledge.

First, M. B., Gibbon, M., Spitzer, R. L., Williams, J. B. W., & Benjamin, L. (1997). *Structured Clinical Interview for DSM–IV Personality Disorders (SCID-II)*. Washington, DC: American Psychiatric Press.

Fiscalini, J. (1994). The uniquely interpersonal and the interpersonally unique: On interpersonal psychoanalysis. *Contemporary Psychoanalysis, 30*(1), 114–134.

Fischer-Kern, M., Doering, S., Taubner, S., Hörz, S., Zimmermann, J., Rentrop, M., et al. (2015). Transference-focused psychotherapy for borderline personality disorder: Change in reflective function. *British Journal of Psychiatry, 207*(2), 173–174.

Fjermestad-Noll, J., Ronningstam, E., Bach, B., Rosenbaum, B., & Simonsen, E. (2019). Characterological depression in patients with narcissistic personality disorder. *Nordic Journal of Psychiatry, 73*(8), 539–545.

Fonagy, P. (1999). Psychoanalytic theory from the viewpoint of attachment theory and research. In J. Cassidy & P. R. Shaver (Eds.), *Handbook of attachment: Theory, research, and clinical applications* (pp. 595–624). The Guilford Press.

Fonagy, P. & Allison, E. (2014). The role of mentalizing and epistemic trust in the therapeutic relationship. *Psychotherapy, 51*(3), 372–380. https://doi.org/10.1037/a0036505

Fonagy, P., Bateman, A. W., & Luyten, P. (2012). Introduction and overview. In A. W. Bateman & P. Fonagy (Eds.), *Handbook of mentalizing in mental health practice* (pp. 3–42). Arlington, VA: American Psychiatric Publishing.

Fonagy, P., & Campbell, C. (2015). Bad blood revisited: Attachment and psychoanalysis, 2015. *British Journal of Psychotherapy, 31*(2), 229–250.

Fonagy, P., Gergely, G., Jurist, E., & Target, M. (2002). *Affect regulation, mentalization and the development of the self*. New York: Other Press.

Fonagy, P., Leigh, T., Steele, M., Steele, H., Kennedy, R., Mattoon, G., et al. (1996). The relation of attachment status, psychiatric classification, and response to psychotherapy. *Journal of Consulting and Clinical Psychology, 64*(1), 22–31.

Fonagy, P., Luyten, P., Allison, E., & Campbell, C. (2017a). What we have changed our minds about: Part 1. Borderline personality disorder as a limitation of resilience. *Borderline Personality Disorder and Emotion Dysregulation, 4*(1), 11.

Fonagy, P., Luyten, P., Allison, E., & Campbell, C. (2017b). What we have changed our minds about: Part 2. Borderline personality disorder, epistemic trust and the developmental significance of social communication. *Personality Disorder and Emotion Dysregulation, 4*(1), 9.

Fonagy, P., Luyten, P., Bateman, A., Gergely, G., Strathearn, L., Target, M., & Allison, E. (2010). Attachment and personality pathology. In J. F. Clarkin, P. Fonagy, & G. O. Gabbard (Eds.), *Psychodynamic psychotherapy for personality disorders. A clinical handbook* (pp. 37–87). Washington, DC: American Psychiatric Publishing.

Fonagy, P., Steele, M., Steele, H., & Target, M. (1998). *Reflective-function manual: Version 5.0 for application to the Adult Attachment Interview*. Unpublished manual, University College, London.

Fossati, A., Feeney, J., Pincus, A., Borroni, S., & Maffei, C. (2015). The structure of pathological narcissism and its relationships with adult attachment styles: A study of Italian nonclinical and clinical adult participants. *Psychoanalytic Psychology, 32*(3), 403–431.

Fossati, A., Maffei, C., Bagnato, M., Donati, D., Donini, M., Fiorilli, M., et al. (2000). A psychometric study of DSM-IV passive–aggressive (negativistic) personality disorder criteria. *Journal of Personality Disorders, 14*(1), 72–83.

Foster, J. D., Campbell, W. K., & Twenge, J. M. (2003). Individual differences in narcissism: Inflated self-views across the lifespan and around the world. *Journal of Research in Personality, 37*(6), 469–486.

Freud, S. (1953). Recommendations to physicians practicing psycho-analysis. In J. Strachey (Ed. & Trans.), *The standard edition of the complete psychological works of Sigmund Freud: Vol. 12. The case of Schreber, papers on technique and other works (1911–1913)*. London: Hogarth Press. (Original work published 1914)

Freud, S. (1957). On narcissism: An introduction. In J. Strachey (Ed. & Trans.), *The standard edition of the complete psychological works of Sigmund Freud* (Vol. 14, pp. 69–102). London: Hogarth Press. (Original work published 1914)

Freud, S. (1964). Analysis terminable and interminable. In J. Strachey (Ed. & Trans.), *The standard edition of the complete psychological works of Sigmund Freud* (Vol. 23, pp. 216–253). London: Hogarth Press. (Original work published 1937)

Frith, U., & Frith, C. D. (2003). Development and neurophysiology of mentalizing. *Philosophical Transactions of the Royal Society London B, 358*(1431), 459–473.

Gabbard, G. (1989). Two subtypes of narcissistic personality disorder. *Bulletin of the Menninger Clinic, 53*(6), 527–532.

Gabbard, G. O. (1997). Finding the "person" in personality disorders. *American Journal of Psychiatry, 154*(7), 891–894.

Gabbard, G. O. (2009). Transference and countertransference: Developments in the treatment of narcissistic personality disorder. *Psychiatric Annals, 39*(3), 129–136.

Gabbard, G. O., & Crisp, H. (2018). *Narcissism and its discontents: Diagnostic dilemmas and treatment strategies with narcissistic patients*. Washington, DC: American Psychiatric Association.

Gabbard, G. O., & Crisp-Han, H. (2016). The many faces of narcissism. *World Psychiatry, 15*(2), 115–116.

Gabbard, G. O., & Westen, D. (2003). Rethinking therapeutic action. *International Journal of Psychoanalysis, 84*(Pt. 4), 823–841.

Gallese, V. (2003). The roots of empathy: The shared manifold hypothesis and the neural basis of intersubjectivity. *Psychopathology, 36*(4), 171–180.

George, C., Kaplan, N., & Main, M. (1996). *Adult Attachment Interview*. Unpublished manuscript, University of California, Berkeley.

Gerber, A. J. (2005). *Structural and symptomatic change in psychoanalysis and psychodynamic psychotherapy: A quantitative study of process, outcome, and attachment*. Doctoral thesis, University of London.

Gerson, M. J. (2009). *The embedded self: An integrative psychodynamic and systemic perspective on couple and family therapy* (2nd ed.). New York: Routledge.

Giacomin, M., & Jordan, C. H. (2016). Self-focused and feeling fine: Assessing state narcissism and its relation to well-being. *Journal of Research in Personality, 63*, 12–21.

Gill, M. M. (1954). Psychoanalysis and exploratory psychotherapy. *Journal of the American Psychoanalytic Association, 2*(4), 771–797.

Gill, M. M. (2013). *Psychoanalysis in transition: A personal view*. New York: Routledge.

Glover, N., Miller, J. D., Lynam, D. R., Crego, C., & Widiger, T. A. (2012). The Five-Factor Narcissism Inventory: A five-factor measure of narcissistic personality traits. *Journal of Personality Assessment, 94*(5), 500–512.

Gore, W. L., & Widiger, T. A. (2016). Fluctuation between grandiose and vulnerable narcissism. *Personality Disorders: Theory, Research, and Treatment, 7*(4), 363–371.

Graf, E., & Diamond, D. (2018). The significance of three evidence based psychoanalytic psychotherapies on psychoanalytic research, psychoanalytic theory and practice. In S. D. Axelrod, R. C. Naso, & L. M. Rosenberg (Eds.), *Progress in psychoanalysis* (pp. 147–172). New York: Routledge.

Grant, B. F., Hasin, D. S., Stinson, F. S., Dawson, D. A., Chou, S. P., Ruan, W., et al. (2004). Prevalence, correlates, and disability of personality disorders in the United States: Results from the National Epidemiologic Survey on Alcohol and Related Conditions. *Journal of Clinical Psychiatry, 65*(7), 948–958.

Green, A. (2002). A dual conception of narcissism: Positive and negative organizations. *Psychoanalytic Quarterly, 71*(4), 631–649.

Grijalva, E., Newman, D. A., Tay, L., Donnellan, M. B., Harms, P. D., Robins, R. W., et al. (2015). Gender differences in narcissism: A meta-analytic review. *Psychological Bulletin, 141*(2), 261–310.

Grilo, C. M., McGlashan, T. H., & Quinlan, D. M. (1998). Frequency of personality disorders in two age cohorts of psychiatric inpatients. *American Journal of Psychiatry, 155*(1), 140–142.

Grover, K., Carpenter, L., Price, L., Gagne, G., Feijo-Mello, A., Mello, M., et al. (2007). The relationship between childhood abuse and adult personality disorder symptoms. *Journal of Personality Disorders, 21*, 442–447.

Gunderson, J. G., & Ronningstam, E. (2001). Differentiating narcissistic and antisocial personality disorders. *Journal of Personality Disorders, 15*(2), 103–109.

Gunderson, J. G., Ronningstam, E., & Smith, L. (1991). Narcissistic personality disorder: A review of data on DSM-III-R descriptions. *Journal of Personality Disorders, 5*(2), 167–177.

Gunderson, J. G., Ronningstam, E., & Smith, L. (1996). Narcissistic personality disorder. In T. A. Widiger, A. J. Frances, H. A. Pincus, R. Ross, M. B. First, & W. W. David (Eds.), *DSM-IV sourcebook* (Vol. 2, pp. 745–746). Washington, DC: American Psychiatric Association.

Hamilton, C. E. (2000). Continuity and discontinuity of attachment from infancy through adolescence. *Child Development, 71*(3), 690–694.

Hare, R. D. (1996). Psychopathy and antisocial personality disorder: A case of diagnostic confusion. *Psychiatric Times, 13*(2), 39–40.

Hare, R. D. (1999). *Without conscience: The disturbing world of the psychopaths among us*. New York: Guilford Press.

Hart, S. D., & Hare, R. D. (1998). Association between psychopathy and narcissism: Theoretical views and empirical evidence. In E. Ronningstam (Ed.), *Disorders*

of narcissism: Diagnostic, clinical, and empirical implications (pp. 415–436). Washington, DC: American Psychiatric Press.

Haslam, N., Holland, E., & Kuppens, P. (2012). Categories versus dimensions in personality and psychopathology: A quantitative review of taxometric research. *Psychological Medicine, 42*, 903–920.

Hatcher, R. L., & Barends, A. W. (1996). Patients' view of the alliance in psychotherapy: Exploratory factor analysis of three alliance measures. *Journal of Consulting and Clinical Psychology, 64*(6), 1326–1336.

Heimann, P. (1956). Dynamics of transference interpretations. International *Journal of Psychoanalysis, 37*, 303–310.

Heisel, M. J., Links, P. S., Conn, D., van Reekum, R., & Flett, G. L. (2007). Narcissistic personality and vulnerability to late-life suicidality. *American Journal of Geriatric Psychiatry, 15*(9), 734–741.

Hendin, H. M., & Cheek, J. M. (1997). Assessing hypersensitive narcissism: A reexamination of Murray's Narcissism Scale. *Journal of Research in Personality, 31*(4), 588–599.

Hengartner, M. P., Ajdacic-Gross, V., Rodgers, S., Müller, M., & Rössler, W. (2013). Childhood adversity in association with personality disorder dimensions: New findings in an old debate. *European Psychiatry, 28*(8), 476–482.

Hesse, E. (1996). Discourse, memory, and the Adult Attachment Interview: A note with emphasis on the emerging cannot classify category. *Infant Mental Health Journal, 17*(1), 4–11.

Hesse, E. (2008). The Adult Attachment Interview: Protocol, method of analysis and empiricalstudies. In J. Cassidy & P. R. Shaver (Eds.), *Handbook of attachment: Theory, research, and clinical applications* (pp. 552–598). New York: Guilford Press.

Hesse, E., & Main, M. (2000). Disorganized infant, child, and adult attachment: Collapse in behavioral and attentional strategies. *Journal of the American Psychoanalytic Association, 48*(4), 1097–1127.

Hilsenroth, M. J., Holdwick, D. J., Jr., Castlebury, F. D., & Blais, M. A. (1998). The effects of DSM-IV cluster B personality disorder symptoms on the termination and continuation of psychotherapy. *Psychotherapy: Theory, Research, Practice, Training, 35*(2), 163–176.

Holdwick, D. J., Jr., Hilsenroth, M. J., Castlebury, F. D., & Blais, M. A. (1998). Identifying the unique and common characteristics among the DSM-IV antisocial, borderline, and narcissistic personality disorders. *Comprehensive Psychiatry, 39*(5), 277–286.

Horowitz, M., 2009. Clinical phenomenology of narcissistic pathology. *Psychiatric Annals, 39*, 124–128.

Horton, R. S., Bleau, G., & Drwecki, B. (2006). Parenting narcissus: What are the links between parenting and narcissism? *Journal of Personality, 74*(2), 345–376.

Hörz-Sagstetter, S., Diamond, D., Clarkin, J. F., Levy, K. N., Rentrop, M., Fischer-Kern, M., et al. (2018). Clinical characteristics of comorbid narcissistic personality disorder in patients with borderline personality disorder. *Journal of Personality Disorders, 32*(4), 562–575.

Hughes, T. (1997). *Tales from Ovid*. London: Faber & Faber.

Hull, J. W., Clarkin, J. F., & Yeomans, F. (1993). Borderline personality disorder and impulsive sexual behavior. *Psychiatric Services, 44*(10), 1000–1001.

Huprich, S. K., Lengu, K., & Evich, C. (2016). Interpersonal problems and their relationship to depression, self-esteem, and malignant self-regard. *Journal of Personality Disorders, 30*(6), 742–761.

Hyatt, C. S., Sleep, C. E., Lynam, D. R., Widiger, T. A., Campbell, W. K., & Miller, J. D. (2017). Ratings of affective and interpersonal tendencies differ for grandiose and vulnerable narcissism: A replication and extension of Gore and Widiger. *Journal of Personality, 86*(3), 422–434.

Imbessi, L. (1999). The making of a narcissist. *Clinical Social Work Journal, 27*(1), 41–54.

Jacobson, E. (1964). *The self and the object world*. New York: International Universities Press.

Jacobson, E. (1971). Acting out and the urge to betray in paranoid patients. In *Depression: Comparative studies of normal, neurotic, and psychotic conditions* (pp. 302–318). New York: International Universities Press.

James, H. (2003). *The portrait of a lady*. New York: Penguin Classics. (Original work published 1881)

James, H. (2005). *The beast in the jungle*. Urbana, Illinois: Project Gutenberg. (Original work published 1903). Retrieved from https://www.gutenberg.org/files/1093/1093-h/1093-h.htm

Jang, K. L., Livesley, W. J., Vernon, P. A., & Jackson, D. N. (1996). Heritability of personality disorder traits: A twin study. *Acta Psychiatrica Scandinavica, 94*(6), 438–444.

Jauk, E., & Kaufman, S. B. (2018). The higher the score, the darker the core: The nonlinear association between grandiose and vulnerable narcissism. *Frontiers in Psychology, 9*, 1305.

Johnson, J. G., Cohen, P., Smailes, E. M., Skodol, A. E., Brown, J., & Oldham, J. M. (2001). Childhood verbal abuse and risk for personality disorders during adolescence and early adulthood. *Comprehensive Psychiatry, 42*(1), 16–23.

Johnson, J. G., Smailes, E. M., Cohen, P., Brown, J., & Bernstein, D. P. (2000). Associations between four types of childhood neglect and personality disorder symptoms during adolescence and early adulthood: Findings of a community-based longitudinal study. *Journal of Personality Disorders, 14*(2), 171–187.

Joseph, B. (1959). An aspect of the repetition compulsion. *International Journal of Psychoanalysis, 40*, 213–222.

Joseph, B. (1985). Transference: The total situation. *International Journal of Psychoanalysis, 66*(4), 447–454.

Joseph, B. (1989). *Psychic equilibrium and psychic change: Selected papers of Betty Joseph*. London & New York: Routledge.

Joseph, B. (2013). Here and now: My perspective. *The International Journal of Psychoanalysis, 94*(1), 1–5.

Jurist, E. L. (2018). Minds and yours: New directions for mentalization theory. In E. L. Jurist, A. Slade, & S. Bergner (Eds.), *Mind to mind: Infant research, neuroscience, and psychoanalysis* (pp. 88–114). New York: Other Press.

Kanninen, K., Salo, J., & Punamäki, R. L. (2000). Attachment patterns and working alliance in trauma therapy for victims of political violence. *Psychotherapy Research, 10*(4), 435–449.

Karan, E., Diamond, D., Grinband, J., & Fertuck, E. A. (2019). Perceptual sensitivity to facial self perception associated with pathological narcissism. *Psychoanalytic Psychology, 36*(4), 303–312.

Kealy, D., & Ogrodniczuk, J. S. (2011). Narcissistic interpersonal problems in clinical practice. *Harvard Review of Psychiatry, 19*(6), 290–301.

Kealy, D., & Ogrodniczuk, J. S. (2014). Pathological narcissism and the obstruction of love. *Psychodynamic Psychiatry, 42*(1), 101–119.

Kealy, D., Ogrodniczuk, J. S., Joyce, A. S., Steinberg, P. I., & Piper, W. E. (2015). Narcissism and relational representations among psychiatric outpatients. *Journal of Personality Disorders, 29*(3), 393–407.

Kelsey, R. M., Ornduff, S. R., Reiff, S., & Arthur, C. M. (2002). Psychophysiological correlates of narcissistic traits in women during active coping. *Psychophysiology, 39*(3), 322–332.

Kendler, K. S., Aggen, S. H., Czajkowski, N., Røysamb, E., Tambs, K., Torgersen, S., et al. (2008). The structure of genetic and environmental risk factors for DSM-IV personality disorders: A multivariate twin study. *Archives of General Psychiatry, 65*(12), 1438–1446.

Kernberg, O. F. (1979). Psychoanalytic psychotherapy with borderline adolescents. *Adolescent Psychiatry, 7*, 294–321.

Kernberg, O. F. (1974). Further contributions to the treatment of narcissistic personalities. *International Journal of Psychoanalysis, 55*(2), 215–240.

Kernberg, O. F. (1975). *Borderline conditions and pathological narcissism*. New York: Jason Aronson.

Kernberg, O. F. (1976). *Object relations theory and clinical psychoanalysis*. Northvale, NJ: Jason Aronson.

Kernberg, O. F. (1984). *Severe personality disorders: Psychotherapeutic strategies*. New Haven, CT: Yale University Press.

Kernberg, O. F. (1986). Factors in the psychoanalytic treatment of narcissistic personalities. In A. Morrison (Ed.), *Essential papers on narcissism*. New York: New York University Press.

Kernberg, O. F. (1970). Factors in the psychoanalytic treatment of narcissistic personalities. *Journal of the american psychoanalytic association, 18*(1), 51–85.

Kernberg O. F. (1992). *Aggression in personality disorders and perversions*. New Haven, CT: Yale University Press.

Kernberg, O. F. (1993). The current status of psychoanalysis. *Journal of the American Psychoanalytic Association, 41*(1), 45–62.

Kernberg, O. F. (1994). *Internal world and external reality: Object relations theory applied*. New Haven, CT: Yale University Press.

Kernberg, O. F. (1995). Latency, group dynamics, and conventionality. In *Love relations: Normality and pathology* (pp. 143–162). New Haven, CT: Yale University Press.

Kernberg, O. F. (1997). The nature of interpretation: Intersubjectivity and the third position. *American Journal of Psychoanalysis, 57*(4), 297–312.

Kernberg, O. F. (1999). *Ideology, conflict, and leadership in groups and organizations*. New Haven, CT: Yale University Press.

Kernberg, O. F. (2003a). Sanctioned social violence: A psychoanalytic view. Part I. *International Journal of Psychoanalysis, 84*(3), 683–698.

Kernberg, O. F. (2003b). Sanctioned social violence: A psychoanalytic view. Part II. *International Journal of Psychoanalysis, 84*(4), 953–968.

Kernberg, O. F. (2004). *Aggressivity, narcissism and self-destructiveness in the psychotherapeutic relationship*. New Haven, CT: Yale University Press.

Kernberg, O. F. (2006). Identity: Recent findings and clinical implications. *Psychoanalytic Quarterly, 75*(4), 969–1004.

Kernberg, O. F. (2007). The almost untreatable narcissistic patient. *Journal of the American Psychoanalytic Association, 55*(2), 503–539.

Kernberg, O. F. (2008a). *Contemporary controversies in psychoanalytic theory, techniques, and their applications*. New Haven, CT: Yale University Press.

Kernberg, O. F. (2008b). The destruction of time in pathological narcissism. *International Journal of Psychoanalysis, 89*(2), 299–312.

Kernberg, O. F. (2009). Narcissistic personality disorders: Part 1 [Editorial]. *Psychiatric Annals, 39*(3), 105–107, 110, 164–166.

Kernberg, O. F. (2010). Narcissistic personality disorder. In J. F. Clarkin, P. Fonagy, & G. O. Gabbard (Eds.), *Psychodynamic psychotherapy for personality disorders: A clinical handbook* (pp. 257–287). Arlington, VA: American Psychiatric Association.

Kernberg, O. F. (2012). *The inseparable nature of love and aggression: Clinical and theoretical perspectives*. Arlington, VA: American Psychiatric Association.

Kernberg, O. F. (2014). An overview of the treatment of severe narcissistic pathology. *International Journal of Psychoanalysis, 95*(5), 865–888.

Kernberg, O. F. (2015). Narcissistic defenses in the distortion of free association and their underlying anxieties. *Psychoanalytic Quarterly, 84*(3), 625–642.

Kernberg, O. F. (2018). *Treatment of severe personality disorders: Resolution of aggression and recovery of eroticism*. Washington, DC: American Psychiatric Association.

Kernberg, O. F. (2019). Therapeutic implications of transference structures in various personality pathologies. *Journal of the American Psychoanalytic Association, 67*(6), 951–986.

Kernberg, O. F., & Caligor, E. (2005). A psychoanalytic theory of personality disorders. In M. F. Lenzenweger & J. F. Clarkin (Eds.), *Major theories of personality disorder* (2nd ed., pp. 114–156). New York: Guilford Press.

Kernberg, O. F., Diamond, D., Yeomans, F. E., Clarkin, J. F., & Levy, K. N. (2008). Mentalization and attachment in borderline patients in transference focused psychotherapy. In E. L. Jurist, A. Slade, & S. Bergner (Eds.), *Mind to mind: Infant research, neuroscience, and psychoanalysis* (pp. 167–201). New York: Other Press.

Kernberg, O. F., Selzer, M. A., Koenigsberg, H. W., Carr, A. C., & Appelbaum, A. H. (1989). *Psychodynamic psychotherapy of borderline patients*. New York: Basic Books.

Kernberg, P. F. (1989). Narcissistic personality disorder in childhood. *Psychiatric Clinics of North America, 12*(3), 671–694.

Klein, M. (1946). Notes on some schizoid mechanisms. *International Journal of Psychoanalysis, 27*, 99–110.

Klein, M. (1950). On the criteria for the termination of a psycho-analysis. *International Journal of Psychoanalysis, 31*, 78–80.

Klein, M. (1957). *Envy and gratitude*. New York: Basic Books.

Klonsky, E. D., & Oltmanns, T. F. (2002). Informant-reports of personality disorder: Relation to self-reports and future research directions. *Clinical Psychology: Science and Practice, 9*(3), 300–311.

Koenigsberg, H., Kernberg, O., Appelbaum, A., Stone, M., Yeomans, F., & Diamond, D. (2000). *Borderline patients: Extending the limits of treatability*. New York: Basic Books.

Kohut, H. (1966). Forms and transformations of narcissism. *Journal of the American Psychoanalytic Association, 14*(2), 243–272.
Kohut, H. (1971). *The analysis of the self*. New York: International Universities Press.
Kohut, H. (1977). *The restoration of the self*. New York: International Universities Press.
Kolb, B., & Whishaw, I. Q. (2003). *Fundamentals of human neuropsychology* (5th ed.). New York: Worth.
Konrath, S., Bushman, B. J., & Grove, T. (2009). Seeing my world in a million little pieces: Narcissism, self-construal, and cognitive-perceptual style. *Journal of Personality, 77*(4), 1197–1228.
Konrath, S., Corneille, O., Bushman, B. J., & Luminet, O. (2014). The relationship between narcissistic exploitativeness, dispositional empathy, and emotion recognition abilities. *Journal of Nonverbal Behavior, 38*(1), 129–143.
Kosson, D. S., Suchy, Y., Mayer, A. R., & Libby, J. (2002). Facial affect recognition in criminal psychopaths. *Emotion, 2*(4), 398–411.
Krause, A. L., Borchardt, V., Li, M., van Tol, M. J., Demenescu, L. R., Strauss, B., ... & Walter, M. (2016). Dismissing attachment characteristics dynamically modulate brain networks subserving social aversion. *Frontiers in Human Neuroscience, 10*, 77.
Lachmann, F. M. (2007). *Transforming narcissism: Reflections on empathy, humor, and expectations*. New York: Analytic Press.
LaFarge, L. (1997). An object relations theory approach to psychoanalysis with narcissistic patients. In E. F. Ronningstam (Ed.), *Disorders of narcissism: diagnostic, clinical, and empirical implications* (pp. 171). Washington, DC: American Psychiatric Association
Lasch, C. (1979). *The culture of narcissism: American life in an age of diminishing expectations*. New York: Norton.
LeDoux, J. E. (1996). *The emotional brain*. New York: Simon & Schuster.
LeDoux, J. E. (2002). *Synaptic self: How our brains become who we are*. New York: Viking.
Legerstee, M. (2005). *Infants' sense of people: Precursors to a theory of mind*. New York: Cambridge University Press.
Lenzenweger, M. F., Clarkin, J. F., Caligor, E., Cain, N. M., & Kernberg, O. F. (2018). Malignant narcissism in relation to clinical change in borderline personality disorder: An exploratory study. *Psychopathology, 51*(5), 318–325.
Lequesne, E. R., & Hersh, R. G. (2004). Disclosure of a diagnosis of borderline personality disorder. *Journal of Psychiatric Practice, 10*(3), 170–176.
Leunissen, J. M., Sedikides, C., & Wildschut, T. (2017). Why narcissists are unwilling to apologize: The role of empathy and guilt. *European Journal of Personality, 31*(4), 385–403.
Levine, H. (2002). *Intrapsychic and symptomatic change in patients with borderline psychopathology*. Unpublished doctoral dissertation, City University of New York.
Levy, K., Kivity, Y., Diamond, D., Kernberg, O. F., & Clarkin, J. (2018). The impact of narcissism on dropout in three treatments for borderline personality disorder.
Levy, K. N., Beeney, J. E., & Temes, C. M. (2011). Attachment and its vicissitudes in borderline personality disorder. *Current Psychiatry Reports, 13*(1), 50–59.
Levy, K. N., Meehan, K. B., Kelly, K. M., Reynoso, J. S., & Weber, M. (2006).

Change in attachment patterns and reflective function in a randomized control trial of transference focused psychotherapy for borderline personality disorder. *Journal of Consulting and Clinical Psychology, 74*(6), 1027–1040.

Lewis, M., Feiring, C., & Rosenthal, S. (2000). Attachment over time. *Child Development, 71*(3), 707–720.

Lichtenberg, J. D. (1988). A theory of motivational–functional systems as psychic structures. *Journal of the American Psychoanalytic Association, 36*(Suppl.), 57–72.

Lichtenberg, J. D. (2003). Communication in infancy. *Psychoanalytic Inquiry, 23*(3), 498–520.

Lichtenberg, J. D., Lachmann, F., & Fosshage, J. (2011). *Psychoanalysis and motivational systems: A new look*. New York: Routledge.

Lingiardi, V., Lonati, C., Delucchi, F., Fossati, A., Vanzulli, L., & Maffei, C. (1999). Defense mechanisms and personality disorders. *Journal of Nervous and Mental Disease, 187*(4), 224–228.

Lingiardi, V., & McWilliams, N. (2015). The psychodynamic diagnostic manual - 2nd edition (PDM-2). *World Psychiatry: Official Journal of the World Psychiatric Association (WPA), 14*(2), 237–239.

Liotti, G. (2004). Trauma, dissociation, and disorganized attachment: Three strands of a single braid. *Psychotherapy: Theory, Research, Practice, Training, 41*(4), 472–486.

Lobbestael, J., Van Vreeswijk, M. F., & Arntz, A. (2008). An empirical test of schema mode conceptualizations in personality disorders. *Behavior Research and Therapy, 46*(7), 854–860.

Loewald, H. (1960). On the therapeutic action of psychoanalysis. *International Journal of Psychoanalysis, 41*, 16–33.

Loewald, H. W. (1979). The waning of the Oedipus complex. *Journal of the American Psychoanalytic Association, 27*(4), 751–775.

Loranger, A. W. (1999). *International personality disorder examination (IPDE)*. Odessa, FL: Psychological Assessment Resources.

Luchner, A. F. (2013). Maintaining boundaries in the treatment of pathological narcissism. In J. S. Ogrodniczuk (Ed.), *Understanding and treating pathological narcissism* (pp. 219–234). Washington, DC: American Psychological Association.

Luchner, A. F., & Tantleff-Dunn, S. (2016). Dysfunctional empathy in vulnerable narcissism. *North American Journal of Psychology, 18*(3), 597–610.

Lunbeck, E. (2014). *The Americanization of narcissism*. Cambridge, MA: Harvard University Press.

Lyons-Ruth, K., & Spielman, E. (2004). Disorganized infant attachment strategies and helpless–fearful profiles of parenting: Integrating attachment research with clinical intervention. *Infant Mental Health Journal, 25*(4), 318–335.

Magnavita, J. J., Levy, K. N., Critchfield, K. L., & Lebow, J. L. (2010). Ethical considerations in treatment of personality dysfunction: Using evidence, principles, and clinical judgment. *Professional Psychology: Research and Practice, 41*(1), 64–74.

Mahler, M., Pine, F., & Bergman, A. (1975). *The psychological birth of the infant: Symbiosis and individuation*. New York: Basic Books.

Main, M. (1991). Metacognitive knowledge, metacognitive monitoring, and singular (coherent) vs. multiple (incoherent) models of attachment. In C. M. Parkes,

J. Stevenson-Hinde, & P. Marris (Eds.), *Attachment across the life cycle* (pp. 127–159). New York: Routledge.

Main, M. (1999). Epilogue. Attachment theory: Eighteen points with suggestions for future studies. In J. Cassidy & P. R. Shaver (Eds.), *Handbook of attachment: Theory, research, and clinical applications* (pp. 845–887). New York: Guilford Press.

Main, M., & Goldwyn, R. (1998). *Adult attachment scoring and classification system, Version 6.0*. Unpublished manuscript, University of California, Berkeley.

Main, M., Hesse, E., & Goldwyn, R. (2008). Studying differences in language usage in recounting attachment history: An introduction to the AAI. In H. Steele & M. Steele (Eds.), *Clinical applications of the adult attachment interview* (pp. 31–68). The Guilford Press.

Main, M., Kaplan, N., & Cassidy, J. (1985). Security in infancy, childhood, and adulthood: A move to the level of representation. *Monographs of the Society for Research in Child Development, 50*(1–2), 66–104.

Maldonado, J. L. (1999). Narcissistic resistances in the analytic experience. *International Journal of Psychoanalysis, 80*(Pt. 6), 1131–1146.

Maldonado, J. L. (2006). Vicissitudes in adult life resulting from traumatic experiences in adolescence. *International Journal of Psychoanalysis, 87*(5), 1239–1257.

Marissen, M. A. E., Deen, M. L., & Franken, I. H. A. (2012). Disturbed emotion recognition in patients with narcissistic personality disorder. *Psychiatry Research, 198*(2), 269–273.

Maxwell, K., & Huprich, S. (2014). Retrospective reports of attachment disruptions, parental abuse and neglect mediate the relationship between pathological narcissism and self-esteem. *Personality and Mental Health, 8*(4), 290–305.

Mayes, L. C. (2006). Arousal regulation, emotional flexibility, medial amygdala function, and the impact of early experience. *Annals of the New York Academy of Sciences, 1094*(1), 178–192.

Meyer, B., & Pilkonis, P. A. (2011). Attachment theory and narcissistic personality disorder. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 434–444). Hoboken, NJ: Wiley.

Mikulincer, M., Shaver, P. R., Cassidy, J., & Berant, E. (2009). Attachment-related defensive processes. In J. H. Obegi & E. Berant (Eds.), *Attachment theory and research in clinical work with adults* (pp. 293–327). New York: Guilford Press.

Miller, J. D., & Campbell, W. K. (2008). Comparing clinical and social-personality conceptualizations of narcissism. *Journal of Personality, 76*(3), 449–476.

Miller, J. D., Campbell, W. K., & Pilkonis, P. A. (2007). Narcissistic personality disorder: Relations with distress and functional impairment. *Comprehensive Psychiatry, 48*(2), 170–177.

Miller, J. D., Dir, A., Gentile, B., Wilson, L., Pryor, L. R., & Campbell, W. K. (2010). Searching for a vulnerable dark triad: Comparing factor 2 psychopathy, vulnerable narcissism, and borderline personality disorder. *Journal of Personality, 78*(5), 1529–1564.

Miller, J. D., Hoffman, B. J., Gaughan, E. T., Gentile, B., Maples, J., & Campbell, W. (2011). Grandiose and vulnerable narcissism: A nomological network analysis. *Journal of Personality, 79*(5), 1013–1042.

Miller, J. D., Lynam, D. R., & Campbell, W. K. (2016). Rejoinder: A construct validity approach to the assessment of narcissism. *Assessment, 23*(1), 18–22.

Miller, J. D., Lynam, D. R., Hyatt, C. S., & Campbell, W. K. (2017). Controversies in Narcissism. *Annu Rev Clin Psychol*, 2017.

Miller, J. D., Lynam, D. R., Vize, C., Crowe, M., Sleep, C., Maples-Keller, J. L., et al. (2018). Vulnerable narcissism is (mostly) a disorder of neuroticism. *Journal of Personality, 86*(2), 186–199.

Miller, J. D., & Maples, J. (2011). Trait personality models of narcissistic personality disorder, grandiose narcissism, and vulnerable narcissism. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 71–88). Hoboken, NJ: Wiley.

Millon, T. (1981). *Disorders of the personality.* New York: Wiley.

Millon, T. (1996). *Disorders of personality: DSM-IV and beyond*. New York: Wiley.

Mitchell, S. A. (1986). The wings of Icarus. *Contemporary Psychoanalysis, 22*(1), 107–132.

Mitchell, S. A. (1999). Attachment theory and the psychoanalytic tradition: Reflections on human relationality. *Psychoanalytic Dialogues, 9*(1), 85–107.

Modell, A. H. (1975). A narcissistic defence against affects and the illusion of self-sufficiency. *International Journal of Psychoanalysis, 56*(3), 275–282.

Modell, A. H. (1980). Affects and their non-communication. *International Journal of Psychoanalysis, 61*(2), 259–267.

Mondloch, C. J., Lewis, T. L., Budreau, D. R., Maurer, D., Dannemiller, J. L., Stephens, B. R., et al. (1999). Face perception during early infancy. *Psychological Science, 10*(5), 419–422.

Money-Kyrle, R. E. (1965). Megalomania. *American Imago, 22*(1–2), 142–154.

Morey, L. C. (1985). A psychometric analysis of five different DSM-III categories. *Personality and Individual Differences, 6*, 323–329.

Morf, C., & Rhodewalt, F. (2001). Unraveling the paradoxes of narcissism: A dynamic self-regulatory processing model. *Psychological Inquiry, 12*(4), 177–196.

Morf, C. C., & Rhodewalt, F. (1993). Narcissism and self-evaluation maintenance: Explorations in object relations. *Personality and Social Psychology Bulletin, 19*(6), 668–676.

Morf, C. C., Torchetti, L., & Schürch, E. (2011). Narcissism from the perspective of the dynamic self-regulatory processing model. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 56–70). Hoboken, NJ: Wiley.

Nenadic, I., Smesny, S., Schlosser, R. G., Sauer, H., & Gaser, C. (2010). Auditory hallucinations and brain structure in schizophrenia: Voxel-based morphometric study. *British Journal of Psychiatry, 196*(5), 412–413.

Northoff, G., Heinzel, A., De Greck, M., Bermpohl, F., Dobrowolny, H., & Panksepp, J. (2006). Self-referential processing in our brain—A meta-analysis of imaging studies on the self. *Neuroimage, 31*(1), 440–457.

Ochsner, K. N., & Gross, J. J. (2005). The cognitive control of emotion. *Trends in Cognitive Sciences, 9*(5), 242–249.

Ogden, T. H. (1994). The analytic third: Working with intersubjective clinical facts. *The International Journal of Psychoanalysis, 75*(1), 3–19.

Ogden, T. H. (2004). The analytic third: Implications for psychoanalytic theory and technique. *The Psychoanalytic Quarterly, 73*(1), 167–195.

Otway, L. J., & Vignoles, V. L. (2006). Narcissism and childhood recollections: A quantitative test of psychoanalytic predictions. *Personality and Social Psychology Bulletin, 32*(1), 104–116.

Panksepp, J. (2003). At the interface of the affective, behavioral, and cognitive neurosciences: Decoding the emotional feelings of the brain. *Brain and Cognition, 52*(1), 4–14.

Panksepp, J. (2008). The affective brain and core consciousness: How does neural activity generate emotional feelings? In M. Lewis, J. M. Haviland-Jones, & L. F. Barrett (Eds.), *Handbook of emotions* (pp. 47–67). New York: Guilford Press.

Panksepp, J., & Biven, L. (2012). *The archaeology of mind: Neuroevolutionary origins of human emotions*. New York: Norton.

Paris, J. (2014). Modernity and narcissistic personality disorder. *Personality Disorders: Theory, Research, and Treatment, 5*(2), 220–226.

Parish, M., & Eagle, M. N. (2003). Attachment to the therapist. *Psychoanalytic Psychology, 20*(2), 271–286.

Patrick, M., Hobson, R. P., Castle, D., Howard, R., & Maughan, B. (1994). Personality disorder and the mental representation of early social experience. *Development and Psychopathology, 6*(2), 375–388.

Paulus, C. (2009). Der saarbrücker persönlichkeitsfragebogen SPF (IRI) zur messung von empathie: Psychometrische evaluation der deutschen version des interpersonal reactivity index. Available online at: http://hdl.handle.net/20.500.11780/3343

PDM Task Force. (2006). *Psychodynamic diagnostic manual*. Silver Spring, MD: Alliance of Psychoanalytic Organizations.

Perry, J. C. (1990). *The Defense Mechanism Rating Scale manual* (5th ed.). Copyright by J. C. Perry, M. D., Cambridge, MA.

Perry, J. C. (2014). Phenomenology in need of treatment: Commentary for the special series "Narcissistic personality disorder—New perspectives on diagnosis and treatment." *Personality Disorders: Theory, Research, and Treatment, 5*(4), 446–447.

Perry, J. C., & Cooper, S. H. (1989). An empirical study of defense mechanisms: I. Clinical interview and life vignette ratings. *Archives of General Psychiatry, 46*(5), 444–452.

Perry, J. D., & Perry, J. C. (1996). Reliability and convergence of three concepts of narcissistic personality. *Psychiatry, 59*(1), 4–19.

Perry, J. D., & Perry, J. C. (2004). Conflicts, defenses and the stability of narcissistic personality features. *Psychiatry: Interpersonal and Biological Processes, 67*(4), 310–330.

Perry, J. C., Presniak, M. D., & Olson, T. R. (2013). Defense mechanisms in schizotypal, borderline, antisocial, and narcissistic personality disorders. *Psychiatry: Interpersonal and Biological Processes, 76*(1), 32–52.

Pfohl, B., Coryell, W., Zimmerman, M., & Stangl, D. (1986). DSM-III personality disorders: Diagnostic overlap and internal consistency of individual DSM-III criteria. *Comprehensive Psychiatry, 27*(1), 21–34.

Pincus, A. L., Ansell, E. B., Pimentel, C. A., Cain, N. M., Wright, A. G. C., & Levy, K. N. (2009). Initial construction and validation of the Pathological Narcissism Inventory. *Psychological Assessment, 21*(3), 365–379.

Pincus, A. L., Cain, N. M., & Wright, A. G. (2014). Narcissistic grandiosity and narcissistic vulnerability in psychotherapy. *Personality Disorders: Theory, Research, and Treatment, 5*(4), 439–443.

Pincus, A. L., & Lukowitsky, M. R. (2010). Pathological narcissism and narcissistic personality disorder. *Annual Review of Clinical Psychology, 6*, 421–446.

Pincus, A. L., & Roche, M. J. (2011). Narcissistic grandiosity and narcissistic vulnerability. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 31–40). Hoboken, NJ: Wiley.

Preston, S. D., & de Waal, F. B. (2002). Empathy: Its ultimate and proximate bases. *Behavioral and Brain Sciences, 25*(1), 1–20.

Pulay, A. J., Goldstein, R. B., & Grant, B. F. (2011). Sociodemographic correlates of DSM-IV narcissistic personality disorder: Results from the Wave 2 National Epidemiologic Survey on Alcohol and Related Conditions (NESARC). In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 165–180). Hoboken, NJ: Wiley.

Raby, K. L., Cicchetti, D., Carlson, E. A., Egeland, B., & Collins, W. A. (2013). Genetic contributions to continuity and change in attachment security: A prospective, longitudinal investigation from infancy to young adulthood. *Journal of Child Psychology and Psychiatry, and Allied Disciplines, 54*(11), 1223–1230.

Racker, H. (2018). *Transference and countertransference*. New York: Routledge.

Raskin, R. N., & Hall, C. S. (1979). A narcissistic personality inventory. *Psychological Reports, 45*(2), 590.

Raskin, R., & Hall, C. S. (1981). The Narcissistic Personality Inventory: Alternate form reliability and further evidence of construct validity. *Journal of Personality Assessment, 45*(2), 159–162.

Raskin, R., & Terry, H. (1988). A principal-components analysis of the Narcissistic Personality Inventory and further evidence of its construct validity. *Journal of Personality and Social Psychology, 54*(5), 890–902.

Rathvon, N., & Holmstrom, R. W. (1996). An MMPI-2 portrait of narcissism. *Journal of Personality Assessment, 66*(1), 1–19.

Reddy, V. (2005). Feeling shy and showing-off: Self-conscious emotions must regulate self-awareness. In J. Nadel & D. Muir (Eds.), *Emotional development: Recent research advances* (pp. 183–204). New York: Oxford University Press.

Reinhard, D. A., Konrath, S. H., Lopez, W. D., & Cameron, H. G. (2012). Correction. Expensive egos: Narcissistic males have higher cortisol. *PloS One, 7*(7).

Ringstrom, P. (2012). A relational intersubjective approach to conjoint treatment. *International Journal of Psychoanalytic Self Psychology, 7*(1), 85–111.

Ringstrom, P. (2014). *A relational approach to couples therapy*. New York: Routledge.

Ripoll, L. H., Snyder, R., Steele, H., & Siever, L. J. (2013). The neurobiology of empathy in borderline personality disorder. *Current Psychiatry Reports, 15*(3), 344.

Ritter, K., Dziobek, I., Preissler, S., Rüter, A., Vater, A., Fydrich, T., et al. (2011). Lack of empathy in patients with narcissistic personality disorder. *Psychiatry Research, 187*(1–2), 241–247.

Ritter, K., Vater, A., Rüsch, N., Schröder-Abé, M., Schütz, A., Fydrich, T., et al. (2014). Shame in patients with narcissistic personality disorder. *Psychiatry Research, 215*(2), 429–437.

Riviere, J. (1936). A contribution to the analysis of the negative therapeutic reaction. *International Journal of Psychoanalysis, 17*, 304–320.

Roche, M. J., Pincus, A. L., Conroy, D. E., Hyde, A. L., & Ram, N. (2013). Pathological narcissism and interpersonal behavior in daily life. Personality disorders: Theory, research, and treatment. *Journal of Personality Disorders, 27*, 270–295.

Ronningstam, E. (2000). *Disorders of narcissism—diagnostic, clinical, and empirical implications*. Northvale, NJ: Jason Aronson.

Ronningstam, E. (2005). *Identifying and understanding the narcissistic personality*. Oxford, UK: Oxford University Press.

Ronningstam, E. (2009). Narcissistic personality disorder: Facing DSM-V. *Psychiatric Annals, 39*(3), 111–121.

Ronningstam, E. (2010). Narcissistic personality disorder: A current review. *Current Psychiatry Reports, 12*(1), 68–75.

Ronningstam, E. (2011). Narcissistic personality disorder in DSM-V—In support of retaining a significant diagnosis. *Journal of Personality Disorders, 25*(2), 248–259.

Ronningstam, E. (2012). Alliance building and narcissistic personality disorder. *Journal of Clinical Psychology, 68*(8), 943–953.

Ronningstam, E. (2013). An update on narcissistic personality disorder. *Current Opinion in Psychiatry, 26*(1), 102–106.

Ronningstam, E. (2014). Beyond the diagnostic traits: A collaborative exploratory diagnostic process for dimensions and underpinnings of narcissistic personality disorder. *Personality Disorders: Theory, Research, and Treatment, 5*(4), 434–438.

Ronningstam, E. (2016). Pathological narcissism and narcissistic personality disorder: Recent research and clinical implications. *Current Behavioral Neuroscience Reports, 3*(1), 34–42.

Ronningstam, E. (2018). *NPD basic: A brief overview of identifying, diagnosing and treating narcissistic personality disorder* (3rd ed.). Washington Township, NJ: National Education Alliance for Borderline Personality Disorder.

Ronningstam, E., & Gunderson, J. (1991). Differentiating borderline personality disorder from narcissistic personality disorder. *Journal of Personality Disorders, 5*(3), 225–232.

Ronningstam, E., Gunderson, J., & Lyons, M. (1995). Changes in pathological narcissism. *American Journal of Psychiatry, 152*(2), 253–257.

Ronningstam, E., Weinberg, I., Goldblatt, M., Schechter, M., & Herbstman, B. (2018). Suicide and self-regulation in narcissistic personality disorder. *Psychodynamic Psychiatry, 46*(4), 491–510.

Ronningstam, E., Weinberg, I., & Maltsberger, J. T. (2008). Eleven deaths of Mr. K.—Contributing factors to suicide in narcissistic personalities. *Psychiatry: Interpersonal and Biological Processes, 71*(2), 169–182.

Ronningstam, E. F., & Maltsberger, J. T. (1998). Pathological narcissism and sudden suicide-related collapse. *Suicide Life Threatening Behavior. 28*(3), 261–271.

Rose, P. (2002). The happy and unhappy faces of narcissism. *Personality and Individual Differences, 33*(3), 379–392.

Rosenfeld, H. (1964). On the psychopathology of narcissism, a clinical approach. *International Journal of Psychoanalysis, 45*(2–3), 332–337.

Rosenfeld, H. (1971). A clinical approach to the psychoanalytic theory of life and death instincts: An investigation into the aggressive aspects of narcissism. *International Journal of Psychoanalysis, 52*(2), 169–178.

Rosenfeld, H. (1987). *Impasse and interpretation: Therapeutic and anti-therapeutic factors in the psychoanalytic treatment of psychotic, borderline, and neurotic patients*. New York: Routledge.

Rosenstein, D. S., & Horowitz, H. A. (1996). Adolescent attachment and psychopathology. *Journal of Consulting and Clinical Psychology, 64*(2), 244–253.

Rosenthal, S. A., Hooley, J. M., Montoya, R. M., van der Linden, S. L., & Steshenko, Y. (2020) The narcissistic grandiosity scale: A measure to distinguish narcissistic grandiosity from high self-esteem. *Assessment, 27*(3) 487–507.

Rudden, M., Milrod, B., Target, M., Ackerman, S., & Graf, E. (2006). Reflective functioning in panic disorder patients: A pilot study. *Journal of the American Psychoanalytic Association, 54*(4), 1339–1343.

Russ, E., Shedler, J., Bradley, R., & Westen, D. (2008). Refining the construct of narcissistic personality disorder: Diagnostic criteria and subtypes. *American Journal of Psychiatry, 165*(11), 1473–1481.

Rycroft, C. (1956). The nature and function of the analysts' communication to the patient. *International Journal of Psychoanalysis, 37*, 469–472.

Samuel, D. B., Lynam, D. R., Widiger, T. A., & Ball, S. A. (2012). An expert consensus approach to relating the proposed DSM-5 types and traits. *Personality Disorders: Theory, Research, and Treatment, 3*(1), 1–16.

Sandler, J. (1976). Countertransference and role-responsiveness. *International Review of Psychoanlysis, 3*(1), 43–47.

Sandler, J. (1987). *From safety to superego: Selected papers of Joseph Sandler*. New York: Guilford Press.

Sandler, J. (2003). On attachment to internal objects. *Psychoanalytic Inquiry, 23*(1), 12–26.

Sandler, J., & Rosenblatt, B. (1962). The concept of the representational world. *Psychoanalytic Study of the Child, 17*(1), 128–145.

Sandler, J., & Sandler, A. M. (1987). The past unconscious, the present unconscious and the vicissitudes of guilt. *International Journal of Psychoanalysis, 68*(3), 331–341.

Schafer, R. (1960). The loving and beloved superego in Freud's structural theory. *Psychoanalytic Study of the Child, 15*(1), 163–188.

Scharff, D., & Scharff, J. (1991). *Object relations couple therapy*. New York: Jason Aronson.

Schore, A. N. (1994). *Affect regulation and the origin of the self: The neurobiology of emotional development*. Hillsdale, NJ: Erlbaum.

Schulze, L., Dziobek, I., Vater, A., Heekeren, H. R., Bajbouj, M., Renneberg, B., et al. (2013). Gray matter abnormalities in patients with narcissistic personality disorder. *Journal of Psychiatric Research, 47*(10), 1363–1369.

Segal, H., & Bell, D. (1991). The theory of narcissism in the work of Freud and Klein. In J. Sandler, P. Fonagy, & E. Person (Eds.), *On narcissism: An introduction* (pp. 149–174). New Haven, CT: Yale University Press.

Sharp, C., & Wall, K. (2021). DSM-5 level of personality functioning: Refocusing

personality disorder on what It means to be human. *Annual Review of Clinical Psychology, 17*(1), 313–337.

Sharp, C., Wright, A. G., Fowler, J. C., Frueh, B. C., Allen, J. G., Oldham, J., et al. (2015). The structure of personality pathology: Both general ("g") and specific ("s") factors. *Journal of Abnormal Psychology, 124*(2), 387–398.

Shaw, D. (2010). Enter ghosts: The loss of intersubjectivity in clinical work with adult children of pathological narcissists. *Psychoanalytic Dialogues, 20*(1), 46–59.

Shaw, D. (2013). *Traumatic narcissism: Relational systems of subjugation*. New York: Routledge.

Simon, R. I. (2002). Distinguishing trauma-associated narcissistic symptoms from posttraumatic stress disorder: A diagnostic challenge. *Harvard Review of Psychiatry, 10*(1), 28–36.

Simonsen, S., & Simonsen, E. (2012). Comorbidity between narcissistic personality disorders and Axis I diagnoses. In W. K. Campbell & J. D. Miller (Eds.), *The handbook of narcissism and narcissistic personality disorder: Theoretical approaches, empirical findings, and treatments* (pp. 239–247). Hoboken, NJ: Wiley.

Singer, T. (2006). The neuronal basis and ontogeny of empathy and mind reading: Review of literature and implications for future research. *Neuroscience and Biobehavioral Reviews, 30*(6), 855–863.

Skodol, A. E., Bender, D. S., & Morey, L. C. (2014). Narcissistic personality disorder in DSM-5. *Personality Disorders: Theory, Research, and Treatment, 5*(4), 422–427.

Slade, A. (2008). The implications of attachment theory and research for adult psychotherapy: Research and clinical perspectives. In J. Cassidy & P. R. Shaver (Eds.), *Handbook of attachment: Theory, research, and clinical applications* (pp. 762–782). New York: Guilford Press.

Smolewska, K., & Dion, K. L. (2005). Narcissism and adult attachment: A multivariate approach. *Self and Identity, 4*(1), 59–68.

Sommer, K. L., Kirkland, K. L., Newman, S. R., Estrella, P., & Andreassi, J. L. (2009). Narcissism and cardiovascular reactivity to rejection imagery 1. *Journal of Applied Social Psychology, 39*(5), 1083–1115.

Sroufe, L. A., & Waters, E. (1977). Heart rate as a convergent measure in clinical and developmental research. *Merrill-Palmer Quarterly of Behavior and Development, 23*(1), 3–27.

Stechler, G. (2014). Too little or too much. *Psychoanalytic Inquiry, 34*(5), 383–393.

Steele, H., & Steele, M. (Eds.). (2008). *Clinical applications of the Adult Attachment Interview*. New York: Guilford Press.

Steele, H., Steele, M., & Murphy, A. (2009). Use of the Adult Attachment Interview to measure process and change in psychotherapy. *Psychotherapy Research, 19*(6), 633–643.

Steiner, J. (1994). Patient-centered and analyst-centered interpretations: Some implications of containment and countertransference. *Psychoanalytic Inquiry, 14*(3), 406–422.

Steiner, J. (2003). *Psychic retreats: Pathological organizations in psychotic, neurotic and borderline patients*. London: Routledge.

Steiner, J. (2006). Interpretative enactments and the analytic setting. *International Journal of Psychoanalysis, 87*(Pt. 2), 315–320.

Steiner, J. (2011). *Seeing and being seen: Emerging from psychic retreat*. New York: Routledge.

Stern, A. (1938). Psychoanalytic investigation of and therapy in the borderline group of neuroses. *Psychoanalytic Quarterly, 7*(4), 467–489.

Stern, A. (1979). *Me: The narcissistic American*. New York: Random House.

Stern, B. L., Caligor, E., Clarkin, J. F., Critchfield, K. L., Horz, S., MacCornack, V., Lenzenweger, M. F., Kernberg, O. F. (2010). Structured interview of personality organization (STIPO): Preliminary psychometrics in a clinical sample. *Journal of Personality Assessment, 92*(1), 35–44.

Stern, B. L., Yeomans, F., Diamond, D., & Kernberg, O. F. (2011). Transference-focused psychotherapy (TFP) for narcissistic personality disorder. In J. S. Ogrodniczuk (Ed.), *Understanding and treating narcissistic personality disorder* (pp. 235–252). Washington DC: American Psychiatric Press.

Stern, D. N. (1985). *The interpersonal world of the infant: A view from psychoanalysis and developmental psychology*. London: Karnac Books.

Steyn, L. (2013). Tactics and empathy: Defences against projective identification. *International Journal of Psychoanalysis, 94*(6), 1093–1113.

Stinson, F. S., Dawson, D. A., Goldstein, R. B., Chou, S. P., Huang, B., Smith, S. M., et al. (2008). Prevalence, correlates, disability, and comorbidity of DSM-IV narcissistic personality disorder: Results from the Wave 2 National Epidemiologic Survey on Alcohol and Related Conditions. *Journal of Clinical Psychiatry, 69*(7), 1033–1045.

Stone, M. H. (1989). Long-term follow-up of narcissistic/borderline patients. *Psychiatric Clinics, 12*(3), 621–641.

Stone, M. H. (1990). *The fate of borderline patients: Successful outcome and psychiatric practice*. New York: Guilford Press.

Stone, M. H. (2003). Borderline patients at the border of treatability: At the intersection of borderline, narcissistic, and antisocial personalities. *Journal of Psychiatric Practice, 9*(4), 279–290.

Stone, M. H. (2016). Long-term course of borderline personality disorder. *Psychodynamic Psychiatry, 44*(3), 449–474.

Storebø, O., Stoffers-Winterling, J., Völlm, B., Kongerslev, M. T., Mattivi, J., Jørgensen, M., et al. (2020). Psychological therapies for people with borderline personality disorder (Review). *Cochrane Database of Systematic Reviews* (Online). *5*.

Symington, N. (1993). *Narcissism: A new theory*. London: Karnac Books.

Talia, A., Daniel, S. I. F., Miller-Bottome, M., Brambilla, D., Miccoli, D., Safran, J. D., & Lingiardi, V. (2014). AAI predicts patients' in-session interpersonal behavior and discourse: A "move to the level of the relation" for attachment-informed psychotherapy research. *Attachment and Human Development, 16*(2), 192–209.

Talia, A., Miller-Bottome, M., & Daniel, S. I. (2017). Assessing attachment in psychotherapy: Validation of the Patient Attachment Coding System (PACS). *Clinical Psychology and Psychotherapy, 24*(1), 149–161.

Talia, A., Taubner, S., Hauschild, S., Duschinsky, R., & Volkert, J. (2018). *Forty-four unresolved/disorganized patients: Their psychopathology and their behavior in psychotherapy*. Paper presented at the 5th International Congress on Borderline Personality Disorder and Allied Disorders, Sitges/Barcelona, Spain.

Tanzilli, A., Colli, A., Muzi, L., & Lingiardi, V. (2015). Clinician emotional response toward narcissistic patients: A preliminary report. *Research in Psychotherapy: Psychopathology, Process and Outcome, 18*(1), 1–9.

Thomaes, S., Brummelman, E., Bushman, B. J., Reijntjes, A., & Orobio de Castro, B. (2013). *Is it time to rethink the pervasiveness and nature of low self-esteem in children?* Unpublished manuscript.

Thomaes, S., Brummelman, E., Reijntjes, A., & Bushman, B. J. (2013). When Narcissus was a boy: Origins, nature, and consequences of childhood narcissism. *Child Development Perspectives, 7*(1), 22–26.

Thomaes, S., Bushman, B. J., Orobio de Castro, B., Cohen, G., & Denissen, J. J. A. (2009). Reducing narcissistic aggression by buttressing self-esteem: An experimental field study. *Psychological Science, 20*(12), 1536–1542.

Thomaes, S., Stegge, H., Bushman, B. J., Olthof, T., & Denissen, J. (2008). Development and validation of the Childhood Narcissism Scale. *Journal of Personality Assessment, 90*(4), 382–391.

Torgersen, S., Czajkowski, N., Jacobson, K., Reichborn-Kjennerud, T., Røysamb, E., Neale, M. C., et al. (2008). Dimensional representation of DSM-IV cluster B personality disorders in a population-based sample of Norwegian twins: A multivariate study. *Psychological Medicine, 38*(11), 1617–1625.

Torgersen, S., Kringlen, E., & Cramer, V. (2001). The prevalence of personality disorders in a community sample. *Archives of General Psychiatry, 58*(6), 590–596.

Torgersen, S., Lygren, S., Oeien, P. A., Skre, I., Onstad, S., Edvardsen, J., et al. (2000). A twin study of personality disorders. *Comprehensive Psychiatry, 41*(6), 416–425.

Torgersen, S., Myers, J., Reichborn-Kjennerud, T., Røysamb, E., Kubarych, T. S., & Kendler, K. S. (2012). The heritability of cluster B personality disorders assessed both by personal interview and questionnaire. *Journal of Personality Disorders, 26*(6), 848–866.

Tritt, S. M., Ryder, A. G., Ring, A. J., & Pincus, A. L. (2010). Pathological narcissism and the depressive temperament. *Journal of Affective Disorders, 122*(3), 280–284.

Tronick, E. (1998). Dyadically expanded states of consciousness and the process of therapeutic change. *Infant Mental Health Journal, 19*(3), 290–299.

Tronick, E. (2007). *The neurobehavioral and social-emotional development of infants and children*. New York: Norton.

Tronick, E. (2008). Multilevel meaning making and dyadic expansion of consciousness theory: The emotional and the polymorphic polysemic flow of meaning. In D. Fosha, M. Solomon, & D. Siegel (Eds.), *The healing power of emotion: Affective neuroscience, development, clinical practice* (pp. 86–110). New York: Norton.

Trull, T. J., Jahng, S., Tomko, R. L., Wood, P. K., & Sher, K. J. (2010). Revised NESARC personality disorder diagnoses: Gender, prevalence, and comorbidity with substance dependence disorders. *Journal of Personality Disorders, 24*(4), 412–426.

Tuckett, D. (2019). Transference and transference interpretation revisited: Why a parsimonious model of practice may be useful. *International Journal of Psychoanalysis, 100*(5), 852–876.

Twenge, J. M., Campbell, S. M., Hoffman, B. J., & Lance, C. E. (2010). Generational

differences in work values: Leisure and extrinsic values increasing, social and intrinsic values decreasing. *Journal of Management, 36*(5), 1117–1142.

Twenge, J. M., & Campbell, W. K. (2003). "Isn't it fun to get the respect that we're going to deserve?" Narcissism, social rejection, and aggression. *Personality and Social Psychology Bulletin, 29*(2), 261–272.

Twenge, J. M., & Campbell, W. K. (2009). *The narcissism epidemic: Living in the age of entitlement*. New York: Simon & Schuster.

Twenge, J. M., Konrath, S., Foster, J. D., Campbell, W. K., & Bushman, B. J. (2008a). Egos inflating over time: A cross temporal meta-analysis of the narcissistic personality inventory. *Journal of Personality, 76*(4), 875–902.

Twenge, J. M., Konrath, S., Foster, J. D., Campbell, W. K., & Bushman, B. J. (2008b). Further evidence of an increase in narcissism among college students. *Journal of Personality, 76*(4), 919–928.

Twenge, J. M., Miller, J. D., & Campbell, W. K. (2014). The narcissism epidemic: Commentary on modernity and narcissistic personality disorder. *Personality Disorders: Theory, Research, and Treatment, 5*(2), 227–229.

Unruh, B., & Drozek, R. (2020). *A mentalization based treatment approach to NPD*. Paper presented at Narcissistic Personality Disorder (NPD) Conference, McLean Borderline Disorder Training Institute (BPDTI), Harvard Medical School, Waltham, MA.

Urbonaviciute, G., & Hepper, E. (2020). When is narcissism associated with low empathy? A meta-analytic review. *Journal of Research in Personality, 89*, 104036.

Vater, A., Ritter, K., Schröder-Abé, M., Schütz, A., Lammers, C. H., Bosson, J. K., et al. (2013). When grandiosity and vulnerability collide: Implicit and explicit self-esteem in patients with narcissistic personality disorder. *Journal of Behavior Therapy and Experimental Psychiatry, 44*(1), 37–47.

Vater, A., Ritter, K., Strunz, S., Ronningstam, E. F., Renneberg, B., & Roepke, S. (2014). Stability of narcissistic personality disorder: Tracking categorical and dimensional rating systems over a two-year period. *Personality Disorders: Theory, Research, and Treatment*, *5*(3), 305–313.

Volkan, V. (2009). Large-group identity, international relations and psychoanalysis. *International Forum of Psychoanalysis, 18*(4), 206–213.

Volkan, V. (2014). *Blind trust: Large groups and their leaders in times of crisis and terror*. Charlottesville, VA: Pitchstone.

Waters, E., Hamilton, C. E., & Weinfield, N. S. (2000). The stability of attachment security from infancy to adolescence and early adulthood: General introduction. *Child Development, 71*(3), 678–683.

Waters, E., Merrick, S., Treboux, D., Crowell, J., & Albersheim, L. (2000). Attachment security in infancy and early adulthood: A twenty-year longitudinal study. *Child Development, 71*(3), 684–689.

Watson, P. J., Grisham, S. O., Trotter, M. V., & Biderman, M. D. (1984). Narcissism and empathy: Validity evidence for the narcissistic personality inventory. *Journal of Personality Assessment, 48*(3), 301–305.

Watson, P. J., & Morris, R. J. (1991). Narcissism, empathy and social desirability. *Personality and Individual Differences, 12*(6), 575–579.

Weinberg, I., & Ronningstam, E. (2020). Dos and don'ts in treatments of patients with narcissistic personality disorder. *Journal of Personality Disorders, 34*(Supplement), 122–142.

Weinfeld, N. S., Sroufe, L., & Egeland, B. (2000). Attachment from infancy to early

adulthood in a high-risk sample: Continuity, discontinuity, and their correlates. *Child Development, 71*(3), 695–702.

Westen, D. (1997). Divergences between clinical and research methods for assessing personality disorders: Implications for research and the evolution of Axis II. *American Journal of Psychiatry, 154*(7), 895–903.

Westen, D., & Arkowitz-Westen, L. (1998). Limitations of axis II in diagnosing personality pathology in clinical practice. *American Journal of Psychiatry, 155*(12), 1767–1771.

Westen, D., Gabbard, G. O. (2002). Developments in cognitive neuroscience: II. Implications for theories of transference. *Journal of the American Psychoanalytic Association, 50*(1), 99–134.

Westen, D., Nakash, O., Thomas, C., & Bradley, R. (2006). Clinical assessment of attachment patterns and personality disorder in adolescents and adults. *Journal of Consulting and Clinical Psychology, 74*(6), 1065–1085.

Westen, D., & Shedler, J. (1999). Revising and assessing axis II, part I: Developing a clinically and empirically valid assessment method. *American Journal of Psychiatry, 156*(2), 258–272.

Westen, D., Shedler, J., & Bradley, R. (2006). A prototype approach to personality disorder diagnosis. *American Journal of Psychiatry, 163*(5), 846–856.

Wetzel, E., Brown, A., Hill, P., Chung, J., Robins, R., & Roberts, B. (2017). The narcissism epidemic is dead: Long live the narcissism epidemic. *Psychological Science, 28*(12), 1833–1847.

Widiger, T. A. (2007). Dimensional models of personality disorder. *World Psychiatry, 6*(2), 79–83.

Widiger, T. A., & Corbitt, E. M. (1993). Antisocial personality disorder: Proposals for DSM-IV. *Journal of Personality Disorders, 7*(1), 63–77.

Wilson, M. (2003). The analyst's desire and the problem of narcissistic resistances. *Journal of the American Psychoanalytic Association, 51*(1), 71–99.

Wink, P. (1991). Two faces of narcissism. *Journal of Personality and Social Psychology, 61*(4), 590–597.

Wink, P. (1992). Three narcissism scales for the California Q-set. *Journal of Personality Assessment, 58*(1), 51–66.

Wink, P. (1996). Narcissism. In I. G. Costello (Ed.), *Personality characteristics of the personality disordered* (pp. 146–172). New York: Wiley.

Winnicott, D. W. (1960). The theory of the parent-infant relationship. *International Journal of Psycho-Analysis, 41*, 585–595.

Winnicott, D. W. (1971). Transitional objects and transitional phenomena. In *Playing and reality* (pp. 1–26). New York: Penguin Books.

Wood, J. M., Garb, H. N., Nezworski, M. T., & Koren, D. (2007). The Shedler–Westen Assessment Procedure–200 as a basis for modifying DSM personality disorder categories. *Journal of Abnormal Psychology, 116*(4), 823–836.

Wright, A. G. (2016). On the measure and mismeasure of narcissism: A response to "Measures of narcissism and their relations to DSM-5 pathological traits: A critical reappraisal." *Assessment, 23*(1), 10–17.

Wright, A. G., & Edershile, E. A. (2018). Issues resolved and unresolved in pathological narcissism. *Current Opinion in Psychology, 21*, 74–79.

Wright, A. G., Lukowitsky, M. R., Pincus, A. L., & Conroy, D. E. (2010). The higher order factor structure and gender invariance of the Pathological Narcissism Inventory. *Assessment, 17*(4), 467–483.

Yeomans, F. E., Clarkin, J. F., & Kernberg, O. F. (2015). *Transference-focused psychotherapy for borderline personality disorder: A clinical guide*. Washington, DC: American Psychiatric Publishing.

Yeomans, F. E., Delaney, J. C., & Levy, K. N. (2017). Behavioral activation in TFP: The role of the treatment contract in transference-focused psychotherapy. *Psychotherapy, 54*(3), 260–266.

Yeomans, F. E., & Diamond, D. (2010). Treatment of cluster B disorders: TFP and BPD. In J. F. Clarkin, P. Fonagy, & G. O. Gabbard (Eds.), *Psychodynamic psychotherapy for personality disorders: A clinical handbook* (pp. 209–239). Arlington, VA: American Psychiatric Association.

Yeomans, F. E., Gutfreund, J., Selzer, M. A., Clarkin, J. F., Hull, J. W., & Smith, T. E. (1994). Factors related to drop-outs by borderline patients: Treatment contract and therapeutic alliance. *The Journal of Psychotherapy Practice and Research, 3*(1), 16–24.

Yeomans, F. E., Selzer, M. A., & Clarkin, J. F. (1992). *Treating the borderline patient: A contract-based approach*. New York: Basic Books.

Young, J., & Flanagan, C. (1998). Schema-focused therapy for narcissistic patients. In E. Ronningstam (Ed.), *Disorders of narcissism—Diagnostic, clinical, and empirical implications* (pp. 239–268). Washington, DC: American Psychiatric Press.

Zanarini, M. C., Frankenburg, F. R., Dubo, E. D., Sickel, M. A., Trikha, A., Levin, A., et al. (1998). Axis I comorbidity of borderline personality disorder. *American Journal of Psychiatry, 155*(12), 1733–1739.

Zanarini, M. C., Weingeroff, J. L., & Frankenburg, F. R. (2009). Defense mechanisms associated with borderline personality disorder. *Journal of Personality Disorders, 23*(2), 113–121.

Zeigler-Hill, V., Green, B. A., Arnau, R. C., Sisemore, T. B., & Myers, E. M. (2011). Trouble ahead, trouble behind: Narcissism and early maladaptive schemas. *Journal of Behavior Therapy and Experimental Psychiatry, 42*(1), 96–103.

Zeigler-Hill, V., Myers, E. M., & Clark, C. B. (2010). Narcissism and self-esteem reactivity: The role of negative achievement events. *Journal of Research in Personality, 44*(2), 285–292.

Zimmerman, M., Rothschild, L., & Chelminski, I. (2005). The prevalence of DSM-IV personality disorders in psychiatric outpatients. *American Journal of Psychiatry, 162*(10), 1911–1918.

Index

Note. *f* or *t* following a page number indicates a figure or a table.

Abuse, 104–106, 329, 368. *See also* Early experiences
Acceptance, 127, 268
Active–aggressive polarity of antisocial behavior, 360. *See also* Antisocial features
Adaptive coping, 213–214
Adaptive narcissism. *See* Normal (adaptive) narcissism
Admiration
  combined narcissistic and borderline pathology (NPD/BPD) and, 329
  love relations and, 394–395, 406
  Structured Interview of Personality Organization—Revised (STIPO-R) and, 203
  transference and countertransference patterns and, 182, 183*t*
Adolescents, 100–103, 105–107, 110–111
Adoption, 100
Adult Attachment Interview (AAI)
  case examples, 334–340
  change in attachment and mentalization and, 119–122
  combined narcissistic and borderline pathology (NPD/BPD) and, 334–340, 341
  overview, 111–116
Adverse childhood experiences, 104–106, 329, 368. *See also* Early experiences
Affect regulation
  borderline personality organization and, 56–58
  defense use and, 83–84
  development of normal and pathological narcissism and, 50
  high-functioning NPD and, 55
  internalized object relations and, 45
  TFP-N and, 145–146
Affective valence, 254, 286–305
Age factors, 25–26. *See also* Developmental processes
Agentic extraversion, 68
Aggression. *See also* Ego-syntonic aggression; Malignant narcissism
  assessment and, 213
  children who present as narcissistic and, 99
  development of normal and pathological narcissism and, 50–51
  eroticized transference and, 176
  interpersonal functioning and, 127
  later stages of therapy and, 300–301
  love relations and, 402
  malignant narcissism and, 379–381
  malignant-level of functioning and, 59
  pathological grandiose self and, 23
  relationship functioning and, 123
  spectrum of narcissistic pathology and, 53*f*
  technical neutrality and, 160–161
  treatment contract and, 241
Ainsworth Strange Situation, 111
Aloofness, 127, 172
"Alternative Model for Personality Disorders" (AMPD) section III of DSM-5, 16–17
Altruistic acts, 54

Ambitions, 53*f*
Ambivalence
  combined narcissistic and borderline pathology (NPD/BPD) and, 335–337, 351
  love relations and, 402, 404–405
  malignant narcissism and, 385, 390
Anaclitic aspect of love relations, 393–394. *See also* Love relations
Anger, 127, 337
Anhedonia, 27
Antagonism, 8, 17
Antisocial features. *See also* Antisocial personality disorder; Malignant narcissism
  case examples, 246–247, 248–249
  children who present as narcissistic and, 99
  comparing normal/adaptive with pathological narcissism and, 8
  empathy and, 135
  facial emotional recognition and, 133
  malignant-level of functioning and, 59
  spectrum of narcissistic pathology and, 53*f*
  treatment contract and, 241, 243–249
Antisocial personality disorder. *See* Antisocial features
Antisocial personality disorder (ASPD). *See also* Personality disorders
  comorbidity and, 27
  differential diagnosis and, 215–216
  ego distortion and, 137–138
  empathy and, 135
  facial emotional recognition and, 133
  pathological grandiose self and, 79
  transference and, 176
Antisocial transference, 175–176, 178*t*. *See also* Transference
Anxiety
  assessment and, 213–214
  borderline personality organization and, 56–57
  case examples, 14–16
  children who present as narcissistic and, 99, 100
  combined narcissistic and borderline pathology (NPD/BPD) and, 330–331
  interpretive process in TFP-N and, 168
  later stages of therapy and, 313–315
  love relations and, 394
  narcissistic resistances and, 256
  narcissistic–masochistic presentation and, 71
  NPD and personality organization and, 17
  oedipal issues and, 317–318
  termination stage and, 157–158
  trauma and, 105
Anxiety disorders, 27. *See also* Anxiety
Approach sensitivity, 44–45
Approval of others, 17, 290
Arrogance, 79–81, 99
Assessment. *See also* Diagnosis; Pretreatment assessment and contracting stage
  adolescents and, 106
  attachment and, 111–116
  case examples, 205–212
  clinical assessment of NPD, 197–200
  development of pathological narcissism and, 102–103
  differential diagnosis and, 215–216
  empathy and, 131, 134–135
  facial emotional recognition and, 133
  grandiose/vulnerable distinction of NPD and, 69
  information from prior therapies, 225–226
  instruments for, 216–218
  levels of narcissistic psychopathology, 213–215
  love relations and, 402–404
  overview, 3–4, 197, 218, 219
  pathological grandiose self and, 79–80
  pretreatment assessment and contracting stage of therapy, 147–149
  prevalence rates when diagnosed by clinicians, 29–30
  Reflective Functioning Scale (RFS), 119
  self-assessment, 26
  sharing diagnostic impressions with patients, 226–229
  Structured Interview of Personality Organization—Revised (STIPO-R), 200–205
  therapeutic relationship and, 192
  vulnerable presentation of NPD and, 61–62
Attachment
  adolescents and, 105–106
  Adult Attachment Interview (AAI), 111–116
  attachment-based treatments, 2
  case examples, 334–353
  change in, 119–122
  combined narcissistic and borderline pathology (NPD/BPD) and, 326, 334–354
  free association and, 160
  integration and, 307–308
  internal working models (IWMs) and, 109–111

internalized object relations and, 40
later stages of therapy and, 291, 307–308
love relations and, 403
malignant narcissism and, 388, 391
malignant-level of functioning and, 59
overview, 107–122, 139–141, 414
personality disorders and, 117–120
TFP for pathological narcissism (TFP-N) and, 33
TFP-N and, 144–145
therapeutic relationship and, 192–193
transference analysis and, 170–171
Attachment theory, 43, 353–354, 411–412
Attention seeking, 17, 44–45, 71
Autistic fantasy, 90
Autobiographical memory, 48–49
Autonomy
adolescents and, 106
development of normal and pathological narcissism and, 50
parent–child interactions and, 97–98
Avoidance, 170, 220
Avoidant–dismissing attachment patterns, 108, 111–116

Behavioral resistances. *See* Resistances
Biological factors, 101
Borderline functioning level. *See also* Functioning
assessment and, 213–215
attachment and mentalization and, 120–122
case examples, 13–14, 58–59, 86–88
combined narcissistic and borderline pathology (NPD/BPD) and, 325
grandiose/vulnerable distinction of NPD and, 68
malignant narcissism and, 367
overview, 56–59
structural features of pathological narcissism and, 53, *53f*
suicidality and, 93
treatment contract and, 241, 247–249
Borderline personality disorders (BPDs). *See also* Borderline personality organization (BPO); Combined narcissistic and borderline pathology (NPD/BPD); Personality disorders
attachment and mentalization and, 120–122
comorbidity and, 27
empathy and, 131–132
pathological grandiose self and, 79
transference and, 176
transference-focused psychotherapy (TFP) and, 9–11, 32–34
Borderline personality organization (BPO). *See also* Borderline personality disorders (BPDs); Personality organization
case examples, 13–16, 58–59
combined narcissistic and borderline pathology (NPD/BPD) and, 330–331
malignant narcissism and, 366–371
overview, 18, 20–21
spectrum of narcissistic pathology and, 56–59
Boredom
levels of personality organization and, 20–21
love relations and, 397, 398–399
Structured Interview of Personality Organization—Revised (STIPO-R) and, 205
Boundaries with narcissistic patients, 221, 225, 236–238. *See also* Therapeutic relationship
Brain structures and functioning. *See* Neurobiological factors
Bully and attack schemas, 126

California Adult Q-Set (CAQ), 102
California Child Q-Set (CCQ), 102
Callousness, 80–81
Capacity to invest, 201–202
Caregiving experiences. *See also* Parent factors
attachment and, 110
caregiver–child interactions, 42–43
combined narcissistic and borderline pathology (NPD/BPD) and, 329
development of narcissistic pathology and, 96–107
internalized object relations and, 40, 44
middle stage of therapy and, 155
Caring, longing for, 290
Childhood attachment experiences, 108, 110–111. *See also* Early experiences
Children who present as narcissistic, 99–103
Clarification. *See also* Interpretation in TFP-N
early stages of therapy and, 261–268
malignant narcissism and, 368
overview, 165–169, 413
transference analysis and, 170–171
Classical Freudian approach, 2
Clinical assessment, 197–200. *See also* Assessment
Cognitive empathy, 130, 131–132. *See also* Empathy

Cognitive-affective representational structures, 40, 42–43
Cognitive-behavioral approaches, 2, 43
Cognitive-behavioral therapy (CBT), 193
Collaboration. *See also* Therapeutic relationship
  responding to resistances and, 258–261
  sharing diagnostic impressions with patients, 226–229
  treatment contract and, 220, 229
Collateral sources of information, 199–200, 225–226. *See also* Assessment
Combined narcissistic and borderline pathology (NPD/BPD). *See also* Borderline personality disorders (BPDs); Comorbidity; Narcissistic personality disorder (NPD)
  Adult Attachment Interview (AAI) and, 334–340
  attachment and, 334–353
  case examples, 13–16, 326–328, 331–353
  characteristics of, 328–331
  overview, 9, 325–328, 353–354
  TFP-N for, 331–353
Communication, 5, 266–267
Communication between sessions, 236–238
Comorbidity. *See also* Combined narcissistic and borderline pathology (NPD/BPD)
  empathy and, 131–132
  overview, 27–28, 325, 414–415
  prevalence rates when diagnosed by clinicians and, 30
Compensatory processes, 220–221
Complimentary identifications in the countertransference, 180–181. *See also* Countertransference
Compromise, 289–290
Concern, lack of, 51
Concordant identification in the countertransference, 181. *See also* Countertransference
Conduct disorders, 134–135
Confidentiality, 236
Conflicts
  defense use and, 84
  internalized object relations and, 41–42
  love relations and, 402–404
  relationship functioning and, 123
  technical neutrality and, 160–161
Confrontation. *See also* Interpretation in TFP-N
  early stages of therapy and, 261–268
  overview, 165–169, 413
  transference analysis and, 170–171
Confused and conflicted state of mind, 116
Connection, 127, 289–290
Consolidation
  adolescents and, 106
  integration and, 306–307
  termination and, 320–321
Containment
  early stages of therapy and, 261–280
  malignant narcissism and, 376, 381–382
  overview, 252–253
Contemporary Freudian approach, 2
Contempt for others
  borderline personality organization and, 56
  case examples, 86–88
  narcissistic–masochistic presentation and, 71
Contracting. *See* Treatment contract
Contradictions
  combined narcissistic and borderline pathology (NPD/BPD) and, 338
  integration and, 305–320
  interpretive process in TFP-N and, 166
  overview, 413
  responding to resistances and, 259–260
Control of others
  children who present as narcissistic and, 100
  combined narcissistic and borderline pathology (NPD/BPD) and, 335
  love relations and, 398
  malignant narcissism and, 358–359, 364–366, 379–381
  transference and countertransference patterns and, 182, 183*t*
  vulnerable presentation of NPD and, 61
Coping, 213–214
Co-regulated patterns, 40
Countertransference. *See also* Therapeutic relationship
  attachment and mentalization and, 122
  case examples, 184–192
  early stages of therapy and, 150
  free association and, 160
  interpretive process in TFP-N and, 162, 174–184, 178*t*, 181*t*, 183*t*, 267
  love relations and, 406–408
  narcissistic defenses and, 88–89
  neo-Kleinian and relational approaches and, 172–174
  overview, 31
  research regarding, 182–184
  technical neutrality and, 160–162
  treatment contract and, 230
Couple relationships and treatment, 54–55, 402–404. *See also* Love relations

Critical attitudes, 182, 183*t*
Critical judgments, 255–256
Criticism reactivity
  combined narcissistic and borderline pathology (NPD/BPD) and, 335, 353
  malignant narcissism and, 359
  vulnerable presentation of NPD and, 61
Cultural factors, 5

Dangerous behavior, 161–162
Dead-mother syndrome, 71–72
Defenses
  case examples, 85–88, 89–90
  clinical assessment of NPD and, 199–200
  combined narcissistic and borderline pathology (NPD/BPD) and, 332–334
  countertransference and, 88–89
  early stages of therapy and, 150
  free association and, 160
  grandiose presentation of NPD and, 62
  high-functioning NPD and, 54, 55
  internalized object relations and, 41–42
  interpretive process in TFP-N and, 167–168, 172–173
  later stages of therapy and, 156
  malignant narcissism and, 365
  middle stage of therapy and, 154–155
  NPD and personality organization and, 17
  oedipal issues and, 316
  overview, 83–90
  pathological grandiose self and, 74, 75
  structural features of pathological narcissism and, 52
  suicidality and, 93
  treatment contract and, 221, 231
  vulnerable presentation of NPD and, 62, 64
Defensiveness, 24
De-idealization, 312–315
Denial, 90, 256
Dependency
  adolescents and, 106
  interpretive process in TFP-N and, 168, 170
  love relations and, 395, 399–400, 402
  narcissistic resistances and, 258
  overview, 412
  treatment contract and, 220
Dependent self, 288–290
Dependent transference, 177, 178*t*. *See also* Transference
Depression
  case examples, 14–16
  combined narcissistic and borderline pathology (NPD/BPD) and, 326, 330–331
  comorbidity and, 27
  grandiose/vulnerable distinction of NPD and, 68
  later stages of therapy and, 303–305, 312–315
  love relations and, 394, 400
  malignant narcissism and, 388–391
  narcissistic depression presentation, 72–73
  NPD and personality organization and, 17
  oedipal issues and, 317–318
  termination stage and, 157–158
  vulnerable presentation of NPD and, 61
Depressive transference. *See also* Transference
  interpretive process in TFP-N and, 177, 178*t*
  later stages of therapy and, 303–305
  malignant narcissism and, 388–391
Destructiveness
  combined narcissistic and borderline pathology (NPD/BPD) and, 330–331
  malignant-level of functioning and, 59
  spectrum of narcissistic pathology and, 53*f*
  technical neutrality and, 161–162
  treatment contract and, 241
Detachment
  interpersonal functioning and, 127
  interpretive process in TFP-N and, 170–171
  later stages of therapy and, 288–290
  middle stage of therapy and, 154–155
  transference and countertransference patterns and, 182, 183*t*
Devaluation. *See also* Valuations of others
  adolescents and, 105–106
  case examples, 85–88, 90
  children who present as narcissistic and, 99, 100
  combined narcissistic and borderline pathology (NPD/BPD) and, 337, 353–354
  countertransference and, 88–89
  depressive anxieties and, 314–315
  depressive transferences and, 303–305
  early stages of therapy and, 149–150, 263–265, 269–270
  later stages of therapy and, 287–289, 307
  levels of personality organization and, 21
  love relations and, 395, 397–399, 400, 403
  malignant narcissism and, 359–360, 362–363, 367, 384
  middle stage of therapy and, 154–155

Devaluation *(cont.)*
overview, 84, 122–123
pathological grandiose self and, 73–84, 74*f*
Structured Interview of Personality Organization—Revised (STIPO-R) and, 204–205
treatment contract and, 221
vulnerable presentation of NPD and, 62
Devalued self–superior other internal representation, 263–265, 413
Developmental processes
adolescents and, 105–107
adverse childhood experiences and, 104–106
children who present as narcissistic and, 99–100
early stages of therapy and, 251–252
internalized object relations and, 40–41
normal and pathological narcissism and, 47–52
overview, 414–415
parent–child interactions and, 96–107
research regarding, 100–103
Diagnosis. *See also* Assessment
borderline personality organization and, 14
challenges common in, 16
clinical assessment of NPD and, 197–200
combined narcissistic and borderline pathology (NPD/BPD) and, 331
comparing normal/adaptive with pathological narcissism and, 8
differential diagnosis, 55, 215–216
empathy and, 128, 135
grandiose presentation of NPD and, 60–61
overview, 3–4, 5, 197, 414–415
pathological grandiose self and, 78–79
pretreatment assessment and contracting stage of therapy and, 147–148
prevalence rates when diagnosed by clinicians and, 28–30
sharing diagnostic impressions with patients, 226–229
Structured Interview of Personality Organization—Revised (STIPO-R) and, 200–205
vulnerable presentation of NPD and, 61–62
Diagnostic and Statistical Manual of Mental Disorders (DSM–III) criteria, 128
Diagnostic and Statistical Manual of Mental Disorders (DSM–III-R) criteria, 28–29
Diagnostic and Statistical Manual of Mental Disorders (DSM–IV) criteria, 25–26, 29
Diagnostic and Statistical Manual of Mental Disorders (DSM–5) criteria
"Alternative Model for Personality Disorders" (AMPD) section III of DSM-5, 16–17
borderline personality organization and, 14
clinical assessment of NPD and, 197–200
comorbidity and, 27–28
comparing normal/adaptive with pathological narcissism and, 8
grandiose presentation of NPD and, 60–61
overview, 3–4
pretreatment assessment and contracting stage of therapy and, 147–148
Dialectical behavior therapy (DBT), 34, 193
Dictatorships, 366. *See also* Political leadership
Differential diagnosis, 55, 215–216. *See also* Diagnosis
Disengagement
interpretive process in TFP-N and, 170–171, 172
transference and countertransference patterns and, 182, 183*t*
Dismissing attachment patterns
adolescents and, 106
Adult Attachment Interview (AAI) and, 111–116
attachment and mentalization and, 120–122
combined narcissistic and borderline pathology (NPD/BPD) and, 337, 353–354
free association and, 160
overview, 108–109
Dismissing detachment, 289–290
Dismissiveness, 220, 397–399
Disorganized attachment patterns. *See also* Attachment
Adult Attachment Interview (AAI) and, 111–116
case examples, 334–353
combined narcissistic and borderline pathology (NPD/BPD) and, 334–353, 354
free association and, 160
interpersonal functioning and, 125
overview, 107–109, 411–412
Disorganized factors, 46*f*
Disrupting states of mind, 354

Dissociative mechanisms
combined narcissistic and borderline pathology (NPD/BPD) and, 337
countertransference and, 179
interpretive process in TFP-N and, 169
malignant narcissism and, 363–364, 377, 379–380, 383
overview, 87
structural features of pathological narcissism and, 52
Dissolution of the grandiose self
case examples, 298–300, 309–312
integration and, 305
later stages of therapy and, 297–300
Distress, 102
Divorce, 100
Dominance of others. *See also* Control of others; Power; Social dominance strivings
grandiose/vulnerable distinction of NPD and, 68
malignant narcissism and, 358–359
vulnerable presentation of NPD and, 61
Doubt, 266–267
Drives, 41–42, 161
Dropping out of treatment, 34, 415
Dyadic mental representations, 49, 151. *See also* Mental representations
Dyads, object relational. *See* Object relational dyads

Early experiences. *See also* Traumatic experiences
attachment and mentalization and, 109, 121
combined narcissistic and borderline pathology (NPD/BPD) and, 329
development of narcissistic pathology and, 96–107
effective interpretations, 255–256
internalized object relations and, 46*f*
malignant narcissism and, 368
overview, 412
Early stage of therapy. *See also* Stages of therapy; TFP for pathological narcissism (TFP-N)
case examples, 185–188, 271–280, 281–284
interpretive process in TFP-N and, 166–167
narcissistic resistances and, 256–261
overview, 149–153, 251–255, 284–285, 413
phase 3 of, 281–284
phases 1 and 2 of, 261–280
role reversals and, 281–284

Eating disorders, 27, 238–239
Ecological momentary assessment (EMA), 79–80
Ego functioning. *See also* Superego functioning
ego distortion, 97*n*–98*n*, 137–138
ego ideal, 49
ego weakness, 53*f*
malignant narcissism and, 357–358, 386–387
technical neutrality and, 161
Ego-syntonic aggression, 59–60, 241. *See also* Aggression; Malignant narcissism
Emotion recognition, 45
Emotion regulation
malignant narcissism and, 377
neurological correlates of NPD and, 135–136
Emotional abuse, 104–106
Emotional detachment. *See* Detachment
Emotional development, 43
Emotional empathy, 45, 130, 131–132. *See also* Empathy
Empathy. *See also* Interpersonal functioning
borderline personality organization and, 58
children who present as narcissistic and, 99
combined narcissistic and borderline pathology (NPD/BPD) and, 329
development of normal and pathological narcissism and, 51
facial emotional recognition and, 132–134
grandiose presentation of NPD and, 60–61
high-functioning level of personality organization and, 19–20
high-functioning NPD and, 56
internalized object relations and, 45
malignant-level of functioning and, 60
mentalization and, 118
neurological correlates of NPD and, 134–136
pathological grandiose self and, 79
pathological narcissism and, 53, 127–136
relationship functioning and, 125–126
research regarding, 130–132
Structured Interview of Personality Organization—Revised (STIPO-R) and, 204
transference and countertransference patterns and, 182, 183*t*
vulnerable presentation of NPD and, 61

Employment functioning, 57, 61, 91
Emptiness, 20–21, 27
Enactments
  countertransference and, 179
  interpretive process in TFP-N and, 164
  love relations and, 403
  malignant narcissism and, 376
Entitlement. *See also* Expectations
  children who present as narcissistic and, 99–100
  combined narcissistic and borderline pathology (NPD/BPD) and, 329
  comparing normal/adaptive with pathological narcissism and, 8
  development of normal and pathological narcissism and, 51
  grandiose/vulnerable distinction of NPD and, 24, 60–61, 68
  interpersonal functioning and, 126
  malignant narcissism and, 360
  NPD and personality organization and, 17
  parent–child interactions and, 98–99
  pathological grandiose self and, 79–81
  transference and countertransference patterns and, 182, 183*t*
Envy
  case examples, 309–312
  combined narcissistic and borderline pathology (NPD/BPD) and, 329
  love relations and, 397, 399, 400, 405–406
  malignant narcissism and, 360, 386
  oedipal issues and, 316
  Structured Interview of Personality Organization—Revised (STIPO-R) and, 204
  transference and countertransference patterns and, 182, 183*t*
Epistemic mistrust, 117–118
Erotic strivings, 394. *See also* Love relations
Erotic transference. *See also* Transference
  case examples, 296–297
  interpretive process in TFP-N and, 176, 177–178, 178*t*
  oedipal issues and, 317–318
Ethical values
  borderline personality organization and, 59
  high-functioning NPD and, 54, 56
  interpersonal functioning and, 136–139
  malignant-level of functioning and, 60
  NPD and personality organization and, 17
  parent–child interactions and, 99
  structural features of pathological narcissism and, 53
Ethnicity factors, 25–26
Evaluation, 402–404. *See also* Assessment
Excitement seeking, 20–21
Exclusion, 127
Exhibitionism, 71
Expectations. *See also* Entitlement
  development of normal and pathological narcissism and, 51
  high-functioning NPD and, 54–55
  NPD and personality organization and, 17
Exploitativeness
  assessment and, 214
  children who present as narcissistic and, 99, 100
  combined narcissistic and borderline pathology (NPD/BPD) and, 329
  comparing normal/adaptive with pathological narcissism and, 8
  grandiose/vulnerable distinction of NPD and, 24, 60–61, 68
  love relations and, 400
  malignant narcissism and, 363
  pathological grandiose self and, 79–81
Exploratory therapy, 238–240, 240*t*
Externalizing behaviors, 105
Extratransferential relationships, 171

Facial emotional recognition, 125–126, 132–134
Failure reactivity, 61, 70
False self-organization, 97*n*–98*n*
Family factors, 5, 44, 100–101, 109. *See also* Parent factors
Family meeting, 248
Fear
  grandiose/vulnerable distinction of NPD and, 68
  integration and, 42
  later stages of therapy and, 289
  transference and countertransference patterns and, 182, 183*t*
Fees, 233–234
Five-Factor Narcissism Inventory (FFNI), 69
Flexibility, 241, 266–267
Fragmented state of mind, 116
Frame, treatment. *See* Treatment frame
Free association
  early stages of therapy and, 149–150
  malignant narcissism and, 388
  overview, 159–160
  treatment contract and, 234–235

Frequency of sessions, 231–233
Freudian approaches, 2
Functioning. *See also* Borderline functioning level; Higher functioning level; Interpersonal functioning; Lower functioning levels; Self functioning
  assessment and, 213–215
  case examples, 11–16
  functional impairments associated with NPD and, 30–31
  high-functioning NPD and, 54–56
  levels of personality organization and, 18–22
  NPD and personality organization and, 16–18
  overview, 96
  self-presentation and, 1–3
Futility, sense of, 27

Gender
  development of pathological narcissism and, 101–103
  love relations and, 395–397, 399–400
  prevalence and, 25–27
  trauma and, 105
Generosity, 54
Genetic factors, 44–45, 101
Goals. *See also* Self-directed aggression; Treatment goals
  borderline personality organization and, 56–57
  combined narcissistic and borderline pathology (NPD/BPD) and, 330
  early stages of therapy and, 151
  love relations and, 402
  malignant narcissism and, 389
  treatment contract and, 220
Grandiose presentation of NPD. *See also* Grandiosity; Presentation of NPD
  attachment and, 115–116
  case examples, 65–67
  combined narcissistic and borderline pathology (NPD/BPD) and, 325–326
  development of pathological narcissism and, 101–103
  early stages of therapy and, 152, 263
  later stages of therapy and, 301–303
  malignant narcissism and, 366–370
  oscillation between vulnerable presentation and, 301–303
  overview, 24–25, 60–70, 94–95, 140
  research support for the distinction of, 67–70
  suicidality and, 94
  trauma and, 105
  treatment contract and, 241
Grandiose–devalued dyad, 150, 154
Grandiosity. *See also* Grandiose presentation of NPD
  attachment and, 115–116
  borderline personality organization and, 56–57, 58
  case examples, 298–300
  challenges related to diagnosis and treatment and, 16
  children who present as narcissistic and, 99
  combined narcissistic and borderline pathology (NPD/BPD) and, 329
  comparing normal/adaptive with pathological narcissism and, 5–8
  development of normal and pathological narcissism and, 51–52
  dissolution of the grandiose self and, 297–300
  early stages of therapy and, 151–152, 251–252
  free association and, 160
  high-functioning NPD and, 54
  internalized object relations and, 46*f*
  interpretive process in TFP-N and, 167–168
  later stages of therapy and, 297–300
  levels of personality organization and, 20, 21
  love relations and, 399, 402
  malignant narcissism and, 357–358, 360–362
  middle stage of therapy and, 153–155
  NPD and personality organization and, 17
  overview, 94–95, 412–414
  pathological grandiose self, 22–24, 73–81, 74*f*
  prevalence and, 26
  research regarding, 78–81
  spectrum of narcissistic pathology and, 53*f*
  TFP for pathological narcissism (TFP-N) and, 31–34
  treatment contract and, 220–221
  vulnerable presentation of NPD and, 61–62
Gratitude, 402
Guilt
  children who present as narcissistic and, 99
  later stages of therapy and, 155–156
  love relations and, 400
  malignant-level of functioning and, 59

Helping Alliance Questionnaire (HAQ), 192
Heritability rate of NPD, 44–45, 101
Higher functioning level
  assessment and, 148, 213–215
  case examples, 85–86
  combined narcissistic and borderline pathology (NPD/BPD) and, 329, 330
  defense use and, 84
  disturbances of identity and, 82
  early stages of therapy and, 150
  malignant narcissism and, 359–360, 362, 367
  pathological grandiose self and, 75–76
  pretreatment assessment and contracting stage of therapy and, 148
  structural features of pathological narcissism and, 53, 53*f*
  suicidality and, 94
  technical neutrality and, 161
  treatment contract and, 220, 241
High-functioning level of personality organization, 18–19. *See also* Personality organization
High-functioning narcissistic disorder. *See also* Functioning
  case examples, 11–13, 55–56
  development of normal and pathological narcissism and, 51
  overview, 54–56
Histrionic personality disorder, 27. *See also* Personality disorders
Histrionics, 71
Hopelessness, 182, 183*t*
Hospitalization, 358, 361
Hostility
  comparing normal/adaptive with pathological narcissism and, 8
  eroticized transference and, 176
  later stages of therapy and, 291
  transference and countertransference patterns and, 182, 183*t*
Humiliation, 70, 93, 117
Humility, 402
Humor, 260–261
Hyper- or overactive mentalization, 118
Hypersensitivity, 24

Ideal self/ideal other. *See also* Idealization; Representations of self and others
  borderline personality organization and, 57
  disturbances of identity and, 81–83
  early stages of therapy and, 253, 262, 263–265
  ethical values and reality testing and, 136–137
  later stages of therapy and, 155–157, 287–288, 290–291
  malignant narcissism and, 378–386
  middle stage of therapy and, 154–155
  narcissistic depression presentation and, 72–73
  overview, 122–123
  parent–child interactions and, 97–98
  pathological grandiose self and, 73–81, 74*f*
  spectrum of narcissistic pathology and, 53*f*
  TFP-N and, 146
  transference and countertransference patterns and, 179–180, 182, 183*t*, 303–305
  vulnerable presentation of NPD and, 62
Idealization. *See also* Ideal self/ideal other
  adolescents and, 105–106
  attachment and, 113
  case examples, 86–88, 90, 292–297
  children who present as narcissistic and, 100
  combined narcissistic and borderline pathology (NPD/BPD) and, 350
  depressive anxieties and, 314–315
  integration and, 305–306, 312–315
  later stages of therapy and, 287–288
  love relations and, 394–395
  malignant narcissism and, 367
  overview, 84, 122–123
  parent–child interactions and, 97–98
  states of mind and, 116
  Structured Interview of Personality Organization—Revised (STIPO-R) and, 204–205
  treatment contract and, 221
Identity
  adolescents and, 106
  borderline personality organization and, 58
  combined narcissistic and borderline pathology (NPD/BPD) and, 329–330
  development of normal and pathological narcissism and, 50, 103
  disturbances of, 81–83
  high-functioning NPD and, 55–56
  identity formation, 83
  malignant-level of functioning and, 59–60
  mental representations and, 49
  NPD and personality organization and, 17

overview, 83
structural features of pathological narcissism and, 52
Structured Interview of Personality Organization—Revised (STIPO-R) and, 200–203
Identity diffusion
borderline personality organization and, 56–57
combined narcissistic and borderline pathology (NPD/BPD) and, 326, 329
mentalization and, 118
spectrum of narcissistic pathology and, 53*f*
Image-distorting defenses, 84–88, 160. *See also* Defenses
Imitation, 130
Impairment, 30–31
Imperfection tolerance, 269–270. *See also* Perfectionism
Impulse control
assessment and, 213–214
borderline personality organization and, 56–57
defense use and, 83–84
internalized object relations and, 41–42
suicidality and, 93
technical neutrality and, 161
Inadequacy feelings, 24
Inconsistencies, 166, 167
Individualism, 5
Inertness, 71–72
Infantile narcissism, 50–51, 399–400
Inferiority, feelings of, 58, 61, 161
Informant data, 26
Inhibition
love relations and, 397
malignant narcissism and, 363–364
oedipal issues and, 319–320
vulnerable presentation of NPD and, 61, 126
Inpatient treatment, 358, 361
Insecure attachment patterns. *See also* Attachment
Adult Attachment Interview (AAI) and, 111–116
combined narcissistic and borderline pathology (NPD/BPD) and, 334–353
internalized object relations and, 46*f*
interpersonal functioning and, 125
malignant-level of functioning and, 59
overview, 107–109, 140–141, 411–412
personality development and, 43
Instability, 19–20
Integration
case examples, 309–312
disturbances of identity and, 81–83
integrated mental representations, 50, 52–53
integrated object relations theoretical framework, 9
integrated representation, 254
integrated self, 49, 73–74
internalized object relations and, 41–42
interpretive process in TFP-N and, 163
later stages of therapy and, 156–157, 305–320
malignant narcissism and, 378–388, 390
mentalization and, 118
object relations approaches and, 315–316
overview, 322–324
pathological grandiose self and, 73–74
termination and, 157–158, 321–322
TFP-N and, 146
Internal objects, 143–144
Internal working models (IWMs)
Adult Attachment Interview (AAI) and, 114
interpersonal functioning and, 125
overview, 109–111
personality development and, 43
Internalized object relations. *See also* Object relations approaches
defense use and, 83–90
development of normal and pathological narcissism and, 47–52
disturbances of identity and, 81–83
early stages of therapy and, 253
overview, 46*f*, 94–95, 139–141
pathological grandiose self and, 73–84, 74*f*
relationship functioning and, 122–139
self-esteem regulation and, 91–94
structural features of pathological narcissism and, 52–53, 53*f*
TFP-N and, 145
Internalizing disorders, 105
Interpersonal functioning. *See also* Functioning; Relationship functioning
borderline personality organization and, 58
development of normal and pathological narcissism and, 48
empathy and, 127–136
ethical values and reality testing and, 136–139
facial emotional recognition and, 132–134

Interpersonal functioning *(cont.)*
functional impairments associated with NPD and, 30–31
grandiose/vulnerable distinction of NPD and, 25, 68
interpersonal transactions, 40
later stages of therapy and, 156
malignant narcissism and, 388–389
NPD and personality organization and, 16–18
overview, 96, 122–139, 140–141
pathological grandiose self and, 73–81, 74*f*
Interpersonal Reactivity Index (IRI), 131, 132, 134–135
Interpretation in TFP-N
case examples, 184–192, 271–280, 281–284
clarification and confrontation, 165–169
combined narcissistic and borderline pathology (NPD/BPD) and, 332–334
countertransference use and, 179–182, 181*t*
effective interpretations, 255–256
malignant narcissism and, 368, 381–383
narcissistic resistances and, 256–261
neo-Kleinian and relational approaches and, 172–174
overview, 162–165, 252–253, 284–285, 413
phase 3 of treatment and, 281–284
phases 1 and 2 of treatment and, 261–280
projections and, 268–270
role reversals and, 281–284
therapeutic relationship and, 192–193
transference and countertransference patterns and, 169–172, 174–179, 178*t*, 182–184, 183*t*
Interview, structural. *See* Assessment; Structural interview
Intimacy. *See also* Love relations; Relationship functioning
borderline personality organization and, 59
grandiose presentation of NPD and, 61
high-functioning NPD and, 56
malignant narcissism and, 358–359
malignant-level of functioning and, 60
overview, 5
Intrafamilial factors, 100–101. *See also* Family factors
Intrapersonal factors, 100–101
Intrapsychic structural features
development of normal and pathological narcissism and, 51
internalized object relations and, 45–46
interpersonal functioning and, 125–126
later stages of therapy and, 156
Introjection
case examples, 90
introjective psychopathology, 72–73
malignant narcissism and, 366
narcissistic resistances and, 256
overview, 84
Invincibility beliefs, 258
Isolation, 240

Jealousy, 405–406. *See also* Envy
Judgments, 255–256

Late stage of therapy. *See also* Stages of therapy; TFP for pathological narcissism (TFP-N)
aggression in the transference and, 300–301
case examples, 190–192, 292–297, 298–300, 309–312
dealing with loss, limitation, and de-idealization, 312–315
depressive transferences and, 303–305
dissolution of the grandiose self and, 297–300
grandiose/vulnerable distinction of NPD and, 301–303
integration and reflection and, 315–316
malignant narcissism and, 386–391
oedipal issues and, 316–320
overview, 155–157, 286, 322–324, 413–414
phase 4 of, 286–305
phase 5 of, 305–320
termination, 320–322
Leadership roles, 61, 359, 364–366
Libido, 50–51
Limitations, 156, 312–315
Loss, 111–112
high-functioning level of personality organization and, 19–20
integration and, 312–315
later stages of therapy and, 156
malignant narcissism and, 390
termination and, 320–321
Love relations. *See also* Intimacy; Relationship functioning; Sexual behavior and issues
countertransference and, 406–408
evaluation of, 402–404

overview, 393–402, 408–409
preoedipal and oedipal conflicts and, 405–406
transference and, 404–405
Low borderline personality organization, 56–59, 86–88. *See also* Borderline functioning level; Borderline personality organization (BPO)
Lower functioning levels, 247–249, 325. *See also* Borderline functioning level

Malignant narcissism. *See also* Malignant-level of functioning; Narcissistic personality disorder (NPD); Pathological narcissism
advanced stages of treatment and, 386–391
case examples, 59–60, 368–391
with comorbid borderline personality organization, 366–371
defining the dominant object relations, 371–374
differential diagnosis and, 215–216
integrating the persecutory and idealized object relational dyads, 378–386
moving into the depressive position and, 388–391
overview, 59–60, 357–364, 391–392
role reversals in the transference, 374–377
social implications of, 364–366
Malignant-level of functioning. *See also* Functioning; Malignant narcissism
assessment and, 213–215
case examples, 14–16, 59–60
overview, 59–60
structural features of pathological narcissism and, 53, 53*f*
suicidality and, 94
Maltreatment, childhood, 104–106. *See also* Abuse; Early experiences
Manipulation, 176, 360
Marital status, 25–26. *See also* Love relations
Mature idealization, 394–395. *See also* Idealization
Meaninglessness, sense of, 27
Medications, 193–194, 313–314
Mental representations. *See also* Representations of self and others
Adult Attachment Interview (AAI) and, 111–116
attachment and, 109–111
development of normal and pathological narcissism and, 47–52
internalized object relations and, 40, 41–42
interpretive process in TFP-N and, 162–165, 172, 267
malignant narcissism and, 383, 391
NPD and personality organization and, 17
structural features of pathological narcissism and, 52–53
Mentalization
attachment and, 117–118
change in, 119–122
cognitive empathy and, 130
combined narcissistic and borderline pathology (NPD/BPD) and, 351–353
countertransference and, 89
internalized object relations and, 46*f*
later stages of therapy and, 307
malignant narcissism and, 388, 391
overview, 107–122
TFP-N and, 33, 144–145
Mentalization-based treatments (MBTs), 2, 34
Middle stage of therapy, 153–155, 188–190, 413–414. *See also* Stages of therapy; TFP for pathological narcissism (TFP-N)
Midlife period, 20
Minority status, 25–26
Mirroring
early stages of therapy and, 266
mentalization and, 117
motor empathy and, 130
transference and countertransference patterns and, 182, 183*t*
Modulated mental representations, 50
Moral superiority, 71
Motivational systems, 110, 151–152
Motor empathy, 130. *See also* Empathy
Mourning, 320–321. *See also* Loss
Multiple perspectives, 48–49
Mutuality, 157, 220, 358–359

Narcissism. *See also* Normal (adaptive) narcissism; Pathological narcissism
case examples, 11–16
development of, 47–52
integration and, 42
overview, 5–8, 411–415
Structured Interview of Personality Organization—Revised (STIPO-R) and, 200–205
Narcissistic aspect of love relations, 393–394. *See also* Love relations
Narcissistic depression, 72–73. *See also* Depression

Narcissistic personality disorder (NPD). *See also* Combined narcissistic and borderline pathology (NPD/BPD); Malignant narcissism
case examples, 11–16
comparing normal/adaptive with pathological narcissism and, 7–8
defense use and, 83–90
disturbances of identity in, 81–83
grandiose and vulnerable presentations of, 24–25
internalized object relations and, 40–46, 46*f*
object relations approach to treatment of, 9–11
overview, 2, 3–4, 411–415
parent–child interactions and, 96–107
personality organization and, 16–18
self-esteem regulation and, 91–94
Narcissistic Personality Inventory (NPI)
empathy and, 135
grandiose/vulnerable distinction of NPD and, 69
overview, 4, 216–217
Narcissistic resistances. *See* Resistances
Narcissistic transference, 175, 176, 178*t*. *See also* Transference
Narcissistic–histrionic presentation, 71
Narcissistic–masochistic presentation, 70–71
Negative affectivity
children who present as narcissistic and, 100
development of normal and pathological narcissism and, 51
grandiose/vulnerable distinction of NPD and, 68
levels of personality organization and, 21
NPD and personality organization and, 17
transference and, 269
Negative narcissism, 71–72
Neglect, 97, 104–106, 329. *See also* Early experiences
Neo-Kleinian approach, 2, 172–174
Neurobiological factors
development of normal and pathological narcissism and, 50
development of pathological narcissism and, 101–102
empathy and, 134–136
internalized object relations and, 45
Neuroticism, 68
Neutrality, technical. *See* Technical neutrality
Nihilism, 27
Nonbeing, 71–72
Nonverbal communications
early stages of therapy and, 150, 152
free association and, 160
interpretive process in TFP-N and, 162, 166
Normal (adaptive) narcissism, 5–8, 47–52. *See also* Narcissism

Object relational dyads
early stages of therapy and, 151, 254, 261–280
integration and, 305–320
interpretive process in TFP-N and, 168, 169
later stages of therapy and, 286–287, 305
malignant narcissism and, 378–386
role reversals and, 281–284
Object relations
malignant narcissism and, 371–374
Structured Interview of Personality Organization—Revised (STIPO-R) and, 203–205
Object relations approaches. *See also* TFP for pathological narcissism (TFP-N); Transference-focused psychotherapy (TFP)
Adult Attachment Interview (AAI) and, 112–113
attachment and, 109–111
integration and reflection and, 315–316
internalized object relations and, 40–46, 46*f*
object relations theory, 3, 412
overview, 2, 6, 39–40, 139–141, 142–143, 411–412
treatment contract and, 222–223
to treatment of NPD, 9–11
Object relations developmental model of narcissism
internalized object relations and, 40–46, 46*f*
overview, 39–40
Object representations, 47–48. *See also* Mental representations
Occupational functioning, 57, 61, 91
Oedipal conflicts, 397, 404, 405–406
Oedipal transference, 316–320. *See also* Erotic transference
Omnipotence
case examples, 85–86, 90
combined narcissistic and borderline pathology (NPD/BPD) and, 335
countertransference and, 179–180
interpretive process in TFP-N and, 168
later stages of therapy and, 156

love relations and, 398, 406–407
malignant narcissism and, 361, 379–381
overview, 84
Openness, 267
Oscillating cannot classify state of mind, 116
Other-directed aggression, 53*f*. *See also* Aggression
Overvaluation, 98–99

Paranoid features. *See also* Malignant narcissism
children who present as narcissistic and, 99
malignant narcissism and, 386–387
malignant-level of functioning and, 59
spectrum of narcissistic pathology and, 53*f*
termination stage and, 157–158
vulnerable presentation of NPD and, 24
Paranoid personality disorder, 27. *See also* Personality disorders
Paranoid transference, 176, 178*t*, 388. *See also* Transference
Parasuicidality, 93, 360. *See also* Suicidality
Parent factors. *See also* Caregiving experiences; Family factors; Parent–child interactions
adolescents and, 106
development of narcissistic pathology and, 96–107
internalized object relations and, 44
overview, 412
Parent–child interactions. *See also* Parent factors
adolescents and, 105–107
Adult Attachment Interview (AAI) and, 111
development of normal and pathological narcissism and, 48–49, 96–107
internalized object relations and, 44
parent–infant interactions, 42–43, 108
research regarding, 100–101
Passive–parasitic polarity of antisocial behavior, 360. *See also* Antisocial features
Pathological grandiose self, 22–24, 73–83, 74*f*
Pathological narcissism. *See also* Malignant narcissism; Narcissism
attachment and, 108
characterological presentations of, 70–73
comorbidity and, 27–28
compared to normal/adaptive narcissism, 5–8
development of, 47–52
empathy and, 127–136
grandiose and vulnerable presentations of NPD and, 60–70
love relations and, 401–402
NPD and personality organization and, 16–17
overview, 2–3, 5–8, 94–95
parent–child interactions and, 96–107
pathological grandiose self and, 22–24, 73–81, 74*f*
structural features of, 52–53, 53*f*
suicidality and, 94
transference-focused psychotherapy (TFP) and, 9–11, 31–34
treatment and, 35–36
Pathological Narcissism Inventory (PNI), 69, 132, 216–217
Patient Attachment Coding System (PACS), 340–353
Patient responsibilities and roles in treatment. *See also* Roles and responsibilities in treatment
roles and responsibilities of the patient, 229*t*, 230–231, 234–235
Patient-centered interpretations, 268. *See also* Interpretation in TFP-N
Peer relationships, 99
Perceptions, 165–166, 330
Perfectionism, 72–73, 97*n*–98*n*, 156
Peroccupied attachment patterns, 108–109
Persecutory sector of experience, 378–386
Personality disorders. *See also* Combined narcissistic and borderline pathology (NPD/BPD)
attachment and, 107–108, 116, 117–120
comorbidity and, 27–28
comparing normal/adaptive with pathological narcissism and, 8
development of normal and pathological narcissism and, 51–52
internalized object relations and, 43
NPD and personality organization and, 16–18
pathological grandiose self and, 79
sharing diagnostic impressions with patients, 226–229
transference and, 170
Personality features
comparing normal/adaptive with pathological narcissism and, 5–8
developmental processes and, 42–43
grandiose/vulnerable distinction of NPD and, 68–69
overview, 141

Personality organization
clinical assessment of NPD and, 198–200
levels of, 18–22
narcissistic personality disorder (NPD) and, 16–18
Perspectives, multiple, 48–49, 385–386
Phone calls between sessions, 236–238
Playfulness, 260–261
Political leadership, 61, 359, 364–366
Positive emotions, 100
Posttraumatic stress disorder (PTSD), 27, 104–105
Power
love relations and, 399–400, 402
malignant narcissism and, 361–362, 363, 364–366, 380–381
pathological grandiose self and, 79–80
spectrum of narcissistic pathology and, 53*f*
Praise seeking, 44–45
Premature termination, 158, 182, 183*t*, 313. *See also* Termination stage
Preoccupied attachment patterns, 111–116, 121–122, 337
Preoedipal conflicts, 404, 405–406. *See also* Oedipal conflicts
Preschoolers, 102–103. *See also* Children who present as narcissistic
Presentation of NPD. *See also* Grandiose presentation of NPD; Vulnerable presentation of NPD
characterological presentations, 70–73
early stages of therapy and, 152
overview, 1–3, 60–61, 415
pathological grandiose self and, 73–81, 74*f*
research support for the grandiose/vulnerable distinction of, 67–70
treatment contract and, 221
Pretreatment assessment and contracting stage, 147–149, 197–200, 221–222. *See also* Assessment; Stages of therapy; TFP for pathological narcissism (TFP-N); Treatment contract
Prevalence statistics
comorbidity and, 27–28
functional impairments associated with NPD and, 30–31
overview, 4, 5, 25–31
when diagnosed by clinicians and, 28–30
Primary role satisfaction, 201–203
Privileges, demands for, 182, 183*t*. *See also* Entitlement
Projections
borderline personality organization and, 57–58
case examples, 90, 271–280
countertransference and, 179–181
development of normal and pathological narcissism and, 51
early stages of therapy and, 265–267, 268–270
integration and, 306–307
interpretive process in TFP-N and, 162, 168, 169, 172–173, 253, 268–270
later stages of therapy and, 155–157, 306–307
malignant narcissism and, 366, 385–386
overview, 84, 413–414
suicidality and, 93
technical neutrality and, 161
Projective identification, 85, 87, 90
Promiscuity, 399–400. *See also* Sexual behavior and issues
Psychoanalytical perspective, 2–3, 5–6, 163–164
Psychodynamic Diagnostic Manual (PDM-2), 17, 61–62
Psychodynamic treatment, 119–120, 142–143
Psychopathic transference, 175–176, 178*t*, 388. *See also* Transference
Psychopharmacology, 193–194, 313–314
Psychotic level of personality organization, 18, 21–22. *See also* Personality organization
Punitive representations, 75

Rage, 53*f*, 403
Reality testing
borderline personality organization and, 56–57
internalized object relations and, 41–42
interpersonal functioning and, 136–139
malignant narcissism and, 386–387
relationship functioning and, 125–126
spectrum of narcissistic pathology and, 53*f*
structural features of pathological narcissism and, 53
Reflection, 53*f*, 309–312, 315–316. *See also* Reflective functioning (RF)
Reflective functioning (RF)
attachment and, 119
combined narcissistic and borderline pathology (NPD/BPD) and, 339–340, 351–353
malignant narcissism and, 387, 391
overview, 114
Reflective Functioning Scale (RFS), 119, 339–340

Regression
combined narcissistic and borderline pathology (NPD/BPD) and, 350
malignant narcissism and, 366, 386–387
termination and, 157–158, 320–321
Reintegration, 315–316. *See also* Integration
Rejection sensitivity
borderline personality organization and, 58
development of pathological narcissism and, 102
early stages of therapy and, 152
facial emotional recognition and, 133
interpersonal functioning and, 126–127
transference and countertransference patterns and, 182, 183*t*
vulnerable presentation of NPD and, 24
Relatedness to others, 73–74, 156, 413
Relational approach, 2, 172–174
Relationship functioning. *See also* Interpersonal functioning; Intimacy; Love relations
assessment and, 213–214
borderline personality organization and, 57, 58
children who present as narcissistic and, 99
depressive transferences and, 303–305
development of normal and pathological narcissism and, 50, 51
empathy and, 127–136
ethical values and reality testing and, 136–139
functional impairments associated with NPD and, 30–31
grandiose presentation of NPD and, 61
high-functioning level of personality organization and, 19–20
high-functioning NPD and, 54–55
integration and, 305
internalized object relations and, 40
later stages of therapy and, 156–157, 288
malignant narcissism and, 358–359, 362–363, 390–391
malignant-level of functioning and, 59
middle stage of therapy and, 155
negative narcissism and, 71–72
oedipal issues and, 316–320
overview, 95, 122–139, 414
pathological grandiose self and, 23, 73–84
prevalence and, 25–26
relational development, 43
self-esteem regulation and, 91
spectrum of narcissistic pathology and, 53*f*
termination stage and, 157
TFP for pathological narcissism (TFP-N) and, 31–34
Representations of self and others. *See also* Ideal self/ideal other; Mental representations; Self functioning; Self-representation
attachment and, 115
change in, 119–122
combined narcissistic and borderline pathology (NPD/BPD) and, 334–335, 350
early stages of therapy and, 152, 254
empathy and, 130
ethical values and reality testing and, 136–137
free association and, 159–160
integration and, 307
internal working models (IWMs) and, 109–111
internalized object relations and, 41–42, 43–44
interpersonal functioning and, 125–126
interpretive process in TFP-N and, 162–165
later stages of therapy and, 156
love relations and, 404–405
malignant narcissism and, 367–368
middle stage of therapy and, 154–155
oedipal issues and, 316–320
overview, 413
pathological grandiose self and, 22–24, 73–84, 74*f*
structural features of pathological narcissism and, 52–53
TFP-N and, 145–146
Resistances
combined narcissistic and borderline pathology (NPD/BPD) and, 331
early stages of therapy and, 256–261
examples of, 240*t*
interpretive process in TFP-N and, 167–168
responding to, 258–261
treatment contract and, 238–240, 240*t*
Responsibilities in treatment. *See* Roles and responsibilities in treatment
Rewards, 44–45, 100, 101
Rigidity, 73–74, 87–88
Risk factors, 93–94
Role responsiveness, 153–154
Role reversals. *See also* Transference
case examples, 188–190, 280, 281–284
early stages of therapy and, 254, 280–284
interpretive process in TFP-N and, 169

Role reversals (cont.)
malignant narcissism and, 374–377
middle stage of therapy and, 153–154
Roles and responsibilities in treatment, 229*t*, 230–231, 234–238, 247
Romantic relationships. *See* Love relations; Relationship functioning

Sadistic behaviors and attitudes, 182, 183*t*
Sadness, 155–156
Sadomasochist transference, 176, 365. *See also* Transference
Schema-focused therapy, 34
Schizotypal personality disorder, 27. *See also* Personality disorders
Scorekeeping, 204
Secondary gains, 241–247, 363
Secure attachment, 108–109, 111–116, 144–145
Secure base, 155, 354
Secure internal working models of attachment, 43
Seductiveness, 176, 360. *See also* Sexual behavior and issues
Self functioning. *See also* Functioning
assessment and, 213–215
combined narcissistic and borderline pathology (NPD/BPD) and, 329–331
development of normal and pathological narcissism and, 48
functional impairments associated with NPD and, 30–31
later stages of therapy and, 287–290
NPD and personality organization and, 16–18
overview, 139–141, 412
pathological grandiose self and, 22–24, 73–84, 74*f*
Structured Interview of Personality Organization—Revised (STIPO-R) and, 201–202
Self regulation, 69–70
Self representations, 47–48
Self-aggrandizement
borderline personality organization and, 57
development of normal and pathological narcissism and, 51
grandiose presentation of NPD and, 60–61
high-functioning NPD and, 54
interpersonal functioning and, 126
pathological grandiose self and, 80–81
Self-assessment, 26
Self-centeredness, 17, 203
Self-conscious emotions, 47–48
Self-criticism, 70–73
Self-definition, 16–17
Self-destructiveness
case examples, 368–391
combined narcissistic and borderline pathology (NPD/BPD) and, 330–331
malignant-level of functioning and, 59
technical neutrality and, 161–162
Self–devalued other internal representation, 263–265, 413
Self-directed aggression, 53*f*, 360. *See also* Aggression
Self-direction, 56, 58, 60. *See also* Goals
Self-doubt, 24, 69–70
Self-effacement, 61
Self-esteem regulation
adolescents and, 106
borderline personality organization and, 58
childhood maltreatment and, 104
children who present as narcissistic and, 100
development of normal and pathological narcissism and, 50, 51
grandiose presentation of NPD and, 61
high-functioning level of personality organization and, 19–20
high-functioning NPD and, 55
malignant narcissism and, 364–365
NPD and personality organization and, 16–17
overview, 91–94
parent–child interactions and, 98–99
pathological grandiose self and, 79–80
relationship functioning and, 123
spectrum of narcissistic pathology and, 53*f*
Structured Interview of Personality Organization—Revised (STIPO-R) and, 203
vulnerable presentation of NPD and, 61
Self-importance
combined narcissistic and borderline pathology (NPD/BPD) and, 329
comparing normal/adaptive with pathological narcissism and, 8
grandiose presentation of NPD and, 24, 61
pathological grandiose self and, 79–81
Self-injury, 59, 238–239, 361, 368–391
Self-loathing, 70–71
Self–object affect dyads
development of normal and pathological narcissism and, 47–52
early stages of therapy and, 261–262

internalized object relations and, 41–42
overview, 144–145
Self–other affect dyads, 42, 51–52
Self-other representations. *See* Representations of self and others
Self-perception, 330
Self-presentation. *See* Presentation of NPD
Self-psychology, 2
Self-punishment, 72–73
Self-regard, 70–71, 126
Self-regulatory model of pathological narcissism, 91–92
Self-reliance, 44–45, 220–221
Self-representation, 24, 163–164, 404–405. *See also* Representations of self and others
Self-states
countertransference and, 179
early stages of therapy and, 152
interpretive process in TFP-N and, 165–166
levels of personality organization and, 20
overview, 412
suicidality and, 93
Self-sufficiency
adolescents and, 106
interpretive process in TFP-N and, 168
narcissistic resistances and, 256
suicidality and, 93
Sense of self, 201–202
Sensitivity to rejection. *See* Rejection sensitivity
Session frequency, 231–233
Sexual abuse, 104–106, 329, 368. *See also* Early experiences
Sexual behavior and issues. *See also* Love relations
interpretive process in TFP-N and, 176, 177–178
love relations and, 399–400
malignant narcissism and, 363–364
narcissistic–histrionic presentation and, 71
oedipal issues and, 319–320
preoedipal and oedipal conflicts and, 405–406
resistances and, 238–239
technical neutrality and, 161
Shame
borderline personality organization and, 58
comparing normal/adaptive with pathological narcissism and, 8
integration and, 42
overview, 92
suicidality and, 93
vulnerable presentation of NPD and, 61
Shedler–Western Assessment Procedure–II (SWAP-II), 61–62, 217–218
Short-term dynamic therapy, 34
Social anxiety, 100
Social cognition, 46*f*
Social dominance strivings. *See also* Dominance of others
grandiose/vulnerable distinction of NPD and, 24, 60–61, 68
malignant narcissism and, 364–366
pathological grandiose self and, 80–81
Social functioning, 75–76, 140–141, 220–221. *See also* Interpersonal functioning
Social inhibition, 126. *See also* Inhibition
Social media, 5, 101
Social rejection, 126–127. *See also* Rejection sensitivity
Society factors, 76–77, 414–415
Sociocultural factors, 5, 45, 46*f*, 100–101
Sources of assessment information, 199–200, 225–226. *See also* Assessment
Specialness, sense of, 61, 79–81, 152
Spectrum of narcissistic pathology. *See also* Borderline functioning level; Higher functioning level; Malignant-level of functioning
borderline personality organization and, 56–59
high-functioning NPD, 54–56
malignant-level of functioning and, 59–60
structural features of pathological narcissism and, 53, 53*f*
Split treatments, 194
Splitting mechanisms
case examples, 90
combined narcissistic and borderline pathology (NPD/BPD) and, 326, 349–350, 353
countertransference and, 179
early stages of therapy and, 152, 254
internalized object relations and, 41–42
interpretive process in TFP-N and, 169, 172–173
later stages of therapy and, 156
malignant narcissism and, 377, 380
overview, 85, 88
spectrum of narcissistic pathology and, 53*f*
structural features of pathological narcissism and, 52
Spontaneity, 260–261
Stability, 400

Stages of therapy, 147, 149–158, 185–192, 322, 413–414. *See also* Early stage of therapy; Late stage of therapy; Middle stage of therapy; Pretreatment assessment and contracting stage; Termination stage; TFP for pathological narcissism (TFP-N)
Standards, 17
States of mind, 79–80, 116, 354
STIPO-R, 200–205, 218
Strange Situation, 111
Strengths, 220–221
Stress-diathesis model, 100–101
Structural diagnosis, 197–200. *See also* Diagnosis
Structural interview. *See also* Assessment
  case examples, 205–212
  differential diagnosis and, 215–216
  overview, 198–200
  pretreatment assessment and contracting stage of therapy and, 147–148
  Structured Interview of Personality Organization—Revised (STIPO-R), 200–205
Structured Clinical Interview for DSM-IV Personality Disorders (SCID-II), 133
Structured Interview of Personality Organization—Revised (STIPO-R), 200–205, 218
Submission, 126
Substance abuse, 27, 193, 238–239
Success, illusions of, 79–80, 99
Suicidality
  case examples, 14–16, 368–391
  combined narcissistic and borderline pathology (NPD/BPD) and, 325–326, 330–331, 338–339, 349
  grandiose/vulnerable distinction of NPD and, 69–70
  malignant narcissism and, 360, 366
  narcissistic depression presentation and, 73
  overview, 238*n*
  resistances and, 238–239
  self-esteem regulation and, 91, 93–94
Superego functioning. *See also* Ego functioning
  children who present as narcissistic and, 99
  high-functioning NPD and, 54
  overview, 137–138
  technical neutrality and, 161
Superficiality, 399
Superiority
  borderline personality organization and, 56
  love relations and, 399
  malignant narcissism and, 359, 362–363
  narcissistic resistances and, 258
  NPD and personality organization and, 17
  parent–child interactions and, 98–99
  suicidality and, 93
Supportive psychodynamic therapy, 34

Technical neutrality, 160–162
Temperamental factors, 100–103
  internalized object relations and, 46*f*
Termination stage. *See also* Stages of therapy; TFP for pathological narcissism (TFP-N)
  depressive anxieties and, 313
  malignant narcissism and, 390–391
  overview, 157–158, 320–322
  transference and countertransference patterns and, 182, 183*t*
TFP for pathological narcissism (TFP-N). *See also* Early stage of therapy; Interpretation in TFP-N; Late stage of therapy; Middle stage of therapy; Pretreatment assessment and contracting stage; Termination stage; Transference-focused psychotherapy (TFP)
  attachment and mentalization and, 120–122
  combined narcissistic and borderline pathology (NPD/BPD) and, 331–353
  combining with other interventions, 193–194
  frequency of sessions, 231–233
  justifying the use of to the patient, 221–222
  malignant narcissism and, 361–362, 367–368, 390–392
  overview, 31–34, 42, 142–146, 194, 322–324, 411–415
  sharing diagnostic impressions with patients, 226–229
  stages of therapy, 147–158
  techniques of, 158–165
Theory of mind, 130
Therapeutic relationship. *See also* Boundaries with narcissistic patients; Collaboration; Countertransference; Transference; Treatment alliance
  combined narcissistic and borderline pathology (NPD/BPD) and, 353
  dissolution of the grandiose self and, 297–300
  early stages of therapy and, 149–150
  interpretive process in TFP-N and, 192–193
  later stages of therapy and, 289

malignant narcissism and, 360, 362–363, 388, 390
middle stage of therapy and, 153–155
narcissistic defenses and, 88–89
neo-Kleinian and relational approaches and, 172–174
oedipal issues and, 316–320
overview, 192–193, 412
pretreatment assessment and contracting stage of therapy and, 148
responding to resistances and, 258–261
technical neutrality and, 160–162
treatment contract and, 225
Therapist responsibilities and roles in treatment. *See also* Roles and responsibilities in treatment
roles and responsibilities of the therapist, 229*t*, 235–238
Thick-skinned presentation of NPD. *See* Grandiose presentation of NPD
Thin-skinned presentation of NPD. *See* Vulnerable presentation of NPD
Threat sensitivity, 353
Threats to treatment, 238–240, 240*t*
Totalitarian regimes, 366. *See also* Political leadership
Transcendence, 408–409
Transference. *See also* Role reversals; Therapeutic relationship
aggression in, 300–301
attachment and mentalization and, 109–110, 122
borderline personality organization and, 57
case examples, 184–192, 271–280, 281–284, 292–297, 298–300, 334–353
combined narcissistic and borderline pathology (NPD/BPD) and, 331, 334–353
early stages of therapy and, 254
free association and, 159
integration and reflection and, 305–320
interpretive process in TFP-N and, 163–164, 169–172, 174–179, 178*t*, 182–184, 183*t*, 267
later stages of therapy and, 157, 300–301
love relations and, 404–405
malignant narcissism and, 362, 368, 374–377, 379, 383–384, 387
middle stage of therapy and, 153–155
neo-Kleinian and relational approaches and, 172–174
oedipal issues and, 316–320
overview, 142–144, 413–414
relationship functioning and, 125–126
research regarding, 182–184
technical neutrality and, 160–162
termination and, 320–321
Transference-focused psychotherapy (TFP). *See also* Object relations approaches; TFP for pathological narcissism (TFP-N)
attachment and, 109–110
narcissistic defenses and, 88–89
overview, 9–11, 31–34
research support for, 32–34
Transient narcissistic symptoms (TANS), 105
Traumatic experiences. *See also* Early experiences
attachment and mentalization and, 111–112, 121
combined narcissistic and borderline pathology (NPD/BPD) and, 329
development of pathological narcissism and, 104–106
malignant narcissism and, 364–365, 368
malignant-level of functioning and, 59
vulnerable presentation of NPD and, 65
Treatment
challenges common in, 16
change in attachment and mentalization and, 119–122
combining TFP-N with other interventions, 193–194
functional impairments associated with NPD and, 30–31
narcissistic defenses and, 88–89
object relations approach, 9–11
overview, 35–36
stages of therapy, 147–158
Treatment alliance, 219–220. *See also* Therapeutic relationship
Treatment contract. *See also* Pretreatment assessment and contracting stage
addressing secondary gains in, 241–247
case examples, 32–34, 223–224, 246–247, 248–249
contents of, 229–240, 229*t*, 240*t*
countertransference and, 230
fees, 233–234
frequency of sessions, 231–233
functions of, 222–225, 222*t*
justifying the use of TFP-N to the patient and, 221–222
lower level of borderline organization and, 247–249
malignant narcissism and, 384
moving from to therapy, 249
overview, 219–221, 250
patient's acceptance of, 240–249
pretreatment assessment and contracting stage of therapy and, 148–149

Treatment contract *(cont.)*
returning to through the course of therapy, 249–250
roles and responsibilities of the patient, 229*t*, 230–231, 234–235
roles and responsibilities of the therapist, 229*t*, 235–238
sharing diagnostic impressions with patients, 226–229
Treatment frame, 222–225, 222*t*, 362
Treatment goals. *See also* Goals
early stages of therapy and, 151
functions of the treatment contract and, 222–225, 222*t*
later stages of therapy and, 287
treatment contract and, 220
Triadic mental representations, 49. *See also* Mental representations
Triangulation
love relations and, 398–399, 400–401, 405–406
malignant narcissism and, 389–390
oedipal issues and, 316–320
Trust, 401–402

Unconscious processes
countertransference and, 179
interpretive process in TFP-N and, 162–165, 254
love relations and, 395–397
technical neutrality and, 161
Unintegrated self, 73–74
Unworthiness, sense of, 24

Valuations of others, 17, 100. *See also* Devaluation
Vengeful destructiveness, 53*f*
Verbal abuse, 104–106
Violence, 362, 365, 366. *See also* Aggression
Vulnerability. *See also* Vulnerable presentation of NPD
attachment and, 115–116
challenges related to diagnosis and treatment and, 16
levels of personality organization and, 20
middle stage of therapy and, 155
overview, 412
Vulnerable presentation of NPD. *See also* Presentation of NPD; Vulnerability
attachment and, 115–116
case examples, 62–65, 67
childhood maltreatment and, 104–105
combined narcissistic and borderline pathology (NPD/BPD) and, 325–326, 329
development of pathological narcissism and, 101–103
early stages of therapy and, 152, 263, 265
interpersonal functioning and, 124–126
later stages of therapy and, 301–303
malignant narcissism and, 362–363, 366–370
middle stage of therapy and, 153
oscillation between grandiose presentation and, 301–303
overview, 24–25, 60–70, 94–95, 140
research support for the distinction of, 67–70
suicidality and, 94
trauma and, 105

Weakness, 69–70
Working through
depressive anxieties and, 314–315
dissolution of the grandiose self and, 297–300
later stages of therapy and, 297–300, 308
oedipal issues and, 316–320